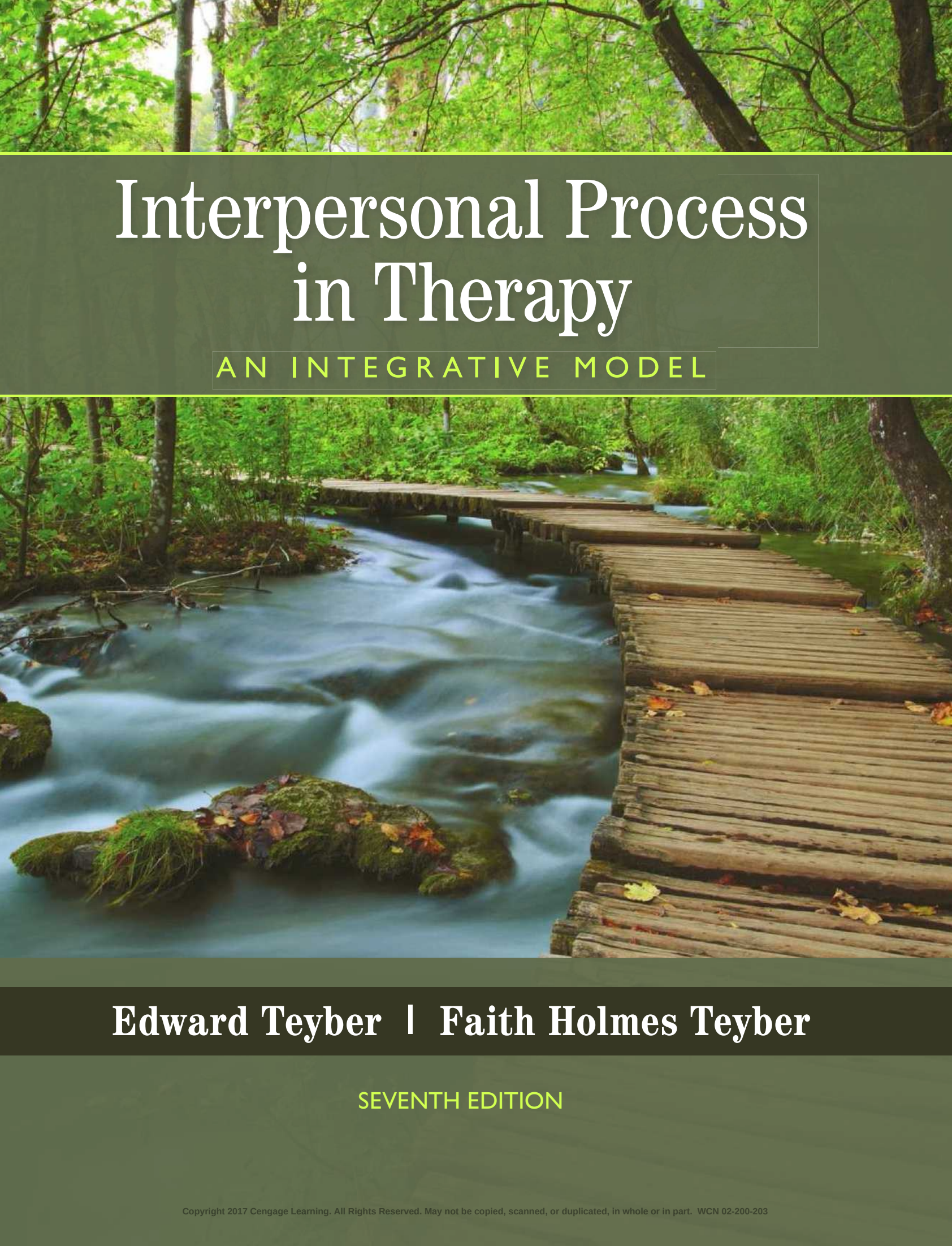




# Interpersonal Process in Therapy

AN INTEGRATIVE MODEL

**Edward Teyber | Faith Holmes Teyber**

SEVENTH EDITION

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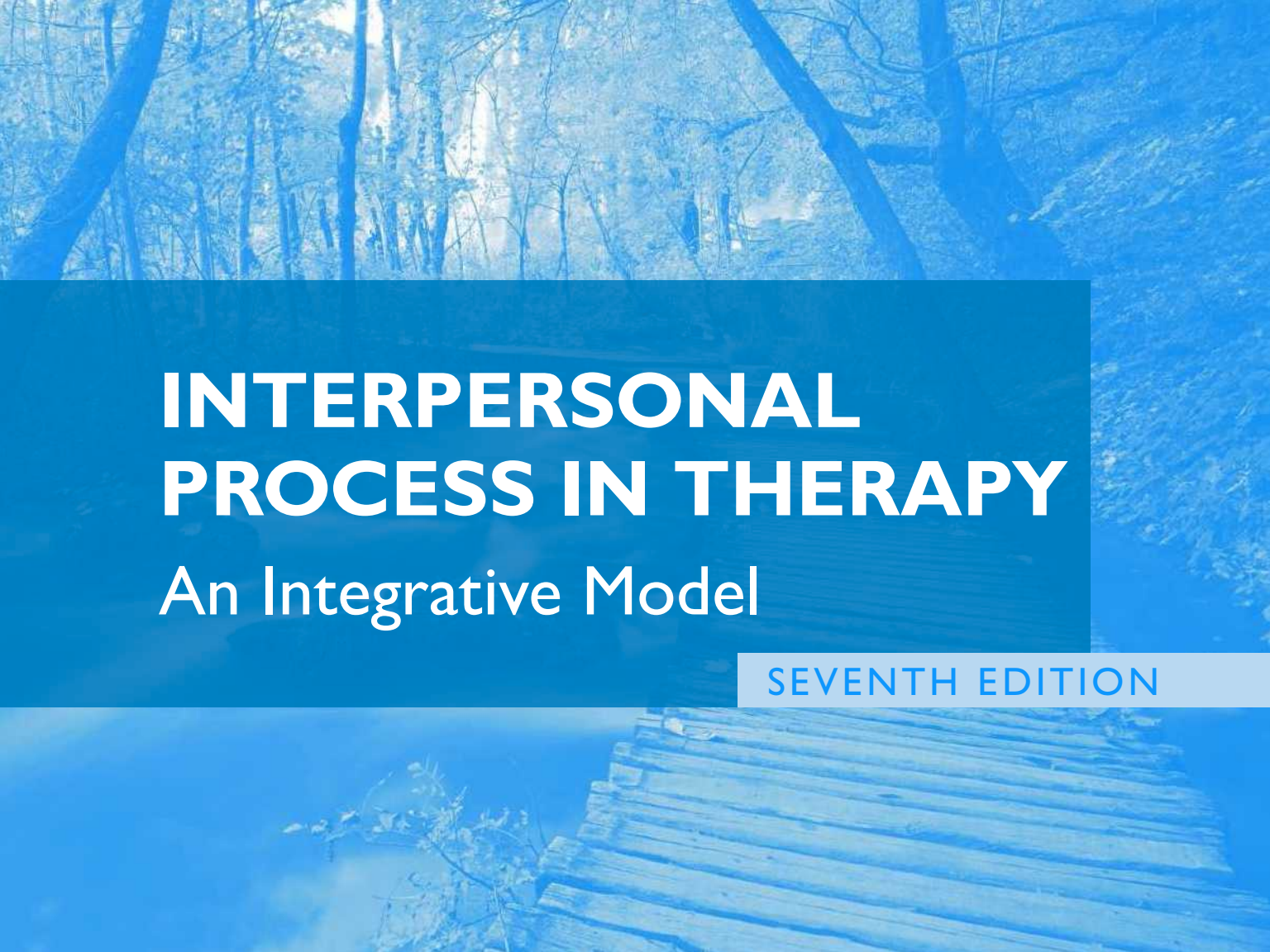
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# INTERPERSONAL PROCESS IN THERAPY

## An Integrative Model

SEVENTH EDITION

**Edward Teyber**

**Faith Holmes Teyber**

*California State University,  
San Bernardino*



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*Dedicated to those who are struggling to change;  
To our children, who have strengthened our  
commitment to making a difference;  
And to all those who have come before  
and taught about the power of relationships to  
bring about change.*







# CONTENTS

PREFACE xi

ACKNOWLEDGMENTS xv

ABOUT THE AUTHORS xvii

## PART 1

### INTRODUCTION AND OVERVIEW 1

#### CHAPTER 1

##### The Interpersonal Process Approach 3

Therapists-in-Training Struggle with Performance Anxieties 4

Therapists are More Effective When They Have a Treatment Focus 7

Core Concepts 8

*The Process Dimension 8*

*The Corrective Emotional Experience 10*

*Client Response Specificity 15*

Teresa: Case Illustration of Core Concepts 19

Theoretical and Historical Context 22

*The Interpersonal Domain 23*

*The Cognitive Domain 24*

*The Familial/Cultural Domain 28*

Closing 33

## PART 2

### RESPONDING TO CLIENTS 35

#### CHAPTER 2

##### Establishing a Working Alliance 37

Conceptual Overview 37

The Working Alliance 38

*Collaboration: An Alternative to Directive and Non-Directive Styles 39*

*Collaboration Begins with the Initial Interview 43*

|                                                                        |    |
|------------------------------------------------------------------------|----|
| Empathic Understanding: The Foundation for a Working Alliance          | 48 |
| <i>Ways in Which Clients Do Not Feel Understood</i>                    | 50 |
| <i>Communicate Understanding Overtly and Specifically</i>              | 54 |
| <i>Identifying Recurrent Themes Facilitates Empathic Understanding</i> | 60 |
| Performance Anxieties Make It Harder to Establish a Working Alliance   | 65 |
| Empathic Understanding as a Precondition of Change                     | 66 |
| Closing                                                                | 67 |

## CHAPTER 3

**Honoring the Client's Resistance** 69

|                                                                   |     |
|-------------------------------------------------------------------|-----|
| Conceptual Overview                                               | 69  |
| Reluctance to Address Resistance                                  | 71  |
| <i>The Therapist's Reluctance to Work with Resistance</i>         | 72  |
| <i>The Client's Reluctance to Work with Resistance</i>            | 75  |
| Identifying Resistance                                            | 80  |
| Formulate Working Hypotheses to Clarify Resistance                | 81  |
| Responding to Resistance                                          | 83  |
| <i>Addressing Resistance During the Initial Telephone Contact</i> | 84  |
| <i>Exploring Resistance at the End of the First Session</i>       | 88  |
| <i>Resistance During Subsequent Sessions</i>                      | 95  |
| Shame Fuels Resistance                                            | 102 |
| <i>Shame Versus Guilt</i>                                         | 103 |
| <i>Shame-Prone Sense of Self</i>                                  | 104 |
| <i>Shame-Rage Cycle</i>                                           | 104 |
| <i>Shame-Anxiety</i>                                              | 105 |
| Success in Treatment Can Be Threatening                           | 107 |
| Closing                                                           | 108 |

## CHAPTER 4

**An Internal Focus for Change** 110

|                                                             |     |
|-------------------------------------------------------------|-----|
| Conceptual Overview                                         | 110 |
| Helping Clients Focus Inward on Their Own Experience        | 112 |
| <i>Therapy Intensifies When Clients Focus Inward</i>        | 116 |
| Reluctance to Adopt an Internal Focus                       | 118 |
| <i>Therapists' Reluctance</i>                               | 118 |
| <i>Points of Entry</i>                                      | 120 |
| <i>Clients' Reluctance</i>                                  | 122 |
| Developing Agency: Placing the Locus of Change with Clients | 125 |
| <i>Fostering the Client's Initiative</i>                    | 125 |
| Enlist Clients in Solving Their Own Problems                | 132 |
| <i>Resolving Client's Conflict</i>                          | 133 |



|                                                         |     |
|---------------------------------------------------------|-----|
| Tracking Clients' Anxiety                               | 136 |
| <i>Identify Signs of Client's Anxiety</i>               | 137 |
| <i>Approach Client's Anxiety Directly</i>               | 138 |
| <i>Observe What Precipitates Clients' Anxiety</i>       | 139 |
| <i>Focusing Clients Inward to Explore Their Anxiety</i> | 140 |
| Closing                                                 | 142 |

## CHAPTER 5

### Helping Clients with Their Feelings 143

|                                                                                   |     |
|-----------------------------------------------------------------------------------|-----|
| Conceptual Overview                                                               | 144 |
| Responding to Clients' Feelings                                                   | 144 |
| <i>Work with Clients about Addressing Their Ambivalence</i>                       | 144 |
| <i>Clients Avoid Feelings Because of Unwanted Interpersonal Consequences</i>      | 145 |
| <i>Approach the Client's Most Salient Feeling</i>                                 | 148 |
| <i>Expand and Elaborate the Client's Affect</i>                                   | 152 |
| Identify the Predominant Affect                                                   | 157 |
| <i>An Old Wound</i>                                                               | 158 |
| <i>Multiple Stressors</i>                                                         | 158 |
| <i>A Characterological Affect</i>                                                 | 159 |
| Recognize the Constellation of Feelings That Clients Frequently Present           | 161 |
| <i>Anger-Sadness-Shame</i>                                                        | 162 |
| <i>Sadness-Anger-Guilt</i>                                                        | 166 |
| Attachment Theory Provides Helpful Guidelines for Responding to Clients' Distress | 167 |
| <i>Containment: Using Attachment Constructs to Guide Intervention</i>             | 170 |
| <i>A Safe Haven Facilitates Change from the Inside</i>                            | 174 |
| <i>The Client's Feelings Often Evoke Countertransference</i>                      | 175 |
| <i>Familial Rules and Roles Shape Countertransference Propensities</i>            | 175 |
| <i>Differentiating the Therapist's Issues from the Client's</i>                   | 178 |
| Closing                                                                           | 179 |

## PART 3

### CLARIFYING THE CLIENT'S PROBLEM AND DEVELOPING A TREATMENT FOCUS 181

## CHAPTER 6

### Familial and Developmental Factors 183

|                                                               |     |
|---------------------------------------------------------------|-----|
| Conceptual Overview                                           | 183 |
| Attachment Style and Clinical Presentation in Adult Treatment | 184 |
| <i>Four Categories of Attachment</i>                          | 185 |
| <i>Secure Attachment Style in Adult Clients</i>               | 186 |

|                                                                                                   |            |
|---------------------------------------------------------------------------------------------------|------------|
| <i>Dismissive Attachment Style in Adult Clients</i>                                               |            |
| <i>Preoccupied Attachment Style in Adult Clients</i>                                              | 193        |
| <i>Fearful Attachment Style in Adult Clients</i>                                                  | 197        |
| <i>Concluding Thoughts on Adult Attachment Styles</i>                                             | 200        |
| Parenting Styles                                                                                  | 200        |
| <i>Authoritarian Parenting</i>                                                                    | 201        |
| <i>Permissive Parenting</i>                                                                       | 203        |
| <i>Disengaged Parenting</i>                                                                       | 205        |
| <i>Authoritative Parenting</i>                                                                    | 207        |
| Love Withdrawal and Conditions of Worth                                                           | 208        |
| Guidelines for Responding to Clients with a Diversity of Parenting and Attachment Styles          | 212        |
| <i>Additional Clinical Guidelines</i>                                                             | 214        |
| Family Interaction Patterns                                                                       | 217        |
| Closing                                                                                           | 222        |
| <br>CHAPTER 7                                                                                     |            |
| <b>Inflexible Interpersonal Coping Strategies</b>                                                 | <b>224</b> |
| Conceptual Overview                                                                               | 224        |
| Interpersonal Framework for Conceptualizing Clients                                               | 225        |
| Unmet Developmental Needs                                                                         | 227        |
| <i>Internal Working Models and Interpersonal Coping Strategies</i>                                | 228        |
| <i>Internal Working Models Shape Symptoms and Problems</i>                                        | 228        |
| Interpersonal Coping Strategies                                                                   | 230        |
| <i>Inflexible Interpersonal Coping Strategies: Moving-Toward, Moving-Against, and Moving-Away</i> | 231        |
| <i>Rigid Interpersonal Coping Styles: A Defense Framed as a Virtue</i>                            | 232        |
| <i>Shoulds for the Self and Expectations of Others</i>                                            | 234        |
| Resolving the Core Conflict                                                                       | 235        |
| Case Study of Peter: Moving-Toward Others                                                         | 238        |
| <i>Developmental History and Precipitating Crisis</i>                                             | 238        |
| <i>Precipitating Crisis, Maladaptive Relational Patterns, and Symptom Development</i>             | 240        |
| <i>Course of Treatment</i>                                                                        | 241        |
| Two Case Summaries                                                                                | 244        |
| <i>Juan: Moving-Against Others</i>                                                                | 244        |
| <i>Maggie: Moving-Away from Others</i>                                                            | 247        |
| Closing                                                                                           | 251        |

## CHAPTER 8

**Relational Themes and Reparative Experiences 253**

Conceptual Overview 253

Three Ways Clients Reenact Their Problems with Others  
in the Therapeutic Relationship 253*Eliciting Maneuvers* 254*Clients' Testing Behavior* 261*Transference Reactions* 272

Finding Interpersonal Balance 280

*Enmeshment* 281*Disengagement* 282*Effective Middle Ground of Balanced Involvement* 284

Ambivalence: Responding to Both Sides of Clients' Experience 288

Closing 291

## PART IV

**RESOLUTION AND CHANGE 293**

## CHAPTER 9

**Working With the Process Dimension 295**

Conceptual Overview 295

Resolving Problems through the Interpersonal Process 296

*Responding to Clients' Conflicts in the  
Therapeutic Relationship* 296*Bringing Conflicts to the Therapeutic Relationship* 297*Using the Process Dimension to Facilitate Change* 299*Using Process Comments to Provide an Interpersonal Solution* 313

Therapists' Initial Reluctance to Work with the Process Dimension 323

Closing 332

## CHAPTER 10

**Working-Through and Termination 333**

Conceptual Overview 333

The Course of Client Change: An Overview 333

Working-Through 336

*The Working-Through Process* 336*Family-of-Origin Work* 345*The Dream: The Future* 351

|                                                       |     |
|-------------------------------------------------------|-----|
| Termination                                           | 354 |
| <i>Client Ends Treatment Early</i>                    | 354 |
| <i>Client and Therapist Talk about Therapy Ending</i> | 357 |
| <i>Ending the Relationship</i>                        | 363 |
| Closing                                               | 363 |
| APPENDIX A                                            | 365 |
| APPENDIX B                                            | 367 |
| GLOSSARY                                              | 369 |
| REFERENCES                                            | 377 |
| NAME INDEX                                            | 396 |
| SUBJECT INDEX                                         | 405 |





# PREFACE

## WHO ARE WE AND WHAT IS OUR PURPOSE?

We are both practicing therapists and clinical instructors who feel so fortunate to have spent our careers doing this work. Throughout the 80 collective years we have been practicing, we continue to find this work deeply meaningful. We have spent our careers involved in every aspect of clinical work: seeing clients, supervising graduate student therapists, directing treatment/training clinics, conducting psychotherapy process research, and carrying out empirically validated intervention projects. As we have grown older, we hold even more respect for the clients we see—often feeling genuine admiration for many who are coping and living as well as they are despite having grown up with so little help for the very big problems they are still contending with.

We find that the problems most clients bring to treatment are not simple or superficial but often complex, painful, and not so easily resolved. However, with helpful training and supervision, student therapists can learn much about how to help and make a meaningful difference in their clients' lives. We have been privileged to have taught and supervised so many rewarding graduate students, most of whom have gone on to become practitioners, teachers, and clinical supervisors. We love this work, feel honored to have had the privilege of entering deeply into our clients' lives, and believe that therapists help clients change through the reparative relationships they offer.

## WHO IS THIS BOOK FOR?

*Interpersonal Process in Therapy: An Integrative Model, 7/e*, is a clinical training text for those who are seeing clients: graduate student therapists in practicum, internship and other applied practice courses. It is also suitable for advanced students in upper division and pre-practicum courses that provide an in-depth, applied introduction to counseling and therapy. Foundational clinical concepts are introduced throughout each chapter and illustrated with numerous clinical vignettes and sample therapist-client dialogues that student therapists will find informative and compelling. Clinically authentic and personally engaging, this text will help developing therapists understand the therapeutic process and how change occurs.

Clinical training is stressful for many new therapists because they are painfully uncertain of what to do and how to proceed with their clients. Although they take helpful courses on counseling theories, helping/micro skills, and psychopathology, student therapists need more specific help as they approach their first clients in session. Fully cognizant of their limited experience and knowledge, and sometimes receiving contradictory input from different supervisors and practicum instructors, these trainees are often aware that they do not really know what to do or how to help their clients. Although bright and caring, many feel inadequate and worry about making mistakes or

doing something wrong that could hurt their clients. New therapists need a conceptual framework to help them understand where they are trying to go in treatment, and why, in order to help their clients change. One of the primary goals of this foundational text is to replace the ambiguity that students often have about treatment with a well-developed framework for understanding how change occurs and what role the therapist-client relationship plays in the change process.

## WHAT IS THIS BOOK ABOUT?

We believe that the relationship between the therapist and the client is the foundation for therapeutic change. Thus, this text offers an integrative treatment approach that draws from different theoretical approaches and teaches student therapists how they can use the therapeutic relationship as a focus for understanding and intervening with their clients. In order to help, developing therapists need guidance learning how to make sense of the complex interactions taking place in the therapeutic relationship, and to find effective ways to talk with clients about what may be transpiring between them. With a better understanding of what is going on between the therapist and the client, the therapist is able to intervene effectively by providing clients with a new and reparative relationship that disconfirms clients' early maladaptive schemas and maladaptive relational patterns. With this *in vivo* or experiential relearning, we are teaching student therapists how to understand and intervene in the current interaction or interpersonal process that is occurring between the therapist and client to provide a corrective emotional experience.

Specifically, this text aims to help developing therapists learn how to use process comments and other immediacy interventions, such as metacommunication, self-involving statements, and interpersonal feedback to intervene in the here-and-now, current interaction with clients. Written in a direct and conversational tone, this text provides an integrative treatment approach that highlights how new therapists can use the process dimension—talking sensitively yet forthrightly about “you and me” and what may be going on between the therapist and client, right now, to facilitate change.

The model teaches readers to (1) identify significant relational themes and cognitive patterns in the client's behavior; (2) help the client recognize how these patterns function both for better and for worse in the client's life—that is, what they provide and what they cost; (3) change how these maladaptive relational patterns and outdated coping strategies that are causing problems with others are also coming into play during sessions in the real-life relationship between the client and therapist; and (4) generalize the *in vivo* relearning from this experience of change with the therapist to others beyond the therapy setting. This treatment model demonstrates a way of being with clients in session that is genuine, actively engaging but not directive, empathic, and highly collaborative. It encourages therapists to explore with clients what is going on between them so they can work together to change faulty patterns that are being enacted with the therapist and others, and provide the experiential relearning that will help clients change.

Describing the course of treatment from the initial session through termination, this text provides practical intervention guidelines for working with the primary issues that emerge during different stages of treatment, and shows how theory leads to practice. With clarity and immediacy, it highlights the challenging clinical situations that new therapists are facing, and captures the questions and concerns that are salient for student therapists as they begin seeing clients. The interpersonal process model is an integrative approach that incorporates client-centered, cognitive-behavioral, family systems, interpersonal-dynamic, and attachment theories; uses the therapeutic relationship as an organizing focus for treatment; and encourages student therapists to draw flexibly from varying theoretical perspectives and develop their own personal styles. This text presents a comprehensive treatment approach

that teaches student therapists how to use themselves, and the relationships they establish with their clients, as the most important way to help clients change.

## WHAT'S NEW IN THIS SEVENTH EDITION?

The goal for improving this foundational text on clinical training has always been the same: to make it simpler, clearer, and more practical for new therapists. Because this material is clinically authentic and so personally evocative for student therapists, we have tried to make the Seventh Edition more concise. Every chapter is shorter. However, we also wanted to develop more fully certain core concepts that run throughout the text, especially empathic understanding, working collaboratively, rupture and repair in the working alliance, and further guidelines for engaging clients in more depth. The Seventh Edition has thoroughly integrated the burgeoning literature linking attachment theory to clinical practice, especially adult attachment styles and clinical presentation. There is further attention to the trans-diagnostic role of shame in many clients' symptoms and problems, and to its role in resistance and premature termination.

This new edition also provides more attention to case formulation and treatment planning, and empirical support for treatment guidelines. A revised and expanded reference section provides improved access both to classic and current clinical and research literature. Finally, it is challenging for new therapists to work “in the moment” with clients and try out the process-oriented interventions that can be so helpful yet intimidating in the beginning. To assist with these immediacy interventions and other core concepts, many new therapist-client dialogues and case vignettes have been added in every chapter to illustrate effective and ineffective intervention. In conjunction with new video demonstrations that accompany the Seventh Edition, these illustrative dialogues will help trainees “see” concepts and apply them to practice. We hope this will help trainees become more effective clinicians as they sort through their own identities as therapists and choose their own personal styles for working with clients.

## SUPPLEMENTAL MATERIALS

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In addition to the benefits of the platform, MindTap for *Interpersonal Process in Therapy: An Integrative Model, 7th Edition* also includes self-assessments, case studies, practice exercises, a chapter quiz, and video demonstrations by the authors. These demonstrations show both authors applying core concepts from the text in clinical situations. The videos focus on critical practice issues discussed in the text that are often challenging for new therapists.

## INSTRUCTOR SUPPLEMENTS

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**Online PowerPoint.** These vibrant Microsoft® PowerPoint® lecture slides for each chapter assist instructors with their lectures by providing concept coverage directly from the textbook.



## ACKNOWLEDGMENTS

I (Ed) started percolating on this text 40 years ago while working at the Student Counseling Center at Michigan State University. I have learned much from good mentors and been fortunate to continue finding wise friends and colleagues over the years who keep teaching me about counseling relationships and helping clients change. Most importantly, I would like to thank my wife, Faith Holmes Teyber, for teaching me so much over the years—usually by her example rather than words. She joined me as a co-author for the Sixth Edition of *Interpersonal Process in Therapy*, and has enriched this Seventh Edition with her understanding and compassion. She is a gifted therapist and supervisor, and a uniquely loving person who offers every child a trustworthy friend. I also wish to thank my family and three loving sisters for their lifetime of support—oldest sister Julie typed my dissertation for me in 1976 and, still typing, helped with this manuscript in 2016! I am especially thankful for two caring and creative sons, Ted and Reed—I am immensely proud of the person each has become. Finally, I wish to thank the clients I have worked with over the years. They have honored me by taking the risk of letting me become someone who mattered to them, and thereby giving me the simple pleasure of being able to help.

I (Faith) have been privileged to have the support of a nurturing family, loyal friends, and affirming mentors. My friends, especially Laura Kamptner, have provided both personal and intellectual fuel over the years. I am deeply grateful for the partnership I have with Ed Teyber—he is a master therapist and provides tremendous safety to those who have the pleasure of being in his circle. I am especially thankful to my sons, Andrew and Cameron, who have taught me so much about the power of relationships. I am also thankful to the many children and families who have given me the honor of letting me into their lives and allowing me to help them with their problems. I have been fortunate to find this meaningful work.

Together, we would like to thank our friend and colleague, Dr. Rayna Markin at Villanova University for her significant help in making this edition better. We would also like to thank four helpful graduate students, Nicoletta Dragan, Cadisha Gravesande, Chelsea Lamb, and Jenn Rizk, who have offered us skillful editing with this revision and helped us stay close to the experience of the beginning therapist. We also would like to thank Chris Vallejo for his help with videotaping. We appreciate how much they, and many other rewarding graduate students over the years, have given us. It will be a great pleasure to watch and share in all they are going to accomplish and offer to others in the years ahead. Additionally, we would like to thank the reviewers who made many helpful suggestions for this revision: Jeffrey Bulanda, Aurora University; Arthur Clark, St. Lawrence University; Matthew Domico, Lewis University; Jeffrey Cornelius-White, Missouri State University; Randi Smith, Metro State University of Denver; Lorraine Guth, Indiana University of Pennsylvania; Mary Houston-Vega, University of Texas at San Antonio; and Michael Chovanec, St. Catherine University/University of St. Thomas.

Finally, thanks to our many good colleagues over the years at Cengage. In particular, we would like to thank Julie Martinez and Elizabeth Momb for their support and partnership for this edition, as well as Kailash Rawat at Lumina Datamatics.







## ABOUT THE AUTHORS

**Edward Teyber** is Professor of Psychology Emeritus, director of the Psychology Clinic, and president of the Foundation Board of Directors at California State University, San Bernardino. He received his Ph.D. in clinical psychology from Michigan State University. Dr. Teyber is also the author of the popular book *Helping Children Cope with Divorce*, and co-author with Dr. Faith H. McClure (now Teyber) of *Casebook in Child and Adolescent Treatment: Cultural and Familial Contexts*. His work has focused on the effects of marital and family relations on child adjustment, attachment and psychotherapy, and counseling relationships. He is interested in clinical training and enjoys maintaining a private practice.

**Faith Holmes Teyber** is Professor of Psychology Emeritus at California State University, San Bernardino, and provides consultation to organizations serving the mental health needs of children. She grew up in South Africa during apartheid and received her Ph.D. in clinical psychology from the University of California, Los Angeles. Her clinical and research interests are in the areas of attachment and psychotherapy and at-risk children, families, and adults, with a particular focus on factors that promote resilience in those who have experienced trauma. She is currently conducting an attachment-based, parent-child therapy intervention project with incarcerated parents. She is co-author with Dr. Edward Teyber of *Casebook in Child and Adolescent Treatment: Cultural and Familial Contexts*. Her publications are in the area of coping with stress and trauma.



PART

# I

## INTRODUCTION AND OVERVIEW

### CHAPTER ONE

The Interpersonal Process Approach







## CHAPTER 1

# The Interpersonal Process Approach

*Claire, a first-year practicum student, was about to see her first client. She had been looking forward to this event for a long time. Like many of her classmates, Claire had decided to become a therapist while working on her undergraduate degree. Counseling\* had always been intrinsically interesting to her. For Claire, being a therapist meant far more than having a “good job”; it was the fulfillment of a dream. How meaningful, she thought, to make a living by helping people with the most important concerns in their lives.*

*At this moment, though, Claire felt the real test was at hand: Her first client would be arriving in a few minutes. Worries raced through her mind: What will we talk about for 50 minutes? How should I start? What if she doesn’t show up? What if I do something wrong and she doesn’t come back? Claire also worried if her client—a 45-year-old Hispanic woman—would have difficulty relating to her, a Caucasian woman in her mid-20s. Needless to say, Claire was anxious. And even though she was painfully uncertain about how to proceed, she was still committed to finding a way to help this client with her problems. She had learned something about therapy in her undergraduate psychology classes and a good deal more from her volunteer experience with callers on the local crisis hotline. But even with these experiences and a supervisor to guide her, Claire was keenly aware of her novice status and the fact that she didn’t know very much about actually doing therapy.*

*Claire’s classmates shared her excitement about becoming a therapist. Many of them were older than Claire and more experienced in life. Some had raised children; others had already had careers as teachers, nurses, and businesspeople. These new therapists were often coming from life roles in which they had already been successful and felt confident. A counseling career held new hopes for these classmates as well but also evoked anxiety about their ability to become effective therapists. Like Claire, they knew that realizing their hopes and plans for a rewarding new career would also depend on their ability to establish their credibility with their clients in order to work effectively with them. And with the arrival of their first clients, their ability to help was about to be tested.*

---

\*The terms “counseling” and “therapy” are used interchangeably throughout the text.

## THERAPISTS-IN-TRAINING STRUGGLE WITH PERFORMANCE ANXIETIES

It is indeed really difficult to begin seeing one's first clients. Those initial sessions can be both exciting and intense for new therapists like Claire, who often struggle with performance anxieties and fears about their own inadequacy. We recall how worried we were, years ago, about making mistakes and as clinical supervisors, we often listen to bright and caring practicum students express concerns about somehow "hurting my client." If new therapists are too worried about making a mistake, or become overly preoccupied about what their supervisor might want them to say or do, they often become too quiet or passive—and the client doesn't feel heard, helped, or responded to. Similarly, when they are trying too hard to figure out what they are going to say or what they are supposed to do next, or grasp the key meaning in what they just said, therapists cannot listen intently and actively engage with the client. In this way, performance anxieties can significantly impede the treatment process. These unwanted performance pressures are generated when new therapists frame what they don't know or can't do yet as a "deficit" or as evidence of their inadequacy, rather than more realistically regarding it as just their own inexperience. A useful starting point for learning how to use the therapeutic relationship to help clients change is to gain a better understanding of these initial performance anxieties that diminish therapists' effectiveness. Let's look at some of the typical concerns practicum students express as they begin seeing their first clients.

Jessica expresses concerns that are fairly realistic for a new therapist: *"My biggest anxiety about being a student therapist is that I really don't have any idea what I am doing—and feel like I've been thrown headfirst into the water! Don't get me wrong, I am definitely enjoying the chance to start seeing clients, but it's just a little scary taking other people's problems into my own hands and trying to help them without a clue of where to go next. I'm OK with exploring things, but when it comes to the 'action stage' and actually trying to help somebody change, well, that concerns me most."*

Chanté brings a more personal concern to her initial sessions—the fear of being found out or exposed as an imposter: *"My biggest anxiety stems from a fear of failing or being incompetent—a feeling that somehow I have managed to sneak through my educational career and land in this great training program without anyone figuring out that I'm really not cut out for this. Perhaps I am just as afraid myself of realizing that I may not be very good at this work. That would be devastating for me because the more I see my clients, the more I enjoy and appreciate every aspect of this. And even though I'm aware that I'm going to make mistakes as I go along, I am afraid of doing more harm than good sometimes. So, with every new case that gets passed out, the first thing I think about is all the reasons why I may not be the right therapist to help that person."*

Carol connects how the anxiety that she experiences in her new role as a counselor links to a problematic role she played in her family. *"My biggest anxiety about becoming a therapist is feeling that I am inadequate. My instructors reassure me that this is a normal feeling, that most therapists experience this in their first year or two of training, and that we're not expected to be perfect. But it doesn't make any difference—it remains my biggest anxiety. I believe it's because I was always second best in my family of origin. No matter what I did, my sister was always smarter...more creative. I learned to feel really uncomfortable whenever I wasn't in complete command and didn't know just exactly what I was supposed to do. So, even though some part of me knows that I'm really not inadequate, it still churns my stomach when I am not good at something right away."*

Dennis relays how his performance anxieties keep him from grasping, in the moment, what his client is really saying or meaning. *"My anxiety level goes way up when I feel lost*

*in the session. Sometimes I feel so lost in there, like I'm going in circles, and I just don't know what to do. And that just makes my anxiety even worse. Then I start wracking my brain about what to do next and I start missing even more issues. It just kills me when I review the videotape after the session and then, when it's too late, I 'get' what they were really saying or think of what I could have said. But why couldn't I get it during the session, right then when it was happening...it's so frustrating!"*

As these promising student therapists are telling us, counselor training certainly is challenging. But new therapists need to be patient with themselves and appreciate that learning to be an effective therapist is a long-term developmental process (Rizq, 2009). It often takes three or four years to find one's own professional identity and feel competent in this work. Within a year or so, most new therapists do become more confident, start to feel like they can be themselves in the session, and begin to enjoy the hour as they find that they can indeed help their clients. Lorraine, a second-year student, captures this rewarding transition: *"Everything considered, I have enjoyed conducting therapy this year. Instead of being nervous in the room, like I felt last year, I feel at home now. I'm actually able to enjoy my clients instead of being caught up in my fear of failure. I'm not nearly as worried about making mistakes, and I view the misunderstandings or ruptures that occur as opportunities to develop a better relationship with my client. So, the best part of therapy this second year is that my performance anxiety has decreased exponentially. I spend a lot more time thinking about what's going on with the client and conducting therapy, and a lot less time thinking about myself and waiting to fail."*

What helps therapists like Lorraine resolve their initial performance anxieties so they can begin responding more effectively to their clients? We introduce three suggestions here, and will return to this important developmental issue throughout the chapters ahead. First, new therapists are encouraged to question the unrealistic performance expectations they often place on themselves, accepting that it is OK to make mistakes—all therapists do. Often, *this means they need to focus more on what they are learning rather than on how they are performing.* This will progress readily for some, especially those who can be non-defensive, learn from their mistakes by being open to constructive feedback from supervisors, and move on. In contrast, this will be more difficult for other practicum students who may have grown up in families where they received too much criticism from their caregivers, were expected to be perfect and could not make mistakes, or were judged too frequently. An important consideration to keep in mind is that we are human; we are all going to make mistakes with our clients. We will be most effective if we focus instead on how to recover from them and can grasp that our mistakes are not indicative of being inadequate. Join in this thought exercise:

Right now, think of the professional you regard as the single most effective therapist you have known—a particular instructor or supervisor, or perhaps a personal therapist you have worked with, whom you trust and admire. Pause now, and reflect for a moment on the reality that this esteemed and seasoned therapist makes mistakes in her or his practice every day. (If you don't think they do, then ask them.) The therapist you are thinking about probably is highly skilled, in part because s/he can recover from mistakes (the inevitable misunderstandings that occur in every relationship or "ruptures in the working alliance") by being non-defensive. That is, she/he regularly checks out potential problems by asking clients about potential misunderstandings, is willing to look at his/her own contribution to whatever problem may be occurring, and is able to talk through or sort out these misunderstandings that occur. For example:

**THERAPIST:** "You seem more distant, Jason. Maybe I said something, that doesn't feel quite right. Can I check in with you about this?"

Second, new therapists will do a better job of managing whatever anxieties they may be experiencing if they can set their intention to de-center and *focus more on the client and what the client is really saying rather than on themselves and their own performance*. Too often, new therapists are self-absorbed, which can, at times, develop to a state of painful self-consciousness or obsessive self-awareness (e.g., **Therapist**: “What should I say when she looks at me like that...what am I going to ask about next...I’m being way too quiet...I keep saying um hmm”). As new therapists develop a reflective self-awareness that is more balanced—that is, process both what is going on for the client, while simultaneously considering what might be going on between client and therapist as well—they will become more effective. We are going to explore this **participant/observer\*\*** stance closely, because it is a challenging stance that involves being in the relationship experientially while simultaneously stepping out of it and observing and thinking about what might be going on between the therapist and client. This often feels elusive in the beginning, but it is an essential therapeutic skill that we will be helping you develop.

Third, to help with these normative and expectable performance anxieties, new therapists need active support from their supervisors and instructors. In particular, they need to be reassured about the acceptable reality that most new therapists do not really know very much about the change process, what to do in their sessions, or how to help clients change. Student therapists also need practical guidelines for how to proceed and suggestions that they can apply, especially in their initial sessions. This support and preparation often includes real-life illustrations or examples such as watching videotapes of instructors conducting intakes and initial sessions, watching actual sessions of your supervisor with a client, and role-playing with instructors who model effective ways to respond and conduct initial sessions. These real-life role models are especially effective and can go a long way toward diminishing the student therapist’s initial anxieties about seeing clients and knowing “what to do.”

In sum, new therapists’ anxieties about their ability to help are not to be dismissed as just neurotic insecurity or obsessive worrying. It is realistic to be concerned about one’s performance in a new, complex, and ambiguous arena. It is simultaneously important that new therapists like Claire not lose sight of the significant personal strengths they already have. Most trainees already possess sensitivity, intelligence, and a genuine concern for others. Such personal assets, and all they have learned from their own life experiences, will prove helpful to their future clients. Although proponents of different theoretical approaches often claim superiority of their approach over others, treatment outcome studies repeatedly find that success rates in treatment have far more to do with the personal characteristics and the skill of the therapist than the theoretical orientation (Norcross, 2002; Norcross & Beutler, 2014; Norcross & Lambert, 2014; Norcross & Wampold, 2011; Seligman, 1995; Wampold, 2011, 2006; Wampold & Budge, 2012).

Personal experience, common sense, good judgment, and intuition are useful indeed. However, in order for a therapist to be effective with a wide range of people and problems, these valuable human qualities need to be wed to a conceptual framework, which provides a compass for the work you are going to do with the client. It is from this compass that you develop a **case formulation** for your client. Therapists working within every theoretical orientation become more effective when they understand specifically what is wrong and where they need to go in treatment, in order to help clients change. When therapists have this kind of focus for treatment, and know where they are going in treatment and why, they can be more consistently helpful. Without a conceptual framework as a guide, however, therapists do not have a focus for treatment and, in each successive session, are just putting out the next fire or wandering around without direction. We will turn to this key competency in the next chapter.

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\*\* Boldface terms are defined in the glossary.



## THERAPISTS ARE MORE EFFECTIVE WHEN THEY HAVE A TREATMENT FOCUS

The interpersonal-process approach is fundamentally grounded in the **common factors** literature (D’Aniello, 2015; Imel & Wampold, 2008; Messer & Wampold, 2002; Wampold & Budge, 2012). They are necessary for change and essential to every brand of successful therapy (Wampold, 2015). For many clients, however, the core conditions of empathy, genuineness, and warmth are not enough, especially for those who have more significant problems such as being violated or betrayed by a primary caregiver. For these and other clients with complex and challenging problems, something more is needed. Therapists will have much greater impact when they are able to conceptualize or discern more precisely what this client’s core problem really is, how it came about developmentally, and how it is being played out and causing symptoms and problems in daily life. The most widely used term for this key competency is a “**case formulation**” or “**case conceptualization**.” Having a case conceptualization to guide treatment can prevent therapists from getting lost in the different stories the client brings in each week—not knowing how to find the unifying thread or recurrent themes as the clients keeps moving from topic to topic.

Clearly, the opportunity for more enduring and significant change is lost without a case conceptualization and the focus for treatment it provides (Ingram, 2012; Persons, Beckner & Tompkins, 2013). Without this focus, clients might feel understood or cared about by their therapists, and may like them very much, but often little change occurs. In support, a burgeoning research literature highlights that greater effectiveness is associated with more focused interventions (Roth & Fonagy, 2005). Distinguishing a general sympathy, warmth, friendliness, or benevolence from a more specific and accurate empathy or conceptual clarity, Allen et al. (2008) suggest the metaphor that “we should visualize psychotherapy as being less like lounging in a warm bath and more like swimming in a cool, crystal-clear lake. Warmth is easier; clarity is harder.” Thus, while core conditions are necessary, they are insufficient—therapists also need to discern more clearly a focus for treatment that clarifies what’s really wrong, how the central problem came about, and what needs to be done to change it. In the chapters ahead, we will further elucidate how therapists can hone in and clarify more precisely the themes and patterns that link together the different problems and concerns the client is presenting.

Additionally, the process of learning how to contextualize or make sense of a client’s problems involves exploring various theoretical frameworks (Fernando, Cohen & Henskens, 2012). Before therapists can find their own professional identities and feel confident in their abilities, they need to integrate a theoretical framework that they can apply with diverse clients and that is congruent with their own personal values and life experiences. Developing therapists cannot claim their own professional identity by simply taking on the same theoretical approach as their mentor or adopting what their instructors and supervisors do. To work most effectively in whatever theoretical orientation they eventually adopt, and to enjoy being a therapist, student therapists are encouraged to actively explore and try out different approaches for several years—eventually integrating and *choosing for themselves* how they are going to work with clients.

Continuing with this concern for the therapists’ personal development, one purpose of this book is to provide a conceptual framework for understanding the therapeutic relationship, and learning how therapists can use the interpersonal process or current interaction with their clients to intervene and help clients change. Throughout this text, readers will be actively encouraged to modify this framework to fit their own personalities and therapeutic styles. The interpersonal process approach is integrative and can be applied by therapists who choose to work within different theoretical orientations (Norcross & Lambert, 2014).

With support from supervisors and instructors, new therapists will be able to personalize this model, integrate it with other theoretical approaches, and modify it to make it their own.

In this approach the emphasis is on providing clients reparative experiences with the therapist that will allow them to create self-narratives that are more coherent, affirming, and flexible. Many clients who come to therapy have suffered significant developmental wounds—their experiences lacked safety, protection, boundaries, and predictability, and in many cases their attempts to differentiate and develop autonomy were met with threats of abandonment or guilt inducement (Cassidy & Shaver, 2008; Lieberman & Van Horn, 2008). As a result, many were left to cope in a variety of symptomatic ways in order to manage developmental challenges and unmet needs—such as adopting rigid coping styles and living their lives in ways that can be characterized as being pervasively compliant or pleasing, strident or rigidly controlling, invisible or without needs, detached or demanding, and so forth. Thus, therapists working within a relational framework, whether they eventually come to identify as cognitive-behavioral therapists, interpersonal/dynamic therapists, existential-humanistic therapists, family systems therapists, narrative therapists, or others, can use the interpersonal process approach to work more effectively.

## CORE CONCEPTS

In the chapters ahead, we are going to apply three core concepts to understand clients and guide our interventions: the Process Dimension, a Corrective Emotional Experience, and Client Response Specificity. In this section, these three orienting constructs are introduced and then illustrated with a case example.

### THE PROCESS DIMENSION

The relationship between the therapist and the client is the foundation of the therapeutic enterprise and shapes the extent to which clients will be able to change (Markin & Hilsenroth, 2014). In order to utilize the therapeutic relationship as a vehicle for change, however, therapists need to understand the **interpersonal process** or the way that the client and therapist are interacting.

What do we mean by the process dimension? The therapist–client relationship is complex and multifaceted; different levels of communication occur simultaneously. It is helpful, for example, to recognize the subtle but important distinction between the overtly spoken *content* of what is discussed and the *process* dimension of how the therapist and client interact (Kiesler, 1988). In order to work with the process dimension, the therapist is making a perceptual shift away from the overt content of what is discussed and is beginning to track the relational process of how two people are interacting as well (Altenstein, Krieger & Holtforth, 2013; Thomas, Hopwood, Woody, Ethier & Sadler, 2014). This means that the process-oriented therapist is going to step beyond the usual social norms at times and talk more directly with clients about their current interaction or what may be going on between them right now. For example,

**THERAPIST:** Since I asked you about your canceling our session last week, and then coming late to our appointment today, you’ve become quiet. Maybe something about my question didn’t feel quite right to you. Can I check in with you about that?

OR

**THERAPIST:** Right now, John, I’m having a little trouble keeping up with you. You’re speaking fast, and sometimes it feels a little like we are almost jumping from topic to topic. Any thoughts here...what do you think might be going on between us?



In both of these queries, the therapist is not addressing what the client is saying (content) but how the therapist and client are interacting with each other (the interpersonal process). Let's look at one more example but this time, we'll extend the therapist's initial process comment into a longer therapist–client dialogue and see how process comments like these can uncover important new issues to explore in treatment, resolve misunderstandings that arise between the therapist and client, and bring greater intensity to the therapeutic relationship by moving the dialogue beyond surface issues:

**THERAPIST:** I'm honored that you choose to share such an important part of yourself with me. What's it like for you to risk sharing this sensitive feeling with me?

**CLIENT:** Uh, I'm not really sure what you're asking me?

**THERAPIST:** You just said so much to me, and I'm wondering how that was for you. What do you think I might be thinking about you as you're telling me this?

**CLIENT:** Well, you probably have to be nice, you know, because you're a therapist so that's how you have to act. But maybe inside you're judging me or something.

**THERAPIST:** "Judging you." That sure wouldn't feel very good. Tell me more about how I might be judging you.

**CLIENT:** Well, come on, you can't possibly respect me now that I've told you about this.

**THERAPIST:** I'm so glad we're talking about this misunderstanding, because no part of me is feeling judgmental toward you. Actually, I was appreciating your courage to bring this up and talk with me so honestly. But maybe you're telling me that others have often judged you?

**CLIENT:** Oh yeah, I know my mom loves me but she was always so judgmental...and now, whenever my husband says something that's the least bit critical, I just shut down inside and completely go away...

We are going to call these types of here-and-now, present-focused interventions, which focus on "you and me" or explore what is going on between the therapist and the client right now, as **process comments** (Hill, 2014). Because it may conflict with familial rules or cultural prescriptions for new therapists to speak directly in these ways, it may feel awkward at first for some. For other therapists, it may seem impolite or disrespectful to use process comments and speak forthrightly or inquire in open-ended ways about what may be going on between "you and me." However, as new therapists become more confident and less concerned about performance anxieties, and their supervisors help them understand better where they are trying to go in treatment with this particular client, they will be ready to begin trying out and exploring these process comments. New therapists soon will see how helpful it can be when they link the problem that the client is talking about with others to their current interaction in the therapist–client relationship—that is, highlighting how the client is utilizing the same problematic thought processes, faulty expectations, or ineffective coping strategies with the therapist, right now, that have been causing problems with others in their lives (Kasper, Hill & Kivlighan, 2008). Or, similarly, the therapist is *non-defensive* and willing to explore and sort through with the client potential misunderstandings, inaccurate perceptions, "mistakes," or other interpersonal conflicts that may be going on between them in their real-life relationship; that is, "restoring ruptures" (Safran et al., 2002). Therapists can **meta-communicate** and talk with clients about how they are interacting together, and use a variety of other immediacy interventions, in an empathic and respectful manner. When we use process comments in this way, clients will welcome them as a collaborative invitation for genuine understanding and honest communication—something they want and need. This can be done in a way that does not make them feel uncomfortable, but brings them closer to their key concerns (Crits-Cristoph & Gibbons, 2002; Hill, 2014; Kiesler, 1996). Consider the following example.

During their first session together, the client tells the therapist that he resents his wife because she is “bossy” and “always telling me what to do.” He explains that he has often had trouble making decisions on his own and, as a result, his wife has come to make decisions for him. Even though she said that she was just trying to help him with his indecisiveness, he resents her “pushiness” and “know-it-all attitude.” After describing the presenting problem in this way, the client asks the therapist, “What should I do?”

Let’s think first about the process dimension in their interaction. Suppose the therapist complied with the client’s request and said, “I think the next time your wife tells you what to do, you should...” If treatment with this particular client continues in this prescriptive vein, the therapist and client will quickly begin to reenact in their relationship the same type of conflict that originally led him to seek treatment. That is, the therapist will be telling the client what to do, just as his wife has been doing. The client has certainly invited this advice, and will probably welcome the therapist’s suggestions at first. In the long run, however, he will probably come to resent the therapist’s directives just as much as he resents his wife’s and will ultimately find the therapist’s suggestions to be as unwanted and of as little help as hers.

As an alternative to this directive response, a more non-directive therapist might respond to the other side of the client’s conflict and say, “I don’t think I would really be helping you if I just told you what to do. I believe that clients grow and learn more when they find their own solutions to their problems.” Frustrated by this non-directive response, which he only finds evasive, the client retorts, “But I told you I don’t know what to do! It’s hard for me to make decisions. Aren’t you the one who’s supposed to know what to do about these things?”

This non-directive response throws the client back on the other side of his conflict and leaves him stuck in his presenting problem—his own inability to make decisions. If this mode of interaction continues and comes to characterize their relationship, their process will reenact the other side of the client’s problem. His inability to initiate, make his own decisions, and be responsible for his own actions will immobilize him in therapy, just as it has in other areas of his life.

Thus, one of the things new therapists are going to observe is that clients do not just talk with therapists about their problems in an abstract manner. Rather, in their interpersonal process with the therapist, they often “enact” or recreate the same types of problematic interactions that originally led them to seek treatment. That is, clients also convey their problems in *how* they interact with the therapist (the process), bringing critical aspects of their problems with others into their current interaction (Hill et al., 2008). *This repetition or replaying of the client’s problem is a regular and predictable phenomenon that will occur, at times, in many therapeutic relationships.* We will return to this in Chapter 3, and throughout the chapters that follow, and see how therapists can use the process dimension to find more effective ways to respond and help clients change.

For now, let’s turn to the second core concept, the **Corrective Emotional Experience**.

## THE CORRECTIVE EMOTIONAL EXPERIENCE

Like most far-reaching and enduring concepts in therapy, the Corrective Emotional Experience (CEE) has many grandparents. It was developed most fully in the 1940s by Franz Alexander and Thomas French (1946), and has become the cornerstone of many short-term dynamic and interpersonally oriented therapies (Bernier & Dozier, 2002; Bridges, 2006). Although it was not appreciated by the psychoanalytic community of its day, this more active, direct, and personal approach provided clients with a real-life experience of change in the here-and-now relationship with the therapist. Accurately anticipating the future of therapy, Alexander and French originally advocated the CEE to shorten the length of treatment, put less emphasis on insight as the primary means of effecting change,

and focus instead on a more experiential, behavioral, or in vivo relearning. They actively encouraged clients to approach or engage in anxiety-arousing behavior that previously they had avoided, and even suggested giving homework assignments to clients between sessions. Initial support for this came from Horowitz and colleagues (1976; 1984; 1993) who found strong empirical support for enduring change in clients who received CEEs in a more supportive therapeutic approach.

Contemporary short-term relational approaches (Binder, 2004; Levenson, 2010; Silberschatz, 2005) describe the same direct, immediate experience of change in the real-life relationship between the therapist and client. The basic concept is that all therapists, working within different theoretical orientations, can help their clients change: they do so by providing a new and more satisfying response to the clients' old relationship patterns than they have usually found with others. That is, working collaboratively with clients, *the therapist helps clients identify maladaptive relational patterns or themes that commonly occur with others, and works with clients to alter this problematic pattern, disconfirm the faulty expectations or schema, or change this familiar but unwanted interpersonal scenario in their real-life relationship or therapeutic interaction* (Castonguay & Hill, 2012). Following Alexander and French, we are going to call this core concept a CEE (some interpersonal and attachment-oriented therapists use the term “reparative relational experience,” whereas some behavior therapists may describe this as “in vivo learning” or “exposure trials”).

When therapy fails and clients terminate prematurely, or treatment bogs down and reaches an impasse, the therapist and client are often reenacting in their interpersonal process some aspects of the same conflict that the client has been struggling with in other relationships, although neither of them may be aware of this reenactment. Based on their schemas and expectations, clients soon come to hold the same misperception or faulty expectation toward the therapist, or respond with the same maladaptive relational patterns, that have been causing problems with others in their lives. For example, even though the therapist has not behaved accordingly, when clients become distressed or vulnerable, they soon may believe the following:

- *They are being controlled by the therapist and have to do everything his or her way, just as they have done with others.*
- *They have to take care of and meet the therapist's needs, just as they have been doing with others in their life.*
- *They must please the therapist and win his or her approval, just as they keep striving to do with significant others throughout their lives.*

When this occurs, *the therapeutic process is metaphorically repeating the same type of conflicted interaction that clients have not been able to resolve in other relationships, and that they have often experienced in formative, attachment relationships*. When clients begin to play out with the therapist the same relational patterns that originally brought them to treatment, the therapist's goal is to respond in a new and more effective way that allows clients to resolve the conflict and change the pattern within their relationship. The benefit to clients from this in vivo experience of change is that fixed or narrow schemas will expand and become more flexible or realistic, and it will become easier to begin changing these problematic patterns with others, such as spouses, children, and friends. Although providing a corrective emotional experience may sound easy, it can be challenging to do—especially when all of this is so new to therapists-in-training. To help, Hill (2014) encourages therapists to be asking themselves the same process-oriented question throughout each session:

**THERAPIST:** Right now, am I co-creating a new and reparative relationship, or am I being drawn into a familiar but problematic interaction sequence that is reenacting for this client?

When the therapist and client are able to resolve these reenactments, which occur in most therapeutic relationships, change has begun to occur. Clients learn that they no longer have to respond in their old ways (such as always having to be in control, be the responsible one, or have to take care of others) or always have to receive the same unwanted responses from others (e.g., being ignored or dismissed, judged or criticized, envied or competed with). In this way, clients develop a wider range of expectations and begin to learn how they can respond in more flexible and adaptive ways—especially in the safety of their relationship with the therapist. And, as their behavior begins to change with the therapist, it is usually relatively easy to take the next step and help clients generalize this experiential relearning with the therapist and adopt similar, more adaptive responses with others outside in their everyday lives.

**Clients believe actions not words.** Long ago, Frieda Fromm-Reichmann (1960) captured this central tenet best by saying that the therapist needs to provide clients with an *experience* rather than an *explanation*. Interpretations, facilitating insight, empathic understanding, cognitive reframing, self-monitoring techniques, education, skill development, and other interventions are utilized in the interpersonal process approach and will be helpful with most clients. They are not the primary seat of action in this approach, however.

This is an experiential learning model. Following Strupp and Binder (1984), clients change when they live through emotionally painful and long-ingrained relational experiences with the therapist, and the therapeutic relationship gives rise to new and better outcomes that are different from those anticipated and feared. That is, when the client re-experiences important aspects of his or her primary problem with the therapist, and the therapist's response does not fit the old schemas or expectations, the client has the real-life experience that some relationships can be another way. This new or reparative *experience* is a powerful type of relearning that can be generalized readily to other relationships (Bandura, 1997). The key point here is that *this behavioral experience of change with the therapist is far more compelling than words alone can provide*.

We are going to be exploring many different ways that therapists can use this corrective emotional experience to facilitate change. In particular, the new or corrective response from the therapist helps clients change by creating *greater interpersonal safety* for the client. This is far more than the good feeling that comes from being taken seriously and treated with respect in the therapeutic relationship. Instead, it is a more significant sense of deep personal safety that results when the client is not hurt again by receiving certain interpersonal responses that are familiar and expected, yet unwanted or even dreaded. As we are emphasizing, *this pivotal moment is not the end point in treatment but a window of opportunity for a range of important new behaviors to emerge*. For example, in the next few moments following a reparative experience with the therapist, clients often feel safer and may be empowered to:

- *Experience or risk feeling more fully certain painful or difficult emotions;*
- *Resolve their ambivalence and make an important personal decision;*
- *Risk trying out new ways of responding with the therapist or others in their lives;*
- *Make significant self-disclosures or bring up relevant new issues or concerns to explore;*
- *Feel bolder and risk addressing unspoken misunderstandings with the therapist or bring up more directly a problem the client has been having with the therapist;*
- *Feel better about themselves or forgive themselves for things they have felt unrealistically shameful or guilty about;*
- *Grasp more fully how much this faulty coping strategy or self-defeating behavior has hurt others or cost them in their own lives;*
- *Have insight or make meaningful links between current behavior and formative relationships where these schemas and patterns originally were learned.*

In these ways, corrective emotional experiences with the therapist begin to expand early maladaptive schemas and problematic relational patterns, and a wider range of responses becomes available to the client—often in the next few moments. A single CEE usually is not sufficient for sustainable change, but it is often the pivotal experience that initiates new behavior. Based on the interpersonal safety that arises as clients find that they reliably receive this new, reparative response, the therapist is seen as someone who can help—earning achieved credibility (Sue & Zane, 2009). Clients then invest further in the therapeutic relationship and in the treatment process, and often become more willing to try out new coping strategies and ways of responding with others. In contrast, when clients elicit the same type of reactions from the therapist that they tend to find with others (e.g., the therapist begins to feel frustrated, controlled, discouraged, or disappointed, as significant others in the client’s life often feel), intervention techniques from every theoretical perspective will falter. In this way, the interaction between the therapist and client provides a “meta” perspective both for understanding what is occurring and for knowing how to intervene in the therapeutic relationship, and it can be integrated readily with other treatment approaches (e.g., schema confirming versus schema disconfirming interactions with the therapist).

### Using the Process Dimension to Provide a Corrective Emotional Experience.

Linking our first two core constructs, the therapist needs to be able to work with the process dimension in order to provide this CEE. Recall the previous example of the client who was complaining that his wife was “pushy” and always telling him what to do. When the client asked his therapist what he should do, one option would have been for the therapist to offer a “process comment” that makes their current interaction overt as a topic for discussion:

**THERAPIST:** Right now, it seems that you’re asking me to tell you what to do. But I’m wondering whether that will only bring up the same problem for us here in therapy that you are having at home with your wife. Let’s see if you and I can figure out a way to do something different in our relationship. Rather than having me tell you what to do, let’s try and work together to understand what’s going on for you when you are feeling indecisive. Where do you think is the best place to begin?

**CLIENT:** I’m not sure.

**THERAPIST:** Take your time, and let’s just see what comes to you.

**CLIENT:** (pause) I’ll get criticized. No matter what I decide, she’ll find something wrong with it.

**THERAPIST:** OK, that’s a good place for us to start. You can’t get it right in her eyes— you’ll always do it wrong. Sounds like there’s a lot of feeling in that for you, tell me more.

**CLIENT:** Well, I hate it because she wants to be the parent—you know, the one in charge, and treat me like a child...

In this dialogue, the therapist’s intention is to ensure that the same problematic but familiar pattern does not play out in the relationship with the client. The therapist is trying not to take the bait and repeat the pattern by telling the client what to do. Instead, the therapist is trying to offer the client a new and different type of relationship—a collaborative partnership in which they can work together. To support this, the client will rehearse, role-play, and try out these new behaviors with his wife and others. Typically, he will have some successes and some failures in his attempts to adopt this stronger way of relating to her and to others. However, the therapist’s initial attempt to offer the client a corrective response will need to be repeated in different ways for change to consolidate. If the therapist can keep finding ways to engage him more actively in their interaction (easier to say



than to do with this client), this new interpersonal process will be corrective, and it will facilitate other therapeutic interventions as well. That is, behavioral alternatives, interpretations, and interpersonal feedback will all become more effective with this corrective interpersonal process. This client's life has been ruled by his Subjugation/Control schema, but now he is a collaborator in the treatment process with his own voice and is not just being told what to do again (Baldwin et al., 2007; Miller & Rollnick, 2013). We aren't all the way around the block yet, but change is under way.

### **Countertransference Issues and Fears about Making Mistakes Diminishes New Therapists' Engagement with Their Clients.**

As we have seen, the guiding principle in the interpersonal process approach is to provide clients with an experience of change (Florsheim & McArthur, 2009). With this in vivo relearning, clients live out a new relationship with the therapist that disconfirms their faulty expectations and expands their schemas for what they can have in life—for what can take place for them in relationships. For example, clients learn that, at least sometimes—first with the therapist, and then with some other people in their lives, they find—they can: Be responded to supportively if they ask for help; Say “no” and have others respect their limits; Give priority to their own needs sometimes rather than always submerging them and going along with what others want; Succeed without always evoking competition or envy; and so forth. Providing this type of CEE is a personally engaging and highly rewarding way to work with clients but—make no mistake about it—it demands much on the part of the therapist. In particular, **countertransference** issues, and our previous topic of new therapists' fears of making mistakes, make it harder to provide a CEE. Let's look further at both.

First, it requires personal involvement from therapists to work with the client in this way. The therapist must be willing to engage genuinely with and risk being personally affected by the client (Rogers, 1980). That is, in order to have the emotional impact necessary to provide a corrective emotional experience and propel change, the relationship must hold real meaning for both participants (Gelso, 2011; Horvath & Bedi, 2002; Kivligan, Gelso, Ain, Hummel & Markin, 2015). In this simple human way, it is the relationship that heals—the relationship itself facilitates “**collaborative, purposive work**” (Hatcher & Barends, 2006). If the therapist is merely an objective technician, psychologically removed and safely distant from the client, the relationship will not hold real meaning for either person. It will be too insignificant to effect change—even if the therapist does indeed respond in new ways that do not follow along with old problematic patterns, beliefs, and expectations.

On the other side of this issue, treatment also falters if therapists over-identify with clients or become too invested in the client's choices or ability to change. In this situation, therapists often lose sight of the process they are enacting with the client and unwittingly begin to respond in problematic ways that reenact aspects of the client's maladaptive relational patterns. Thus, a prototypic situation for many new therapists is to find that they are experiencing the client's predicament or concerns as similar to their own. This occurs, for instance, when the therapist is thinking, “She's just like me!” The solution to this lack of differentiation is a supportive yet honest supervisory relationship that helps therapists see their own countertransference issue and recognize the numerous ways that their own experience really does differ significantly from the client's. There may be important similarities between them, yet, in reality, the client's problems—and the broader context that shapes their meaning—are never really the same as the therapist's. Once therapists are able to differentiate their own issues from those of the client, they will often be able to see how the therapeutic process has been reenacting aspects of the client's conflict (e.g., perhaps the therapist also grew up being a confidant and best friend to her parent and also felt they were too close and the same). In parallel, the multicultural literature is also instructive here in highlighting “zones of difference and similarity within and between

cultures” (Pedersen et al., 2008). This understanding that each therapeutic relationship involves a range of similarities and differences often enables therapists to change this reenactment (such as over-identifying with or over-reacting to the client) and establish instead a more resolving and differentiated interpersonal process.

Second, we have already seen that new therapists should expect to make many “mistakes” with their clients. For example, to become over-involved or under-involved with certain clients, to reenact the client’s maladaptive relational patterns at times, and so on. Such mistakes are an inevitable part of the therapeutic process for beginning and experienced therapists alike. Few clients are fragile, however, and therapeutic relationships are often remarkably resilient. Unfortunately, most student therapists do not know this and, as emphasized earlier, concern about making mistakes remains one of their biggest anxieties. New therapists will do better therapy—and enjoy this work so much more—as they discover that *mistakes can be undone*. Mistakes can be worked through or resolved with clients, and mistakes can provide important therapeutic opportunities, when therapists are willing to work with the process dimension—that is, *when therapists are willing to talk with clients about the misunderstandings or potential problems that may be occurring between them*. For example,

**THERAPIST:** I’m wondering whether I might have misunderstood something you said. I’m asking about that because it seems as if you have become more distant from me in the last few minutes, and you’ve mentioned that other people often don’t understand you. What do you see happening between us here?

**CLIENT:** Yeah, I guess I do feel a little bit distant. But I don’t feel “misunderstood,” I just don’t really like it when you don’t say very much—you’re pretty quiet, you know.

**THERAPIST:** That’s helpful to hear—I appreciate your honesty. So, let’s change things—I’ll start speaking up more and sharing more of my thoughts with you. And let’s check back in on this later and see how it’s going. How’s that sound?

**CLIENT:** Thanks, that would be better for me.

**THERAPIST:** Good, I want to hear your concerns, and work together in ways that feel most helpful to you. Can you tell me a little more about what’s it like for you when I’m quiet or don’t say very much?

**CLIENT:** I get uncomfortable...I don’t feel like I’m getting enough direction or help, and maybe then therapy isn’t going to work for me and I’m just going to stay depressed. And I’m not really sure what a client is supposed to do. I’ve never been in therapy before, you know, so I worry about what I’m supposed to be talking about or what you expect me to be doing...

As their performance anxiety declines, new therapists will find that they are better able to identify and work with the process dimension. The very best way to realign the therapeutic relationship when it has gone awry is for therapists to talk through and resolve potential misunderstandings or mistakes. Following Hill (2014), student therapists are encouraged to be non-defensive and recognize their mistakes, apologize if necessary, and *talk through the event with the client*. This restores the therapeutic relationship, and it provides clients with an effective model for how to deal with problems. We will return to process comments and this all-important topic of “rupture and repair” in the next chapter. For now, we turn to the third core concept: Client Response Specificity.

## CLIENT RESPONSE SPECIFICITY

We have already begun to see that new therapists need a theoretical framework to guide their therapeutic interventions. Let’s look further at what a theory needs to provide if it is to be of help to therapists and their clients.

One feature of an effective clinical theory is that it has the flexibility and breadth to encompass the diversity of clients who seek treatment. For many student therapists, clinical training changes their fundamental **worldviews** in profound and enduring ways. For instance, new therapists begin to recognize—to a far greater extent than they had appreciated before—that there are unifying patterns in personality and behavior (Levenson & Strupp, 1997). In parallel with this new organizing perspective, however, it also is clear that every client really is unique and different. Each client has been genetically endowed with a unique set of features and each has been raised with different values and beliefs in his or her familial and cultural context. Socialization is different for women and men, individuals growing up in varying cultures have vastly different political experiences and religious training, and economic class shapes opportunity and expectations. To be helpful, a clinical theory needs to help therapists work effectively with diverse clients—who also are entering treatment with different levels of motivation and differing readiness to change (Prochaska & Norcross, 2006). Moreover, each therapist is a different person. Like clients, therapists differ in age, gender, ethnicity, sexual orientation, and developmental background. Therapists also bring diverse values, subjective worldviews, and personal styles to their clinical work. How can any theory help such a diversity of therapists respond to the extraordinary range of human experience that clients present?

Our third core construct, **Client Response Specificity**, will be one of our best tools. With this construct, therapists need to tailor their responses to fit the specific needs of each individual client—one size does not fit all! There are no cookbook formulas or generic techniques for responding to the complex problems, diverse developmental experiences, and multicultural backgrounds that clients present. In this regard, we are going to see that the same therapeutic response or intervention that helps one client make progress in treatment will only serve to hinder another (Bachelor, 1995; Silbererschatz, 2005). For example, some clients respond well to warmth in the counselor, whereas others do better with objectivity. More specifically, a warm and expressive therapist can put off a distrustful or avoidant client; a businesslike therapist can fail to engage an anxious client in crisis. Clearly, this work is not easy. It's complex—and it will be simply maddening if you must have rules and can't tolerate ambiguity! The key point is that therapists need to have the flexibility to listen to the cues, assess clients' responses, and search for the best way to respond to a particular client (Lazarus, 1993; Sampson, 2005). As we are going to see, however, *therapists often fail to hear and accommodate to clients' feedback about what they want and don't want from them* (Waldin & Imel, 2013).

As we consider client response specificity, the operative word is *flexibility*. This approach requires therapists to respond on the basis of each client's own personal history and ways of viewing the world. **It is certainly helpful to be familiar with the experiences of particular groups** (e.g., to know that African Americans' history includes almost 375 years of slavery, and 150 years of subsequent racial discrimination) and the defining criteria and symptomatology for DSM disorders (e.g., to know that bipolar clients in a manic phase are at risk to act out with aggression toward others, sexual promiscuity, or suicide). **Additionally, however, with client response specificity, our abiding intention is to attempt to understand and respond to each client as a unique individual.** Thus we will not seek treatment rules or intervention guidelines that apply to all clients, or even to specific diagnostic categories or groups, such as men, Latinos, or Christians. Rather, we will be trying to find case-specific recommendations that help us understand what it means to this individual sitting in front of us, to be depressed, lesbian, or biracial. Let's look more closely at what client response specificity means and see how it can help us be more effective therapists.

Interpersonal process therapy is a highly “**idiographic**” approach; it emphasizes the personal experience or subjective worldview of each individual client. Diagnostic

categories will tell us something about the client, but they are global and lack specificity for a particular client. Personality typologies also highlight potential directions that therapy might take, but the actual therapeutic process and issues covered will be client-specific. For example, the two clients on your caseload who both have generalized anxiety disorder, an avoidant personality disorder, or who are sexual assault survivors certainly will share some common ground, but the differences between the two—their uniqueness—is highly significant. Throughout each session, therefore, *therapists are encouraged to join the client in a process of mutual exploration to try to find the subjective meaning that each particular experience holds for this client.* For example,

**THERAPIST:** Let me check this out with you and make sure I'm understanding what you really mean here. When I hear you say that, it sounds more like you are feeling sadness, or maybe even grief, than depression. Am I getting that right—what are your thoughts?

From the first session through the last, the therapist is taking on the personal challenge to be *accurately* empathic or, in different terms, to have the cognitive flexibility and perspective-taking ability to de-center and enter the client's subjective worldview. The meaning that a particular experience holds for the client often differs greatly from the meaning that this same experience holds for the therapist, or for other clients that the therapist might see. Therapists are encouraged to listen carefully to clients' language and words choices, and to explore clients' own metaphors collaboratively. For example,

**THERAPIST:** Help me understand what you mean, or what you might be saying about our relationship, when you tell me that you just want to “sail right out of here?”

By continuously reaching for this specificity, and not taking for granted that we know what the client really meant, we are not belaboring the obvious. When someone cares about you enough to listen seriously, and works hard to understand just exactly what it is that you are trying to say, meaning is created.

With client response specificity, there is no standardized treatment protocol. Each individual client's developmental history, cultural context, and current life circumstances serve to guide the therapist's treatment plans and intervention strategy. The interpersonal process approach does not advocate a particular therapeutic stance toward clients in general, such as being directive or non-directive, problem-solving or exploratory, supportive or challenging, veiled or self-disclosing, and so forth. Each of these stances will be helpful with a particular client at times but ineffective with another. Thus, this approach asks something different and more challenging of therapists:

- *To conceptualize the specific relational experiences that this client needs in order to change (e.g., a client who grew up with intrusive caregivers, and now has a boyfriend who checks her cell phone and email, will benefit from a therapist who repeatedly supports her personal boundaries and privacy by saying things like, “Tell me as much or as little about that as you wish...”; “We can address that at your pace...”; “We are going to do things your way here—I don't need to know anything about you that you don't wish to tell me...”; etc.);*
- *To be flexible and modify their interventions in the ways that this particular client requests or seems to utilize more productively.*

This recommendation may sound obvious or easy to do but, unfortunately, researchers tell us that most therapists do not do this very well. Therapists continue to respond to clients in the same manner, based on their theoretical orientation—even when their preferred brand is not working! That is, psychotherapy process studies find that most therapists do not demonstrate *flexibility* in their technical approach to the client, or have the interpersonal range to modify their interventions, and *provide the responses that each*

*client could utilize best* (Najavits & Strupp, 1994; Sampson, 2005). With client response specificity, our aim is to assess—strive to discern—how clients are responding to our interventions in an ongoing way, and flexibly adapt. To help with this, the counseling literature on “intentions” suggests that therapists ask themselves the following questions:

*Where am I right now and what do I want to accomplish?* (E.g., recognize and respond to the feeling or concern that the client is expressing right now.)

*OK, what is the best way to do that?* (This might include validating or providing an empathic reflection of the client’s distress or concern.)

In this way, therapists are encouraged to think about their intentions—*what they are trying to accomplish with each response or turn of the conversation* (Ivey, Ivey & Zalaquett, 2014). Therapists will be better prepared to respond to the specific needs of this particular client, at this moment, when they are asking themselves in an ongoing way,

*What is needed right now, and how can I provide that?*

Let’s use an example to illustrate client response specificity more concretely, and then examine how therapists can assess clients’ positive or negative responses to the different interventions and varying responses they provide. We’ll also look at how therapists can modify or tailor those responses, as necessary, to work better for clients. Consider an often debated question: Should therapists self-disclose to their clients? client response specificity emphasizes that self-disclosure (or any other intervention) *will hold very different meanings for different clients*. That is, the same intervention or response may have the opposite effect on two different clients with contrasting developmental histories and cultural contexts. For example, if a client’s caregiver was distant or aloof, the therapist’s judicious self-disclosure may be helpful for that client. In contrast, the same self-disclosure is likely to be anxiety-arousing for a client who grew up serving as the confidant or emotional caretaker for a depressed, anxious, or alcoholic parent. Greater self-disclosure from the therapist may help the first client learn that, contrary to her deeply held beliefs, she does matter and can be of interest to other people. In contrast, for the second client, the same self-disclosure may inadvertently impose the unwanted needs of others and set this client back, as in her mind, she experiences herself in her unwanted caretaking role again—this time with the therapist. This problematic reenactment occurs because the therapeutic relationship is now paralleling the same maladaptive relational theme that this client struggled with while growing up. Considering client response specificity in this way, therapists working within every theoretical orientation will be more effective if they consider how the client’s cognitive schemas or developmental experiences are likely to shape the impact of their interventions on this particular client.

Like the previous two core constructs, client response specificity is a complex and multifaceted clinical concept that will take time to learn and put into practice with clients. Let’s illustrate it further by posing another common question: Should therapists give opinions when clients ask for advice? As before, the therapist’s response is informed more by each client’s particular circumstances, than by the brand of therapy standardly employed by the therapist. For example, offering advice may be counterproductive for a compliant, intimidated female client who is trying to become more independent and assertive with her dominating and controlling husband who undermines her confidence and actively fosters her dependency on him. For such a client, it changes the relational pattern for the therapist to suggest,

*OK, why don’t we start with your thoughts about this, and then I’ll add mine. What do you think might work best for you here?*



In contrast, giving advice and directives may work well with a different client whose caregivers did not help him solve his problems, perhaps because the caregivers were too self-absorbed or preoccupied with their own lives to be very interested or responsive to him. **To withhold suggestions or problem-solving advice from these clients may only reenact developmental deficits and impede their progress in treatment as, yet again, they do not get the help or guidance they need and want.** Here, the same response from the therapist will have very different effects on different clients. From this meta-perspective, techniques or interventions from any theoretical approach may help or hinder the client; it depends on whether the response serves to reenact or resolve maladaptive schemas and relational patterns for this particular client.

Finally, let's highlight another aspect of client response specificity— assessing clients' responses to therapists' interventions. Therapists can learn how to assess the effectiveness of their interventions by paying close attention to how the client utilizes or responds to what they have just done. For example, if the therapist observes that the client becomes burdened, irritated, or worried in response to the therapist's self-disclosure, the therapist is learning how to respond more effectively to this client. It's not that self-disclosure is always ill-advised, or an ineffective way to respond in general, but it would be problematic rather than helpful for this particular client. As noted earlier, when the therapist provides a corrective response, many clients will make progress right away by feeling safer and may act more boldly, bring forth relevant new material, become more engaged with the therapist, and so forth. In contrast, when the therapist responds in a way that repeats problematic old patterns or confirms maladaptive schemas, clients will behaviorally inform the therapist of this by acting "weaker" in the next few minutes—for example, by being more passive, indecisive, or compliant (Silberschatz, 2005). Thus, if therapists track clients' immediate responses to the varying interventions they employ (e.g., providing interpersonal feedback or making process comments, suggesting thought records or self-monitoring strategies as homework assignments, making interpretations or cognitive reframing, etc.), clients will behaviorally inform therapists how best to respond. However, as emphasized earlier, therapists then must be flexible enough to adjust their interventions when necessary and try out different ways of responding that this particular client might be able to utilize more effectively.

To sum up, working in this highly individualized way adds complexity to the therapeutic process and places more demands on the therapist. However, such an approach gives therapists the flexibility to respond to the specific needs and unique experiences of the diverse clients who seek help. Client response specificity is a core construct in the interpersonal process approach. Tools to assist with this construct include guidelines for keeping process notes (see Appendix A) and for writing case conceptualizations (see Appendix B). These will help therapists identify developmental experiences that are likely to have caused faulty beliefs and relational difficulties, which need to be addressed along the process dimension in a corrective way, with specificity for this particular client. The following case example illustrates these three core concepts.

### TERESA: CASE ILLUSTRATION OF CORE CONCEPTS

The following vignette illustrates our three core concepts: the Process Dimension, Corrective Emotional Experience, and Client Response Specificity. In particular, the vignette highlights how the process dimension (the way the therapist and client interact) can contribute either to resolving or reenacting important aspects of the client's problem. Usually, therapists do not literally reenact with clients the same hurtful or problematic responses they have received from others (in this case, the problem is sexual abuse).

Unintentionally, however, the way in which they interact often thematically evokes the same issues, feelings, or concerns that the client has been struggling with in other relationships (in this case, *compliance* and having to “go along” with the counselor and others). As you read this example, think about what you would do similarly or differently if Teresa were your client.\*\*\*

A first-year practicum student is talking for the first time with her 17-year-old client, Teresa, about Teresa’s sexual contact with her alcoholic stepfather. Understandably, this new therapist is anxious. Her heart is breaking for this girl, and she wants to help Teresa so much, but the therapist has never in her life spoken directly with anyone about the experience of being violated sexually. The *content* of what Teresa and the therapist are beginning to talk about is sexual molestation. Depending on the interpersonal process they enact, however, the effectiveness of this discussion is going to vary greatly. On the one hand, suppose the counselor is initiating this discussion and pressing Teresa for further disclosure about what happened to her. The graduate student therapist is deeply concerned about Teresa’s safety. However, her “need to know” is intensified by her concerns about her legal responsibilities as a mandated reporter, and her concern that her supervisor will want to know more details or facts about Teresa’s molestation.

In response to the counselor’s continuing press for more information, Teresa complies with the counselor and reluctantly speaks further about what happened. Useful information may be gained under these circumstances (i.e., content), but the opportunity for therapeutic progress is diminished or even lost because aspects of Teresa’s problem are being reenacted with the counselor in the way they are interacting (i.e., process). How is this a problematic **reenactment** along the process dimension? Teresa is again being pressured to obey an adult, comply with authority, and do something she doesn’t want to do. Of course, being pressed to talk about something she doesn’t want to discuss in no way re-traumatizes her as the original abuse did. The new way of thinking student therapists want to start developing, however, is how the interpersonal process of Demand/Comply is awry, and painfully familiar to Teresa, and will evoke in Teresa similar types of feelings and concerns that the abuse initially engendered. That is, her helplessness will lead to depression and her compliance (having to go along) to feelings of shame. Because the therapeutic process is thematically or metaphorically evoking aspects of her original mistreatment, her compliance with the therapist is likely to hinder her progress in treatment and actually slow the process of reempowerment.

This unwanted situation can be further complicated if Teresa belongs to an ethnic group where family loyalty is highly prized, or if she is a member of a religious community where obedience to authority and hierarchical relationships are emphasized. In this cultural context, Teresa is being asked to violate rules sanctioned by her family’s ethnic or religious group. Other family members and friends may strongly disapprove of the stepfather’s behavior. However, they may not provide Teresa with the validation and support she needs because she took this information to someone outside the family (the therapist) or because she violated their religious proscriptions that emphasize obedience and forgiveness. Thus, although disclosure violates family rules and loyalties for most victims, this may take on additional significance and add more distress for Teresa if she were a Hispanic adolescent who belonged to a conservative religious community.

What should the counselor in this example do instead? Wait non-directively for Teresa to volunteer this information—while she may continue to suffer ongoing abuse

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\*\*\* The clinical examples in this text are based on actual cases; however, identifying information, including the gender of the client or therapist, has been altered to preserve confidentiality.

at home? Of course not. But by attending to the process dimension, the therapist may be able to begin providing Teresa a corrective experience while gathering the same information. That is, instead of pressing for disclosure, what if the therapist *honored* Teresa's reluctance or cultural prescriptions? For example, instead of pressing for more information and trying to get Teresa to talk about things she doesn't want to talk about, the therapist could "meta-communicate" and make a process comment by compassionately or supportively inquiring about Teresa's reluctance to speak:

**THERAPIST:** All of this is so difficult right now. What's the hardest thing about talking with me?

OR

**THERAPIST:** It seems hard for you to talk with me right now—maybe something doesn't feel safe. I'm wondering what might happen if you share this and let me try to help you. What might go wrong?

OR

**THERAPIST:** Perhaps talking with me about this violates family rules, and you're concerned what your mother or stepfather might think. Would you feel OK talking about your family's rules and what's OK or not OK to talk about outside your family?

OR

**THERAPIST:** Let's work together and try to find a better way to talk about this. Can you tell me one thing that we might be able to change or do differently that might make this a little bit easier for you?

The purpose of process comments like these is to create a different and more reparative interpersonal process—one that helps Teresa feel less that she has to comply with the therapist—as she has with her stepfather and others. In response to these invitations, Teresa is likely to present a number of reality-based concerns that make it harder for her to speak further. For example, Teresa might reply that:

- *her mother won't believe her;*
- *her stepfather will be sent away and she will be blamed for breaking up the family;*
- *she will be told by some people that it is her fault;*
- *some will tell her that she is being selfish and should stop causing problems;*
- *some will tell her to be more forgiving, and not talk to anyone about this;*
- *she will be ostracized or cut out for not keeping it in the family and trying to resolve it there;*
- *the counselor may not believe her or may take her parents' side;*
- *the counselor may want to remove her from her home;*
- *the counselor or others will regard her as "dirty," "ruined," or shame-worthy in some way;*
- *the therapist may feel uncomfortable talking about this sensitive issue and will not want to do anything about it, and so forth.*

If the therapist can help Teresa identify and resolve her concerns, then she can find it safe and empowering to begin talking about what happened—which does not occur if the therapist simply directs her to disclose what happened. Although this process difference may seem subtle, its effect is powerful and will have a defining impact on the course and outcome of treatment. Based on the way she shares her trauma with the therapist, Teresa either will begin the empowerment process or confirm her problematic expectation that she must always comply and go along with what others want. With the process comments and invitations suggested earlier, Teresa is able to *participate* in the decision

process with an authority figure, have her concerns expressed and taken seriously, and, to the extent possible, accommodated as best they can. For example:

**THERAPIST:** Yes, Teresa, you don't have to do this "alone." Your aunt Nora brought you to see us today, and she is waiting in the lobby. If you like, we can invite her to sit in with us while we talk.

In a way that is new and different, Teresa finds that she can at least have some shared control over what she says, and to whom, and still remain supported. *She is not "alone" or **disempowered** in her interaction with the therapist—as she had been while keeping this secret about her stepfather.* This **corrective interpersonal process** with the therapist will help Teresa to begin acting in similarly empowered and self-affirming ways in some of her other relationships as well. In this and subsequent sessions, the therapist can help Teresa discern other people in her life with whom it is safe to be similarly assertive (her aunt Nora, her minister, and certain friends and family members), and those who will punish or thwart this and demand that she merely comply again (her stepfather, her grandmother, and certain friends and family members).

But what if the therapist's efforts to create a different interpersonal process don't work so well and Teresa still does not want to speak or discuss the secret any further? The therapist is a mandated reporter and still will need to contact Child Protective Services. In some cases, the therapist's attempts to empower the client by giving her more choice or participation in the reporting/treatment process clearly will have helped, and other times it may seem as if it hasn't made much of a difference. *Even if not, however, Teresa has found that the therapist is sincerely trying to find ways to include and empower her rather than merely demand that she comply again and do what the therapist wants.* In this way, a small but significant difference is occurring in their relationship that will facilitate her recovery, as Teresa begins to see that the therapist's intentions are different than what she has come to expect from others.

In sum, *techniques from differing theoretical orientations all can be helpful, but they are not likely to be effective unless the interaction or process that transpires between the therapist and client is enacting a solution to the client's problems.* In the chapters ahead, a variety of therapeutic interventions will be used, including providing empathy and a sense of being understood, helping the client recognize thought processes or behavior patterns that are maladaptive, and role-playing new behaviors. Per client response specificity, however, the effectiveness of these interventions will depend on whether the client's maladaptive schemas and problematic interpersonal patterns are reenacted or resolved along the process dimension.

## THEORETICAL AND HISTORICAL CONTEXT

The interpersonal process model presented here is an Integrative approach (Ingram, 2012). It highlights relational components from differing theories and synthesizes a common focus on the therapeutic relationship—which has emerged in the empirical literature as the most consistent predictor of successful treatment outcome (Baldwin, Wampold & Imel, 2009; Horvath & Bedi, 2002; Norcross, 2002; Norcross & Lambert, 2014). As we have begun to see, this treatment approach clarifies how therapists can use their current interaction—the interpersonal process they are enacting with their clients—to help clients change. Theorists and researchers from different theoretical orientations all have contributed to our understanding of the therapeutic relationship and how therapists can use it to guide what we do with our clients. In this regard, varying theories have focused on different aspects of clients' functioning and helped therapists respond to different aspects of people's problems. In particular, the Interpersonal, Cognitive, and Familial/Cultural

domains have been emphasized in different treatment approaches. This text highlights the process dimension in each of these broad theoretical traditions, and links these concepts about the current interaction between the therapist and client to clinical practice. These three domains are introduced below and provide a theoretical context for the clinical approach presented in the chapters ahead.

## THE INTERPERSONAL DOMAIN

Beginning with a historical perspective, the Interpersonal dimension was originally highlighted by Harry Stack Sullivan and has been articulated further by Hans Strupp, Irvin Yalom, and many others. Sullivan (1968) first brought the interpersonal focus to psychotherapy in the 1940s, and he remains an enormously influential but insufficiently recognized figure. A maverick, Sullivan radically broke away from Freud's biologically based libido theory and was one of the first major theorists to argue that the basic premises of Freud's drive theory (e.g., sexual and aggressive instincts) were inaccurate. Sullivan emphasized clients' current behavior and relationships with others rather than developmental fixations or how clients were stuck in the past. He and other relational theorists pressed for a more behavioral or reality-based focus on the child's actual experience with parents, or real-life interactions with caregivers, and stepped away from Freud's emphasis on fantasy and intra-psychic processes. Sullivan focused on what people do to avoid or manage anxiety in close relationships and, prefiguring John Bowlby, was especially interested in the hurtful or anxiety-rousing interactions that repeatedly occurred in formative parent-child interactions. Following from this, Sullivan conceptualized personality as the collection of interpersonal strategies that the individual employs to avoid or minimize anxiety, ward off disapproval, and maintain self-esteem.

According to Sullivan, children develop their personality, or "self-system," through repetitive interactions with their parents. For example, children may develop internal images of themselves as helpless or insignificant and expectations of parents and others as demanding or critical. Alternatively, more fortunate children who enjoyed better developmental/familial experiences may evolve images of themselves as love-worthy and expectations that others can be trustworthy.

Having learned these self-other relational patterns, people systematically behave in ways that avoid or minimize the experience of anxiety. For example, suppose that particular aspects of the child, such as feeling sad or crying, consistently result in parental responses of irritability or indifference, or even more problematically, ridicule or overt rejection. The child learns that these parts of oneself, (including vulnerability or emotional needs), are unacceptable and represent a "bad self." These unacceptable, anxiety-arousing aspects of the self are split off or disowned and the child develops interpersonal coping styles (e.g., a pleasing/placating manner, a macho/dominating style, or a rigidly self-reliant and distant stance in all or most relationships) that preclude being subjected to anxiety-arousing rejection or ridicule again. *These coping styles are interpersonal defenses* (as opposed to Freud's intrapsychic defenses such as denial or projection) *that originally were necessary to protect the self in early parent-child relationships*. Unfortunately, these interpersonal defenses are rigidly applied and overgeneralized to other relationships in adulthood and become habitual behavior patterns as the child, now grown to an adult, inaccurately expects that new experiences with others will repeat the same hurtful, anxiety-arousing patterns of the past. Sullivan's early formulations have been greatly enhanced by attachment researchers (using the terms "internal working models" and "relational templates") and by cognitive behavioral therapists (using terms such as "early maladaptive schemas," "faulty expectations," and "selective filters"). In a historical perspective, however, this was pioneering work.



Sullivan influenced seminal thinkers in the interpersonal field, such as Carl Rogers, Eric Fromm, and Erik Erikson, and inspired nothing less than a new direction in psychotherapy. He even prefigured family systems theory—two of the founding pioneers in family therapy, Don Jackson and Murray Bowen, trained with Sullivan. Finally, he also set the stage for a host of contemporary approaches to short-term treatment that were shaped by this interpersonal approach and its more active and engaging therapeutic stance toward the client. In sum, many of the basic concepts in the interpersonal process approach can be traced back in some form to Sullivan and other interpersonal/relational theorists.

## THE COGNITIVE DOMAIN

Approaches as seemingly diverse as object relations theory, Bowlby's attachment theory, and cognitive behavioral therapy each help us understand and intervene with the faulty thinking that is central to clients' symptoms and problems. Let's begin with object relations theory—with its remote and alienating jargon, it is usually the most difficult theory for graduate student therapists to integrate. Following this overview, we will link the attachment theorists' core concept of "internal working models" and the schema-oriented approaches of contemporary cognitive therapists—which are concerned with the client's beliefs and expectations, and how they impact affect and interpersonal relationships.

**Object Relations and Attachment Theory: Internal Working Models.** In the present context, *objects* are people or, more precisely, internal representations of important caregivers. Object relations theory is about close interpersonal relationships—especially those between parents and young children, and how those important early relationships are internalized as enduring mental representations of parent–child relations or **internal working models**. In particular, object relations theorists try to account for the meaning of attachment and the human need for secure, enduring relationships with a few significant others who can respond to our emotional needs. Object relations theorists are interested in understanding how formative interactions between parents and children become internalized by the child and, akin to cognitive schemas, serve as mental representations that shape or guide how children establish and carry out subsequent relationships with others. These representational models of self and other provide basic expectations or road maps for what will transpire in relationships (e.g., Others are dependent and needy—I must take care of them; Others are critical and overwhelming—I need to protect myself by withdrawing). Although these early Internal Working Models (IWMs) for relationships will become more complex and evolve over time, object relations theorists argue that these formative IWMs provide the basic structure for developing a sense of self, organizing the interpersonal world, and shaping subsequent relational patterns (Ogden, 2002).

Extending these constructs, attachment theorists further clarify the important role of early parent–child relationships, and the security or anxiety that children find in their parents' responses to their attachment needs (see Cassidy & Shaver, 2008). When parents accurately register and respond to their children's emotional needs, children develop attachments that are labeled *secure*. In contrast, when parents do not accurately read the child's emotional cues, or do not hear or respond to their young child's distress or bids for engagement, the child's attachment will be labeled *insecure*. Attachment researchers describe three types of insecure attachment in children: Avoidant, Ambivalent, and Disorganized.

When parents consistently ignore, dismiss, reject, or otherwise fail to console a distressed child, that child will be insecure and develop an *Avoidant* attachment style. The child learns early on to cope with the parent's unresponsiveness by developing a pseudo or false form of "independence." This counter-dependency conveys that the child has

absolutely no need of others, regardless of his or her actual experience or genuine emotional need. As an adult, this child is likely to have difficulty accessing and expressing his/her emotions and will run the risk of being isolated and emotionally disengaged from both him/herself and others.

At the other end of the continuum are the children who develop an *Ambivalent* attachment style. This occurs when children have caretakers who are intrusive, respond inconsistently, and have difficulty supporting the child's independence. Since caretakers are sometimes responsive and sometimes unresponsive because they are self-absorbed or preoccupied, the child is uncertain if the parent will be there when needed. Thus this *Ambivalently* attached child (and later, adult) is often clingy and needy, demonstrates significant worry and anxiety, and expresses contradictory or mixed messages (come close/go away) toward the parent and others. Relationships will be challenging for these children as they enter adolescence and adulthood—partners will experience them as dependent and needy, and reciprocity or mutuality will be lacking.

Finally, children who develop the *Disorganized* attachment style have no organized attachment pattern or coping style to deal with their very troubled attachment figures. They have often experienced trauma or abuse, neglect, or other confusing dissociative or frightening behavior from their parents. Hesse and Main (2000) articulate the maddening and unresolvable paradox that these children struggle with: the attachment figures who are their primary source of comfort but at other times, are hurtful and a realistic source of fear. These Disorganized children therefore have difficulty sustaining consistent relationships and are at high risk for more serious psychopathology, such as Borderline Personality Disorder. Often, because they have experienced so much invalidation and frequently been traumatized, as adults they may engage in self-destructive behaviors (e.g., binge eating and purging, substance abuse, self-mutilation, etc.) and may also have dissociative episodes. They may, in the relationships they establish, often have difficulty trusting yet also be extremely vigilant for loyalty or commitment and be highly demanding of friends and partners. In sum, each of these differing attachment histories is going to shape expectations and behavior in subsequent relationships, which, as we will see, includes clients' responses to the therapist and the therapeutic relationship.

**Internalizing the Attachment Relationship.** Object relations and attachment theorists believe that the child's primary motivation is to establish and maintain emotional ties to parental caregivers—a very different approach from that used by psychoanalytic and behavioral theorists (Eagle & Wolitzky, 2009). Parental caregivers are important in that these attachment figures are a source of comfort and protection. From their point of view, the greatest conflicts in life are threats to, or disruptions of, these basic attachment ties (separation anxieties or abandonment fears). In this regard, anxiety is a *signal* that emotional ties to caregivers are being threatened. If parents are **attuned** and consistently responsive to the child's distress, the child becomes secure in those attachment ties (parents don't need to do this perfectly—they just have to be “good enough”).

What's so interesting here is that through the course of development, these secure children increasingly “internalize” their parents' emotional availability and responsiveness and come to hold the same constant or dependable loving feeling toward themselves that their parents originally held toward them. Said differently, cognitive development increasingly allows securely attached children to internally hold a mental representation of their emotionally responsive parents when the attachment figures are away and they can increasingly soothe themselves as their caregivers have done—facilitating the child's own capacity for affect regulation and independent functioning. Thus, as these children mature cognitively and emotionally, they become increasingly able to soothe themselves when distressed, function for increasingly longer periods without emotional refueling,

effectively elicit appropriate help or support when necessary, and become the source of their own self-esteem. As adults, they are more likely to be able to recreate their internal working models in subsequent real-life relationships—such as successfully choosing similarly responsive and supportive partners and enjoying a better marriage (Hinnen, Sanderman & Sprangers, 2009).

But what if the child cannot maintain secure emotional ties to the primary caregivers? In some families, parents do not respond well to the child's distress or attachment need because they are anxiously preoccupied, depressed or withdrawn, or even abusive and frightening. When attachment needs are compromised by caregivers who are unreliable or unresponsive, these children are trapped in an unsolvable dilemma. They are unable to elicit help from their caregivers when they are distressed. What do they do with the anguish this creates? Object relations theory tries to account for the different compromise solutions and coping adaptations these children must make to this poignant human dilemma. They may, for example: 1) become anxious and overly preoccupied with their caregiver (Ambivalent Attachment); 2) distort the reality of their circumstances by denying certain needs and feelings of their own, or that reality-based problems with others really do exist, and idealize others in unrealistic, problem-free ways (Avoidant Attachment); or 3) exhibit a confusing combination of both (Disorganized Attachment). Below, we illustrate how an Avoidantly Attached child is likely to present in treatment as an adult.

*Alert and friendly, Bob entered treatment telling his therapist that he didn't have any real problems—things were “great,” but his fiancée wasn't ready to go forward with their wedding plans unless he talked with a therapist about his “short fuse” and a recent DUI. As the therapist explored these symptoms and tried to learn more about them, Bob gave very general explanations that minimized the significance of his alcohol-related problems and his temper outbursts—trying to convey that things really were OK by repeatedly using the phrase “No problem.” Looking for a way to move their dialogue beyond the surface, the therapist suggested that, because Bob was thinking about getting married and starting a family of his own, it would be helpful if they could talk about his own childhood and get a feel for his parents' marriage and what it was like growing up in his family. Bob gradually unfolded a narrative that included a threatening and explosive father who yelled and hit his children when he drank, and a mother who provided little protection from this— or even acknowledged that these frightening and humiliating interactions were occurring. Typical of an Avoidant Attachment style (reabeled Dismissive in adulthood), Bob described these experiences without much feeling, and minimized these disturbing family interactions as insignificant. They were “no big deal” because he had a “close and loving family” and “everybody came out great anyway.”*

Next, let's describe and briefly illustrate the Disorganized or maltreated child. When caregivers are more severely unresponsive or abusive, some children resort to “splitting defenses” to maintain ties to the fear-provoking caregiver. The child internalizes the “bad” (threatening or rejecting) aspects of the parent in a sharply separate or split-off way from the “good” (loving or responsive) aspects of the parent. This preserves the necessary image of an idealized, “all good,” responsive caregiver with whom the child is internally connected or attached. However, the price is high in that reality is distorted, the self lacks coherence, and the child comes to believe that he or she is the one who is “bad.” The frightening or maltreating caregiver is no longer “bad,” allowing the child to cope by viewing the external world and the attachment figure as safe (Fonagy, 2000). The child is left believing that, if only he or she were different or better in some way, parental love would be forthcoming (Benjamin, 2003).

This situation is illustrated most poignantly by the physically abused child who, though repeatedly hit and afraid, nevertheless continues to idealize and defend the maltreating parent. For example, a child does this by believing that he or she is bad and

maintaining that the punishment is actually justified (e.g., seven-year-old child: “My dad hit me because I wasn’t being nice to my brother.”). This self-blaming distortion enables the child to cope with the attachment dilemma (needing care from a hurtful or frightening person) and believe that he or she has some control over events and is not helpless. Early in their training, many new therapists will have clients in their caseloads who were physically mistreated, sexually violated, or rejected and held in contempt at times by attachment figures. New therapists will often be surprised to observe that these clients commonly blame themselves for their mistreatment and unrealistically assume responsibility for what went wrong.

Understanding that the child must find some way to maintain ties with a good, loving parent helps therapists appreciate why many clients persist in maladaptive behavior and self-defeating relationships. New therapists sometimes will observe that when clients resolve a symptom, succeed in achieving a personal goal, or make healthier choices for themselves, some will feel anxious or distressed and may undo their progress. Thus, loyalty and allegiance to symptoms—maladaptive behaviors originally developed to manage the “bad” or painfully frustrating aspects of parents—are not maladaptive to insecurely attached children. Such loyalty preserves “object ties,” or the connection to the good or loving aspects of the parent. Attachment fears of being left alone, helpless, or unwanted can be activated if clients disengage from the symptoms that represent these internalized “bad” objects (e.g., if the client resolves an eating disorder or terminates a problematic relationship with a controlling/jealous partner). The goal of the interpersonal process approach is to help clients modify these internal working models by providing them with experiential or in vivo **relearning** (i.e., a corrective emotional experience). Through this real-life experience with the therapist, clients learn that, at least sometimes, some relationships can be different and do not have to follow the same familiar but problematic lines they have come to expect.

**Cognitive Behavioral Therapy: Early Maladaptive Schemas.** In his early pioneering work, Beck (1967) emphasized the importance of schemas in depression. For Beck, schemas are the basic components of cognitive organization—the fixed or patterned ways we interpret, categorize, and evaluate our experiences. Put most simply, schemas can be thought of as broad organizing principles for making sense of one’s life or experience. Beck emphasized that repetitive themes occur in clients’ thinking through “schema bias”—a consistent and selective bias in organizing information and interpreting events that results in “the typical misconceptions, distorted attitudes, invalid premises, and unrealistic goals and expectations” that clients present (1967, p. 284). Cognitive behavioral therapy originally emphasized three aspects of cognition—automatic thoughts, cognitive distortions, and underlying assumptions.

More recent theorists expand Beck’s groundbreaking work and further emphasize the child’s developmental experiences with caregivers in shaping affectively charged “**Early Maladaptive Schemas**” (Young et al., 2003).

In his rich and substantive approach, Jeffrey Young integrates cognitive behavioral therapy with object relations and psychodynamic approaches, and places more emphasis on the therapeutic relationship, affective experience, and early life experiences. Following Young (1999), Early Maladaptive Schemas (EMSs) are defined as stable and enduring themes that develop from ongoing patterns of parent–child interaction during childhood that are significantly dysfunctional, and which are elaborated and extend on into adulthood. These schemas serve as templates for the filtering of later experience, and are taken for granted by individuals as *a priori* truths. They feel familiar and are not questioned by the individual—they are just the givens of one’s existence and form the core of a client’s self-concept and conception of the environment. EMSs are self-perpetuating and, unfortunately, resistant to change because the individual distorts information to maintain the

validity of these schemas. This occurs, for example, as the client magnifies information that confirms the schema and minimizes or negates information that is inconsistent with the schema.

For example, a husband with a Rejection/Shame schema focuses on the one complaint that his wife expressed toward him but does not really hear the genuine appreciation she also voices. Clarifying further how EMSs selectively filter for corroborating experience, another client with a Rejection/Shame schema may instead select a mate or spouse who repeatedly confirms his or her self-perception as inadequate and shame-worthy. Strong feelings accompany these EMSs, and intense anxiety (or other feelings) will be activated when life situations trigger the client's EMS. Similarly, imagine a graduate student with a Failure schema, which developed from repeated interactions with highly critical caregivers who were, in reality, very difficult to please. She is likely to worry and experience significant anxiety as comprehensive exams, dissertation defense, applications/interviews for job or internship placements, and other evaluation/performance situations that activate her Failure schema loom on the horizon.

The cardinal issue for us here, which will be emphasized throughout the interpersonal process approach, is that it is important to discuss schemas or misperceptions *in the moment, when they are being played out or experienced in response to the therapist*, and are accompanied by strong affect. For example, Aaron grew up with drug-involved parents who often were away and, when they were home and physically present, were often altered and emotionally absent. At age eight, Aaron was placed in a series of three foster homes by Child Protective Services until he was finally adopted and settled into a stable, predictable life with a supportive family. At the core of many issues that Aaron is going to be struggling with in his life will be an Abandonment schema. Aaron is vulnerable or prone to feel anxious and depressed and struggle with other familiar symptoms that will disrupt his life, particularly at times when separation experiences are occurring, such as when his girlfriend suggests that she wants to date others or break up, or when his daughter heads off to the first grade or college.

Similarly, feelings of emptiness, loneliness, and despair ("Abandonment feelings") may be evoked toward the therapist at critical times, such as at the end of an especially powerful session, when the therapist is preoccupied about a personal problem in his own life, just before the therapist leaves for vacation, when they discuss termination, and so forth. The key refrain here and in the chapters ahead is this: The real opportunity to help clients change is when the problem they are struggling with is occurring *between therapist and client*—not just being talked about in the abstract. This *immediacy* is anxiety-arousing for new therapists, for sure, but also the potent place where we have the chance to make a real difference in the client's life.

In sum, we can see that theoretical approaches as disparate as object relations, attachment theory, and cognitive therapy can all teach us much about the schemas or internal working models that shape clients' lives. As we go on, we will explore further how these faulty beliefs and problematic expectations, which once made sense and accurately fit what really occurred in formative family relationships, are overgeneralized now and no longer serve clients well in most current relationships.

## THE FAMILIAL/CULTURAL DOMAIN

Children must adapt to their attachment figures, and they do so within a familial and cultural context. Family systems theory offers therapists an invaluable way of understanding their clients' strengths and problems—clarifying the familial rules, roles, myths, communication patterns, and boundary issues that defined their clients' development. Cultural norms and values are imparted through daily family interaction and, in this and other



ways, we develop our identity or “self” and become who we are in our families of origin. In particular, family process researchers have learned much about family interaction and relationships by adopting a multigenerational perspective.

**Family Systems Constructs.** In the late 1950s, Gregory Bateson, Virginia Satir, Murray Bowen, and other pioneers began studying communication patterns in families (Goldenberg & Goldenberg, 2012). They found that communication patterns in families followed definitive but often unspoken rules that determined who spoke to whom, about what, and when. These covert or *unspoken rules* also governed affective expression in the family, such that for certain family members, being sad or disappointed, irritated or angry, or even excited and happy, may not be allowed. Years later, these unspoken family rules about communication or expressing certain feelings are still in play for most clients and contribute to their problems in current adult relationships. For example, clients often continue faulty communication patterns they learned years before in their families of origin, as they:

- *address conflicts through a third party rather than speaking directly to the person involved (e.g., Father: Tell your Mother that I’m sick and tired of...);*
- *allow others to speak for them and define what they are thinking or feeling (Parent: “No, that’s not what she really means...”); and*
- *never make “I” statements and communicate directly what they want (e.g., “You never give me a chance to say what I think” versus “I would like to complete my thought”).*

Early family researchers also illuminated how children in dysfunctional families are scripted into narrow roles, such as the responsible or good child; the problem or bad child; and other common roles—including the family hero or star, the invisible or lost child, and the rescuer or peacemaker. Following object relations theory, the most common configuration is the good child–bad child role split that occurs in so many dysfunctional families. Some therapists were the “good child” in their families of origin and had a sibling who fulfilled the “bad child” role and seemingly couldn’t do anything right or was always viewed as the problem—independent of actual behavior. Especially apt for therapists-in-training, many therapists, and others who enter the helping professions such as nurses or ministers, were **parentified**. In this reversal of parent–child roles, they fulfilled a caretaking role for their parent. That is, they often were scripted to meet the emotional needs of their parent, rather than having their own age-appropriate needs met, by serving as a confidant or best friend to their parent. This might have included trying to protect their parent or shore up the emotional needs of an anxious, depressed, dependent, alcoholic, or otherwise vulnerable caregiver. For others who choose counseling careers, they may have been assigned to the role of mediator or go-between in family or parental conflicts.

Why is this family systems perspective so important to new trainees? Most clients are still fulfilling these limiting childhood roles, which deny many aspects of who they are and hold them back from becoming more of who and how they want to be in life. Thus, in the process of adapting in their families of origin, these children “internalized” these roles as self-schemas. These problematic familial roles have become their identity—who they are and what they do. And, even though their parents now may live far away or even be deceased, they continue to place their families’ original expectations on themselves and recreate these problematic roles and relational patterns with others in their current lives. For example, many clients who have played the good-child role in their families are still making perfectionistic demands of themselves, feeling guilty about doing things for

themselves, worrying excessively about others' needs, fearing their appropriate assertiveness and anger, and being confused by their sad and empty feelings. The “bad child,” on the other hand, may be defiant and act out personal conflicts externally. He or she may, for example, abuse substances, be promiscuous, or behave in other rebellious ways. In contrast, the “good child” is more likely to seek treatment and enter therapy, often presenting with vague complaints of guilt, anxiety, and an inability to make decisions or know one's own mind.

There is cultural variation in the balance of “separateness/relatedness” or the degree of continuing responsibility that young adult offspring are expected to hold for their parents versus pursuing their own adult interests and needs. Within these varying cultural contexts, many families also have unspoken rules about how, in late adolescence, offspring can leave home or **individuate** and further establish their own beliefs and values, work and careers, and marriage and families. For example, the oldest daughter may not be allowed to grow up and leave home successfully on her own. She may seek emancipation through pregnancy, only to find herself even more dependent on her parents than before and forced to live at home again. A son may only be able to leave home through rejecting confrontations with parents, and then may live in another part of the country and have little or no continuing contact with his family. However, in better-functioning families, young adults find support and guidance for launching their own lives, while continuing to maintain strong family ties and connections. Additionally, family rules, roles, and faulty communication patterns often serve to maintain family myths that, in turn, function to avoid anxiety-arousing issues in the family (Satir & Bitter, 2000). For example, common family myths include:

- *Dad doesn't have a drinking problem.*
- *We are a happy family and nobody is ever sad.*
- *Mom and Dad never fight, and are very happily married.*

In sum, family myths and these other family characteristics are rule-bound **homeostatic mechanisms** that govern family relations and establish repetitive, predictable patterns of family interaction. They are homeostatic in the sense that they tend to maintain the stability of the family system—often making change uncomfortable or even threatening.

**The Cultural Context.** Salvatore Minuchin and other family researchers have also explored the alliances, coalitions, and subgroups that make up the structure of family relations (Minuchin & Nichols, 1998; Minuchin, Lee & Simon, 1996; Minuchin, Nichols & Lee, 2007). In some families, for example, the maternal grandmother, mother, and eldest daughter are allied together, and the father is the outsider. This structural road map for reading family relations becomes even more illuminating when these family dynamics are examined in a three-generational perspective (Bowen, 1993; Kerr & Bowen, 1988). It is fascinating—and sometimes disturbing—to draw and see how these same family rules, roles, myths, and structural relationships can be reenacted across three or four generations in a highly patterned, rule-governed system (McGoldrick, Gerson & Petry, 2008). Adding to the complexity of this tapestry, these family rules, roles, and communication patterns operate within a broader cultural context. For example, the oldest son in a traditional Chinese family or the oldest daughter in a traditional Latino family may be expected to assume significant, ongoing responsibilities for elders or other family members. It will be necessary for therapists to establish the extent to which these roles and behavior patterns are culturally sanctioned or determined. New therapists will see many clients in therapy who are struggling with balancing their own needs versus the needs of their families of origin. Therapists help by working with clients to identify healthy role models within their extended family or cultural context who have achieved a better balance between their own personal needs versus obligations and loyalty to the family.

To help, Sue and Sue(2012) describe cultural competence in working with clients as including:

- (1) *the therapist's awareness of his/her assumptions, values, and biases;*
- (2) *the therapist's level of knowledge regarding the worldview of culturally different clients; and*
- (3) *the therapist's skill in developing interventions that are culturally sensitive.*

Sue and Zane (2009) suggest that one key consideration in working with clients is “achieved credibility”—the skills exhibited by the therapist that garner the client’s sense of hope and confidence, which would include working collaboratively, actively engaging the client, and having the ability to clarify what the client’s core concerns really are (Baldwin et al., 2007). Thus, knowledge of the client’s culture is used in the service of developing credibility; for example, using language that resonates with the client’s experience is associated with better treatment outcomes (Draguns, 2007). In this way, the client’s cultural features or cultural group do not define the client but alert the therapist to potential credibility issues that can arise. Pedersen et al.’s (2008) informative book, *Inclusive Cultural Empathy*, provides student therapists with helpful exercises to increase self-awareness and sensitivity to a range of cultural issues.

The interpersonal process approach draws on these basic family systems and inclusive cultural concepts, and considers the effects of familial experience and contextual factors on clients’ personality strengths and problems. Therapists want to help clients make realistic assessments of the resiliencies and vulnerabilities that coexisted in their families of origin. However, for many clients and for many therapists, it is culturally taboo to speak critically of parents or talk about problems with someone outside of the family. Alice Miller (1984) observes how, for some, this even breaks the Fourth Commandment: Honor thy parents. Herein, though, is one of the great strengths of family systems theory. The family systems therapist is *trying to understand hurtful familial interactions, not to place blame or scapegoat any family member*. The family therapist is concerned about the well-being of every family member, parents as well as children. In parallel, only as clients realize that their individual therapist wants to understand—rather than to blame their caregiver or make them bad in some way—do clients have the safety they need to explore threatening material and make significant gains in treatment. At times, however, therapists’ own splitting defenses or dichotomous thinking leads them to blame or reject parental figures. For example,

**INEFFECTIVE THERAPIST:** I can’t believe your mother did that to you, she’s awful!

Out of guilt and threat to ties with their caregivers, it will be far more difficult for clients with this type of therapist to make progress in treatment. In contrast, it will be far more effective if therapists eschew blame and try only to understand and empathize with their clients’ experience. For example,

**EFFECTIVE THERAPIST:** I’m sorry you were hurt so much when she did that.

There are some good things in almost every family, even in very troubled or abusive families, just as there are limitations and problems in the healthiest families. When clients continue to idealize parents and deny real problems that existed, as many do when they enter treatment, they are complying with binding family rules and protecting their parents at their own expense. On the other hand, however, clients cannot simply reject hurtful parents and emotionally cut off from them, because they will simultaneously want to remain connected to the healthier aspects of their caregivers. This means that therapists need to help clients find ways to keep caregivers (or the healthier, more benevolent parts of them) alive inside themselves as “partial identifications.” How can clients accomplish this?

In order to resolve problems and make progress in counseling, clients want to be able to come to terms with both the good news and the bad news in their families of origin. As already noted, however, clients cannot achieve this integration if the therapist also employs his or her own splitting defenses or dichotomous thinking. In other words, the therapist does not want to identify with or idealize the wounded child that exists within some clients and to reject the hurtful parent as “bad.” Neither does the therapist want to support clients’ continuing denial of familial problems and idealization of parents. Instead, the therapist’s appropriate role is to try to understand what actually occurred in clients’ development and help them realistically come to terms with both the good news and the bad in their experience.

Family systems concepts and other cultural sensibilities inform the interpersonal process approach. In particular, they help us identify the problematic interaction patterns that clients are enacting with the therapist and others, and help both therapists and clients understand the familial and cultural context that shaped many current problems. This awareness empowers clients to stop reenacting problematic familial roles with others in their current lives without breaking off important, but conflicted, familial relationships.

**Developmental Experience Helps Therapists Understand Clients.** Let’s pause here for a moment to appreciate two reasons why familial experience has such a profound, lifelong impact on the individual. First, the same types of affect-laden interchanges are reenacted thousands of times in daily family life. To illustrate, suppose a caregiver has difficulty responding positively to her child’s success experiences because of her own depression, envy, or competitiveness. When the young child enthusiastically seeks this caregiver’s approval for an accomplishment, the caregiver may ignore the child, change the topic, compare the child’s achievement unfavorably with a sibling’s greater accomplishment, turn away and look vaguely sad or hurt, or take the success away from the child by making it her own. The same type of caregiver response usually occurs whenever the child enthusiastically shows the caregiver a drawing, makes a new friend, wins a race at school, or earns a star from her teacher. This transactional pattern becomes a powerful source of learning when it continues over a period of years or even decades. As an adult, this child is likely to feel conflicted when, for example, she completes her degree, takes pleasure in a promotion at work, or even enjoys her marriage or has good friends who are appreciating her. In this way, the most significant and enduring problems in people’s lives develop from these habitual response patterns. Contrary to popular belief and portrayals in movies, long-standing symptoms and problems are shaped far more by repetitive family transactional patterns (*strain* trauma) than by isolated traumatic events (*shock* trauma) (Kerig & Wenar, 2005). This represents a paradigm shift for many therapists-in-training, who are beginning to learn to listen for patterns and themes in daily interaction, rather than keying on crisis events.

Second, the impact of these repetitive transactions is magnified because of the intensity of the feelings involved. Attachment figures are the pillars of the child’s universe, and children depend on them with a life-and-death intensity. These repetitive transactional patterns have been reenacted in highly charged, affective relationships with the most important people in one’s life. The child in the preceding example may well feel a desperate need to find some way to try and win the caregivers’ approval yet simultaneously feel increasing anxiety about trying to succeed or approaching the non-affirming caregiver. Thus, our sense of self in relation to others is learned in the family of origin and, in many ways, will carry over to adulthood.

In sum, family interaction patterns may be hurtful and frustrating, validating and encouraging, or, commonly, a mixture of the two. For better or worse, these repetitive patterns of family interaction, roles, and relationships are internalized and become the

foundation of our sense of self and the social world. Of course, other factors are influential as well. Familial relationships are molded within a broader cultural context that can affirm or repudiate caregivers' responses.

Familial experience provides our first, most long-lasting, and affectively charged model for what goes on in close relationships. It will figure significantly in the individual's choice of marital partner and career, in how adult offspring will, in turn, raise their own children, and in many of the other enduring problems and satisfactions of adult life. Although family-of-origin influences are profound, problematic rules, roles, and communication patterns can be relearned. Change is indeed possible, although often not easy, and it occurs in part through the relational process of *experiential relearning*, which we have begun to consider.

## CLOSING

Student therapists reading this book need to have realistic expectations for themselves. New therapists who are seeing their first clients are going to find many practical guidelines to help them in their initial work with clients. However, more information is presented here than a new clinician can fully integrate and apply. The concepts presented in the interpersonal process approach are easy to say but challenging to master. New therapists will be able to incorporate these concepts into their practice—but are encouraged to do so gradually, and at their own pace. Especially during their first year of seeing clients, some may begin to feel that the more they learn, the less they know. With more experience and confidence, however, second-year students often begin to find that they are effectively employing many of these concepts with their clients. Typically, trainees are able to make these process-oriented concepts their own and apply them easily and naturally with their clients, in two or three years. Because it is going to take some time to learn how to work in this way, the best approach is to be patient with yourself and enjoy the learning.

In closing, new therapists are encouraged to *be themselves* with clients rather than trying to fulfill the role of a therapist. Perhaps Kahn says it best:

*When all is said and done, nothing in our work may be more important than our willingness to bring as much of ourselves as possible to the therapeutic session.... One of the great satisfactions of this work comes at the moment students realize that when they enter the consulting room, they don't need to don a therapist mask, a therapist voice, a therapist posture, and a therapist vocabulary. They can discard those accouterments because they have much, much more than that to give their clients (1997, p. 163).*

We turn next to beginning the initial session and using empathic understanding to establish a working alliance.

## CHAPTER EXERCISE

We invite you to write out your response to the following Self-Reflection question: As you begin seeing clients, you may be concerned about your ability to help others (e.g., "I'm worried that I don't know enough to help my clients"). At this point in your clinical training, what is the primary source of anxiety evoked in you by seeing clients? Try to be specific about your concerns. Are you, for example, not sure how to proceed or what your role is in the counseling session? Are you concerned that your own unresolved issues may interfere with your ability to be helpful, you will "take your client's problems home with you," or be too affected by their feelings? Are you worried about somehow doing something wrong that will hurt your client or of making "mistakes" and disappointing your supervisor?



## SUGGESTED READING

Student therapists begin their clinical training with different levels of experience. Less experienced readers may wish to read Clara Hill's (2014) text, *Helping Skills: Facilitating Exploration, Insight and Action*, 4/e. This practical text provides new therapists with the basic helping skills they need for responding to their clients, such as active listening, reflecting, exploration, reframing, communicating empathy and so forth. See especially Chapters 6–14.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes. Also included with this chapter is a video demonstration by the authors showing how therapists can make process comments effectively and role-play a supervision session with a new therapist struggling with performance anxieties.

# PART II

## RESPONDING TO CLIENTS

### CHAPTER TWO

Establishing a Working Alliance

### CHAPTER THREE

Honoring the Client's Resistance

### CHAPTER FOUR

An Internal Focus for Change

### CHAPTER FIVE

Helping Clients with Their Feelings





## CHAPTER 2

# Establishing a Working Alliance

### CONCEPTUAL OVERVIEW

Therapy is a profession based on trust. Clients enter treatment with a need: They are often in pain and asking for help with something they have not been able to resolve on their own. A trustworthy response is to respect clients' requests for help by responding nonjudgmentally—with compassion and empathic understanding—to their concerns. Yet to be most effective, the therapist also wants to respond in a way that helps clients achieve a greater sense of their own agency or voice. That is, the goal is not only to resolve specific situational problems but to do so in a way that leaves clients empowered with a greater sense of their own ability to manage the situations and stressors that are likely to cause problems in the future. Following Erik Erikson (1968), crisis provides an opportunity for growth. The Interpersonal Process approach aims to help clients resolve their presenting problems in a way that also leaves them with a greater sense of their own personal power—reducing their vulnerability to anxiety and depression.

Clients cannot attain these twin goals of resolving problems and achieving a greater sense of **self-efficacy** in a hierarchical or one-up/one-down therapeutic relationship. To become empowered, *clients need to share ownership of the change process* and be active participants who work collaboratively with the therapist, rather than being passively “cured” or told what to do. Clients need help from a responsive and actively engaged ally. Thus, this chapter presents a model for a collaborative relationship or **Working Alliance** that accepts the client's need for understanding and guidance while equally encouraging the client's own initiative and responsibility. As highlighted by the work on Motivational Interviewing (see Miller & Rose, 2009) and the extensive research on the working alliance (Baldwin et al., 2009), a collaborative engagement between therapist and client is an essential component of therapy—contributing to the client's motivation and commitment to change and enhancing the overall effectiveness of treatment.

Let's look at what researchers find predicts successful psychotherapy outcome. Although researchers have tried long and hard, they have not been able to find consistent empirical support for the long-term superiority of any one treatment approach over another (Lambert & Barley, 2002; Norcross, 2010; Norcross & Lambert, 2014). In contrast, researchers consistently have found *strong evidence of great variability in the*

*effectiveness of individual therapists within each treatment approach* (Wampold, 2011). That is, greater differences in treatment effectiveness are found between therapists of the same theoretical orientation (within-group differences) than between groups of differing orientation (e.g., interpersonal versus cognitive behavioral orientations). Arguably, the strongest finding in the psychotherapy outcome literature is that the most common feature of these effective therapists across differing theoretical approaches is the therapist's ability to establish a strong working alliance early in treatment (Safran et al., 2009; Horvath & Bedi, 2002). Indeed, even when manual-based treatments are used and therapists are supposedly doing the exact same things with their clients, therapist effects on outcome are large (Malik et al., 2003). Thus, this chapter explores how therapists can use empathic understanding to help establish a strong working alliance, and then use process comments to restore it when the misunderstandings and problems that inevitably occur rupture the alliance.

## THE WORKING ALLIANCE

At each successive stage of treatment, therapists have a different overarching goal to guide their interventions. In the first stage, the therapist's principal goal is to establish a working alliance with the client. A working alliance is established when clients perceive the therapist as a capable and trustworthy ally in their personal struggles—someone who is interested in, and capable of, helping them with their problems. To achieve this credibility with clients and become someone who matters to them, the therapist's intention is to consistently communicate that he or she has empathy (Nienhus, 2014) and:

- *grasps their predicament and recognizes their distress;*
- *feels with them and is empathic to their pain;*
- *is an ally on their side who has their best interests at heart;*
- *has an abiding commitment to help them through this predicament.*

The concept of the working alliance originally was developed by Ralph Greenson (1967). Subsequently, Bordin (1979) and others clarified that there are three separate but interrelated components in all therapeutic relationships: the **Real Relationship** (Gelso, 2011); **Transference** and **Countertransference**; and if that can be done easily, the **Working Alliance that impact outcome** (Friedlander et al., 2011; Shirk & Carver, 2011).

Researchers have defined the working alliance as a collaborative process whereby both client and therapist:

- (1) agree on what's wrong—what the problem is, and share the same therapeutic goals;
- (2) collaborate and agree about the ways to go about meeting these goals; and
- (3) develop a collaborative relationship based on trust, acceptance, and competence.

Of these three components, Horvath (2006) reports that the relationship component—connection or feeling between the therapist and client—is most important and accounts for most of the variance in treatment outcome. In addition to the working alliance, researchers also find strong evidence for the impact of therapeutic presence and a collaborative relationship on treatment outcome (Fluckiger et al., 2012; Greenberg, 2014; Hersoug, Hoglend, Gabbard & Lorentzen, 2013; Horvath et al., 2011). The therapist's ability to establish a successful working alliance in the initial sessions has emerged as a strong predictor of effective treatment outcomes in both short- and longer-term treatment. A robust ingredient common to all psychotherapies, the working alliance is correlated with successful outcomes across a wide range of clients' symptoms and problems, and across differing approaches and theoretical orientations (Horvath & Bedi, 2002;



Horvath, Del Re, Fluckiger & Symonds, 2011). Although present in some form in almost every theoretical orientation, the working alliance is most closely linked to Carl Rogers's core conditions of genuineness or congruence, positive regard or acceptance, and especially, accurate empathy (Rogers, 1957).

A strong working alliance that predicts successful treatment outcomes develops only when the client feels safe with, and understood by, the therapist. Clients develop this sense of security when therapists are emotionally present and empathically engaged, and respond genuinely to clients in non-judgmental and validating ways (Geller & Greenberg, 2012). New therapists' ability to develop and apply these relational skills with their clients will shape their effectiveness more than the theoretical orientation they eventually adopt (Wampold, 2011). In the next section, and throughout the chapters ahead, we will try to clarify and illustrate how therapists can apply these constructs in the hour with their clients. Let's turn now to collaboration: long established as an important aspect of therapeutic relationships (Horvath & Greenberg, 1994b).

### COLLABORATION: AN ALTERNATIVE TO DIRECTIVE AND NON-DIRECTIVE STYLES

One of the best ways for therapists to establish a strong working alliance with clients is to collaborate with them. In the initial sessions, the therapist's primary aim is to give clients the *experience* of working together in partnership. If a collaborative relationship is maintained throughout treatment, this interpersonal process will go a long way toward our twin goals of helping clients resolve their presenting problems and achieve a greater sense of their own self-efficacy. Thinking of the working alliance as a collaborative partnership, therapy is not something therapists "do" to clients, it is a shared interaction that requires the participation of both parties in order to succeed (Aron, 1996). In their Motivational Interviewing approach, Miller and Rollnick (2013) contrasted two counseling styles. Counselors who acted as experts focused on persuading clients to change, and actively confronted clients' reluctance to change, actually increased clients' resistance to change. In contrast, collaborative counselors who focused instead on understanding clients' perspectives through empathy, drawing out and evoking the clients' own concerns or motivations to change, and then joined clients in exploring actions they might want to take, had better outcomes.

How does a beginning therapist go about fostering this collaborative process? At the outset, the therapist can ask clients what they know about counseling, explore their expectations of the therapist and the treatment process, and invite their thoughts and suggestions about how the therapist and client could best work together. When clients have been in treatment before, it is necessary to ask what was helpful and what wasn't. This is especially important if clients previously dropped out of treatment prematurely, felt they didn't change, or experienced problems or difficulties with the therapist that were not discussed or resolved. When therapists ask clients why they dropped out or what they didn't like about previous treatment, they often respond in one of two ways:

**CLIENT:** She was a nice person and I liked going to see her in the beginning. But she didn't say very much—she was pretty quiet and I never really knew what she was thinking. I guess I needed more input or feedback because, after a while, I realized that not much was changing for me. I did feel kind of bad about stopping though.

OR

**CLIENT:** The therapist didn't listen to me very well. Sometimes it almost seemed like he wanted to start telling me what to do before he even really heard what was wrong. I just stopped feeling like going.

Let's look at how both of these directive and non-directive styles often fail. A widely held misconception among clients is that the therapist is a doctor who will prescribe their route to mental health. Often, the therapist is perceived as simply telling the client what to do. Instead, it is the therapist's job to help clients understand the therapy process and actively engage clients so they become partners or collaborators. The research supports this collaborative stance and finds better client outcomes when working with therapists who promote rapport, instill hope, and encourage **change talk** (i.e., who are collaborative and are more active but not directive) (Miller & Rollnick 2013; Miller & Rose, 2009; Moyers et al., 2007). Therapists can engage the client collaboratively by assessing together:

- *the issues and concerns that are most important—what's most painful or distressing right now;*
- *what the client and others have done in the past that has been helpful and what has not been helpful;*
- *shared treatment goals—brainstorming and sorting through together what's really wrong, what they would like to be able to change, and how they can best work toward those goals.*

Therapists may also explain that, even though the client has not been able to change on his or her own, the therapist will be an active partner and the client's willingness to work with the therapist to understand problems and explore solutions will be essential. Unless the therapist and client discuss how they will work together, a hierarchical doctor-patient or teacher-student relationship is more likely to develop, which undermines the client's self-efficacy and fosters dependency. As part of educating the client about the treatment process, especially for clients from a family or culture that is hierarchically structured, actively inviting partnership and collaboration becomes an even more important treatment process. For example,

**THERAPIST:** Serena, tell me your thoughts and concerns about both sides of this choice. What is the good news and bad news if you decide to stay with him, and what are the advantages and disadvantages for you if you choose to leave?

A one-up/one-down relationship brings with it a number of problems. As we would expect from client response specificity, a doctor-patient relationship may work well with some clients in the short run. Based on early maladaptive schemas and developmental experiences, many clients who enter treatment believe they have to be *compliant* in close relationships. Regardless of what the therapist says about collaborative relationships, these clients still believe that the therapist, like others in their lives (such as Serena's current boyfriend), actually wants them to just "go along" and follow the therapist's lead. Such clients will passively follow the therapist early in treatment and, commonly, even go so far as to actively seek direction from the therapist (e.g., Client: "What do you think I should do?"). However, this compliance will ultimately evoke shame over being controlled and anger toward the therapist, just as it does in other relationships. Although most clients will not recognize or understand what is transpiring, this faulty interpersonal process will prevent them from utilizing the therapist's help, making progress on their problems, or even remaining in treatment. Even if clients don't have **compliance issues**, they remain dependent on the therapist if this hierarchical mode is enacted. They will not be able to gain a greater sense of their own personal power as long as they hold the dysfunctional belief that the source of adequacy or responsibility for success resides with the therapist, rather than in themselves. Let's explore this further.

So, if the interpersonal process approach isn't directive, is it non-directive instead? No. Many clients bring cognitive schemas to the treatment setting that do not encompass collaborative relationships or mutuality, and these clients may insist that the therapist assume a directive or leadership role in the beginning. For example, if clients bring to therapy an authoritarian upbringing and hierarchical schemas for relationships, then it would be fruitless to insist that they go along with a more egalitarian process. They have not experienced this in the past, and many have been punished for acting stronger, speaking up and having their own mind, or being more assertive and independent. Thus the therapist can be flexible, meet the client on his or her terms, and accept the client's request to provide more direction or advice—sometimes this is all the client can do in the beginning. At the same time, however, *the therapist can start the change process by giving this client overtly spoken permission to initiate, and by enthusiastically joining this client whenever he or she does speak up or initiate.* For example,

**THERAPIST:** That sounds like an important issue for us to work with, Sue, I like what you're bringing up. What's the best way for us to go further with this—what comes to mind as you bring this up?

The therapist can also find others ways to actively engage the client in a more collaborative treatment process. For example,

**THERAPIST:** Tell me about your strengths and successes, John. When you've had this problem in the past, what have you done, or how have others responded, that was helpful?

In particular, the therapist can also change this unwanted, hierarchical relational pattern that is causing problems with others *by using process comments to begin talking with the client about how they are interacting together.* For example, if the client seems to be rejecting the advice and direction he has just elicited from the therapist—as often occurs—the therapist can begin to talk with the client about their current interaction or interpersonal process:

**THERAPIST:** I'm a little confused here, John, let's try to figure this out together. You keep asking me what to do, but, whenever I suggest something, you say "Yes, but..." I'm wondering if one part of you feels like you are supposed to go along with what I say, but another healthier part of you doesn't want to be told what to do. What do you think might be going on between us here?

Whereas a directive stance may work in the short run, a purely non-directive approach will often sputter right from the start. Understandably, most clients feel frustrated when their requests for help or direction are merely reflected back (the "hot potato" game). A negative cycle may ensue: The client becomes increasingly frustrated and seeks some direction from the therapist, who may further eschew this role and talk about inner direction and finding one's own answers. This, in turn, further frustrates the client, who may not feel that he or she has the answer to anything at that moment, and only sees the therapist's attempts to be non-directive as evasive.

Many clients drop out of therapy prematurely because therapists were too quiet, hesitant to say what they were observing, share what they were thinking, ask further about what seemed important or didn't quite make sense to them, and so forth. As we saw in Chapter 1, therapists' concerns about making mistakes often inhibit them in this way and prevent them from becoming more actively involved and responsive—as most clients need and want. Therapists need to be an engaging presence in the relationship—so clients can feel they have a helpful collaborator with them in the room. If therapists want to

adopt an *active stance* toward the client, and not be passive or inhibited in the therapeutic relationship, how can they do so without taking over and being directive? To illustrate this active but not directive stance, therapists can respond in these ways:

- *Provide feedback about the relational or cognitive patterns they observe (for example: “As I listen to you, it often sounds as if you are working hard to take care of everybody else. That leaves me wondering who meets your needs?”);*
- *Help clients consider alternative frames of reference and consider situations from new perspectives that expand their schemas (for example: “I can see why you didn’t like that—what do you think her intentions might have been when she said that to you?”);*
- *Offer empathic understanding of the client’s feelings and validate their experiences (for example: “I see why you felt so unimportant there—your feelings were being minimized. It’s painful to be told you are overreacting—when your needs actually are being dismissed as insignificant.”);*
- *Provide interpersonal feedback (for example: “James, you’re speaking to me in a really loud harsh voice right now. I’m wondering if others have ever given you this feedback before? How do others usually respond when you talk this way?”);*
- *Use process comments to make the current interaction overt and utilize the therapeutic relationship as a social learning laboratory (for example: “You just asked me a question, Tina, and then interrupted me as I was answering. That’s happened a couple of times, and I’m wondering what might be going on inside for you when that happens. Any ideas?”);*
- *Check out the client’s reactions toward the therapist (for example: “You just took the risk of sharing some sensitive information with me, and I wonder if that could make it harder for you to come back to see me next week?”).*

We will further illustrate this balanced therapeutic stance that is active, responsive, and engaging—yet cannot accurately be characterized as directive or non-directive. This middle ground we are introducing contains both a following or reflective therapeutic mode, and at other times shifts to a guiding mode, where the therapist is actively helping the client move forward. This balance or integration of directive and non-directive styles is skillfully demonstrated in two exceptional books: Miller and Rollnick’s *Motivational Interviewing* (2013) and in Greenberg’s *Emotion- focused Therapy* (2002). Most beginning counselors have not been exposed to this balanced approach.

New therapists often lose clients in their first practicum because of their own inactivity or lack of responsiveness, and report feeling afraid of “making a mistake” or feeling too worried or preoccupied about “what to do.” More subtly, new therapists’ self-consciousness or inhibition may be prompted by their own concerns about acting more strongly and expressing their own thoughts and observations—in other words, having their own voice in the hour. Others hold themselves back because they feel apprehension about becoming someone important to the client and accepting the responsibility of trying to have an impact on the client’s life. For these and other reasons that we have already begun to explore, new therapists often are painfully concerned about doing something wrong and, as a result, become passive, quiet or emotionally distant and lose the opportunity to have an impact. This is unfortunate, because clients benefit from active engagement with their therapists. Further, clients are less likely to drop out if their therapist engages in role induction and provides education about the therapy process (Reis & Brown, 2006). For now, it is important to recognize clients do need direction at times—especially when they are in crisis (James & Guilliland, 2000). However, the therapist can provide guidelines for how to proceed without falling back on a hierarchical or one-up/one-down relationship. How can this best be accomplished? Beginning with the initial interview, *the therapist can make an overt bid to establish a collaborative relationship in which the therapist and*

*client work together to resolve the client's problems.* For example, this stance may be conveyed by the following overture:

**THERAPIST:** Let's work together and see if we can figure out what's been going wrong. Tell me your thoughts about what the problem has been, and I'll follow along and join in.

**CLIENT:** I'm not sure where to begin with all of this.

**THERAPIST:** It sounds like there's been a lot going on. Out of all the things that have been happening, what's been the hardest thing for you?

**CLIENT:** Well, I guess that would be my wife and I. We're not getting along very well anymore, and I guess I'm worried about that.

**THERAPIST:** "Worried." Tell me more about that feeling...

The therapist is trying to communicate, in words and in actions, that he or she will be an active and responsive ally. The therapist needs to actively work to create a collaborative partnership where clients are encouraged to shape the treatment agenda by articulating the concerns that are most important to them and contribute their own ideas about the problems. The therapist shows interest in the client's perspective and input by saying things like:

*"What was your intention when you responded that way?"*

*"What do you think she was thinking when she said that?"*

*"What do you understand her to mean by that?"*

When actively invited to participate, clients will begin to experience that their input is listened to, valued, and incorporated—essential as so many clients enter treatment feeling disempowered and disconnected. Creating a working partnership is a treatment goal that establishes a new, middle ground of shared control in the therapist-client relationship. This balanced therapeutic stance is active, responsive, and engaging—but still collaborative. There is a spacious middle ground here between the polarized positions of prescriptively taking charge versus passively following the client's lead.

For many clients, this collaborative partnership, in itself, provides a Corrective Emotional Experience (Gostas, Wyberg, Neander & Kjellin, 2013; Wampold, 2010). This interpersonal process is a new way of interacting that many clients have not experienced in other important relationships. This will challenge and expand clients' early maladaptive schemas, and provide a practical role model to help them see how they can begin to respond differently and more effectively in their relationships with others. Regardless of the therapist's theoretical orientation or techniques, one of the most important determinants of treatment outcome is whether the therapist and client can continue this process of collaboration. Let's look further at ways to work collaboratively so therapists can begin facilitating a strong working alliance right away—even in the initial session.

## COLLABORATION BEGINS WITH THE INITIAL INTERVIEW

One of the most important concerns for new therapists is whether their clients will return after the first session and remain in treatment. Many clients do indeed drop out after the first session(s) (Garfield, 1994; Reis & Brown, 2006), and such experiences exacerbate new therapists' concerns about their own adequacy and performance. One of the most helpful guidelines for conducting a successful first session is for therapists to initiate a collaborative relationship in their initial contact with the client. As we will illustrate, the therapist structures the session by providing the client with guidelines and direction for what is going to occur in the interview (**role induction**) (Henderson 2010; Swift, 2013).



However, *it is essential that the client feels his or her distress and concerns have been understood and responded to in a collaborative manner.* The therapist's intention is to attend to the issues that the client views as most important. In other words, the therapist is actively trying to facilitate clients clarifying what is really wrong, and then joining clients in addressing their concerns. Certainly, there will be further ideas about what the problem is, *but the focus for treatment and ideas about what most needs to change must remain closely connected to the client's personal agenda*—his or her subjective experience of what the problem is—or we will lose partnership with the client through this empathic failure. This process is best accomplished by cultivating an attitude and presence that is respectful, warm, open, and alert to all that the client is presenting (Ackerman & Hilsenroth, 2003; Greenberg, 2014). With this stance in mind, therapists can help clients begin by offering open-ended invitations to talk about the issues or concerns that matter most to them.

**THERAPIST:** What's the difficulty that brings you to treatment? Help me understand what's wrong.

**CLIENT:** There's so much going on, it's not easy to explain.

**THERAPIST:** Things are complicated. Maybe you can start by telling me about the concerns you feel are most pressing or important right now.

This type of inquiry communicates several important things to the client. It ends the opening phase of social interaction that occurs as the therapist and client are introduced and walk to the interview room. The client realistically knows that most people in life do not really want to hear about their problems in depth, so *it is important to define this new relationship by saying, in effect, this is the right place—and I'm the right person to talk to about what's really wrong, what you are feeling, and what you need.* It also tells clients that the therapist is someone who is willing to talk directly about their personal problems as they experience or perceive them—and is ready to respond to their need for help by listening. However, it does so in a way that still gives clients the freedom to choose where they want to start and leaves them in charge of how much and what they want to disclose. From the outset, clients are sharing control of the interview by choosing what they want to talk about, yet the therapist is an active participant who has offered some direction for where they are heading. This type of collaborative interpersonal process does not occur if the therapist gives the client a more directive cue and begins telling the client what to talk about:

**INEFFECTIVE THERAPIST:** When we talked on the phone, you said you were having trouble with your partner. What is the problem there?

This type of opening bid may seem benign, but it is not. It is important to communicate that clients should talk about what they want to talk about and not feel that they have to follow the therapist's agenda. From the start, that is, we want the client to take an active role in directing the course of treatment, while still feeling that the therapist is participating as a supportive ally. Enacting this interpersonal process in the initial session is more important than the content of what is discussed, and it will fundamentally shape the course and outcome of treatment.

In most cases, clients will readily accept the therapist's open-ended invitation and begin to share their concerns. The therapist can then follow the client's lead and begin to learn more about this person and his or her difficulties. Therapy is under way when this occurs. Before we go on to the next step, however, we need to examine three exceptions in which the client does not accept the therapist's offer to begin:

- 1) when another therapist has conducted an initial screening interview,
- 2) when the supervisor's presence is felt, and
- 3) when the client has conflicts over initiating.

**Previous Screening Interview.** If another therapist has previously conducted the intake interview, the client may not be so ready to begin.

**CLIENT:** (*impatiently*) I've already been through all of this in the intake with Dr. Smith. Do I have to go over it all again just for you?

OR

**CLIENT:** (*hesitantly*) I don't know how much you already know about me. What has Dr. Smith told you about me?

Our initial goal is to develop a working alliance between the therapist and the client. In these two examples, the previous intake therapist is a third party who, psychologically, is still in the room with them. This is especially problematic for clients who have come from families in which a third person was repeatedly *triangulated* into the marital relationship, especially when too much conflict was occurring (Kerr & Bowen, 1988). For many clients, that is, when parents or other family members became uncomfortable with the level conflict or independence/separateness, a third family member would be drawn in to distract from the conflict or be a peacemaker (Haley, 1996; Minuchin, Nichols & Lee, 2006). To keep therapy from reenacting this common but dysfunctional family pattern, the therapist and client want to begin their own relationship as a stable dyad. As the family therapists teach us, it is important to maintain a dyadic therapeutic relationship that does not allow others to disrupt the therapeutic coalition—as so often occurred for many clients in their families of origin.

**THERAPIST:** I know that you have already spoken with Dr. Smith, and I have learned a little bit about you from his intake notes. But just you and I are going to work together from now on, and I'd like to hear about you in your own words. It may be a little repetitious for you, but this way we can begin together at the same point.

**Supervisor's Presence Is Felt.** Triangulation in the therapist-client dyad may also occur in the initial therapy sessions for practicum students because of the unseen but felt presence of their clinical supervisor. As we will see, this can occur from either the therapist's concerns or from the client's. Through insecurity, compliance issues, or other factors, new therapists may triangulate their supervisors into the therapeutic dyad. For example, the therapist might tell the client, "I'll have to ask my supervisor about that." More commonly, new therapists may silently invoke their supervisors—by wondering what they would say or do at a particular point in the session or how the supervisor would evaluate the therapist's performance at that moment. This self-critical monitoring does not facilitate treatment—and it takes the fun out of seeing clients! It diminishes both the therapists' self-confidence and their ability to be emotionally present with the client. In particular, it disempowers new therapists by inhibiting them from being themselves in the session—from finding their own words, acting on their own ideas and utilizing their own perceptions, listening to their own intuitions, and developing their own therapeutic styles. Thus, new therapists are encouraged to trust themselves enough to be a person who is simply sharing the life story of another. Supervisory feedback and guidance, though necessary and helpful, usually will be more productive when it is processed outside of the therapy hour.

Although we don't want the therapist to triangulate the supervisor into the counseling dyad, clients may be curious or concerned about what the supervisor thinks of them or the progress they are making. This is a different situation and requires a therapeutic response. This is especially likely when a videotape or audiotape is being used during

sessions. It can be informative for the therapist to explore clients' concerns about the recording equipment or thoughts about the supervisor—such inquiries usually reveal the cognitive schemas or transference distortions they bring to relationships. For example, if the therapist asks clients what they imagine the supervisor might be thinking about them, clients often reveal key concerns:

- *I'm worried that he might be talking about me to other people.*
- *He probably thinks I'm a weak person because I'm so anxious about this, you know. He doesn't respect me because I'm so worried about everything all the time.*
- *She's probably bored with me—like everybody else.*
- *I think she's probably critical—you know, judging me.*

By having the client reveal these problematic schemas or expectations in this way, the therapist has the opportunity to highlight, address, and resolve them in treatment. The first benefit of this is keeping some clients from dropping out of therapy prematurely. Second, it also helps the client begin to change by disconfirming the early maladaptive schema as it is playing out in their current interaction, thus providing a CEE. For example:

**THERAPIST:** No, I don't think she's feeling "critical" or judging you at all. Actually, I think she probably feels compassion for what you've been coping with all of these years, and respects your willingness to come here every week and work so hard on your problems—as you've been doing.

Third, the therapist can make a process comment and begin making connections and working on how the client's faulty schemas or problematic expectations may be contributing to problems with the therapist and others. For example:

**THERAPIST:** You feel he's judging you critically. Well, that sure wouldn't feel very good. I'm wondering if there are others in your life right now who might be judging you, or maybe you've been concerned sometimes that I might be feeling critical of you as well? Can we check this out together for a minute?

Additionally, some clients may be unsure about the trainee's ability to help them, and provocatively challenge or test them. For example:

**CLIENT:** So, does your supervisor tell you what to say, or do they let you say what you think sometimes?

**THERAPIST:** I consult with a supervisor, but I have my own mind and will say what I think and believe.

Finally, many clients grew up parentified—taking care of the emotional needs of their caregiver at the expense of their own. Continuing to live out this familial role, these clients often will try to protect or take care of the student therapist as well. In order to make the therapist feel competent or look good to the supervisor, they may try extra hard to improve in treatment, talk about positive changes they are making and minimize areas that are not improving, express their appreciation for how helpful or understanding the therapist is being, and so forth. Following the primary theme of this text, this problematic but common reenactment, if not addressed and resolved in the therapeutic relationship, will prevent clients from changing this caretaking role with others in their everyday lives. In all of these ways, clients make progress in treatment when they can talk through with the therapist such concerns and ensure that the therapeutic relationship does not subtly reenact long-standing but unwanted relational patterns. In sum, whenever therapists sense the psychological presence of a third party in the room, they are encouraged to make this presence overt, explore the impact this third-party may be having on their relationship, and reestablish the therapeutic relationship as a dyad.

**Conflicts Over Initiating.** There is a second circumstance in which the client does not respond to the therapist's initial open-ended request to begin. The problems that some clients bring to treatment involve conflicts over initiating. This client cannot begin at the therapist's invitation to choose where he or she would like to begin. By asking these clients to begin talking about whatever is most pressing or important for them, the therapist has inadvertently presented them with their central or core problem. It is difficult—and often highly anxiety arousing—for this client to decide what to talk about, take the first step and initiate any activity, or to share responsibility for the course or direction of treatment. Recalling Client Response Specificity, a therapist—with the very best of intentions—who non-directively waits for the client to lead places an immobilizing demand on this client.

On the other hand, a directive therapist who takes charge and begins the session by telling this client where to begin or what to talk about is often replaying the same unwanted scenario the client has experienced with others. Whether communicated in a genuinely friendly tone, or impatiently after waiting awkwardly for a while to get started, the therapist—like others in the client's life—is again telling him or her what to do. Thus, treatment stalls right from the start if the client has problems with initiative and the therapist responds in either a directive or non-directive manner. What can the therapist do instead with this inhibited client? One of the best ways to find this more effective middle ground between directive and non-directive extremes, and to establish a more collaborative relationship, is to make a process comment, such as wondering aloud or asking about what may be occurring at that moment.

**THERAPIST:** It seems like it's hard for you to get started. Maybe we can begin together right there. Is it often hard for you to begin, or is there something about me or this situation in particular that is difficult for you?

By first identifying this as a problem, and then encouraging the client to explore it, the therapist has offered the client a focus and assisted the client in moving forward. However, the therapist has provided this focus without taking over and telling the client what to do, which would only reenact the familiar but unwanted relational pattern. The therapist's open-ended inquiry is supportive, in that it responds to the client's immediate concern. However, the client can take this issue of initiating wherever he or she wants, and is sharing responsibility for the direction of treatment. This type of response provides a new opportunity for clients to explore their problems in a supportive relational context. Our first goal is met as the client *experiences* a collaborative interaction with the therapist, rather than merely discussing the need to work together—which usually doesn't take the client very far. In contrast, such a new, collaborative experience from the outset often engenders hope about the therapist, the therapeutic relationship, and the possibility of change.

In keeping with Client Response Specificity, it will be more challenging for some cultural groups to engage initially because family structure or culture endorses a more hierarchical teacher-student (therapist-directed) interaction. Age, race, economic class, gender, religion, and other issues of diversity need to be considered. For some, it may be seen as disrespectful to presume to lead with one's elders, an educated person, or an authority figure such as a therapist. Therapists can best respond to such concerns by acknowledging differences and inviting clients to express or explore the concerns together. While it is always important to ask clients, "What is it like for you to be here?" this question can be especially important for clients who come from different cultural backgrounds, especially backgrounds where family information is closely guarded and talking about family issues with others will often feel disloyal (Sue & Sue, 2012; Vasquez, 2007).

To illustrate, one 20-year-old Korean client came to treatment—as it turned out, to get help for her 12-year-old brother. Her parents had been divorced for two years but no one in the family actually knew about the divorce because they went to family events “as a family.” However, her father had a separate residence with another woman and her brother was acting out both at home and at school. It took her most of the session to state her real reason for coming to counseling:

**CLIENT:** There are problems in my family but no one knows about them.

**THERAPIST:** It is hard to talk about your family to someone you don’t know.

**CLIENT:** Yes...

**THERAPIST:** What are you most concerned about if you talk to me?

**CLIENT:** I’ll be telling a family secret.

**THERAPIST:** OK, it doesn’t feel right to talk about a family secret. I’m wondering if there may be another side to this: what might be helpful about telling this family secret?

**CLIENT:** I wouldn’t be alone with the secret anymore.

**THERAPIST:** Yes, family secrets can be a heavy burden, a big load to carry alone. You can decide as you get to know me better if you want to share this secret with me. For now, is there anything about your family that feels safe, that doesn’t feel like a secret, that you feel OK talking to me about?

**CLIENT:** Yes, can I just describe them? Like how old they are and where they were born?

**THERAPIST:** Absolutely. That sounds like a great place to start. I’d like that.

As the client began talking about her family, safety in the therapeutic relationship developed. This helped her realize that she also had another safe relationship in her family: an aunt. She was then able to use the therapist’s help to reach out to her aunt and get this family member to join her to help her brother.

## EMPATHIC UNDERSTANDING: THE FOUNDATION FOR A WORKING ALLIANCE

As previously discussed, the therapist begins the session by giving the client an open-ended invitation to talk about whatever feels most important. Responding to this bid, most clients begin to share their concerns with the therapist and clarify the background and context of their problems. The therapist’s intention now is to find the subjective meaning that each successive vignette holds for the client and grasp what is most significant to the client from the client’s point of view. Here, the therapist is striving to have the flexibility to de-center, enter into the client’s subjective experience, and capture the *core meaning* that this particular issue holds for the client (McClure & Teyber, 2003). The client will feel deeply understood when:

1. *the therapist repeatedly captures and reflects the most basic feeling, key concern, or central meaning in what the client just said; and*
2. *the therapist can identify a common theme or pattern that links the client’s varying concerns into a more coherent narrative—helping the client better make sense of this particular issue in the broader context of his or her life, and now that it is recognized or “named,” begin to change it.*

These themes are often informed by developmental experiences with early caregivers and attachment figures and have come to shape the client’s worldview (Levenson,



1995). As the therapist and client together explore these themes and developmental experiences, the **Empathic Understanding** that ensues earns credibility with the client. Perhaps more than any other component, such Empathic Understanding is the basis for establishing the working alliance: Clients feel seen and understood and become more engaged with the therapist, invest further in the treatment process, and risk exploring their problems more fully (Angus & Kagan, 2007). Let's explore this pivotal sequence more closely.

As used here, *empathic understanding* connotes a genuine feeling of warmth and concern for the client—and that the client's distress matters to the therapist (Raskin, 2005). It is not a technique but rather a respectful attitude and nonjudgmental stance toward the client. This has nothing to do with being friendly or nice. In sharp contrast, empathic understanding behaviorally demonstrates to clients that the therapist “gets it” or sees them in a way that others generally do not. Like many important concepts we are going to explore, the attachment people often capture it best. When therapists can accurately discern and grasp their dilemma and articulate or express this understanding, they are providing the **attuned responsiveness** to contain or soothe the client's distress (Bowlby, 1988). Although this is not sufficient to resolve clients' problems, such empathic understanding will often ease clients' initial distress and, for some, their presenting symptoms may even abate. A secure attachment configuration is established, and the therapist's attuned responsiveness helps create the **Safe Haven** that often begins to relieve the entire family of attachment affects:

- *Anxiety may diminish from this reassuring emotional contact, where the client no longer feels alone.*
- *Depression may lessen; the experience of being seen and accepted, rather than dismissed or judged, engenders hope in the client.*
- *Anger from frustration over not being seen or feeling invalidated by others may be assuaged by the therapist's attuned responsiveness.*
- *Shame over having emotional needs revealed by entering therapy and asking for help may be lessened by the therapist's empathic, respectful response.*

Empathic understanding is the basis for establishing a successful working alliance, and a key concept for change in the Interpersonal Process approach. However, researchers consistently find that some therapists are significantly more effective than others in providing the therapeutic skill of empathic understanding (Catley et al., 2006). To illustrate some of these studies, Lafferty, Beutler, and Crago (1991) summed results across groups of more and less effective therapists from 11 different studies based on the extent of clients' symptom change. They found that clients of less effective therapists *felt less understood* by their therapists, whereas more effective therapists were seen as more empathic by their clients. Similarly, Miller and Rollnick (2013) report that the same manualized treatment program for Motivational Interviewing had very different treatment outcomes—based on therapists who were rated as more or less empathic.

Empathic understanding is highly challenging for therapists to provide consistently or reliably. Again, we are not talking about being nice or friendly—those are good things but not at all what we are getting at here. Empathic understanding goes beyond the surface and reaches the client's hidden, needy, or unacceptable self. *It is a highly discriminating response that requires the therapist to discern closely the key concern or central meaning in what the client just said.* Here, the therapist accurately:

- (1) *discerns the client's thoughts and feelings, and*
- (2) *effectively communicates his or her understanding of this to the client, engendering in the client the feeling, “My therapist really gets me!”*

Many clients cannot enter their own experience deeply—cannot tolerate exploring and sharing painful feelings and experiences, without the safety that this deep understanding provides. Let's illustrate what this looks like:

*Yolanda was drifting through community college without purpose or direction, and was working in a “nowhere” job. She had as beautiful a face as a girl could have but weighed nearly 300 pounds. Sometimes a cutter during stressful phases in her life, she would “scrape” herself on her arms. Developmentally, Yolanda had been molested by her godfather as a young girl. When she told her mother about what was occurring, her mother said she “didn’t want to hear this.” She made six-year-old Yolanda feel guilty about voicing this problem—telling her that it would “cause a lot of problems in the family” if she talked any more or to anyone else about it. Confused why her mother wouldn’t help her, and feeling painfully alone and “bad” about herself, Yolanda complied and continued to suffer her violation silently. Two years later, when now her mother’s boyfriend began “touching” her, Yolanda knew it was useless to try and speak up.*

*Striving to make a connection with this endearing but vulnerable young person, the therapist soon felt that she had heard enough to grasp Yolanda’s dilemma and tried to offer her empathic understanding:*

**THERAPIST:** Yolanda, you have kept so many painful experiences to yourself—you have been so alone with them. I hear you talk about how much you hate how you look, especially your weight. Is there anything about being heavy that makes sense to you now?

**CLIENT:** What do you mean?

**THERAPIST:** What happens when you begin to lose weight?

**CLIENT:** Guys begin to make remarks, pay attention; I hate that. They’re just gross...

**THERAPIST:** So, when you are heavy, what happens to that attention from guys?

**CLIENT:** It stops.

**THERAPIST:** So I’m wondering, does being heavy make sense to you?

**CLIENT:** *(crying)* Ya, I know, it protects me. I guess I always sorta knew but never quite talked about it. After my godfather, I got chubby, then my mother’s boyfriend, I got really...*(cries harder)*, Ya, I guess I gotta deal with this first before I can lose the weight...I did try to tell my mom the first time but she just...*(trails off)*

**THERAPIST:** Yes Yolanda, I can see how alone you’ve been with so much. You didn’t cause the violation in any way. You were a precious little girl who deserved to be protected. I’m so glad you are bringing me in on it.

Therapists wish to treat clients with respect, restrain their rush to judgment, and *look instead for the life story or context that helps clients make sense of their symptoms and problems*. As we have just seen with Yolanda, new therapists’ ability to help clients will largely depend on their ability to use empathic understanding to establish a strong working alliance. The four sections that follow examine different aspects of this key concept.

## WAYS IN WHICH CLIENTS DO NOT FEEL UNDERSTOOD

Many clients are concerned that others do not really listen to them, take them seriously, or understand what they are saying. Clients often describe themselves as feeling alone, unseen, different, unimportant, or dismissed. Many clients feel this way because their subjective experience was not *validated* in their family of origin. While growing up, most

clients with significant and enduring problems repeatedly received messages from caregivers that denied their feelings and invalidated their experience:

- *You shouldn't feel that way.*
- *Why would a silly thing like that make you mad?*
- *You can't possibly be hungry now.*
- *I'm cold. Put your sweater on.*
- *How can you be tired? You've hardly done anything.*
- *You shouldn't be upset at your mother. She loves you very much.*
- *You don't really want to do that.*
- *We don't talk about those things.*
- *What's the matter with you—how could you feel that way!*

Or, sometimes, family members simply changed the topic or did not respond when the client expressed a certain feeling or concern. As Carl Rogers, Virginia Satir, and Irvin Yalom model so effectively, one of the most effective ways therapists can help their clients change is to affirm their subjective experience. The pioneering family therapist R. D. Laing goes so far as to suggest that people stop feeling “crazy” when their subjective experience is validated (Laing & Esterson, 1970).

**Consistent invalidation in the client's family of origin, or misattunement in attachment terms, has profound, long-lasting consequences.** In its severe forms, some authors describe it as “soul murder” because clients lose themselves—their sense of self or their own voice—when they lose the validity of their own perceptions or experience (Schatzman, 1973). Whereas invalidation occurs to some extent in most families, it is pervasive in most symptomatic families. For example, in alcoholic, eating disordered, highly authoritarian, and abusive families, children repeatedly have experienced invalidation of most important experiences in their daily lives (e.g., Caregiver: “Of course he didn't do that. Don't you ever say anything like that again!” or poignantly, simply change the topic or continue as if nothing was said). One of the most serious consequences of such systematic invalidation is inefficacy or disempowerment. When reality-based feelings and perceptions are repeatedly denied, children become incapable of setting limits, saying “no,” and refusing to go along with what does not feel right to them. More significantly, consistent denial of their experience leaves clients unsure of what has actually happened to them and of the subjective meaning that events hold. That is, they lose the ability to “have their own mind”—to trust their own intuition, to listen to their own gut feeling and know what they know. They no longer trust their own perceptions of what may be occurring or making them uncomfortable. Typically, they won't register that something someone said or did bothered them until well after the problematic interaction is over, if at all. Denying the validity of their own experience, these clients characteristically say to themselves, “Oh, nothing really happened”; “It wasn't that bad”; or “It doesn't really matter,” when it actually was significant to them. **Routinely, they cannot find words to communicate their own experience or point of view with any clarity or specificity and, even if they can, do not expect others to understand or be interested in what they say** (Linehan, 1997).

When their subjective experience has been denied repeatedly, clients live confused, vulnerable, and anxious—not knowing what they are feeling, what they really like or value, or what they want to do. In many dysfunctional families, such invalidation is a pervasive, everyday experience that continues throughout childhood and adolescence, and often still characterizes how they continue to live. In place of clear feelings and confident perceptions, a vague, painful feeling of internal dissonance results. Fortunately, this global feeling state can be replaced with clarity and a stronger sense of voice or personal identity if the therapist consistently listens to clients with respect, captures what is most central or

key in what they just said, and validates their experience. When therapists provide such reflection and affirmation throughout the course of treatment, they will provide these clients with a powerful CEE. Therapists will be better able to grasp the core meaning in what the client just said when they relinquish the need for the session to proceed in a particular way and, instead, enter the client's subjective worldview and, together with the client, explore and clarify the issues presented.

Helping clients understand the themes presented within the context of their developmental life experiences will make the current concerns and issues make sense. Clients feel affirmed and empowered when they are listened to intently, taken seriously, and have their concerns understood and reflected back with specificity (Miers & White, 2010). Although such simple empathy and validation may sound like common or ordinary human responses, they are not. Many clients have not had their most important feelings and perceptions validated in their significant relationships—and in turn, they do not expect to be seen or understood by the therapist either (Linehan, 1997). Thus, the therapist's intention is to try to provide this validation consistently. To illustrate, in the following reflections, the therapist is not agreeing or disagreeing with the client, but trying to capture the emotional meaning or distill the key issue in what the client has just said.

- *It didn't seem fair to you.*
- *I'm wondering if you were frightened when he did that?*
- *It's been too much for you, more than you can stand, sometimes.*
- *It felt great to be so effective and in charge!*
- *It was disappointing; you wanted more than that.*
- *Here again, are you having to take care of everyone else?*
- *Yes, I hear the quandary you are struggling with there—you are damned if you do and damned if you don't.*

Stepping back from unrealistic performance demands, the therapist does not have to be an exceptionally perceptive person to understand the client's experience, and new therapists are reassured that, of course, they will not be accurate all the time. For example, in discussing parenting, Winnicott (1965) has a wonderfully reassuring phrase to diminish our performance anxieties: Children need only “good enough mothering,” just as clients, too, need their therapists only to be “good enough.” What clients need is that therapists convey their sincere striving to better understand them and “get” what this experience means to them. The therapist's empathy, genuine concern, and effort to foster a working alliance mean more to the client than being “right.” Thus, when therapists feel they do not quite understand what the client is saying, they should not feign understanding by saying “Yes,” “OK,” or “I know what you mean” when they really don't. Instead, they can acknowledge the uncertainty and ask the client to try to clarify or restate something. This will convey that the therapist is not just going through the motions but is trying hard to really understand what the client means. This furthers a partnership and becomes an opportunity to get more connected to the client. By inviting this clarification, the therapist is intervening with a more collaborative approach to empathy. Here is an example of how this might sound with a client:

**THERAPIST:** That sounded important, Susie, but I didn't understand it as well as I wanted. Can you say that again, or put it differently?

OR

**THERAPIST:** As I listen, it sounds as if you feel so discouraged that you just want to give up. Am I saying that right—or can you help me say it more accurately?

In both of these examples, the therapist is giving the client an invitation for a dialogue and trying to *jointly clarify the client's meaning with this back-and-forth interaction*. This collaborative approach to empathy takes the performance pressure of having to be “right” off the therapist, diminishes unwanted concerns about making mistakes or being “wrong,” and strengthens the working alliance.

Carl Rogers and other pioneers in client-centered therapy originally highlighted the cardinal role of accurate empathy in the change process (Truax & Carkhuff, 1967). Unfortunately, empathy came to be regarded as if it were a stable or enduring personality characteristic of the therapist. Instead of thinking about empathy as a relatively fixed personality trait, researchers have found that effective empathy comes from attempts to *collaboratively* understand the client's experience within an emerging, shared frame of reference (Barkham & Shapiro, 1986). That is, measuring across client-centered, cognitive, and dynamic therapists, *the therapeutic mode of mutual exploration—characterized by active negotiation between therapist and client—is crucial to the client's experience of being understood* (Allen, Fonagy & Bateman, 2008). For example,

**THERAPIST:** What did he do when you took the risk to say that?

**CLIENT:** He just went on talking, as if I hadn't said anything at all.

**THERAPIST:** I'm sorry he wasn't able to listen better—where did that leave you inside?

**CLIENT:** I don't know...(pause)...I hated it.

**THERAPIST:** Uh-huh, you “hated” it. I can sure see why...like you were invisible or didn't count?

**CLIENT:** Yeah, something like that.

**THERAPIST:** Hmm, it sounds like “invisible” doesn't quite capture it. What would be a better word?

**CLIENT:** I'm not sure...(pause) maybe just erased. Yeah, like my whole existence was being erased by him!

**THERAPIST:** “Erased.” That captures it much better—you say that with a lot of feeling. Tell me more about being erased—help me understand what that's like for you.

**CLIENT:** Oh, I hate that feeling more than anything, and it happens over and over again...

**THERAPIST:** Help me understand why being erased is so significant for you.

**CLIENT:** Well, being an African-American woman, you know...I've worked so hard to be where I am, to earn my place...and...have some dignity...

*In this collaborative exploration, the therapist and client work as partners to clarify the specific meaning this experience held for her. In this way, accurate empathy is less a personality trait or characteristic of the therapist than an interpersonal process characterized by mutual exploration and collaboration.*

To sum up, therapists intervene by validating their clients' experience, grasping the core messages, and affirming the central meaning in what they relay. Being accurately empathic is sometimes misunderstood as merely being nice, friendly, or reassuring. These are benevolent, well-intended responses, yet they do not help most clients change. Instead, with accurate empathy, we are looking for something more specific and far more potent—the ability to repeatedly identify what is central and discern what is most important—in what the client just said to us. Almost universally, an accurate, empathic understanding of clients' experience and validation of their feelings leaves them with both the feeling of being seen or understood, and with the sense of being accepted and not judged.

To reiterate, providing *validation* is especially important when working with minorities, LGBT clients, physically challenged individuals, those who are economically



disadvantaged, and others who feel they do not identify with a majority group. These clients will bring issues of oppression, prejudice, and injustice into the therapeutic process because their personal experiences have often been invalidated by the dominant culture. These clients, in particular, will not expect to be heard or understood by the therapist. The first step in working with all people is to enter their subjective worldview, listen empathically, and hear what is important to them—from their point of view (Pedersen et al., 2008).

### COMMUNICATE UNDERSTANDING OVERTLY AND SPECIFICALLY

To engage a client in a working alliance, the therapist listens to the client's experience, finds the feeling and meaning that each successive vignette holds for this client, and accurately reflects back or captures what is most significant or key in the client's experience. Clearly, this is not just a rote parroting of what the client has said (simple reflection). An effective reflection is more akin to an accurate interpretation or creative reframing. It demonstrates behaviorally that the therapist understands the core message, registers the emotional meaning, or distills what is most important from the client's frame of reference (complex reflection). For example, an Asian student, age 19, comes to the student health center feeling suicidal because he got an A- on his mid-term. One therapist may respond by saying, "*You are catastrophizing...the reality of an A- is not that awful.*" Alternatively, a different therapist would, upon taking into account the client's *life context* (e.g., the importance of education in his culture, being seen as a failure if he has a grade below an A, etc.), might enter more gently and wonder aloud:

**THERAPIST:** So, in your family, anything less than perfection is unacceptable? Am I saying that accurately?

**CLIENT:** Yes, my failure brings shame on me and my entire family.

**THERAPIST:** I can see what a big problem you are struggling with here—you are telling me you want to be a good son and keep your parents' respect and approval. What goes on, inside of you, right now as I try to join you so you are not alone in this painful experience?

Trying to understand the client's broader developmental/life context makes this seemingly smaller issue hold greater personal weight. Rogers (1975) believed that communicating such understanding in a nonjudgmental way provides a deep acceptance that is a prerequisite for meaningful change. New therapists will find that they achieve credibility with their clients—that is, prove their competence or ability to help, and strengthen the working alliance when they can articulate or *demonstrate* their understanding in this tangible way.

According to one unfortunate stereotype, a therapist is someone who remains on the surface and says superficially, "I hear you," "I know what you mean," or "I understand." However, this type of global, undifferentiated response is not effective and, paradoxically, often furthers the client's sense of not being seen or heard. **The therapist does not simply say, "I understand," but demonstrates that understanding by articulating the central meaning of what the client has said.** Twenty-four-year-old Susan, with a history of child abuse (perpetrated by her maternal grandfather and minimized or denied by family members who continued with regular "family gatherings"), struggled with the approaching Thanksgiving get-together:

**CLIENT:** I wish someone would just tell me what to do...give me guidelines...

**THERAPIST:** Do you have thoughts and feelings about what you would like to do but feel reluctant to act on your own wishes?

**CLIENT:** Yes...Because I'm not always sure what's real...I can be convinced that what I think and know is wrong...

**THERAPIST:** From what you have told me about your family, I can see why this is a struggle... you have described how they repeatedly invalidated what you experienced and said...it makes sense that sorting through what is real and what you know is difficult.

**CLIENT:** That's it! I know what I feel, what has gone on...then, on the outside, it all seems so perfect, like this perfect, together family...yet my cousin was also molested. But then they go on as though all is OK...I start to feel as though I'm the crazy one, especially when they have these family gatherings and Grandpa is treated like this special person...

**THERAPIST:** I can see how confusing that would be...

**CLIENT:** Yes, like, what's real?

As we see in this dialogue, therapists show the client they can be helpful when they capture and express the specific meaning and validate the client's experience, rather than offering well-intended but vague reassurances. And by checking with the client to see if that understanding is accurate, therapists are working collaboratively, which in turn strengthens the working alliance. For example, to illustrate this differently:

**INEFFECTIVE THERAPIST:** Yeah, I know just what you mean—that's happened to me too.

## VERSUS

**EFFECTIVE THERAPIST:** Let's check this out together and see if I'm getting what you're really saying. As soon as you realize that others at work are taking you seriously—listening to your ideas and suggestions, you become anxious—worried that you're going to be dismissed or put down. That's when you get quiet, go along with others' ideas and stop putting out your own, and end up just hating yourself when you leave that meeting. Am I getting that sequence right—can you help me say it better?

Let's look more closely at how trainees can start to put these ideas into practice. In the following illustration, we explore a brief case study of a client who initially had the unwanted experience of not feeling heard by her therapist, but subsequently felt understood with a second therapist.

While growing up, Marsha did not feel heard or seen by her parents. Her father was distant—uncomfortable talking with his adolescent daughter, and believing his wife should “handle the children.” Struggling with her own ongoing anxiety and dysthymia, her mother too often was critical, angrily demanding, and intrusive. In particular, whenever her mother felt or believed something, she expected her children to see it the same way—differing points of view were not permitted. For example, if Marsha felt something that her mother did not feel, her mother would chide her, “That's ridiculous. What's wrong with you?” By the time she was an adolescent, Marsha was painfully self-conscious and insecure. However, growing up with so much invalidation left her confused and uncertain about her own subjective experience, such that she didn't really understand why she felt so badly about herself. Sadly, Marsha often thought to herself, “There's just something wrong with me.” Marsha frequently cried alone in her room without really understanding what she was so distressed about.

Marsha always thought that everything would get better and be OK when she went away to college. To her dismay, though, she found herself becoming even more depressed during her first semester away. Frequently, she had to fight back tears over seemingly nothing, found it hard to reach out or talk to others, and started gaining

weight. More insecure and confused about herself than ever, Marsha began seeing a counselor at the Student Counseling Center. Although she did not really have words for what was wrong, Marsha tried to be a good client and help her therapist understand her problems:

**MARSHA:** I'm not sure what's wrong with me. Maybe I'm just lonely...but I've always had this sort of empty feeling inside.

**INEFFECTIVE THERAPIST:** Do you have any friends? What are your classmates like?

**MARSHA:** I guess I sort of have friends. My roommate in the dorm is nice.

**INEFFECTIVE THERAPIST:** What do you do with your friends—go to the movies, shopping?

**MARSHA:** Yeah, I do those things. I belong to a foreign language club, too.

**INEFFECTIVE THERAPIST:** Do you like your friends? You're new to the university; maybe you need some new friends here at school. There are lots of ways to meet new people here...

**MARSHA:** Well, I've had friends, but it's just hard being with people sometimes. So, maybe I'm lonely, but I don't know what's wrong with me. I just keep crying—like a big baby all the time.

**INEFFECTIVE THERAPIST:** But you've just left your family and come to college. You must miss your family and feel lonely. It's natural to feel lonely when you move away from home. Most of the other kids in the dorm feel that way, too. I sure did when I moved away to college.

**MARSHA:** Oh.

**INEFFECTIVE THERAPIST:** This is not unusual at all. You're going to be just fine once you get through this transition.

**MARSHA:** I hope so. Maybe my family is different, though. In high school, I always thought that my family had more problems than my friends' did. And it seemed like my mom was always mad at me for doing something wrong.

**INEFFECTIVE THERAPIST:** Yeah, but like I said, it's normal for you to be kind of emotional at this time in your life.

**MARSHA:** It is? But I still feel different from everybody else—like there's something wrong with me...

**INEFFECTIVE THERAPIST:** Sure, it's natural for you to feel different from everybody else. Late adolescence, moving away to college—it's a tough time. You're going to be just fine. Do you have a boyfriend?

**MARSHA:** Yeah, sort of. I try to tell him what's wrong, but that never works. And then he gets frustrated with me because nothing he suggests helps. I don't know why I get so depressed a lot (*pauses*)...maybe my mother used to yell at me too much...or maybe I'm just "too sensitive"—that's what my family always said.

**INEFFECTIVE THERAPIST:** Do you have trouble eating or sleeping?

**MARSHA:** No trouble eating, as anybody can see, I've gained 12 pounds and look like a pig. I wake up at night sometimes—worrying about things that don't really matter, trying not to start crying, wondering what's wrong with me...

**INEFFECTIVE THERAPIST:** Are you eating alone? Maybe you should be eating with friends.

**MARSHA:** (*with resignation*) Maybe that would help.

Marsha could not say exactly why, but she did not like seeing the therapist and, after canceling and then missing a few sessions, she did not go back. However, as the semester went on, she became more depressed, continued to wonder what was wrong with her, and began to struggle in her classes as well. During advising for second semester, a concerned

professor saw her distress, listened to her disappointing experience in her previous therapy, and encouraged her to go back and try again with another therapist. Reluctantly, Marsha agreed to try it when the professor suggested that she could go back, for just one session, and if she didn't like it again, just stop.

Therapy began more slowly this time. Marsha found herself feeling impatient and testy with the therapist and was reluctant to share much. She alternately acted blasé and then distressed but would never allow the therapist to remain joined with her on the same topic for very long. The therapist was not frustrated by her mixed message of come closer—go away, however, and was effective at communicating his continuing interest in her. After a few weeks, Marsha sensed that this therapist might be different than before and began disclosing her feelings again.

**MARSHA:** I'm just a mess—I don't know what I'm feeling—or why I keep crying all the time.

**EFFECTIVE THERAPIST:** There are a lot of difficult feelings there, and you can't sort them out yet. Let's just sit together for a minute, and see if one of them comes to you.

**MARSHA:** OK, uh, well, I guess I feel sort of anxious and sad and who knows what else.

**EFFECTIVE THERAPIST:** Uh huh, several difficult feelings there. Which one is strongest or most important for you right now? Can you tell me a little bit about that one?

**MARSHA:** I don't know...I just feel anxious...jittery inside—I always have.

**EFFECTIVE THERAPIST:** This anxious and jittery feeling is important—it sounds like it's been hurting you for a long time. Let's work together and try to get closer to it. Bring me in a little more on this—I don't want you to be alone with it anymore.

**MARSHA:** (*long pause*) I don't know what to say...how to begin.

**EFFECTIVE THERAPIST:** Sure, it's hard to get started. Maybe you can just pick an adjective, or make some gesture, that will help me see this anxious jitteriness. Or maybe you've noticed when you start to feel it—who you are with or what's going on?

**MARSHA:** I'm just anxious that I'm always doing something wrong and people don't really like or want to be around me.

**EFFECTIVE THERAPIST:** You feel anxious because you're doing something wrong, and it feels like nobody wants you. This is so painful. As if people don't like you—and it's somehow your own fault?

**MARSHA:** (*doesn't speak; nods and becomes teary; looks at therapist*)

**EFFECTIVE THERAPIST:** (*holding her gaze kindly*) I can see how much you are hurting right now as you share this with me. It's sad that things have happened to make you feel this anxious and unwanted, but I'm honored that you are willing to risk sharing all of this with me.

**MARSHA:** (*cries harder*) There's just something wrong with me; there always has been.

**EFFECTIVE THERAPIST:** Uh-huh. It's sad for me to hear you say that—that there's always been “something wrong” with you. Tell me more about what's been wrong...what's felt so wrong.

Marsha was heard and felt understood this time. Breaking the old relational pattern, she was not crying alone in her room anymore. In subsequent sessions, the therapist continued to listen well and kept responding to what Marsha thought was most important. There were ups and downs, of course, but this empathic understanding forged a working alliance between them that ultimately allowed Marsha to begin clarifying her own thoughts and feelings. As she increasingly found her own voice in this relationship, she started to become more assertive, felt more confident and relaxed with others, and her lifelong dysthymia diminished.

The concept of understanding the client sounds simple enough—as in, sure, can we move on to the next topic now.? After all, the therapist just has to listen carefully, grasp the central feelings or core message, and find a way to articulate or reflect this understanding to the client. In actual practice, however, demonstrating our understanding in this way is not so easy to do. *Most of us have been strongly socialized to “hear” in a limited, superficial way that minimizes the emotional meaning and avoids the interpersonal messages embedded in clients’ remarks.*

Marsha’s first therapist could not hear what she was really saying and repeatedly steered her away from the conflicted feelings and personal concerns she was trying to formulate and communicate. Reflecting her resiliency, however, Marsha kept trying to come back to her core message—something’s wrong. By keeping things on the surface and reassuring her, in effect trying to talk her out of her feelings and orienting her toward superficial problem-solving before he really knew what was wrong, the first therapist’s interpersonal process reenacted Marsha’s developmental history of not being seen or heard.

Most new therapists are aware that they possess a “third ear”—an ability to hear the key issue or core meaning in what people say. However, they usually feel they must avoid acknowledging the true meaning of these underlying, and often nonverbal, messages because of their emotional content. Therapists can downplay or avoid the emotional message or relational statement about you-and-me that’s embedded in the subtext for many reasons:

- *Feeling awkward themselves or concerned that they might embarrass the client*
- *Feeling inadequate or unsure of how to respond if they do go beyond the surface and respond to the bigger feeling or issue*
- *Feeling reluctant to violate cultural norms or familial rules against forthright or more direct communication*
- *Feeling a need to protect or take care of others by shielding them from their own pain or distress*
- *Being afraid of potential boundary violations or feeling too close to the client if they risk real personal engagement*
- *Having their own problems or issues activated by entering the client’s pain or distress*

As a result, many therapists are adept at switching automatically to a more superficial, social level of interaction to avoid the client’s (and sometimes their own) vulnerability. This also occurs when clients convey **embedded messages** about the therapist–client relationship that therapists may not want to hear or approach. That is, *clients are often making covert statements about the therapeutic relationship or indirect references to what is going on between them—especially about problems or concerns they are experiencing right now in their current interaction with the therapist.* Let’s provide an example of both an ineffective and an effective response for therapists faced with embedded messages about problems in the therapist–client relationship:

**CLIENT:** Do therapists who work with clients ever see their own therapists for their own personal problems?

**INEFFECTIVE THERAPIST:** Yeah, they probably do, but let’s get back to what you were just saying about...

## VERSUS

**EFFECTIVE THERAPIST:** Yes, most therapists seek help with their own issues at times. But I’m wondering if you may be having a question about me as well, or about something related to our work together, that we might talk about? If so, I welcome talking about that—it would help our work.



In contrast to her first therapist, Marsha's second therapist broke the social rules and responded directly to her core affective message, "I feel anxious." As a result, Marsha felt understood and responded to. In a small yet significant way, this was a corrective emotional experience. The therapist was willing to join meaningfully with her in her experience—demonstrating his genuine interest and concern for what was really going on for her—which significant others had not been able to do as she was growing up. Disconfirming her problematic relational expectations, he did not move away from her feelings or try to talk her out of them; in sharp contrast, he genuinely welcomed them and actively encouraged her to enter them more fully. When the therapist is able to provide such corrective experiences consistently, trust builds as clients learn that their previous, early maladaptive schemas do not fit in this relationship—and perhaps this will be true for some other relationships as well. Specifically, behavior change begins first as clients become more confident in the expectation that the therapist will see and hear them. Then, with the therapist actively scaffolding, this new, expanded schema and more flexible way of relating is extended to others in clients' everyday lives. *New therapists are encouraged to try responding in this more direct, empathic way; use the ability they already possess to hear what is most important to the client; and while still being respectful and diplomatic, judiciously take the risk of saying what they see and hear.* Most student therapists are overly inhibited—they hold themselves back too much. We can tentatively wonder aloud with the client and share what we are hearing and observing, or what we want to ask further about, in this active way without taking over, giving advice, or telling clients what to do.

**THERAPIST:** Gina, you just told me three stories about your boyfriend Robert and I wonder if you find anything in common in them. He wants you to stop working and let him take care of you. Later, he flirts with another girl in your presence and when you get upset, tells you that you are "stupid" and overreacting. Then when you try to remove yourself from this escalating argument, he physically blocks your way. Any thoughts about this?

**CLIENT:** I feel disrespected and pushed to get angry...

**THERAPIST:** Disrespected...that makes sense. You have asked him not to flirt and he is doing that and calling you "stupid" when you try to address it.

**CLIENT:** I hate it when he calls me stupid; it makes me feel like he is better than me.

**THERAPIST:** Better than you—can you say more about that?

**CLIENT:** Like he knows more, or is stronger. You know has more power...

**THERAPIST:** More power...that's a huge word...

**CLIENT:** Yes, like when he won't let me leave when I get mad. I feel so powerless, I just want to scream and hit at him but I don't want to give him a reason to hit me or say, "See why I flirt with other girls, they are nicer than you."

**THERAPIST:** You are unable to go someplace else to calm down, but, it also sounds as though speaking up for yourself has been difficult—and perhaps not felt safe?

**CLIENT:** Yes, he starts to yell, calls me names, it can feel a little scary, I don't want him to lose his temper and blame me for it...

**THERAPIST:** So you have felt disrespected, and scared, and yet he wants you to quit your job and become financially dependent on him. How does that sound to you as you hear me say it?

**CLIENT:** Wow, you know, as we talk about it, I start to wonder why I keep going back to him—why I stay with him? Because he'll eventually say, "Babe, I'm sorry I called you a bitch," but it will happen again after a while...

**THERAPIST:** There's a lot going on here that has been hurtful. And I'm also feeling some concern for your safety. It has been hard in the past to leave...

**CLIENT:** Yes, it has been hard...it helps to talk about it without having to hide it. When I try to talk to my mom, she immediately says, "Leave that asshole," but doesn't let me sort out what I want to do. Yet she's still with my dad, who uses drugs and has never worked...

**THERAPIST:** I'd like to hear all your thoughts...on both sides of this complicated problem. What has kept you with him and what might make you not want to be with him? Together, we can work out what you want and what's in your best interest.

**CLIENT:** I like that...

At its best, therapy offers clients an opportunity to be understood more fully than they have been in other relationships. When this understanding is offered in the initial sessions, clients feel that they are no longer invisible or alone, different or defective, dismissed or unimportant, and so forth. At that moment, clients begin to perceive the therapist as someone who is different from many others in their lives, and possibly as someone who can help. In other words, hope is engendered when the therapist understands and articulates the personal meaning that each successive vignette holds for clients. With this goal in mind, let's look at guidelines to help therapists clarify the central meaning in what clients present and learn how to be more accurately empathic.

### IDENTIFYING RECURRENT THEMES FACILITATES EMPATHIC UNDERSTANDING

Most clients will not share deeply or risk exploring their vulnerabilities with a therapist who does not provide the security that comes from feeling that the therapist understands them. One of the best ways to help therapists understand their clients better is to identify *recurrent patterns and themes* in the narratives they relate (Daly & Mallinckrodt, 2009; Levenson, 1995; Levenson & Strupp, 1997). Let's explore pattern recognition closely—it will also help us develop a more useful treatment focus.

The therapist's goals at the beginning of treatment are to:

- *Support clients' initiative and help them take ownership of the treatment process by encouraging them to lead and choose what they feel is most important to discuss*
- *Actively join clients in exploring the concerns they bring up and help them tell their story*
- *Identify the core message or discern the central meaning in what clients are communicating*
- *Demonstrate this by accurately capturing and reflecting this empathic understanding to the client*
- *Begin identifying the common patterns or themes in the issues presented*

Although these basic intervention goals may sound simple, they are not. What's so hard about it? *It requires the therapist to give up a great deal of control over the direction of therapy and over the timing and content of what the client brings up.* The "letting go" of what the client discusses—not knowing what the client is going to say or do next—is anxiety-arousing for many. Most student therapists are interpersonally skilled in their everyday lives and readily able to subtly take charge and, with good intentions, quietly shape or direct many of their social interactions. As student therapists try to cope with the anxiety and ambiguity of doing therapy they often fall back on the familiar role of "friendship"—which they often have learned to provide easily and successfully. However, being a friend is not a good model for how to be with clients. The same social skills that may facilitate friendships and other relationships outside of therapy do not work well with clients. Subtly leading or shaping where the conversation is going, filling awkward

moments, keeping the conversation moving or staying on comfortable topics, and having our own emotional needs met in a reciprocal manner help friendships but usually limit clients' progress. For example, in treatment the novice therapist is placed in the new and far more demanding position of responding to the diverse and unpredictable material that the client produces. A therapeutic stance that encourages the client's initiative and ownership of the treatment process fosters greater agency and self-efficacy in clients. However, it also requires the therapist to have the **flexibility** to be able to:

- *Relinquish control over what will occur next in the therapeutic relationship;*
- *Tolerate the ambiguity of not knowing what clients will produce or where the current topic will lead; and*
- *Make sense of the varied and unpredictable material that clients present.*

No doubt, this is a tall order. There are two important dimensions to working in this way. First, on a personal level, therapists need to “know themselves” (Bromberg, 2006). This self-awareness, also emphasized in the multicultural literature (e.g., Sue & Sue, 2012; Pedersen et al., 2008) will allow them to recognize more fully, when their reactivity is evoked, if it is something in their own life or if it is grounded in the client's experience. Supervisors help when they assist therapists differentiate their own experience from that of the client. Secondly, one of the best ways to help is to begin tracking *recurrent themes* in the client's vignettes or stories. Learning to identify patterns will help therapists make sense of the client's experience and better understand what is most important or central in the wide-ranging material the client presents. This empathic understanding, in turn, provides therapists with the only legitimate control they can have in therapeutic relationships—over their own responding—as opposed to ineffective attempts to subtly direct or control the material that clients produce. Additionally, the appropriate or legitimate internal controls that come from this understanding will allow therapists to *tolerate the ambiguity* inherent in this work. Thus, our challenge is less to shape or subtly lead where clients are going, as occurs so commonly, than it is to follow wherever they lead, and be able to collaboratively make sense of whatever concerns they bring to us.

**Patterns and Themes Help Us Understand.** To illustrate, suppose the client tells the therapist about his or her reasons for coming to therapy. As the therapist follows the different recollections and descriptions the client chooses to relay, the therapist's goal is to discern an *integrating focus* for the wide diversity of material the client presents. The best way to do this is by identifying the patterns or unifying themes that recur throughout. Typically, these integrating themes occur in three interrelated domains—let's take a look at each:

#### 1. *Repetitive relational themes or interpersonal patterns.*

Terminology varies, but the bedrock of interpersonal and contemporary relational treatment approaches is to identify the interpersonal scenarios that keep reoccurring for the client and working with the repetitive relational patterns that weave throughout clients' symptoms and problems. Therapists-in-training are working to develop their pattern recognition abilities—that is, to develop the eyes and ears to recognize the interpersonal themes that keep reoccurring throughout the varying narratives the client relates. For example,

*They expect so much from me, but no matter how hard I try, it's never enough. I always have to give up what I want in order to be close. It's a lose-lose situation; go along and do what others want—or be alone.*

The therapist provides deep and reassuring understanding, and begins the change process, by *highlighting or naming* these relational patterns. This helps clients begin to recognize and anticipate when they are coming into play, and explore new and better ways to

respond that could change the usual but unwanted scenario. The client's relational pattern, for example, might be to feel controlled by others, let down, or abandoned. Or, perhaps the client readily feels criticized and put down, ignored or dismissed as unimportant, or even idealized by others. It is compelling for clients when the therapist understands and can identify the same interpersonal themes occurring in three different spheres:

*In current interactions with others in their daily lives;  
In developmental relationships with family members; and  
In the current, here-and-now interaction with the therapist.*

The therapist's observations are shared in a *tentative* manner—so the client can easily modify, refine, or even reject them. Exploring them collaboratively in this way, clients feel understood in a way they have not in other relationships. As a result they,

- 1) *often develop a more specific awareness of just how these problematic patterns typically go;*
- 2) *often begin observing when these familiar but unwanted situations are coming into play with others; and*
- 3) *often begin to recognize their participation in or contribution to this interaction sequence. For example (client to herself):*

*“Here I am again. I’m starting to feel that he really doesn’t want to be with me, and I’m trying soo hard to figure out what he wants and how to please him. I need to change what I’m doing here, and instead start focusing more on what I need.”*

For now, let's go on to identify themes in the cognitive and affective domains—all three are equally important and inextricably entwined.

## 2. *Pathogenic beliefs, automatic thoughts, faulty expectations, and schema distortions.*

Just as the therapist can better provide empathic understanding by identifying unifying themes in interpersonal relations, cognitive patterns and thought processes also provide an essential component for accurate understanding and effective intervention (Beck, 1995; Bjorgunsson & Hart, 2006; Lazarus, Lazarus & Fey, 1993). For example, clients' maladaptive relational patterns are closely associated with their problematic beliefs about themselves, inaccurate perceptions of others, and faulty expectations for what is going to occur in relationships and in the future. Ellis (1999) suggests that two of the most common dysfunctional beliefs concern being liked and loved (I must *always* be loved and approved of by the significant people in my life) and being competent (I must *always*, in all situations, demonstrate competence). Working in an integrative cognitive-psycho-dynamic approach, Weiss (1993) and Silberschatz (2005) highlight **pathogenic beliefs** that help to create and sustain the maladaptive relational patterns introduced above. For example, regarding clients who feel excessive or unrealistic guilt, the client's pathogenic belief might be:

*I am being selfish whenever I say no or do what I want.  
Others are hurt by my independence.  
I must not enjoy my own success or surpass important others.*

Similarly, a pathogenic belief for clients who are prone to feeling shame, or that others will ignore or reject them, might be:

*I am unimportant and do not matter.  
I am inadequate, and if others really know me, they will see that I am lacking or weak.  
Others would ignore me if I asked for anything, or resent my needs as selfish and demanding.*

Therapists-in-training are encouraged to begin listening for faulty beliefs such as these. If we attend closely, familiar motifs can be heard across the widely varying vignettes that clients relay. When therapists can capture these themes, and collaboratively engage the client in identifying and clarifying them, clients will feel understood and find that this therapist can help them.

### 3. *Recurrent affective themes or central feelings.*

Therapists also can identify unifying themes in clients' emotional reactions. Recognizing and responding to these recurrent feelings can be nothing less than a Gift that therapists offer to their clients (Sue & Zane, 2009). Often, a primary or core affect comes up again and again for the client in many different situations (Greenberg, 2002; Greenberg & Goldman, 2008). As the therapist listens to the client, an overriding feeling such as sorrow, bitterness, distrust, or shame may pervade the client's mood or characterize different experiences the client relates. To capture this theme, for example, the therapist might then reflect:

**THERAPIST:** As I listen to you, it sounds as if you have lived your life with the feeling of being hopelessly burdened. Am I capturing something here?

OR

**THERAPIST:** This doesn't quite sound like worry or anxiety that keeps coming up, it seems much bigger than that! Am I hearing more of a sickening feeling of dread?

When the therapist can *identify and accurately name* this **characterological affect**—the central or core feeling that some clients may experience as nothing less than the defining aspect of their existence—it has a profound impact. Clients feel that the therapist understands who they really are and is seeing them and what their life is like in a way that others have not done. A key in establishing the therapist's credibility and fostering the working alliance is the therapist's ability to identify these central or core feelings that keep coming up, and accurately reflect the far-reaching meaning it holds in the client's life.

Responding to these primary feelings and capturing the recurrent emotional themes that keep coming up in the client's life is one of the most significant interventions therapists can provide (Diener & Hilsenroth, 2009; Markowitz & Milrod, 2011). Unfortunately, new and experienced therapists often feel insecure about responding to strong feelings in their clients (Williams et al., 1997). Trainees need their supervisors to help them recognize and respond to these affective themes, especially by role-playing or otherwise demonstrating what trainees might say or do when these primary feelings emerge. Because this topic is so important, and often receives too little attention in clinical training, specific guidelines for responding to clients' feelings will be provided in Chapter 5. Completing the process notes provided in Appendix A will also help therapists identify recurrent patterns in the affective domain.

*Identifying unifying patterns in these three domains, which will be present in most clients' narratives, helps therapists understand their clients' problems and provide empathic understanding.* The disparate material that clients present may initially seem unrelated and disconnected, and it can evoke in the therapist the unwanted feeling of being lost, confused, or even overwhelmed. With practice and the sustained intention to develop this skill, however, new therapists will improve their ability to listen for and recognize the synthesizing themes that commonly are present in all three domains. As we see in the case illustration below, attending to developmental and familial experiences (contextualizing) is often key to understanding the patterns and themes that clients present.



*Sarah, age 28, was referred to treatment by her physician. She was the oldest of three children and her mother was chronically depressed and hypochondriacal. By the time Sarah was 10, she often made the family dinner and school lunches for herself and her siblings. She was “reinforced” for this by her father for being “such a great helper” and throughout her adult life she gravitated toward jobs where the demands were high and she excelled. Although she was appreciated and praised, her financial compensation was not commensurate with the time and energy she invested. She also often chose partners who, while initially attentive, soon demanded extensive caretaking from her. By the time she was 28, she was having severe migraines and had developed high blood pressure. Her boss had just given her a “promotion”—a title change with additional responsibilities, but again no additional staff/assistance and no increase in pay. The boss’s familiar statement had been, “You are absolutely my best worker. Don’t worry, in time we’ll give you a raise and more help. Be patient, at least you have a better title now.”*

*Consistent with her developmental story, Sarah felt conflicted. The praise was reminiscent of what she had experienced with her father and she felt she should be appreciative, yet she felt used and resentful. She had just been asked to take on more of her boss’s responsibilities but with no help to complete the work and no financial compensation for it. Amplifying her dilemma, her personal life was becoming “overwhelming.” Her boyfriend expected her to be the primary provider in their relationship, yet he resented all the hours she devoted to her work.*

**THERAPIST:** It’s sounding to me as if, here again, you have to be the responsible one who takes care of everything—and wind up feeling tired and resentful in the end. I’m wondering if this sounds like a familiar pattern in your life—does something like this “used” scenario go on a lot?

**CLIENT:** (crying but angry) It doesn’t sound like “something” in my life—that is my life! My work, my personal life, my childhood! That’s been my role my whole life—take care of everybody—as if that’s just my job in life. But if I don’t stop doing this, it’s going to kill me—I’m exhausted! And it’s not fair—no one ever responds to me.

In this example, the therapist “gets” her and provides empathic understanding. Sarah’s developmental experiences provide an illuminating context for grasping the patterns and themes that are leading to the poignant life circumstance she currently is in. Further, the therapist’s empathic understanding created safety and allowed Sarah to explore more deeply her own needs and the consequences of the choices she was making. Therapists can help clients make sense of their long-standing problems by listening for and highlighting integrating patterns in their thoughts, feelings, and behavior. This linking helps to locate current symptoms in a broader context—which helps clients change by developing a more coherent life story or narrative for who they are and where they have come from. When developmental life experiences are taken into account and symptom presentation and repetitive patterns are clarified, therapists can then integrate the themes and share these tentatively, as hunches or possibilities, to provide empathic understanding and to help formulate a shared treatment focus (Curtis et al., 1988; Smith, 2006).

How can new therapists begin to develop this essential skill of recognizing patterns and understanding presenting symptoms in a lifespan perspective? Writing process notes after each session is one way that student therapists can learn to identify patterns and recognize integrative themes in their clients’ narratives and in the therapeutic relationship. Guidelines for doing this are provided in Appendix A, and we will develop these concepts further in succeeding chapters. To sum up, if the therapist can identify the maladaptive interpersonal patterns, pathogenic beliefs, and primary feelings that recur throughout

clients' lives, and link experiences with problems, clients can better make sense of things. In this way clients are reassured to know that the therapist understands them in a way that others do not. Providing this highly differentiated or specific empathy that “hits the nail on the head” can be profoundly helpful for clients. *This is not an isolated intervention or technique, but a consistent stance that characterizes the therapist's way of being with the client—throughout each session—over the course of treatment.*

## PERFORMANCE ANXIETIES MAKE IT HARDER TO ESTABLISH A WORKING ALLIANCE

Let's let therapists be human, too. They cannot always hear or fully assimilate what their clients have just told them or be accurately empathic. Understandably, all therapists will miss the feeling and meaning in the client's experience at times, but this takes on greater salience for therapists-in-training. Thus, this section returns to performance anxieties and new therapists' worries about mistakes, which make therapists too self-conscious to be able to listen well and be present with their clients.

As we saw in Chapter 1, new therapists will not be as effective with clients when they are burdened by their own excessive performance demands—which can be amplified by reading about all the conflicting ideas and differing ways of responding. We have already seen that beginning therapists often are trying too hard (e.g., to be helpful, to win approval from a supervisor, to prove their own adequacy to themselves, to be liked by the client, to avoid making a mistake and hurting the client or being criticized, etc.). When therapists are trying too hard in one of these ways, it is almost impossible to de-center, enter the client's subjective worldview with accurate empathy, and be emotionally present (Skovholt & Rennestad, 2003).

Too often, novice therapists are also preoccupied about where the interview should be going, wondering how best to phrase what they are going to say next, trying to formulate alternatives or suggestions for the client, or worrying about what their supervisors would expect them to do at this point in the interview. Such self-critical monitoring commonly immobilizes student therapists, blocks their own creativity, and keeps them from enjoying this rewarding work. Moreover, it prevents therapists from stopping their own inner chatter, listening receptively to their clients, recognizing patterns and making connections, and understanding as fully as they could. Thus, new therapists are encouraged to slow down their self-monitoring process and focus more on the moment—on the client rather than themselves and their own anxiety, on their current interaction with the client, and on grasping the meaning that this particular experience seems to hold for this client right now. That's how we begin to establish this all-important alliance.

Finally, student therapists feel anxious about their helping abilities. New therapists often experience an unwanted, internal press to *do* something to make the client change. They frequently believe they are supposed to make the client think, feel, or act differently within the initial therapy session(s). As a result, they often change the topic abruptly (at least from the client's point of view) and pull clients away from their own concerns to what the therapist thinks is most important—losing any sense of collaboration or shared exploration. The most appropriate way for therapists to manage their realistic concerns about their performance is to discuss them openly with a trusted supervisor and to normalize them as common and expectable. Therapists do not need to labor under the burden of always knowing the right thing to do or what is going on with the client. They need only to foster a collaborative dialogue with clients in order to explore and understand better what is going wrong in their life and what actions to consider which will clarify what they wish to try to change.

## EMPATHIC UNDERSTANDING AS A PRECONDITION OF CHANGE

The therapist does need to help the client change, of course, but change is most likely to occur if the client first experiences the therapist as someone who sees and understands him, and treats him with respect. As previously stated, in the initial sessions, the therapist's primary goal is to establish a working alliance by repeatedly providing empathic understanding and emotional presence and communicating a genuine interest in the client's safety and well-being. This understanding of the client's experience is done in a way that also communicates compassion and nonjudgmental concern. The ability to articulate the client's experience in an accurate and caring way is illustrated elegantly in a classic article, "Ghosts in the Nursery," by Selma Fraiberg and her colleagues (Fraiberg, Adelson & Shapiro, 1975). The therapist in this case study is working with a depressed young mother whom social services has judged to be at risk for physically abusing her infant daughter. Early in treatment, the therapist is disconcerted as she observes the mother holding her crying baby in her arms for five minutes without making any attempt to try to soothe her. The mother does not murmur comforting things in the baby's ear or rock her; she just looks away absently from the crying baby. Trying to formulate a case conceptualization and treatment plan, the therapist asks herself, "Why can't this mother hear her baby's cries?"

As the young mother's own neglectful and abusive history began to come out in treatment, the therapist realized that no one had ever heard or responded to the mother's own profound cries as a child. The therapist hypothesized that the mother "had closed the door on the weeping child within herself as surely as she had closed the door upon her own crying baby" (p. 392). This conceptual understanding led the therapist to a basic treatment plan: When this mother's own cries are heard, she will hear her child's cries.

The therapist set about trying to hear and articulate compassionately the mother's own childhood experience. When the mother was five years old, her own mother had died; when she was 11, her custodial aunt "went away." Responding to these profound losses and the mother's resultant feeling that "nobody wanted me," the therapist listened and put into words the feelings of the mother as a child:

*How hard this must have been.... This must have hurt deeply.... Of course you needed your mother. There was no one to turn to.... Yes. Sometimes grown-ups don't understand what all this means to a child. You must have needed to cry.... There was no one to hear you. (p. 396)*

At different well-timed points in treatment, the therapist accurately captured the mother's experience in a way that gave her permission to have her feelings. As a result, the mother's grief and anguish for herself as a cast-off and abused child began to emerge. The mother sobbed; the affirming therapist understood and comforted. In just a few more sessions, something remarkable happened. For the first time, when the baby cried, the mother gathered the baby in her arms, held her close, and crooned in her ear. The therapist's hypothesis had been correct: When the mother's own cries were heard, she could hear her baby's cries. The risk for abuse abated as this beginning attachment flourished.

This poignant case study illustrates how the therapist can use empathic understanding to resolve the client's symptoms and, in this case, stop the intergenerational transmission of abuse. A corrective emotional experience occurred when the therapist responded to the mother's pain with compassion and understanding—something she desperately needed as a child but did not receive. This case study illustrates how powerful it is when

therapists can articulate their understanding of the client's experience, and do so in a way that also communicates their compassion for the client.

Using role models such as this, therapists-in-training are encouraged to explore their own ways of communicating that they are touched by the client's pain and are concerned about the client's well-being. Clients' inability to care about themselves is central to many of their problems, and most clients cannot care about themselves until they feel someone's caring for them (Gilligan, 1982). Therapists provide this care when they recognize what is important to the client, express their genuine concern about the client's distress, and communicate that the client is someone of worth and will be treated with dignity in this relationship. These are the therapist's goals in the initial stage of therapy.

## CLOSING

The Interpersonal Process approach is integrative and draws on concepts and techniques from differing theoretical traditions. The basic concept in this chapter—establishing a working alliance through empathic understanding, is grounded in Rogers's client-centered approach and, especially, accurate empathy.

The Interpersonal Process approach tries to resolve problems in a way that leaves clients with a greater sense of their own self-efficacy. This independence-fostering approach to therapy can best be achieved through a collaborative partnership (Myers & White, 2010). The client needs to be an active participant throughout each phase of treatment—not a good patient who waits to be cured or told what to do by the doctor. This process dimension of how the therapist and client work together is more important than the content of what they discuss or the theoretical orientation of the therapist.

In this chapter we also have explored the profound therapeutic impact of listening with presence and working collaboratively to discern what is most important to the client. Offering the deep understanding described here is a Gift to clients—one of the most important interventions that therapists of every theoretical orientation can provide. Perhaps because they are so simple, these basic human responses are too easily overlooked. They are the foundation of every helping relationship, however, and the basis for establishing a working alliance.

## CHAPTER EXERCISE

When significant others in your personal life are distressed or in crisis, how have you typically responded? Would you characterize your responses more as attempts to provide reassurance or problem-solving, or as offering validation and empathic understanding? Looking back, do you think your response was helpful?

In parallel, recall a significant crisis in your own life that occurred in recent years. Looking back, what kinds of responses from others were helpful to you, and which were not? What might validation or empathic understanding have offered you during that stressful time?

## SUGGESTED READING

Egan, G. (2014). *The skilled helper*. Belmont, CA: Brooks-Cole. Chapters 3 and 4 provide helpful guidelines for listening with empathic presence and responding with empathy.

Beginning therapists need to observe role models demonstrating the concepts they are reading about. Readers are encouraged to watch a training DVD that illustrates powerfully

the concepts presented here. Readers can observe Hanna Levenson providing empathic understanding to a client dealing with domestic violence. This skillful therapist will help readers understand a relational approach to treatment in this DVD series produced by the American Psychological Association.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes. Also included with this chapter is a video demonstration by the authors illustrating empathic understanding and working collaboratively with clients.





## CHAPTER 3

# Honoring the Client's Resistance

*Joan was nervous about seeing her first client, but the initial session seemed to have gone well. The client talked at length about his concerns and, to her relief, she found it was easy to talk with him. The client shared some difficult feelings and Joan felt that she understood what was going on for him. She was thinking they had begun a good therapeutic alliance; the client seemed so friendly and appreciative when he left. One week later, however, Joan received a brief telephone message from the client saying only that he was “unable to continue therapy at this time.” Confused and dismayed, Joan sat alone in her office wondering what had gone wrong.*

### CONCEPTUAL OVERVIEW

Just when the therapist feels that something important is getting started, some clients put their feet on the brakes. The client cancels or doesn't show up for the next second appointment, shows up 25 minutes late, or asks to reschedule for Sunday at 7:00 A.M. This resistance is puzzling and frustrating for the novice therapist: “Why didn't she return?! We had such a great first session—she shared so much....” Although most clients will not be resistant in this particular way, other forms of **resistance** will occur regularly throughout therapy—even for motivated, responsible clients who are trying hard to change. Although most new therapists do not anticipate this as they begin their training, **working with resistance is an expectable component of the treatment process**. Thus, the purpose of this chapter is to help new therapists learn how to recognize and respond effectively to resistance so more clients can remain in and benefit from treatment.

Most clients have both positive and negative feelings about entering therapy, although the positive feelings are more apparent at first for most clients. Clients seek therapy in order to gain relief from their distress. However, we want to look further into the complexity of clients' feelings. Often, as clients seek help and genuinely try to change, they may simultaneously resist or work against the very change they are trying to attain, or have difficulty returning to sessions—even though they like and find the therapist helpful. From the earliest beginnings of psychotherapy (Breuer & Freud, 1893, reprinted in 1955) and continuing throughout with contemporary integrative treatments such as Motivational Interviewing (Arkowitz, Miller & Rollnick, 2015; Miller & Rose, 2009), clinicians have been trying to understand and address this paradoxical, shadow side of the change process. It occurs for

many reasons. For many clients, **shame** is associated with asking for help or with having an emotional problem they cannot solve on their own. In some cultures, revealing problems to people outside one's family meets with strong disapproval or is seen as *disloyal*. Other clients feel **guilt** about asking for help for themselves or doing something to meet their own needs. Still other clients, acting out of their schema distortions, have *anxiety* evoked by the expectation that the therapist, like significant others in their lives, will respond in familiar but unwanted or hurtful ways. Thus, if clients have a problem or need to ask for help, then shame, guilt, anxiety, and other difficult or threatening feelings are often evoked, especially the deeply held belief that the therapist, too, will ultimately respond in the same problematic ways that others have in the past. In addition to difficulties in entering treatment, we will see that **ambivalent feelings** can also be evoked when clients feel better, improve in treatment, and make successful changes in their lives (Miller & Rollnick, 2013).

Therapists are trying to identify and work with the specific issues that make it difficult for this particular client to enter treatment, and then to sustain behavior change later in treatment. In many instances, *the resistance is a coping strategy that once was adaptive for a client*. For example, being completely self-reliant as a child and never needing or asking for anything from a caregiver, kept the child from being abandoned, ridiculed, hit, or in some way hurt, but is no longer needed in adult life. The therapist's aim is to find a way to honor this coping strategy. For example, acknowledge its developmental adaptive value with a statement such as: It makes sense that asking for help was not safe because you were ridiculed as being 'too needy' and derided as 'demanding' when you showed vulnerability. Thus therapists help clients see that this coping strategy is no longer functional (it is now actually impeding healthy relationships). Following client response specificity, specific concerns about entering therapy and asking for help, talking to the therapist or someone outside the family, or making changes and feeling better about themselves must be clarified for each individual client. However, common themes include:

- *If I let myself trust or depend on the therapist, he might criticize or judge me, try to control me, start to depend on me or need me, or in some unwanted way take advantage of me—just as others have done when I needed them.*
- *I cannot ask for help because I must be independent and in control all the time.*
- *I don't deserve to be helped by anyone; I don't really matter very much.*
- *I cannot need anything from others—if I'm not perfect I will bring shame to my family.*
- *Asking for help is admitting that there really is a problem, and that proves there really is something wrong with me.*
- *I am afraid of what I will see or what a perceptive therapist will learn about me if I stop and look inside myself.*
- *If I cannot handle this by myself, it means that I really am needy and too sensitive, just as they always said.*
- *I'm a therapist. I can't have problems—I'm supposed to have the answers.*
- *I'll start crying and won't be able to stop—it will be humiliating.*
- *If I ask for help, strings will be attached and I'll lose my independence.*

In these and a thousand other ways, difficult or resistant feelings will be activated for many clients by the simple act of calling the therapist and coming in for the initial session (Young et al., 2001). Although reasons will vary greatly for each client, *clients' resistance and defenses are driven most frequently by shame*, as seen in many of the examples above. To help us gain some understanding for this resistance, and therein find more compassion for what it's about, pause now for a moment and think about your own life during a time of real crisis. Ask yourself, "What has it been like for me to have a personal or emotional problem and need to ask someone for help?" For many, sadly, it's not easy. For some, it's simply unthinkable to even imagine talking to a caretaker or others about a significant problem (Egan, 2014).

This profound issue of how someone copes with vulnerability leads many places but, in particular, it takes us to the heart of the Attachment story. What do we really mean when we say that children are securely attached? It's this simple: *Securely attached children are secure in the expectation that, when they are in distress, their caregivers will consistently register or hear their distress and reliably orient to try and help them with it.* In other words, they are not alone with their problems—they have a trustworthy partner or ally who genuinely wants to know when something is wrong and who dependably wants to help them with whatever problem they are having (Bowlby, 1988; Cassidy & Shaver, 2008). With resistance, in contrast, we are usually working with insecure attachment histories. These clients lacked both components of a secure attachment: a **Safe Haven**—someone to whom they could come to for comfort in distress, and a **Secure Base**—someone from whom they could find support to venture out, explore, and become more independent. That is, based on their real-life experience, many clients have learned realistically to expect that the therapist and others will not really be very interested in, or be capable of, responding to them when they are distressed or in need; nor will their growth and independence be encouraged if/when it conflicts with the other person's need. This belief, originally learned in repeated real-life experiences with primary caregivers, and often confirmed in subsequent important relationships, is going to come in play with the therapist at different points—especially as treatment begins (Gold, 2011).

It is easy to respond to the approach side of clients' feelings—the fear or pain that motivates them to seek help and enter treatment. If you just scratch the surface, it is usually plain to see. One part of the client wants you to respond to it but, commonly, another part doesn't. This shadow side—the contradictory or ambivalent feelings that arise from the unwanted responses the client has received from others in the past for having a problem and needing help—acts as a countervailing force. If unaddressed, this resistance will keep some motivated and highly workable clients from being able to engage successfully in treatment, as in our opening vignette.

## RELUCTANCE TO ADDRESS RESISTANCE

Imagine your client has missed, come very late for, or twice rescheduled the second appointment. Perhaps this has no significant psychological meaning at all. Cars break down; traffic jams occur; children get sick; employees get called for work at the last minute. However, if this behavior reflects the client's ambivalence about some aspect of being in treatment, and not just reality-based constraints, the therapist needs to find an effective way to address this or the client is far more likely to terminate prematurely. Initially, therapists usually do not know whether the client's behavior is reality-based, psychologically motivated, or both. As we will see, however, by finding non-threatening or non-blaming ways to explore this behavior with the client, therapists can greatly increase the chances of the client's continued participation. In these explorations, therapists' curious, nonjudgmental manner is as important as what they say. Otherwise, the client may misunderstand the therapist's observations as blame or criticism, which never help and only serve to exacerbate client resistance.

To be more specific, let's follow a two-step sequence for working with resistance. First, the therapist can enter supportively into clients' frame of reference, *affirm or validate the reality-based constraints they perceive*, and respond flexibly by doing whatever is reasonable to accommodate or help resolve the problem:

**CLIENT:** My child gets out of school at 2:30 and...

**THERAPIST:** Well, I can't meet at a different time on Wednesdays but I could meet on Thursdays at 4:30. Would that give you the shuttling time you need?

Second, only after taking seriously clients' reality-based concerns, as they see them, can therapists begin to inquire about other conflicted feelings that entering treatment may hold. The therapist can begin this joint exploration by wondering aloud, *in a tentative manner*, whether entering treatment may be evoking other psychological concerns as well.

**THERAPIST:** I'm wondering how it is for you to call and come in to see me. What's it like for you to have this problem and come here to see if I can help?

OR

**THERAPIST:** What's the biggest fear or concern that coming here and talking to me brings up for you?

If therapists approach resistance without following this two-step sequence, most clients will feel misunderstood or blamed, impeding their ability to engage in treatment. For example, an accusing therapist might say, "You didn't show up for our last session. Why not?" At the other extreme, an avoidant therapist might say, "Oh, you're late again, but I'm so glad you still came. Great, let's get to work, where should we begin?" Let's look now at both the therapist's and the client's shared reluctance to talk about potential resistance—and how this leads to losing clients.

### THE THERAPIST'S RELUCTANCE TO WORK WITH RESISTANCE

For several reasons, resistance is not a welcome concept to most new therapists. It can feel uncomfortable or be misunderstood as confrontational, but it is a necessary skill and we can start learning sensitive ways to work with it right now. To a greater or lesser degree, every client will be ambivalent, defensive, or resistant at times. Resisting clients are simultaneously struggling with a conflict between their genuine wish to change and need to maintain the status quo (Gabbard, 2007). This push-pull may occur at the beginning of treatment and it can continue to wax and wane throughout treatment. In their informative work on stages of change, Prochaska et al. (1992, 2006) report that most clients who terminate prematurely can be described as "precontemplators." Precontemplators do not "own" their problems, and usually enter treatment because of pressure from others. That is, they feel coerced to change by a spouse who threatens to leave, an employer who threatens to dismiss them, judges who threaten to punish them, or a parent who threatens to withdraw support.

Resistance become an even bigger issue when courts mandate clients to attend treatment. Although mandated clients can be helped by working collaboratively to find some aspect of the problem they genuinely wish to change, they are generally far more challenging and not well suited for new therapists. With every client seeking treatment, however, therapists want to approach or address potential signs of client resistance—and not avoid or deny them. Researchers find that clients progress in treatment and have better outcomes when therapists recognize resistance and respond to it in ways that restore clients' active collaboration in the treatment process (see Beutler et al., 2002; Orlinsky et al., 2004). Let's examine three reasons why many beginning—and experienced—therapists often find it difficult to approach and explore the resistance, ambivalence, and defense that is part of the treatment process.

First, many new therapists—especially if they have never been in therapy themselves—are not so aware of the conflicted feelings associated with the decision to enter treatment. These therapists are often surprised to find that, in the initial sessions, some clients actually resist the help they are overtly seeking. Commonly, therapists are reluctant to explore and work with resistance because they fear it can only be addressed in a blaming or critical way that will anger clients or make them feel guilty. They may also

believe that resistance has to be addressed in a manner that puts therapists in a superior position and denies the validity of the client's own experience—which we never want to do. This occurs, for example, when a therapist ineffectively says to a working, single mother of three:

**INEFFECTIVE THERAPIST:** Hmm, I notice that you are five minutes late for our appointment today.

Problematically, this therapist is ignoring the first step of our sequence: validating the reality-based constraints of this client's life-context. Resistance does not have to be associated with this unwanted type of hierarchical therapist–client relationship. Therapists can work with resistance and still be sensitive to the social context or reality-based constraints of the client's life (Egan, 2014). For example, clients who use public transportation will not be able to arrive on time consistently. Clients from cultures with varying views of time may not think that being five minutes late for a 50-minute appointment is of any significance. For some, it may even be impolite to come on time; arriving late allows the host extra time to prepare for the visit. Therapists can educate clients about the procedures of therapy and, as we will see, respond to resistance in collaborative, non-blaming ways that strengthen the client's commitment to therapy, enhance the therapeutic alliance, and empower rather than invalidate the client.

A second reason why therapists are reluctant to address their clients' resistance is more personal. Most new therapists have strong needs for their clients to like them. Understandably, student therapists want very much for their new clients to find them helpful and to keep coming to counseling. If the client does not show up or comes late, new therapists often struggle with feelings of failure. If this occurs with other clients as well, new therapists often become painfully insecure about their ability to help, place even more performance demands on themselves, and become overly invested in pleasing or performing adequately for the next client. This is not a good spot for new therapists to find themselves in.

Third, therapists often do not inquire about signs of potential resistance in order to ward off unwanted criticism. Not wanting to hear what they may be doing wrong, it becomes especially hard for new therapists to make process comments and invite the (seemingly) bad news. The examples below, and throughout this text, are intended to give new therapists a language to use with clients that is forthright and direct but *never confrontational* in any way (Kiesler, 1988; Wachtel, 2011):

**THERAPIST:** What's it been like for you to talk with me today? What's felt good, and what hasn't?

OR

**THERAPIST:** Is there anything about our work together that doesn't feel quite right to you? If so, I would welcome hearing about it. You know, so we could make any changes we needed to have this work better for you.

Like their clients, most therapists are not eager to approach issues that arouse their own anxiety—such as bringing up potential conflict with others and talking it through forthrightly. Thus, new therapists may be hesitant to invite clients to express any negative reactions they may be having toward the therapist's response, the concerns evoked for them by having to ask for help, or their ambivalent feelings about needing to be in treatment. It is difficult for most people to invite critical feedback or approach interpersonal conflict, but it is so very helpful when the therapist can be *non-defensive* enough to do so. Many clients have not found this in any other relationship.



Exploring potential resistance or ruptures becomes far more important if the client makes an embedded message about the therapist or some aspect of the therapeutic relationship and alludes to some conflict with the therapist or difficulty about being in treatment:

**CLIENT:** Where I grew up, we don't talk about problems outside the family.

**THERAPIST:** Thanks for telling me that. So, I guess you're saying it's hard to come and talk to me also. Let's talk about that.

OR

**CLIENT:** My family really values self-reliance. You're not supposed to have any problems—ever. And if you do, you just handle them on your own. You'd never tell anyone about them or see a therapist.

**THERAPIST:** OK, so you're telling me that it's not so easy to come here and do this—it violates a family value. I appreciate your willingness to risk doing it differently with me. How has it been so far for you to tell me about this problem when your family has always said that's unacceptable?

**CLIENT:** Kind of mixed. I like talking to you because you seem to understand me, but I guess I feel kind of guilty, too.

**THERAPIST:** I appreciate both sides of your feelings. I'm glad that you feel understood by me, and think that will help you. But I also appreciate that you want to honor your family and not be disloyal to them. I'm wondering if you can decide to do some things differently than your family does, you know, that work better for you, but still find other ways to continue to honor and respect them.

Continuing further, it becomes even more important for the therapist to address potential problems or misunderstandings when the client implies there is some problem or personal concern with the therapist. For example:

**CLIENT:** So, how old are you anyway—have you been practicing long?

For many new therapists, *learning to address interpersonal conflict in this straightforward, non-defensive way—rather than avoid it as if nothing meaningful has been stated—is one of the biggest challenges in their clinical training.* Such a challenging or provocative comment from the client is highly anxiety-arousing for most new therapists. However, we need to anticipate it, and prepare to respond therapeutically. Most new therapists did not expect that being forthright and addressing interpersonal conflict straightforwardly was part of the job description when they signed on for this career! The paradox is, *if therapists allow their own anxiety to keep them from asking clients about such potential signs of resistance, ambivalence, or rupture, their clients will be far more likely to act on these concerns and drop out of treatment prematurely.* New therapists need to take a breath, gather themselves to respond non-defensively, and take the risk to meet this concern head-on. They do this effectively by simply inviting the client to discuss this concern more fully:

**THERAPIST:** I'm happy to respond—but I'm wondering if you are asking because you have concerns about whether I have enough experience to help you, or if there is something in particular that happened between us that brought that question to mind?

An effective approach is to make a process comment that suggests the possibility that something about entering treatment or talking to the therapist may be difficult for the client, and invites the therapist and client to explore this possibility together. If the



client is much older and is just focused on age, being forthright is often what is needed and responding to the question directly is appropriate. Usually, however, the question is really about competence. Before long, new therapists will find that they can do this in an affirming way that does not make the client feel accused or blamed, and that their non-defensiveness and willingness to engage so honestly enhances the working alliance.

**THERAPIST:** You seem discouraged today, and you've had some trouble getting here. I'm wondering if something isn't going right between us. Any ideas?

**CLIENT:** I'm not sure what you mean.

**THERAPIST:** I'm wondering if something about our relationship—or being in treatment—isn't working very well right now. We seem to be missing each other in some way. Any thoughts about what could be going on between us, or what I could do differently to make this work better?

**CLIENT:** Well, maybe you could talk a little more or give me some more feedback. You're pretty quiet, and I don't really know what you're thinking most of the time.

**THERAPIST:** I'm glad you're telling me this—thanks for your honesty. Sure, I'll be happy to start sharing more of my thoughts with you—that'll be easy to change. I'm wondering what it's like for you when you don't know what I'm thinking?

**CLIENT:** Well, maybe when you're quiet it's like you're kind of judging me or something.

**THERAPIST:** “Judging you.” That sure wouldn't feel very good. Let me clarify that no part of me has felt judgmental toward anything you've said, and if I ever was, it would certainly be my limitation or problem. And, maybe you're also telling me that others have judged you too much in your life?

**CLIENT:** Oh yeah, my husband is so critical. And I guess my mother disapproved of just about everything I ever did...

In this way, taking the risk to explore the client's resistance will often “uncover” or reveal key concerns (e.g., “judging”) that (1) impede the client's participation in treatment, and (2) are central to his/her problems and clarify the treatment focus.

### THE CLIENT'S RELUCTANCE TO WORK WITH RESISTANCE

Unfortunately, the client usually shares the therapist's reluctance to address resistance. Clients often are unaware of their resistance and externalize it to others or outside events. For example, the client may say,

**CLIENT:** Yeah, I'm late for our sessions, but the traffic is always so bad.

Although there usually is some truth to this statement, if this is a *consistent* behavior pattern, it begs the need for further exploration. To acknowledge both the reality-based constraints that are possibly contributing, and the more personal or psychologically based resistance that is often in play as well, the therapist can ask whether leaving earlier for the appointment is possible or if an alternative session time needs to be negotiated. In tandem, the therapist also begins to explore any concerns or other thoughts the client might be having about treatment or the therapist:

**THERAPIST:** Can you recall what you might have been thinking about our session last week as you walked out to your car or as you drove home afterwards?

OR

**THERAPIST:** I'm wondering if you had any thoughts about me, or concerns about our work together, during the week?

OR

**THERAPIST:** I'm wondering what people in your family, or friends at church, might think about you coming to counseling and talking about personal problems?

There is no criticism or blame here. And, even if the client is not ready to recognize or talk about whatever psychologically based resistance may be operating, the therapist has invited the client to consider the possibility of psychological factors and laid the groundwork for further discussions. This is part of educating the client about the treatment process and changing the social rules that define their interaction. That is, the therapist is making it “normal” to talk about their relationship and explore what goes on between them—setting new expectations that are different from what goes on in most other relationships in the client’s life. To illustrate:

**THERAPIST:** I know other people don’t usually talk this directly, but one of the best ways I can help you is if we can talk together about any concerns you might have about our relationship or the way we’re working together.

Sometimes clients recognize that it is hard for them to engage in treatment, and they usually feel frustrated and confused by their own contradictory behavior. For example, clients may exclaim with dismay:

- *Why do I go to all the trouble and expense of coming here to see you, then I can’t think of anything to say as soon as I walk in the door?*
- *Why would I forget our next appointment after we had such a great session last week? It doesn’t make sense!*
- *Why do I keep asking you for advice, and then say, “Yes, but...” to whatever you suggest? What’s wrong with me?*

If it becomes clear to clients that they are resisting in some way and **sabotaging** their own efforts, they typically evaluate themselves harshly—as being bad or failing in some way—and assume that the therapist shares this critical judgment. Thus, when the therapist begins to inquire about resistance, most clients want to avoid the topic because they are afraid the therapist is going to blame them—for not trying hard enough, failing as a client, being unmotivated, and so on. Because it is never helpful for the client to feel blamed or criticized, the therapist tries to make this self-critical attitude overt and helps clients reframe their critical attitude toward their own resistance. How do we do this? First, *the therapist wants to help clients learn how to honor their resistance as an outdated coping strategy that originally served a self-preservative and adaptive function for them*—and actually was the best possible response they had available at an earlier time in their development. Second, the therapist helps clients realize that they no longer need this coping strategy in most current relationships, such as the present relationship with the therapist. Instead, they can work with the therapist to learn more flexible ways of responding, first with the therapist and then with others in their everyday lives. Reframing clients’ resistance with this contextual or developmental understanding helps in many ways, especially by inviting self-acceptance and understanding rather than self-criticism and shame (Neff, 2011). This is a complex and far-reaching intervention. Addressing the resistance or coping style with compassion and providing understanding for why it developed is essential—it provides clients with an opportunity to have compassion for themselves and thereby begin the process of change.

Often, clients' resistance, and even the symptoms that bring them in for treatment, once served as a survival mechanism that was necessary, adaptive, and even creative. For example, anxiety signals danger—perhaps not a current threat but a past or developmental problem that really did exist. Similarly, depressive symptoms may reflect clients' attempts to cope with painfully unfulfilled longing for protection or love from a caregiver. It may also be a way to cope with the exploitation or derision that they experienced and continue to expect from others (e.g., shut down, avoid and withdraw, and stop feeling). In other words, formative experiences with caretakers and current interactions with significant others have given clients very good reasons for not wanting to ask for help, share a vulnerable feeling, or risk trusting someone. If the therapist and client explore patterns in how significant others are responding now, and have responded in the past, the client's resistance will make sense. It's not "irrational"—it's understandable developmentally, which is enormously validating and empowering for the client.

To illustrate, suppose a client is having trouble entering therapy. The therapist is encouraged to use process comments and work in the moment to highlight or clarify these unwanted but expected relational patterns:

- *If you and I begin to work together on your problems, what could go wrong between us?*
- *What could I do that might hurt you, or make things worse, if you take the risk to let me try and help you?*
- *How could our religious, ethnic, or gender differences get in the way and be a problem for you and me—for our work together—here in therapy?*

Routinely, questions like these reveal the most meaningful issues in clients' lives. In particular, they highlight what the client is afraid of or doesn't want, yet expects, because that is how significant others have indeed responded in the past. In other words, these process questions serve an assessment function and help shape the treatment focus. They clarify the key issues in the client's life, making clear to therapist and client alike what's really wrong.

Such process-oriented questions help the therapist and the client *identify* the aversive consequences that followed when the client asked for help in the past. Routinely, clients will have very specific answers to these questions and describe in detail the interpersonal scenarios that typically transpired in the past. When they asked for help in their family, for example, they were ignored and felt powerless, were made fun of and felt ashamed, or were told that they were selfish or too demanding. Some have been invalidated and told that they didn't really need or want what they asked for—and they should just stop whining or complaining about this. Others may have learned that anything they received brought unwanted obligations or that the nonverbal communication they received was that anything they needed or asked for burdened their caregiver and induced guilt. In this way, therapists and clients together can clarify together the very good reasons why clients originally learned to respond in these ways that once made sense but now have outlived their usefulness and become maladaptive (Tsai, Yard & Kohlenberg, 2014; Wei & Ku, 2007).

*Marjorie, a 45-year-old schoolteacher, came in because of increasing depression that now involved long periods of isolation, crying bouts, and worrying about how long she could "keep up appearances." During the first session she described a long history of emotional deprivation and neglect: her mother, who was probably suffering from a Major Depression, drank herself to sleep most days by mid-afternoon. By the time Marjorie was 10 years old, she generally made her own dinner, ate alone front of the TV, and put herself to bed. Desperate for connection with others, Marjorie found herself helping many of the neighbors with their chores and errands as she became the "neighborhood helper." Some saw this young girl's dilemma and were kind, at times inviting her to dinner. The reality, though,*

*was that whenever someone was kind to her, Marjorie overextended herself—to use her own word, she “enslaved” herself in gratitude for their attention. By age 45, Marjorie was “depleted” by her “enslavement.” As the session ended, the therapist, aware that Marjorie had seemed relieved to tell her story, wondered how Marjorie would later feel after being responded to with such validation and compassion. Might this support raise concerns for her about having to give back and respond to yet another person?*

**THERAPIST:** Marjorie, our time is almost over. How was it for you to be here today, and how will it be for you to come back and see me next week?

**CLIENT:** You listened and I felt understood.

**THERAPIST:** Good. I’m glad you felt that way.

**CLIENT:** Yes, it was good to talk with you. I’ve been isolated for a long time, afraid to get close to people again...

**THERAPIST:** Given what we have been talking about today, I’m wondering if it might be hard to come back next week, if you might be afraid that you’re going to have to take care of me, too?

**CLIENT:** Hmm, that’s a thought...I do like talking to you—I feel so alone. But that’s right—I could get worried. How would I ever repay you? I know this is your job, but I shouldn’t be leaning on you so much, I’d owe you too much...(tearful)

**THERAPIST:** Yes, and then you are “enslaved” again—but this time with your therapist. I’m so glad we are talking about this so we can make sure it doesn’t happen here between us. You and I can change this old pattern that has depleted you so much. That pattern helped you as a child—it gave you connection to some caring neighbors, so it makes sense. But—you don’t need it anymore, not with me at least. You and I can be connected without depleting yourself and without having to “enslave” yourself to me.

**CLIENT:** I’d like that—I want to come back, but I am scared. How will I know if this will be different?

**THERAPIST:** Let’s keep checking in to make sure you’re not feeling like you “owe” me, or that I am doing anything that makes you feel like you have to take care of me—like you’ve had to with others.

In this way, exploring the client’s resistance commonly reveals an *outdated coping strategy*—not a lack of motivation (Wei & Ku, 2007). Marjorie continued in treatment and made significant progress. But without exploring what could go wrong for her with the therapist, Marjorie may have been challenged to return to the next session, even though she liked her therapist very much. To begin learning how the therapist can start to change this outdated or maladaptive coping strategy, we introduce a four-step sequence therapists can follow: *Identify, Validate, Track, and Generalize*.

First, the therapist is trying to help clients *Identify* their outdated coping strategies with others (such as being perfect, withdrawing, intimidating, complying, etc.). As we have just seen with Marjorie, for example, the client who always takes care of everyone else, at the expense of her own needs, and feels burned out and “used” at work and resentful at home. *As part of this sequence, the therapist also helps clients understand why they originally needed to cope or protect themselves in these ways.* For example, the client’s caregiver was anxious, drug-involved, or depressed, and she grew up taking care of her parent rather than being taken care of. Another aspect of identifying this outdated coping strategy is to clarify how clients may be continuing to do this now in their current interactions with others—including the therapist. For example, such clients commonly don’t share difficult feelings because they don’t want to “bother” or “burden” the therapist.

Second, the therapist *Validates* the protection this interpersonal coping strategy once provided. This strategy (always taking care of others, not meeting one's own needs), is now causing problems in the therapeutic relationship—as it is with others. However, once it was a strong and adaptive response to a reality-based problem. For example, Marjorie taking care of others preserved something of an attachment to her unresponsive caregiver, and it became the basis of her identity and the source of what little self-esteem she could garner through her ability to meet the needs of others. Affirming the validity and original need for this coping strategy is often an empowering liberation to clients as this new understanding leads them to feel more self-acceptance rather than self-blame (e.g., Client: “So, I guess I’m not just stupid and hopeless. I learned to sabotage my successes for very good reasons”).

Third, the therapist holds a steady intention to *Track the process dimension and ensure that the therapeutic relationship does not repeat this problematic pattern* that has transpired so often with important others. The therapist can provide a corrective emotional experience—rather than an unwanted reenactment—by checking this out with the client and asking:

**THERAPIST:** At work you take care of everybody and everything, at home you do the same—and understandably feel resentful and exhausted. So, as we begin to clarify this unwanted life-script of yours, it makes me wonder if it ever feels like you are taking care of me in some way, too?

Therapists often sit in disbelief as many clients respond “yes” to process questions like this and go on to explain how aspects of the old coping strategy they are describing with others are indeed being reenacted between them in ways the therapist never dreamed were possibilities. For example:

**CLIENT:** Well, I guess sometimes I worry that I’m burdening you with my problems. I can see from the picture on your desk that you’re a mother, so I know that you go home to a lot of needs yourself...

Fourth, the therapist’s ultimate goal is to *help clients Generalize or transfer this new experience or relearning with the therapist and apply it with others* in their lives. For example, this caretaking client could also respond differently and say “no,” the pattern isn’t repeating with the therapist:

**CLIENT:** No, you’re different than my boss and husband and daughter—I don’t have to take care of you. It’s nice, I can be responded to.

**THERAPIST:** Good, I’m glad this is one place where you can let that depleting role go and allow yourself to be responded to. Who else in your life can you break this pattern with—you know, respond to your needs mutually?

**CLIENT:** Well, I could probably do more of this with my sister, and one of my coworkers is pretty responsive.

**THERAPIST:** Let’s try to build on those relationships, and start looking actively for others where things could be more balanced.

By following this four-step sequence, over and over again, therapists provide clients with the real-life experience that, at least here, they no longer need to keep using the outdated coping strategy. More precisely, therapists can help them learn to *discriminate* between when they still need to respond in this old way (for instance, at work with their rigid and demanding supervisor) and when they don’t (such as with their supportive friend or cooperative coworker).



There is a great deal of complex but important information here—we'll return to this pivotal topic of outdated interpersonal coping strategies and work further with this four-step sequence of change. To sum up for now, we have seen that clients are far more likely to become stalled in therapy, or to drop out prematurely, if their resistance, ambivalence, or rupture is not addressed in a supportive and collaborative manner (see Bender, 2005; Knox, Adrians, Everson et al., 2013; Ogrodniczuk et al., 2005). One of the most important ways to engage clients in a successful therapeutic alliance, and to identify the key conflicts in their lives that need to be worked on in treatment, is to invite clients to discuss any concerns they may have about any aspect of being in treatment or problems with anything the therapist does. In this regard, Rhodes et al. (1994) asked clients who were satisfied with treatment what their therapists had done to resolve misunderstanding between them. More satisfied clients reported that therapists asked them how they were feeling about what was going on in their relationship, listened non-defensively and were willing to hear what the clients thought they were doing wrong, and apologized if they made a mistake or hurt clients' feelings. In sum, *the guiding principle is that if we make it easy and safe for clients to talk about their fear, ambivalence, or concerns about entering treatment, they will be less likely to act on them and drop out prematurely* (Cartwright, 2004). Before going on to illustrate further how to do this, we first need to learn more about recognizing when resistance may be in play for our clients, and how to distinguish it from reality-based constraints that need to be affirmed.

## IDENTIFYING RESISTANCE

Especially as they begin their training, it is common for new therapists to have clients who drop out of treatment within the first few sessions. This doesn't feel very good, of course, but as we have begun to see, there is much we can do to prevent these unwanted **premature terminations**. Too often, therapists do not consider whether they have been effective in providing empathic understanding, or invited clients to speak forthrightly about any questions or concerns they may have about entering treatment or the therapist (Kegel & Fluckiger, 2015). Instead therapists may just blame clients and conclude that they weren't really motivated or ready to look at their problems (which usually isn't the case, unless clients are entering treatment at someone else's behest). Or, therapists may blame themselves and think that they unwittingly made some irrevocable mistake in the previous session—which is usually untrue as well. Often, something else is going on with these premature terminations. Researchers have interviewed clients after treatment and found that *clients often report having some negative or conflicted feelings toward the therapist, which they hid from the therapist* (Hill et al., 1992, 1993). All of us need to remember and learn from this. More specifically, clients report that it often *doesn't feel safe* to bring up these negative feelings on their own and, generally, therapists do not want to talk about “you and me” and ask about potential problems or misunderstandings between them. As a result, many clients terminate prematurely because they are acting on, rather than talking about, their conflicted feelings about entering therapy, or unspoken misunderstandings or difficulties with the therapist that were not talked through and resolved. This unfortunate but common outcome is not necessary. There is so much therapists can easily do to help clients successfully engage in treatment and resolve the misunderstandings and difficulties that commonly arise. For example, the therapist can:

- *help clients identify when resistance may be occurring,*
- *approach this issue in a noncritical, welcoming manner that actively encourages clients to express fully any concerns they may have about the therapist or the treatment process, and*
- *help clients resolve these concerns by listening in an accepting and non-defensive manner, expressing appreciation for clients' willingness to talk about problems, and communicating a willingness to be flexible, take their concerns seriously, and work out mutually agreeable solutions.*

Beginning with the first bullet item, how does the therapist identify or know when resistance is occurring? Therapists never really know what any behavior means for a particular client, but resistance may be operating when clients *consistently* have difficulty participating in treatment. For example, after *reality-based constraints* have been considered (such as scheduling conflicts or fees), *psychologically based resistance* (e.g., getting help is inconsistent with familial or cultural norms, or the client is afraid of being judged or controlled by the therapist—as they have been in other significant relationships) is probably occurring when any of the following happen repeatedly or in combination:

- *The client misses multiple appointments or repeatedly comes late.*
- *The client has very limited hours available for therapy.*
- *The client asks to reschedule frequently or often cancels appointments.*
- *The client does not want to make a firm commitment to attend the next session (“I’ll probably be seeing you next week”).*
- *The client attends but remains vague or elusive about problems.*
- *The client becomes reluctant to talk freely (“I don’t feel much like talking about anything today”).*
- *The client avoids certain topics that have come up in treatment and have been important, or brings up highly significant topics only at the end of the session.*
- *The client has been working productively but abruptly suggests stopping and terminating before change has occurred.*

It is important to stress that the same behavior often means very different things to different clients. However, when clients have trouble attending or participating fully in sessions in one of these ways, some form of resistance is usually at work (Callahan, 2000; Meissner, 2007). In such cases, the therapist can begin to generate working hypotheses about the possible meaning this resistance holds.

## FORMULATE WORKING HYPOTHESES TO CLARIFY RESISTANCE

What is the threat or danger? On the basis of information the therapist has started to gather about the client’s current and past relationships, the therapist can begin speculating about the different ways that entering treatment could be aversive for the client. Therapists can formulate **working hypotheses** such as the following about the issues or concerns that are likely to make it difficult for this particular client to enter treatment.

- *Does this Christian client believe that her inability to resolve her problems through prayer means that she has failed in her faith?*
- *Is it guilt-inducing for this highly responsible parent, who grew up taking care of her own anxious and dependent mother, to ask for something for herself?*
- *Is it awkward or embarrassing for this older Latino male to seek help from this younger female therapist?*
- *Is it shameful evidence of his own weakness or inadequacy if this blue-collar worker has an emotional problem that he cannot solve by himself?*
- *Is it disloyal for this Asian client to discuss personal problems with someone outside of the family?*

Therapists may also keep in mind that resistance and defense are attempts to manage unwanted feelings of shame, guilt, disempowerment, and anxiety. Although clients may be unaware of it, *they often struggle with the worry that, if they continue in treatment or engage more fully with the therapist, the therapist will hurt them as others have in*

*the past*. This schema-driven concern won't come up in everyday interactions, but it will be activated when clients are upset, vulnerable, or in distress (when "hot cognitions" are activated during crisis). We will see how therapists can use process comments to: (1) help clients clarify and better understand these concerns about their psychological safety with the therapist; (2) learn how to assess whether the therapist, unwittingly, may indeed be responding in a familiar but unwanted way that the client has learned to expect in close relationships (reenacting); or (3) make overt how they are co-creating a new and different type of relationship than the client has come to expect (corrective or resolving). Thinking in this way is one of the most significant learning curves for new therapists. As they begin seeing clients, most trainees do not realize that many of the clients' strong perceptions and reactions to them are not based on the reality of what they have said and done with the client, but often is shaped by the clients' internal working models and what has transpired in their attachment relationships.

Next, let's consider one of the bigger meanings conveyed in the key question: "What is the threat or danger if you talked with me about that—what could go wrong between us?"

Therapists don't want to "play it safe" with clients and keep things on the surface. Instead, therapists are trying to be someone who is willing to go beyond everyday social conversation. Being friendly and nice is appreciated, especially in the beginning, but it doesn't carry us very far and won't be enough to help most clients make progress with enduring or serious problems. Thus we hold a steady intention to be looking for opportunities to engage clients more significantly. *We do this by reflecting the deeper meaning, inviting the stronger feeling, and approaching the central concern implied or alluded to in what the client just said.* Therapists need to be willing to risk personally engaging with what is most important in the client's life. Exploring the client's resistance, and asking the here-and-now, **immediacy** questions suggested throughout this text, is not for the faint of heart. These questions may include:

- *What's the threat here? What's going to go wrong between us if you disagree with me?*
- *What might I say or do that could make things worse for you if you risked sharing that with me?*
- *Is there a way I might respond to what you will share that you might expect but don't want?*

This way of responding will highlight clients' core concerns and put them on the table for discussion. It will also strengthen the working alliance by bringing real engagement, shared meaning, and greater intensity to the therapeutic relationship—which we have seen is the key to positive treatment outcome (Horvath & Bedi, 2002). Further, Ackerman and Hilsenroth (2003), clarify the association between being active, interested, and engaged with clients and establishing strong alliances. Thus, new therapists are encouraged to take the risk of responding to clients in this stronger, more forthright manner from the start to the end of treatment—even though it may go beyond their familial and cultural norms. **If the therapist has held a consistently empathic and nonjudgmental attitude toward the client, and has been willing to break the social rules and check out with the client how it is to talk together in this more straightforward way, most clients will welcome such honest communication.** In fact, most clients find it reassuring because the therapist is behaviorally demonstrating that they aren't alone with their problems anymore but have found someone who is ready and willing to join them and address what's really wrong. Like all of the ideas suggested here, watching a supervisor demonstrate this, and then role-playing and practicing with a classmate is usually necessary before trainees can put this into practice.

Therapists also can formulate hypotheses that anticipate how clients are most likely to play out or enact their concerns about treatment and/or the therapist. For example,

suppose a therapist has observed that her depressed client feels guilty and believes he is selfish whenever he does something for himself or someone does something for him. The therapist doesn't know but is tentatively hypothesizing that this client may withdraw from treatment as soon as he starts to improve and feel better, or distance himself by disengaging when he realizes that the therapist genuinely cares about his well-being and their work together.

The therapist knows that these initial hypotheses may not be accurate and is ready to revise or discard them as more is learned about the client. However, the therapist will be able to respond more effectively if she can try to anticipate the concerns that might cause each client to drop out of treatment and the way each client is likely to express these concerns. Prepared with working hypotheses such as these, student therapists will be able to “get it” more quickly and be able to recognize better in the moment what is happening between them—not two hours or two days later when they are reviewing tapes of their sessions. They are then able to respond more effectively by helping clients explore their own resistance before they act on it and leave treatment prematurely.

Extending this further, as the therapist identifies the repetitive issues that keep arising throughout the client's life, the therapist is also better prepared to center treatment on these recurrent themes. Again, the therapist does this by highlighting them tentatively—observing or wondering about them aloud to the client as they occur. These patterns, tentatively suggested so the client is invited to modify or discard them, readily become useful and of interest to most clients when offered collaboratively—creating a shared treatment focus and providing a compass for the ongoing course of therapy (Binder, 2004; Levenson, 2004). This is part of the ongoing process of formulating and refining case conceptualizations (that is, what's really wrong) that will guide treatment plans (i.e., what the therapist wants to do to help with this problem) (Persons, Beckner & Tompkins, 2013). We are going to return to this related topic of treatment focus. However, let's turn now to applying these concepts about resistance in sessions with our clients.

## RESPONDING TO RESISTANCE

We need defenses in our lives; they often are adaptive and necessary to cope. Most clients we see in treatment certainly needed certain coping strategies to lessen anxiety, sustain attachment ties, and to defend or protect themselves from real threats and dangers, when they first learned them as children. And, in a reality-based way, clients often need them now in some situations and relationships as adults. A primary treatment goal for therapists with almost all clients is to help clients learn to *discriminate* between when certain defensive coping strategies are needed and when they are outdated and no longer serve us well. Most clients do not do this well—contributing strongly to most of the symptoms and problems they present in treatment. Throughout this text, we are going to illustrate how the therapist can help clients assess when their coping strategies are still necessary and helpful, and when they are no longer useful or needed. For example, becoming quiet and developing the chameleon-like ability to make oneself almost invisible in a room full of people—in order to go unnoticed and stay out of the line of fire—once was an effective coping strategy. It was necessary and adaptive while growing up with a hot-tempered parent and still is useful at the office for the same client who now is working for a critical, demanding, and volatile supervisor (at least until we can help this client find a different job or supervisor and stop this reenactment of a childhood dilemma). In contrast, however, being invisible and going unseen in this way is no longer adaptive or needed in the client's current relationship with the therapist, and will almost certainly cause problems in other personal relationships.

In this section, we learn how therapists can respond effectively to common expressions of resistance. Sample therapist–client dialogues illustrate effective and ineffective responses at three critical points: during a telephone conversation in which the client has difficulty scheduling the initial appointment; at the end of the first session; and during subsequent sessions with a client who has difficulty keeping appointments. As before, student therapists should find their own words to express the principles embodied in these dialogues.

## PART I: ADDRESSING RESISTANCE DURING THE INITIAL TELEPHONE CONTACT

While scheduling the initial appointment, the therapist wishes to obtain a definitive commitment to attend from the client. The client, however, may be ambivalent about entering therapy and, overtly or implicitly, express this during the initial telephone contact. The therapist’s aim is to attend to potential signs of resistance and find noncritical and non-blaming ways to broach this with the client if some aspect of resistance seems to be in play. If not, there will be a higher rate of no-shows for the first session.

**An Uncertain Commitment.** In the first example, the client expresses only minimal uncertainty about attending the first session.

**THERAPIST:** It’s been good talking with you, and I’ll plan on meeting with you on Tuesday at 4:00 P.M.

**CLIENT:** OK, I guess I’ll see you then.

**THERAPIST:** I’m wondering if you feel a bit uncertain as you say, “I guess.” Maybe there’s nothing to it, but I’m wondering if we should talk together for a minute about how it feels to come in? Do you have any questions or concerns I can try to answer for you?

**CLIENT:** Well, maybe I’m a bit cautious about most new things I try, but I do want to come and will be there on Tuesday.

**THERAPIST:** Good, I’m looking forward to meeting with you. We can talk then about that little bit of cautiousness, if you’d like, and any questions you may have about me or treatment. See you next Tuesday at 4:00 P.M.

An effective response to the client’s ambiguity (“I guess...”) is modeled in this dialogue. The therapist hears potential indecision in the client’s vague commitment, and approaches it directly but sensitively by asking about it in an exploratory, non-blaming manner. An ineffective response would be for the therapist to let it pass by on the assumption that it probably doesn’t mean much. Perhaps it doesn’t, and chances are that most clients will still arrive at the appointment time. However, the chances of the client not showing up for the first appointment are far greater if the therapist does not acknowledge these signs of uncertain commitment and invite a non-defensive dialogue about this potential resistance. Furthermore, although this ambiguous comment may seem too subtle or small to be significant, it is a statement about the counseling relationship—about you and me. Treatment benefits when therapists track clients’ comments about the therapeutic relationship and offer open-ended bids to explore them further, even though this is not how most new therapists have been socialized.

In addition, the therapist can mentally note this indecision as a possible sign of resistance—that something about beginning may not feel quite right. This is an instance where the therapist can begin to generate working hypotheses, derived from our three orienting questions, about the possible meaning of this “cautiousness.” As therapists meets this type



of client and listen to the narratives of his or her life, they can begin to consider tentative working hypotheses about other material related to “uncertainty”—or discard it if they don’t hear this theme in other arenas of the client’s life.

**A More Ambivalent Client.** Now imagine a similar telephone conversation, but with a different and more resistant client this time.

**CLIENT:** OK, I guess I’ll see you then.

**THERAPIST:** I’m wondering if there’s a bit of uncertainty when you say, “I guess.” Can we talk for a minute about how it feels for you to come in and see me?

**CLIENT:** Well, in my family, we don’t really talk to others about family problems.

**THERAPIST:** I respect your concern about being loyal to your family—I can see they are important to you. But I’m also hoping that we can find a way to balance things. Maybe one of the things we can talk about is how you can ask for help from others and get your own needs met in a way that still respects your family—you know, a way that leaves you with the feeling that you aren’t being disloyal.

**CLIENT:** I’d like that. My family is really important to me and maybe another part of this is that sometimes it seems like psychologists just want to blame parents for everything. That doesn’t feel right to me.

**THERAPIST:** Your concern makes sense to me. I’m glad you brought it up. That wouldn’t feel right to me, either. I’m hoping that we can better understand what’s wrong and causing problems in your life, so you can make the changes that you choose to make. So, I’m looking to understand problems better, not blame. That’s a big difference.

**CLIENT:** I like that. OK, I’ll see you on Tuesday.

**THERAPIST:** Good. I’ll look forward to meeting you Tuesday at 4.

An ineffective response would be to accept the client’s uncertain commitment (“OK, I guess I’ll see you then”) and avoid the possibility that an important issue about what it means for this client to have a problem, ask for help, or enter treatment is being played out. In this dialogue, the therapist has tried to “hear” and respond to the client’s concern instead. By affirming both sides of the client’s ambivalent feelings (balancing personal needs with family loyalties); differentiating herself from other psychologists in the client’s worldview (trying to understand not blame); and simply being accepting and respectful of the conflict, the therapist has done much to help this client take the first step and enter treatment. Without the therapist’s follow-up on a small, ambiguous statement, however, this highly workable client likely would not have been able to enter treatment successfully.

Let’s look further at why therapists in both dialogues would take this stance and press for a clear commitment to attend. Wouldn’t it be more supportive to let clients leave the appointment a little bit tentative, if that is what they need to do? No. Little therapeutic change will occur until clients take responsibility for the decision to enter therapy and work on their problems. Therapists do, however, need to be flexible. As we will see shortly, for example, therapists can give clients permission to make only a very limited commitment—for only one session or just a few sessions—to get a better sense of whether treatment, or this particular therapist, is right for them. Without this commitment, however, the therapist has little to work with. In fact, it is better for the client to remain out of therapy than to enter without commitment and have an unsuccessful therapeutic experience. As Yalom (1981) emphasizes, the therapist wants to be concerned about preventing failed hope in certain clients. If clients have one or more unsuccessful therapy experiences, it may discourage them from trying to seek help with their problems again. When

a client makes a commitment—even if only to attend one session, the therapy process becomes a joint endeavor and therapist and client become partners or collaborators in the work—which facilitates positive treatment outcome (Tyrone & Winograd, 2002).

**Clients Who Try to Make the Therapist Responsible.** Let's continue with a different telephone conversation that new therapists are also going to encounter with prospective clients. Some clients will try to make the therapist (and others) take responsibility for their decisions.

**CLIENT:** I'm not sure if I should start therapy or not. What do you think I should do?

It is important to reach out to clients and actively welcome them into treatment, but the therapist does not want to resolve clients' ambivalence by taking responsibility for their decisions or being a cheerleader and trying to provide the motivation for treatment. With some of these externalizing clients, a helpful response might sound something like this:

**THERAPIST:** From what you've just been telling me, it really does sound like this has been a difficult time for you, and I would like to try and help. But, since you are uncertain about coming to therapy, I suggest that you decide if you would like to come in for just one session and see how you like it. As we talk about what's going on for you, I think you'll get a good sense of treatment, and what it's like to talk with me, and you will be able to decide for yourself whether you'd like to continue or not. How does that sound?

OR

**THERAPIST:** I can't offer you any guarantees of course but, yes, I do believe that therapy may be of help. I'd like to try and work with you, but I suggest that we agree to meet for just one time. After we've had the chance to talk together, I think you'll have a good sense of me and how I work, and will be able to decide for yourself what you want to do. At the end of the session, we can check in on this and talk forthrightly about how it's been to meet and whether you want to continue. If we're not a good match, I can easily give you other referrals.

As we would expect on the basis of client response specificity, such direct invitations may be useful at times. When the therapist discerns that this client may have felt unseen, dismissed, or even rejected while growing up, or otherwise come to expect that others are uninterested and do not really care, it becomes important for the therapist to express genuine interest or even enthusiasm in working with the client. However, even when therapists are trying hard not to repeat the problematic relational patterns the client is describing, they still want to respond in ways that ensure clients take responsibility for their own decisions to enter treatment. Therapists do not want to cajole, coerce, or win clients into therapy. At the same time, therapists do not want to be aloof or indifferent. Thus the therapist is trying to meet clients where they are—working supportively with their ambivalence or conflicts about entering—yet without taking responsibility for the clients' choices. In many cases, an effective response to the client's question about entering treatment may be as follows:

**THERAPIST:** It seems as if one part of you wants to be in therapy, but another part of you doesn't. Tell me about both sides of your feelings.

OR

**THERAPIST:** It sounds like a part of you wants to try this and a part of you doesn't. Why don't we agree to meet for just one or two sessions? Let's see if we can work together and sort through both sides of this, so you can decide for yourself what you'd like to do.

The next client in the following dialogue is providing the therapist with potentially important information. As before, the therapist can begin generating working hypotheses about what this behavior might mean. Having generated hypotheses, the therapist can then be alert for evidence of these themes in other areas of the client's life. If, for example, there are narratives about uncertainty or avoiding personal responsibility that keep coming up and prove to be enduring relational themes, the therapist can use these working hypotheses, which have now gained further support, to help provide the client with a focus for treatment. For example:

**THERAPIST:** You know, Marie, I'm hearing the same theme again with your landlord that you were talking about last week with your boyfriend—you become quiet and “go along,” rather than speak up for yourself when you disagree. What do you think about this possibility?

The therapist does not want to become wed to these early hypotheses, however, and needs to be ready to discard them if the theme does not keep coming up or hold real feeling for the client. New therapists can be reassured that as they become more experienced, their initial hypotheses will become more fruitful. This developing skill will become an especially important aid to counselors as they begin working in short-term and crisis intervention modalities that require quicker, more accurate assessments during the initial phase of treatment.

As we are emphasizing in the interpersonal process approach, the therapist's overarching intention is to try not to get stuck in an ongoing way that reenacts the client's faulty patterns in the therapeutic relationship. However, this can easily occur in a metaphorical or encapsulated way in the initial negotiations between therapist and client. This is another reason why the therapist does not want to take over and tell the client seeking advice what to do. For example, the following response is well intended, but it is not going to be helpful to many clients:

**INEFFECTIVE THERAPIST:** On the basis of what you've told me, I think therapy will help you and believe you should give it a try.

Assuming such a directive stance reenacts a problematic relational scenario for many clients and confirms a maladaptive schema. Many clients are hampered in life by the faulty belief that they cannot act on what they want, pursue their own goals, or advocate on their own behalf. Others originally learned in attachment relationships that caregivers needed them to remain dependent to assuage the caregiver's own anxiety. Now, as adults, these clients continue to act dependently and allow others to take responsibility and make decisions for them in order to remain connected.

Although such clients will also believe that they need to ask the therapist to tell them what to do, paradoxically, the healthier part of them that wants a more independent Self will resist the control they have just elicited. Clients with these **compliance dynamics** (sometimes referred to as “excessive accommodation”) then have to resist the therapist—and reject the good help the therapist may be offering as well—or they are further complying and losing even more of themselves by being helped and getting better in treatment. As this common compliance scenario continues, clients come to feel badly about themselves—confused as to why they are rejecting the help they have just asked for, and guilty about rejecting the therapist who was trying to help. Therapeutic responses that give the client responsibility for deciding whether to follow through and enter treatment prevent this reenactment from occurring, and will provide some clients with a new and corrective response that challenges the old schema and begins to change a long-standing problem.

## PART II: EXPLORING RESISTANCE AT THE END OF THE FIRST SESSION

Toward the end of the first session, no matter how well it seems to have gone, the therapist is encouraged to ask clients how the session felt to them, and whether they have any concerns about the treatment process or the therapist. Unless the therapist is willing to inquire about potential problems in this straightforward way, such concerns are probably going to remain unspoken. Recall that follow-up research with clients after treatment indicates that many do indeed have some disappointments or difficulties with the therapist or some aspect of treatment, *but they usually do not bring these up or address these on their own* (Hill et al., 1992). Failing to inquire about and then sort through whatever concerns the client might be having is an opportunity lost. Many clients feel some reticence about entering treatment. And, at times, almost all clients will misperceive what the therapist is thinking about them or feel that the therapist misunderstood something important they have said. Such misperceptions and misunderstandings are an inevitable part of all relationships—and may occur because of the distortions shaped by the client’s maladaptive schemas or because the well-intended therapist simply misunderstood or did not grasp something that was meaningful to the client. Similarly, most clients will be uncomfortable at some points in treatment with how the therapist is responding to them (feeling, for example, that the therapist is being too quiet, too directive, etc.). All of these clients will be less likely to drop out prematurely and will be better able to make progress and change in treatment, if the therapist has:

- *explained that these misunderstandings are expectable and given the client overtly spoken permission to talk about whatever concerns may come up,*
- *overtly welcomed hearing any concerns and listened non-defensively and fully, and*
- *taken the client’s concerns seriously and demonstrated a sincere effort to sort through them and resolve whatever the client risked expressing.*

**Addressing Interpersonal Conflict.** To illustrate, five or 10 minutes before the end of the first session (to allow clients enough time to talk about their reactions to the therapist and the session), the therapist can check in with the client:

**THERAPIST:** How was it to be here today?

Most clients will answer, “Fine.” Perhaps thinking that the student therapist is feeling insecure and may be seeking some reassurance, clients may go on to say how helpful the session has been. If so, the therapist might respond as follows:

**THERAPIST:** Good, I’m glad you’ve found our first session helpful. You’ve told me a lot about yourself today, and I feel we have gotten off to a good start, too. As part of our work together, I think it will help us if we can establish an important ground rule and talk about our relationship more directly than others usually do in your everyday life. I’m hoping that it will feel safe enough for you to be able to tell me about what does and doesn’t feel good about our interaction, and things that we could change or do differently to make our work more helpful. So, I’m wondering if there was anything about coming today, or anything about our work together, that didn’t feel quite right? If so, I would really welcome talking about it—you know, so we could change things and make it better.

**CLIENT:** Oh no. I was eager to begin, and you have been very nice and understanding.

**THERAPIST:** Good. In the future, though, if something does come up—and it probably will—it will help our work together if we could talk about it. How would it be for you to do that? Would that be OK or maybe kind of hard for you?

**CLIENT:** Well, I don't know...that might be kind of difficult for me. We were taught to be polite in my family, you know.

**THERAPIST:** Absolutely. It is important to be polite, but I'm hoping we can be both polite and honest with each other, so we can solve any problems that might come up between us. One of the things I value about therapy is that this is a place where we can talk about whatever is most important to us, including our relationship and what's going on between us that does and doesn't feel good. Often that can be things we can't do so easily with others out in the world but, in here with me, I welcome it. That way, we can always be up front, respectful, and also know where we stand with each other. How does that sound to you?

(Here, clients might respond in a variety of ways)

**CLIENT:** I like the idea of things being up front...I'm not used to doing that so it might be hard at first;

OR

**CLIENT:** Hmm, what if I have something to tell you about you that I don't like?

OR

**CLIENT:** Well, I don't know. I'm a Christian and I believe it's important to be respectful.

OR

**CLIENT:** So you're saying we can just get down to brass tacks here and talk about what's really important, and not beat around the bush and worry so much about what you might be thinking—or try so hard to be nice.

In this vignette, the therapist is educating the client about the treatment process and trying to establish an important set of expectations for what is going to occur in therapy.

The therapist is telling the client that, in contrast to many others, the therapist wants to hear about the client's wishes and concerns, and address potential problems between them straightforwardly. Because straightforward communication and, especially, talking directly about conflict or personal problems between “you and me,” was not allowed in their families of origin, many clients will be surprised by, but welcome, this invitation. Here, the therapist is laying the groundwork for a relationship that is more direct and authentic than most clients have enjoyed in the past. Therapists want to establish these important expectations with their actions, and not just their words, early in treatment. As we emphasized earlier, there will be ruptures or misunderstandings in the therapeutic alliance. It's part of the treatment process—not a mistake (Safran et al., 2002). Effective therapists repair the inevitable ruptures that occur by making process comments, asking about potential problems, and sorting them through (Safran, Muran & Eubanks-Carter, 2011).

Therapeutic relationships will remain superficial unless therapists are willing to broach these expectable ruptures and take the risk to talk them through. This is challenging for most new therapists, so further help with this far-reaching issue of rupture and repair follows—it is essential for successful treatment.

**A More Assertive Client.** As previously noted, approaching interpersonal conflict in these direct ways is often new and anxiety-arousing for student therapists because it may violate familial and cultural norms. Let's continue with an assertive and critical client.

**THERAPIST:** I see that we're almost out of time. How has it been for you to talk with me today?

**CLIENT:** Well, OK for the most part, but I did feel you were trying to hurry me a few times.



Clinical training holds many challenges, but for many student therapists none is bigger than learning to approach interpersonal conflict—in the moment as it is occurring between the therapist and the client. We do not want to minimize or avoid it by moving on to another topic, truncate it with a punitive response, or vaguely express hurt and communicate that the client's complaint is in any way too much to handle, or unwanted. Instead, the best way to approach conflict is by encouraging clients to express their concerns more fully and, as hard as it may be to do, strive to remain as *non-defensive* as you can and listen with a receptive, open mind.

**THERAPIST:** Thanks for speaking up—it helps our work together when you bring up problems like that. Let's look at this together and see if we can figure out what happened for each of us. Tell me more about your feeling of being “hurried up.”

Of course, it is often difficult for new therapists to do this—just as it is for many managers in the workplace, teachers in the classroom, and, unfortunately, many experienced therapists as well. Such direct, conversations about conflict between “you and me” may arouse the therapist's own anxiety, especially when the therapist has strong needs for the client's approval. If therapists are to approach such anxiety-arousing ruptures and restore the therapeutic alliance, however, *they must first become aware of their own characteristic responses to interpersonal conflict*. For example, the automatic, knee-jerk reaction for some therapists is to avoid the client's complaint and move on as if nothing significant was said. For other therapists, the initial response tendency is to agree readily with the client and quickly apologize, in order to abate the criticism. Still other therapists will automatically begin to justify themselves and offer lengthy explanations aimed at clarifying their good intentions and stopping the criticism. Disturbingly, a long line of researchers find that a significant number of highly experienced therapists actually counter with their own hostility and punitiveness, and criticize the client in turn (Ackerman & Hilsenroth, 2001; Binder & Strupp, 1997; Najavits & Strupp, 1994). *The first step in learning better ways of responding is to become more aware of how we typically tend to react to criticism, negative evaluations, or unwanted confrontations*. Research shows that rupture resolution is important to retaining clients in treatment (Muran et al., 2009), and is related to treatment outcome across different types of therapy (Safran & Kraus, 2014).

One of the best ways to develop this self-awareness and learn more about how we usually respond to interpersonal conflict is to *recall how it was dealt with in our own family of origin*. Regardless of age, our initial reactions to conflict often follows closely how our parents dealt with conflict in their marriage and how each parent dealt with conflict with in his or her childhood. Rather than automatically following their initial response propensity, therapists can learn to approach conflicts with more openness. With help from a supportive supervisor, new therapists can learn to do this effectively in a year or two and, before too long, feel comfortable and actually welcome misunderstandings and conflict as a therapeutic opportunity.

After inviting clients to express fully their concerns, *the therapist is not in a hurry to explain “what really happened” between them or disagree with the client's perception*. Often the client is right and the therapist indeed was “hurrying” the client and, most commonly, the client's perception is partially accurate and partially distorted—there is something to it. Thus the therapist's intention is to be non-defensive, examine his or her own behavior, and try to consider what accuracy there may be to the client's perception. If we look from the clients' perspective the therapist does not need to be afraid of acknowledging this to clients.

**THERAPIST:** You know, I think you might be right. I was aware of the time going by, and I wanted to touch on some other issues before we had to stop. I probably was hurrying you there, and I can see why that bothered you. I'm glad you made me aware of that; I'll try not to do it again. You know, I like your honesty—it's a real strength of yours.

We have been keying on how, throughout their first years of training, many student therapists suffer significantly over their fears of making mistakes, hurting clients, failing to help, and so forth. Sadly, these worries keep many from enjoying their clinical training, and others from taking personal risks and trying out new ways of thinking about and responding to clients. Some caring and responsible students simply do not enjoy their years of clinical training because of these worries. It is liberating as new therapists learn that they do not need to be afraid of making mistakes; they need only to remain open to the possibility that they may have erred and be willing to acknowledge mistakes. In the previous example, the therapist is responding to the client in a reality-based and egalitarian way. The therapist validates the client's perception, which tells the client that the therapist is willing to have a genuine dialogue and risk entering into a real relationship. It also indicates that the therapist will respond to clients' concerns with respect. Too often, therapists and clients enact a relationship in which the client fulfills the role of the weak or needy one and the therapist is the healthy one who does not make mistakes or have problems. The responses suggested here discourage that illusion. The therapist's willingness to risk having an authentic relationship is a gift to many clients. The client finds that, at least sometimes, problems with others can be talked through and resolved. The relearning from such corrective experiences with the therapist often has a powerful effect on clients and precipitates change with others in their lives. For example, clients routinely come back to the next session with reports of having acted stronger with someone in their everyday lives, as they have just done with the therapist; such as, using their own voice and speaking up for themselves with someone who previously had been intimidating.

In contrast, therapists can handle the negative feedback in a way that keeps the relationship superficial or puts them in a superior position.

**INEFFECTIVE THERAPIST:** Oh, you thought I was hurrying you up, how was that for you?

OR

**INEFFECTIVE THERAPIST:** That's interesting. Do you ever feel this way in other relationships, too?

These responses are a misuse of the client-centered reflection or therapeutic neutrality. With this type of *deflection*, the message to the client is, "It's always your problem. I will not look at my own contribution here. This will not be an honest or real relationship." Such a response limits the relationship to a superficial encounter and sets up a covert power battle between client and therapist. Little therapeutic gain will be realized as long as their interpersonal process continues in this problematic way.

But what if the therapist does not agree with the client's complaint? We don't want to say anything we don't really mean. Thus, the therapist can still accept the validity of the client's perception, yet without agreeing to the comment.

**THERAPIST:** I'm sorry you saw me as being impatient with you; that wouldn't feel very good. I wasn't aware of being in a hurry or trying to rush you, but I'll watch out for that in the future. If you ever feel that happens again, stop me right then and we'll look at it together.

In this example, the therapist is telling the client:

- *I will take your concerns seriously even if I do not see it the same way.*
- *We can disagree and have differences between us, and still remain connected and work together respectfully.*
- *Your feelings about us, and our work together, are important to me.*

This type of response doesn't just tell clients that problems between them can be resolved. It behaviorally shows this to them. In turn, this lends hope that the therapist

and client may be able to resolve the client's problems with others as well. By addressing conflicts between the therapist and the client directly, the therapist provides the client with an important model of having interpersonal conflicts resolved in a new and more constructive way. Different theoretical orientations use varying terms for this powerful type of **experiential relearning**, which includes exposure trials, modeling, in vivo relearning, corrective emotional experience, and others. Especially for clients who have learned to avoid interpersonal conflict because it has been threatening or simply unresolvable, such experiential relearning is empowering indeed. Perhaps because this experiential or relational re-learning is more procedural or implicit than only verbal, it can lead quickly to behavioral changes (Renn, 2012).

**CLIENT:** Guess what? I spoke up at my big meeting at work! I didn't even think about it, I just jumped in and said what I thought. It was great—my boss seemed really surprised—and I think he's treated me with a little more respect since.

Finally, it is also possible that the client is systematically misperceiving the therapist's behavior in line with an early maladaptive schema. For example, this client may readily experience the therapist (and most other authority figures) as demanding more than the client would like to produce, and then being dissatisfied with whatever the client does produce. If so, this client may have grown up with a parent who repeatedly demanded that the client do everything on the parent's timetable or ignored the child's own accomplishments. Even if the therapist has gained further evidence to support such a hypothesis, this type of historical or transference interpretation should not be posed before the therapist and client have resolved the dispute in their real-life relationship. Therapists stand to lose a lot if clients see them as sidestepping a reality-based confrontation. However, the therapist can use this information to generate working hypotheses about cognitive schemas and faulty expectations that can be utilized later.

**Clients Who Test the Therapist's Adequacy.** Because it is so challenging for most new therapists to approach interpersonal conflict directly with their clients and work with ruptures in the therapeutic alliance, let's continue with another example. In the following situation, the client questions the therapist's competence during the initial session—which cuts quickly into an anxiety-arousing area for most new therapists. As we see, however, this sturdy therapist is able to remain non-defensive enough to approach the issue forthrightly and respond effectively by inviting the client to express that concern more fully.

**THERAPIST:** As you're saying that, I'm wondering if there might be something about our relationship, or being in therapy, that just isn't feeling good for you right now?

**CLIENT:** Well, you did say you were a student here. Isn't that what you said when we first spoke on the phone?

**THERAPIST:** Yes, I'm a second-year graduate student, working on my master's degree.

**CLIENT:** Well, I don't really feel like a guinea pig or anything, but you really are just practicing on me, aren't you? I don't want to be unkind or anything, but I'm wondering if you think you've had enough experience to help me?

**THERAPIST:** Thanks for bringing this up. Let me rephrase and make sure I'm hearing you accurately. Maybe you're saying that you are worried that if you go to all of the trouble of coming here each week, and talk with me about problems that are difficult to bring up, that in the end I just won't know enough to be able to help you?

**CLIENT:** Well, yeah, that's pretty much it. After all, you really are just a beginner, and I must be 20 years older than you. Do you really think you can help me?

**THERAPIST:** I cannot offer you any guarantees, of course, but I will do my best. And as we continue to meet together for two or three sessions, you will get a better sense of me, and what it's like to work together. I think you will be able to decide for yourself soon whether this is the right match for you—and we can talk together about that honestly. But for now, let's see if there are any questions I can answer for you about my training status, and how our age difference might get in the way.

In this vignette, the therapist was able to approach the client's concern directly. The therapist responded effectively by remaining non-defensive, even though this encounter was anxiety-arousing. The therapist did not act on an initial impulse, which was to try to reassure the client. Instead of responding to a personal need to seem competent to the client, the therapist was able to respond to the client's concern—by inviting the client to express reservations directly and fully. The therapist tolerated personal discomfort well enough to be able to discuss the client's concerns and, in so doing, behaviorally demonstrated competence. This is always more effective than offering verbal reassurances, which would only sound hollow to the client and still leave the burden of proof on the therapist.

**Clients' Concerns about Entering Therapy.** In the previous section, the therapist has inquired about resistance and the client has expressed concerns that personally challenged the therapist. In the following example, the therapist asks about the client's potential resistance (e.g., “How is it for you to come in today and begin talking with me about these problems?”) and the client expresses concerns about entering treatment—not about the therapist. Student therapists can be reassured that such concerns are far more common than challenges to the therapist and, of course, much easier for new therapists to address.

**CLIENT:** It's kind of hard. I guess I'm not used to talking about myself very much, and I've never liked asking for help.

The therapist's aim is to let clients know that he or she has heard their concerns and is trying to grasp the key message, core meaning, or biggest feeling. The therapist's intention is also to demonstrate that he or she takes the client's concerns seriously and is willing to be flexible and try to do something about them if possible. Again, the best way to demonstrate this empathic responsiveness is to invite clients to discuss their concerns more fully.

**THERAPIST:** You're saying a lot right there, and it does sound important. Tell me more about what it means for you to focus on yourself, on your own experience or needs, and bring your concerns to me for help.

**CLIENT:** I don't even know where to begin...it's like I've always been so concerned about everybody else that I've never really stopped to pay much attention to what was going on for me. Or maybe I just wasn't supposed to do that...you know, be selfish...

**THERAPIST:** I can see how important your family is to you, and how much you want to honor them. So why don't we agree that, here in therapy, you decide what we talk about and how much you want to share. You set the pace and we'll go at your speed in here. We don't need to talk about anything that you don't want to. And I'll join you there—with the things that you decide you want to talk about. How does that sound?

**CLIENT:** Yeah, I like that. That sounds good. If I feel like I'm betraying my family, I don't have to bring it up, but if I decide I need to talk about something anyway, then it's OK with you.

**THERAPIST:** Yes, that sounds like a good way for us to work together.

**When There Are Cultural Differences.** Listening and responding to the client's concerns with such affirming responsiveness is also important when ethnic or cultural differences exist between the therapist and client (McClure & Teyber, 2003). Cultural differences can be a sensitive issue; neither client nor therapist may feel comfortable addressing them. Sometimes it almost seems as if there is an unwritten rule against recognizing differences. As before, however, the best way to work with these important differences—which often are central to the client's identity and sense of self—is to acknowledge them in an open-ended way. Therapists want to give clients overtly spoken permission to be able to express any concerns they may have. To illustrate, suppose it becomes clear that the client and therapist are of different ethnicity, age, socioeconomic status, or religion:

**THERAPIST:** I wanted to check in with you about what it's like to meet with me. Was there anything about coming today, or talking with me, that could get in the way of our work together?

**CLIENT:** Well, I don't mean to be disrespectful, but I don't know if you can understand my situation because you're a different race...

**THERAPIST:** I'm glad we're talking about this, because our cultural backgrounds really are different. At times, there probably will be important things about you that I won't understand as well as I want. When that happens, help me out and tell me when I'm not getting it. We'll talk it through together until we've got it right. Was there something today that I missed or didn't understand that we should revisit?

The therapist responds affirmingly by accepting the client's concern, validating the client's point of view, and being willing to explore it further. It is important for therapists to be aware of differences between themselves and their clients as they work with clients who differ from themselves in terms of sexual orientation, social class, spiritual beliefs, race/ethnicity, and other factors. For example, some minorities may feel different in a predominantly White school or workplace—especially if few attempts are made to reach out and include them. In some settings, people who are different may be regarded as deficient. The therapist's willingness to acknowledge differences and invite clients to express any concerns or misgivings they may have will go a long way toward establishing a therapeutic alliance and keeping clients who may feel different from dropping out prematurely.

As we have seen, when obvious ethnic, religious, or other cultural differences exist, enlisting the client's help is even more apt. Ethnic, class, gender, and religious differences are complex, and it is counterproductive for therapists to labor under the misconception that it is their responsibility to understand everything about a client whose culture of origin differs from theirs. However, therapists do have a responsibility to educate themselves about clients from different social contexts, for example, by reading and consulting with informed colleagues (see Pederson et al., 2008; Sue & Sue, 2013). In particular, the therapist can invite the client to educate the therapist whenever the client feels the therapist is misinformed or does not understand. By welcoming this clarification instead of feeling threatened by it, the therapist signals acceptance of their very real differences, demonstrates openness to a genuine dialogue, and offers an invitation for a real relationship. In so doing, the therapist gains credibility with the client and strengthens the working alliance.

Therapists also need to explore more fully with clients how they believe their cultural or social context influences their current problems and shapes their subjective experience (Comas-Diaz, 2006; Constantine & Sue, 2005).

**THERAPIST:** Would you help me understand this problem in terms of the expectations from your family or community?

**CLIENT:** I can't discipline my children the way I want without being disrespectful to my mother. I'm Latina and my mother lives with us—she thinks I'm still her daughter, you know, and that it's her role to help out by taking care of the children. I love her very much and appreciate her help, but she's spoiling my son and she won't listen when I ask her to stop. She almost gets mad at me when I speak up—like she thinks she's their mother and she decides how to raise them—not me!

Harkening back to our core concepts in Chapter 1, even when obvious cultural differences are not evident, therapists will be more effective as they try to enter their clients' subjective worldview more fully (Speight et al., 1991; Wachtel, 2014).

To sum up, the therapist's intention is to encourage clients to express any concerns about treatment or the therapist that arise, and then sincerely try to accommodate those concerns. This will go a long way toward resolving them and allowing many more clients to enter treatment and successfully receive help.

### PART III: RESISTANCE DURING SUBSEQUENT SESSIONS

**Understanding Clients' Inability to Change: The Three Rs.** As treatment progresses, some motivated clients will have phases when they find it hard to enter or stay with certain issues that come up, or feel stuck and simply can't make progress in treatment. Too often, supervisors observe that student therapists disengage—quietly giving up on clients at this point—often doubting whether the client is “motivated” or questioning whether this client is ready to take action and start making real changes. More effective therapists, however, welcome these expectable impasses and use them as opportunities to explore re-enactments in the therapy relationship or reveal or identify new issues that can take the client further. Rather than blame or judge the client, and give up and stop trying to make a difference, effective therapists can use process comments and invite the client to join them in a collaborative, non-blaming exploration to understand the resistance or block:

**THERAPIST:** It seems to me that we've kind of gotten stuck. You know, like we just aren't moving forward the way we were before. What are your thoughts—how do you see things going for us at this point?

As we would expect from client response specificity, the reasons for clients' resistance are complex and varied. However, when therapy has stalled or clients have stopped making progress on a problem, therapists can begin making sense of things by formulating working hypotheses around three issues: *Ruptures*, *Reenactments*, and *Resistance*.

First, we have already seen that *Ruptures* occur when there has been a misunderstanding or conflict between the therapist and client that has disrupted the working alliance. The therapist repairs or “restores” these expectable ruptures by bringing them up and talking with the client about the potential problems with treatment or between them. This active engagement, open-ended questioning, and validation of the client's experience is also likely to facilitate the therapeutic alliance and client retention (Sheehan & Friedlander, 2015). For example,

**THERAPIST:** You sound irritated with me as you say that, James. I'm wondering if there may be a problem between us? If so, I'd sure like to talk it through—you know, so we can understand what's wrong and work it out.

Again, an enduring problem in the counseling field—cutting across every level of clinical experience and within every brand of treatment—is therapists' reluctance to address the



misunderstandings and misperceptions that so commonly occur and routinely disrupt the therapeutic alliance. As we see in the previous dialogue, the therapist is speaking directly to potential conflict but doing so collaboratively—in a non-blaming, noncritical manner. We want student therapists to be role-playing and practicing this skill in their training courses.

The second R, *Reenactments*, also hold back clients from being able to succeed in treatment and make meaningful changes in their lives. As emphasized, therapists are encouraged to consider the possibility that in some actual or metaphorical way, the same type of problem or unwanted interaction that the client is having with others is being played out between them. That is, *the treatment block is occurring because the interpersonal process between the therapist and client is reenacting, rather than resolving, some aspect of the client's problem*. For example, based on early maladaptive schemas, the client believes the therapist is feeling critical toward her, or privately has become absolutely certain that the therapist is deeply disappointed in her—just as her spouse and parent have been. As before, the therapist intervenes best by working in the moment and exploring the current interaction between the therapist and client:

**THERAPIST:** What do you think I might be thinking about you right now, Jenn, as you're telling me about this?

Or, suppose a different therapist chose to self-disclose a personal problem of her own—without realizing how pervasively this client had been parentified while growing up. Now, this client feels she is in her old familial role again and has to “take care of” the therapist, too. When such reenactments occur—as they predictably do for all therapists—the therapist can use process comments to help change the therapeutic interaction once it becomes clear that they have become stuck or progress has slowed. In the following example, a student therapist shared some personal family information with a client who was struggling with her parents' recent divorce and who was feeling shame about this family “failure.” In discussing this self-disclosure with her supervisor, the therapist explained her intentions and said she'd shared her own experience of divorce to “normalize” the reality of divorce and minimize the stigma the client appeared to be struggling with. Her supervisor appreciated her good intentions, but cautioned that the client seemed to have been parentified in her family of origin and even now, having moved out and left for college, **was still called frequently by her mother to get emotional support**. The supervisor suggested that she be alert for an opportunity to explore “caretaking” from this client:

**THERAPIST:** You asked about my own life and parenting, and I told you that I got divorced this past year. I'm not sure, but it seems like you have gotten quieter since. Maybe it's just me, but you seem a little more distant or reserved. Can you help me out here...could my feeling be telling us anything important?

**CLIENT:** Well, I guess I'm feeling sad for you, and concerned that you have had a lot of pain and suffering in your life.

**THERAPIST:** I appreciate your genuine concern for me, but I'm doing fine, and wondering right now if this is putting you in that very familiar but problematic caregiving role with me as well?

**CLIENT:** Well, maybe...that's what I do, I guess. I've been taking care of others for a long time.

**THERAPIST:** Uh-huh, so this has kind of put you in your old, familiar spot of having to take care of me, too. Well, if we continued that, it sure would end your opportunity to learn about and change that old role, wouldn't it? I'm so glad we're talking about this, so we can make sure that doesn't happen here. You've been a caregiver for others, at your own expense, long enough in your life and you don't need more of it with your therapist, too. But I can sure see, now, how talking about my own divorce triggered that familiar role for you. Have there been other times when this has come up between us—when you have felt that you needed to take care of me, too, in some way?

By being able to explore and talk together in this way about what *may* be occurring between them, the therapist and client can readily restore their working alliance, stop reenactments from continuing to play out, and resume productive work.

Early in their training, most student therapists will not possess the objectivity necessary to identify these predictable but subtle reenactments and will need assistance from their supervisors to identify when potential reenactments may be occurring. As well, new and experienced therapists alike benefit from personal exploration in their own therapy, *so they can become more aware of their own response styles and better anticipate how their own tendencies to respond could interact with clients' interpersonal patterns.* When reenactments are occurring between the therapist and the client, clients are set back in treatment because their maladaptive schemas are behaviorally confirmed rather than experientially disproved. When maladaptive schemas are confirmed, clients lose the *interpersonal safety* they enjoyed with the therapist and cannot risk further exploring their problems, so treatment commonly becomes stalled and proceeds on a more superficial or intellectualized level. However, once the ship has been righted and the therapeutic relationship again is enacting a solution to clients' maladaptive relational patterns, they can resume changing as these faulty beliefs and family roles are challenged and expanded through the interaction with the therapist, rather than confirmed (Silberschatz, 2005).

The third R, *Resistance*, will be re-evoked throughout treatment. As different issues are explored in treatment, clients will not be fully aware of all of the varying conflicts and feelings that have been activated by a certain topic. *Especially for clients whose feelings and perceptions were consistently invalidated as they grew up, they soon become unaware or "confused" about many aspects of their own subjective experience—what they are feeling or seeing.* Resistant clients are not lying or deceiving the therapist, and they have not lost their motivation—these can occur but they are different treatment issues. Resistance occurs when clients are simply unaware of the multiple or contradictory feelings that have been activated by acknowledging realistically that certain problems really exist, asking for help, exploring difficult topics with more specificity, and—surprising to most new therapists—even by succeeding in treatment, making meaningful changes, and getting better. For example:

- *Getting further help is relieving but may also arouse fears of being needy or weak.*
- *Feeling cared about by the therapist's genuine concern is comforting but can also evoke sadness about the many times this need went unfulfilled.*
- *Being listened to and understood in a consistent, reliable manner is reassuring but may also evoke fears of being controlled if they get closer to the therapist.*
- *Becoming more successful is empowering but arouses the fear of being envied, undermined, or isolated.*

Unless addressed and clarified, these commonly occurring reactions will lead to resistance. They can cause a few clients to leave treatment prematurely, and many more to sabotage the success or undo the changes they have achieved. For others, therapy will stall and becomes repetitious or intellectualized. At the beginning of their clinical training, however, many therapists are not familiar with the ambivalent, push-pull, or paradoxical nature of these conflicts. Let's explore this further by recalling Marsha from Chapter 2.

It was profoundly reassuring for Marsha when her second therapist approached her feelings directly and "heard" her and validated her experience. At the same time, however, this very positive experience also aroused other contradictory feelings that she "hated." Marsha now felt heard and understood but, at times, also felt:

- *sad, as years of unacknowledged loneliness welled up;*
- *angry, at not being heard, and being denigrated, so many times;*
- *guilty, for feeling angry at her parents and being disloyal to them; and*
- *anxious, for breaking unspoken familial rules and talking to someone about these problems and naming what was really wrong in her family.*

Marsha did not want to experience any of these feelings and, without being very aware of it, it was hard for her to stay with these treatment issues related to her family when these feelings were evoked. She did not understand them and felt threatened by them, yet she could not make them go away. Fortunately, her therapist was comfortable with the push-pull nature of conflict and consistently helped her make sense of the different, contradictory aspects of her experience that emerged. His accepting response helped her to recognize and integrate these conflicted feelings and gradually resolve them. In these ways, the second therapist was providing Marsha with **affective containment**. In attachment terms, this is a “holding environment” that provided her with the psychological safety she needed to address, explore, more fully experience or feel, and then share—these cardinal issues for the first time (Greenberg, 2014). As these changes occurred in the therapeutic relationship, Marsha began telling the therapist that she found herself “feeling more confident” than she had before and that she was getting better grades and making new friends at school.

At different points in treatment, clients will often be unaware of, and anxious about, the sequence of ambivalent or conflicted feelings evoked in the change process. As a result, they may feel misunderstood or even blamed when the therapist inquires about potential signs of resistance. Some clients may feel that the therapist’s questions imply that they are doing something wrong or are not trying hard enough. Concerned that the therapist is frustrated or disappointed with them, clients may try to justify their good intentions:

**CLIENT:** Oh no, you don’t understand. I really do want to see you and get here on time. It’s just that...

As already emphasized, therapists never want clients to feel blamed or judged. Rather, *the therapist is inviting the client to join in a collaborative exploration to understand the threat or danger that some aspect of treatment has aroused*. Once clients understand that the therapist’s intention is to explore the threat or danger, and not criticize, most readily join in a productive exploration.

Therapists can best approach potential signs of resistance with curiosity and a spirit of mutual exploration. Although new therapists are often concerned about being too blunt or being misperceived as critical, the problem actually goes the other way. *Because so many therapists are too worried about hurting their clients’ feelings or evoking their disapproval, some avoid these issues and others do not respond as forthrightly as they could*. Many new therapists dilute what they say to the client in order to mute their impact. If therapists are worried that their explorations may be seem intrusive or unwanted to the client, they can shift gears and find another way to respond. Better yet, they can check this out by just asking the client directly. For example,

**THERAPIST:** I like talking about these issues that come up between us—I think it helps our work together. But I wanted to check in and make sure that you’re feeling comfortable with this forthrightness, too. How is it for you to talk with me like this—is this going OK for you?

**CLIENT:** It was different at first, I wasn’t used to talking like this. But I like it—I feel like I’m finally talking to somebody about what’s really going on. My wife likes it, too. She says I’m speaking up and talk about things more than I used to, and she feels “in the loop.”

Researchers evaluating the effectiveness of the widely utilized treatment approach of Motivational Interviewing find that it is especially counterproductive when therapists push or argue against resistance, whereas collaboration and empathy from therapists facilitated change (Arkowitz et al., 2015; Moyers, 2014; White & Miller, 2007). Thus, the therapist’s intentions are to inquire about potential signs of client resistance in a curious, exploratory manner that invites a collaborative dialogue. As before, a tentative approach is needed. Therapists can merely wonder aloud with clients about the possible meaning certain behavior may hold; there is no argument or insistence on what a client’s potential

resistance might mean. Therapists can approach resistance without arousing defensiveness through a series of gradual steps that are progressively more direct; a three-step sequence is often effective.

### *Step One: Permission-Giving and Educative Response*

In the first step, the therapist offers a *permission-giving and educative response* to encourage clients to talk about the positive and negative reactions that come up for them about treatment or toward the therapist. In particular, the therapist explains that problems or misunderstandings with the therapist are inevitable, and the client can help by bringing them up:

**THERAPIST:** Can we talk about what it's like for you to come and see me? I noticed that you were 20 minutes late today and had to reschedule your appointment last week. Maybe it doesn't mean anything, but I was wondering if something about our work together might not be feeling right for you. Any thoughts come to mind?

**CLIENT:** Oh no, I really do want to be here. Things have just been crazy at work. You've been very helpful, really. I had a deadline last week and today it's just that my boss called as I was leaving.

**THERAPIST:** OK, but if in the future you ever find yourself having any concerns about therapy or the way we are working together, I would really appreciate it if you would speak up and tell me about them—you know, let me know about any problems or misunderstandings that might come up. This can happen between us just as it does in other relationships. How would it be for you to tell me about something I did that you didn't like or something about being in treatment that was uncomfortable for you?

**CLIENT:** Well, that might be kind of hard for me to do.

**THERAPIST:** Uh huh...tell me, what do you think might be hard about that for you?

**CLIENT:** Well, I wouldn't want to hurt your feelings or anything, especially because you're such a nice person. It's hard for me to tell people things that are not nice—I try to be nice to everybody, I guess. It's important for me to be sure that I'm being respectful and nice. Maybe I worry about that too much.

**THERAPIST:** Yeah, maybe being nice all the time has become too much of a worry for you. I can honestly say that I would welcome hearing any concerns you might have about us or about counseling. I'd think you were working with me to help you. I certainly wouldn't think that you weren't being nice to me. I would welcome that kind of openness between us. But maybe you're telling me that's been different with other people that have been important in your life.

In this first example, the therapist offered a permission-giving and educative response in the hope that the client would then feel free to talk about any ambivalent feelings about treatment or conflicts with the therapist that may emerge. For many, this is all that is needed. When this invitation is insufficient, the therapist can take a second step and encourage the client to explore the potential threat. That is, if the client continues to show signs of resistance without being able to talk about it (e.g., missing, canceling, rescheduling, coming late, discussing only superficial issues, etc.), the therapist moves to the second step by exploring the interpersonal threat.

### *Step Two: Explore the Danger/Identify the Threat*

**THERAPIST:** It's important in our work together that you and I are able to talk about any difficulties that might come up between us—they're bound to occur in any relationship. I'm wondering if you would be able to tell me if something about our relationship or work together was troubling you?

**CLIENT:** I suppose.

**THERAPIST:** Well, OK, but based on what you've told me about some other parts of your life, I'm wondering if that might be kind of hard for you to do. You know, thinking that it might be kind of new or difficult for you to tell me if I did something that you didn't like, or if there was something about being in treatment that wasn't feeling very good for you. Sometimes people believe they shouldn't have frustrated or disappointed feelings, or at least they don't think they should express them.

**CLIENT:** Well, yes, that's probably true.

**THERAPIST:** Let's try to figure this out together. It seems that you are having some trouble being in treatment but don't feel comfortable talking with me about that yet. I'm wondering what the threat or danger might be for you if you were forthright and took the risk of telling me about something here that didn't feel right. What's gone wrong with others in the past, or what might go wrong between us?

**CLIENT:** Well, you probably wouldn't want to see me anymore—I'd be too much trouble, too critical, too demanding, or too something. And whether you said it or not, you'd rather I just went away and didn't bother you anymore.

**THERAPIST:** You're saying so many important things right there, but maybe the biggest thing you're saying is that I'm going to leave you or walk away from our relationship if you speak up and have your own voice—and don't just go along with what I say or comply with what I want?

**CLIENT:** Well, yeah, I guess that's kinda how it's been for me. Isn't that how people really are?

In this example, the therapist is trying to identify the threat that keeps the client from being able to talk about feelings or concerns that contribute to resistance and are central to the problems she is having with others. *Emphasizing process rather than content, the therapist is not trying to find out why the client is not showing up for therapy, but why the client is having trouble talking about it.* Here, the client reveals her pathogenic belief that if she disagrees with the therapist, or speaks up and expresses what she wants or doesn't like, the therapist (and others) will leave her. This may not seem like a welcome opportunity to the new therapist, but the potential for significant change is present at this moment. The therapist is beginning to provide a corrective emotional experience by clarifying that he does not wish to end their relationship as soon as they have a disagreement. As this pathogenic belief of “comply or be left” is highlighted and then disconfirmed in their relationship, it will also reveal how this belief had in fact been a reality-based expectation in other important relationships (Tsai et al., 2014). As we will see, this client goes on to recall how the abandonment or rejection she expects from the therapist is what she behaviorally experienced while growing up in her family and in her first marriage (Client: “In my family it was always, go along or be alone”). By first highlighting this outdated coping strategy of compliance in the therapeutic relationship and changing it in their relationship, the therapist can then take the next step in the change process and help the client generalize this to her everyday life. This occurs as the therapist helps this client begin exploring current relationships with others where she is also anxiously pleasing, unable to speak up and say what she thinks, and “goes along” in order to keep others connected to her, and begin to question and challenge this problematic way of relating with them as well. For example:

**THERAPIST:** Tell me how this poignant problem of either going along or being alone might be a problem with others in your life, too, like it's been with me?

**CLIENT:** I think it makes it hard for me to say “no” to anybody. Right now, as we're talking, I'm thinking about what a bad mom I'm being because I just can't discipline my kids. I guess I don't want them to be unhappy with me either—so, yeah, I guess I'm going along with what they want and just trying to please them, too.



**THERAPIST:** You're doing great. Now that we're starting to understand this, let's see if we can help you change it. You know, take a stronger stand with them, tolerate their disapproval rather than cave in to that fear of being left, and follow through and enforce the rules you set. How does it sound to start speaking up more assertively in here with me, and to start working together to help you discipline your kids more effectively?

**CLIENT:** It's like my life would change—I'd be a different person if I could do that.

This client continued to work productively in treatment and did not miss or cancel another appointment after this session. Similarly, with most clients, *exploring the threat or danger in this way will both resolve their resistance to treatment and uncover key concerns that are central to their presenting symptoms and problems.*

For example:

**THERAPIST:** Nadine, again today, it seemed to me that you kept jumping around from topic to topic. It was especially evident after you said you had dinner with your sister. I'm wondering what the threat or danger might be for you if you didn't change the topic so frequently but stayed with one issue longer and allowed me to join you there?

**CLIENT:** I don't know—I hear that a lot. I guess I just don't want to think about anything for too long...

**THERAPIST:** You don't want to think about anything for too long—I'm wondering what feels unsafe about that – what would go wrong?

**CLIENT:** Well...(pauses) I think maybe I'd start to cry or something.

**THERAPIST:** Uh huh, something about crying isn't safe—it doesn't feel OK. Help me understand that better. If you were sad and started crying, what might I do that would be unwanted or make things worse for you?

**CLIENT:** Well, I'm not sure why I'm thinking this, but I'm afraid you might put me down or something.

**THERAPIST:** Put you down?

**CLIENT:** Yeah, think I'm feeling sorry for myself, acting like a baby...but you wouldn't say that out loud cause therapists aren't supposed to say that.

When repeated attempts to focus on the threat do not work—the therapist and client cannot successfully collaborate and identify the threat or danger—therapists can move to the third step. As the next alternative, the therapist can try to interpret the content of the client's resistance. For example, if the therapist thinks the client's resistance is about to pull the client out of treatment, and the client cannot join with the therapist and talk about the difficulty, the therapist can draw on working hypotheses and tentatively suggest possible reasons or interpretations for the resistance.

### *Step Three: Tentatively Interpret Potential Reasons for the Resistance*

**THERAPIST:** It's been hard for you to get here; you've missed the last two sessions. Maybe you just "forgot," but I'm wondering if there is something about coming here to talk with me that doesn't quite feel safe—or something that might have gone on between us or that I said that didn't feel quite right to you. I know I've asked you this before...but do any possibilities come to mind?

**CLIENT:** No, not really.

**THERAPIST:** Well, OK. Let me suggest some possibilities I've wondered about, and you tell me what you think of them. As I think about some of the things you've told me about yourself, I'm wondering if you're concerned that I too am going to "judge" you, like too many others in your life have done?



**CLIENT:** Yeah, I guess that might be true. But it's not so much that you would judge me, it's more like you wouldn't respect me if you knew more about me.

**THERAPIST:** Thank you for clarifying that. It's not about judging, it's that I wouldn't "respect" you. I really appreciate that you are willing to risk sharing this with me. Has that ever happened between us—has there ever been a time when you felt that maybe I wasn't treating you respectfully?

**CLIENT:** Well, no, it hasn't happened yet, but I haven't told you very much either.

**THERAPIST:** Good, I'm glad that hasn't happened yet. I really don't ever want you to feel disrespected by me and, if that ever comes up here with me, I want you to stop us right then and tell me so we can sort it out. But I hear you—you're afraid I'm going to lose respect for you if you tell me more about yourself. I see how that would make it difficult for you to come here each week and talk with me—and maybe make it hard to get close to others, too.

**CLIENT:** Yeah, I think that respect thing has gotten in the way a lot in my life. But I'm just not going to put up with anybody putting me down...

**THERAPIST:** What would make it safe for you to talk about all you need to say in here?

Why does the therapist take this more interpretive stance (tentative as it is) only as a last resort? The interpretation, accurate or not, is the therapist's issue and puts the ball in the therapist's court. Whenever possible, it is better to try to follow the client's lead, or focus on the therapeutic process, rather than to pull the client along in the therapist's direction. In other words, it is usually more effective to explore why it might make sense that the client is resisting, or make a process comment and address the current interaction, than to make interpretations or tell the client what to do. Why? The goal is to maintain a working partnership and keep the relationship as collaborative as possible. Whereas Steps One and Two will be highly effective with most clients, Step Three—tentatively suggesting possible reasons or interpretations for the resistance, and asking the client to modify or improve the interpretations to make them fit better, can be productive at times as well.

## SHAME FUELS RESISTANCE

Misunderstandings, resistance, defense, ambivalence, and ruptures will occur throughout every phase of treatment—it's just part of the work. The most common source of resistance, however, is *shame*. Only in recent years have therapists begun to recognize the pervasive, trans-diagnostic role of shame in many clients' symptoms and problems (Dearing & Tagney, 2011). Shame is more apt than any other issue to generate resistance, lead to premature termination, hold clients back from initiating change, and lead to relapses. In a highly unfortunate interaction, it is also the feeling that therapists are most likely to avoid with their clients. Simply put, it can be terribly difficult for therapists to bear sitting with their clients as they suffer such an excruciating feeling. Routinely, new and experienced therapists alike feel inadequate to help and painfully uncertain of what to do. Predictably, it evokes their own shame as well—which they wish also to escape as much as their clients do. As therapists read and learn more about this cardinal issue, they will develop the eyes and ears to recognize the *shame motifs* that pervade so many of their clients' narratives. For example, therapists will become better able to recognize the many different faces of shame that clients present: perfectionism; frequently blaming or criticizing others; having temper outbursts; being edgy and explosive; being judgmental or contemptuous of others; being self-critical and having low self-esteem or chronic depression; struggling with eating disorders and other addictions; living anxiously preoccupied about appearances, presentation, or approval; social withdrawal and avoidance; an inability to accept compliments or tolerate

constructive criticism of any type; never feeling vulnerable or in need of anyone; being preoccupied about always being seen as strong, powerful, or independent; and many others (Ahmed & Braithwaite, 2004; Potter-Efron, 2011; Sanfer & Tantillo, 2011). With guidance from supportive supervisors, therapists-in-training can begin listening for and recognizing shame-related themes in their clients' concerns about entering therapy, as well as in the issues and concerns that block their progress later in treatment.

## SHAME VERSUS GUILT

Let's start with the basics: shame is different than guilt—and a far more significant problem to deal with. With guilt, clients feel only that they have done something wrong. **The guilty client often feels regret and may look for ways to apologize or make reparations. With shame, in sharp contrast, it's not that I have done something bad, it's that I am bad** (Lewis, 1971; Tangney, 1998). The sense of “badness” that is activated with shame generally leads clients to want to hide and avoid others' perceived scrutiny, and there is no thought or hope of resolution or repair as there is with guilt (Wells & Jones, 2000). A more primary and pervasive feeling than guilt, shame is a total or all-encompassing feeling about one's Self. With the toxic or core shame discussed here, as opposed to everyday, innocuous experiences of self-consciousness or embarrassment, clients report feeling essentially flawed in who they really are. Throughout this discussion, **keep in mind that the key emotion in all forms of shame is contempt** (Miller, 1984; Tomkins, 1967). To suffer shame is to feel that the true self with all of its defects is exposed, naked, and vulnerable to the damning judgment or criticism of others (Spiegel & Alpert, 2000; Teyber, McClure & Weathers, 2011). Clients may be without words to communicate their experience when they are suffering a profound shame reaction (“My mind went blank.”). Sometimes all they can say is something like, “I'm just hideous,” “I don't matter,” “There's something wrong with me,” or simply, “I hate myself.” A full-blown shame reaction is agonizing to suffer, and very difficult for compassionate therapists to remain co-present with and share. During these shame-filled moments, when the Self is being annihilated, some clients may express a wish to die.

Often, shame can be expressed by clients along two different themes or within two distinct domains. **First, there is a dimension of self as bad that predominately reflects feeling worthless, flawed, unlovable or unwanted, or defective in the core of their being.** This client, sometimes referred to as a “hated child,” often has been overtly rejected or angrily denigrated by caregivers and struggles with a sense of disgust for, and anger toward, the self. Perhaps the most heartbreaking consequence of child sexual abuse is this **internalized contempt** and resulting sense of self as stigmatized (“ruined,” “dirty”) (Finkelhor, 1990). **The second dimension is one of self as inadequate and reflects enduring feelings of being small or weak, helpless or disempowered, incompetent or lacking, or a failure.** In particular, this client regularly feels defeated, inadequate, and inferior (e.g., “stupid”).

**Researchers also differentiate internal and external shame.** Clients who struggle more with external shame are concerned about how one exists in the minds of others—how others are seeing or evaluating them. Those with internal shame hold a devaluing and self-critical attentional focus on themselves, and some clients cope with both internal and external shame (Lewis, 1995). Another related distinction in the rich literature on shame is that some clients are quite unaware of their own propensity to feel shame, whereas others are highly sensitive to it. Some clients, who may be variously described as narcissistic, aggrandizing, or controlling, are unaware of their own shame but often are critical or judgmental and readily evoke shame in others. In contrast, other clients are more aware of their own low self-esteem (Joireman, 2004). They are intra-punitive, harshly self-critical, and too readily please, take care of, or comply with others (Wells & Jones, 2000). Sadly, these clients also *withdraw* from many promising social relations and good opportunities in life, thinking, “She won't want me,” “They won't hire me,” and so forth.

### SHAME-PRONE SENSE OF SELF

Let's use the term **shame-prone** to clarify the difference between **the toxic or core shame that is central to so many clients' symptoms and problems**, and the everyday embarrassments that are just an awkward part of life for everyone but are not important treatment issues. It is universal to feel embarrassed in awkward situations and common to feel ashamed in certain moments. This is unpleasant, of course, but not important to our work as therapists or relevant to this discussion. In contrast to these situation-specific feelings, clients who have been frequently shamed in their important relationships—that is, **repeatedly held in contempt by their attachment caregivers, develop a shame-based sense of self and are shame-prone** (Mills, Imm, Walling & Weiler, 2008).

This shame-proneness pervades most types of psychopathology and is central to our work with many clients. Shame-prone individuals, for example, will not be able to accept constructive criticism because it threatens to expose their deeper, pervasive, and long-standing feelings of shame. Even good-natured teasing from family or friends can evoke these feelings. Humiliating feelings of being profoundly diminished, rejected or abandoned, or exposed are readily triggered for shame-prone clients. As noted, these intense and sometimes volatile reactions are triggered in many different situations in their daily lives that would not evoke such strong reactions in others. In response to the intense feelings of shame that are continuously evoked, shame-prone clients employ a wide variety of coping strategies to keep their shame concealed from others and, perhaps more importantly, from themselves as well. Routinely, such clients protect themselves from having their underlying sense of *shame-worthiness* revealed by:

- *acting arrogantly and self-righteously toward others;*
- *intimidating, controlling, or inducing shame in others;*
- *adopting perfectionistic standards around their own work, cleanliness, or religious practices;*
- *developing eating disorders or other addictions;*
- *becoming preoccupied with appearances and social approval;*
- *becoming derisively self-critical and withdrawing socially;*
- *becoming judgmental and critical of others;*
- *withdrawing from others and avoiding many interactions.*

### SHAME-RAGE CYCLE

Therapists will observe other faulty coping strategies that clients have developed to cope with these dreaded feelings. Perhaps most disturbing to witness is the *shame-rage cycle* that characterizes “road rage” and most domestic violence episodes of battering and physical abuse (Gilligan, 2009; Simon, 2002). The shame-prone individual explodes in rage in the seconds after feeling diminished or demeaned by someone, often in response to a completely innocuous, benign comment, such as his wife asking him to take out the trash (or a child asking “why?”). The temper outburst or rage-filled response is a desperate but futile attempt to disprove shame-worthiness (“I’m not weak”; “I’m not a piece of garbage”; “You can’t boss me around like that!”). **This is an artificial or defensive attempt to restore a sense of personal power and Self—often through aggression or contemptuously diminishing others** (Potter-Efron, 2007).

To emphasize, clients who have grown up suffering *contempt* from caregivers (e.g., a parent who often says, in a disgusted tone of voice, “What’s wrong with you? Can’t you do anything right?” or “You’re ruining my life...I wish you were never born”), develop a shame-based sense of self. These clients usually remember the painful developmental experiences that were so crushing of their sense of self but have split-off or defended against the accompanying feelings. For example, the client whose drunken father made

him lick the kitchen floor, while frightened siblings looked on helplessly, can usually remember what occurred behaviorally—what was said and done, and can recount this humiliating experience to the therapist. However, *he does so without being able to feel the shameful affect or experience the unwanted emotions that accompany this abuse*. That is, the behavioral events are often accessible and remembered whereas the accompanying feelings are too painful to acknowledge or experience and may be split off or sequestered away. Similarly, this client often cannot recognize or identify the current experience (feeling bossed around) that has just triggered a temper outburst. In the following dialogue, the therapist is helping a shame-prone client make the connection between the triggering event (feeling put down or “dissed” by someone) and the presenting problem (losing close relationships because he keeps losing his temper).

**THERAPIST:** As I listen to the sequence here, it sounds like you lost your temper just after your wife asked you to take out the trash. Last week you told me you lost your temper when your coworker decided which part of the job you should do. You said afterwards that in reality it wasn't a big deal but your first response was to “go off” at him. I wonder if that's telling us that you lose your temper when it seems as if others have put you down by telling you what to do or dominated you in some way?

**CLIENT:** I hadn't thought of that, maybe... You know, that reminds me, I almost got in another fight this week, when this worm flipped me off on the freeway. I wanted to kill him—my buddy, the one who calls me “pit-bull,” said my face was so red he could see the veins bursting out of my neck while I was screaming at him.

**THERAPIST:** That fits with my suggestion that when someone flips you off, or even just tells you what to do in a reasonable way, you feel shamed—and then the rocket goes off. It's like it almost becomes a matter of life and death for you—as if you are saying: I'm being put down, I can't stand it—no matter what, nobody is ever going to treat me like this again—and you explode.

**CLIENT:** Yeah, that's right. I hadn't thought of the word “shame,” but I do go kind of crazy when someone puts me down.

**THERAPIST:** Yes, I think you do, too, and I'm concerned that you are embarrassing your wife and scaring your son. Maybe you feel shamed sometimes even when others aren't putting you down. It sounds to me like your wife really was just asking you to take out the trash. So, as you think about it now—here with me—do you think she was trying to boss you around or put you down? Let's think about what she might have been thinking—what could have been going on in her mind, as all of this began.

**CLIENT:** She's a good person...she wasn't trying to put me down. (long pause) I've got to change this—it's ruining my life.

**THERAPIST:** Yes, it is ruining your life, and I'd like for us to keep working together to help you change this. So, let's keep talking about “shame”—that's the feeling that seems to trigger or set off your temper. When I say “shame,” what comes to mind?

**CLIENT:** ...Well, my stepfather used to beat us...he's dead, but I still hate him...

## SHAME-ANXIETY

Instead of temper outbursts and rage, other clients who are shame-prone have intense anxiety evoked in many different situations that hold the ever-present threat of revealing their basic unworthiness, inadequacy, or inherently flawed and defective self. As the dreaded feeling of shame is evoked, yet again in this new situation, therapists will commonly witness *shame-anxiety*. For example, something as innocuous as asking someone for a date, applying for a job, needing to ask for help with a flat tire, getting lost and needing to ask for directions, or making a simple mistake at work evokes the anxiety that

the clients' shameful inadequacy, neediness, demandingness, and so forth will be exposed. Repeatedly, throughout these clients' lives, strong anxiety is evoked by the threat of having their inadequate, flawed or bad self revealed. **In many shame-prone clients, this occurs so frequently that they live with a pervasive, anxious vigilance that may be expressed in the presenting symptom of anxiety attacks, social phobia, avoidant personality disorder, a generalized anxiety disorder, and others.** In other words, the anxiety signals that clients' defenses against their shame are being threatened in this situation, and raising the specter that their shame-worthiness is going to be revealed to others right now. To manage this anxiety and protect against the potential exposure of their shame, clients may cope by withdrawing or avoiding, being pleasing or complying, trying harder to be perfect, never needing anybody or asking for anything, and so forth, *which becomes a characteristic or habitual coping response that characterizes their lives* (Greenberg & Iwakabe, 2011).

As we will continue to explore, therapists can help clients recognize and change these problematic coping strategies for defending against their shame (e.g., yelling and hitting, avoiding and withdrawing, controlling or dominating others, inhibiting or constricting oneself, complying with others and going along when they want to say no, becoming intoxicated or getting high every day, bingeing, etc.). Therapists help clients resolve their shame-proneness, and the many varying symptoms that accompany it, by allowing them to experience and share the self-hatred or contempt they feel toward themselves. Change occurs as clients have a corrective emotional experience and find that the therapist is not critical/judgmental but compassionate and the client, in turn, *begins to have some empathy or compassion for themselves and the plight they once were in* (Gilbert, 2009; Gilbert, McEwan, Matos & Rives, 2011; Gilbert & Procter, 2006).

The biggest problem with these interventions, however, is that therapists often have as much difficulty witnessing and responding to the raw experience of shame as clients have in bearing it. Of course, many therapists are shame-prone themselves and have their own unresolved shame dynamics activated by the client's shame. Too often, these therapists try to cope with their own activated shame by reassuring the client, minimizing the problem, offering intellectual explanations or interpretations, changing the topic or avoiding the issue altogether, and providing other well-intended but ineffective responses. Further guidelines for responding to shame and helping clients contain these and other painful feelings will be provided in Chapter 5—we need to learn about this challenging topic in manageable doses. For the time being, therapists can begin listening for the presence of shame in their clients' narratives and begin considering the possibility that **resistance often serves to protect clients from their shame and from having seemingly unacceptable parts of themselves revealed.** This is a simple concept to say but, **because of our own shame dynamics and cultural prohibitions against approaching such taboo issues, takes most new therapists several years to integrate and apply.** Having a supportive supervisor who is attuned to shame dynamics is essential to help new therapists begin working with their clients' shame. Consider the following example:

*Susie's boyfriend did not show up for their date. Feeling desperate and empty, she found herself in the kitchen spooning peanut butter out of the jar and into her mouth until the jar was empty. The next day in therapy, telling her therapist about what had happened, she became overwhelmed with shame—feeling like a “disgusting pig.” Susie's therapist wanted to look away; her mind raced trying to think how she could make her client feel better and stop this sickening pain. By asking about the bingeing episode, Susie's therapist felt she would be causing her client incredible self-loathing that only seemed to be accelerating out of control. Almost immediately, the therapist's shame about her own weight and appearance began to flood her as well. The therapist wanted to do anything to change the conversation and end this excruciating moment. It was too painful and too close. Fortunately, she was able to recall her supervisor's words of support and take a slow, deep breath instead. Rather than flee from*



*her own shame and try to stop the client from feeling hers, the therapist was able to provide a corrective emotional experience by somehow getting out the words, "I'm honored that you are able to share such a vulnerable part of yourself with me. It is a privilege for me to be with you right now." Susie cried hard and spent the next three sessions sharing her history of experiencing rejection, feeling alone, and her desperate need for some support in her life. Susie and her therapist were able to begin addressing her profound shame related to issues of worthiness as well as how to cope without bingeing when these feelings were activated.*

Learning to work with clients' shame is not easy, certainly, but therapists will find their clients remain engaged in treatment, and make real progress with their problems, as they become more effective in working with shame. More specifically, clients resolve their shame as therapists acknowledge or "name" it and respond empathically, as Susie's therapist demonstrates so effectively. It's exciting to see the important changes that commonly follow as the client's shame-based sense of self improves ("low self-esteem" is often a euphemism for shame). Shame-prone clients, for example, who were too preoccupied with their appearance or weight become more self-accepting; others who didn't care enough about how they looked take pride as they lose weight, begin fixing their hair, or dress better. Anxiety over having their shame-based sense of self revealed, and depression over feeling so bad about one's self, diminish markedly as clients find that their therapist can see this unacceptable part of them and still respond with compassion rather than the criticism and judgment they expect.

## SUCCESS IN TREATMENT CAN BE THREATENING

Many clients will experience anxiety and conflict over succeeding in treatment and achieving these new behavioral changes. This paradox or self-defeating behavior is perplexing to most new therapists. Resistance predictably returns, even as clients make healthy new changes. For example, some clients will experience feelings of guilt or disloyalty over making progress and having success in treatment. Therapists are going to observe that some clients retreat from progress in treatment or undo successful changes they have just made. **Often, clients cannot sustain positive changes they have just achieved because the healthy new behavior is inconsistent with their cognitive schemas.** Following attachment theory and internal working models, acting in this successful new manner (e.g., stopping binge-eating episodes, leaving a nonsupportive relationship, graduating from college, etc.) **threatens their attachment ties to caregivers who did not support their independence or success**—and for a few other clients, actively sabotaged or punished it. Becoming stronger or improving in therapy makes some clients feel cut off from parental approval and affection, disloyal to caregivers, or guilty about hurting, leaving, or surpassing the parent. As clients make significant behavioral changes, or report feeling better about themselves, therapists can expect some clients to report feeling either:

- (1) alone, empty, or disconnected following meaningful change or
- (2) guilty, selfish, or bad.

When these reactions occur, **clients need to be reassured of the therapist's continued presence and support for this stronger Self** (Gilbert, 2010). If not, they will have to manage their guilt, or restore their insecure and threatened attachment ties, by sabotaging their own success. For example, the therapist challenges the client's maladaptive schema, disconfirms the problematic expectation, and provides a corrective emotional experience by saying:

**THERAPIST:** You've just had a big success at work—and I'm wondering if that has something to do with why you feel so anxious and alone right now. How does that sound to you?

**CLIENT:** As you say that, it makes so much sense...You know, I called my brother last night, really excited, thinking he'd be happy. You know, when he was promoted last month I took him out to a fancy dinner. All he said was, "oh...nice" and quickly he had to go. Usually he talks more. It made me sad. I do feel that somehow I should not be succeeding...or succeeding so fast...

**THERAPIST:** I am glad we are talking about this. I hope your experience with me will be different than with your brother—and maybe from others you have been close to in the past as well, because I am very happy for your new promotion. I'd like to hear all about it and spend as much of the session as you like talking about this success and what it is like for you to be doing so well.

**CLIENT:** I'd like that; it would be good to have someone who can help me learn to celebrate the good things in my life too.

These commonly occurring sources of resistance and impediments to change will be explored further in the chapter ahead. In particular, we will return to the cardinal affect of shame in Chapter 5.

## CLOSING

Listening to the client with presence and respect is the most important intervention the therapist can make in the beginning of treatment. Sustaining this empathic stance toward the client and consistently grasping the most important feeling or key concern in what the client just said is the basic tool the therapist uses to establish a working alliance. The next step in the treatment process is to be attentive for ambivalence and find non-blaming, exploratory ways to inquire about potential signs of resistance in order to maintain the relationship that has just begun. Resistance will be more of an issue with some clients than with others, but it will occur to some extent for every client. As we have seen, if the client can talk about these conflicted feelings, and the therapist responds affirmingly, these feelings will be far less likely to pull the client out of treatment prematurely. Resistance will wax and wane throughout therapy, often reemerging as clients enter more deeply into difficult issues and as they try out new behavior and make successful changes. To sustain progress in treatment, therapists need to continue responding effectively to the many types of resistance that occur. If therapists formulate working hypotheses about potential client resistance, it will help them to anticipate when it is likely to occur and to respond more effectively when it does. In sum, the therapist's goals are:

- *to encourage the client's lead and ownership of the treatment process;*
- *to provide accurate empathy to foster a working alliance;*
- *to remain alert for potential signs of resistance and address it collaboratively through process comments, and;*
- *to begin formulating treatment plans by generating working hypotheses.*

Resistance, ambivalence, and defense provide a window to observe the fascinating workings of internal conflict. People do not possess a unified self; we are so complex and multifaceted that it often feels as if one part of ourselves is working against another part. By resolving an internal conflict like the ones discussed here, a person becomes a little more integrated or whole. Integrating disparate parts of the Self is usually an important part of behavior change and the avenue to greater self-efficacy. Working successfully with resistance shows clients that they possess the internal resources, and have the help they need, to make chosen changes in their lives.

## CHAPTER EXERCISE

If I start to feel bad, worthless, inadequate, or one of these shame-based feelings, what is most likely to have triggered this reaction...(e.g., when my partner is not meeting my needs, when I feel vulnerable or criticized, etc.)?

When this shame-based feeling is evoked in me, my initial response is to (e.g., withdraw, eat, drink, become pleasing, lose my temper, etc.)?

## SUGGESTED READINGS

R. Dearing & J. Tagney (Eds.) 2011. *Shame in the therapy hour*. Washington, D.C.: American Psychological Association.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes. Also included with this chapter is a video demonstration by the authors illustrating working with potential resistance at the end of the first session and working with the client's reluctance to address an issue.



## CHAPTER 4

# An Internal Focus for Change

Year after year as we supervise bright, promising student therapists, we are continually pointing out how they are keeping the conversation on the surface. Almost more than any other issue, we are giving some therapists permission, and challenging others, to go beyond surface, everyday conversations and engage clients with more depth—to repeatedly strive to join the client at the most meaningful point possible. This chapter gives therapists tools to achieve this goal. Consider the following situation:

*Dana was presenting a new client she was seeing to her first-year practicum class. She showed a videotaped segment from her previous session in which her client was complaining about her past two husbands, her mother, her previous therapist, and others. Lost in trying to grasp what all of this meant, Dana half-pleaded with her practicum instructor, “What should I do?” With her usual good sense of humor, her instructor jokingly replied, “OK, you guys are always asking me what to do, and this time I’m actually going to tell you. Next week, Dana, tell your client that she has two choices. Either everyone she has ever known needs to be in therapy—so she should refer every person in her life here to the clinic for treatment. Or, she can just meet with you herself and begin to explore the decisions and choices she makes and her contributions to these problems.” The class laughed and got the point.*

### CONCEPTUAL OVERVIEW

The first stage of therapy is complete when therapist and client have established a working alliance and begun to work together on the client’s problems. This collaborative partnership is a necessary prerequisite to the second stage of therapy: the client’s journey inward. **In order to change, clients need to become less keyed on the problematic behavior of others and begin to explore their own role in problems.** As real and compelling as these problems with others usually are, many clients need to stop focusing exclusively on historical events, past relationships, and the problematic behavior of others in their lives. Instead, these clients change when they begin to clarify their own thoughts, feelings, and reactions to the troubles they are having. Why? Clients will usually fail in their attempts to change others in their lives, whereas they can often resolve problems by changing themselves—the way they respond and their own participation in problems. In this process, clients:

- (1) Examine their habitual response patterns in problematic situations, (e.g., am I withdrawing and avoiding this situation),
- (2) Evaluate the patterns' usefulness in their current lives (e.g., am I becoming quiet and walking out of the room without addressing the problem) when there is conflict with someone, and
- (3) Begin trying out new and more adaptive ways of responding (e.g., trying to dialogue with my mother about the problem we are having).

This internal focus on changing their own responses, rather than focusing on or trying to change the other person, leads clients toward greater self-efficacy, which diminishes symptoms of anxiety and depression. The therapist's task is to help clients make the transition from seeing the source and resolution of problems in others to adopting an **internal focus** for change. This is a two-fold process. First, the therapist helps clients begin to look within. Without invalidating clients or using language that makes them feel "blamed" in any way, the therapist looks for opportunities to focus clients away from complaining about or trying to change others, and toward understanding and changing their own problematic reactions. Psychotherapy process studies demonstrate that across differing types of treatment, positive outcomes are facilitated by interactions with the therapist that encourage clients to engage in self-observation and introspection (Beitman & Soth, 2006; Watson, Goldman & Greenberg, 2011). The clinical approach presented in this chapter is grounded in Carl Roger's client-centered theory (1967), and informed by talented colleagues who delved deeply into helping clients focus inward and explore their own subjective experiencing (Gendlin, 1996) and feelings (Greenberg, 2002).

Second, the therapist's intention is to help clients assume more responsibility for change. This occurs when clients move the dialogue from everyday surface issues towards their core struggle in the treatment process—by becoming active agents in their own therapeutic work. As clients become more aware of how their cognitive schemas are not always accurate and their usual coping styles and behavioral patterns are not effective in many current situations, they begin the change process. With greater awareness of their own responses, and how they may be participating in or contributing to some of their problems, clients increasingly recognize that they do not have to keep responding in old and familiar ways that are no longer effective (Goldman, Watson & Greenberg, 2011). With this new perspective, clients often join readily with the therapist in exploring new behavioral options and begin working actively to change the problematic ways they are responding both to themselves internally and to others. As clients become more aware of their contribution to problems and how changing their responses may change the reciprocal patterns that have not been working in some relationships, they are more able to commit to a change process that is empowering, and this will, in turn, contribute to behavioral change (see Miller & Rose, 2009 for research on empowering clients to commit to change). Throughout this chapter, we will see that when clients change their part in problematic scenarios, the interaction sequence changes—even if the other person doesn't change and keeps giving the same hurtful or disappointing response (Wachtel et al., 2005). By modifying their role in internal and interpersonal problems, and using the therapist's support for clients' own self-direction and initiative within the therapeutic relationship, clients are empowered to change.

Self-efficacy develops out of this collaborative process between therapist and client (Bandura, 2006). The therapist is not fixing or curing the client—the client is sharing ownership of the treatment process. Why is this interpersonal process so important? The therapist's goal is to empower clients rather than just give them answers or tell them how to live their lives. Our goal is to help clients know their own mind—to give them permission to have their own voice and trust their own intuition or gut—and thereby support them to become better able to make their own decisions. When given answers or



prescribed solutions, most clients do not make them their own—they cannot apply the advice in the next situation or learn how to solve future problems or self-soothe, without having someone else tell them what to do or how to do it. By helping clients look inside at what they are feeling, expecting, and doing, we are teaching them skills that they can carry over to their everyday lives and use on their own when treatment is over. In this way, enduring change results when clients participate actively in treatment and feel ownership of the change process (Tufekcioglu & Muran, 2015).

New behavior and thinking come about when the client has been an active participant rather than when the therapist has just explained a connection or interpreted what something means. **When this process occurs, treatment gains can be better maintained after therapy has stopped.** Per client response specificity, the externalizing stance we are describing does not fit all clients. For example, some clients feel solely responsible for what is wrong in their lives. These clients are so preoccupied with their own self-criticism and self-blame, or a pathogenic belief that they are “bad,” that they cannot even consider the realistic ways that others may be contributing to their distress. For many other clients, however, it will be difficult to shift their focus away from others and begin looking at their own contribution or role in problems. For some clients, looking inward without the partnership that the therapist provides can be challenging. **In order to take this rewarding journey inward, clients need a relationship with a therapist who is affirming yet willing to repeatedly invite them to look within and collaboratively explore.** In this chapter, we will see how therapists can do both.

## HELPING CLIENTS FOCUS INWARD ON THEIR OWN EXPERIENCE

**Early in treatment, many clients see the source of their problems in others.** Clients often want to spend more time describing others’ problematic behavior than discussing their own experience of and response to the problems. For example, many clients begin the first few therapy sessions by announcing that the problem is really with another person:

- *My husband won’t pay any attention to me.*
- *My wife is always on my back.*
- *My children are impossible; they won’t do anything I say.*
- *My boss is a demanding tyrant. I never seem to do enough.*
- *My mother won’t stop criticizing me; I can’t do anything right in her eyes.*
- *I’m 27 years old, and my father treats me like a child.*
- *My boyfriend keeps cheating on me.*

It is essential for the therapist to affirm these complaints as genuinely troubling concerns. The therapist’s sustained intention is to address the problem that is most pressing, “alive,” or salient for the client right now—to find and begin with the client’s “point of urgency.” Even though these concerns and complaints may include over-reactions or schema distortions, from the client’s point of view, they are the truth. As emphasized in Chapter 2, the therapist joins the client there, validating and even helping articulate more clearly how the problematic behavior of others is indeed troublesome.

In some cases, however, the therapist’s assessment of the situation is so different that such affirmation would not be genuine for the therapist. **In these situations, therapists can at least affirm the subjective reality of the client’s perceptions.** For example:

**THERAPIST:** It sounds as though you feel your boss, like your girlfriend, is overly demanding. So, from your point of view, it seems that they are being unfair again.

If the therapist does not provide this affirmation first, many clients will feel that the therapist doesn't understand the reality of the other person's problematic behavior, doesn't believe them, or doesn't care about what's really troubling them. This unwanted outcome is counterproductive and will often reenact clients' developmental histories—they are not listened to, taken seriously, or believed again. Thus, to keep this potential invalidation from occurring, the therapist's first aim is to *hear and validate clients' concerns*. Throughout treatment, there is a consistent intention here: We are repeatedly trying “to meet clients where they are at.” Only after making this genuine empathic connection, which often requires the therapist to de-center and see issues from the clients' subjective viewpoints, can the therapist take the next step.

The second step is to begin focusing clients back on their own thoughts, feelings, and reactions to the problematic behavior of others, rather than joining clients in focusing exclusively on the external problem. Often, it will be easy for therapists to pair these first two steps.

**THERAPIST:** I'm sorry your father keeps doing that to you. I can see, as you talk about it, how affected you are by it. *How do you respond—what do you say and do, when he...?*

OR

**THERAPIST:** I can see how frustrating it can be to have your boss respond that way over and over again, no matter what you do. *How do you feel, what's going on inside, when he...?*

This principle is especially important when working with individuals who have been disempowered or disenfranchised, including racial minorities, women, gay men, and lesbian women. Often, they are accurately describing the social inequities that realistically contribute to their problems. Here again, the therapist wants to join the clients in the reality of their social context, as they experience it, before focusing inward on their reactions to, and ways of coping with, these inequities. Validation and empathy need to precede exploration.

Why is therapy more productive when therapists focus clients inward? In many cases, clients who feel anxious, distrustful, angry, or helpless because of the behavior of another person will try to enlist the therapist in criticizing, blaming, or trying to change the other person. However, therapy will not progress very far if the therapist merely joins clients in focusing on the other person's behavior, no matter how problematic this behavior may be. Why? As the existential therapists inform us, *clients' attempts to change the other person will usually fail; clients are much more likely to resolve the problem by changing their own way of responding and finding their own authentic voice* (Jacobsen, 2007). Yalom (2003) emphasizes that once clients recognize the role they may play in creating their life predicament, they realize their power to change it. Working in a supportive and non-blaming way, therapists are trying to help clients consider the question: *How might I be contributing to my own distress; or How can I change my response when she does that?* Once they grasp this, they are able to chart a new course with a different internal dialogue and different behavioral responses. For clients whose problems stem from trauma or abuse, looking inward with the partnership of a therapist to begin questioning their schemas of shame or self-hatred and move toward greater self-compassion will be a significant part of the treatment process.

Thus, the therapist's task often is to *(1) expand clients' focus beyond the other person and include themselves as well and (2) help clients be compassionate as they explore their internal working models* (i.e., internalized views of themselves and others). The therapist should be *flexible* in adopting an internal focus. The goal is to clarify *both* the reality of what the other is doing, as well as the client's ineffective internal and interpersonal

responses to it. Most clients will benefit greatly from the therapist's help in understanding the behavior or more flexibly considering the potential intentions of problematic others (Fonagy & Allison, 2014). For example, if a client has grown up being befuddled by a parent with a Borderline or Narcissistic Personality Disorder, it may be deeply validating for the client to read the descriptions of these disorders in the *Diagnostic and Statistical Manual* (DSM-V, 2013). This type of external *validation* may help these clients realize that all of this was not their fault and did not happen solely to them:

**CLIENT:** That list of behaviors was so familiar and accurate. My father really was unpredictable and self-centered—it wasn't just me! I wasn't making it up or imagining it.

In tandem, however, therapists are working to increase clients' awareness of their own thoughts, feelings, and reactions in challenging situations and make sense of them. Recall the externalizing comments cited at the beginning of this section. Each of the following questions can be used to focus those clients back on themselves, help them begin to consider their role in the problem as well, and find better ways to manage their responses:

- *What do you find yourself thinking when your husband is ignoring you?*
- *How do you feel when your wife is nagging you?*
- *What do you do when your children won't listen to what you say or do what you ask?*
- *When your boss is being demanding and critical, how do you typically respond?*
- *What thoughts were you having as your mother was criticizing you?*
- *How would you like to be able to respond when your father diminishes you like that?*
- *Tell me about the thoughts and feelings that are evoked in you and then what you would like to be able to say or do when you find that your boyfriend is cheating again?*

Simple inquiries of this type serve two important functions. First, they tell clients that the therapist is listening and grasping their core concerns. The therapist has not changed the topic and is responding directly to clients' concerns as the clients see them. Second, while inviting clients to say more about their concerns, the therapist is also expanding the clients' focus away by encouraging them to look more closely at themselves as well. For clients, becoming more aware of their own cognitive, affective, and behavioral reactions is a powerful intervention that will elicit strong feelings and reveal the issues that are most important and troubling to them. This pivotal step toward clarifying what their central and enduring problems really are provides the basis for a shared treatment focus.

In many cases, clients will welcome the therapist's offer to talk more about themselves. Their active exploration of their own expectations and responses in problematic situations will lead to increased awareness of the narrow range of responses they repeatedly employ, and help them consider a wider range of options. This increased self-awareness of their old response patterns, coupled with learning new or more flexible ways of responding with the therapist, leads to more problem resolution skills and increased feelings of self-efficacy. However, not all clients will respond so positively to the therapist's initial invitation to look more closely at themselves. A few clients may avoid an internal focus and continue to talk about the problem out there in others. In that case, the therapist does not want to start an argument or rigidly demand that clients look inward, because those clients are behaviorally telling us they can't quite do this yet. Fortunately, a range of options exists. For example, the therapist might wait for the next opportunity to enter the client's experience:

**CLIENT:** My husband is impossible to live with. He complains constantly; nothing ever pleases him.

**THERAPIST:** It sounds like living with him is difficult. What's the hardest thing for you?

**CLIENT:** Do you know how hard it is to live with an angry, demanding husband who keeps trying to tell you what to do all the time?

**THERAPIST:** You sound very angry. I can see how hard it is for you to have someone after you like that.

**CLIENT:** He just won't stop—he just keeps at it.

**THERAPIST:** How do you cope when he is doing this? What do you do when he keeps at it?

**CLIENT:** I get really mad, and I suppose I start to yell back some.

**THERAPIST:** You get really mad and start to yell back at him?

**CLIENT:** He just won't stop...

**THERAPIST:** What does the yelling mean to you? How do you make sense of all his yelling?

**CLIENT:** That he is just an angry person. It's not just him, you know; his whole family is like that—always finding fault with others. It's just tiresome. I'm tired of constantly hearing what's wrong with me—that I'm doing everything wrong and failing all the time.

**THERAPIST:** “Doing everything wrong and failing”—ouch! Sounds like his family's constant criticism taps into your own painful feelings about yourself of not measuring up or not being enough. And having those shameful feelings of inadequacy aroused all the time could be infuriating...

**CLIENT:** Yeah, I hate them for making me feel this way. If I could just make them stop, I think my whole life would be better.

**THERAPIST:** This is very important for you, and we need to work together on it. **I would like to understand better what you do when they criticize you, so that I can help you learn some more assertive, limit-setting responses.** But your own feeling of not measuring up is also a part of this problem that we need to work on, so that you can stop charging at the red flag they are waving. If we can change the internal part of you that reacts so strongly—as if you believe what they are saying is true,—it would be much easier for you to handle this than it has been in the past. I can see how their criticisms can upset you, but what's more important is the way you end up feeling inside, about yourself, that becomes the bigger problem.

**CLIENT:** I'm not really sure what you mean.

**THERAPIST:** There is relatively little we can do about their expectations. But we can work on your feelings about yourself—separate from their feelings about you—and on your responses to them, which could help you.

**CLIENT:** Well, maybe, I see what you mean a little bit, but I'm not sure what to do.

**THERAPIST:** Tell me more about your feeling of not measuring up. Let's work together to modulate that feeling better, so you don't lose credibility by going off at them—and then we can role-play some more limit-setting responses to their criticisms and demands.

**CLIENT:** Well, I guess I've always sort of felt like I'm not really good enough...

In this dialogue, the therapist validated the client's experience and encouraged the client to look at her own reactions and role in the problem. Although the client kept trying to focus on her husband and his family, the therapist's repeated, patient but modest invitations to have the client look at herself as well, soon diminished her defensive, externalizing stance. **As a result, the client moved closer to her own problematic feelings and beliefs about herself that made her vulnerable and contributed her reactivity to the marital conflict.** By gaining a better understanding of her own feelings of inadequacy, which are painfully amplified by her husband and in-laws, the client can become less shame-prone and

reactive to others' criticisms. This ability, in turn, will be an important part of learning to respond more assertively and set limits more effectively, rather than alternating between merely yelling back or withdrawing—which only exacerbate the conflict.

Many clients have not been encouraged to focus on themselves before. Some clients may resist this internal focus initially because it asks them to come face-to-face with difficult feelings that they have not been able to understand or resolve alone (in this case, for example, “I feel inadequate.”). For many others, focusing on the other fends off the blame or criticism they have learned to expect from others (e.g., “Can’t you do anything right!”). However, the paradox is that as long as clients avoid the internal or personal aspects of their relational conflicts, by externalizing their problems onto others, they will feel powerless and frustrated.

In particular, many clients who enter treatment are overly invested in changing others as a means of managing their own problems or insecurities. That is, they often are seeking an external solution to a problem that may better be managed internally. The reality is that, beyond clearly expressing our preferences and personal limits (“I would like...”; “I will not...”), we cannot readily influence how others think, feel, and act. In contrast, however, we can change problematic interpersonal scenarios that keep playing out by changing our own responses to others. For example, clients routinely fail in their attempts to make their spouse stop drinking, smoking, or overeating. Similarly, some clients try for decades to win the approval or recognition from others that they never received from a parent. Others try unsuccessfully for years to have their grown offspring choose a different mate, religion, or career. As a result of these failed attempts to change others, many clients enter therapy with feelings of helplessness, hopelessness, and depression.

In sum, to gain a greater sense of self-efficacy and feel less anxious or depressed, many clients will need to focus more on understanding and changing themselves. Feelings of helplessness are inevitable when one tries to change others who do not want to change—whereas empowerment can come from recognizing and changing one’s own responses and addressing one’s own internal reactions and responses. Thus, *a consistent therapeutic intervention is to pair an empathic or affirming response with an invitation to focus clients back onto themselves* (such as, “I’m sorry that happened. How would you like to be able to respond the next time he does that?”). The following types of questions will help clients explore their own responses.

- *What is the main feeling you are left with when...?*
- *What were the thoughts you were having when...?*
- *What was the most difficult thing for you when...?*
- *How would you like to be able to respond when...?*

## THErapy Intensifies When Clients Focus Inward

Focusing clients on their own behavior often reveals how they are contributing to, or participating in, their own problems. As we will see, clients who can focus on themselves and see their own participation in a conflict are usually motivated to change their own part in it. This, in turn, often changes the usual sequence and may allow the other person to respond differently as well. To illustrate, let’s return to our example of the shame-prone wife who complained about her critical husband and in-laws.

Although the client did begin to explore her own lifelong feelings of inadequacy and the ineffective ways she responded to her husband, she kept complaining about her “obnoxious” husband and “superior” in-laws. As before, the therapist was empathic, validated her experience, and affirmed her anger. At the same time, however, the therapist did not join the client in focusing exclusively on her husband or blaming him as the sole source of her problems. Instead, the therapist looked for opportunities to focus the client



away from her singular preoccupation with her husband's behavior and toward her own reactions to him as well. For example:

**THERAPIST:** Rather than just insulting him back, which isn't making things better for you, what could you say instead the next time he criticizes you like that? What would be a realistic way that you could speak up for yourself and say what you don't like?

What would have happened if the therapist had not taken this approach? If the therapist had responded to the client's **eliciting pull** to blame her husband, the client would have remained an angry but helpless victim. In contrast, if the therapist had emphasized just the client's contribution to her marital conflict, without first validating her experience, it would have repeated the problematic relational pattern for her of always being blamed and "sided against." Exploring the client's feelings of inadequacy revealed a broader developmental context for these feelings of shame that stemmed from rarely being supported by her caregivers and consistently being blamed for whatever went wrong—in her recollections, "everything was always my fault." Thus, by validating the client's experience first and, second, focusing her inward, the therapist helped the client adopt a new and more assertive response to the old problem of excessive criticism from her husband (and eventually her in-laws and own family of origin as well). For example, the client rehearsed with the therapist and later tried saying to her husband:

**CLIENT:** You're saying, in so many words, that I did it wrong, but I disagree. I think I handled it okay. I guess we just see this differently.

AND

**CLIENT:** I feel put down when you talk like that. It's hurtful. Please stop or I'm leaving the room.

While helping the client clarify and become more aware of her own internal and interpersonal responses to her husband, the therapist was also suggesting different ways she could respond that would change her part in their marital conflict. Specifically, the wife began to express more directly to her husband when she would like to talk or interact and when she would prefer to be alone. For example, she clarified with her husband that when she came home from work she wanted about 30 minutes to herself because she feels tired and "frazzled" and wants to unwind while she is getting dinner started. But after dinner, she would like to sit down together and share their day—without the TV or the phone—to just be with each other for a while.

More important, as the wife gained a better understanding of her tendency to feel responsible for everyone's needs—which left her feeling overwhelmed by others' demands—she felt less "controlled" and reactive to her husband's requests. Significant change occurred as she increasingly realized that she felt shamefully ruled by him because she could neither say no to his requests nor express her own wishes or set boundaries. On the one hand, if her husband wanted something from her, she believed that she was failing if she didn't comply and do it his way and on his timetable. The therapist was also able to help the client progressively recognize that she expressed this resentment to her husband indirectly—in her own critical, shaming, and withholding ways. Over the next few months, this new awareness allowed her to respond more often to her husband's requests without feeling like she was complying with him or being dominated. As she increasingly realized that she too readily felt that she must comply with him and others, she began to challenge this faulty belief and started trying out more assertive responses. She found that her resentment and irritability decreased. This was not a simple "aha" experience that came about quickly, of course, but her situation gradually improved as she continued to set limits with

him and speak up for herself, which the therapist helped her to do by actively role-playing new ways of responding that she had never really been exposed to. Making this important intervention more specific for new therapists, the therapist demonstrated this by playing new assertive ways of responding while the client played the role of her husband and said the things to the therapist that he frequently said and that bothered her most. In this way, as clients begin to understand that more alternatives are available than they had realized, clients become more hopeful and see that their lives can change.

## RELUCTANCE TO ADOPT AN INTERNAL FOCUS

In the three sections that follow, we explore issues that can make it difficult for some therapists and clients to focus inward. For most student therapists, it is “different” to approach and try to enter, rather than minimize, reassure or avoid, pain, conflict, or strong personal distress. As we will highlight in the first of the following sections, trainees say, “I’m not used to this—it’s not how we talked in my family.” For other trainees, making bids to approach the fear, shame, or loss that the client just vaguely alluded to evokes feelings of anxiety that they will not know “where to go” or how to join with the client in this more significant struggle. In the second section, we will provide tools to help move therapists toward core issues by attending to “Points of Entry”. In the third section, we will consider reasons why some clients will be reluctant to stop externalizing and offer suggestions and illustrations for how best to respond.

### THERAPISTS’ RELUCTANCE

Most new therapists feel comfortable talking about things that are on the surface—akin perhaps to how they have learned to interact socially, and do not hear or respond to the significant emotional and relational messages that are compressed in what the client just said. Routinely, new therapists bring their familial and cultural socialization, and try to reassure clients about their fears or concerns, engage in problem-solving, or move away from and avoid the fear or pain, vulnerability or distress, that the client has just voiced or implied. These are well-intended, benevolent social skills, but this familiar role of “friend” does not apply well to the role of therapist. Thus, *focusing the client inward often means that the therapist is drawing out and highlighting—rather than reassuring or minimizing—the client’s core concerns:*

**INEFFECTIVE THERAPIST:** Don’t worry, your doctor will figure out what’s wrong—it’ll be all right. Have you gotten a second opinion yet?

### VERSUS

**EFFECTIVE THERAPIST:** Your doctor can’t make a diagnosis yet and tell you what’s wrong. You just don’t know what to expect or prepare for—so it makes sense that you’re scared right now. *Let’s explore together the fear and potential consequences you are most concerned about.*

There are many reasons why therapists often have difficulty approaching, and even just registering and empathically reflecting, the personal meaning or distress in what the client just said. For many, such forthrightness goes against unspoken but strongly held familial rules or cultural norms. In treatment, however, therapists want to do more than follow the limiting social rules and norms they may have grown up with. *With every turn of the conversation that seems to hold something meaningful for the client, the therapist is trying to register, reflect, or highlight the most important issue or key concern—what’s really being said here—rather than keeping things conversational or on the surface.* Doing so helps clients feel heard, seen, and understood—that the therapist “gets” them, and, as the attachment researchers put it, has the client’s thoughts, feelings, and experiences in

mind (Allen et al., 2008). That is, the therapist is fully present and attending—trying to discern the most salient message or grasp what the client is really saying or trying to convey right here (Groth, 2008). *When therapists can repeatedly capture the most important feeling or key issue in what the client just said,* clients leave the session with the feeling, “We’re really getting down to it” or “I’m not exactly sure what I want to do about this yet, but we’re sure talking about what’s important.” *This is not at all about leading clients toward more threatening material or pressing for difficult feelings or further disclosure; rather, it is more about approaching the significance of the material that clients have just volunteered or already chosen to present on their own.*

Another reason why new therapists may have difficulty engaging the client consistently in this substantive manner involves feelings of incompetence or inadequacy. While this approach brings meaning and intensity to the therapeutic interaction, *new therapists often feel unsure of what to do as clients focus inward and get closer to their feelings, choices, and meanings in their core concerns.* These unwanted performance anxieties often arise because new therapists inaccurately believe they need to “fix” or “solve” the client’s problem that is now etched more clearly, when what is really needed is a continuing partnership that first focuses on clarifying what the key concerns are and the meaning they hold for the client, and then collaboratively setting treatment goals. This process involves helping the client articulate:

*“This is what keeps hurting me...”*

*“This is what it means to me...”*

*“This is what I need or want...”*

*“This is what I want to change...”*

*“This is what I would like to stop doing...”*

*“This is what I would like to be able to do...”*

There are other reasons for not hearing or approaching the client’s emotional distress or bigger meaning. For example, some new therapists may equate this attempt with being intrusive or exposing because in the therapists’ personal experiences they have seen such honest communication utilized only in non-empathic, hurtful ways. Still other therapists are reluctant to hear or respond to their clients’ emotional messages, or to help clients explore their participation in relational conflicts, because they don’t want to make others “feel bad.” *These therapists try to make clients feel better by reassuring them about their insecurities and by emphasizing only their strengths and successes.* Stepping back from the accurate reflection of clients’ concerns or distress is usually ineffective. Although well intended, *it keeps things on the surface and prevents clients from being able to explore and clarify what’s really wrong.* When this happens, clients lose the opportunity to determine what the real problem is, explore what they want to do about it, and develop the self-efficacy that comes from being able to address and resolve problems. Our therapeutic intention should be to provide Carl Roger’s accurate **empathy**. We do not want to make the client’s problem bigger or smaller than it really is. To achieve accurate empathy we need to work collaboratively and ask the client, *“Did I capture that accurately or can you help me say it better—what are your words for how that was?”* In this vein, Farber et al. (2009) write compellingly about clients who have experienced trauma and who can, through self-disclosure with a supportive and affirming therapist *who listens to all aspects of their story*, develop greater self-awareness and a more integrated identity or cohesive sense of self.

Still other therapists may avoid an internal focus in order to please clients and maintain their approval. These therapists may fear that clients will feel awkward or uncomfortable and become angry as the therapist reflects or invites them to explore the difficult feelings or choices they have just identified. In particular, many new promising therapists may also be concerned that they may not be able to remain present and responsive to the client when strong emotions are expressed. It is understandable that some therapists will

struggle with this and it is important that they find help by sharing these concerns with a supportive supervisor. They should bring to supervision those emotions that might impact their work with clients so that they can discuss, role-play, and manage them with supervisors. When concerns are very strong and linked to significant personal issues, student therapists' strong reactions to the material can be best resolved in their own personal therapy.

Student therapists need their instructors to model and role-play responses, and they also need permission and overt support from supervisors to risk not staying on the surface with clients and instead to enter the real meaning in what the client has just presented. As we have already seen, fears of the clients' feelings—what they might reveal, or of not knowing what to do or say—often interfere. What is most important to clients, however, is that their therapist *join* them where they are and convey a willingness to try and grasp what is most difficult or important in what they just said. This willingness to be present in their need or vulnerability means more to our clients than having an “intelligent” or “learned” answer or solution—it gives clients a sense of partnership and feeling that someone wants to understand and respond to their need. Attachment theorists have often noted that this type of **attuned responsiveness** contains or helps to modulate distress (Shore & Shore, 2008). **For clients, the safety or security provided by such attuned responsiveness will facilitate an internal focus and lead to try new ways of thinking and behaving.**

Therapists collude with the client in externalizing problems when they repeatedly:

- *give advice and tell the client what to do or how to respond,*
- *interpret or explain what something means,*
- *reassure clients that their problems will go away or are not something to be concerned about, or*
- *disclose what the therapist has done to cope with a similar person or problem.*

As we would expect from client response specificity, each of these responses will be effective at times. However, if they come to characterize the ongoing course of treatment, they will hold clients back from learning about themselves and, ultimately, from being able to resolve their own problems. **It is important that throughout treatment therapists repeatedly try to help clients become more aware of their own internal reactions and interpersonal responses in problem situations.** Although it can be awkward or uncomfortable for new therapists to focus clients inward at times, it routinely strengthens the working alliance as the therapist gains credibility by demonstrating the ability to join the client in what's most important for them a way that others in their daily lives cannot.

## POINTS OF ENTRY

As longtime Practicum instructors and supervisors, we do demonstration initial interviews and cases for our students. We do not wish to be immodest, but the most common response we hear from trainees is how quickly we are talking with clients about substantive and compelling issues and sharing the deep feelings that accompany it. Even though they are watching the interview through a one-way mirror or on videotape, **students don't “get” how we got to the bigger feelings and significant issues so quickly.** We and many other therapists are able to do this, in part, because we “pick our spots.” We realize that we often can't make much of what clients just said, so we follow along, joining and helping them unfold their story. But we are always listening with full attention for opportunities to approach **Points of Entry that commonly take us into clients' core struggles.** These points of entry include listening for:

- *Painful feelings that accompany what the client is discussing (especially fear, sadness or loss, and shame);*
- *Distressing experiences (such as being disappointed or let down, helpless or alone with a problem, and invalidated or not heard);*

- Topics that link or connect in big or small ways to developmental/familial issues (such as growing up witnessing domestic violence or chronic parental conflict, physical or sexual abuse, or nonsupportive caregivers who undermine the child's confidence or sabotage the child's friendships or individuation and emancipation);
- Interpersonal patterns or relational scenarios that keep recurring and causing conflict for the client (such as feeling left or betrayed, having to prioritize the needs of others and ignore their own, being subjugated or controlled in relationships and having to just go along and comply, or having to be the responsible one and take care of others);
- Internal conflict or ambivalence (such as the client saying, "I want to lose weight but keep losing my resolve" or "Everybody tells me this relationship isn't good for me, and I agree, but I just can't break it off").

Developing the "ear" to hear how a seemingly small issue can be a point of entry because it connects to an affect-laden issue or more significant life-theme is a challenging skill for instructors to teach and for new therapists to develop. In the following section, we try to help by illustrating with two case vignettes where the clients present a promising point of entry but the supervisees miss the opportunity. In the first vignette, the trainee simply doesn't hear the far-reaching themes of rejection and loss the client presents in an encapsulated way and, in the second vignette, the trainee "sort of" hears the bigger problem and deeper meaning evoked in the concern the client is presenting, but is not confident enough to meet the client there. We will also provide effective responses to both of these points of entry that illustrate how to move beyond surface dialogue and engage the client.

*Sarah, a 19 year-old Caucasian female, came to the Student Counseling Center because she was "depressed." Sarah's parents had both been drug involved and she spent many of her elementary and high-school years in foster-care homes. When Sarah was 16, her aunt decided to take her in but, as soon as Sarah turned 18 and was no longer eligible for foster-care support, her aunt told her she was "old enough to take care of herself." Sarah felt deeply rejected by her parents, not really wanted by the various foster homes she'd been in, and used and then "thrown away" by her aunt.*

**CLIENT:** Last week my friend said she didn't want to hang out with me—didn't have enough free time to be with me as much anymore {*Point of Entry*}. I was mad at first but I got over it. I decided I should take care of myself and clean my room—but it's such a HUGE mess I don't even know where to start.

**INEFFECTIVE THERAPIST:** Well, I like that you are trying to figure out how to take care of yourself—and I have some ideas that could help you get going making your room nice again. The whole room sounds overwhelming. Maybe you could start with just one section at a time. Which part of your room is the worst, or which part do you think you could successfully tackle first?

**EFFECTIVE THERAPIST:** Oh Sarah, I'm so very sorry to hear that you may be losing this friendship. Yes, of course you were mad, and maybe it was hurtful too? **Can you bring me in some more about this loss or rejection?**

Losing a friend is nothing like losing one's parents, but this friendship loss does evoke all of the painful losses that have defined Sarah's life. Even though the ineffective, problem-solving therapist knew Sarah's developmental history, she missed this theme or link of rejection and loss that provided a point of entry. Like most new therapists, she has not yet learned to listen for, or recognize in the moment as it is being said, how the seemingly small issue being presented also expresses a larger life-pattern that holds great meaning for the client. In contrast, the effective therapist "heard" the painful theme of loss and rejection occurring yet again in Sarah's current life, saw this as a point of entry, and *approached* it with empathic understanding.



In the second vignette, the client has a history of domestic violence. She grew up hearing her parents fight and saw them hit each other. She was the oldest daughter and her mother confided in her:

**MOTHER:** Your father has just become a terrible old man! When we were dating and first married, he was so loving and considerate. Then he lost his job and became jealous and controlling.

The client, a 22 year-old Mexican-American woman, recently married to a police officer, felt anxiety as she heard her mother's story. Her husband was loving and thoughtful when they were dating but, on returning from Iraq, he was showing signs of jealousy and control over her time and activities and had begun checking her cell phone and emails. She brought this up with her therapist:

**CLIENT:** My husband is becoming controlling. He is jealous of any man who looks at me—even old men. But I can't control who looks at me. He says I dress in ways that attract attention. He is even jealous of my going to school—he says I won't want him when I'm done with school. He has even begun checking my phone and emails when he comes home from work. *{Point of Entry}*

**INEFFECTIVE THERAPIST:** He is jealous of you going to school? What reason does he have that going to school will affect your marriage? Have you been less available to him since you started school?

**EFFECTIVE THERAPIST:** I appreciate you sharing this with me. I'm so sorry you are faced with a jealous and controlling husband. What feelings come up for you when he acts this way?

**CLIENT:** I'm afraid we're going to end up like my parents—fighting all the time. I don't want the same thing to happen to my children and have them grow up the way I did, scared all the time. *{Point of Entry}*

**THERAPIST:** That makes sense to me. Let's work together to lessen the likelihood that your marriage will go down the same path as your parents' marriage.

**CLIENT:** How do we do that?

**THERAPIST:** The first three things that come to mind are: One, that you are not isolated and have a support system that knows about your concerns and is willing to respond when you need help; Two, that we figure out how you can talk to him about your concerns; and Three, that we can role-play how to set limits and boundaries and see if he can respond adequately to those. Let's address those red flags you identified so you are not at risk for more difficulties in your relationship. Those are my thoughts, how do they sound to you?

## CLIENTS' RELUCTANCE

Why do some clients tend to avoid an internal focus? As we have seen, a shift away from their focus on others can make some clients feel that the therapist does not really understand them. Clients can become frustrated because it seems as if the therapist is not grasping how difficult the other person really is behaving or is just not sympathetic to their concerns. Other clients fear that if they give up their attempts to change others, they will have to either accept the blame for the problem and be the “bad” one, or remain forever resigned to defeat. Still other clients, not yet feeling that a working alliance is solidly in place, do not have the safety they need to approach the difficult feelings or choices that looking inside entails. Clients also will be unable to enter certain topics that have evoked their vulnerability if the therapist and client are *reenacting* some aspect of the client's conflict in their interpersonal process as they have been in developmental and current

attachment relationships. Finally, the most important reason for clients' reluctance is mistrust or expectations based on previous experience that others don't want to hear their experience or will respond in unwanted ways.

**THERAPIST:** It sounds as though you feel misunderstood by your husband, your children, and by others often...

**CLIENT:** Yes, no one seems to get me. *{Point of Entry}*

**THERAPIST:** No one seems to get you—that's a real problem. Let's make it different here. I want to get you, so please tell me more about you—what you wish they, or I, would "get."

**CLIENT:** Like with my husband?

**THERAPIST:** Sure, what does your husband not know or see in you that you wish he did?

**CLIENT:** That I like my quiet time...that I can deal with problems better in the morning than I can after a long day at work...that I'm tired too...

Cultural factors may also impede an internal focus. For example, Native-American, Asian, and other clients from communally centered cultures may initially react negatively to invitations to look within because it may sound self-centered. If clients perceive a difference in values like this, they may question the therapist's credibility. Therapists can help by educating clients about the treatment process and responding in a manner that is congruent with the clients' worldview. Sue and Zane (2009) highlight the constructs of **credibility** and giving in working with culturally diverse clients. They note that therapists can achieve credibility by intervening in a way that shows sensitivity and conveys to clients that they are trustworthy and capable of providing assistance. In a family where roles are hierarchical, for example, **it would be helpful to role-play respectful ways to respond or see if there are culturally sanctioned ways to express one's own views or differing opinions.** They also identify "gifts" as features that convey hope or provide some benefit in the treatment process. This could include many different interventions that clients find useful with problems as they see and experience them, and that are still culturally congruent for them. For example, this may include helping a client reframe a parent's unwanted, hurtful behavior as a wounded expression of their own painful immigration story and unresolved loss—rather than the parent just "not loving me."

Perhaps the best way to work with clients who are reluctant to look at their own behavior or shared participation in problems is to *explore—in a non-blaming, curious manner, their reasons for not wanting to look within.*

**THERAPIST:** When I ask you about your feelings or responses when your mother is critical, you often change the subject. Help me understand what could go wrong if we talked about those feelings?

**CLIENT:** I'm afraid I'd get sad and maybe mad too, you know, like I might get too upset. *{Point of Entry}*

**THERAPIST:** Uh huh, afraid of getting too upset. **What's the danger—help me understand what you are afraid is going to happen or could go wrong if you get too upset?**

**CLIENT:** Well, I'd start out acting mad, but then I might start crying too hard and not be able to stop. I'd just be completely out of control—just humiliating...

**THERAPIST:** **Angry, and also sad, both very big feelings—that take away your sense of control and your dignity and leave you with this terrible sense of humiliation. There's so much here—tell me more about all of this...**

When a client talks about big feelings such as this, it provides the therapist with a point of entry, which is an opportunity to explore these feelings more fully. For example, where her belief about crying and not being able to stop comes from, if this has ever happened and, if so, how others responded to her in this vulnerable/exposed moment. Externalizing is often a defense against feelings that might, at the surface, include anger but often cover profound fear, sadness, or shame. These might include sadness and/or shame about being unseen, not cared about, demeaned, and so forth. However, empowerment comes from owning these feelings and then choosing to respond to oneself and to others, rather than repeating the cycles of behavior that have simply fueled the feelings of hopelessness and helplessness. Thus, focusing inward to explore these feelings will help clients understand that the feelings may have made sense for a child whose vulnerability was ignored or whose longing for comfort was ridiculed as weak. Clients might then be able to link their externalizing defense to a disconnection from family, Self, and other aspects of their presenting problems.

In most cases, however, clients will welcome and accept the therapist's invitation to say more about themselves. However, if clients remain unable to respond, the therapist can simply give clients permission to disclose or proceed at their own pace. For example:

**THERAPIST:** It doesn't seem comfortable for you to share very much of yourself with me yet, but I do think it's important that you choose what and how much you share. You know, you are in the driver's seat. How does that sound to you?

Gradually, however, rather than sit and wait, and the client's externalizing stance does not begin to change, the therapist can make a process comment that makes their interaction overt—as a topic that is now acceptable for discussion. For example:

**THERAPIST:** I've noticed that you talk very easily about your husband and your daughter, but you don't say very much about yourself. Are you aware that that happens?

OR

**THERAPIST:** I think we've been missing each other the last two sessions. I keep asking what you were thinking about or trying to do in a particular situation, and you usually respond by telling me more about the other person. What do you see going on between us?

Such process comments will help clients become aware of their externalizing style with the therapist and learn that they probably respond in the same way to other people as well. For example, clients who cannot share themselves in a personal way will often be perceived by others as boring or aloof. In that case, part of the clients' presenting problem, such as the clients' complaint of loneliness or lack of meaningful relationships with others, may also be reenacted in their relationship with the therapist. While it's often initially discouraging for clients to realize that they are re-creating their conflict in therapy, this reenactment provides the opportunity to begin resolving this problem by changing the distancing pattern first within the safety of the therapeutic relationship. This interpersonal feedback from the therapist is often a gift that helps clients become aware of how they interact with others and recognize—often for the first time—*how others frequently experience them*. Now, the therapist's intention is to begin working to help create a different and more effective way of relating in their relationship. In the following example, the therapist uses a process comment (specifically, a self-involving comment) to disclose how the client is affecting him at this moment, and uses this immediacy intervention to move beyond their surface, everyday conversation and invite a more meaningful dialogue and relationship:

**THERAPIST:** I feel that I am being held away from you when you talk so much about others but so little about yourself. I would like it if you found me safe and we could talk more about your concerns or what matters to you. Can we work together on this?

When the therapist uses a process comment or bid to focus clients inward like this, clients often reveal new and important concerns they have not talked about previously. As we see in the following sample client responses, meaningful issues commonly become accessible for collaborative exploration and work in treatment:

- *Well, I guess there really isn't very much about me that people would want to know.*
- *I'm not used to telling people what's going on for me. I guess I usually take care of them.*
- *Every time I try to get close to someone, they betray me.*
- *You wouldn't like me very much if you knew more about me.*
- *When I start to get close to others, they try to take control. It's better to just be alone.*

In each of these differing ways, the therapist's process comment brings out important new concerns—that are actually life-defining for the client—that can now be addressed in treatment. The therapeutic relationship has intensified, and highly significant new information that is central to the client's symptoms and problems has been identified. Clearly, this process of helping clients focus inward, and exploring their resistance to looking within, provides some of the most important material to be addressed in therapy. These far-reaching concerns are unlikely to be brought out and addressed in treatment unless the therapist uncovers them by working in the moment with different types of process comments and focusing clients inward. When therapists can provide this kind of safety and understanding in the therapeutic relationship, clients can let down their guard and allow themselves to know and feel things they have tried to keep at bay.

## DEVELOPING AGENCY: PLACING THE LOCUS OF CHANGE WITH CLIENTS

### FOSTERING THE CLIENT'S INITIATIVE

Effective therapy of any treatment length should foster the client's self-efficacy—we want the client to develop a greater sense of agency through the treatment process (Cervone, 2000; Galassi & Bruch, 1992). Therapists cannot just talk with clients about choice, responsibility, and personal power, *however; they want to co-create a relationship in which clients are behaving in stronger ways with the therapist in the session.* When clients can first do this with the therapist, the therapist can readily help them transfer this stronger way of acting to the rest of their lives. Thus, the therapist first gives clients permission to follow their own interests and actively encourages them to introduce the material that seems most relevant to them. Consider the following example:

**THERAPIST:** Where would you like to begin today?

**CLIENT:** I'm not sure—what do you think would be best?

**THERAPIST:** I'd like to join you in exploring whatever you think is most important to work on right now. What might that be for you?

**CLIENT:** I'm not sure.

**THERAPIST:** Sure, let's sit a moment, let you take a breath, settle into yourself, and think about what you would like to have different in your life. What seems most alive for you right now?

**CLIENT:** *(Pause)* I think I want to talk about my wife. We had the best weekend together we've had in a long time.

**THERAPIST:** Wonderful, I'm happy for you. What was different this weekend—were you doing anything differently?

**CLIENT:** I think I talked to her more in the way that you talk to me. I asked about what she thought a few times and what she wanted to do, and I could tell she liked it. Maybe I should keep doing this...

After giving clients permission and actively encouraging them to begin pursuing their own agenda in this way, the therapist can be an active participant who helps clients to explore their own concerns more fully, to understand their problems better, and to generate potential solutions and behavioral alternatives. Why is it essential to encourage clients' own initiative and meet them at their point of urgency, even in the time-limited modalities within which therapists increasingly work? In one way or another, many clients have been unable to act on their own interests or have not been supported in pursuing their own goals. In past relationships, these clients have not had significant others support their own interests or help them do what they want. As a result, these clients will feel encouraged but often anxious as well, if the therapist supports their own self-direction and tries to discern what matters most to them, regardless of theoretical orientation. **therapy becomes a more intense and productive experience when the therapist can successfully engage clients in pursuing their own interests and clarifying what matters most to them.** When this occurs, clients take more ownership of the change process. Routinely, this will be evidenced as they invest more fully in the working alliance.

When the therapist can help clients explore and better understand the material they choose to focus on, rather than direct them to the therapist's agenda, change often begins to occur and presenting problems improve (Csillik, 2015). Whatever interventions or techniques the therapist uses, they will be far more effective when given in response to the client's own interests or concerns, rather than the agenda of the therapist, employer, or spouse. This process dimension is one of the most important characteristics of the therapeutic relationship and, because it is so challenging in the beginning, we will explore it further. Once new therapists "get" this interpersonal process dimension and can apply it to their clients, it can help make every type of therapeutic intervention more effective. When therapist and client can engage in this type of mutual interchange and sustain it throughout several sessions, most clients become committed to the treatment process and will report that meaningful changes with others are beginning to occur. Facilitating this growth process may be the primary challenge and satisfaction of being a therapist. Let's see how this sequence might actually sound with a more challenging or provocative client:

**CLIENT:** I need more direction from you. What should I do here? You're the expert.

**THERAPIST:** I have a couple of ideas to share with you, but I think it would work best if I heard yours first.

**CLIENT:** (*impatiently*) If I knew what to do, I'd just do it. I come here to get help from you!

**THERAPIST:** (*bidding for client initiative*) All right, let's swap ideas. You tell me what you think is going on; then I'll tell you what I see occurring; and let's see what we can put together.

**CLIENT:** (*Client rejects bid—presses therapist to lead*) Like I just said, I don't know—I don't really have any ideas.

**THERAPIST:** (*Making a second bid for the client's initiative*) **Let's wait for just a minute and see if anything comes to you. If not, I'll be happy to go first and start us off.**

**CLIENT:** (*Client accepts bid and focuses inward*) (20-second pause) Well, maybe I'm afraid of being alone or something like that.

**THERAPIST:** (*Client focused inward and revealed vulnerability, and therapist recognized this point of entry*) That's interesting and fits with what I'm thinking about. Tell me more about what it means to be "alone"; it sounds important.

**CLIENT:** (*Client responds to bid and elaborates further*) I think I've always been worried about that.



**THERAPIST:** (*With client now working collaboratively, therapist tentatively suggests a possibility*) From other things you've said, it makes me wonder if your parents cut off or disengaged from you emotionally whenever you disappointed them by doing what you wanted rather than what they expected. I'm wondering if that contributed to this feeling of being all alone, even though others were physically present—which would have been very confusing to a child.

**CLIENT:** (*Collaboration continues as client refines therapist's hypothesis*) Yeah, maybe...and the worst thing about it was that it seemed like it was all my fault. They were going away because I let them down. What do you think?

**THERAPIST:** (*Therapist validates client's strength, and uses it as a further point of entry for exploration*) Uh huh, you know, I think you have very good ideas, but something often seems to hold you back from expressing them in the strong way you have just been doing with me.

This collaborative interaction, in which the therapist and client each build on what the other has just produced, is an independence-fostering approach to treatment. Clients have the experience of sharing responsibility for the course of treatment, as the therapist actively encourages and facilitates addressing their own concerns. As therapists support clients in achieving this more active stance, they are simultaneously contributing their own ideas and suggestions in a way that creates a working partnership. *The therapist in the previous dialogue is highly engaging and active—but not very directive.* She is following the client's lead, but she is not being non-directive, either. The interpersonal process we are striving to achieve, and has long been an empirically supported aspect of effective therapeutic relationships, is this collaborative effort (Horvarth & Greenberg, 1994).

A common misconception is that longer-term, dynamic, or relationship-based therapies are dependency fostering, whereas short-term, problem-solving, or strictly behavioral ones are not. Actually, whether therapy fosters dependence or independence is determined by the therapeutic process and not by the length of treatment or the theoretical orientation of the clinician. In short- or longer-term therapy, the client's dependency is inappropriately fostered when the therapist repeatedly directs the course of therapy, gives advice, and prescribes solutions for the client. Let's examine this collaborative alliance further, because it offers an effective middle ground between the less productive polarities of directive and non-directive control.

**Shared Control in the Therapist–Client Relationship.** As we have begun to see, treatment will be most successful when the therapist and client **share control over the agenda and direction of therapy.** In most cases, it is overtly controlling for the therapist to play the predominant role in structuring and directing the course of treatment and it is ineffective to non-directively abandon clients to their own confusion and conflicts. In a more productive relationship, therapists will encourage the client to take the lead but will actively contribute their own understanding and guidance about the material that the client has produced. **It will be therapeutic for many clients to experience a relationship in which both participants share responsibility.** This collaboration in the therapeutic alliance is the key to greater self-efficacy and provides clients with the support they need to act in new and stronger ways with others in their lives. **As previously discussed, the therapist is (1) encouraging clients to take the lead and initiate whatever they believe would be most useful to discuss; (2) following clients' lead and joining in their sense of the problem or what's most important; and (3) looking for points of entry where clients have expressed a significant feeling, concern, or issue, and making active bids to enter this significant topic more fully.** Doing so will move the client and therapist from everyday conversation to exploration of meaningful material. Consider this statement:

**THERAPIST:** I would like to begin each session by having you bring up what you want to talk about. I would like to join you in working on what seems most important to you. I will be sharing my own ideas, reactions, and suggestions as well, but I think this is the best way to begin a good partnership.

Some therapists will be frustrated by this approach and direct clients toward taking action and finding solutions, especially if they are feeling pressed to fix the problem quickly because of the limited number of sessions available.

Responding to these arguments, the solution-oriented or directive therapist might counter, “What if you follow the client’s lead and it takes you nowhere?” This type of therapist might also express concern that a non-directive approach requires an unrealistic amount of time from the therapist and results in a disorganized therapy that is lacking in focus. Thinking in terms of reenacting patterns and themes, a purely non-directive approach may be more likely to reenact problematic relational patterns for clients who grew up in a permissive home, or with passive/disengaged caretakers. In parallel, a purely directive approach may reenact familiar patterns for some clients who grew up with controlling parents in an authoritarian home. **As we are repeatedly trying to illustrate in our sample dialogues, however, there is an effective middle ground of shared therapist-client control or collaboration, and it is especially well-documented by Miller & Rollnick’s Motivational Interviewing (2013).** With this middle ground of shared control and collaboration, the therapist in the following example does not wait non-directively for the client to bring up more relevant material, or directionally lead the client to a new topic. Instead, the therapist makes a process comment about their current interaction and invites the client to join in reshaping it:

**THERAPIST:** It doesn’t seem to me that this is taking us anywhere right now. Maybe I’m not getting what you are trying to tell me, or maybe there’s a better way to use our time. Do you think there’s another way to talk about this or is there something else that matters more to you? What are your thoughts about what I’m wondering?

Therapists need treatment plans and intervention strategies with short- and longer-term goals. However, **these are most effective in producing enduring or sustainable change when they develop out of a collaborative interaction, where the client shares responsibility for successes and disappointments and feels ownership of the treatment process.** To implement this interpersonal process, the therapist needs to be able to tolerate ambiguity and refrain from subtly controlling the interaction. Instead, the therapist is trying to create opportunities for clients to voice their own concerns and to act on their own initiative. The therapist’s task, then, is to be able to de-center; enter into the client’s subjective worldview; and empathically *highlight* the core meaning, central feeling, or relational pattern that this issue seems to hold for the client. This is what we mean when we talk about the therapist looking for a point of entry to move beyond surface dialogue. This approach requires an active therapist who is neither directive nor non-directive but has the flexibility to tolerate ambiguity and share control. In this approach, therapists respond actively by:

- *communicating their understanding and affirming the client’s experience;*
- *joining the client in exploring further the key meaning in what the client just said;*
- *using self-involving comments or their own experience of the client to provide interpersonal feedback about how the client affects the therapist—and may be contributing to problems with others;*
- *affirming new behavior that disrupts old patterns and demonstrates change with the therapist or with others; and*
- *providing a focus for treatment by highlighting the repetitive interaction sequences, central feelings, and faulty beliefs that recur throughout the client’s narratives.*

Furthermore, the therapist is using process comments to address treatment impediments—such as resistance or an externalizing focus—and to prevent unwanted reenactments or unrecognized ruptures between the therapist and client (e.g., the client is silent about her faulty belief that the therapist is impatient, bored, or burdened by her—as others often have been).

Therapists do this by working in the here and now and asking about what may be going on between them in their relationship or current interaction. For example:

**THERAPIST:** What's it like to be talking with me about this?

**CLIENT:** Well, you're probably feeling impatient with me—like everybody else does—because I'm still stuck on this.

In this way, the therapist intervenes by taking the client's problems out of abstract discussions about others “back then and there.” Instead, the therapist is looking for opportunities to create *immediacy* by linking the client's problems with others to what may be occurring between them in the way they are interacting right now. For example:

**THERAPIST:** No, I'm not feeling “impatient” with you at all. Is there anything I'm doing or that is happening between us that makes you feel that way? If so, let's sort that through together and change it. I can see this is a complicated problem for you, and I appreciate how hard you are trying to do something about it.

**CLIENT:** Really, you're not frustrated with me? You know, that makes me almost want to cry—it feels like I've been a disappointment to just about everybody in my life....

Working in the here-and-now with clients is a big new step for most therapists. *It brings therapists and clients out of the mode of everyday social interaction and, as we see above, it quickly takes us to the heart of what's wrong.* Significant feelings come up and key issues that the therapist didn't know even mattered to the client now take center stage. And, just as important, the relationship becomes more intense and begins to matter more to both the therapist and the client.

As we will explore in Chapter 6, an Authoritative parenting style that combines both parental affection and communication with firm limits reflects a more effective middle ground between the better known and more widely adopted Authoritarian and Permissive parenting styles. In parallel, therapists will find that shared control offers most clients a more productive alternative than either directive or non-directive approaches. Trainees need role models of how therapists and clients can work together in this actively engaged and collaborative way, so let's examine interventions that foster a strong working alliance and illustrate this middle ground of shared control.

As we are seeing, the therapist is trying to establish and sustain the interpersonal process throughout each session and encourage the client's lead while still participating actively in shaping the course of treatment. The key is for the therapist to intervene without taking the impetus away from the client. The following examples illustrate ineffective and effective ways to do this.

**Ineffective Interventions.** Suppose that the client is filling the therapy session with seemingly irrelevant storytelling. The therapist cannot find a common theme to any of the client's narratives or understand the personal meaning that these vignettes hold for the client. It seems as if nothing significant is occurring. At this point, it is easy for the therapist to stop the client and direct him or her toward a specific topic that the therapist thinks would be more fruitful. This refocusing can be effective at times, but therapists will succeed more often if they revitalize the therapeutic interaction without shifting the impetus away from the client and onto the therapist. In the following dialogue, the onus for therapy comes to rest with the therapist, and the client loses an internal focus for change:

**INEFFECTIVE THERAPIST:** I'm wondering where this is taking us. Have I lost the focus here?

**CLIENT:** I'm not sure where I'm going with this, either. What do you think I should talk about?

**INEFFECTIVE THERAPIST:** You've had trouble asserting yourself in the past, and I think we need to look more closely at that. Last week, you said you wanted to ask your boss for three weeks of vacation instead of two. How are you going to handle that confrontation?

**CLIENT:** I'm not sure. What do you think I should say?

**INEFFECTIVE THERAPIST:** To begin with, you need to arrange a face-to-face meeting with him. It is important that only the two of you are present, so that you can have his full attention and there is less threat for either of you to lose face. Then use the "I" statements we have practiced to state directly what you want.

**CLIENT:** Sounds good, but what would you say to him first?

Assessing the client's responses in this dialogue, it is clear that this intervention is not productive, even though the therapist has moved the client to a more salient topic and provided useful information about effective negotiations. The fulcrum of therapeutic movement has tipped from the client to the therapist, and a hierarchical teacher-student process has been established. Most clients will not be able to utilize the therapist's useful information until they pick up the momentum and begin to participate actively again. Furthermore, because the client remains in a passive role as the therapist continues to inform, the client does not gain the increasing sense of self-efficacy that comes from participating more collaboratively in treatment. A more productive intervention might be to make bids to engage the client more actively in treatment rather than continuing this faulty interpersonal process. However, *if the client does not become actively engaged again, therapists do not want to just sit and wait non-directively*. Instead, they can actively intervene by simply observing this with a process comment. Let's illustrate this more effective approach.

**Effective Interventions.** The therapist can refocus clients toward more productive material—but in a way that keeps the momentum for treatment with the client. **Again, process comments that make the current interaction between therapist and client an overt topic for discussion are often effective in these circumstances.**

**EFFECTIVE THERAPIST:** I don't have the feeling that what you're talking about is really very important to you. Am I missing the point here, or is this really important? Help me out.

**CLIENT:** Yeah, you're right, I'm not sure where I'm going with this, either. What would you like me to talk about?

**EFFECTIVE THERAPIST:** I think that we should try to identify what would be most important to you and talk about that. What might that be right now?

**CLIENT:** Uh, I'm not really sure.

**EFFECTIVE THERAPIST:** OK, Let's just sit together quietly for a few moments, and let you check in and be with yourself, and see what comes to you.

**CLIENT:** *(pauses)* Well, there is something that I've been ashamed to talk about. Do you think it would be a sin if I was bisexual, or if I had sexual feelings toward men sometimes?

New therapists often feel uncomfortable with silences and may find themselves anxiously filling them. If therapists can refrain from doing so, even just for a moment or two, these are opportune moments for clients to establish their own agenda or introduce new material that is far more meaningful—as in the preceding dialogue. Again, this type of process comment invites the client to approach more substantial material but, in line with our goal, without taking the impetus away from the client. The therapist has directly intervened by sharing an observation and asking about the client's perceptions of the

current interaction, but the therapist has been skillful in leaving the client an active collaborator in revitalizing the discussion. Routinely, clients produce far more significant material when they are invited to lead in this way rather than to follow. As always, try it out with your clients, assess their reactions, and see what works best for you.

Let's look at a different illustration of these treatment guidelines, but this time with a provocative client who would be challenging for new therapists. Imagine the following situation: A 19-year-old White client in a college counseling center had been telling his therapist about his attempts in early adolescence to observe his stepmother undressing. The client had been detailing his voyeuristic efforts at great lengths, but the therapist did not feel that this was genuinely of much concern to the client. Unless the therapist could find the relevant meaning for the client, he wanted to move on to other, more salient issues in the client's current life. In the following dialogue, the therapist uses a process comment to refocus the client in a way that places the locus for change more fully with the client and, in the process, reveals a new and more salient treatment issue.

**THERAPIST:** As I listen to you, I don't get the feeling that you are really very interested in what you are telling me. Is that the case, or am I not understanding the meaning this holds for you?

**CLIENT:** I thought therapists were interested in this oedipal stuff. I figured you would want to hear about it.

**THERAPIST:** I'm struck by the fact that you are telling me what you think I want to hear, rather than talking about what's most important to you. Maybe that's something we should discuss. I wonder if you find yourself doing something like this in other relationships as well?

**CLIENT:** (*sarcastically*) Well, duh! Of course—isn't that what everyone has to do with teachers and parents and therapists like you? Figure out what you want and go along with it?

**EFFECTIVE THERAPIST:** Well, it's what some people have learned they have to do with some teachers and some parents and some therapists. But it's not what you have to do with everyone, and it's not what you have to do with me. I like people who have their own mind.

**CLIENT:** (*sarcastically again*) Do you now?

**THERAPIST:** Yes, actually, I do. So, tell me, who in your life can you be honest with and talk about what you really care about—and who do you have to superficially comply with and talk their talk? While perhaps simultaneously feeling resentful as you may be doing with me right now?

**CLIENT:** So you think I'm being a bad boy and dis-sing you

**THERAPIST:** No, I don't think you are being a bad boy—but I do feel you are being provocative with me. Are there others that you talk to in this way?

**CLIENT:** Well, (*pause*) maybe my mom, sometimes.

**THERAPIST:** What happens if you to talk to her about what matters for you?

**CLIENT:** She gets mad—like I embarrass her by being who I am.

**THERAPIST:** Ouch! I'm sorry for both of you. Have you ever tried talking with her about this? You just said it so clearly.

**CLIENT:** (*with disdain*) Of course not—do you believe in Santa Claus and the tooth fairy, too? She's not going to change!

**THERAPIST:** As you talk with me like that, especially in that contemptuous tone of voice, I feel insulted—as if you think I'm an idiot. Is that what you meant to convey?

**CLIENT:** (*pauses*) Uhh...



**THERAPIST:** I wonder if you are aware of sometimes talking to people in that sarcastic or contemptuous way?

**CLIENT:** I guess I sort of do and sort of don't—but as you say it, I guess it could be a problem.

**THERAPIST:** Yeah, I don't think others would like that much. Some might respond in kind, you know, spit back, and some might step away a bit—or even end their relationship with you.

**CLIENT:** Yeah, all that's happened...(pause, silence) My mom and I do this to each other sometimes too.

**THERAPIST:** I'm sorry, I can see that would not go very well for either of you. I don't know if your mother can change or not, but I think it's worth testing the waters to see. Maybe she could respond better if you began talking to her differently—in a more respectful and straightforward way about what's really going on between you, rather than in the sarcastic way you were just talking to me. That improves her chances of being able to respond better. What do you think?

**CLIENT:** Yeah, maybe, but how can things get better for me if she still thinks I have to do everything her way and be the way she needs me to be all the time?

**THERAPIST:** I have several ideas about what you could do differently with her—whether she is capable of change or not—that could leave you feeling better about yourself.

**CLIENT:** How would things be better for me if it turns out to be hopeless with my mom? I think she'll just say what she always does—everything's my fault; I'm always doing it wrong.

**THERAPIST:** She very well might, sadly, but if you tried talking with her about this in a more respectful way, I think you'd feel differently about yourself and your part in this hurtful interaction. Maybe you'd even stop believing that all of the problem is always your fault. So, other relationships in your life might go a little better, too, even if it doesn't change with your mom.

**CLIENT:** Like with my girlfriend?

**THERAPIST:** Yes, exactly. Tell me how this goes with your girlfriend...

In summary, the therapist in this dialogue is taking the risk to talk judiciously about “you and me.” The therapist provides interpersonal feedback about the client's insulting manner in a non-blaming manner and invites the client to take more responsibility for causing and solving the problem. Carkhuff (2008) calls this **personalizing**, and encourages therapists to respond in ways that help clients realize that they usually have some shared responsibility for creating and maintaining their problems. Therapists who avoid making this challenge for greater personal responsibility generally lose credibility with the client, and the working alliance is diminished. The therapeutic dialogue may be supportive and understanding, but it doesn't go very far—it isn't enough to lead to change for a client such as this. Thus, therapists are encouraged to respond in this personalizing way and say, for example, “You're upset because they took advantage of you, and because you didn't stand up for yourself when it happened.”

## ENLIST CLIENTS IN SOLVING THEIR OWN PROBLEMS

As noted earlier, clients benefit most from looking within, sharing their inner world, and engaging in understanding and resolving their own problems; that is, taking ownership of the change process where they can learn from their mistakes and gain from their successes. Too often, new therapists hold unrealistic expectations that they must be experts who possess insightful solutions to their clients' problems; they then suffer under these performance demands with painful feelings of inadequacy. Here again, this misconception places the impetus for change in the therapist's lap and gives clients the role of passive

recipients, waiting to be cured. Rather than adopting this hierarchical model, a more effective approach is to extend the collaborative alliance into the problem-solving phase as well. Therapists will be more effective when they elicit clients' active involvement in resolving their own problems. For example, a therapist might ask:

- *What is the most important thing you want to change to make your life better?*
- *What have you tried in the past that has worked for you?*
- *Looking back, what have others done to make things better—or worse—for you, when you were in this situation?*
- *What could you and I do together to try and help with this?*
- *How do you understand the way things have gone in your life to get you in this position?*
- *If you were to wake up tomorrow and your problems had improved, what would you have changed to make things better?*

Therapists too often establish expectations that set the stage for the therapist to tell clients how to lead their lives—by explaining what it all means, giving advice, and prescribing solutions. However, when clients participate in a collaborative partnership, it is empowering and gives them the opportunity to become more capable of managing their own lives. Rather than just providing answers, therapists are most effective when they help clients learn how to think about and explore their own problems, consider things from different perspectives, and generate their own solutions and alternatives (Arkowitz et al., 2015). Real change occurs when therapy has not only resolved clients' presenting problems but has fostered their self-efficacy. To illustrate this, let's look at two different ways of responding to a client's dream. Although the interpretation of the dream remains the same, the first example reenacts the client's problems whereas the second enacts a different interpersonal process and provides a CEE.

## RESOLVING CLIENT'S CONFLICT

*Anna, a 24-year-old client, lived at home with her embittered and chronically embattled parents. For several years, Anna had been struggling with the developmental transition of emancipation from her family of origin. She could not establish her own adult life. Anna had few friendships, dated little, and had no serious educational or career involvements. Anna's "martyr-ish" mother and alcoholic father wrangled constantly, and Anna felt that it was her responsibility to stay home and help her mother cope with her failed marriage. Anna entered therapy complaining of depression.*

After several sessions, Anna recounted to her therapist a dream that was of great importance to her. As the dream began, Anna was riding a beautiful horse across an open savannah. She felt as one with this graceful animal as they glided effortlessly across the broad grasslands. They sped toward a distant mountain, past sunlit rivers, birds in flight, and herds of grazing elk. Anna felt strong and free as she urged the tireless animal onward.

The distant mountains held the promise of new life amid green meadows and tall trees. Anna felt their promise quicken inside her as she urged the horse onward. But as the mountains drew near, Anna and the horse began to slow. The horse's legs became her own and grew heavier with each step. Anna desperately tried to will them on, but their footing became unsure and they began to stumble. At that moment, she was surrounded on all sides by menacing riders, ready to overtake and capture her. As she awoke, Anna choked back a scream.

"What do you think my dream means?" Anna asked.

Her bright, concerned, young therapist offered a lengthy and insightful explanation. The interpretation focused on Anna's guilt over leaving home. The therapist suggested

that, if Anna went on with her life and pursued her own interests, she would feel powerful and alive—just as she had in the dream. But before Anna could reach her goal and experience the satisfactions of having her own adult life, she would have to free herself from the binding ties of responsibility that she felt for her mother and her parents' marriage. Her loyalty to her mother threatened to entrap her in guilt and prevent her from being able to live her own adult life. The following dialogue then took place between Anna and her therapist:

**ANNA:** (*enthusiastically*) Yes, you're right, I do feel like I'm doing something wrong whenever I leave my mother and do what I want. Is that why I dreamed that?

**INEFFECTIVE THERAPIST:** We've been talking about whether you are going to move into an apartment next month. I think the dream reflects your guilt over taking this big step on your own. You know, like we've been talking—that by becoming more independent—in a normal or appropriate way, you're being selfish and hurting your mother.

**ANNA:** I want to move out, but I can't leave my mother with my father. She says she will divorce him if I move out, and it'll be my fault. What should I do?

**INEFFECTIVE THERAPIST:** I can't make that decision for you. It's important for you to be responsible for your own decisions.

**ANNA:** But I don't know what to do, and you always know what's best. You're so much smarter than me. I thought about that dream a lot, and I didn't know that's what it meant.

Throughout the rest of the session, Anna continued to plead for advice and expressed her discouragement about being able to resolve her own problems. The well-intended therapist did not want her to become dependent on therapy and kept refusing to tell her what to do—giving her explanations about autonomy, independence, and the need for Anna to find her own solutions. At the end of the hour, Anna felt agitated and depressed, and the therapist was still trying to explain the need for Anna to make her own decisions.

Although the therapist was astute in linking Anna's dream to her separation guilt and the current manifestation of her emancipation problems, this was an unproductive session. How did their interpersonal process go awry?

In this session, Anna experienced a relational pattern with her therapist similar to the pattern she was struggling with at home. In her family, Anna was indeed trained to be dependent and believe that she didn't have the wherewithal to live successfully on her own. Further, she was made to feel guilty whenever she did act independently or on her own behalf. In the beginning of their session when the therapist interpreted the dream so accurately, the therapist acted as the knowing parent who gave all the necessary answers to the needy child. This interaction behaviorally communicated the message that Anna will be dependent on the therapist's superior understanding. On another level, however, this message was contradicted by the therapist's verbal message about independence. This mixed message from the well-intentioned therapist immobilized Anna. The therapist's direct interpretation could have worked well for some clients but, recalling client response specificity, such an approach was problematic for Anna.

Anna reacted so strongly because this interaction cut to the quick of her problem. On the one hand, she needed and greatly wanted permission from the therapist to become more independent but, on the other, she expected that the therapist needed her to remain dependent, just as her mother did. When the problematic relational pattern of having to remain dependent on the authority was reenacted with her therapist, Anna's faulty belief that she was incapable of making her own decisions and having her own life was confirmed. Anna remained depressed until the therapist successfully reestablished a more collaborative interpersonal process with her. With the help of a supervisor, the therapist did this in the next session by soliciting Anna's ideas about something they were discussing.

The therapist expressed genuine pleasure in watching her be so insightful and, following this, shared some related ideas. This time, Anna readily picked up on the therapist's ideas and used them to further her own thinking—it was a partnership.

Recalling “rupture and repair,” Anna and her therapist soon corrected their reenactment and restored their working alliance, and treatment successfully progressed. When therapists can acknowledge a misattunement on their part, it allows the client to enter the relationship more fully and make progress (Safran & Kraus, 2014). The client is given the opportunity to clarify his or her experience and need. In this way the client becomes further empowered to elucidate what is going on and what he or she wants to have change—the client is now more actively engaged as an agent of the treatment process (Benjamin, 2009).

With this in mind, let's consider other, more effective ways of responding to Anna's question, “What do you think my dream means?” Interventions that invite a collaborative partnership—that engage clients in exploring issues with the therapist—will usually be more effective than explanations or guidance provided by the therapist, no matter how accurate (Kazantzis & Kellis, 2012). Therapists can choose to respond in many different ways and can engage clients in exploring their own dreams or any other material they present. In such a collaborative interpersonal process, therapists might ask the following questions:

- *It sounds like a very important dream to me, too. Let's work on it together. Where should we begin?*
- *What was the primary feeling you were left with from the dream? Can you connect that feeling to anything going on in your life right now?*
- *What was the most important image in the dream for you? What does that image suggest to you?*
- *Let's exchange ideas. I'll give you a possibility I'm thinking about, and you share one with me. It'll be interesting to see what we can come up with together. Who should go first?*

Each of these varying responses encourages the client to participate actively with the therapist. Ultimately, the therapist may want to give the same interpretation of the dream but will do so with *skillful tentativeness* by collaboratively inviting the client to modify or refine the interpretation to make it fit better (Kiesler, 1996).

Even though the therapist provides an explanation, the process will be significantly different and far more enlivening for clients if it evolves out of their joint efforts and is integrated into their continuing collaboration. This approach gives clients a relationship in which they are neither dependent on the therapist, nor unable to move forward on their own. Instead, clients are encouraged to exercise their own abilities, and selectively utilize suggestions, and refine possibilities from the therapist. When this occurs, clients feel more ownership of the therapeutic process, their motivation to work in treatment increases, and their self-efficacy is enhanced. Facilitating such self-efficacy provides a CEE for clients like Anna, who get the permission they need to grow out of their dependence or binding ties of guilt and act capably in their relationship with the therapist—something they may not have experienced in past relationships (Silberschatz, 2005; Weiss, 1993). To illustrate this more balanced approach, let's return to Anna's situation and see how readily the therapist can make it more collaborative and productive:

**ANNA:** (*enthusiastically*) Yes, you're right, I do feel like I'm doing something wrong whenever I leave my mother or do what I want. Is that why I dreamt that?

**EFFECTIVE THERAPIST:** We've been talking about whether you are going to move into an apartment next month. I'm wondering if the dream has something to do with you feeling guilty about taking this big step on your own. What comes to mind for you as I wonder aloud about this possibility?

**ANNA:** I want to move out, but I can't leave my mother with my father. She says she will divorce him if I move out, and it'll be my fault. What should I do?

**EFFECTIVE THERAPIST:** Tell me what you think as I slowly reflect back what you just said: "It will be my fault if my mother leaves my father."

**ANNA:** (*pauses*) It sounds crazy...it's ruining my life.

**EFFECTIVE THERAPIST:** That's right—it's ruining your life and it's not true. Children are not responsible for their parents' marriages, and it's not fair when they are pulled into taking sides in marital conflict or expected to rescue a parent from their problems like this—it keeps you from having your own life.

**ANNA:** I feel better when you say that...but what should I do?

**EFFECTIVE THERAPIST:** What would you like to be able to do about this?

**ANNA:** I think I want to speak up, but I just don't want to hurt her feelings...

**EFFECTIVE THERAPIST:** Yes, I get the maddening conflict you've been talking about: you want to be able to have your own life, but you don't want to hurt your mother because you love her. So, tell me about both sides of "speaking up." What would you want to say, and how would it be for you if your mother felt hurt by that?

**ANNA:** I'd like to say that I don't want her to complain to me about my dad anymore. I don't want to hear what's wrong with him. I don't want to have to take sides. But she'd definitely feel hurt and totally betrayed if I said anything like that.

**EFFECTIVE THERAPIST:** It would be great to set boundaries with her like that, but the guilt over hurting her would be very hard for you. You've struggled with this dilemma for a long time now...saying and doing what you need "hurts" her.

**ANNA:** Yeah, it does, but I would like to find a nice way to speak up—you know, respectfully...but she'll still be hurt.

**EFFECTIVE THERAPIST:** Sure, let's find respectful ways to do this, but also keep realistic expectations about how she's likely to respond—and how that's going to affect you.

**ANNA:** Right, I know what you're thinking. I can probably stand up better for myself, but my mother's probably not going to give me any support for moving out or doing what I want with my life. She's going to feel hurt and betrayed no matter how I say it.

**EFFECTIVE THERAPIST:** I'd love to be wrong but, sadly, I think that's a realistic possibility too.

**ANNA:** Yeah, it is sad, really sad. So I'm stuck here and have no clue what to do next.

**EFFECTIVE THERAPIST:** Well, in the past you've liked it when we role-play things. Why don't we practice and explore this some more by role-playing how the conversation might go, and anticipate the trouble spots that are likely to come up. I could role-play a more assertive but still respectful Anna, and you could be your mother, you know, and say the things she will probably say if you decide to talk to her about this some time. And we can both watch out for the guilt, and the sadness, as we go along...

## TRACKING CLIENTS' ANXIETY

Another way to help clients turn inward is to develop the skill of being able to *track the client's anxiety* (Sullivan, 1970). Although the subjective experience of anxiety is distressing and may even be painful, we want to show that it can still be the therapist's ally. How so? **Anxiety serves as a signpost that points to the heart of the problem—a signal that the real threat or danger is at hand.** Tracking precisely what makes a particular



client anxious will help therapists identify the client's key concerns and provides a point of entry for the therapist (Wolfe, 2005). Therapists do not fully understand clients' problems—or what needs to occur in counseling to resolve them—until they discern what makes a client anxious. Why do certain situations or interactions make a particular client anxious, and how has the client learned to cope with this threat or insecurity? Therapists are encouraged to begin formulating working hypotheses to this question in order to clarify a treatment focus (Levenson & Strupp, 2007). As we will see, focusing clients inward on their anxieties—exploring together what activates or triggers this unwanted feeling—will lead the therapist and the client to the client's central concerns.

Therapists can use a four-step sequence to track clients' anxiety and better understand what's really wrong. In this sequence, the therapist:

1. Identifies manifest and covert signs of the client's anxiety;
2. approaches signs of client anxiety;
3. notes the topic presently under discussion and considers the interpersonal process currently transpiring that may have precipitated the client's anxiety; and
4. focuses the client inward to explore the meaning of the client's anxiety.

Let's look at these four steps more closely.

### STEP 1. IDENTIFY SIGNS OF CLIENT'S ANXIETY

Clients become anxious in the session when the issues they are discussing hold real meaning and touch on their key concerns. Clients develop certain interpersonal coping strategies, which they employ over and over again in different situations, to ward off or diminish this anxiety (e.g., pleasing, controlling, withdrawing, etc.). **The therapist is trying to identify what makes this particular client anxious** (e.g., needing to ask for help and expecting the other to attach “strings” to their response; not knowing how to fix or do something and expecting the other to be impatient, critical, or disdainful, etc.). When therapists can do this, they are better prepared to recognize the unifying themes or repetitive patterns that have triggered the client's anxiety (Levenson, Butler, Powers & Beitman, 2002). Thus, throughout treatment, therapists are continually asking themselves, and inviting the client to join them in identifying, **“What makes this client anxious? When does the client feel threatened or insecure? Where do safety and danger lie?”**

To identify the patterns and themes that are causing problems in clients' lives, the therapist tracks their anxiety and remains alert for signs indicating that something they are experiencing is making them anxious right now. **Clients may express their anxiety in a thousand different ways**—through nervous laughter, for example, or through nail biting, hand gesticulation, agitated movement, stuttering, hair pulling, speech blockage, and so on. These “anxiety equivalents” will be expressed in endlessly varied ways across clients, and therapists and clients together can learn how each particular client tends to express anxiety. This insight will be especially important when working with clients from cultures in which certain behaviors—such as gaze aversion—may be normative rather than a signal of anxiety (Ridley & Kelly, 2007).

When most clients become anxious, they employ the same interpersonal coping strategies in a repetitive or characteristic way Johnson, Popp, Schacht et al. (1989). As noted above, these strategies routinely include becoming critical or controlling, acting helpless or confused, withdrawing from people or avoiding certain situations, and so forth. We will explore these strategies closely in Chapter 7. For present purposes, it is enough to note that the first step in tracking the clients' anxiety is to observe and note when they become anxious (e.g., when they have to assume a leadership role or follow someone's lead, when they succeed or make a mistake, when they have to establish boundaries with

someone, when they have to disagree with someone or advocate on their own behalf, when they become intimate and begin to trust someone, when they feel distressed or need to ask for help, etc.).

## STEP 2. APPROACH CLIENT'S ANXIETY DIRECTLY

In working with resistance in Chapter 3, we encouraged therapists to ask about and work with the feelings and concerns that clients may have around asking for help, having a problem, or entering treatment. Our key question was, “What is the threat or danger if...?” Further along in treatment, therapists similarly are encouraged to approach clients’ anxiety about an issue they are discussing or about what may be occurring with the therapist at that moment. Following Greenberg (2004), therapists can focus clients on their immediate experience in an open-ended way (e.g., by asking, “What are you feeling, right now, as you talk with me about this?”). Therapists can also hold the steady intention to be looking for opportunities to better discern and name the anxiety more specifically. For example:

**THERAPIST:** Something seems to be making you uncomfortable right now. Any ideas about what that may be?

**CLIENT:** Not really—I am restless but I’m not sure. Why...?

**THERAPIST:** Let’s do that sentence completion exercise again. What’s the first thing that comes to mind when you ask yourself, “Right now I’m afraid that \_\_\_\_\_.”

**CLIENT:** ...You’re mad. I think I’m afraid that you’re mad or disappointed with me somehow.

By focusing clients inward on their anxiety *as they are experiencing it*, and helping them explore it further and discern it more precisely, the therapist is following the clients’ lead and helping them get closer to the source of their problems. For example, Greenberg (2010) writes, in an illuminating way, about the importance of attending to emotions, and shows how the therapist and client together can use this to facilitate change. Although some clients may be ambivalent about the therapist’s request to look within and explore their anxiety further, many clients will welcome this opportunity to explore their anxiety with the therapist—and the sadness or loss, shame or, or pain that usually follow close behind. On the other hand, clients may also recognize that approaching their anxiety will bring them closer to their **internal working models (IWMs)**—**faulty beliefs about themselves and unwanted expectations of others that evoke further anxiety** (Pietromonaco & Feldman, 2000).

Although clients often experience the therapist’s focus on their anxiety as helpful, at other times it may intensify anxiety and even arouse resistance. As discussed in the previous chapter, therapists need to respond to the client’s resistance when it emerges by exploring what about this anxiety-arousing topic is threatening or does not feel safe to approach. This process will be far more effective than ignoring the client’s reluctance as if nothing significant is occurring, or “pressing” and trying to persuade the reluctant client to talk further about the difficult issue. This respectful, non-demanding, and collaborative approach honors the client’s autonomy, fosters motivation and empowerment, and contributes to positive outcomes (Csillik, 2015; Guame et al., 2008). Before the therapist and client address the specific issue, they first can explore the resistance and together learn what doesn’t feel safe and engenders anxiety. For example:

**THERAPIST:** OK, let’s pause and stop talking about this. Instead, help me understand what’s the threat or danger for you if you did talk to me about that? You know, what’s going to go wrong between us—or how are you going to end up getting hurt again—if you kept talking to me about it?

The therapist's intention here is to respond in a way that honors the client's resistance. Recall from Chapter 3 that the therapist does this by:

- (1) trying to understand the original, aversive experiences that led the client to behave in this particular way;
- (2) helping the client appreciate how this "resistant" or "defensive" response was once, at a different time and with different people, a necessary and adaptive coping strategy; and
- (3) disconfirming the schema or IWM by providing the client with a different and more reparative response (CEE) than the client has come to expect, as in the following example:

**THERAPIST:** No, I don't find you "needy" or "demanding" for asking me if I would call you and touch base for a minute before you make this big presentation on Tuesday. It would be easy for me to take a few minutes to do that, and I'm pleased that you are willing to risk asking for this.

### STEP 3. OBSERVE WHAT PRECIPITATES CLIENTS' ANXIETY

As we have seen, the therapist first observes when the client is anxious, and then helps the client focus inward to explore and try to understand what the client is experiencing. **At the same time, the therapist has a third task: to try to recognize what issue was just being discussed (content) or what type of interaction between the therapist and the client may have precipitated the client's anxiety (process).** If the therapist can identify what it was that made the client anxious, the therapist will be better able to identify the faulty belief, interpersonal scenario, or difficult feeling that may have generated the anxiety. When the therapist and client can collaboratively discern the precipitating or triggering event, it will help them understand what's really wrong and needs to change. Thus, approaching anxiety is a point of entry for engaging the client more meaningfully.

Generally, the client will be able to discuss comfortably a range of different issues with the therapist. However, when the client becomes anxious, the therapist's aim is to try to identify the issue that has just precipitated the anxiety. Which topic was the client just talking about? The answer will highlight the concerns that are most central to the client's problems. As we know, therapists can help the client explore this more effectively if they have already formulated working hypotheses about the possible issues that may be generating the anxiety. Keeping process notes after each session, as suggested in Appendix A, will help therapists formulate their working hypotheses more effectively.

In addition, the client's anxiety may also be a signal that the interpersonal process between the therapist and client is reenacting a developmental problem or cyclical relational pattern for the client. For example, the therapist might observe client anxiety in the following circumstances:

- *As the therapist was expressing uncertainty about what was going on in the session at that point, the client became anxious, perhaps because her alcoholic father would start demeaning her whenever he felt uncertain, confused, or felt insecure or unsure of himself.*
- *The client had just made a significant insight and the therapist applauded his achievement. The client then became anxious, perhaps believing that now he would always have to be so insightful and perform so well, as an aggrandizing and bragging parent had expected of him.*
- *The therapist had just been supportive, and perhaps the client felt anxious because "strings" had always been attached to what she had been given in the past.*
- *Following her IWMs, the client may have become concerned that the therapist would be irritated and withdraw from her following her request to have her appointment changed to a more convenient time, just as a caregiver used to do when she asked for something.*

The therapist can generate working hypotheses such as these about how the current therapist-client interaction may have triggered a developmental problem or unwittingly reenacted a maladaptive relational pattern for the client. These hypotheses can then be used to help therapists and clients explore together what evoked the client's anxiety, understand what this threat really did mean and with whom, and find more effective ways to respond when similar issues emerge in the future.

#### STEP 4. FOCUSING CLIENTS INWARD TO EXPLORE THEIR ANXIETY

As we have seen, the therapist is alert for signs of client anxiety and approaches it whenever it seems to be occurring. Simultaneously, the therapist is trying to identify what precipitated the client's anxiety—especially what has just transpired between them—and begins to generate working hypotheses about the relational patterns, faulty beliefs, or difficult feelings that may have triggered the anxiety. Finally, the therapist also focuses the client inward—to explore more specifically what the threat or danger seems to be for the client right now. If the therapist can help clarify the thoughts and feelings that are associated with the anxiety, the client's vague discomfort will usually become more specific. That is, once the client's basic concern is highlighted, the client can address the anxiety-arousing problem more directly. (Client: "I guess I'm afraid they won't like it if I speak up, or say 'no'—but maybe that's a consequence I can live with.") The following dialogue illustrates this process:

**THERAPIST:** It seems like something's going on for you right now. I wonder what's happening?

**CLIENT:** You know, this doesn't sound very nice to say, but I don't think my mother really wants me to change very much. I don't think she's a mean person or anything, but I do think I'm getting better in therapy, and she's not completely happy about that.

**THERAPIST:** Uh huh. How so?

**CLIENT:** Well, I haven't been depressed for a while now, and I've actually been feeling pretty good the last month or so. Maybe it's just coincidence, but it seems that as I've gotten better and started spending time with friends again, my mother has withdrawn and been harder to talk to. Like she's always got something else to do, or is thinking about something else, when I try to talk with her. And I think this has gone on between us before.

**THERAPIST:** Yeah, you know, I think that's a very good hypothesis. You have been doing good work in here, and getting better, and it does seem that your increasing confidence and independence have been kind of hard for your mother sometimes.

**CLIENT:** (fidgets, begins picking at her nail, and then starts talking about another topic)

**THERAPIST:** Maybe something just made you feel uncomfortable?

**CLIENT:** I don't know. How much time is left?

**THERAPIST:** I'm wondering if something we're talking about, or something that might be going on between us, could be making you uncomfortable right now?

**CLIENT:** (*pauses*) Well, I don't really know why I'm saying this, but maybe I'm afraid that you're going to go away or something?

**THERAPIST:** Well, that actually makes sense to me. We were just talking about how your mother seems to act hurt and withdraws when you are feeling stronger, and I just told you that you were doing good work in here.

**CLIENT:** Well, I guess so, but that just makes things worse. You're not going to stay with me either if I get better—isn't that the point of this whole therapy thing anyway?

**THERAPIST:** That sounded important, but I didn't understand it as well as I wanted to. Can you say that again or say it differently?

**CLIENT:** Well, if I get better, then we stop—just like with my mother. Then I won't have my mother, or you, or anybody. Don't you see? It's hopeless.

**THERAPIST:** You're saying this so clearly. I didn't understand at first, but you're right—the issue we are struggling with right now is like what's gone wrong before. To feel better and act stronger has meant that you have had to be alone. In the past, you have had to be sad and depressed and needy in order to be close or connected to others. I think you've been struggling with this dilemma most of your life, and, right now, it feels like it has to be the same old unwanted story again for you and me.

**CLIENT:** Yeah, it kinda does.

**THERAPIST:** But I wonder if you and I can do this differently for once. Can we work together and try to find a way to make it come out better this time?

**CLIENT:** I don't know, it's sure not working very well so far.

**THERAPIST:** OK, I hear you. But one difference I'm thinking about is that we're talking about it—naming it and sharing it—and that hasn't happened before.

**CLIENT:** What difference does that make?

**THERAPIST:** Maybe it's different because I can see what's happening for you and get how discouraging it's been for you to have your autonomy and success undermined in this way so many times. And right now, I'm still feeling connected to you—still “for” you—as you are changing this and acting stronger in here with me.

**CLIENT:** Well, that's probably true, but these don't ly seem like huge differences to me.

**THERAPIST:** OK. I see these differences as more significant than you do right now, but let's keep talking about this.

**CLIENT:** Why? What's the point?

**THERAPIST:** If you can stop holding yourself back in here with me and find that I, and maybe some other people in your life too, don't go away like your mother does but stay connected to you when you succeed, it might help you act stronger with others in your life. Like your boyfriend, and your professor, and others we've been talking about.

**CLIENT:** You really think that could happen?

**THERAPIST:** Yes, I do. How do you think your boyfriend will respond to you if you let yourself be as intelligent and capable with him as you are in here with me? Would he be threatened, like your mother has been and some others in your life, or would he enjoy that stronger part of you, as I do and some others will?

In this way, tracking the client's anxiety can help the therapist and the client focus inward and clarify the client's key concerns. In particular, *it will often reveal how aspects of the same core problem that the client is having with others has been activated in the therapeutic relationship*. This will allow the therapist to intervene in two important ways. First, therapists can use a process comment to change their interaction with the client and ensure that the therapeutic relationship does not continue to repeat the same problematic patterns that have occurred with others. Second, as we have just seen, they can link the client's positive new behavior with the therapist to others in the client's life. In the chapters ahead, we will explore further how clients can begin trying out new ways of relating with the therapist and then generalize this to other people in their lives.



## CLOSING

We believe therapists from every theoretical orientation can help their clients change through experiential relearning and enacting solutions to their problems in their real-life relationship with the therapist (Friedlander, Sutherland, Sanders et al., 2012; Goldfried, 2012; Levenson, 2010). We find that helping clients adopt an internal focus for change is one important way to move beyond surface conversation to their most important problems. By codetermining what issues will be pursued in treatment, and by actively participating in finding solutions to their problems, clients take more ownership of the treatment process. When this interpersonal process sustains, we find the working alliance is strengthened, and clients are more likely to change and gain an increasing sense of self-efficacy in the process. Perhaps Bowlby (1988) has captured this most succinctly by saying that the basic therapeutic stance toward the client is not “I know; I’ll tell you” but “You know; you tell me.”

It is important to remember that we are not pressing clients for further disclosure. In supervision, as we watch our trainees’ videotapes, we routinely find that the client is presenting significant material and the trainee doesn’t hear the feeling or grasp the greater meaning in what the client has just said, or doesn’t meet the client at that significant point but moves to surface issues. In this chapter and throughout this text, we are trying to highlight this as a significant topic for clinical training. Instructors need to work with trainees to help them develop the capacity to hear and respond to the key message in what the client just said. *The intention is to give clients the feeling that they have been fully seen and fully met.*

As clients begin to reflect on their own reactions and become less focused on the problematic behavior of others, their entire emotional world opens up and becomes more accessible. This affective unfolding is a pivotal step in the change process. However, the strong feelings commonly evoked by pursuing an internal focus can be difficult for some clients—and often present a challenge for new therapists. The next chapter explores how therapists can help clients with the significant feelings evoked by looking within.

## CHAPTER EXERCISES

What does it evoke in you to try to help clients move beyond everyday, surface conversation to their core concerns and feelings?

What are the familial/cultural rules that influenced or may have restricted your ability to enter more emotionally charged topics?

## SUGGESTED READING

Hill, C. (2014). *Helping skills, (4/E)*, Washington, D.C.: American Psychological Association. See chapters 8–10.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes. Also included with this chapter are video demonstrations by the authors. In the first vignette they contrast an effective and ineffective response to an externalizing client. In the second vignette they demonstrate several different points of entry that trainees can use as guides for entering more significant issues with their clients.



## CHAPTER 5

# Helping Clients with Their Feelings

*Jennie, a first-year practicum student, was off to a good start with her client, Sue. However, at their third session, Sue entered in true crisis. “I have breast cancer!” she cried. “My doctor says I have breast cancer!” With undisguised anguish, she went right to her biggest fear: “My girls may not have a mother to help them grow up!” Taken aback by the intensity of this raw emotion, Jennie didn’t know what to do at first. Thinking of what she could say, Jennie jumped in and began doing what she had always done best: getting the facts, making plans, and solving problems: “Have you got a second opinion? Have they talked to you about your treatment options yet? They can do so much more now than they used to; chemo has become so much more effective....”*

*The next day, in supervision Jennie’s supervisor tried to help Jennie see that she had moved right into a problem-solving and reassuring mode without responding directly to Sue’s profound fear. The supervisor commented, “Such big feelings, Jennie, such vulnerability. It sounds like you wanted to help Sue figure out what to do. Instead, what do you think would have happened if you had just tried to stay with her feelings longer?” Jennie was rather quiet, and vaguely suggested that she wasn’t sure how to do that. Her supervisor tried to be more specific: “I’m thinking of several ways you might be able to do that in the future—like expressing more directly your own compassion or concern for her. Or perhaps helping her clarify or name her feelings that seem so overwhelming, or maybe just affirming how frightening all of this is right now? I think the sequence is important here. If you respond to her feelings first with this type of understanding or empathy, I think that then she could better use your suggestions about what to do.”*

*As their discussion unfolded, however, Jennie disagreed and said, “The client wasn’t ready to go that deep yet—we haven’t been working together long enough to push her into feelings like that.” Jennie looked puzzled as the supervisor replied, “But you weren’t ‘pushing’ her to ‘go deeper’ than she wanted. Sue initiated this—she brought these feelings to you.”*

*It was too new and too much for Jennie to absorb now, but, by the end of her practicum year, Jennie had become better at letting her clients feel what they were feeling.*

## CONCEPTUAL OVERVIEW

Painful feelings lie at the heart of enduring problems, and therapists help clients change when they respond effectively to them. When the therapist focuses clients inward on their own experience, clients will begin to feel more intensely, and express more directly, the difficult feelings that accompany their problems. This affective unfolding highlights or reveals the conflicted feelings that are central to the clients' problems—clarifying what's really wrong. These important feelings, which have not been responded to well in the past, are now accessible for the therapist to respond to and work with. On the one hand, clients will welcome the promise of having therapists make contact with them on this deeper, more personal level. But on the other hand, clients still fear and may want to avoid feelings that have been unacceptable in other relationships—or that just seem too painful, shameful, or hopelessly unsolvable. Thus the therapist's response to the challenging feelings that clients bring will have a significant impact on the outcome of treatment (Elliott, Watson, Goldman & Greenberg, 2004).

Therapists facilitate change by providing a more helpful response to clients' feelings than they have come to expect from others. That is, the therapist validates these, takes them seriously, and does not minimize them or offer superficial reassurances (Linehan et al., 2002). When therapists avoid or do not respond to clients' feelings, the therapeutic relationship loses its vitality and meaning—treatment is reduced to an intellectual pursuit (Greenberg, 2010). The purpose of this chapter is to help therapists learn to respond effectively when clients present important feelings.

Let's begin this chapter with a note of encouragement and an eye toward realistic expectations for new therapists. The material presented in this chapter is more personally evocative and challenging for many therapists than the material presented in other chapters. Initially, these principles are often difficult to apply with clients. Understanding these processes intellectually is a realistic goal for first-year therapists; being able to utilize these interventions effectively in ambiguous therapeutic interactions is not. Be patient: In a year or two, you will be able to integrate these suggestions and make many of these interventions your own. Like so many things, learning this comes with practice.

## RESPONDING TO CLIENTS' FEELINGS

### WORK WITH CLIENTS ABOUT ADDRESSING THEIR AMBIVALENCE

Many of the problems that clients present can be resolved by trying out new coping strategies or behavioral alternatives, such as questioning faulty beliefs or reframing problems in a new perspective. These brief interventions are sufficient to help with many of the difficulties clients present, and they are an important aspect of change for most problems. However, for the more enduring and pervasive problems that most clients present, the conflicted emotions that accompany their difficulties need to be addressed as well. Psychotherapy process studies have demonstrated that positive outcomes are associated with therapist interventions that facilitate clients' emotional experiencing and encourage verbalization of affect (Diener et al., 2007; Greenberg, 2014). Clients resolve their problems better when they are in touch with their feelings (Elliott et al., 2004).

At times, clients may be reluctant or ambivalent about exploring more fully the difficult feelings that emerge in treatment. Although clients hope that the therapist will be able to help with their emotional reactions, they may also want to avoid the difficult

feelings that they have not been able to resolve on their own. This reluctance emerges in the beginning for many different reasons. To clients, it may seem pointless to:

- *Share the same painful feelings again when—based on what has happened before—they have scant hope of receiving support;*
- *Suffer shame by revealing secrets, failure experiences, or feelings of inadequacy;*
- *Risk unwanted but expected judgment from the therapist for having feelings that have always seemed unacceptable;*
- *Struggle with their own fear of “losing control.”*

As a result of these and other concerns, clients sometimes resist the therapist’s attempts to reach the emotional core of their experience. At the same time, however, they often long for this empathic understanding. The hope is that:

- *Someone may, at last, truly listen to them with deep understanding or compassion rather than criticism or blame;*
- *Their experiences might, in fact, be validated;*
- *They no longer have to be alone with the shame, anxiety, anger, guilt, or other form of distress;*
- *They can begin the process of connecting meaningfully with another person and have a relationship characterized by trust.*

Once the therapist recognizes the client’s relational schemas and understands how significant others have characteristically responded to the client’s vulnerability in the past, the fear or shame that motivates the client’s ambivalence will make sense. As outlined in Appendix B, therapists can generate working hypotheses about the client’s problems, their genesis, and what the treatment focus should be—and try to anticipate the client’s potential response to conflicted feelings and attempt to work with those feelings rather than avoid them. Otherwise, treatment will not be sufficiently potent to propel meaningful and enduring change. Affective arousal is needed in order to reorganize meaning schemas, expand subjective worldviews, and find meaning (Goldfried, 2006; Lieberman, 2007; Schore 2012). In sum, the therapist can appreciate the client’s reluctance to approach certain feelings yet also keep in mind the client’s simultaneous wish to be responded to in this deeply personal way.

Although difficult emotions are central to most clients’ problems, the therapist’s affirming response to these feelings also provides the avenue to resolution and change. The Corrective Emotional Experience (CEE) that the therapist provides to clients—the experience of sharing important feelings with another who remains connected and validating—loosens the hold of faulty beliefs and expectations. As rigid schemas become more flexible and expand, clients are empowered to make the changes they desire. At this critical juncture, when clients experience a reparative or corrective response to such significant feelings, the treatment process springs forward. This is the point when we see clients typically become more receptive to a wide range of interventions from varying theoretical orientations (Friedlander et al., 2012; Gold, 2006; Renn, 2012).

## CLIENTS AVOID FEELINGS BECAUSE OF UNWANTED INTERPERSONAL CONSEQUENCES

In Chapter 3, we saw that both therapists and clients may be reluctant to address the client’s resistance. As we have just seen, however, treatment doesn’t progress far—it soon stalls and loses meaning—when strong feelings are not dealt with. As with resistance to treatment, the therapist does not simply want to push through these defenses in order to reach clients’ sadness, anger, or other significant feelings. We don’t want to press clients

to feel or disclose anything they don't want to share. So what do we do instead—just wait and try to be patient? No, as before, *therapists can collaborate with the client in trying to understand the threat or danger if they shared a certain feeling*, rather than press for it. Let's see how therapists can take this more effective approach.

The common notion is that clients avoid sharing or experiencing feelings because “it hurts too much.” We suggest something entirely different: We suggest that the defenses are against *expected* but unwanted *interpersonal consequences*. For example,

**CLIENT:** You wouldn't respect me if you really knew how afraid I am to leave him, and how much I put up with just so I don't have to be alone. It would be humiliating for you to see what a doormat I really am.

This contrasts to the more traditional notion of intrapsychic dynamics:

**CLIENT:** It hurts too much; I can't stand it.

We propose that *clients often minimize or avoid difficult feelings because they expect to receive familiar but unwanted responses from the therapist (e.g., disapproval, disappointment) and others in their lives that evoke fear, shame, or guilt*. Therapists create a more affirming interpersonal process, with greater safety for clients, if they enlist clients' collaboration in trying to understand the very valid or realistic reasons why it has been threatening to experience or share certain feelings (Geller & Greenberg, 2012). For example, rather than pressing the client to disclose a difficult feeling, the therapist may use responses such as these to explore the relational threat instead:

- *OK, let's talk about what might go wrong for you, or between us, if we talked about your feelings.*
- *If you did cry or let me see your vulnerability, what might I be thinking about you?*
- *I'm wondering if something has happened with others that you don't want to happen again here with me?*
- *What is the danger for you if you let me see your sadness? Help me understand how you have been hurt in other relationships—what have others said or done?*
- *Growing up, how did your parents respond to you when you were feeling \_\_\_\_\_? What did each of them typically say and do when you were feeling that way? Can you recall the look on their faces, or talk about what they might have been thinking or feeling toward you as you were feeling \_\_\_\_\_?*

*In these ways, the therapist does not push for the content—the specific feeling—but works with the client to clarify the threat or danger—the reality-based reasons for why it has not been safe to experience or share the feeling.* As in Chapter 3, the therapist is honoring the client's resistance by collaborating with the client in trying to understand why this reluctance once made sense. *However, the therapist can make it clear that he or she (and later, that some others in the client's life) will not respond in these familiar but unwanted ways.* With this new understanding, it will soon be safe enough for the client's threatening or unacceptable feelings to emerge.

To emphasize, there are often interpersonal reasons why clients want to avoid difficult feelings. Although they may use vague terms like it's too painful or overwhelming, they expect to be responded to in familiar but unwanted ways. Such interpersonal threats are usually far more important than their own subjective discomfort. Although it is difficult for most beginning therapists to grasp this, *clients also hold these same expectations toward the therapist—even though the therapist has never responded in any of these problematic ways.* The key issue is for therapists to be empathic to clients' feelings and expectations, and help clients clarify the valid reasons they learned that certain feelings were not safe to experience or share (Moyers & Miller, 2013; Tsai, Yard & Kohlenberg, 2014).



Therapists can also help the client differentiate “then” from “now” and clarify how they would like to respond differently now in their current therapist–client relationship than others have in the past. Although it may seem to the therapist that these distinctions are obvious to the client, they are not; the client will need to have these differences highlighted. For example:

**THERAPIST:** I’m wondering if it has ever seemed like I was responding to you in that kind of judgmental way—you know, like your husband does sometimes, and your father did, at times?

OR

**THERAPIST:** How do you think I am going to respond if you risked sharing that feeling and letting me see that part of you?

OR

**THERAPIST:** Tell me two things. If you chose to share that with me, how would you hope I would respond? What response from me would you be most afraid of receiving?

Many clients avoid or minimize their own painful feelings in order to protect their caregivers from seeing the hurtful impact they are having or have had on them. However, this loyalty to the caregiver is carried out at clients’ own expense. As adults, these clients are still *complying* with familial rules and colluding with their caregivers in denying the impact of hurtful parental actions. These clients are preserving insecure attachment ties (“Everything’s fine—no problem”) at the cost of disavowing their own fear or deprivation. When clients lose the validity of their own experience in this way, they also lose their own voice. This protects the (internalized) caregivers from seeing what they did that hurt the client (e.g., embarrassing or ridiculing the child, repeatedly ignoring him or her or favoring a sibling, needing the child to be “perfect” or fulfill the family role of “hero,” hitting or exposing the child to domestic violence, etc.). If clients participate in this denial and dismiss the problems that actually were occurring, homeostatic family rules are maintained. Clients remain attached to internalized caregivers who often remain idealized or “good” while, in this splitting defense, the clients remain “bad” and feel that something is wrong with them. However, if painful feelings are permitted expression in therapy, and *the therapist can affirm the clients’ experience without blaming or rejecting the attachment figure*, chronic symptoms such as lifelong dysthymia or generalized anxiety often improve. For example:

**THERAPIST:** Yes, it makes sense to me that you feel anxious and worried “all the time.” It must have been frightening to hear your parents fight like that—threatening and demeaning each other. Your mom might get really hurt—your dad might go away, and you couldn’t stop them or fix this big problem. And you and your sister listening to them at night through your bedroom door like that...it’s such a heartbreaking spot for a little girl to be in. So, no, I don’t think you’re making a “big deal out of nothing.” This was hard for you, and probably for your parents as well.

The therapist’s intention is not just to help clients *get their feelings out*. This does not help them connect their feelings to the specific person with whom they are sad, mad, or hurt, and just what it was about that interaction that evoked their feelings. Venting in this general way does not help most clients make progress on their problems (Lohr et al., 2007). Too often, instead, it serves only to divert clients away from the real feelings or concerns they are struggling with. In attachment terms this is called a “miscue.” A more useful approach can be for the therapist to help clients identify the *problematic*

*interaction sequences* that repeatedly occurred and gave rise to their difficulties with experiencing or sharing certain feelings. For example, the client feels upset about something and tells her mother, who exaggerates the issue and then subtly shifts the focus to her own distress; the client, feeling confused and alone, finds herself walking toward the refrigerator to eat ice cream. By clarifying and naming what actually did occur over and over in other important relationships, the therapist provides the interpersonal safety that clients need to enter and experience their feelings more fully (Geller & Greenberg, 2012). With the safety that comes from this empathic understanding, therapists can also help clients identify the problematic relationships in their current lives where these same unwanted scenarios are recurring, and discern other relationships where better responses are occurring or could be developed.

### APPROACH THE CLIENT'S MOST SALIENT FEELING

With each turn of the conversation, therapists are challenged to select how they want to respond to what the client has just said. For example, the therapist might respond by seeking more information, clarifying what the client has said, making connections between this issue and other material the client has presented, or linking it to what is occurring between the therapist and client at that moment. As a general principle, however, *the most productive intervention is to approach the most salient feeling in the material that the client is talking about right now*. In other words, the therapist's initial focus or priority is to acknowledge the affective component—the primary feeling or emotion—in the client's response (Elliott et al., 2004).

To illustrate, imagine that the therapist and client have just sat down to begin their first session:

**THERAPIST:** Tell me, Mike, what's the difficulty that brings you to therapy?

**CLIENT:** I'm having a lot of problems with my 15-year-old son. We disagree about everything and can't seem to talk to each other anymore. He doesn't do what I ask him to, and I don't like his values or his attitude. I guess I'm pretty ANGRY (*salient feeling*) with him. His mother and I are divorced and have been sharing custody, but I'm thinking it may be time for him to go live with her. Do you think it's OK for a teenage boy to live with his mother?

**THERAPIST:** I don't think either of us understands what's going on between you two well enough to make any decisions yet, but you said you were "pretty angry" with him. Tell me more about your anger.

The client quickly has presented many different issues that the therapist could have chosen to pursue. Following the general guideline, however, the therapist approached the primary affect or feeling that the client presented. The therapist could have responded differently and inquired further about the issues that the father and son disagreed about, worked on values clarification or communication skills, obtained more background information about the father-son relationship or the divorce, provided research findings about the effects of mother custody versus father custody on boys, and so forth. These and many other responses will often work well for some clients. However, *responding first to the central or most salient feeling that the client expresses will usually produce the most meaningful information and intensify the interaction* (Whelton, 2004).

In this dialogue, the therapist responded when the client spoke directly about his anger. Clients often communicate their salient feelings through nonverbal cues and therapists also need to respond to the covert or unverbally expressed feelings that clients experience sometimes (Richmond, McCroskey & Hickson, 2012; Schore, 2012). For instance, the

therapist in our example might have chosen to respond instead to a salient but more covert affect. The therapist could have done this by tentatively wondering aloud, “*As I listen, it sounds like you’re feeling really discouraged about being a dad right now—maybe so discouraged that you just feel like giving up sometimes? Am I saying this accurately?*”

When clients begin talking about what matters most to them, important feelings usually come up. Such nonverbal affective signs as tearing, sighing, grimacing, or blushing are signals to the therapist that the client has entered a significant topic that holds real meaning. Responding to these nonverbal cues, the therapist invites the client to share the feeling or deeper meaning that this situation holds. For example, imagine that the client has been discussing her marital problems:

**CLIENT:** (*grimacing*) I don’t know if I should stay married or not. I haven’t been happy with him for a long time, but I can see how hard he’s trying to make our relationship work. And our four-year-old son would be devastated if we broke up. I don’t know what to do, and I have to make the right decision.

**THERAPIST:** As you speak about this, your face tightens. (*nonverbal communication*). I’m wondering what you might be feeling right now as you tell me about this?

When the therapist responds to the client’s feelings in this *open-ended* way that invites exploration, it often serves to clarify the client’s key concern. Often, clients will respond by expressing more specific concerns. Continuing with the previous dialogue, the client may respond:

- *I’m so sad about hurting the people I love.*
- *I’m afraid that everyone will think I’m selfish for leaving, and they’ll blame me for the divorce. They’ll think I haven’t been a good mother.*
- *I don’t want to be alone. I don’t want to be married to him anymore, but I’m afraid of leaving and trying to make it on my own.*
- *I’m furious that I’m the one who has to make this decision. I’m responsible for every decision we make, and I always pay for it in the end. It’s not fair!*

Throughout each session, clients have emotional reactions to the issues and concerns that trouble them most. Sometimes the client’s affect will be presented forthrightly; at other times, it will be subtle or more covert. Cultural, class, and gender factors will further shape how each client displays affect (Pedersen, Crethar & Carlson, 2008; Sue & Sue, 2016). For example, some clients may overtly state their reactions, whereas those from other cultural or familial backgrounds may simply divert their eyes or become noticeably quieter when certain feelings are evoked. Additionally, when clients from some cultures are angry and want to disagree, they feel anxious, become quiet, and shut down because it would be disrespectful to express their disagreement. The therapist’s abiding intention, however, is to *keep giving invitations or open-ended bids to explore the feelings that seem most important to the client right now*. As the therapist joins the client in sharing and understanding the central feelings that hold the most meaning, it will:

- *facilitate further disclosure from the client,*
- *highlight key concerns, and*
- *provide a clearer direction or focus for treatment.*

The next example illustrates the prototypic sequence we go through with clients. It shows how the therapist draws out clients’ feelings and validates them, which leads clients to go below the surface and begin clarifying their core concerns, which in turn provides a sharper focus for treatment.

Ella is a 20-year-old client who is talking about her on-and-off again boyfriend and two-year-old son's father, Edward.

**CLIENT:** I'm just like my mother...

**THERAPIST:** How so, Ella? You have tears in your eyes as you say that.

**CLIENT:** I choose men who hurt me, dis me, and I can't seem to leave them.

**THERAPIST:** You see this pattern yet something keeps you going back...

**CLIENT:** Yup, that's me, I just don't know what's wrong with me.

**THERAPIST:** Let's just be with this and see if we can figure it out together. Maybe if we explore how the sequence works we might understand what needs are being met.

**CLIENT:** OK.

**THERAPIST:** Tell me about the last time you and Edward had a problem. What happened? How did you feel after it happened?

**CLIENT:** It was my son's birthday and he didn't even call to say "happy birthday" to my son, our son. So I called him the next day to ask why he didn't call.

**THERAPIST:** How was it for you on your son's birthday when you realized that he wasn't going to call?

**CLIENT:** Sad, alone, no support.

**THERAPIST:** Alone, very alone.

**CLIENT:** (*tears*) Yes, I just feel so alone, I do everything alone, I am alone.

**THERAPIST:** You have been alone a lot. I am so glad that you are letting me in on your experience right now so you don't have to be alone with your sadness.

**CLIENT:** I think I keep taking him back when he "sweet-talks" me because I'm so afraid of being alone. Yet I know he is not right for me. He is so much like my dad. I don't want to end up like my mother, all doped up to deal with the abuse from him.

**THERAPIST:** You are getting help now and that takes a lot of courage. These feelings of being afraid to be alone make sense. You have had to take care of yourself for so much of your life and have had to do so much on your own.

This led Ella to talk about her history of multiple foster-care placements during the times when her mother was incapacitated by her drug use, which helped Ella recognize how her fears of being alone are impacting her willingness to tolerate Edward's behavior. In this way, Ella was able to address her needs and how these affect other relationships.

To emphasize, the therapist wants to find *collaborative* ways to invite and explore the client's affect. We don't want to "push" for feelings, of course, or become invested in the client having sad, angry, or any other feelings during the session. However, for most therapists in training, the problem usually goes the other way. That is, the client introduces a feeling and the therapist does not "hear" it or, even if the therapist does register it, the therapist doesn't approach or respond to it.

All of this may sound easy enough to do, but it is not. Researchers find that even experienced therapists often do not approach sensitive feelings and concerns very effectively (Hill & O'Brien, 1999). Recalling the discussion of "rupture and repair" in Chapter 2, we have already seen that clients routinely hide or withhold their negative feelings and reactions toward the therapist (Regan & Hill, 1992), and that many clients

leave one or more important things unsaid during sessions (Hill et al., 1993). Similarly, in another study, about one-half of the clients sampled had major life secrets that they had not told their therapists, even in long-term therapy (Kelly, 1998). They withheld these secrets for two reasons. First, they reported feeling embarrassed and ashamed. Second, these clients also believed that either they themselves would be “overwhelmed” by their emotions or clients worried that their therapists “would not understand” or could not handle the disclosure!

Further into therapy, Ella tells the therapist:

**CLIENT:** I have been afraid to tell you about Edward and me having unprotected sex because I was afraid you’d think, “How stupid is she.” But I really need help talking to him. I know, after all the ways he’s treated me, how could I take him back, and even have sex with him? And you know, I didn’t even ask him if he is having sex with other women. I’m afraid if I ask him if he is sleeping with other women, he might get mad and, you know, what if he decides to leave me?

**THERAPIST:** Being alone is really frightening for you...I can see that. You are also worried about safe sex—that’s a very reasonable concern, Ella. It sounds as though you’ve also been afraid of being judged by me if you talk about how you have managed your relationship with Edward.

**CLIENT:** Yeah. I’ve let him walk all over me. And I never used to worry about safe sex before but now—since I’ve been talking with you, I want more for myself, and I do worry about my son, too. But, I really don’t want Edward to leave me; I don’t want to be alone. I’m ashamed to admit that I’m risking getting HIV because I’m afraid to be alone. But when I think about it, that I could leave my child with a sick mother—that would make me just like my mother! I would end up sick and unable to be a mom and maybe abandon my child because of sickness.

**THERAPIST:** You’ve just said a lot here, Ella. You are afraid of being alone. You want more from the relationship than you are getting. And you want to be responsible to yourself, your health, and to your child. These are all important, Ella. Your needs and your feelings do matter. I’m glad we are talking about these things together.

**CLIENT:** I’m glad you make it OK for me to talk about it. I don’t want to be alone but I also don’t want to fail my child. Can you help me find the words to talk to Edward? I want to talk to him about having sex only with me, even though I know he could get mad about it. My son does need me, and you’re right, my fear of being alone is keeping me from making good choices for me and for my son. But I don’t want to end up like my mother—with a disease, and my son could end up in foster homes if I can’t take care of him.

**THERAPIST:** How is it for you to be talking to me about this right now, Ella?

**CLIENT:** It’s really helpful.

**THERAPIST:** What am I doing that’s helpful to you?

**CLIENT:** It helps that you haven’t made me feel like I’m a horrible mother. I feel like I can tell you things I can’t talk about to most people. They would think I’m just a loser. What should I say to Edward?

**THERAPIST:** Let’s think about what you want him to hear and practice that together...

The troubling research findings referred to earlier (that clients fear therapists’ responses to their revelations) tell us a great deal about approaching clients’ feelings. Recalling Jennie and her client with a breast cancer crisis at the beginning of this chapter, we note that *therapists often shy away from strong feelings and keep things on the surface by not acknowledging or capturing the most significant feeling in what the client just said.* Actively



intending to “hear” and approach the feeling that seems to be most sensitive, meaningful, or intense is a new way of responding for most therapists-in-training. For example:

**THERAPIST:** You’re saying so much here. Out of all of this, what’s the most important feeling you’re left with right now?

It often feels awkward to approach feelings so directly because it breaks the social norms that most of us grew up with—we are not used to talking to friends and family like this in everyday interactions. However, with help from supervisors and practice with colleagues, new therapists can learn how to approach their clients’ feelings more directly and respond effectively to the sensitive material they choose to share.

### EXPAND AND ELABORATE THE CLIENT’S AFFECT

Let’s look at open-ended questions to explore further how therapists can best respond to clients’ feelings. At times, it can be helpful to suggest or tentatively name a client’s feeling (e.g., by saying, “It sounds like you were just furious” or “I’m wondering if you’re feeling disappointed right now?”). However, as a general guideline, a more effective response is to give clients a more open-ended invitation to explore the feeling further:

- *I’m wondering what you might be feeling right now?*
- *Can you tell me a little more about that feeling?*
- *How do you feel now as you are telling me about what happened with them?*
- *Help me understand some of the feelings you’re experiencing as you tell me this.*

These open-ended invitations encourage clients to explore whatever it is that they feel is central in their affective experience, rather than what the therapist thinks is most important (Fitzpatrick et al., 2001). In particular, these open-ended responses also bring more *immediacy* to the therapeutic relationship. That is, clients’ feelings are explored and dealt with in the here-and-now with the therapist, which usually intensifies them (Teyber & Teyber, 2014).

In contrast, we don’t want to ask too many closed questions, which lead us to get all the facts and details about a problem and away from the feelings and what this event really means to the client (Barkham & Shapiro, 1986). For example, suppose the client says, “I got my chemistry mid-term back.” Much more is gained if the therapist responds with the open-ended bid, “Tell me your thoughts about it” rather than with the closed question, “What did you get on it?” This takes practice, of course, because most of us have not been socially trained to respond in this open-ended, exploratory way that “unpacks” or expands the personal meaning this holds.

An open-ended invitation also is more effective than trying to label the client’s feeling in an either-or fashion—for example, by saying, “Were you feeling \_\_\_\_\_ or \_\_\_\_\_ in that situation?” Such a choice may restrict the client rather than invite a free range of responses. Clients often experience something that the therapist has not anticipated (no matter how much experience we have, clients always seem to surprise us), and the client may have several different feelings about the same situation. Thus, open-ended queries are more effective in drawing out exactly what the client is feeling, and are more collaborative.

In addition, open-ended responses usually are more effective than asking clients why they are experiencing a particular feeling. For example:

**INEFFECTIVE THERAPIST:** Why were you so upset and crying?

**CLIENT:** I don’t know.

Although *why* questions can be effective at times, as a rule of thumb, therapists are advised to minimize them. Clients often don't really understand why they feel, think, or do something. Asking them may only make them feel "on the spot" or defensive—as if they are failing because they don't have the answer. The therapist will usually elicit more information and collaboration from clients by offering a more open-ended invitation—“*Help me understand what you were upset and crying about?*” or “*Tell me more about your feelings of being upset?*”—than by asking, “*Why were you upset about that?*” By inviting clients to express more fully whatever feeling they are experiencing, therapists are giving the message that they are interested in clients' own subjective experience and are comfortable with whatever clients say. As we have seen, clients frequently withhold affect-laden material from their therapists. Most clients have not had permission to share their emotions so genuinely in other relationships, which is why these open-ended invitations are so important. As they discover that they can be authentic with the therapist, and that it is safer than it has been in other relationships to feel whatever they are feeling, clients become willing to risk disclosing further, try out new behavior, and invest further in the therapeutic relationship.

Giving clients an open-ended invitation to explore their feelings will also help them clarify what each particular feeling means to them. Too often, therapists assume that a particular affective word, such as “angry” or “sad,” means about the same thing to the client as it does to them. Therapists should not assume that they understand a particular affective word without clarifying what it means to this particular client. Although this may be especially apt when there are cultural differences between therapist and client, it applies with every client. In this process of mutual exploration, just as we did with empathy as a collaborative exchange in Chapter 2, therapists share their understanding of what the client meant and then encourage clients to refine or further clarify the personal meaning that a feeling holds for them. This clarification and mutual exploration is especially important if clients repeatedly use the same compacted affective word to describe themselves (e.g., boring, too much). When this occurs, often several different and important meanings can be encapsulated within that single affective word. For example:

**THERAPIST:** Here again, you're describing yourself as feeling “erased” and “dismissed.” It's sounding to me like you feel totally insignificant when he does that—that you are completely unimportant so nothing you say or do matters. Am I saying that right—or can you help me capture it better?

Having the client elaborate an experience in this way—*clarifying the subjective or personal meaning it holds more specifically*—will also safeguard therapists against becoming over-identified with the client and misperceiving the client's situation as being the same as their own, as so commonly occurs. When therapists over-identify with clients, they are unable to maintain boundaries as participants/observers, and often find themselves taking clients' problems home with them.

Because they are frequently not listened to or are invalidated, many clients do not know what they feel. In many families, children grew up learning that it was not acceptable to have a certain feeling—such as being sad, angry, or even happy. As a result, many clients are uncomfortable with, or unaware of, what they are actually feeling. Thus, many clients need help to clarify the broad, “undifferentiated” feeling states that they often experience (Client: “I don't know, I guess I'm just mad all the time”). Too often, new therapists try to approach or invite their clients' feelings by repeatedly asking, “How did that make you feel?”—which usually leads to a dead end. Instead, therapists can help

clients learn more about themselves, their own inner life, and the emotional reactions that are linked to their problems by asking questions such as these:

- *Can you bring that feeling to life for me...help me understand a little better what it's like for you when you're feeling that way?*
- *Do you have an image that captures that feeling or goes along with it?*
- *Is there a particular place in your body where you experience that feeling?*
- *Is this a familiar or old feeling? When is the first time you can remember having it? Where were you? Who were you with? How did the other person respond to you?*
- *How old do you feel when you experience that emotion? Can you attach an age to it, such as 7 years old or 13 years old?*

Some of these exploratory questions will work well with a particular client and not at all with another. Following client response specificity, therapists want to assess the client's reactions to how the therapist approached the client's feelings and discern what types of bids work best for a particular client. In general, however, these are all ways of approaching clients' emotions, entering their subjective worldview, and clarifying the meaning this feeling holds for them. These types of bids also tell clients that the therapist is concerned about their feelings. Furthermore, they behaviorally inform clients that the therapist is different from many other people they have known. The therapist is comfortable sharing whatever emotions the client has and, in addition, wants to know the client's authentic experience more fully—as good or as bad as it actually is for them—without minimizing or exaggerating it as others have often done. In attachment terms, this highly accurate empathy is known as attuned responsiveness.

When the therapist works with clients to clarify their emotions, it creates an opportunity for the therapist to see and respond to the clients' most personal experience. This also provides clients with the opportunity to know themselves, especially when significant others have repeatedly invalidated, ignored, or disdained certain feelings that the client has had. This occurs, for example, when caregivers say:

**CAREGIVER:** (*invalidating*) Oh, come on sweetheart, you're not really mad about that.

OR

**CAREGIVER:** (*threatening*) One more look like that, and I'll give you something to be sad about.

As old expectations are disconfirmed by the therapist's empathy and validation, the client receives an important, real-life experience of change—a CEE.

**Clients Make Progress When They Experience, Rather Than Simply Talk about, Their Feelings.** As stated earlier, our treatment goal is not to just help clients get their feelings out. Venting does not usually help much or lead to behavior change (Lohr et al., 2007). On the other hand, however, therapists want to do much more than just cognitively label, explain, or interpret a feeling—instead, they want to help clients experience it more fully in the moment, and then respond in a different and corrective way (i.e., schema disconfirming) that provides validation and helps the client feel seen and understood. In attachment terms, this is known as containment. Little change will occur for most clients until they are able to stop talking about their emotions in an intellectualized manner and actually experience or feel their feelings in the presence of the therapist (Carkhuff, 2008).

Gendlin (1996), an early research collaborator with Rogers, has linked treatment outcomes with the importance of clients tuning in on their own emotions and their level

of “focusing” on their own experiencing process (see Hendricks [2002] for an extensive research review). With consistent validation and accurate empathy, the therapeutic alliance continues to develop, and clients feel more *safety*. In turn, this interpersonal safety allows clients to be able to risk “feeling what they feel” and “knowing what they know”—something that often has been too anxiety-arousing and not permitted in the past. The exploratory responses we have been discussing, especially the interventions that create immediacy, will intensify clients’ experience and allow them to enter their own experience more fully—as long as the therapist can contain the affect and doesn’t over-react or become upset, dismiss or minimize, or in some other way not be able to modulate his or her own experience and be empathically present with the client. In this process, the therapist provides the client with a relationship that is characterized by, and cultivates, being **reflective** (i.e., an increasing ability to see things from a variety of perspectives, which is part of helping clients respond in new and different ways). That is, the therapist resonates with, is attuned to, and experiences the client’s experience as fully as possible in the here and now, and in doing so provides a restorative experience for the client (Allen, 2013; Fonagy et al., 2002).

Therapists may also draw out the client’s affect by addressing any **incongruence** that they perceive between the client’s narrative and the accompanying affect. That is, in relaying a story that the therapist finds poignant or disturbing, the client may display no feeling at all or an incongruent affect, such as laughter. Therapists are subtly but powerfully “pulled” by social norms to match the client’s incongruent affect. New therapists, in particular, often feel they are violating unspoken social rules if they do not respond in kind and laugh—even though the story seemed sad to them. However, clients are usually relieved when the therapist risks breaking the social rules and makes a process comment by acknowledging this discrepancy between what was said and the feeling that accompanied it. By working with the process dimension and finding a diplomatic way to make this discrepancy overt, therapists make an important intervention: they begin to invite clients’ true feelings or congruent experience. For example:

**THERAPIST:** I’m a little confused. What you are saying seems so sad to me, yet you are almost smiling as you tell me this. What are your thoughts as I share this with you?

OR

**THERAPIST:** You’re telling me how angry you are about this, Lee, but your voice is so pleasant as you speak—so soft and quiet. What you’re saying doesn’t seem to quite match the way you are saying it. What do you think—is there a discrepancy here that might be telling us something?

Our intervention goal here is to provide *interpersonal feedback* that helps clients register the feelings that accompany their actual experience—to become more congruent. So many symptoms and problems stem from clients’ inability to feel what they feel: to be sad about what was lost or missed, afraid of what really was once frightening, or angry about what actually was hurtful. No small intervention, this is about identity, personal power, and the capacity to be who you really are. *Simply allowing clients to be able to have the feelings that are commensurate with what actually happened to them is a lifelong gift to many.* Recalling the concept of invalidation, it allows clients to reclaim their own experience. It means nothing less than empowering clients to know their own mind so they can live their lives without feeling confused, uncertain, or “mixed up” anymore.

The very best opportunity to provide a corrective emotional experience and help clients change occurs *at the moment when they are experiencing the full emotional impact of their problem*. Unfortunately, however, this is also the moment when therapists are most likely to feel uncertain about what to do. If therapists can communicate their accurate

understanding and genuine compassion to clients in the moment, as they are experiencing difficult emotions, it will help clients resolve the affective component of their problems. In particular, this in vivo relearning changes their expectations of how some others in their lives can respond to them, and it changes their selective biases in filtering and distorting how others are actually responding to them. However, as the following case example illustrates, clients cannot make such progress when they are psychologically alone with their feelings or can't let their true feelings emerge.

*During one session, Jean had been telling her therapist about her hopelessness. She described her life as a merry-go-round of endless ups and downs that always returned to the same discouraging spot. This past week she had dropped out of school, as she had done “so many times before,” and had given in to pressure from her self-centered, jealous, and possessive boyfriend to let him move back into her apartment again. Jean described her hopelessness about ever being able to do or have anything for herself.*

The therapist could feel Jean's despair and was moved by it. The therapist tried to work with Jean's feelings by acknowledging her despondency and validating her experience.

**THERAPIST:** You sound sad and hopeless right now—I hear you say how painful it is to feel so defeated. You haven't been able to follow through and do what you wanted to do for yourself again, and that feels so discouraging. You are again feeling that you have to go along and meet someone else's needs rather than say no and do what you want for yourself.

As in their previous sessions, however, Jean was not able to let the therapist be emotionally connected to her while she was experiencing this. She talked about these feelings but, both agreed, she held the therapist at arm's length. Because the emotional connection to the therapist was missing, it was hard for Jean to use the therapist as a Safe Haven. Even though she was talking to the therapist, she was still alone with her feelings and hopelessly alone in her life.

Developmentally, Jean had grown up with a self-centered mother whom Jean had often found herself parenting. Jean could never take her own needs to her mother—Jean's mother had stifled her ability to launch and enjoy an independent and successful life. This had made it difficult for Jean to share her struggles or concerns with her mother (or anyone else) because her mother regularly used these to highlight why Jean should live at home and attend the local college, and in other ways foiled Jean's trajectory toward independence. Jean's inability to stay in school, or to set boundaries with her boyfriend, were thus part of her struggle with making choices for herself because her mother often called her “selfish” when she began to make choices that were for her own advancement. However, when Jean acted on others' behalf and squelched her own needs—as her mother dictated—she experienced depression and feelings of hopelessness, such as she was now experiencing. *Separation guilt* kept her from advocating on her own behalf, yet, *compliance* kept her disempowered and depressed. Clearly, Jean was stuck.

The setbacks of the previous week had intensified Jean's distress. The therapist recognized that this crisis provided an opportunity to be more connected to Jean in her distress—as she was feeling her feelings—for the first time:

**THERAPIST:** Jean, you've dropped out of school and let Josh move in. Right now it seems your needs have evaporated. It makes sense that you feel discouraged and hopeless.

**JEAN:** (*long pause, nods*) Yes, it seems hopeless. I'm just hopeless—a doormat. I don't matter.

**THERAPIST:** You feel unimportant...that you don't matter...too discouraged to even try.

**JEAN:** (*nods*) I always have to do it their way.



**THERAPIST:** This is familiar...

**JEAN:** Yeah, my mother, my boyfriend, even at work—I'm just pathetic...

**THERAPIST:** Yes, you've felt this way a lot before. I can see how painful this is for you right now.

**JEAN:** (*looks away, puts hands over eyes*)

**THERAPIST:** I'm feeling with you in your sadness right now...

**JEAN:** Don't you think I'm just pathetic for letting this jerk move back in with me again?

**THERAPIST:** No, I don't see you as pathetic or weak. But I do see your sadness right now and how much all of this has hurt you. I think you are showing great courage in sharing this with me; and I see how hard you are trying to sort out what you need and want—I respect that.

**JEAN:** I just don't know what to do next, I feel so hopeless...

**THERAPIST:** Yes, it strikes me that "going along" leaves you feeling disempowered—it evokes feelings of hopelessness.

**JEAN:** I hate all of this. I wish I felt different...was different.

**THERAPIST:** What would you like to be different? What would you like to change if you could?

**JEAN:** I wish I could tell Josh to just leave but I don't know how to stand up for myself...and I worry about hurting his feelings.

**THERAPIST:** So their feelings matter and yours don't?

**JEAN:** (*hands over face*) Yes, yes.

**THERAPIST:** You do matter, your needs are important.

**JEAN:** (*pause*) It is OK to stand up for myself, isn't it?

**THERAPIST:** Yes, Jean, it is. I know it has been hard to set limits and it has been hard to let your needs be more important than other people's needs. Let's spend more time together exploring the feelings that come up when you let yourself matter, when you let yourself be a priority in your own life. It's absolutely OK and actually terrific when you stand up for yourself.

**JEAN:** (*looks at therapist and slowly begins to cry*) Well, maybe I'm not completely hopeless... I want more for myself. I do want to make more of my own choices. I've been worried about being called selfish. But I don't want to live the rest of my life taking care of everyone else and feeling so used and depressed...

For the first time, Jean was able to let the therapist be emotionally connected with her *while she experienced her despondency*. The therapist acknowledged the risk that Jean had taken, and they went on to discuss how it felt for her to share her sadness, discouragement, and disdain for herself so openly. Jean said that she was surprised to hear that the therapist didn't judge her and still respected her, and that made her feel "lighter." At their next session, Jean said that her depression had been a little better and mentioned, almost in passing, that she had told her boyfriend she wasn't ready to live together yet and that he needed to move out.

## IDENTIFY THE PREDOMINANT AFFECT

Clients often feel confused by their emotions; their feelings seem to occur for no reason and often are stronger than, or different from, what the situation realistically seems to call for. Clients' inability to understand their own emotional reactions undermines their self-confidence and engenders doubt. Helping clients make sense of their emotional reactions

is an important pathway to their greater self-efficacy (Bandura, 1997). One way therapists can do this is by working with the client's **predominant affect**. As we will see, the client's experience often revolves around a central feeling state, such as being shame-prone or guilt-prone, and therapists help when they can identify and highlight this predominant feeling.

Clients often enter therapy in response to one of two situations. First, a current life crisis echoes an emotional problem that originally occurred years before—the current stressor taps into an old wound. Second, clients also enter therapy in response to feeling overwhelmed by too many stressors in a short period. These multiple stressors have overwhelmed the clients' usual coping strategies, and symptoms such as anxiety, depression, sleep disturbances, and increasing conflict in relationships develop.

### AN OLD WOUND

Many clients enter therapy because of an old wound. For example, a client who has suffered a painful loss through divorce or death will find it more difficult to cope with the stress of that loss if it simultaneously evokes unresolved or painful feelings from previous losses in the client's life (compacted grief) (McClure & Teyber, 2003). The current loss may be associated with a predominant feeling—perhaps regret, loneliness, or sadness. *The feeling becomes understandable, and far more manageable, when it can be linked to similar feelings that have been evoked by other losses in the client's life.*

For example, Nora entered therapy at the student-counseling center for depression. She had been having intense crying spells since her aunt died two months ago—even though she hadn't known this relative very well or been especially close to her. Things began to make sense for Nora when her therapist began to ask about other times in her life when she similarly felt like “crying all the time.” Nora surprised herself by replying to the therapist, “When my Mother died... when I was 11.”

In this way, the therapist is trying to identify the core or primary affect that the current crisis has evoked and explore if it might be linked to the original wound. Connecting this primary affect to the way clients felt in the past enables them to make sense of their seemingly irrational feelings. When this occurs, clients change by feeling more accepting or forgiving of themselves (Gilbert, 2010). They are better able to appreciate the bigger problem they are actually dealing with—a story playing in two times—and gain a greater sense of self-efficacy. Although this is important in all treatment modalities, it is one of the most significant interventions therapists can make in crisis intervention or short-term therapy.

### MULTIPLE STRESSORS

*When his company downsized, Jim was laid off from work. Unable to find a new job during the economic downturn, he felt like a failure—but his problems were just beginning. As his self-esteem decreased and his financial worries increased, his wife described him as “negative” and said he was “impossible” to put up with. His marriage had never been the best but now, living all day together at home, day after day, with this increasingly short-tempered, pessimistic husband became too much for her. Six months after Jim's layoff, she had had enough of his complaining, criticism and irritability and moved out. Because his wife had always created their social life, and because he no longer had coworkers, Jim became even more isolated and lonely. Eating and drinking too much to cope with his anxiety and depression caused Jim's blood pressure to rise markedly. At his doctor's prompting, Jim reluctantly entered therapy, describing himself as a “loser.”*

Most individuals have adaptive coping mechanisms that allow them to manage a single stressful event, unless it taps into the type of preexisting vulnerability or old wound

described earlier with Nora. However, if a second or third stressful event follows from the first, as it did for Jim and many others, it becomes much harder for individuals to cope. Stretched too thin, the client's usual coping strategies fail and the cumulative stressors precipitate symptoms. In this way, most significant and enduring problems are not caused by simple, isolated events. They usually develop over time out of a sequence of events or interaction cycles—as they did for Jim.

When subjected to multiple stressors, clients may describe themselves as “overwhelmed,” “burned out,” or even “broken.” Many repeat a **compacted phrase** that encapsulates their primary emotional reaction to the stressful events:

- *I want to go away and just be alone.*
- *It's too much for me; I can't stand it.*
- *I don't care anymore; there's no point in trying.*
- *I've had it, I'm done, I can't face any more.*

As introduced in Chapter 2, therapists are looking for *recurrent affective themes* and trying to discern the most central or unifying feeling that is encapsulated in these compacted sentences. Underlying each compacted sentence are one or two core feelings that keep coming up and capture the essential impact these stressors have had on the client. Generally, the various stressors are arrayed around one or two over-riding feelings, like spokes around the hub of a wheel. For example, in response to losing too many people or things, the client may have an over-riding feeling of being alone or powerless, or the client may be afraid of experiencing further losses or changes or taking on new commitments. When therapists are able to articulate and capture the underlying feeling that links the impact of several different crisis events, clients often feel seen and deeply understood. For example:

**THERAPIST:** As I listen to what happened over the weekend, it seems that in each case—with the soccer coach, the telephone call from your boss, and then the argument with your husband—you were not heard. It seems like you became so upset because you were never really being listened to—that you weren't being taken seriously.

When the therapist has been accurately empathic in this way, and able to identify and articulate the integrating theme, a measure of hope and security may be engendered. In response to this attunement—which many clients have not found elsewhere—their motivation to explore and risk in treatment increases and they invest further in the therapeutic relationship. To succeed in short-term treatment, therapists need to develop the skill to make this type of engaging empathetic connection, relatively consistently, early in treatment in order to establish credibility and initiate a working alliance. Of course, this ability only comes in time with supportive training and the benefit of observing role models; it is not a realistic goal or expectation for beginning therapists.

## A CHARACTEROLOGICAL AFFECT

As clients relay their narratives, the therapist's intention is to try to discern and respond to the most important or meaningful feeling that clients are experiencing. Therapists can also listen for a recurrent feeling that keeps coming up in the client's life—a **characterological affect**.

As treatment proceeds, and as the therapist is accurately empathic, the therapist can often identify a predominant feeling that captures the client's current conflict and also characterizes his or her life more broadly. Clients have long experienced this familiar feeling as nothing less than part of the fabric of their lives; it pervades clients' lives or returns when they are distressed or in crisis. Clients often have one or two of these core feelings

that have been continuous throughout much of their lives. When the therapist can identify these core feelings that come up again and again in different situations, clients often respond emphatically. For example, they might say:

- *Yes, that's how it's always been for me.*
- *That's what it's like to be me—that's my life.*
- *That's me—that's who I really am.*
- *That's it! That's just how it is.*
- *You've got it!*
- *Yes—I often feel inadequate—like I'm an imposter waiting to be exposed.*

Clients may describe these core feelings as their “fate,” because it feels as if they have always been there and seems like they always will be. One of the most significant interventions therapists can make is to identify these characterological feelings and accurately reflect them to the client. For example, after listening to the client for a while, *and grasping the integrating affective theme*, the therapist may respond:

- *As I listen, it seems as if you've always felt burdened by all the demands you feel you have to meet.*
- *I'm getting a sense that you've always been afraid—that you live with the fear that people are going to find you out and point out your true inadequacies.*
- *Are you saying that you've always felt resentful; that no matter how much you do, it's never enough?*
- *It sounds as if you've always felt wary...perhaps that others are trying to put you down or take advantage of you.*
- *I'm wondering if you're worried that if people really knew the true you, they couldn't possibly love or choose you?*

Therapists are holding a sustained intention to hear and respond to each of the varying emotions that clients present as they relay their narratives. However, therapists will be most effective when they can identify one or two feelings that have recurred throughout a client's life and that the client considers to be central to his or her sense of self. This often will be one of the most important interventions that therapists can make to establish their credibility with clients and strengthen the working alliance—it demonstrates that the therapist can help.

Twenty-year-old Jose came to counseling after being mugged. He struggled with revealing his characterological affect to others—his lifelong core feelings of inadequacy—which he repeatedly defended against by presenting as strong.

**THERAPIST:** What do you think I'm thinking as I see you cry?

**CLIENT:** That I'm weak.

**THERAPIST:** I'm thinking how strong it is that you can allow yourself to have such deep feelings.

**CLIENT:** (*silent for a moment*) Most people see the “in control” part of me...

**THERAPIST:** Yes, I get that...thanks for showing me the feeling part of you.

**CLIENT:** (*crying more*) No one knows this part of me...In public, I only show the social part of me, the strong and competent part of me...I just hate being weak...

**THERAPIST:** Yes, in public they see you, but only the social part, yet in private it often seems you feel lonely because of these unacceptable feelings of being weak. I'm wondering if you wish you could share with some safe people, like you are doing with me right now?...

**CLIENT:** Yes, it does help to talk about all of this with you. I'd like to be able to be more myself with others. I do get along good publicly, yet I go home and I'm all alone. I want to be more open with others...I need to do it differently. It's a lot of pressure to show only the strong part of me.

**THERAPIST:** I think it is great that you feel so deeply and want more balance, more relationships where you can be open and share more of yourself. We can work on that together if you like. Actually, you have already started that process—right here with me. You are already sharing more of yourself to a new person, and I'm respecting how courageous you are right now.

**CLIENT:** I told my parents how scared I've been feeling after being mugged, but they... I don't know...I was hoping for something different from them.

**THERAPIST:** How so?

**CLIENT:** Well, my dad did his usual thing and started talking about being tough—and how tough he is. And my mom did her usual thing, and went on and on about how she was scared too. And then they started talking about themselves and all the things they always worry about. I wished they had listened to me, you know, let me speak more.

**THERAPIST:** That makes sense, you needed to be heard, your fears acknowledged, rather than hear about them and their problems?

**CLIENT:** Yes! How did you know? It always becomes about them...

**THERAPIST:** I didn't know, I was inquiring, wondering out loud with you.

**CLIENT:** That's exactly what I wanted—they to have just listened to me, to my needs, given me the feeling that I could come to them, have a need, and not always seem so much in control; that they would just be with me without going into their own stuff so much.

**THERAPIST:** You are putting it so well. You know, here with me, you can say exactly what you need. It makes sense to me that you were afraid. Being mugged would make most people really afraid. Wanting to be heard and responded to by your parents makes a lot of sense to me.

**CLIENT:** I'm glad you understand. (*Tearing up more*) Thank you for not making me feel bad for feeling this way. I did need to be heard and not have them go into their stuff or have it become all about them. I feel like I always have to be "in control" and "just fine" even when I'm not doing OK...

This interaction was especially significant because Jose came from a Mexican-American family. Men of Mexican descent are often expected to assume a dominant, strong, provider role (sometimes included in the "machismo" definition) in the family. Crying in front of others is generally seen as shameful, and sharing or talking about problems can also be seen as weak. Thus, while there is variability in men from this culture, many feel the need to be on guard and typically present as strong. This means presenting as stoic ("in control" for Jose) and not showing vulnerability to others—even in response to violent assault (Archineiga & Anderson, 2008; Fragoso & Kashubeck, 2000; Sue & Sue, 2016).

## RECOGNIZE THE CONSTELLATION OF FEELINGS THAT CLIENTS FREQUENTLY PRESENT

It is deeply rewarding but sometimes challenging to work with the painful feelings that clients bring. Some clients want to downplay or avoid their emotions altogether; others may be overly preoccupied with them and talk obsessively about the same feelings or



worries without any progress. When clients are not progressing in treatment, one reason may be that therapists have only responded to a single presenting affect. In this section, we will see what to do about this by learning about the constellation or sequence of feelings (the “firing order”) that clients often present.

If the therapist acknowledges the client’s current affect and invites the client to explore it further, a sequence of interrelated feelings will often occur in a predictable way (Greenberg, 2002, 2010). For example, suppose the client feels anger, which occurs in response to feeling hurt or sad about something that happened. However, the hurt or sad feeling is unacceptable to the client and is dismissed or disavowed because it seems “weak” to this client and evokes shame. To ward off the unwanted feeling of shame, the client artificially or defensively “restores” self-esteem and a sense of personal power or worth by returning to the “stronger” or safer feeling of anger again (which usually was an acceptable feeling or even scripted role in the family). As a result, others oftentimes perceive the client as being an angry person. This constellation of emotional reactions is central to the client’s problems, and each successive affect in the sequence needs to be addressed. Although different sequences occur for each client, the therapist can often identify a triad of interrelated feelings that cycle repeatedly when clients are distressed. Making a similar point, Lazarus (1989) writes informatively about tracking a client’s “firing order.” The process of change involves coming to terms with each of the feelings in this affective triad. In this section, we examine two **affective constellations** that commonly occur. We will see that therapists help clients change by responding to the entire sequence of feelings in their affective constellation, rather than just responding to the first or initial feeling in the sequence—the client’s primary presenting affect.

### ANGER-SADNESS-SHAME

The first affective constellation we will examine consists of anger, sadness, and shame. The predominant or characteristic feeling state for some clients is anger. These clients often experience and readily express a range of angry feelings including irritation, impatience, criticism, and cynicism. However, the client’s anger is usually *reactive*; that is, it is a secondary feeling that occurs in response to an original experience of sadness or pain.

The therapist needs to acknowledge the client’s anger, collaboratively work to clarify more specifically who and what the client is really angry about, and help the client find appropriate ways to express it. For example, becoming more appropriately assertive, setting limits, or telling someone what it is that you don’t like rather than losing your temper, yelling, or demeaning someone. This first step is important but not sufficient. If the therapist stops at this point, the client may still remain stuck in an angry feeling state and more substantial change will not result. Instead, *the therapist’s intervention goal is to help the client identify and stay with the primary emotional response of sadness or hurt that led to the reactive feeling of anger*. This primary feeling is more threatening or unacceptable for the client than the anger that it prompts.

To reach the original affect, the therapist might wait for clients to express their anger spontaneously and, immediately afterward, invite them to attend to what they are feeling now. Following the expression of the reactive feeling, there is often a point of entry to the primary feeling of pain or sadness. For example:

**THERAPIST:** OK, you’ve just been sharing how angry this made you, and that sure makes sense to me. But let’s stop here for a moment and check in on what else you might be feeling right now, on the other side of your anger.

Often, the original feeling (e.g., vulnerability, abandoned, embarrassment, helplessness, etc.) will often surface next. By joining the client in this feeling and working together

to accurately name it, the therapist can often dislodge the client from the familiar (but safer and nonproductive) reactive feeling of anger.

Alternatively, if this approach does not work, the therapist can inquire directly about the original experience (such as a client's husband criticizing her in front of friends) that led to the client's reactive or secondary feeling of anger.

**THERAPIST:** Something must have hurt you very much for you to remain so angry. Tell me how you felt when that first happened.

If the client again begins to express anger or indignation about what occurred (“It really made me mad when he put me down like that...”), the therapist focuses the client back on the original experience:

**THERAPIST:** Yes, I can see why it made you so mad, but I’m also wondering how it felt right then, when he was putting you down in front of everybody?

**CLIENT:** Well, I guess maybe it hurt my feelings a little.

**THERAPIST:** Yes, it hurt your feelings. Of course it did. I’m so glad you can share that hurtful feeling with me. You’ve just made a big change, you know: you’ve never been able to risk sharing the hurt part of this before. How does it feel to taking that big step with me right now?

**CLIENT:** (*emphatically*) I just hate feeling so weak that he can hurt me like that.

Thus, in response to this query, the client’s original feeling of “hurt” was expressed for the first time. The therapist responds affirmingly as the client begins to experience and share the internal aspect of her problem—that is, the feeling of hurt or sadness that is so unwanted or unacceptable for her. Recalling our internal focus for change, *it is this internal aspect of her problem—her sense of herself as weak or shame-worthy because he has been able to hurt her—that has kept leading her back to her safer, reactive feeling of being angry all the time.* This is a complex sequence for new therapists to follow in the beginning; let’s keep going and try to make it clearer.

As we have just seen, when this client experiences hurt or sadness about what happened, these vulnerable feelings are held away. Although her anger is easily experienced and readily expressed, the client has learned that it is not safe to experience or share the original hurt or vulnerability. To avoid the original painful feeling, the client defensively returns to the reactive feeling of anger in a repetitive manner. Again, this defensive coping pattern often leads others to regard this client as an “angry person”—and contributes to the problems the client keeps having with others at work and at home.

The goal of the therapist’s intervention is to help such clients experience the sadness, hurt, or vulnerability that precipitates their reactive/repetitive anger. However, as soon as the original hurt is evoked, clients now feel ashamed and will defend against this unwanted feeling by reflexively returning to the safer expression of anger that, in their belief, is “stronger.” Thus, the therapist needs to explore clients’ resistance to the original feeling of being hurt. Often, clients avoid the original painful affect because it arouses a third, aversive, feeling of anxiety, guilt, or—most frequently—shame, as we saw earlier.

For example, as the therapist inquires about clients’ reluctance to feel the original hurt, they will often reveal faulty beliefs and say something like:

- *If I let the pain be there, it’s admitting that he really can hurt me.*
- *If I acknowledge that it hurts, it means he won.*
- *If I feel sad, it shows that I really am just a baby feeling sorry for myself.*
- *If I admit that part of this was my fault, then EVERYTHING becomes my fault!*

Thus, if the therapist draws out the original pain that underlies the reactive anger, a third feeling of shame will often be aroused. To defend against the original sadness, and the shame associated with being sad, the client has learned to return automatically to the “stronger” feeling of anger that is safer or more acceptable for this client to reveal.

Although shame is a common reaction, other clients will experience anxiety or guilt in response to feeling the original hurt. For example, some clients will say that if they let themselves feel sad, hurt, or vulnerable, then “no one will be there,” “others will go away,” or “I will be alone.” These clients feel painful separation anxieties upon experiencing their hurt or pain and avoid this anxiety by reflexively returning to their anger. Still other clients will say that, if they let their sadness be real, they would be admitting that they really do have a need, which would make them feel selfish or demanding. For these clients, guilt is the third element of their affective constellation.

This triad of feelings exists for many clients: *Frequent anger defends against unexpressed sadness, which, in turn, is associated with shame, guilt, or anxiety.* Enduring change results when therapists can help clients better come to terms with each feeling in the triad. More specifically, clients resolve such conflicted emotions when they can:

1. allow themselves to experience or feel each feeling;
2. share or be able to disclose the feeling with the therapist so they are no longer alone with it or hiding it; and
3. with the therapist’s support, be able to stay with the difficult feeling for a moment or two (i.e., tolerate, hold, or contain it) rather than have to move away from it, as they have in the past.

This is what it means to integrate or come to terms with unresolved feelings.

Clients integrate their conflicted emotions when they can progress through these three steps for each feeling in their affective constellation; they are internally resolving their conflict. With this resolution, clients no longer need the outdated coping strategies or defenses that they have employed in the past. Furthermore, at this point clients are prepared to adopt new, more adaptive responses with others in their lives that they were previously unable to incorporate. For example, rather than always get mad and alienate others, they can opt to respond differently sometimes. In the following example, the client is becoming more direct and assertive without elevating:

**CLIENT:** (*to husband*) I’m feeling criticized right now and I don’t like it—I’m asking you to find a better way to talk about this.

**HUSBAND:** (*contemptuously*) Umm, I’m not really so concerned about what you think. Why don’t you just settle down and be quiet.

**CLIENT:** I’m feeling disrespected right now and it’s not good for me to continue this conversation. I’d like to talk about how to manage our money with more civility. Let me know when you are willing to hear my ideas without telling me that everything I say is stupid.

Clearly, this affective integration is a pivotal experience in the process of change.

Before going on to examine another common affective constellation, let’s pause to highlight further the special role of *shame*. Although much has been written about anxiety, depression, and guilt, shame has long been eschewed in clinical training. Correspondingly, clients could be anxious, guilty, or depressed with their therapists, but not ashamed. Thankfully, shame is no longer the taboo topic it has been; indeed, this most unwanted and painful feeling state is now regarded by many theorists as the “master emotion” (Nathanson, 1987; Scheff, 2000). The cardinal role of shame in most resistance and defense, and in so many of the symptoms and problems that clients present, was first

illuminated by the groundbreaking work of Helen Block Lewis (1971) and later developed by many others (Tangney & Dearing, 2002; Tracey et al., 2007).

As we began exploring in Chapter 3, new realms of understanding will open up to therapists as they allow themselves to begin “hearing” the family of shame-based emotions that pervade many of their clients’ narratives. To feel shame is to feel fundamentally bad as a person—inadequate or worthless in the essence of your being (Dearing & Tangney, 2011). Clients directly reveal their shame-based sense of self by repeatedly using words such as bad, stupid, worthless, self-conscious, sheepish, embarrassed, low self-esteem, humiliated, and so forth. Less direct indications of shame may include the repetitive use of words such as should, ought to, must, and perfect.

On an interpersonal level, some clients defend against their own shame, and defensively attempt to restore their injured sense of self, by inducing shame in others. That is, these shame-prone clients routinely try to disprove their feelings of shame through rage and violent temper outbursts (“You can’t tell me what to do. I’ll show you!”); contempt for others or giving strong judgments (“You’re stupid” or “What’s wrong with you!”); provocation of others with bravado (“Just try me”); and automatic blame or counter-criticism of the other—especially in chronic marital conflict (“No, it has absolutely nothing to do with me, it is *all* your fault”). In contrast, other clients defensively try to restore themselves and deny their feelings of shame by rigidly striving for perfection, withdrawing from others or going away inside (e.g., “I just wish I was invisible”), and engaging in other avoidant and shame-anxiety cycles (Balcom et al., 1995; Tangney & Dearing, 2002).

When therapists are able to break cultural taboos and begin approaching shame reactions sensitively but directly, significant progress often follows. However, most therapists have been socialized strongly to avoid or to rescue clients from these shame-based affects (i.e., ignore, make light of, or change the topic). As a result of these social norms, new therapists will need the support and guidance of their supervisors, and additional reading, to begin responding effectively. Bromberg (2006) writes elegantly about the importance of approaching and addressing shame—so it is challenged with empathy rather than given legitimacy—in the safety of a supportive therapeutic relationship. In a nut shell, however, therapists help clients resolve their shame when they demonstrate compassion for clients’ feelings of inadequacy or incompetency, harsh or contemptuous judgments toward themselves, and feelings of worthlessness. Often, these shame-based emotions are referred to euphemistically as “low self-esteem,” masking the far-reaching emotional wound that is actually at hand.

Therapists help clients resolve such shame-based reactions in two steps. First, by providing an unambiguously kind and accepting response to clients as they are experiencing these feelings of shame, and then following up and processing this with clients by clarifying that the therapist was not feeling judgmental or critical toward them in any way (as they expect and typically misperceive). Second, the therapist is working (over time) to help clients *internalize* the therapist’s compassion for their shame and begin to feel the same empathy for themselves and their own suffering that the therapist felt toward them (Elliott, Bohart, Watson & Greenberg, 2011). In other words, clients break the identification with the attachment figure who was rejecting or contemptuous and, instead of despising themselves in the same way the caregiver once did, clients come to adopt the same compassionate or empathic stance toward themselves and their own humanness that the therapist provided. This sequence resolves the internalized contempt at the core of shame dynamics (Teyber, McClure & Weathers, 2011). It is very difficult for most of us to bear witness to our clients’ shame, especially as we begin our training. With supportive supervision and role models, however, therapists can soon learn how to offer clients this great gift.

### SADNESS-ANGER-GUILT

Whereas some clients lead with their anger and defend against their hurt, others lead with their sadness and avoid their anger. This type of client may present with a general (“undifferentiated”) feeling state of sadness, helplessness, vulnerability, or depression. These clients do not experience or express anger, they avoid interpersonal conflict, and they tend to respond to others’ needs at the expense of their own. In addition to this caretaking role, some may present as acting like a martyr or a helpless victim. To illustrate this affective constellation, we return to the case of Jean (page 156).

Earlier in this chapter, we examined a dialogue in which, for the first time, Jean was able to let the therapist respond to her sadness and be emotionally connected to her while she was experiencing it. Subsequently, the therapist explored what had made it difficult for Jean to share her vulnerable feelings with others. Jean explained that historically her mother would listen to her. However, her mother would then tell her she “shouldn’t be that way,” especially when Jean tried to assert herself or act on her own behalf. She’d tell Jean to be “considerate” of others and put others’ needs first—that doing so was the “Christian” thing to do. (Jean subsequently clarified in treatment that she partly agreed with her mother and truly desired to be a good Christian, but her mother’s dictum was out of balance and led her to deny other important parts of her self—it didn’t support her wish to honor both God and herself.) Thus, in these ways, Jean had come to keep her feelings to herself, because she felt sharing them was no use anyway—she’d just be criticized, told she was being selfish, and in the end, her needs would not be met because the needs of others would always be deemed more worthy than hers. Following these early maladaptive schemas, Jean had continued in adulthood to reenact the affective themes and relational patterns of her childhood in the problematic love relationships she kept reconstructing.

Recall that when Jean returned to therapy the following week, she had made a significant change: she had told her boyfriend she wasn’t ready to live with him yet. Two weeks later, Jean’s boyfriend borrowed her car for the day and arrived late to pick her up from work, and this time she expressed her indignation. Further, in an uncharacteristically strong move, she told him that she deserved to be treated with more respect and consideration and, soon afterward, ended the relationship. Jean had remained locked in her sad and helpless victim role, in part, because the direct expression of anger and the assertive expression of her own needs were unacceptable—selfish and hurtful to others. As often happens, Jean began to claim her own personal power when her feelings were expressed and she received affirming responses to them from the therapist. However, we need to look more carefully at the sequence of feelings (sadness-anger-guilt) that unfolded for Jean.

Although Jean began the next therapy session by sharing her good news about ending the problematic relationship, she had difficulty sustaining this stronger stance. As the session progressed, Jean retreated from her indignation at how badly her boyfriend had treated her, began to feel concerned about how she may have “hurt” him, and wondered whether she had been “selfish” to end the relationship because he “needed me so much.” However, the therapist’s working hypotheses had prepared her for the possibility that Jean would feel guilty upon experiencing her anger, setting limits, or responding to her own needs. With this awareness, the therapist was able to recognize Jean’s guilt and engage her in questioning its validity. Jean had defensively avoided her anger because it aroused guilt, which served to return her to her characteristic affect of sadness and interpersonal presentation of helplessness. As the therapist helped Jean identify and consider all three of these interrelated feelings, she was able to relinquish her helpless/victim stance for increasing periods.



With the therapist's support, Jean could acknowledge the painful reality of the deprivation and invalidation that she had originally experienced in her family. *As she stopped denying the reality of these painful experiences, she stopped reenacting them in her current relationships*; she no longer recreated these familiar relational patterns that kept evoking the same disheartening emotions. By responding to each feeling in clients' affective constellations, therapists help clients make sense of themselves and their lives with a new and more realistic self-narrative, and change how they respond to themselves and to others.

This is a lot of material for most new therapists to absorb, so let's pause and try to tie things together a bit. Stepping back for a moment, let's see how the previous topic, the predominant characterological affect, relates to a client's affective constellation. The predominant **characterological affect** will typically contain the leading edge of the affective constellation or triad. For example, clients who present affectively a predominant sense of helplessness can easily access and express sadness, which is first in their triad. Often, *the first emotion in the triad is the one that was allowed—or even prescribed as part of the child's role—in the family of origin*. Other feelings became subsumed under this emotion. Often, the second affect in the triad was too threatening to a caregiver—and therefore was not allowed to be experienced or expressed. Over time, the client came to see these legitimate feelings as unacceptable as well, and the client's own authentic voice was muted. The third affect in the sequence, shame or guilt in the two examples here, is often avoided because it is too disruptive or aversive for the client. Hence, the client repetitively returns to the first or leading affect that is safer and more familiar.

The therapist's intention is to validate the legitimacy of each feeling in the affective triad and remain empathic or emotionally connected to clients as they are experiencing them. This gives clients the experience of expressing themselves authentically without receiving the unwanted responses they have come to expect. Meaningful changes are set in motion for many clients as they find that their previously unacceptable feelings can be shared without the familiar but unwanted consequences occurring. In particular, many begin to feel that they can center in themselves and “live inside their own skin,” rather than being so anxiously preoccupied with appearances and trying to manage how others see and respond to them. This is especially problematic for clients who are shame-prone because *shame involves such insidious self-consciousness*, negative self-evaluation, and the fundamental feeling that something is wrong with one's *self*, which seemingly is being negatively viewed by others (see Dearing & Tangney, 2011; Parker & Thomas, 2009). Thus, outgrowing these limiting familial rules about affective experience and expression—which often are embedded within cultural expectations—brings clients into a more coherent personal identity. Feeling safer and stronger, they are able to risk more with the therapist and are empowered to initiate changes in their own responses to others in their lives.

## ATTACHMENT THEORY PROVIDES HELPFUL GUIDELINES FOR RESPONDING TO CLIENTS' DISTRESS

In attachment terms, the best way for therapists to help clients manage and resolve challenging feelings is to provide a Safe Haven that contains their distress. Before exploring this treatment goal of affective containment, and the interventions therapists can use to provide it, let's refresh and clarify core constructs in attachment theory that help therapists understand and respond to their clients' emotions.

Caregivers who provide a secure attachment for their children offer both a Safe Haven (consistently respond with empathy and emotional responsiveness to the young child's distress) and a **Secure Base** (actively support the developing child's exploration and increasing independence). Both are necessary for a secure attachment, but the Safe Haven

construct is especially apt here and will help us learn how we can help our clients with their conflicted feelings.

Children develop the capacity to successfully modulate or “self-regulate their own emotions without defenses or symptoms when they receive attuned responsiveness from caregivers who recognize their changing states of affective arousal (Fonagy et al., 2007). Children who receive sensitively attuned responses have caregivers who consistently discern their child’s emotional needs or messages, and respond accurately to what the child is experiencing or needing. These children have the experience of, and receive, being protected, comforted, and supported in times of need. When parents and children function in this mutually attuned or synchronous way where they each learn to read and respond to the other, children develop the secure feeling that they are seen and understood. They also learn that they matter to someone and that their feelings—especially their problems or distress—are important and will be responded to.

Within a well-functioning interactive dyad like this, the attuned parent sees or registers the child’s experience and responds to the child’s need *before* the child becomes overly distressed. The parent and child are smoothly coordinated with each other—conflict and distress is minimal and readily resolved. This fortunate child is not indulged and does not become dependent, but increasingly learns to cope with and modulate distress successfully. Researchers find that these Safe Haven features actually make children stronger or more resilient (Sroufe, 2005). Experiencing this nurturing and attuned relationship also teaches the child that *moments* of misattunement between them can be overcome. That is, recovery from ruptures or miscues is possible in their relationship with caretakers, which often makes distressing emotions easier to manage. Indeed, short-term dysregulations (where a caretaker and child “miss” each other) can be used as opportunities for repair (e.g., Mom: *You were trying to tell me that you were upset, and I thought you were just being difficult. I was in a hurry and wasn’t listening very well. I’m sorry I got so impatient—let’s get back on track*). In contrast, long-term or chronic states of misattunement, where the attachment figure consistently ignores or misreads the child’s attempt at cueing or communicating their need or distress, are highly problematic for child adjustment. It results in deficits in affect modulation and emotional self-regulation that will come to characterize many of the symptoms our clients present in treatment. In the following section and in the chapters ahead, we are going to see that attachment researchers identify two broad types of affect dysregulation:

- 1) **Deactivation** is where feelings are consistently down-regulated and clients are out of touch with their own and others’ feelings. These clients ignore or minimize reality-based threats in the environment; deny problems, needs, and other negative emotions; and tend to suppress feelings of vulnerability.
- 2) **Hyperactivation** is where clients typically escalate emotions or over-react because they fear abandonment and ruminate on distressing emotions.

For children to feel secure or contained, their caretakers need to *sustain* the consistent intention to provide this empathic attunement—characterized by responsiveness to the child’s distress and the willingness to engage in repair when misattunement occurs. Parents who provide this attuned responsiveness, where children can bring any need or feeling, are considered a Safe Haven—which is central to the secure attachment that helps children modulate and manage their affects resulting in better mental health and buffering them from symptom development such as anxiety, mood disorders, eating disorders, temper or rage outbursts, and so forth (Aromur, Elklit & Shevlin, 2011; Bakermans-Kranenburg & Van IJzendoorn, 2009; Tasca, Ritchie, Zachariades, et al., 2013).

Why are we emphasizing these attachment/developmental constructs to therapists-in-training? As already mentioned, there is strong empirical support for them and we are

going to try and show how therapists can use these attachment concepts to guide and inform how they respond to their clients' emotions. Let's continue a little further before turning to intervention strategies.

To establish a secure attachment, an attuned caregiver who provides a Safe Haven also needs to provide the child with a **Secure Base** from which to venture out and explore the world with increasing confidence and autonomy. The child often uses a caretaker's emotional expressions as a "signal" or reference for what is safe and what is dangerous in the environment. When the child feels secure and is "cued" that it is safe (and permitted) to venture out, he or she is more likely to explore, develop mastery, show flexibility in dealing with others and the world, and develop a more integrated identity and coherent self-narrative. The child is able to individuate or become more independent and to have a more balanced approach to affective experiences. This individual is able to experience feelings without being symptomatic—such as frequently losing his or her temper or feeling overwhelmed. Thus, *full and successful engagement in treatment is associated with clients' experience of their therapists as both Safe Havens (support for their distress and emotional needs) and Secure Bases (support for their differentiation and independence)* (Parish & Eagle, 2003).

In contrast, clients who have not had secure attachment histories often come to therapy having had developmental experiences that were painfully isolating, anxiously unpredictable, aggrandizing or disempowering, and so forth—because their caregivers did not consistently respond to their fear, problems, or distress with the affirmation and understanding that children need in order to learn how to manage or self-regulate their own distress (Brisch, 2002; Cassidy & Shaver, 2008). To be more behaviorally specific, **containment** means that the child's concerns were heard, taken seriously, and understood by a caregiver who remained emotionally present and responsive. The child's distress was consistently registered with the caregiver and was taken seriously, without being diminished or exaggerated. The child who felt secure has learned that the caregiver would be "wiser and stronger" and remain available to help manage the problem, if only by expressing interest and concern (Cooper, Hoffman & Powell, 2013). The key concept is: *the securely attached child is secure in the expectation of getting help or being responded to when distressed, vulnerable, or in need*. It does not mean that the child is indulged or spoiled in any way, that the child gains the upper hand in the parent-child relationship and can successfully be demanding or manipulative, or that the caretaker can always solve the problem. But it does mean that the child is not alone with distress or emotional need—there is an empathic connection.

Before children (and clients) can develop the capacity to manage disruptive feelings on their own and function more independently, someone must first provide this Safe Haven or holding context for them. However, student therapists are quickly going to recognize that, for most of their clients, this essential developmental experience of having a parental Safe Haven to provide them with **affect regulation** was not provided. For example, suppose the child is sad or hurt. In some families, the child's sadness:

- *arouses the parent's own sadness, and the parent withdraws, which leaves the child emotionally alone;*
- *makes the parent feel anxious or guilty about not knowing how to respond, and the parent denies the child's sad feelings and tries to cheer the child up, which leaves the child confused and alienated from his or her own authentic experience; or*
- *makes the parent feel inadequate as a mother or father, and the parent responds punitively or derisively toward the child's sadness, which leaves the child ashamed of his or her vulnerability.*

In such commonly occurring scenarios, the child's sadness cannot be heard and responded to; as a result, these feelings cannot run their natural course and come to their

own natural resolution. Young children cannot contain strong, painful affects—such as fear—by themselves without support from an emotionally available caregiver. As a result, children will have to find ways to deny or avoid this painful feeling (deactivate) or exaggerate the feelings to get the caregiver’s attention (hyperactivate). Years later, when these children are adult clients and risk sharing the unacceptable or threatening feeling with the therapist, *the therapist’s intention is to provide the holding environment or Safe Haven that was missed developmentally* (Fosha 2003; Johnson, 2004; Fosha, Siegel & Solomon, 2009). Working within this attachment framework, the therapist’s role is to provide a safe harbor for a while. And later, when clients feel “contained” and are functioning better, they use this safety to explore—to try out new behavior and allow themselves new emotional experiences (i.e., use the therapist as a Secure Base from which to launch). Providing clients with this attachment security is so important because it is crucial to the client’s affect regulation—that is, the client’s ability to manage anger, sadness, loneliness, anxiety, and other disruptive feelings without acting out or developing symptoms such as depression, over-eating, and alcohol or substance abuse (Shaver & Cassidy, 2008). We have already seen two instructive examples of containment in Chapter 2: Marsha’s second therapist heard and responded to her experience of “emptiness,” (Page 59) and the therapist of the depressed young mother sensitively articulated the mother’s own “unanswered cries” (Page 392). Both therapists successfully provided a Safe Haven, which allowed their clients to make important changes in the next few sessions.

### CONTAINMENT: USING ATTACHMENT CONSTRUCTS TO GUIDE INTERVENTION

With Safe Haven and affect regulation in mind, the therapist’s next concern is how to put this in practice and respond most effectively. What can we say and do to help our clients in these special moments when they are feeling deeply their pain and distress? Perhaps nothing evokes the new therapist’s insecurities more than this moment:

- *I’m so uncertain about what to do...how can I make him feel better?*
- *What if I make a mistake or say something wrong—and make things even worse for her?*
- *How can I help with these feelings? I want to help her so much but I really don’t know what to do...*

As we have just seen, attachment-informed therapists respond effectively to clients’ distressing feelings by providing a Safe Haven or **holding environment** of empathic understanding and attuned responsiveness to contain the client’s affect. Therapists provide this *is by maintaining a steady presence in the face of the client’s distress*, and do so with the tone of their voice, the way they maintain eye contact, and may even use touch (Levensen, 2010). The therapist’s verbal and nonverbal communications are:

- *I can hear these difficult feelings and register how important they are to you;*
- *I can tolerate the feelings and stay steady with you without having to minimize your distress to keep in my own comfort zone (deactivating therapist) or become overwhelmed or disrupted myself (hyperactivating therapist);*
- *Together, we can get through this; you don’t need to be alone with it; and,*
- *I understand that this has felt undoing for you. I’m OK hearing it, feeling with you, and choosing to stay present and supportive as we explore and clarify further what you need and want.*

Clients who hyperactivate need help tolerating distress. They have difficulty calming or soothing their affect and can benefit from mindful-based approaches like those used

in dialectical behavior therapy, where there is a focus on the client's thoughts, beliefs, and behavior in a way that validates their experiences with compassion (Linehan, 1993). Conversely, clients who deactivate need therapists who can help them identify and become aware of their feelings and learn to express their emotions. When these clients can process what is threatening about being emotional or have close, intimate relationships, they will become less critical of their emotions and feelings of vulnerability. These clients need therapists who will use emotion-focused or experiential approaches (Elliott, Watson, Goldman & Greenberg, 2004). Clients have a CEE when the therapist responds in a new and safer way that resolves rather than metaphorically reenacts old relational patterns—*especially around affective experiences*. As emphasized earlier, however, unless the therapist debriefs with the client afterwards and clarifies or makes overt what the therapist was thinking and feeling as clients were experiencing or sharing these conflicted feelings, many clients will subsequently misperceive the therapist's response to fit their old schemas. Consider the following example of an immediacy intervention:

**THERAPIST:** Right now, as you are feeling so upset about this, what do you think is going on inside for me?

**CLIENT:** I don't know what you mean—what are you asking for?

**THERAPIST:** As you share this with me, what might I be feeling or thinking about you?

**CLIENT:** Well, since you're a therapist, you have to act nice and supportive so you'd probably never say anything. But you're probably thinking I'm over-reacting again. You know, acting like a child, and I should just stop feeling sorry for myself and grow up. Like my husband says.

**THERAPIST:** Oh, no, I'm not thinking that at all. Actually, I was feeling how sad it is that this has hurt you so much for so long; and that you have had to be alone with it.

Without such exploration and clarification, many clients will settle back into the inaccurate belief that the therapist is privately judgmental, disappointed, frustrated, burdened, and so forth, just as significant others have been so many times before. Without such clarification, some clients may have difficulty attending the next session.

In sum, as an emotionally responsive and affirming therapist continues to provide these CEEs and clients' feelings are contained, distressed clients will discover that they can increasingly restore their own emotional equilibrium. Real comfort comes from being seen—empathically understood and validated—and psychologically “held” in this way, which leaves clients feeling secure and empowered (Geller & Greenberg, 2012). Clients responded to in this way become independent, have greater capacity for self-reflection and better perspective-taking, can access both positive and negative affects, and develop a mature sense of self as they learn how to self-regulate their emotional reactions through the experience of feeling containment with the therapist (Greenberg, 2002; Schore, 2003). We are going to explore how, in the moments following such containment experiences, clients often step forward and make progress in treatment. For now, however, let's look further at how therapists can provide containment when their clients are distressed.

Too often, therapists minimize, reassure, explain, or seemingly don't hear and simply move away from clients' painful feelings. In so doing, they commonly reenact the clients' developmental problem and again leave clients disconnected from others while they are distressed and experiencing difficult feelings. This occurs for many reasons, but one of the most common is because *therapists assume too much responsibility both for causing the client's pain and for alleviating it*. Erroneously, new therapists may think, “I got the client into this, so now it's up to me to get him out of it.” Both parts of this belief are



false. By responding in the ways suggested here, therapists *reveal* feelings that are already present in the client; they do not *cause* them. Moreover, it is patronizing for therapists to think they can “fix” clients. In actuality, the therapist never has the power to manipulate another’s feelings in this way, and attempts to do so commonly reenact the caretakers’ controlling or dependency fostering stance with the client. When these ineffective responses occur, there is no Safe Haven or holding environment.

Let’s look more closely at how therapists’ inability to hear and join clients in their core conflicted emotions, as reflected in these ineffective responses, often thematically replays the clients’ developmental predicaments. For example, by trying to talk her out of her feelings, Marsha’s first therapist (page 58) reenacted her experiences in her family. Marsha’s first therapist went even further to avoid Marsha’s feelings by, in effect, sending her away: When Marsha got near her sad, empty feelings, the therapist suggested that she join a group and talked to her about seeing a psychiatrist for antidepressant medications. Such referrals are certainly necessary at times, yet in this case they were used more to distance the therapist from his own discomfort with her feelings. Unfortunately, this occurs commonly and, as we will explore closely in the next chapter, is more likely for therapists who themselves have a Dismissive attachment style (i.e., avoid commitment in relationships, minimize their feelings and deny their needs, and rigidly prize their self-reliance). The therapist’s aim is to welcome the client’s feelings and approach them directly. For example, the following responses offer a point of entry and would be more effective ways to respond to Marsha’s strong feelings:

**THERAPIST:** I’m glad you’re taking the risk of telling me about these sad and empty feelings. Let’s keep talking—I want to understand them better.

OR

**THERAPIST:** I know that talking this way and being together with such painful feelings breaks the norms you grew up with, but I appreciate being able to share all of this with you.

Therapists want to find genuine ways to express their care and concern. Therapists can do this in words, of course, but what we communicate nonverbally—in our tone of voice and our manner—is equally important (Richmond et al., 2012). There are many ways to be emotionally present with clients, of course, and much of this communication is nonverbal or involves only a few words. Next, we provide some guidance for successfully providing the holding environment that gives clients the reparative experience of containment. However, therapists will want to tailor their responses to fit each particular client and make them congruent with their own personal styles.

Initially, the therapist’s primary goal is to stay emotionally connected or present with clients as they are experiencing painful feelings (Geller & Greenberg, 2012). As discussed in Chapter 2, the therapist may also want to reflect what clients are feeling and affirm the reality of their experience. This can be done just by acknowledging, “You’re very sad right now.” With some clients, it may be helpful to further validate their experience by saying, “Of course you are feeling mad right now. It hurt you very much when he did that.” As an important part of developing their own professional identities, therapists-in-training are encouraged to explore and try out their own ways of expressing their compassion or concern. Role models are very important, yet instead of trying to be like their favorite supervisor or therapist, new therapists are encouraged to find their own style with clients in these special moments. For example:

**THERAPIST:** I can see how much you miss your grandfather and how sad you are about his passing—he was so important to you. I feel honored that you choose to share such a special part of yourself with me—thank you.

To provide a Safe Haven for clients, therapists also want to demonstrate behaviorally that they can tolerate the full intensity of clients' feelings. In other words, they must communicate by their manner—and overtly in words with some clients—that they are in no way hurt, burdened, or undone by clients' feelings (as most clients expect).

**THERAPIST:** I'm privileged that you choose to share this with me. I'm with you right now, and we're going to stay together in this until we've worked our way through it.

As clients regain their equilibrium from a strong emotion, the therapist may also want to clarify how caregivers or significant others tended to respond when they felt this way. This is often illuminating as it makes clear to the client and therapist alike how it was simply unimaginable for the client, as a child, to take distress to a caregiver. The therapist may also want to highlight:

- (1) how their current relationship is different from the client's relationships with others (e.g., the therapist remains emotionally present and affirming rather than becoming impatient, withdrawing, minimizing, or making it about him or herself, as others have tended to do) and
- (2) that he or she is accepting and welcoming of what the client has just revealed. Again, it is essential to debrief clients by asking them how it has been to share these feelings or personal disclosures with the therapist.

As emphasized above, too often therapists miss this important opportunity. Genuinely touched by the client, they fail to appreciate that it is at these intense and sensitive moments that the client is most likely to misperceive the therapist's caring or affirming response in terms of earlier schemas. They can correct these expectable distortions by making a process comment. For example, the therapist might ask clients what they think the therapist was thinking, feeling, or going through while they were feeling sad, angry, and so forth. Early in their training, it can be awkward for new therapists to broach this, and it just doesn't seem necessary to check in with the client like this. However, long-standing schema distortions, which also cause problems and disrupt relationships with others, can often be identified and disconfirmed at these moments. For example, the therapist might clarify the client's distortion by means of a self-involving comment:

**THERAPIST:** No, I wasn't thinking that you "looked silly" or "sounded weird" in any way. In fact, I was touched by how alone you felt in that awkward, embarrassing situation. Your feelings sure made sense to me.

When offered sincerely, simple human responses of validation and respect mean much to clients and help to restore their dignity and composure. Unfortunately, new therapists often believe that they have to do much more than this to respond adequately to clients' pain. These concerns may reflect the therapist's own unrealistic expectations that there is an absolute right or wrong way to respond. In reality, the client does not need the therapist to provide eloquent, crafted responses in a calmly self-assured manner. Such perfectionism is as unnecessary as it is unrealistic. It often serves only to make therapists more preoccupied with evaluating their own performance and less focused on just being present with clients and grasping what they are really saying (Greenberg, 2014). As Winnicott (1965) says, the child needs only "good enough mothering." Therapists provide a "good enough" holding environment when they find their own genuine ways to enact these guidelines. Clients will be appreciative of therapists' sincere efforts and find them helpful, even if they are not always expressed articulately.

### A SAFE HAVEN FACILITATES CHANGE FROM THE INSIDE

When clients feel safe, they experience the sense that positive outcomes are possible. In this safe relationship, they can bring their needs, clarify their beliefs about themselves and others, and begin to forge with selected others the kind of supportive relationship they are having with their therapist. Indeed, as a result of clarifying who they are, their likes and dislikes, and how they want to be in the world, they are empowered to become their own agents—to develop their own voice. The acceptance and safety they experience from the therapeutic relationship fosters a sense of who they are in a stable, mature way that helps them develop autonomy—where they are now able to embrace all aspects of themselves (including their imperfections and insecurities, as well as their strengths). In contrast to how they presented at the initial sessions, clients no longer have to be detached or cut off from parts of themselves (deactivate), or overwhelmed (hyperactivate). Getting the experience that they have access to a supportive other, who provides the safety to sort through the feelings, thoughts, and experiences that were previously too threatening to explore, experience, and share, is invaluable (Moyers, Houck, Rice et al., 2015). Their feelings become more modulated and flexible and can become part of a coherent or more integrated personal identity.

Many clients will be profoundly relieved to discover in the therapeutic relationship that neither they nor others need be hurt, burdened, or overwhelmed by their feelings. When the therapist can encompass clients' most threatening emotions, *and it is made clear to the client that the unwanted consequences they expected did not ensue this time*, clients forge a more authentic connection to the therapist—and to themselves. They discover that it is safe this time to be seen and known as they truly are. Thus, the feelings that previously had to be sequestered away can now be experienced, understood, and integrated. Furthermore, when the therapist can hold clients' threatening feelings and allow them to tolerate experiencing and sharing them, they, in turn, will become more able to contain these feelings on their own, which greatly changes how they can respond to others (Elliott, 2011). This is change from the inside out. This interpersonal process often propels a far-reaching trajectory of enduring behavioral, cognitive, and affective change, and long-standing symptoms often improve at this point. Ironically, when clients risk exposing their pain, vulnerability, or shame, and the therapist responds with compassion, they feel more powerful. They are more flexible, more accepting of their vulnerabilities and imperfections, and approach relationships in a more reflective way, that is, they become flexible and can see things from a variety of perspectives (Fonagy et al., 2002). Being able to facilitate such self-acceptance and self-efficacy is one of the most satisfying aspects of being a therapist.

When difficult feelings emerge, therapists have the best opportunity to help clients change. *The most important CEEs usually occur around the therapist's new, more satisfying response to the client's emotions. On the other hand, however, familiar but unwanted patterns are most likely to be reenacted with the therapist in these affect-laden moments as well.* Why? Vulnerability activates our schemas or IWMs and leads clients to expect the familiar but unwanted response from the therapist they have received in the past. Per Beck's "hot cognitions," it is when strong feelings have been triggered that clients are most apt to distort or misperceive the therapist's response and slot it to fit old expectations. Compounding this situation, the therapist's own personal issues or countertransference reactions are also most likely to be evoked in these affective-infused interactions. Thus, all therapists need to pay attention to their own countertransference propensities. To help, we turn now to personal factors in the therapist's own life that can make it difficult to respond to clients' feelings.

### THE CLIENT'S FEELINGS OFTEN EVOKE COUNTERTRANSFERENCE

Significant feelings about issues in therapists' own lives are often evoked by the clients' feelings. This is normative and expectable but throughout their careers, therapists need to be willing to consider and work with their own personal reactions to the material clients present. In much the same way that it is sometimes easier for clients to externalize and focus on others rather than themselves, it can be easier for therapists to focus on the client than to look within at their own issues and dynamics (Ladany & Inman, 2012). If, however, therapists are not willing to work with their own emotional reactions to the material clients present, they will tend to avoid dealing with clients' feelings and engage clients only on an intellectual level.

The purpose of this last section is to help therapists responsibly manage their own reactions to the evocative material that clients present by learning how to anticipate or recognize their own countertransference propensities, and what to do when this occurs. Further, neither a working alliance nor a Safe Haven will sustain unless therapists are accurately empathic and can communicate their genuine respect and compassion for the affect-laden conflicts their client is struggling with. However, it will be usually difficult for therapists to respond to all of their clients' varying feelings, because all therapists bring their own developmental histories and current life stressors to the session. In their families of origin and their cultural contexts, therapists often learned unspoken rules about which feelings could be expressed, and how, and many were scripted into familial roles that now shape how they respond to their clients. Next, we explore how these personal factors can keep therapists from responding effectively to their clients' emotions.

### FAMILIAL RULES AND ROLES SHAPE COUNTERTRANSFERENCE PROPENSITIES

Most trainees who are drawn to clinical practice are generous people, genuinely concerned about others, and ready to give of themselves. Often nurturing by nature, they can readily empathize with others and easily enter their experience. At the same time, however, *many who choose this work also have strong needs to be liked* (Hayes, Nelson & Fauth, 2015). Perhaps some grew up with too much emphasis on "appearances" or an anxious preoccupation with what others might be thinking. Other therapists grew up with caregivers who were committed and responsible parents but, because of their own upbringing and life experiences, were simply too judgmental or critical. Some had caregivers who cut-off or disengaged emotionally when they disapproved of the child's behavior or, most commonly, the child grew up anxiously striving to win or earn the attention or affection of their well-intended but rigid caregiver. For these and other reasons, such developmental experiences lead to too much need for approval—which makes it anxiety-arousing for this new therapist to explore resistance and ask about what might not have felt good in the session, risk breaking the social rules and venturing a process comment about what may be going on between them, or approaching the client's distressed feelings more directly.

Most new therapists—but especially those with strong needs to be liked—will have great difficulty responding non-defensively to clients who present with a provocative, angry, or challenging interpersonal style, which these clients have been enacting with others in their lives as well. That is, some clients characteristically respond in problematic ways to others in their lives who are critical, confrontational, controlling, competitive, dominating and so forth—and of course, many will begin to replay these same hostile/alienating interpersonal patterns with the therapist, as well. Researchers report that therapists of every theoretical persuasion routinely respond to such "client negativity" personally—with their own counter-therapeutic emotional reactions of anger, criticism,

withdrawal, and, most frequently, a punitive and judgemental response to the client (Binder & Strupp, 1997; Gold & Stricker, 2011). Unfortunately, most clinical training programs do too little to help new therapists learn how to respond non-defensively or constructively to this negativity that trainees are going to encounter. It is especially problematic for therapists who need their clients to like them.

While some therapists grew up overly concerned about approval or being liked, many others fulfilled the role of “caretaker” in their families of origin. Some, for example, are drawn to clinical practice in part by their anxious childhood need and family role to rescue their vulnerable or distressed caregiver, and later others, from a life of alcohol or substance abuse, depression and an unhappy marriage, dependency and a generalized anxiety disorder, and so on. In particular, this common countertransference issue of *parentification* makes it harder for therapists to address clients’ vulnerable feelings of sadness or hurt. Those feelings can evoke for parentified therapists the familiar predicament of needing to protect or take care of their caregiver. Because, in reality, this was always an impossible task for children to succeed with, it threatens them once again with the painful prospect of disappointing their caregiver (and now their client). These therapists often struggle with feelings of inadequacy because they have failed somehow, and their anxiety is evoked in much the same way it was when their attachment ties to their vulnerable caregivers were threatened because they could not rescue or solve the caregivers’s problems.

For these and many other countertransference issues, all therapists need to reflect on the roles they played in their families of origin. *The issue is not whether therapists have their own countertransference tendencies, but how they deal with them.* Every therapist has certain countertransference reactions—it’s just human. Self-examination on our own or with a colleague, with a supervisor, or, when this is not sufficient, with our own therapist, are responsible ways to manage them. What we do not want is for therapists to act on their countertransference propensities. Therapists do this when they try to win the client’s approval by trying to be a “nice” person and make the client like them, do not risk moving beyond the surface and utilize points of entry, or approach difficult feelings, or are reluctant to make process comments to sort out misunderstandings or awkward interactions because the therapist wants to avoid interpersonal conflict. On the other hand, countertransference is also in play when therapists press clients to enter into, disclose, or stay with a certain affect. Instead, therapists repeatedly want to offer invitations or “bids” and allow clients to enter and leave difficult emotions *as they wish*. Rather than press clients to express a feeling they are reluctant to share, explore what the threat or danger may be for them if they did. For example:

- *What does it say or mean about you if you feel \_\_\_\_\_?*
- *What could happen between us, or what could go wrong for you, if you let yourself feel \_\_\_\_\_?*
- *How have important people in your life responded to you in the past when you felt \_\_\_\_\_?*

Letting clients have it their own way avoids the issues of coercion/demand and compliance that *ruled* so many clients in their families of origin, and that people who have been marginalized often have experienced in the dominant culture. Therapists provide an effective middle ground when they work with clients to understand their resistance or reluctance and explore together the threat or danger that once existed. *With understanding comes safety*, and clients soon will be able to explore feelings that needed to be held at bay in the past.



Therapists often assume too much responsibility for their clients' feelings. When this happens, therapists place on themselves the unrealistic expectation that they have to fix or solve the problem, and the usual outcome is for therapists to feel inadequate. Or, based on family rules (e.g., sadness is weak and unacceptable for males in our family) and roles (e.g., mothers have to ignore their feelings and take care of everyone else's feelings), therapists may have learned to be uncomfortable with their own sadness, anger, shame, or other feelings that have been evoked by their clients. When such countertransference is in play, therapists are likely to respond to clients' feelings in ineffective ways, such as:

- *Interpreting what the feelings mean and intellectually distancing themselves*
- *Becoming directive and telling clients what to do*
- *Reassuring clients that everything is OK or will work out all right*
- *Becoming anxious and changing the topic*
- *Falling silent or emotionally withdrawing*
- *Self-disclosing or moving into their own feelings*
- *Diminishing clients by trying to rescue them*
- *Over-identifying with clients and becoming controlling—pressing clients to make some decision or take a particular action in order to truncate the therapist's own unwanted feelings*

If therapists do not shoulder this inappropriate responsibility for causing or fixing clients' feelings, what should they do instead? The therapist's response to clients' feelings is threefold: *to identify, to join, and to affirm*. First, the therapist tries to help clients identify and express more fully the significant feeling they may be having. Second, the therapist's intention is to be empathic or emotionally responsive to clients, so that clients can share their feelings with a concerned other rather than having to experience them alone or have them dismissed, as they often have in the past. Third, the therapist wants to validate clients' feelings, by helping clients understand or contextualize their feelings so they can make sense of why they are experiencing this feeling now in this particular situation (White, 2007). In this way, the therapist will be free to respond effectively to whatever feelings clients present.

Let's take this a step further. Even therapists who are well aware of the pitfalls of assuming inappropriate responsibility for clients' feelings are prone to struggle with this issue when the clients' affect becomes intense. When working with the profound emotions that accompany life-threatening health crises, or trauma and abuse, many new therapists struggle with the feeling that somehow they are responsible for causing the clients' suffering, or need to fix or do something to take away their pain. Most commonly, as a result, they tend to avoid or explain away these intense feelings (Ladany, Friedlander & Nelson, 2005). To prevent this, it is especially important for therapists to consult with a supportive supervisor or colleague to help them manage their own exaggerated feelings of responsibility at times. Otherwise, these countertransference reactions will stop therapists from responding effectively and providing clients with the validation and compassion they need. One of the most important ways supervisors help their supervisees is to explore together and better understand how the supervisees' emotional responses to their clients is affecting the treatment process. Supervisors also help when they provide the Safe Haven experience of containment for their supervisees who are trying, in parallel, to provide this for their clients who are struggling with profound emotional reactions (Falender & Shafransky, 2004).

## DIFFERENTIATING THE THERAPIST'S ISSUES FROM THE CLIENT'S

We have explored a great deal of complex and evocative material in this chapter, and we are going to end by highlighting a final countertransference issue that is especially far-reaching. Early in their training, many new therapists, and virtually all therapists at some points, lose their healthy or appropriate **differentiation** and become over-identified with the client. The first way this occurs is when situational stressors in therapists' own lives keep them from providing Sullivan's "participant/observer" stance and the therapist becomes either too distant from, or overly reactive to, the client.

Focusing first on over-identification, sometimes personal problems in therapists' own lives will be similar to those their clients are experiencing (e.g., experiencing marital conflict or divorce, caring for aging or declining family members, coping with financial setbacks, dealing with parenting and childrearing problems, facing health crises, etc.). When this occurs, some therapists will lose their healthy differentiation and not be able to remain both a participant and an observer in the therapeutic relationship. They lose their "observing self" and get lost in over-identification with the client—becoming only a participating self. This occurs, for example, when the supervisee exclaims to her supervisor, "She's just like me! She finds out she's pregnant, and then her husband just got laid off too!" Although such similarities or parallels in the therapist's and the client's life circumstances will occur sometimes, *very real differences between them also always exist*. For example, just because the client is a White female and is a psychology major like the therapist, their personal histories and other aspects of their lives make them different. If these differences are not kept in mind and differentiation is lost, the therapist is likely to have difficulty approaching clients' affect, does not discern what clients just said, and cannot be accurately empathic. This may not allow clients to stay with the feelings they are experiencing that are uncomfortable for the therapist, and become personally invested in the choices or decisions the client makes.

This lack of differentiation, or difficulty remaining both a participant and an observer with the client, is an expectable or normative problem for new therapists. Most new therapists will struggle with this at times, and it usually gets better with more experience. As seen throughout this chapter, however, supervision is the best place to sort out the very real ways the client's experience is both like and different from the therapist's. With this clarification, the supervisee can return to the next session with a more balanced or optimal interpersonal distance. However, taking this further, what if this doesn't occur only with similar situational problems? What if the new therapist has a more enduring concern about "taking clients' sadness home with me"? Here again, the therapist is struggling countertransferentially with a lack of self-other differentiation. When this situation occurs, commonly, a client's affect—such as sadness, has evoked the therapist's own longstanding personal feeling of sadness. Therapists like this are not being accurately empathic or entering their clients' pain. Instead, they are experiencing their own personal sadness or pain but not recognizing that they are managing it through their clients. Sometimes this can be sorted through in supervision, but often the supervisee's own therapy is the best place to work through this unresolved feeling. Learning to set boundaries between what clients share and how therapists respond will help therapists move from being a participant to being a participant/observer in working with clients.

Therapy is a deeply human and personal interchange. Therapists' own personal qualities and emotional responsiveness are the greatest asset in helping clients change, but they can also be an obstacle at times. Although we have focused on problems stemming from a lack of differentiation here, we prefer to see supervisees working with these

over-identification issues than supervisees who are too distant or removed in the role of therapist, and unwilling to risk entering a relationship and being personally affected by clients. It is easier to help supervisees develop differentiation than it is to foster genuine empathy or a willingness to engage with clients. If therapists are just a distant observer and not emotionally engaged participants, we don't believe the therapeutic relationship will matter much to the client and a reparative relationship or CEE can't occur.

In sum, countertransference reactions will occur for every therapist—the issue is not whether they occur but how they are dealt with (Hayes et al., 2015). When countertransference reactions are in play, therapists should consult with a colleague or supervisor to understand and manage their own personal reactions. When therapists find that they are repeatedly having difficulty with the same type of affect (such as shame, anger, or sadness), or when supervision does not help or free them up to respond effectively with more neutrality, therapists are encouraged to seek treatment for themselves. *Because change is predicated on the relationship therapists create with clients, therapists are encouraged to make a lifelong commitment to working on themselves and their own personal development.* Without this non-defensive openness to the possibility that their own countertransference issues may be activated, and ongoing willingness to work on their own personal reactions to clients, the help therapists can provide will be limited (Bromberg, 2006; Robbins & Jolkovski, 1987). Furthermore, therapists who are unwilling to acknowledge or work on persistent countertransference reactions are those most likely to have a negative therapeutic impact on their clients (e.g., “It's the client's problem—it doesn't have anything to do with me”). It is a privilege to be able to help people change, and therapists honor that privilege by acknowledging their own limitations and personal involvement in the therapeutic process.

## CLOSING

An essential component of facilitating change is to help clients resolve difficult emotions by integrating or coming to terms with feelings that have been too painful, shameful, or unacceptable to tolerate in the past. Many clients have experienced tragedies in their lives, and painful feelings will be entwined with the symptoms and problems they present. The therapist helps clients resolve their problems by providing a Safe Haven and containment for feelings that others have not been able to understand, accept, or encompass. Perhaps the most important relearning occurs when clients risk sharing certain feelings and find that, this time, they do not receive the same problematic response they have come to expect. A CEE occurs as old expectations and unwanted relational patterns are disconfirmed. However, therapists also need to be aware that the biggest obstacle to providing clients with a relationship that can produce change will be their own discomfort with certain feelings that are based on rules and roles learned in their families of origin. Thus, therapists are encouraged to make an ongoing commitment to working with their own countertransference propensities and how they influence their work with clients.

What's ahead? In the next three chapters we will focus on case conceptualization and treatment planning, and see how therapists can clarify clients' core problems and better understand the key issues that need to change. To do this, we will learn more about how clients' problems originally developed, and how they are being enacted now with the therapist and others. For now, this chapter has presented a great deal of complex and personally evocative information that often takes several years to integrate and make your own. This is challenging work—be patient with yourself and find help when you need it.

## CHAPTER EXERCISE

Which feeling is most difficult for you to approach in your clients? What makes this feeling difficult to approach? How was this feeling responded to in your own family of origin?

## SUGGESTED READINGS

To help student therapists respond to clients' feelings, see especially Chapters 9 and 10 of Clara Hill's *Helping Skills*, 4/E, (2014). Washington, D.C.: American Psychological Association. More experienced readers are encouraged to read the classic in the field: Greenberg, L.S. (2002) *Emotion-Focused Therapy*. Washington, D.C.: American Psychological Association.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes. Also included with this chapter is a video demonstration by the authors in which they role-play a therapist responding to each feeling in a client's affective constellation and then debrief by discussing each of the three feelings the client transitions through in this sequence.

# PART III

## CLARIFYING THE CLIENT'S PROBLEM AND DEVELOPING A TREATMENT FOCUS

### CHAPTER SIX

Familial and Developmental Factors

### CHAPTER SEVEN

Inflexible Interpersonal Coping Strategies

### CHAPTER EIGHT

Relational Themes and Reparative Experiences







## CHAPTER 6

# Familial and Developmental Factors

### CONCEPTUAL OVERVIEW

The previous chapters in Part I and Part II focused on the interaction between the therapist and the client, and provided guidelines for working with the process dimension. In Part III, the next three chapters will explore more fully how therapists can better clarify or conceptualize their clients' problems and develop a more specific focus for treatment. We are trying to become more specific in our thinking about just what the central problem really is for a particular client, and clarify the focus for treatment that will provide the most help. Therapists are more effective when they can formulate an accurate assessment of the problem or case conceptualization that leads to specific treatment plans (Levensen, 2010, Chapter 4). To do this, therapists seek to understand:

- *How clients' problems originally developed* (Chapter 6);
- *How these developmental problems are being played out now in problematic interactions with others and amplifying current symptoms and problems that clients are bringing to treatment* (Chapter 7); and
- *How clients' problems with others are brought into the therapeutic relationship and played out along the process dimension or way in which the therapist and client interact together* (Chapter 8).

The more specifically therapists can clarify their thinking and understand their clients in these three ways, the easier it will be to formulate the **treatment focus** therapists need to help clients change. We find, however, that new therapists have had little training or experience conceptualizing clients, and this is the most undeveloped and challenging new skill for most student therapists. Early in their training, it is difficult to clarify a focus to guide treatment—that is, *to identify the core problem and understand where they are trying to go and what they are trying to accomplish or do with each client*. To help, guidelines for meeting these professional demands are provided in the next three chapters.

In this Chapter, we will see that one important component for helping therapists conceptualize clients and formulate treatment plans is to recognize the familial, cultural, and developmental antecedents of clients' problems. In particular, therapists earn credibility with their clients, and help them engage in treatment, when they can provide empathic

understanding that helps clients grasp why certain situational problems or seemingly modest stressors can become amplified and engender more significant symptoms or distress than the current situation seems to warrant. We value and utilize a developmental perspective to shed light and help us better *understand* current problems and maladaptive relational patterns, rather than to gain psychodynamic insight. Although we are exploring and trying to assess what has gone well and what has been problematic in family interactions, we do not seek to blame or “bash” parents, or make caregivers responsible for clients’ own lives and choices, as is so commonly misunderstood. We begin with the rapidly expanding literature that utilizes attachment theory to inform clinical practice.

## ATTACHMENT STYLE AND CLINICAL PRESENTATION IN ADULT TREATMENT

In previous chapters we have introduced basic constructs in attachment theory such as attachment styles in children, Safe Haven/Secure Base, affect regulation, reflective capacity, hyperactivating and deactivating coping strategies, and attuned responsiveness. A rich empirical literature is now linking attachment theory to clinical research and practice with adult clients and helping therapists understand therapeutic relationships (for research reviews see Cassidy & Shaver, 2008; Daniel, 2006; Gold, 2011; Obegi & Berant, 2009; Shaver & Mikulincer, 2010). John Bowlby launched a generation of research in 1988 when he wrote for the first time about therapists serving as reparative attachment figures—providing many clients with a relationship, for the first time, that offered the Safe Haven and Secure Base experiences they had missed developmentally. In this way, he viewed the therapeutic relationship as having the potential to be a far-reaching and corrective attachment relationship, rather than merely a teacher-student, coach-trainee, or instructional relationship (1988). An important caveat: this reparative relationship is not “re-parenting” and does not give clients what they missed emotionally from their caregivers. Instead, it helps them develop a more realistic narrative for their lives, develop self-compassion for what they wanted and did not receive developmentally, expand rigid IWMs that are limiting and disrupting current relationships, and develop the skill and capacity to establish new and more satisfying relationships in their current lives.

With an integrative approach, psychotherapy researchers and practitioners from every brand of treatment are drawing on strong empirical support for utilizing attachment theory to guide treatment and provide a foundation for therapeutic relationships (Mallinckrodt, 2010). For example, researchers find that establishing a secure relationship with the client first will then allow behavioral, cognitive, and other active interventions to become more effective (Connors, 2011; Gold, 2011). More far-reaching, many posit that effective therapists across diverse treatment approaches are working in the first stage of treatment to develop Security—both a Safe Haven and a Secure Base—within the therapist-client relationship. In the second stage of treatment, therapists are utilizing this CEE, and the new relational skills developed from establishing it, to transfer this experiential relearning and expanded social skills with the therapist to others in their daily lives (Mallinckrodt, 2015).

In this way, the therapist and client are working to develop Security within the client-therapist relationship. Fostering change via this “earned security” occurs as the therapist *consistently* facilitates (1) CEEs that provide the experience of a Safe Haven when the client is vulnerable and distressed (this is not a one-time, dramatic, or peak experience but a dependable way of relating that characterizes their ongoing interaction), and (2) a Secure Base that sustains a watchful eye for points of entry to support the client’s exploration and autonomy, recognizes and highlights their personal strengths, and celebrates their successes (Feeney & Van Vleet, 2010; Mallinckrodt & Joeng, 2015;

Sroufe, 2005). Having first struggled to achieve some measure of security in the therapist-client relationship with individuals who usually begin treatment with limited experience and capacity for establishing relationships with these adaptive features, we are going to see that treatment then often enters a second (and usually easier) phase. Here, the therapist draws on the client's experiences of change; to help the client understand that some people can respond to the client in different and better ways than before. Having *experientially* disconfirmed the schema or expanded the IWM in the client's real-life relationship, the therapist works actively to help the client transfer or generalize these new and more effective ways of relating to others. In this way, multiple sequences of CEEs occurring in and out of treatment contribute to successful outcomes (Holtforth & Fluckiger, 2012). This integrative and experiential model of change is the relational treatment approach developed and illustrated throughout.

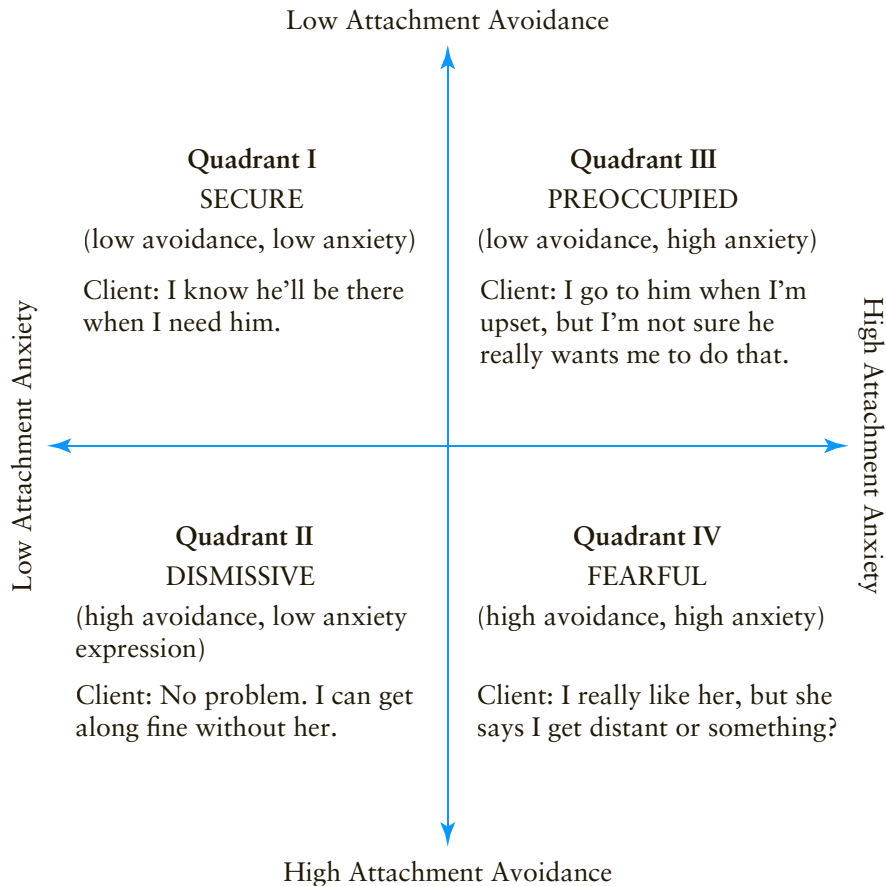
We begin this chapter with the research on four categories of adult attachment. This discussion will (a) describe how these four different types of adult clients will present in treatment and interact with the therapist, and (b) provide guidelines for developing a treatment focus and intervening with each. Be patient as you begin reading—complex ideas need to be introduced before we can illustrate how they can be applied to help with your current clients.

#### FOUR CATEGORIES OF ATTACHMENT

Following research by Bartholomew & Horowitz (1991), Mikulincer and Shaver (2007; 2008) and others, adult attachment researchers measure **adult attachment styles** along two orthogonal dimensions of *Anxiety* and *Avoidance*. Fear is the “privileged” or primary emotion that attachment-informed therapists are attending to in their clients (Slade, 2004). Parallel to children, when adults feel threatened, vulnerable, or distressed, they tend to seek comfort or support from their current, adult attachment relationship(s). Anxiety over the security of attachment relationships refers to worry or concern about the availability and willingness of our significant others to recognize and respond when we are injured or sick, threatened or vulnerable, facing loss, or have suffered trauma (Janzen, Fitzpatrick & Drapeau, 2008; Shevlin, Boyda, Elklit, & Murphy, 2014). That is, clients can be rated high or low on a continuum of *Anxiety* over the availability or responsiveness of their “go-to” person or significant other.

The other measured dimension is *Avoidance* of attachment relationships when distressed. This refers to a continuum of comfort versus discomfort with intimacy or closeness, emotional needs, and commitment in relationships. Individuals rated high on Avoidance see themselves as self-sufficient, view others as unavailable or undependable, and fear being excessively or arbitrarily controlled. This leads to an avoidance of both one's own—and others'—feelings and needs. These individuals avoid sharing with others and emotional engagement, as well as subjectively or privately experiencing their own emotional vulnerability or distress (Mallinckrodt, 2010; Marmarosh, Kivlighan, Bieri, et al., 2014). The intersection of these two dimensions of Anxiety and Avoidance portrays four adult attachment prototypes: Secure, and three types of insecure attachment: Preoccupied, Dismissive, and Fearful.

As seen in Figure 6-1, adult clients with **Secure** attachment styles are low in anxiety about seeking help and understanding from others, and low in avoidance of having emotional needs of others and relationships. When distressed, these adaptive adults seek help comfortably from well-chosen go-to people in their lives. **Dismissive** clients are low in *overt* or expressed attachment anxiety about needing others (although they experience high physiological distress or cortisol activation when their needs come into play in relationships), and high in avoidance of approaching others when distressed. Dismissive clients do not easily seek support when distressed, and cut off from or avoid their own



**FIGURE 6.1** | ATTACHMENT STYLES IN ADULTHOOD

and other's feelings. **Preoccupied** adults are high in anxiety about seeking support and understanding from others, but are low in avoidance of expressing this need. These clients seek support when distressed, but are anxious that the attachment figure will not sustain interest in them or remain sufficiently responsive. Finally, **Fearful** adults are high on both anxiety about expressing their needs for support and avoid and are fearful of being vulnerable in attachment relationships. That is, they show intensely conflicted approach-avoidance. They want relationships, but often because of abuse histories, approaching others evokes unwanted or frightening feelings and memories about their unresolved trauma or loss, so they simultaneously avoid these relationships. The two-dimensional schema in Figure 6-1 is adapted from Bartholomew & Horowitz (1991). We will use it as an orienting compass to explore the four clinical subtypes that new therapists are going to quickly recognize in their caseloads.

### SECURE ATTACHMENT STYLE IN ADULT CLIENTS (QUADRANT I: LOW AVOIDANCE/LOW ANXIETY, P. 186)

Researchers find that *Secure clients* value close attachment relationships, and provide consistent memories and descriptions of their childhoods in undefended ways. These coherent "attachment narratives," which the other three attachment styles do not provide, are based on having received a Safe Haven and Secure Base from their caregivers, which leads to a more integrated identity or coherent sense of Self. In their Adult Attachment Interviews (AAI), which are the most widely used measure of adult attachment styles,



Secure clients discuss relationships clearly and openly, exhibit reflectiveness (i.e., the adaptive capacity to think flexibly about their own—and other’s—thoughts, feelings, and intentions) and show empathy (See Bretherton & Munholland, 2008). Secure adults acknowledge the impact of childhood experiences realistically and, when these experiences were not positive, they are not denied or glossed over, as Dismissive clients will do as they enter treatment. In this way, Secure clients assess reality more accurately. They don’t engage in idealization of caregivers (or their therapists), or denial of real problems that currently exist or actually existed in the past—which we will see is prevalent and maladaptive in the other three types of Insecure clients.

Sometimes misunderstood (especially by Dismissive individuals), Secure adults were not indulged or over-protected in childhood. Their attachment needs were simply met in a way that has helped them become more resilient and autonomous in adulthood. Their responsive emotional environment included attuned responsiveness. That is, empathically seeing and validating the child’s experience—and responsively helping to resolve the distress or problem as necessary, has provided the opportunity for the child to remain connected to his or her own experience and know him or herself. It has also helped the child to develop the significant psychological strengths of emotional self-regulation, impulse control, empathy, and agency (Allen, 2012; Fonagy & Target, 1997). Although secure individuals will seek treatment at times, they possess a positive IWM of Self as being good or worthy of care, and that some carefully filtered Others are positive or trustworthy (Agrawal, Gunderson, Bjarne, et al., 2004; Bowlby, 1969).

Researchers find that secure adults are more efficacious, more flexible cognitively and interpersonally, and more resilient—better capable of dealing with stress than the other three attachment styles. Thus, when secure adults enter therapy, they are less conflicted about having a problem and seeking help, and are more able to work collaboratively with the therapist. Meta-analyses find that Secure clients have the capacity to establish a working alliance, possess the psychological resources to enter into exploration of their problems with more self-disclosure, feeling, and depth, and are more capable than the other three subtypes to risk exploring ruptures and negative reactions toward the therapist (Brenecker, Levy, & Ellison, 2014; Diener & Monroe, 2011; Romano, Fitzpatrick, & Janzen, 2008; Rubino et al., 2000).

Perhaps capturing best the adaptiveness of having a secure attachment style, researchers find that *secure adults are effective in finding trustworthy social supports when needed and, especially meaningful, are usually able to avoid, or readily terminate, maladaptive relationships* (Mallinckrodt, 2000). So, if one didn’t have the good fortune to grow up securely, are these health features unattainable? No, attachment styles do have significant continuity, but changes also occur in some individuals over time (Lewis, Feiring, & Rosenthal, 2000). Those who did not have secure attachments as children, but become secure as adolescents or adults, have addressed their problematic experiences realistically and worked through them. These individuals have attained the goal we hold for many clients in treatment—achieving or earning security (**earned security**). This has been achieved through subsequent reparative relationships with others in their lives (such as therapists, extended family members, or other corrective relationships) that provided the responsiveness they needed. These individuals have “reprocessed” and made peace with their challenging childhood experiences (Jacobvitz, 2008; Schore, 2014).

Thus, as noted in Figure 6-1 (on page 186), the Secure client in Quadrant I is low on Avoidance and low on Anxiety. When distressed, these clients do not avoid relationships or their emotional needs. Rather, without being dependent, demanding, or needy, they appropriately seek out help or understanding in healthy adult ways from a few carefully selected significant others who have proven trustworthy over time. They do this without anxiety or excessive worry about being controlled, rejected, or abandoned, or other threats that the other subtypes will exhibit. In this regard, research finds that accepting a

degree of dependence and allowing oneself to be comforted and supported by dependable others when distressed, paradoxically, allows a person to become more autonomous and self-confident (Feeney & Van Vleet, 2010; Mallinckrodt, 2015 ). When working with Secure clients, therapists will hear prototypic sentences like these that capture a felt sense of trust, safety, and security:

- *I know that he will be there when I need him.*
- *I can trust that she won't hurt my feelings when I get close to her.*
- *He is able to comfort me when I'm distressed.*
- *I enjoy it when she gets emotionally close to me because I feel close to her.*

The adult attachment categories that follow reflect the three different subtypes of Insecure attachment (Dismissive, Preoccupied, and Fearful). Recalling affect regulation from Chapter 5, one of the key concepts to help therapists understand the symptoms, defenses, and clinical presentation that these three types of Insecure clients present is the *Hyperactivating and Deactivating strategies to cope with feelings when distressed*. Depending on their differing attachment histories and subsequent relational experiences, Insecure clients have learned in formative relationships that their attachment needs will not be met and that they need to adopt these defensive coping strategies to regulate their affect (Daly & Mallinckrodt, 2009).

Clients who use a hyperactivating strategy usually have an IWM of Self as negative—being fundamentally unlovable and unworthy of others' love and responsiveness—and a more positive but often inaccurate or unrealistic IWM of Others. In contrast, those with a **deactivating** strategy usually have an IWM of Self that is *unrealistically* positive and competent, and a more negative and inaccurate IWM of Others as hostile and competitive, unavailable, or incompetent/incapable to respond effectively. Those who are Fearful alternate between using a hyperactivating and a deactivating strategy, which is confusing to themselves, to others in their lives, and often to their therapists. They are fearful of the person they want and need to go to in time of distress (the attachment figure who sometimes helped them but at other times hurt them in significant ways). So, as they start to approach someone they almost simultaneously begin to retreat and avoid—and lead very difficult lives as they cycle through this approach-avoid loop repeatedly. Poignantly, they have an IWM of Self as negative and Others as negative, and do not have the stable and more adaptive coping strategy to cope with anxiety that the Dismissive and Preoccupied client can utilize.

## DISMISSIVE ATTACHMENT STYLE IN ADULT CLIENTS (QUADRANT II: HIGH AVOIDANCE/LOW ANXIETY EXPRESSION P. 186)

Let's begin with a clinical vignette that describes a prototypic client with a Dismissive attachment style. This vignette, and the one to follow in the next section on Preoccupied clients, is reprinted from a highly informative study on treatment planning and case conceptualization for clients with differing attachment styles (Daly & Mallinckrodt, 2009).

*R is seeking counseling for relationship difficulties. R is very uncomfortable opening up to others on a deep personal level and finds it equally uncomfortable when others begin to get too close, revealing more personal emotions. This is causing problems in relationships because R tends to back away when relationships reach a certain level of intimacy. R feels more comfortable in relationships where there is some distance and it isn't necessary to depend and rely on the other person involved in the relationship. It actually makes R nervous to think about having to depend on another. It is difficult for R to rely on others, and relationships become draining quickly if partners begin to insist that their concerns and needs be met in the relationship. R admits that this pattern of relating to others has evoked problems in past relationships and usually results in R being driven away by demands of a partner that seem overwhelming.*

In contrast to Secure clients, those who are dismissive use a deactivating strategy and, as noted earlier, have an unrealistically high IWM of Self as positive and competent, and inaccurately hold a negative model of Others (under stress, that is, the Dismissive client will characteristically *distort* the Other as being controlling, hostile, intrusive, or undependable). Their learning histories have taught them that other people do not offer help very often or well. As young children, their caregivers consistently ignored and rejected their overtures and withdrew when they showed distress—and this pattern typically continued throughout their development (Ainsworth et al., 1978). Their internal working model of Others leads them to readily slot people as being potentially harmful or hostile, weak or unreliable, or intrusive, demanding, and controlling because this is what they have routinely experienced, year after year, in primary attachment relationships. Understandably, this history results in the fixed perception that seeking closeness with others is futile, and that it reflects personal weakness to ever have or approach others with a need. Seeking support when distressed can be injurious because it will allow others to undermine their autonomy and arbitrarily control them, or to ignore or even reject them. For a client with this developmental history of emotional deprivation and painful aloneness, seeking closeness or sustained intimacy is threatening, indeed. Therefore, minimizing or denying any need of anyone is the safest coping strategy—they expect rejection and *have learned to modulate their distress by ignoring and not even recognizing it* (Main, 1995).

Dismissive clients inflate their self-esteem and competence unrealistically in response to threat, and also minimize or deny real dangers that actually exist—these clients do not assess the environment accurately (Mikulincer et al., 2003). Further, just as they will not register or be very aware of their own fear or distress in current threatening situations, they also will divert their attention away from thoughts or memories about disappointing or hurtful experiences with their attachment figures (Mallinckrodt, 2010; Mikulincer & Shaver, 2007). Thus, we are going to see that Dismissive clients (the adult version of the Avoidant child) damp down or minimize their anxiety and distress with a deactivating strategy. And, in turn, tend to show less sensitivity, empathy and nurturance toward others and are averse to intimacy (Simpson & Rholes, 2010).

Early in treatment, it doesn't usually work well for therapists to ask very directly or often about Dismissive clients' childhood or familial experience. These clients may be unwilling to share, and often are unable to remember specific aspects of their childhoods, saying:

**Dismissive Client:** My parents...my family...I don't know—I don't remember much about growing up...

When they do speak about developmental and familial experiences, they will usually present their childhoods in overly ideal or positive terms or, occasionally, a few may be overtly derogating. Consider these responses:

**Dismissive Client:** I had a great family—yeah, everything was just great.

OR

**Dismissive Client:** I didn't need them—they were useless. I managed just fine on my own.

When the Dismissive client does briefly allude to childhood experience, however, it will readily become apparent to the therapist that everything really wasn't "great." *Therapists need to pay close attention to the details or behavioral specifics of what others actually said and did when these clients communicated to a caregiver that they were distressed or had a problem.* Typically, careful listening reveals a different and painful narrative that caregivers were unresponsive, overtly rejecting, or sometimes even physically abusive. For Dismissive clients, however, these painful, shaming, and anxiety-arousing experiences

are held away or defended against by being cognitively and affectively minimized or dismissed. This may serve as a point of entry for therapists to explore the behavioral specifics of how others responded—what was actually said and done. For example:

**THERAPIST:** What was the tone of voice, or expression on her face, as you were telling her that?

Like the Avoidant children they often once were, Dismissive clients fundamentally do not trust others. Their experience repeatedly has been that emotional or social support will not be available when they need it (Connors, 1997, 2011). Therefore, the defensive interpersonal presentation is usually one of pseudo-independence or complete self-sufficiency, always appearing “strong” and never “weak.” Many of these clients will be seeking dominance and control in relationships. The mantra their therapist will hear about almost any situation is, “No problem.” These painful developmental experiences, that consistently received no validation, compassion, or assistance from caregivers, are the basis for their signature detachment from their own feelings, blunted empathy for the experience and feeling of others, and their limited capacity for emotional closeness and commitment in relationships (Mikulincer & Shaver, 2007). However, it is also the basis for the central importance of the therapist’s empathy and willingness to continue working to engage these clients in a new and different way. The central theme for these counter-dependent clients is that sharing vulnerability with anyone or expressing a need (or even just internally or privately experiencing their own pain or distress in response to highly stressful circumstances) is unacceptable and seen as being shamefully weak. These clients are often initially resistant to treatment and commonly refuse help when it is offered (Dozier, 1990; Dozier, Lomax, Tyrrell, & Lee, 2001).

If you are seeing a Dismissive client in therapy, and you are not dismissive yourself, you may initially feel bored with him or her in sessions. Of course, just as these clients avoid closeness in relationships and look down on reliance on others, they will minimize the significance of the therapist as well (which is challenging for most new therapists, and for Preoccupied therapists, who need and seek approval). Dismissive clients minimize the therapist’s importance by missing sessions, coming late, and generally just not allowing the therapist to become someone who matters much. They will keep therapy superficial by avoiding pain and real exploration of problems, remaining “compulsively independent” and not utilize the therapist as a source of support or for exploration in treatment (Mikulincer & Shaver, 2007).

Therapists should not give up on these non-responsive, difficult-to-engage clients when they do not talk much about themselves, may be intellectualizing or speak in global or terse ways that communicate little, and are reluctant to disclose or share feelings. We can help by joining them where they are—letting them have the interpersonal distance they need in the beginning of treatment, but watching for points of entry where we can respond when they bring up seemingly small problems, disappointments or dissatisfactions, or when any slight wish or motivation for something to change is voiced. Therapists generally do not want to adopt a directive stance, assign homework assignments, press clients to share feelings or disclose further, and so forth. Despite the therapist’s best intentions, Dismissive clients are likely to regard such responses, which may work well with other clients, as familiar, expectable, and unwanted control. Many Dismissive clients are oriented to attaining power and control in relationships and, in turn, they will expect the therapist to be dominating and controlling (Bartholomew & Horowitz, 1991; Chen & Mallinckrodt, 2002). Their competitiveness and orientation to power and control hierarchy is often difficult for new therapists, who will need support from their supervisors. These clients sometimes seek to gain the upper hand in interactions with the therapist or to diminish or dismiss the therapist, just as they do with spouses, partners, coworkers, and others in their lives.

So, with all of this counter-dependency, what would bring dismissive individuals to treatment? Their partner/spouse may be threatening to leave them because they cannot be intimate or responsive to the feelings and concerns of their partner or make a commitment to the relationship (i.e., keeping themselves “independent” and not controlled). Their spouse/partner also may be threatening to leave them unless they enter treatment because they are being too controlling, dominating, or possessive in their relationship (Levine & Heller, 2010). Additionally, we find that Dismissive clients need help grieving successfully. They struggle when facing loss—sadness is simply an unacceptable “weak” feeling. However, when a parent or loved one dies, their feelings of loss can be too intense and topple their deactivating defenses. Amplifying their predicament, their childhood feelings of deprivation—what they wanted but did not receive—often emerge when parents or other attachment figures die, leading some to enter treatment at this point. Research also suggests that affective disorders such as depression may be seen in individuals with a dismissive style (Fonagy et al., 1996; Patrick et al., 1994; Tyrrell, Dozier, Teague, & Fallot, 1999). This makes sense in that they will not seek support during times of distress. Additionally, while they invest in work and may be successful professionally, when stressed they are vulnerable to alcohol abuse and addictive disorders—a coping strategy that is non-relational (Connors, 1997).

When these or other problems lead them to therapy, Dismissive clients will still want to focus on others’ problems rather than their own at the beginning of treatment. These clients will not want to use the therapist to work on themselves—that is, they will not look within and explore with much depth, and want to externalize rather than consider their own contribution to problems. Thus, therapists should anticipate that they often will be feeling a strong pull to join in blaming or criticizing others. Researchers find this is especially problematic when the therapist shares a complementary dismissive attachment style and too readily settles for the surface dialogue and externalization of problems that is being elicited (Tyrrell, et al., 1999). This poor therapist-client match makes sense because these clients deny their need for help and their therapists remain in their comfort zone by joining in this defense and avoiding real engagement and pain (Dozier, Lomax, Tyrrell, & Lee, 2001). However, we will see that treatment can also fail if the therapist goes too far the other way and presses too hard for closer personal engagement, and does not allow Dismissive clients to have the dismissive defense they need and allow them to be as distant as they need when they initially enter treatment.

Within clients who fit a dismissive category—and within each of the other two Insecure attachment styles to follow—therapists will meet a wide range of differing individuals. Therapists do not want to compartmentalize and respond to their client as merely a “Dismissive client,” of course, but first and foremost as a unique person with diverse qualities and experiences that we want to get to know. Like any classification system, these attachment features will help therapists form working hypotheses and clarify important aspects of their functioning. However, putting this client into a dismissive box, or any other classification system, will never really capture the full person. Attempting to do so, ironically, will actually be a dismissive response itself! With this caveat in mind, the extent or severity of the client’s dismissiveness will vary widely along a continuum of severity. For example, therapists are going to see high-functioning clients, like “R” in our introductory vignette, who cope well in many arenas of life but not with intimacy or in committed relationships. In particular, mid-range dismissiveness includes many self-centered clients with Narcissistic features, as well as some with Obsessive-Compulsive and even Paranoid features. Moving further toward more severe psychopathology, a dismissive state of mind is often associated with Antisocial Personality Disorder and criminality (Allen, 2012; Allen et al., 1996; Mikulincer & Shaver, 2008). Additionally, Dismissive clients will often be described as having Authoritarian personalities and, as we will see in the next section, be authoritarian parents.



Looking back at Figure 6-1 on page 186, the Dismissive client in Quadrant 2 is high on *Avoidance* (i.e., the client avoids his or her own emotional needs and avoids seeking help from others) and low on *Anxiety* (i.e., *he or she does not present with OVERT anxiety or concern when faced with real problems, although physiological measures of distress are measured*). They are out of touch with fear and other vulnerable feelings. In this way, these clients are blocking out their own, and others', attachment thoughts and feelings. When you are working with Dismissive clients, you are going to hear prototypic sentences like these:

- *I don't care if she doesn't love or want me.*
- *I don't tell him when I'm upset because I take care of my feelings myself.*
- *I'm used to doing things on my own, so I don't ask for help.*
- *I can get along just fine without him.*
- *No problem. Everything's fine.*

In response to these prototypic comments, it's not so useful to ask Dismissive clients, "How do you feel?" As treatment progresses, they can do more with this invitation, but not in the beginning. Instead, therapists are encouraged to be listening for points of entry where any conflict, need, or distress is even minimally expressed, and then trying to make modest empathic bids, such as:

- *It seems as though you've had to do a lot on your own.*
- *I'm wondering if, just sometimes, it might have been a little hard for you to be so alone with your problems?*
- *Maybe others tried their best, but haven't always been able to be so helpful to you, or responded quite in the ways that might have worked best for you?*

Why are empathic bids—small but consistent attempts to provide empathic understanding—prioritized in this way? After all, these clients are not going to welcome or accept them initially. They are going to dismiss them, and sometimes the therapist as well. Therapists want to be mindful to not respond counter-transferentially—that is, to take *personally* the client's dismissive rejection of the empathic bid or dismissal and respond defensively with criticism, judgement, withdrawal and so forth (as others in the client's life have done). Instead, a new and different response with this client is to stay steady, and continue to look for points of entry when the therapist can venture another modest empathic bid, or tentatively venture a process comment and inquire about what others tend to do when the client responds in this dismissing way.

Why is this intervention plan important or more effective? The primary treatment goal with Dismissive clients is to provide a CEE by repeatedly facilitating the Safe Haven experiences they missed. Typically, these clients had better developmental experiences with a Secure Base (exploring on their own with few supports and "miscuing" their caretakers about their *seeming* lack of need for help). That is, they defensively conveyed to their caretaker they didn't need help when they actually did, and pretended they could handle everything on their own. An example would be a student going off to college and needing help from a caretaker but, anticipating rejection or lack of assistance or a non-response, communicated that he or she can handle it alone. As children, Dismissive clients were too alone with their feelings when fear and pain was evoked (they lacked a Safe Haven). So, learning that their caretakers were consistently unresponsive to their emotional distress, —they managed on their own and conveyed that they didn't need help (Hoffman, Cooper & Powell, 2013).

Providing such empathy is not just a nice thing. It goes right to the core of Dismissive clients' central problem: the lack of attunement, validation, or empathic understanding that once was so profoundly lacking. But now, sensing when these clients are distressed or have a problem, therapists will actually "hear" it and consistently join these clients in a validating and supportive way—perhaps for the first time in their lives. This provides a CEE that, if ongoing, will expand Dismissive clients' rigid IWMs and limited interpersonal capacities.



Therapists will be most helpful when they can: provide clients a place where they can develop trust by finding empathy and understanding; explore what clients need and clarify how their attachment experiences inform their current problems; help clients identify when and how they are being dismissing toward themselves and others; and help clients change their dismissive behavior by rehearsing and practicing together new and better ways of responding. Dismissive clients are ready for termination when they are able to be more emotionally engaged and capable of expressing a need; first, with their therapists and then, with a few carefully selected others in their lives.

### PREOCCUPIED ATTACHMENT STYLE IN ADULT CLIENTS (QUADRANT III: LOW AVOIDANCE/HIGH ANXIETY)

As before, let's begin by reading a clinical vignette that describes a prototypic client with a Preoccupied attachment style (also reprinted from Daly & Mallinckrodt, 2009).

*J is seeking counseling for relationship concerns. J has always feared abandonment in relationships and usually experiences a lot of anxiety even at the slightest sign of rejection or loss. In fact, J has pretty much been in a relationship since adolescence and often feels insecure and unsettled when not involved in a romantic relationship. J is beset by constant worries of being alone. J constantly has the feeling of not getting enough from partners and tries to get closer and closer until the point of being completely merged with the other person. J becomes angry and frustrated when partners do not show enough interest, are unavailable, or disregard J's emotional needs. J resents it when partners desire time apart. J invests everything in relationships and just wishes partners would provide the same commitment and reassurance that J needs and is willing to give. However, J admits that the continuous desire to be close and connected in relationships often scares and drives others away.*

The defining feature for a Preoccupied client is a hyperactivating style with an *exaggeration* of emotional needs. These highly anxious clients often are suffering significantly in their daily lives. They are highly insecure about the availability or responsiveness (i.e., the Safe Haven) of their relational ties. In treatment, they frequently will be worrying about being left or unwanted, and anxiously pursue closeness with others in order to evoke as much attention, responsiveness, and reassurance as possible. Therapists can readily observe that Preoccupied clients will: magnify expressions of distress (often creating drama in their lives for the purpose of eliciting reassurance or as a strategy to reestablish closeness with their partner); be vigilant and over-reactive to potential signs that the therapist—like their romantic partners and significant others—is about to leave them or become unavailable; and can be clinging or demanding of further time and attention from the therapist and others (Brennan, Clark & Shaver, 1998; Mikulincer & Shaver, 2007).

Adult clients who are preoccupied tend to be entangled in angry, idealizing, or worrisome preoccupations about others in current and past relationships. Also many are characterized by a rambling style of speech that goes on and on and are seemingly unable to finish a sentence or complete a thought (Dozier, Stovall-McClough, & Albus, 2008). They are often preoccupied with whether their significant others will be reliably available and will not leave them or otherwise disappoint or let them down. Typical statements can include:

**PREOCCUPIED CLIENT:** Why won't he text me back? I haven't heard from him since lunch!

OR

**PREOCCUPIED CLIENT:** I just can't stop thinking that she might be cheating on me!

These clients' anxious preoccupation with issues related to the dependability or availability of their significant others is so all-encompassing that *they don't seem to have room in their minds for their own mind*—their own thoughts, goals, and interests (Wallin,

2007). Worries and concerns about what is going on with the other person and what he or she may be thinking or feeling about them dominates these clients' subjective experience, often leaving them feeling overwhelmed.

In addition, Preoccupied individuals lack self-confidence and doubt their own abilities. They perceive themselves as weak and believe neither in their own capacities to cope nor in others' ability to help in a trustworthy and reliable way (Mikulincer et al., 2003). They also see themselves as vulnerable and unlovable. This neediness in Preoccupied clients comes from having caretakers who were often self-absorbed or preoccupied themselves. As a result of their own self-preoccupation, these caregivers could not keep their child's "mind in mind." They responded to their child unpredictably or inconsistently, when it crossed their mind, rather than when their child was trying to communicate distress. There is a confusing ambivalence here which serves to keep these clients embroiled or enmeshed with their caregiver. Contrary to the way that the Dismissive client had consistent unavailability—at times the caregiver of the Preoccupied client could be responsive and loving (Ainsworth et al., 1978). This caregiver's self-absorption and inconsistent, unpredictable responsiveness resulted in children exhibiting intense neediness and developing the coping strategy of *maximizing their emotions in an attempt to get through to their caregiver* (Main, 1995).

The cardinal fear for Preoccupied clients is losing relationships, and these abandonment threats readily disrupt Preoccupied clients and consume their lives with drama-laden crises. They will often enter treatment in response to a recent breakup, the inability to end a relationship that is highly problematic, or the inability to get over a relationship that ended long ago. In fact, their relationships often end because of their extreme neediness, jealousy, and dependence on partners (Feeney & Noller, 1990). More than most, Preoccupied clients are often self-absorbed and vacillate between a self-centered obliviousness toward others and, at other times, an intrusive and demanding over-involvement with others that elicits the withdrawal they fear. Many Preoccupied clients grew up enmeshed (and often parentified) with an unpredictable parent who was too often caught up in his or her own emotional upheavals to be a reliable Safe Haven and provide containment and affect regulation for the child (Fonagy & Target, 1997). And for some, when the caregiver did initially respond to the child, a confusing role reversal may have subtly transpired where the caregiver's need for a confidant or emotional support, and not the child's need, was actually met.

If you are seeing a Preoccupied client, he or she will often be intense and engaging in the initial session with you. There will often be a high level of communication and emotional expression as part of a strong press to engage and "feel close" with the therapist. These clients readily feel overwhelmed and they can feel overwhelming to the therapist, just as they tend to overwhelm others in their lives. Although they will often engage in considerable self-disclosure, it may be almost too indiscriminant or too unfiltered for someone they have just met—they are "too friendly" (Chen & Mallinckrodt, 2002). During treatment, they will often want to rely heavily on the therapist, just as they want to rely excessively on others. Similarly, these clients will often press the therapist for reassurances rather than joining with the therapist and going to work in earnest on their problems. Because of their insecurity about the therapist's responsiveness as well, they will focus on the therapist (anxiously extending or entertaining to ensure the therapist remains interested and engaged with them) at the expense of their own exploratory work.

In sum, these Preoccupied clients will have overly intense reactions to perceived threats, pervasive fears of abandonment, and paradoxically, even though they are often communicating a high degree of distress and need for reassurance, can simultaneously *resist the help they are actively eliciting* and cannot easily be contained or helped (e.g., Client: "Yes, but..."). Although this doesn't make sense to our supervisees at first, researchers find that Preoccupied clients are harder to treat than Dismissive clients

(Fonagy et al., 2006; Levy, Ellison, Scott, & Bernecker, 2011; McBride et al., 2006). This client, who can seem so appealingly communicative, engaged, and responsive as treatment begins—especially to a Preoccupied therapist—can soon evoke counter-transferential feelings of frustration or incompetence in new therapists who, understandably, have high needs for their clients to find them helpful.

Thus, in Figure 6.1 on page 186, the Preoccupied client in Quadrant III is low on Avoidance and high on Anxiety (i.e., he or she does not avoid emotional needs or avoid seeking help from others, although he or she is overtly anxious and worried when approaching others with problems or concerns). As we have seen, these clients may engage in excessive or inappropriate self-disclosure, lean heavily on others, or share strong emotions with others to be their Safe Haven and Secure Base (Bartholomew & Horowitz, 1991; Chen & Mallinckrodt, 2002). This is the adult version of the Anxious/Ambivalent child. If you are working with a client who is Preoccupied, you are going to hear prototypic sentences like these:

- *I can't get along without him, even though being with him isn't working.*
- *I'm often wondering whether she really cares about me or not.*
- *I often feel really dependent on him for emotional support.*
- *I find it hard to forgive her when she lets me down.*
- *I turn to him when I'm upset, but it doesn't really help me feel much better.*

In response to prototypic statements like these, which Preoccupied clients often make, therapists may find the differentiating, boundary-setting reflection below to be helpful because it goes right to *the core problem of not having room for a mind of one's own*:

**THERAPIST:** Shanika, it's almost like all the people in your life sort of fill up or take over your thoughts so much that there isn't room for your own mind. You know, like you're so caught up in how other people are seeing you, what they might be thinking about you, or if your boyfriend is going to return your call soon enough, that there isn't room in your head for your own thoughts. Why don't we get those others out of your mind for a while and set them aside while you're in here with me? Let's draw an imaginary golden circle around you—a protective boundary that keeps out what others are doing or thinking about you, and let's focus instead on what it is that you are thinking and feeling and wanting. How does that sound to you?

**PREOCCUPIED CLIENT:** *(slow deep breath and relaxing sigh)* Ahhh, I really like that. It makes me feel like I can let down and just breathe .

As within each of the attachment styles, there is a continuum of preoccupation for clients with this insecure attachment style. The neediness and dependency seen in more seriously Preoccupied clients makes it harder for them to use therapy productively (Dozier, et al., 2001; Levy et al., 2011). While it is hard for Dismissive clients to enter therapy, once they do and begin to develop a working alliance, they get to work—perhaps because therapy helps them connect to their missed emotional experiences. In doing so, they become more capable of connecting to others. In contrast, Preoccupied clients enter treatment more easily, but their ongoing cycles with patterns of abandonment fears and anxious pursuit leave them in frequent crises. Seemingly entrenched in their enmeshed relationships, they struggle to step back and reflect on their own feelings or behavior—making it difficult to see and change what is not working for them. Thus, they tend to do less well in therapy than clients with a dismissive style because of the variable reinforcement schedule they grew up with, and their less reflective or perspective-taking capacity—both with themselves and with others. Preoccupied clients also have more symptoms and are more unresolved with regard to their attachment figures, with whom they have such mixed emotions (Agrawal, Gunderson, Holmes, & Lyons-Ruth, 2004; Daniel, 2006; Weston, Nakash, Thomas, & Bradley, 2006). Being highly ambivalent

about a primary attachment figure- having occasional closeness which is also paired with more conflict, seems to leave clients more stuck and makes it harder for them to make progress in treatment (Benjamin, 2006).

Moving out further on a continuum of severity, individuals who are Preoccupied may have more trauma and abuse in their history and under distress may move toward the Fearful category. Also, they often are anxiously preoccupied with anger, especially with their partners for not being sufficiently available and responsive. However, this anger is simultaneously anxiety-arousing for them as it creates their basic fear: driving away the go-to person they feel so in need of. Thus, they often present with high rates of internalizing problems such as affective disorders (e.g., Dysthymia), internalizing forms of anxiety such as generalized anxiety and, most seriously, symptoms or features of Borderline Personality Disorder with confusion and fearfulness (Bernier et al., 2005; Fonagy et al., 1996; Larose & Bernier, 2001; Patrick, Hobson, Castle, Howard, & Maughan, 1994). When relationships end, individuals with a preoccupied style may threaten suicide—perhaps from depression or from dramatic attempts to re-engage a departing partner (Adams, Sheldon-Keller, & West, 1996). Some studies also suggest that the need for approval, their extensive focus on perfectionism, and difficulty regulating affect may lead Preoccupied clients to develop eating disorders (Tasca & Balfour, 2014). In contrast, Dismissive clients are found to exhibit more externalizing problems such as conduct disorders and substance abuse (Lasca, Mitjavila, Ochoa, & Balluerka, 2015).

What sorts of therapists would work well with Preoccupied clients? These clients need therapists who can provide a Secure Base which will allow them to explore their relational patterns, develop more autonomy in relationships where they can have more “voice” or assertiveness, and actively support their differentiation and boundary-setting via instruction and role-playing with the therapist. They also benefit from therapists who can make process comments and talk about their relationship and interaction together. This is especially important as therapists need to be able to inquire about and approach clients’ negative feelings about the therapy they are receiving, respond to therapist-client ruptures—such as “needing more” from the therapist—and be empathic and non-defensive about their contributions to misunderstandings in their relationships with their clients. This is not easy, however, because Preoccupied clients are often pleasing and do not want to risk “offending” and losing the therapist by talking forthrightly about problems or dissatisfactions (Safran, Muran, & Eubanks-Carter, 2011).

Researchers find better outcomes when Preoccupied and Dismissive clients work with Secure therapists because they are more flexible in responding to these differing interpersonal styles. Secure therapists are less likely to get countertransferentially caught up in either attachment style and over-react to feeling disengaged or enmeshed. Further, therapists who can assist both Preoccupied and Dismissive clients develop more reflective capacity will lead to more successful outcomes (Fonagy, Gergely, Jurist, & Target, 2002; Katznelson, 2014). Clients have more success in relationships when they (a) are able to consider their own thoughts, feelings, and reactions flexibly without just acting on them or becoming stuck in just one feeling state, and (b) become more interpersonally aware, empathic, or better able to envision others’ possible mental states in terms of intention (i.e., the ability to consider what the other person might be experiencing, needing, or wanting). To illustrate, effective therapists foster this reflective capacity and help clients adopt this perspective-taking by asking:

**THERAPIST:** What do you think he was intending when he did that?

OR

**THERAPIST:** Yes, that’s one good possibility for what was going on for her in that situation. Can you think of other possibilities that might explain this as well?

Initial research by Dozier and colleagues had suggested that non-complementary client-therapist attachment styles yielded better outcomes. That is, a Dismissive client was thought to do better with a Preoccupied therapist and, in parallel, a Preoccupied client was thought to do better with a Dismissive therapist (Dozier et al., 1994; Tyrrell, 1999). The rationale was that therapists with a non-complementary style address and are responsive to what was missing in these clients' histories (Bernier & Dozier, 2002). The issue of having therapist and client have non-complementary attachment styles for Preoccupied clients suggested that these clients benefit from a therapist who will use a structured approach that does not further entrench them in their emotional preoccupations, while Dismissive clients may benefit from therapists who will challenge them to become more emotionally available and empathic (i.e., if Dismissive therapists can tolerate Preoccupied clients' neediness and dependency, and Preoccupied therapists can manage to "hang on" with Dismissive clients who are hesitant to self-disclose or engage with the therapist).

Recent research, however, does not support this. Instead, the idea of "optimal therapist-client distance" is a better predictor of successful outcomes (Daly & Mallinckrodt, 2009; Marmarosh, 2015). That is, Preoccupied clients need a therapist who is not threatened by or over-reactive to their intense attachment anxiety. Being able to tolerate their need for a strong early connection helps them successfully enter treatment. Having met these clients on their terms, however, the primary treatment goal is to gradually challenge them toward increasing interpersonal distance and autonomy over the course of treatment. Similarly, therapists who can meet clients who are dismissive "where they are" and pace the therapeutic distance—beginning with greater distance, but looking for points of entry when they can make bids to move toward greater vulnerability and emotional engagement at a pace the client can manage—will do best (Mallinckrodt, Choi & Daly, 2015).

How do we know when it is time to consider termination with clients who have a preoccupied style? We know this when clients are less afraid of interpersonal conflict, when they can self-regulate their emotions, create less drama or crises when they have a need, can manage increased therapeutic distance where they are less dependent on the therapist—and subsequently with others—and act more independently rather than being anxiously preoccupied about the availability of others (Daly & Mallinckrodt, 2009).

## **FEARFUL ATTACHMENT STYLE IN ADULT CLIENTS** (QUADRANT IV: HIGH AVOIDANCE/HIGH ANXIETY P. 186)

Some Preoccupied and Dismissive clients may present with serious problems, but their deactivating and hyperactivating coping strategies are "good enough" defenses that allow many of them to function with only modest symptomatology. With the *Fearful* client, in contrast, we see more vacillating coping strategies that are less adaptive. These clients are over-represented among psychiatric clients (Dozier et al., 2008; Shevlin, Boyda, Elkit, & Murphy, 2014). As children, there was no patterned defense structure or stable coping strategy: the "Disorganized" child becomes the Fearful adult who now alternates between the two dismissive and preoccupied coping styles—which frustrates and confuses these clients and others in their lives. Therapists are going to see a wide range of symptoms and contradictory or back-and-forth/approach-avoid behavior in Fearful clients. Main & Hesse (1990) highlight the key dilemma: the caregivers of disorganized children are simultaneously viewed by their child as both a source of security or help at times, while also viewing them, realistically, as a threatening source of danger at other times. Disturbingly, these Disorganized children describe their caregivers as Frightened (e.g., moving away from their young child and saying "D-don't come to me"), Frightening or threatening (e.g., hovering over their child), Dissociative (e.g., talking to the child in a "haunting" voice), Sexual (e.g., overly intimate in their touching or caressing of their child), and



simply responding to the child's movements or behavior in disorganized or strange ways that are not understandable (Fonagy & Target, 1997; Main & Hesse, 1990, 1998).

All of this results in the child being affectively labile, relationally unstable, lacking in empathy, and having a diffuse identity, characteristics often noted in clients with Borderline Personality Disorder (BPD) (Herman, Perry, & van der Kolk, 1989; VanDijke & Ford, 2015). Further, some researchers describe clients who were Disorganized in childhood, and now fearful as adults, as having multiple IWMs of Self: Self as frightening to Others, rescuer of Others, loved child, and rejected or hated child. They also may have multiple models of their highly contradictory caregivers: Caregivers as helpless victims, angry or exploitative perpetrators, and as competent or helpful. These markedly disparate relational experiences from the same caregiver make integration of a Self difficult and treatment challenging (Liotti, 2004).

Sadly, with Fearful clients we are in the realm of maltreatment and abuse where two themes predominate. First, many Fearful clients suffered significant parental hostility and overt rejection (e.g., *I wish you were never born...you are ruining my life...we just hate you*). Second, some clients with a fearful attachment style have suffered sexual abuse and/or witnessed or experienced physical abuse (Fonagy, Leigh, Steele, et al., 1996) but have not resolved or come to terms with their maltreatment or traumatic loss (again, this is the adult version of the Disorganized child, also referred to as "Category D"). These clients can be incoherent in their speech when recalling trauma or loss. They may also exhibit disorientation and intense fear when distressed, as a result of having experienced their caregivers, confusingly, as sometimes frightening and sometimes helpful or benevolent (their attachment figures often have their own unresolved trauma and are most likely to be frightening to the child while dissociated or under the influence of alcohol/drugs) (Fonagy, 2001; Solomon & George, 1999).

Understandably, these clients often struggle with serious psychopathology, including intense anxiety, PTSD, acting out symptoms such as criminal behavior, substance abuse and addiction, or self-injurious behavior such as cutting. Some also will experience altered states such as dissociation or depersonalization as a result of trauma and abuse (Carlson, 1998; Stovall-McClough & Cloitre, 2006). Therapists will observe that these significant symptoms are especially likely to occur under conditions of attachment stress (e.g., when a relationship is ending via an important person moving away, dying or abandoning). Frightening memories or feelings about their unresolved trauma or loss also occur when cued by the smell of alcohol on someone's breath or the threat of invasive medical procedures. Because these painful events happened *and the child received no help with the maltreatment* (e.g., compassion, validation of their experience and useful explanations that helped them understand what occurred; coupled with realistic expectations for what is going to occur next; and overtly alleviating any self-blame for maltreatment or loss that occurred), these experiences remain unresolved and contribute to significant adult pathology such as frequent episodes of Major Depressive Disorder (Dozier et al., 2008). These findings suggest that fearfully attached clients may present with both externalizing symptoms (like criminal behavior) and internalizing diagnoses (like Major Depression and BPD) as well as with dissociative symptoms (Lyons-Ruth, et al., 1997, Ogawa, Sroufe Weinfield, et al., 1997; Sroufe, 2005).

Fearful clients are challenging in treatment because they often evoke strong countertransference reactions in therapists, and because therapists may need to manage acting-out behavior to keep these clients safe. They often need longer treatment for improvement because their experience has led them to feel others, including the therapist, may be rejecting, unavailable, or threatening (Reis & Grenyer, 2004). These clients certainly can be helped in treatment, but are not appropriate for new therapists.

In treatment, clients with Fearful attachment styles avoid close involvement with an intense fear of rejection. They are low in self-disclosure, emotional engagement, and reliance on the therapist and others. Often, they have a stubborn, recalcitrant *distrust* of others, and also view themselves as unlovable and unworthy of care. Although they are interested in approaching others and in having closeness, they behaviorally avoid relationships for fear of rejection (Bartholomew & Horowitz, 1991). Thus, they see both themselves and others negatively and, confusing to themselves and others, are simultaneously both high on avoidance of attachments while still wanting them but also highly anxious while approaching others. These clients cannot easily use the therapist as a Safe Haven (to bring their distress to) or as Secure Base (for working-through and exploring new ways of relating), and the reason they tend to move slower in treatment is because they cannot easily trust. Highly shame-prone, they possess a sense of personal unworthiness and interpersonal insecurity that leads them to mis-hear the therapist's benevolent responses through this distorting lens of expecting to be rejected or exploited.

Key to understanding Fearful clients is their shifting or alternating presentation from a Preoccupied to a Dismissive coping style. These clients can look similar to a Dismissive client at times and temporarily avoid relationships but, at other times, confusingly, may look preoccupied as they become immobilized by intense anxiety at the prospect of being rejected or exploited again upon approaching an attachment figure. Under stress, some Preoccupied clients with sexual abuse histories could develop more serious dissociative symptoms, and some Dismissive clients could seem depersonalized in their extreme disconnection from their own experience as they talk about a traumatic experience such as physical abuse. Although trauma, abuse, neglect, and unresolved loss may be found within any of the four attachment categories (including secure), it is central in the fearful attachment style. These clients need therapists who can manage and tolerate the shifting and contradictory presentations they exhibit.

Thus, in Figure 6.1, the Fearful client in Quadrant IV (page 186) is high on *Avoidance* and high on *Anxiety* (that is, these clients avoid their own emotional needs and they present with overt anxiety when they approach attachment figures). If you are working with Fearful clients, you are going to hear prototypic sentences that communicate their profound shame—the legacy of their rejection and abuse histories, and the maddening double binds they have been struggling to decipher, such as:

- *There's just something wrong with me.*
- *I'm losing it—I can't live without her, but I'm not sure I can live with her either.*
- *I don't matter—I just hate myself.*
- *They wouldn't want someone like me, although I sometimes wish they would.*

In treatment, therapists are going to observe Fearful clients enact their core conflict—a maddening double bind that leaves them stuck in a seemingly unresolvable approach-avoidance conflict. The Fearful client has often grown up with the childhood dilemma of sometimes being genuinely helped by the same person who, at other times, is frightening and rejects or betrays them (Main & Hesse, 1990, 1998). Fearful clients want to approach others at times but are quickly made anxious (or terrified) by the fear of further rejection or abuse, and this back-and-forth will play out with the therapist as it does with others in their lives. Thus, they cannot get close and they cannot get away, and so live their lives in a constant state of stress and physiological arousal (adrenal fatigue), leading them to suffer from all of the medical and psychological problems engendered by this “fear without resolution.” To capture their difficult lives most simply, *the rigidly controlled Dismissive client holds fear of connection, the impulsive Preoccupied client holds fear of differentiation, and the Fearful client fears both.*

How will we know when Fearful clients are ready to terminate therapy? They will have resolved their inability to manage trauma and loss. This often means letting the bad news of their developmental history be as painful and real as it was—and being able to tolerate the feelings (experience and express them) that once accompanied their maltreatment. They will have developed the reflective ability to view events and experiences from a variety of perspectives (i.e., not slot varied experiences into the same schema) and be able to regulate their affect rather than acting out on it, even under stress. Finally, they will be able to tolerate both connection and autonomy in a flexible way. Usually, this will occur first with their therapist, and then (with the therapist's guidance) with selected individuals outside the therapy setting. In other words, they will have achieved “earned security.” This will best be accomplished through experiential relearning with therapists who can provide a relationship that serves as a reparative attachment.

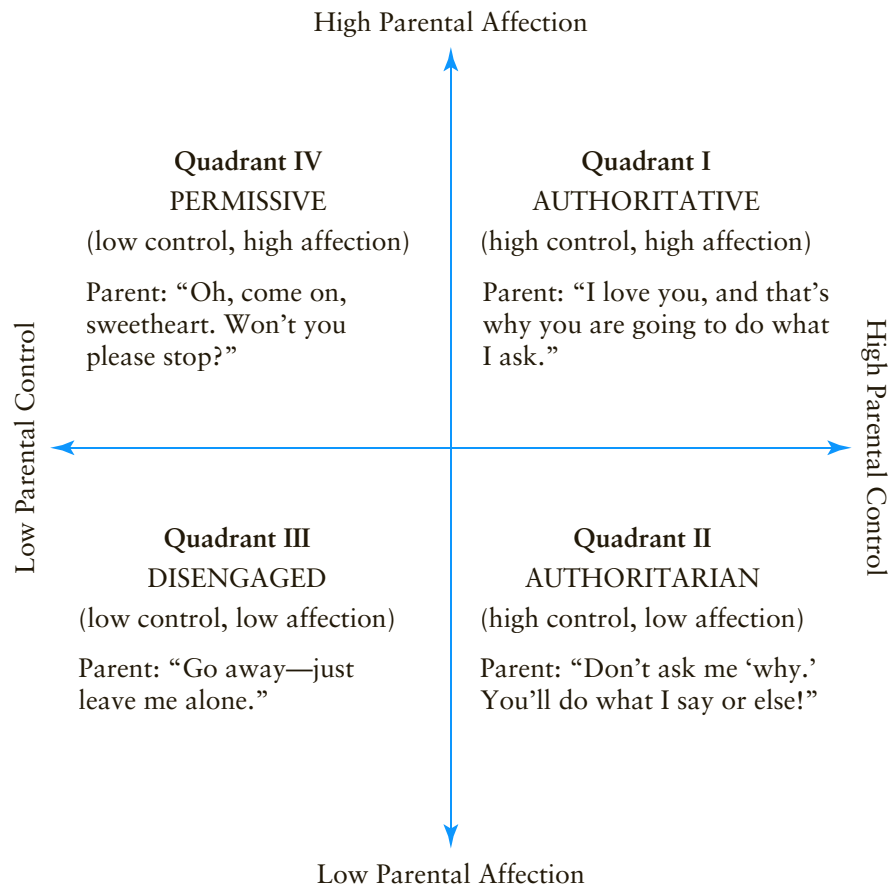
### CONCLUDING THOUGHTS ON ADULT ATTACHMENT STYLES

Attachment styles teach therapists much about their clients but, like all classification systems, they also have real limits. Rather than thinking of your clients as pure types or attempting to fit them neatly into one category, it is best to *appreciate the variability that stems from many factors*: clients will have these attachment styles to different degrees—they occur on a spectrum of severity; there are several different subtypes within each of the four primary attachment categories presented; many clients will share characteristics of more than one attachment style; and some children and adults simply do not fit any category (Cannot Classify). Thus, some clients will present Dismissive or Preoccupied features in different relationships and at different points in their lives. These presentations are also influenced by cultural features such as ethnicity, religious background, having multiple caregivers, and how autonomy is viewed and encouraged in different cultural contexts (Fiori et al., 2009; Keller, 2013; Wang & Mallinckrodt, 2006). When distressed, however, many individuals are going to present with one predominant style of attachment in their most important relationships.

Having a secure attachment style is a profound buffer or resiliency factor, but it does not mean that securely attached adults will not have symptoms or problems that result from challenging life circumstances. However, having secure attachments in childhood does mean that you are protected from real threats and buffered from real dangers in the world in a way that the three types of Insecure children are not. Children simply grow up safer when they have a responsive caregiver to go to when distressed. In contrast, children with Ambivalent/Anxious, Avoidant, and Disorganized attachment styles are less protected and more vulnerable to peer pressure and invitations from predators, and are more likely to miss danger signs in the environment that can lead them to walk too readily into threatening situations and relationships that hurt them. (Excellent reviews of the attachment literature can be found in Cassidy and Shaver, 2008; and Obegi and Berant, 2009.)

### PARENTING STYLES

In addition to the attachment experiences already described, another dimension of parent-child interaction that significantly influences children and the symptoms and problems adults later present in treatment are parenting styles (see Baumrind's research: 1971, 1991). In this section, we discuss the child-rearing practices children have experienced that shape their presentation when they enter therapy. Baumrind and others (for example, Bender et al., 2007; Berk, 2005; Slicker and Thornberry, 2002) have conducted extensive research programs on the impact of different parenting styles on children's self-esteem, development of autonomy and initiative, social and communication skills, and so forth.



**FIGURE 6.2** | FOUR STYLES OF PARENTING

To better understand the different styles of child-rearing that parents use, and how many adult clients' problems originally developed, consider the dimensions of Control and Affection that are diagrammed in Figure 6.2. As reflected on the horizontal axis, parents can vary along a continuum from firm discipline (high control/structure) to permissive or lax discipline (low control/structure). The vertical axis represents how parents can vary on a continuum from much warmth, emotional responsiveness, and communication (high affection/support) to little approval, acceptance, or interest (low affection/support). Using the high and low points along these two dimensions, parents typically discipline their children using one of four approaches. The first three that we will discuss are problematic and the fourth is effective: *Authoritarian (strict/cold)*, *Permissive (indulgent)*, *Disengaged (neglectful)*, or *Authoritative (loving and firm)*.

### 1. AUTHORITARIAN PARENTING

One of the most common but ineffective methods of discipline is the **Authoritarian** approach, which is controlling and often punitive (see Quadrant II: high control, low affection on page 188). These parents are low in warmth, communication, and emotional responsiveness but high in control and demands. They are strict disciplinarians who give children clear expectations about what behavior is acceptable and unacceptable. Parental rules and expectations are clear and the consequences for violating them are consistently enforced. Authoritarian parents are demanding and have high expectations

for their children to behave in a responsible and mature manner. However, they rarely engage in mutual, respectful communication that involves explanation for their rules—their children are simply expected to obey and conform to parental dictates. Children are also expected to be competent and perform up to their abilities, and to be responsible and contributing family members.

However, without the emotional support and affection so important to the development of a sense of belonging and security, especially during moments of distress, children of authoritarian parents learn to hide any signs of vulnerability from their parents and, sadly, eventually from themselves as well. While many are able to attain success in school and later in their chosen careers, they tend to keep people at a distance and their own emotions under tight control. Experiencing vulnerability of any kind may activate fear or a Disapproval/Rejection schema, something these clients have often struggled with throughout their lives. They often internalize anger and have difficulty coping with frustration and may respond with angry outbursts. Because they do not have close, affectionate relationships with their strict and rather cold caregivers, unless they have compensatory adult mentors, they will lack healthy role models with whom to identify. Given the extent to which their emotional needs were not of importance in their families of origin, they often become detached from relationships or appear pseudo-independent and some may end up with Avoidant attachment styles in childhood or dismissive styles as adults.

Authoritarian parents' control is on a continuum from being simply strict and firm to using harsh, intimidating discipline. Thus, in addition to a lack of emotional support or warmth, these parents often instill a fear of rejection in their children by their methods of discipline. These children regularly hear their parents make statements such as, *"Don't ever ask me why you can't go out. I am your father, and you'll do what I say or else!"* Children are not encouraged to suggest alternatives or to explain their side of the story. These parents are often committed to the parenting role, however, and usually provide an "instrumental" form of love in the sense that they responsibly feed and clothe their children, help them with their homework, and may play sports or games with them.

Authoritarian parents mean business, and their children know this and behave in their parents' presence. The threat of parental power and the fear of rejection keep children in line, especially while they are young. This strict, no-nonsense approach is better for children than having no discipline at all, but it has major drawbacks. *These children are obedient and achieving, yet they are also anxious, depressed, and often insecure; they comply out of fear* (Gerlsma et al., 1990; Kerig, Ludlow & Wenar, 2012; Sakado et al., 2000). Often, lack of parental nurturance results in children and adolescents who are uncooperative and immature compared to their peers who are raised by warm and protective families. Similarly, other data supports the findings that children who grow up in over-controlling, non-nurturant families are more likely to be anxious, depressed, engage in self-defeating behaviors, have low self esteem and feel inadequate (Fryers et al., 2012; Love & Thomas, 2014; Uji et al., 2014). Further, this subjugation can also lead to eating disorders, such as anorexia, as one of the few avenues for control (Enten & Golan, 2009; Haycraft & Blissett, 2010). Although most children remain intimidated by their authoritarian parents and live anxiously with little positive self-regard, some become aggressive and defiant as they grow older and more verbal. Authoritarian children are socialized to endorse traditional gender roles where men are seen as dominant and stoic, and women as emotional, submissive and dependent (Peterson & Zurbriggen, 2010).

Another drawback to the authoritarian approach is that it limits the growth of children's critical thinking abilities. When children are not given reasons to help them understand why parents have set certain rules and are not encouraged to suggest alternatives or compromises, they do not learn to exercise their language and reasoning skills and exhibit less cognitive development and achievement (Hamre & Watkins-Lewis, 2012; Kathleen et al., 2012; Sternberg et al., 1994).



In contrast, within healthy families, children learn to obey without sacrificing their own initiative and positive self-regard. Healthy children become self-controlled and self-reliant without losing their sense of being prized by their parents. However, for children of authoritarian parents, the trade-off between acting on their own wishes and maintaining parental approval is too severe. Because authoritarian parents provide too little nurturance and affection, *too much of their children's initiative and positive self-regard is lost in order to try to win parental approval*. These anxious children are worried about doing everything right and never making a mistake, in order to ward off parental criticism and painful disapproval. By the time the children are of school age, these relational patterns with the authoritarian parent have been internalized as a cognitive schema: What was originally an interpersonal conflict becomes an internal conflict that shapes subsequent relationships with other adults, and with themselves, in problematic ways. More specifically, these well-behaved, insecure children become harsh, critical, and demanding toward themselves, just as their parents have been with them. Many will seek therapy as adults, presenting with symptoms of guilt, depression, unassertiveness, anxiety, and low self-esteem. These personal and emotional problems will be present even though many of these clients are responsible, hardworking, and successful adults—they suffer internally.

Having come from families with this strict, cold environment, therapists' treatment goals will be to:

- 1) Validate and affirm these clients' experiences because they have been so deprived of empathy and understanding that they will be out of touch with their own feelings, including bodily experiences;
- 2) Provide choice and help them develop their own voice, which they missed developmentally;
- 3) Help them lead and initiate in the therapy hour and actively teach and support more assertive responses to the therapist and others because they have been subjugated and will have either aggressive or compliance propensities;
- 4) Be a Safe Haven that provides compassion and warmth for their distress which they have not experienced before, and will increase their capacity for empathy and success in relationships;
- 5) Help them to develop communication skills so they can become emotionally intimate with others (especially with their partners/spouses and own children) and become more assertive and exert more control over their lives so they don't need to exert excessive self-control via disordered eating.

## 2. PERMISSIVE PARENTING

Although authoritarian parents recognize that children need to know the rules and that the consequences for violating them will be enforced, their discipline is rigid and their parenting lacks empathy. In contrast, other parents may err on the side of permissiveness and lax control. Although **Permissive** parents are often warm, nurturing, and communicative, they do not place appropriate expectations for mature or independent behavior on their children. Most importantly, they are also indulgent and set few rules, and when they do, they fail to follow through and enforce them consistently (see Quadrant IV: low control, high affection on page 197). Children in these families make their own decisions and receive too little age-appropriate guidance from their parents.

Permissive parents' inability to take charge and discipline effectively, which causes so many significant problems for children and later adults, may occur for many reasons. Some permissive parents may falsely believe that if they are firm they are acting harshly and, in their own minds, being just like their own (authoritarian) parents, who were too harsh or intimidating (e.g., Adult Male Client: "*My father was a tyrant, and I knew when I was 10 years old that I wasn't going to be anything like him*"). Or, sadly, some

insecure parents (often Preoccupied) fear their children will not love or be close to them if they say “no” and/or tolerate their child’s disapproval if they follow through. Still other parents (often Fearful), perhaps so invalidated and disempowered by their own parents in childhood, don’t really believe they have the right to be the one in charge or to instill their own values and standards and teach their children how they want them to behave or act in life.

For these and other reasons, *the balance of power has tipped in Permissive families, and children wield too much control in the parent-child relationship*. Permissive parents often feel disempowered vis-à-vis their children and may be heard negotiating, bargaining, or even pleading with their children to behave (“*Oh, come on, sweetheart, won’t you please stop?*”). Parent and child both begin to suffer when control shifts from the parent to the child in this way. These children don’t learn how to self-regulate as they have had no guidance in setting limits. Problems begin to develop early as these young children become bossy toward other children, angry with playmates when they can’t dominate, and are argumentative and disrespectful toward adults when they don’t get their way. And years later, too often, these same relational patterns will be tested in the therapeutic relationship, as these clients will try to evade limits and rules and try to manipulate and control the therapist—as they once learned to do with their caregivers.

Children of Permissive parents do not know what behavior is expected of them or what will happen if they violate parental norms. These children learn that they do not have to obey because their parents will not *consistently* enforce the rules. Without parental expectations that they perform to the best of their abilities, children do not develop the skills or internalize the discipline necessary to succeed on their own. While these children can at times exhibit self-confidence and initiative—like children raised by authoritative parents (Shaw et al., 2004)—they are routinely viewed as self-centered, dependent, and demanding (Baumrind, 1991; McCrae & Costa, 1988). In addition, they often become less engaged with school over time, and it is also common for these children to become involved in school misconduct (Steinberg et al., 1994). Lacking the guidance to develop appropriate self-regulation, they also may become involved in drug and alcohol use (Lamborn et al., 1991), and engage in disordered eating behaviors (Coccia et al., 2012; Haycraft & Blissett, 2010). Behavior problems may develop involving school authorities for truancy, the police for reckless driving, or drugs and alcohol. In addition to being demanding, selfish, and angry, these children are likely to fail with peers and friendships, and to report lower levels of well-being (Love & Thomas, 2014).

Children and adolescents with permissive parents are right in knowing that they are not safe and cannot be protected by a parent who cannot say “no” to them, cannot tolerate their disapproval, or gives them too much power and control over the parent-child relationship. *Underneath the demanding, angry and manipulative presentation is an anxious child who can benefit from clear boundaries, limits and predictability*. Children do not respect parents (or, later, therapists) whom they can manipulate or who cannot say “no” and discipline effectively because they need the child’s approval.

Later, as adults when they enter treatment, they continue to be self-centered, demanding, and dependent in their relationships, and less capable of making commitments and following through responsibly on obligations. Often, these clients will be mandated to treatment by judges and courts and will be seen in alcohol and substance abuse treatment programs. Therapists working in Employee Assistance Programs will also work with these clients. Typically, they will be referred for treatment by a supervisor at work who is dissatisfied with their inability to be a good team player and get along with others in their work group. When seeking therapy as adults, they tend to avoid taking responsibility for themselves and try to blame others for their problems. They also have learned that they

can break the rules and escape the consequences of their own behavior by manipulating others. Of course, these same interpersonal themes that are causing problems with others will appear in the interpersonal process they begin to play out with the therapist, usually within the initial telephone contact or first session.

Finally, in one subtype of Permissive families, parents are acting more as friends than as parents to their children. In this subtype, clients may describe their family of origin as a group of siblings living under the same roof—signaling the absence of clear intergenerational boundaries. When parental guidance or help is required, such parents may appear inept or even incompetent. To remain attached to these parents, children in these families become parentified and take on an adult role. During emergencies, for example, these parents may fall apart, requiring the children to come to their assistance. Children in these families become responsible for the emotional (and sometimes physical) well-being of one or more parents. In families with more serious problems, children may be given the role of “rescuer” in a family where one parent is physically abusing the other parent. Children may receive love and attention in the form of gratitude; however, *the love is conditioned on the child’s ability to aid the parent*. It is a “binding” love that ultimately smothers children and prevents them from developing a secure attachment, a viable identity, and the ability to differentiate from the parent and successfully emancipate from the family and establish their own independent adult life (Teyber, 1983; 2001).

As adult clients, they may superficially present as self-confident but, having had no limits and guidance and being indulged, they will lack emotional regulation. As a result, they typically will be demanding and lack perspective for the impact of their behavior on others. Therapist can help these clients by:

- 1) Having clear limits and expectations (e.g., coming to sessions and paying fees on time; having limits on missed sessions, cancellations, and re-scheduling; taking responsibility for their behavior; attending sessions sober and not intoxicated) that are consistently enforced;
- 2) Assisting them to de-center and become more aware of the impact of their behavior (especially of their demandingness) on others (such as their spouse or their own children);
- 3) Without being judgmental or critical, consistently clarify that in real life, people are not going to respond to them and/or meet their needs automatically;
- 4) Teaching them about the importance of reciprocity in relationships, and looking for opportunities to address this when it is occurring in your current interaction with clients;
- 5) Encouraging them to develop self-control and emotional regulation by highlighting both the triggers and consequences of their behaviors;
- 6) Giving them the sense that as they learn self-control and emotional regulation, their problem behaviors such as temper outbursts, lack of follow-through, disordered eating, and alcohol and drug difficulties will diminish; and
- 7) Providing a relationship where clients get the sense that the therapist is genuinely concerned about their well-being yet is not afraid of their disapproval, and clear therapeutic boundaries will be maintained but without being shaming or punitive.

### 3. DISENGAGED PARENTING

The third parenting style is **Disengaged** (also termed Neglectful) parenting. Especially problematic, these caregivers are disengaged from their children (Quadrant III: low control, low affection on page 193). Looking at Figure 6.2, one sees that these uninvolved parents are low on both axes—they are doing little for their children. Whether passively

unresponsive or overtly rejecting, this neglectful parent says in word or deed, “Go away—just leave me alone.” These parents have little investment in their role as caretaker and engage in inconsistent and erratic discipline. Within different subtypes of Disengaged families, children may be rejected, suffer neglect, be physically or emotionally abandoned, and some will end up in foster placements. For some, their very existence seems to irritate or burden their parents. In order to adapt to these painful situations, children of Disengaged or Neglectful parents often learn how to hide and not make waves, often limiting their ability to form a personal identity.

How does this disengaged/neglectful parenting style come about? Often caught up in their own drug or alcohol abuse, disengaged parents may be too self-absorbed to attend to their child’s needs and may expose their children to predatory individuals in their own homes. Another type of disengaged parent may be chronically depressed and unresponsive to children, who grow up unnoticed, and in later years describe themselves in treatment as having been “invisible.” Still other children may have grown up with a caregiver who had a Narcissistic, Borderline, or Paranoid Personality Disorder, and may be overtly rejected (e.g., “*I wish you were never born!*”). These children may also be pushed away by an angry parent, or blamed for all of their parents’ problems. Not surprisingly, these children are less mature, less competent in all spheres of life, and are more troubled when compared with children raised by authoritative parents (Lamborn, Mounts, Steinberg, & Dornbusch, 1991; Steinberg et al., 2006).

The research further indicates that children from disengaged/neglectful families have more problems with substance abuse, report lower psychological well-being including higher rates of depression (Rothrauf, et al., 2009), and that these problems are evident across all ethnic groups, SES levels, both genders (Steinberg et al., 1994, 2001), and across cultures (Ozer et al., 2011). This lack of parental care can result in the child being uncooperative, having problems with authority, and with developing personality disorders such as Antisocial Personality Disorder (Carter et al., 2001). Unsupervised and unwanted, this disaffected child becomes at risk for many different problems, including antisocial behavior and the peer influences of drugs, delinquency, and early sexual contact. These clients are often in the child welfare system, frequently being seen in group treatment homes and juvenile probation programs. Because they have not had a caregiver who monitored them and tried to keep the child’s “mind” in mind—that is be aware of their needs, activities, and decisions, they are less socially competent, have more accidents, are unprotected from exploitative individuals in the environment, and too often suffer sexual molestation and physical abuse (Allen, Fonagy, & Bateman, 2008; Benbassat & Priel, 2012; Katznelson, 2014).

These at-risk youth bring strongly held schemas for rejection and distrust to the therapeutic relationship. In therapy, they will present as though they need no one because they expect the therapist to reject or be disinterested in or disdainful of them, and see them as unworthy of the therapist’s attention, as they were in their family of origin. The therapist, working with clients from this background, needs to:

- 1) Establish a working alliance based on showing genuine personal interest and giving these clients a sense that the therapist finds them worthy of attention and takes pleasure in being with them, in order to build trust;
- 2) Validate the reasons for their lack of trust and difficulty establishing a relationship with the therapist by “getting” their developmental history;
- 3) Work with the clients’ understandable anger and the underlying shame of not being chosen or prized by their family of origin;
- 4) Help them establish an attachment to the therapist—primarily through empathic understanding—and then to seek selective others in their lives who may be able to assist them in learning prosocial values and making better life choices;

- 5) Work with them to understand both the triggers and the consequences of their problematic behaviors; and
- 6) Assist them in developing reflective functioning skills which include the capacity for self-awareness and self-understanding that leads to emotional regulation (the intrapersonal aspect), and to see events, situations, emotions, thoughts, and needs from others' perspectives (the interpersonal aspect of reflective functioning).

#### 4. AUTHORITATIVE PARENTING

Finally, the **Authoritative** child-rearing style is most effective and produces well-adjusted children (see Quadrant I: high control, high affection on page 186). The authoritative approach combines firm limits and reliably enforced rules with affection and communication. These parents are sensitive and responsive to their children's needs. These positive caregiving experiences facilitate the development of children's emotional, cognitive, and brain development, and contribute to their academic success and social competence (Morgan et al., 2012; Piko & Fitzpatrick, 2013; Roisman & Fraley, 2008; Spieker et al., 2011; Sroufe et al., 2005). These parents establish relationships with their children that involve clear communication and clear guidelines, and they provide explanations and reasons for parental rules and decisions. Parents have high expectations for responsible and mature behavior, and where possible, give the children the opportunity to be involved in decisions and have choices. This process helps children become confident independent decision makers themselves. In addition, because these parents set reasonable limits and are actively engaged with their children in warm and mutually respectful ways, this positive parenting is associated with secure attachment. Children who grow up in households such as this are more likely to adopt and internalize their parents' morals and values, and behave in prosocial ways (Schroeder & Mowen, 2014).

Thus, while authoritative parents believe in discipline and responsible behavior, unlike the authoritarian parents, they combine this with physical affection, spoken approval, and explanations for their rules. For example, authoritative parents expect good behavior yet also tell their children stories, roll with them on the floor, hold them in their lap, praise them when they do well, and look in their eyes and say, "*I love you.*" Children more readily cooperate with requests from an affectionate parent than from one who is threatening or distant.

Authoritative parents tell their children what they would like them to do and explain why certain behavior is encouraged or discouraged. They also encourage children to offer alternatives or compromise solutions. In contrast, the authoritarian parent provides clearly defined and enforced limits but with no room for compromises, alternatives, or explanations. While permissive parents may offer reasons, explanations and compromises to lessen their children's disapproval, they ultimately do not take a firm stance or convincingly enforce limits.

Clearly, the authoritative parent exercises a wide range of parenting skills. Although many parents falsely believe that they have to be either strict or loving (i.e., authoritarian or permissive), *authoritative parents are more effective because they have the flexibility to be both at the same time*. The data is consistent: Authoritative parenting fosters self-reliance, a sense of competence, achievement motivation, academic success, and the ability to socialize more effectively than non-authoritative parenting (Henderson et al., 2006; McKinney & Renk, 2008; Rothcraft, Cooney, & An, 2009; Schroeder & Mowen, 2014; Steinberg, 2001). Children who grow up in authoritative homes report greater psychological well-being, exhibit greater success, and engage in less substance abuse than children from non-authoritative homes (Hoeve et al., 2009; Morgan et al., 2012; Pico & Fitzpatrick, 2013; Shah & Walter, 2000).



As therapists listen to their clients' narratives and learn more about their developmental histories, they will hear relational patterns and themes derived from authoritarian, permissive, and disengaged parenting styles. But what about children of authoritative parents—is this a “perfect” family without any problems? Parenting and development are always challenging, even in the best of circumstances, and conflict-free families and individuals are a myth. On average, however, children of authoritative parents are better adjusted and, like Secure clients, they will be less likely to be seen in treatment for enduring problems. They will be more likely to seek help in crisis situations (such as a child's major illness) or when negotiating developmental hurdles (such as seeking premarital counseling). And because they have learned that some others can respond responsibly to their emotional needs, they are able to seek help, enter treatment when necessary, and utilize it effectively.

In thinking about these four different styles of child-rearing, we also need to recognize the enormous complexity of raising children and all of the different factors that contribute to family interaction. For example, birth order, gender, and temperament greatly influence how parents respond to children. Family functioning is fundamentally shaped by cultural values and beliefs, as all child-rearing practices are embedded in a social context (Domenech-Rodriguez et al., 2009; Huntsinger & Jose, 2009). Because of these and other influences, the four parenting styles we have discussed become far more complicated in everyday life. For example, some children grow up with two parents who have conflicting styles (e.g., authoritarian father and permissive mother), and children may learn that they can escape consequences—to their detriment—by playing one parent off against another. Or, to illustrate this complexity further, in a different family children may do well with an authoritarian and a permissive parent—because in combination the children are receiving the warmth and communication they need from one parent, and clear limits with expectations for mature and responsible behavior from the other. Suppose, however, that these two parents divorce, and the authoritarian father does not sustain an active parenting role in the years following the divorce. Now, the children have lost their source of discipline and are raised primarily by their permissive mother. Typically they will stop functioning as well (unless this mother grows through the experience of divorce and becomes better able to tolerate her children's disapproval and discipline them more consistently) and the children will become more demanding, angry, and less achievement-oriented (Teyber, 2001). As a rule, authoritative parenting (a combination of limits and warmth) is best for all children and disengaged parenting yields the worst outcomes.

## LOVE WITHDRAWAL AND CONDITIONS OF WORTH

Many caregivers who are not authoritative often employ discipline techniques based on **love withdrawal**. Instead of communicating that they disapprove of the child's *behavior*, these parents respond with anger or rejection and *communicate disapproval of the child's basic self*. While disciplining, these parents withdraw their warmth and *emotional connection* to the child, engendering in the process the anxiety of an attachment disruption (even though the parent and child are not physically separated). This parental communication is often nonverbal; the withdrawal of love from the child is expressed as much by tone of voice and inflection, gesture, and facial expression as in words. Parents who use psychological control in this harsh way have children who suffer negative consequences and even increase the likelihood of their children becoming delinquent (Hoeve et al., 2009). Central to his theory, Carl Rogers (1959, 1980) referred to this far-reaching developmental issue as **conditions of worth**—what children learned they had to do to maintain their parents' approval (e.g., be perfect, achieve and be a star, remain dependent and child-like, take care of their parents' emotional needs).

Parents respond with love withdrawal to punish the child and communicate their anger—which too often is accompanied in tone and expression by the more deeply wounding affect of contempt. They may say with disgust, for example, “*Get out of here! I don’t even want to have to look at you. What’s wrong with you, anyway?*” Better-functioning parents may respond in these hurtful ways occasionally, when they are tired or upset, and may later apologize or clarify that they have over-reacted (“*Daddy got too mad and shouldn’t have said that. I’m sorry*”).

In contrast, for some parents, withdrawal of love and emotional disengagement occurs routinely, and the child’s emotional ties to the parents are disrupted regularly (van Zeijl et al., 2006). For example, the parent may say overtly, “*I can’t stand to be around you. Get away from me.*” In contrast to these overtly rejecting messages, caregivers may emotionally withdraw in more *covert* ways. For example, martyrish parents may say nothing, sigh painfully, and turn their backs and walk away from the child, silently shaking their heads in disappointment and disgust. The child develops symptoms and defenses to cope with the painful separation anxieties and the shame-based sense of self that these attachment disruptions engender. For these children, self-schemas develop that leave them shame-prone throughout their lives as they come to believe, *I’m a bad boy, There’s something wrong with me, I need too much, or Mom doesn’t want me.* Everything changes—and gets better, when whatever just happened can be acknowledged or spoken about—rather than having to act as if nothing significant just occurred.

Whereas authoritarian and disengaged parents tend to use love withdrawal disciplinary techniques frequently and create insecure attachments and separation anxieties in their children, permissive parents, in contrast, often become emotionally entangled or overinvolved with their children. At times, they will smother them with unwanted love and attention, which is better described as aggrandizement, specialness, and indulgence. This strategy is to draw the child closer to the parent, but often with the purpose of making the parent, not the child, feel better.

Love withdrawal techniques and conditions of worth, like all of the parenting attributes we are presenting, occur on a continuum of severity. In some families, disruption of ties from love withdrawal may not be severe. Or, as noted earlier, they may occur infrequently—only when caregivers are stressed or fatigued. If love withdrawal does not occur routinely, and if other opportunities for emotional connection are available, ties can soon be restored. Although the anxiety may still be painful, the child can often fashion a reconnection through *compliance* and taking full responsibility for causing the disruption. For example, the child of a disengaged parent may adopt the interpersonal coping strategy of being quiet and “going away inside” so she needs nothing from her parent. Or, the child of an authoritarian parent may learn that by compulsively achieving or anxiously attempting to be perfect, this coping strategy will help preserve as much as possible of their insecure attachment. As new therapists gain more clinical experience working with diverse clients, *they will begin to recognize the varied coping strategies their clients adopted as children to employ in their efforts to restore ruptures and maintain ties to attachment figures.* Such interpersonal coping strategies (or rules of attachment) may sound like one of these statements: *It’s my fault they’ve “had it” with me. I’ll be OK if I could just:*

- *Play baseball better;*
- *Be nicer all the time;*
- *Do more for my mom;*
- *Always get As in every class;*
- *Be thinner and look prettier;*
- *Help my dad stop being so unhappy;*
- *Be quiet and not ask for anything.*

Why is it helpful for therapists-in-training to learn about all of these developmental issues in order to clarify the client's problem and develop a treatment focus? As we will see in Chapter 7, these attempts to win attention or affection and gain more security often become a *pervasive coping style* that clients continue to use with others throughout their lives. That is, these developmental coping strategies come at a significant personal price, and they become central to the symptoms and problems that clients subsequently present in treatment. Furthermore, because of the schemas that have developed from these childhood experiences, these clients also are going to believe that they similarly have to elicit, or be responsible for, the therapist's interest in them—as they have with caregivers and others in their lives.

In families that are more dysfunctional, however, the attachment disruption will occur more often and more severely. In daily interactions around discipline and control, *children in these families will be exposed regularly to experiences of interpersonal loss and emotional isolation, even though the parent and child remain in physical proximity*. As we move further along this continuum, ridicule and rejection occur more overtly. In disengaged families, gross neglect and even actual abandonment may ensue. In highly authoritarian families, these behaviors may erupt into physical abuse. In these painful situations, children experience their parents' anger and contempt as assaults on their basic sense of self, leaving them ashamed of who they are and psychologically alone. It is important for new therapists to know that they are going to work with clients with these painful developmental experiences, and that they will need to help *these adult clients who still hold the pathogenic belief that they are somehow to blame* for their parents' neglect, rejection, physical domination, and contempt. In other words, as attachment-seeking children, these clients assumed responsibility and blamed themselves for what their caregivers did, said, or thought (and now, as adult clients, they are still doing this internally as well) and make self-blaming statements such as:

- *If I hadn't fought with my brother so much, my dad would have stayed with my mom. It's my fault she cries all the time now.*
- *If I had done all of my chores, like Mom asked me, she wouldn't have hit me so hard. I was bad.*
- *If I hadn't dressed like that, he wouldn't have touched me down there. I feel dirty.*

In conjunction, when caregivers act irresponsibly in these ways, routinely they overtly blame the child for their own inappropriate behavior. Compounding this, unfortunately, the attachment researchers suggest that this sense of shame and feeling blame-worthy are adaptive if these children are to experience some sense of psychological control in their lives (Janoff-Bulman, 1979). Clients hold on to such false beliefs tenaciously because the unwanted alternative would be to view their caretakers more realistically as rejecting and punitive, which would leave the child feeling even more unattached, unprotected and alone. In sum, when the child experiences significant love withdrawal as a disciplinary technique and conditions of worth, it is the genesis of a shame-based and blame-worthy sense of self, with presenting symptoms of anxiety and depression.

**Mystification Amplifies Problems.** Love withdrawal as a method of discipline and conditions of worth occur in many ways and in different types of families. In these moments of parental love withdrawal, we see that the child's emotional connectedness to the parent is temporarily broken, and the child's attachment ties are situationally disrupted—engendering separation anxieties (and often shame) until the child can find a coping strategy, such as compliance, to restore the tie. Of course, all children's ties to their parents can be threatened at times. However, significant problems occur when disruption of ties is more severe and so frequent as to *characterize the relationship*.

Let's add another dimension here and take this an important step further. The child's long-term problems are greatly amplified when these rupturing interactions are *disavowed* by the parent. That is, **the parents** act as if nothing significant happened—even when intense temper outbursts, hitting walls, or domestic violence just occurred. Rather than acknowledging the behavioral reality, that something frightening just occurred, the parent avoids saying, "I'm in a really bad mood today, and I got way too upset just now; that was my fault, not yours." The result leaves the child **mystified**: confused, disempowered, and—left more vulnerable to anxiety, depression, and identity diffusion by having his or her own experience and perceptions *invalidated* so profoundly (Laing & Esterson, 1970; Schatzman, 1974). In this way, therapists often will find that clients who grew up with these more serious problems lived with a covert or **unspoken family rule** *that hurtful interactions like these could not be talked about—named or made overt—as if the child could not say or even know what just transpired*. Just as therapists and clients need to do in the therapeutic relationship, parents and children need to be able to acknowledge, sort through, and restore ruptures in their relationships.

Unfortunately, new therapists are going to find that many of their clients were unable to do this in their families. This is one reason why working to restore ruptures in the therapeutic relationship, and validating the client's experience, can provide such a profound CEE for clients with these undoing developmental experiences. Developmentally, even when there was a highly significant interpersonal problem, there was no way to reconnect. For some others—usually children who grow to become Fearful clients—there could not even be overt acknowledgement of what was painfully occurring.

Finally, a constellation of significant emotional reactions occurs when parents cut off their emotional connection to children in anger or disgust (Johnson, 2008). As we are seeing, even though the caregiver is physically present, the child is psychologically alone and suffers painful separation anxieties. This withdrawal also stifles the child's sense of self-efficacy and gives rise to feelings of helplessness and hopelessness because the child in this predicament cannot really win or earn the parent's love and remains helpless to reconnect until the parent initiates "normalcy." The child is angry at being abandoned and wants to **protest**, of course, but this reaction would only elicit further domination or intimidation from the authoritarian parent or ostracization and further isolation in other families. Through power assertion, the authoritarian parent usually does not allow the child to disagree, let alone find appropriate means of expressing anger. The child may be told, for example, "I'm your father. You are never angry at me. Do you understand that? Look at me and say, 'Yes, sir.'" Thus, the child cannot protest behaviorally, or even experience anger internally, because such reactions will further threaten already tenuous ties to the parent. As a result, this child often becomes intra-punitive by turning the anger inward—and then frequently struggles with a low frustration tolerance and difficulty controlling anger outbursts. This tendency toward punitive self-blame is exacerbated as children commonly *adopt the same critical or contemptuous attitude toward themselves that the parent originally held toward them*—which often becomes a primary treatment focus.

Just as this type of parent loses touch with the child's feelings or experience in these angry moments, the child, in turn, loses the clarity and authenticity of his or her own internal experience. Such children also lose touch with important aspects of themselves and become "incongruent" in Roger's term. When authoritarian parents are rigidly demanding of conformity and obedience, permissive parents over-react to success or problems, and disengaged parents are so invalidating or non-empathic to their children's feelings and needs, children soon lose touch with what they feel and believe internally. Years later, as they present in adult treatment, some may not have much clarity about simple aspects of their subjective experience, such as what they like and dislike, and may even be uncertain about what does or does not feel good to them. Commonly, their

own subjective experience can be so completely over-ridden that they have no basis for developing their own belief systems and clarifying their own spiritual, political, or sexual values, or formulating occupational/career interests in late adolescence and young adulthood. In other words, they haven't been allowed to develop an identity or, more basically, a Self (i.e., have their own mind or voice). New therapists will find themselves working with many clients with these identity problems that stem from varying degrees of these developmental experiences.

Taking this one generation further, even now as adults, the clients who grew up with caregivers who used severe love withdrawal and conditions of worth may continue to be subjected to the same ongoing threats and controls they experienced as children. Even in their current adult lives, if they do not comply with what parenting figures are now demanding, they will (seemingly) destroy the relationship. Thus, therapists will hear these clients present with crises such as these:

- *If you marry him, we won't come to the wedding or visit you anymore.*
- *If you get a divorce, we are going to disown you and take you out of the will.*
- *If you do that, no one in this family will ever speak to you again.*

The theme here is that caregivers who use severe love withdrawal techniques to control their children are not just setting limits on unacceptable behavior, as authoritative parents do; they are instead *threatening to cut off fundamental relational ties*. As attachment-seeking children try to cope with the intense anxiety this arouses, compliance becomes a generalized trait, pervasive personality constriction and inhibition occur, and obsessive/compulsive symptoms and other control issues, such as eating disorders and alcohol/drug abuse, often develop.

With these clients, the therapist's initial treatment goals are:

1. to validate their subjective experience, which has been so pervasively invalidated;
2. to encourage their initiative, which has been undermined—for example, by following their lead in treatment and supporting their own initiative with others; and
3. to provide a treatment focus that helps them clarify their own preferences, pursue their own interests, and act on their own goals whenever possible.

## GUIDELINES FOR RESPONDING TO CLIENTS WITH A DIVERSITY OF PARENTING AND ATTACHMENT STYLES

The meaning in concepts like Early Maladaptive Schemas or Internal Working Models is that roles, expectations, and relational patterns learned in repetitious, affect-laden familial interactions don't just dissipate or go away when late adolescents and young adults move out on their own. Thus, new therapists will find themselves working with many clients who are still struggling with aspects of the problematic developmental experiences discussed here. For therapists who themselves enjoyed better developmental experiences with their own Authoritative or Secure caregivers, it may be hard to appreciate: the emotional severity and blunting rigidity of highly Authoritarian parenting; the anguish of rejection in Disengaged families; the aloneness, marginalization, and crippling anxiety in severe love withdrawal; the painful self-consciousness and identity confusion of growing up with caregivers who fostered a Preoccupied attachment style; or the lack of a coherent sense of Self and the propensity to feel immobilizing terror and dread that has been engendered in Fearful clients.

Early in their training, these more fortunate therapists may wonder how the ostensibly normal and, in many other aspects of their functioning, genuinely decent parents of these clients can be so rejecting or obviously self-centered on occasion, and can



have caused such profound insecurities or even self-hatred in their clients. Other student therapists may have difficulty engaging and being fully present with these clients because they evoke the therapists' painful feelings and unwanted awareness of their own developmental experiences and how they were raised. Clearly, *these issues can be challenging to approach and enter because they routinely evoke such strong countertransference reactions in many new and experienced therapists*. Thus, therapists-in-training need sustained empathy and overtly expressed support from their own classmates, practicum instructors, and clinical supervisors to help them respond effectively to their clients, understand themselves better, and find more self-compassion (Angus & Kagan, 2007; Teyber & Teyber, 2014).

From this starting point of an empathic stance toward the new therapists' changing worldview, increasing awareness of the pervasiveness of these problematic developmental issues in the human experience, and the countertransference reactions they normatively evoke in the first years of training, let's turn to intervention. How can therapists explore and assess parenting styles and attachment histories with their clients, and find ways to utilize this developmental understanding to help with their clients' current presenting problems? As a rule of thumb, it usually doesn't work well for therapists to try and lead clients back to developmental/familial issues. It just doesn't seem relevant to most clients or meaningfully linked to helping with their current problems. Instead, *a far more productive approach is to watch for points of entry when the client has initiated something that directly connects to or seems somehow related to a childhood/familial experience*. With this approach, the client has ownership of this topic and leads the therapist to it, and the therapist is merely joining and expressing interest by: reflecting the key word or phrase the client just used; offering an open-ended bid to explore this further; and becoming a collaborator who facilitates the client's own exploration and agenda. In most cases, this interpersonal process is best. It will routinely lead to meaningful issues that the therapist and client can productively use to make sense of and better understand current problems.

In the three illustrations that follow, the therapist has just heard and reflected a point of entry, and uses this to ask more directly about the client's developmental experience while remaining responsive to whether or not the client is receptive to these invitations. For example:

**THERAPIST:** I see, so that's how you've been disciplining your child. *{Responds to point of entry}*  
I'm wondering how your parents usually responded to you when they were upset with or disciplining you?

**CLIENT:** I'm not sure what you mean.

**THERAPIST:** What did each parent tend to say and do, and how did that usually leave you feeling?

OR

**THERAPIST:** Both dating partners you just told me about minimized your feelings. *{Responds to point of entry}* That makes me wonder, how did you expect your parents to respond to you when you were distressed or needed help? Would you seek help from your parents and talk to them when you had a problem?

**CLIENT:** No, not really.

**THERAPIST:** Help me understand your reluctance to go to them. What were they likely to say or do that you didn't want to occur again?

OR

**THERAPIST:** I'm so happy for you. This promotion at work is a really meaningful success—something that you ought to feel really happy and proud. *{Responds to point of entry}*  
Did you share this with your mother or father and if so, how did each respond?

**CLIENT:** Uh, I don't think so.

**THERAPIST:** Uh huh, let's try a thought exercise together. Will you close your eyes for a moment and visualize the biggest success experience of your childhood. Now imagine sharing this success with them, and describe the look each of them would have on their faces, and what they might say to you?

Especially in response to point of entry bids, but also in response to more direct queries at well-timed moments, many clients will relay narratives of troubling parent-child interactions that include themes of anxious love withdrawal, painful isolation, invalidation, fear, and shame. To emphasize, therapists can anticipate that often their clients will be able to remember and talk about these difficult or even disturbing interactions, yet the painful feelings that accompany them are unavailable. In particular, therapists will hear that many clients learned not to ask for or expect help and, after a while, *didn't even think of seeking assistance with substantive problems from their caregivers as an option or possibility*. To return to the heart of the attachment story: They were not secure in the expectation that their caregiver would be their ally when they were distressed and try to help them with their problem, even if that meant only to hear their concern and be with them in it.

### ADDITIONAL CLINICAL GUIDELINES

Parenting is probably the most challenging task in life. The influential family therapist, Salvador Minuchin, says, forgivingly, that parenting has always been more or less impossible. Almost all parents are trying to do the best they can for their children. Even many of the highly ineffective parents we have been discussing, who do indeed engender significant, lifelong problems for their children, are not cruel or ill-intended people in most cases. In child-rearing, as in other aspects of personality, people are uneven in their development. Most of these parents do other things well for their children, live by certain moral standards or codes, believe they are trying to do what is best for their children much of the time, and usually are treating their children better than they themselves were treated. Although children certainly are hurt by such developmental experiences, almost universally they still love their parents and seek their approval.

Therapists will do well to appreciate adult clients' willingness to give their caregivers another chance and their lifelong efforts to improve or repair these flawed but all-important relationships (Sampson, 2005). The therapists' aim is to help clients *realistically assess* whether or not caregivers have changed over the years and to what extent they are capable of responding better now than before. Some caregivers have gotten better and can respond more constructively now than when clients were young, whereas others continue to respond in the same problematic ways. The therapist's goal is to foster realistic expectations in their clients and help the current relationships between adult clients and their own aging caregivers become as good as they can be. In treatment, therapists encourage clients to express the full range of their positive and negative feelings toward their caregivers, and aim to be equally receptive to both. However, therapists can foreclose on this process by quickly echoing and remaining stuck on just the negative or positive aspects of the caregivers. This bias will rob their clients of the important experience of integrating and more realistically coming to terms with both the beneficial and the problematic aspects of their attachment figures and, in turn, reworking the IWMs that are shaping the relationships they have been choosing and co-creating. Over the long

term, clients who cannot resolve their ambivalence and integrate both the rewarding and the problematic aspects of their relationships with their caregivers will also have difficulty accepting the good and the imperfect parts of themselves—and of their own children.

As family theorists underscore, the therapist's role is to understand, not judge or blame (Satir & Bitter, 2000), and to help clients formulate a new narrative for their lives that more realistically fits what they have experienced (White, 1997). Thus, on the one hand, the therapist's role is not to foster splitting defenses by bashing parents who have been hurtful and making them bad, taking sides, or encouraging clients to reject attachment figures and break off contact. This problematic approach is often accompanied by a subtle inducement for clients to replace parental figures with idealizations of the therapist. Nor, on the other hand, is the therapist's aim to deny or in any way to minimize the real impact of hurtful interactions that have occurred. This is most commonly acted out by pressure from the therapist (or the family of origin) to forgive. Although this may sound resolving or healing in the abstract, too often the subtext really means just deny and forget. This leaves the client vulnerable to ongoing mistreatment from the same caregiver in their current relationship, or giving in to family pressure to “stop making trouble” and failing to protect the client's children now from the same people and problems that the client once suffered. Instead, the therapist's treatment goals are to:

1. Help clients come to terms more realistically with the good news and the bad news in their family of origin, and formulate a more accurate or realistic narrative for their shaping life experiences;
2. Help clients change their own responses to problematic others in current relationships; and
3. Actively coach and facilitate clients' current attempts to establish new relationships with others that do not repeat the problematic relational patterns that have come before.

Therapists provide a far-reaching CEE for clients who grew up with insecure relational ties when they can sustain a working alliance and remain emotionally present and available throughout the course of treatment and, of course, especially in those moments when clients are distressed or reaching for greater independence. Doing so provides many of the clients discussed in this chapter with the Safe Haven and the Secure Base they missed developmentally. Such consistent emotional availability usually has more effect on client change than do more dramatic but isolated incidents of compelling insight, important self-disclosure, or other significant therapeutic interventions (one trial learning versus a dependable pattern) (Holtforth & Fluckiger, 2012). However, countertransference derived from the therapist's own family of origin commonly makes it challenging for therapists to provide this consistent, corrective presence.

The clients whom we have been discussing share the experience that caregivers withdrew from them in one way or another, or were not emotionally available to them at important times of need. This developmental deficit—and the maladaptive relational patterns that result from the clients' childhood attempts to cope with these circumstances—can readily yet subtly be reenacted in the therapeutic relationship. For example, some therapists may have trouble being emotionally responsive to clients' pain and vulnerability. They may not be able to remain present with certain affects, such as the clients' raw shame or intense sadness, and move readily to problem-solving, reassuring, or other distancing defenses. The clients' healthy individuation may make other therapists uncomfortable because autonomy was not supported in their own development or because the clients' success evokes the lack of fulfillment in their own work or marriage. As clients improve in treatment or progress toward termination, other therapists may feel loss or anxiety about losing the caretaking role they were scripted to assume in their family of origin.

Most important, perhaps, as supervisors we watch many videotaped interactions where therapists disengage or give up on clients when they don't change or somehow disappoint the therapist, or become frustrated by clients' ambivalent, mixed signals to the therapist. This occurs, for example, with Preoccupied or Fearful clients who simultaneously *resist* the help they actively seek (e.g., Client: "Yes, but..."). Therapists too often give up on this back-and-forth client—routinely saying that "the client wasn't ready to change yet," rather than making a process comment and talking with the client about their current interaction. Therapists may feel they are failing or being controlled when the client is saying, "Help me," but not allowing anything the therapist does to help. Although from the vantage point of supervisors we often see these ambivalent clients subsequently trying to re-engage the therapist they have just succeeded in pushing away, rejected therapists often miss this approach side of their ambivalence and respond countertransferenceally by becoming irritated or blaming of the client and eventually withdrawing—which oftentimes reenacts the client's developmental experience.

When these common types of countertransference reactions occur, clients are again left alone and unconnected in their experience. Clients' pathogenic beliefs or maladaptive schemas are confirmed rather than resolved by this problematic interpersonal process. When such reenactments occur without being addressed and sorted through, old schemas and problematic relational expectations are confirmed by the interaction that is being played out in the therapeutic relationship.

In addition to their own countertransference tendencies, certain features in the client also make it difficult for therapists to remain consistently available. For example, many clients will report that they like and trust the therapist but they may also believe that, "if the therapist *really* knew me," the therapist would not respect or care about them. In line with their faulty schemas and problematic coping strategies, these clients believe they have deceived the therapist or manipulated the therapist's positive feeling for them. This is why it is necessary for therapists to help clients adopt an internal focus, and to use process comments to draw out the full range of clients' feelings toward the therapist—including the clients' perceptions of the therapist's reactions to them. For example:

**THERAPIST:** What do you think I am feeling toward you as you tell me this?

Sadly, clients often hold the pathogenic belief that their despised, weak, or otherwise unacceptable emotions constitute their real or true self. This far-reaching problem is resolved when the therapist sees the vulnerable, dependent, demanding, shameful, or other parts of themselves, which clients believe are the damning proof or confirming evidence of their basic unacceptability, and still remains affirming and emotionally connected. *Clients' encounter with compassion and understanding, rather than the judgment, emotional withdrawal, and criticism, that they have learned to expect, is one of the most powerful relearning experiences therapists can provide.* Thinking about conditions of worth, Carl Rogers (1951) emphasized long ago that clients do not change until they accept themselves, and that *they begin to accept themselves when they feel such acceptance from the therapist.* By providing a safe holding environment that contains these conflicted feelings, as clients are experiencing them, the therapist helps clients resolve them.

Finally, some clients are adept at *eliciting the same problematic responses from the therapist that they have received in the past.* As we have seen, a basic tenet of the interpersonal process approach is that aspects of most clients' problems will be temporarily reenacted with the therapist along the process dimension, and need to be identified, made overt, and reworked. It is easy to be empathic, warm, and genuine with clients who are usually cooperative, friendly, and respectful. However, it is far more challenging for therapists to remain committed to a working alliance with negative clients, whose interpersonal coping strategies (e.g., being provocative or dominating, critical

or controlling, passive or dependent, and so forth) may eventually alienate, intimidate, or frustrate the therapist (Najavitts & Strupp, 1994). With these negative clients, it is especially important for therapists to formulate working hypotheses about the clients' maladaptive relational patterns (Levensen & Strupp, 2007). Therapists need to keep process notes that include *hypotheses about what these clients have tended to elicit from others in previous relationships, and what they are likely to evoke or “pull” from the therapist*. When therapists prepare such case formulations, they increase their chances of being able to sustain an empathic stance, and are better able to “think before they act.” This allows the therapist to provide a CEE, rather than responding automatically to the client in the same types of ways that others typically do—which merely reenacts the same problematic scenario with which the client is so familiar (see Appendixes A and B). This is challenging for new therapists to see and do with their clients in the beginning, so we will explore it further in the next chapter.

In sum, clients will present with a rich diversity of developmental and familial experiences, and some clients' problems will be unrelated to the issues discussed here. However, insecure attachments, faulty child-rearing practices, love withdrawal disciplinary techniques, and conditions of worth will be heard pervasively throughout the narratives clients relay. They will shed light on current symptoms and problems, and help both therapists and clients make sense of what is going on in their lives.

## FAMILY INTERACTION PATTERNS

Linking family systems theory with attachment and parenting styles helps therapists discern and understand their clients' central problems and better formulate a focus for treatment. **Structural family relations** are the relatively enduring patterns of alliances, coalitions, and loyalties that exist in the family, and the clarity of **intergenerational boundaries** between these sub-systems helps shape how families function (Cowan & Cowan, 2005). These relationships are the basis for family organization and define how the family operates as a social system (Minuchin & Nichols, 1998). Such family patterns are strongly influenced by cultural values and beliefs that, in turn, shape norms and define acceptable behavior in family functioning. Thus, structural family relations, and the cultural context within which they are embedded, influence how members interact, as well as family communication patterns and the roles that family members adopt (Goldenberg & Goldenberg 2013; Teyber, 2001).

In families that function well, clearly defined intergenerational boundaries separate adult business from child business: that is, *roles and responsibilities are clearly differentiated between adults and children*. This means that adult caregivers provide an organized household with predictable daily routines for children, make decisions and plans for the family, and set limits and enforce rules (Minuchin, 1984). *Although parents have legitimate needs for companionship and support, these emotional needs are met primarily in same-generational peer relationships rather than through the children*. In contrast, children will have more problems when adult and child roles are not clearly distinguished, when intergenerational boundaries are blurred, and when too many adult needs are met through the children (Beavers & Hampson, 1993). With this general framework in mind, let's explore two common problems that new therapists are going to see every day in their caseloads: emancipation and parentification (Perrin, Ehrenberg & Hunter, 2013).

**Emancipation Conflicts.** Many children caught in cross-generational alliances with blurred intergenerational boundaries have problems with differentiation—especially in late adolescence and young adulthood as they try to emancipate from their family



of origin, explore career options, and begin making new relational commitments. If caregivers have a primary marital/partner coalition, or a single parent has supportive peer relationships, children usually have more support to grow up and successfully launch their own independent lives. With clear intergenerational boundaries, young adults usually find it easier to leave home successfully while still preserving a lifetime of close ties and shared involvement with their families of origin. They do not have to disengage emotionally or physically break off contact with the family in order to have their own independent life, or engage in more risky or extreme behavior such as drunk driving or teen pregnancy to separate and “get away.” This is possible because their caregivers do not need them to remain dependent or centered on them in order to fulfill their own lives or manage their own problems.

In contrast, if there is a primary cross-generational alliance between caregiver and child (e.g., mother and oldest daughter), these offspring may feel **separation guilt** over leaving their parent alone or forsaking their parent to the unfulfilling relationship with the other spouse/partner (Weiss, 1993). These offspring who lack a Secure Base find more sabotage than support for exploration and autonomy, and they are especially likely to feel dissatisfied with their achievements, to be unable to establish fulfilling relationships or make commitments to others, or to be chronically depressed. Guilt over emancipation frequently underlies academic failure in college, as well as many other symptoms and problems that late-adolescent and young adult clients present in college counseling centers (Teyber, 2001). Thus, when cross-generational alliances become primary, they often impose binding loyalty ties that make young adults feel guilty about leaving home, pursuing their own career interests, establishing satisfying love relationships, and in general, succeeding. Although emancipation conflicts are expressed differently in varying cultures, symptomatic guilt and depression often ensue when offspring are not permitted *culturally sanctioned avenues for individuation and couple formation*. This issue is especially challenging for many first- and second-generation immigrants whose new family and cultural values may differ significantly from those of their adopted country (Varghese & Jenkins, 2009).

Many clients, like Anna in Chapter 4, struggle with this binding separation guilt that stems from a lack of clear intergenerational boundaries. These clients, who often grew up with Preoccupied caregivers, or who are Preoccupied or Fearful themselves, may feel guilty about being happy, succeeding in life, or even getting better in therapy! It is an important part of providing a Secure Base with all clients, but with these clients in particular, it is essential that *therapists unambiguously enjoy clients' happiness and overtly express pleasure in their successes*. Some trainees are unsure of how to respond or what their role is when clients begin the session by saying happily, “*I feel really good today; I don't have any problems to talk about right now.*” Yes, when that happens, it may be a form of resistance, but for many clients struggling instead with separation guilt, they are apt to (inaccurately) assume, on the basis of their IWMs, that their happiness, success, or independence has somehow hurt the therapist or is unwanted. Other clients, who grew up with caregivers with Narcissistic or Borderline features, or in other ways were nonsupportive, competitive, or threatened by the client's differentiation and success, may be deeply concerned that the therapist also covertly feels resentful or envious of the happiness or success they have just expressed. The therapist provides a CEE and disconfirms such clients' guilt or faulty expectations—and expands the healthier and stronger ways in which they can be connected to others—by welcoming the client's good feeling and enthusiastically responding. For example:

**THERAPIST:** That's great! What's the best thing going on for you this week?

Without receiving such unambiguous affirmation of the successful aspects of their lives, these clients do not find a CEE in their real-life relationship with the therapist. Treatment will bog down and become repetitive as clients with these binding developmental experiences begin to retreat from their success. In line with their old expectations, they will keep sharing an inability to activate or initiate; feelings of discouragement, uncertainty, or inefficacy; and a lack of follow-through or inability to complete undertakings. Little or no progress in treatment will occur until this re-enactment is resolved.

As we will explore further in Chapter 10, these conflicts over emancipation and success often will be activated for clients who grew up with blurred intergenerational boundaries and binding cross-generational alliances during the Termination phase of treatment. In particular, clients who struggle with separation guilt may feel especially bad or worried about no longer needing the therapist and heading off successfully on their own. *Therapists provide clear and reparative boundaries when they give these clients permission not to have to need them, offer support for leaving treatment when they are ready to go, and overtly encourage them to be able to enjoy and pursue their own successful lives.* Although this would be obvious and unimportant to many clients, per client response specificity, these are profound and liberating words for clients with the developmental experiences we have described. Guilt over growing up and becoming stronger, succeeding in work and love, or even surpassing the parent or the therapist with more happiness or success is a common issue that many clients will covertly express in treatment, although few clients will be able to recognize it on their own. Fortunately, it is relatively easy to help many clients dispel these binding guilt-related beliefs by communicating repeatedly and consistently in words and behavior that the therapist enjoys their success, takes pleasure in their competence, and is in no way hurt or threatened by their stronger and more independent stance.

Finally, cultural factors play an especially important role in balancing the complex dialectic of emancipation and family loyalty. Family connectedness and enduring family ties may be of special concern to clients from strong relational cultures—for example, Asian or Latino. Entering the subjective worldview of these clients, therapists can learn to appreciate the subtle balance between being loyal to family and culture yet still possessing their own authentic self. To become familiar with the culturally sanctioned avenues for differentiation and emancipation that exist in every culture, therapists may find it helpful to consult with others who are knowledgeable about the client's cultural context (Jung, 1998). Similarly, it is important to ask clients how they self-identify, especially because variability exists within groups and in ways that individual members relate to their families and identify with their cultures (Cardemil & Battle, 2003). With collaborative exploration, therapists can help clients find role models within their own extended family, or culturally sanctioned ways to differentiate that are acceptable within their own cultural context (Pedersen, Crethar & Carlson, 2008).

**Parentification of children.** When intergenerational boundaries are blurred and the primary coalition in the family is a cross-generational alliance between a caregiver and a child, the result is often the *parentification* of children (Pasternak & Schier, 2014; Perrin, Ehrenberg & Hunter, 2013). A role reversal occurs: Rather than adults responding to the child's needs, the child takes on the role of meeting the caregiver's emotional needs (Boszormenyi-Nagy, et al., 1991). That is, when a parent's emotional needs are not met by his or her partner or other (same-generation) peer adults, the caregiver inappropriately turns to one or more of the children to meet his or her own adult needs for: affection and intimacy; approval and reassurance, or stability and control. Therapists are alerted to the likelihood of such parentification when they hear adult caregivers describe their children as their “best friend,” “lifeline,” or “confidant” (Teyber, 2001).

Although this may seem benign on the surface, it is highly problematic for children when they become responsible for, or take care of, the emotional needs of their caregiver. The problem with this role reversal is that such children must give to their parents rather than receive, and their own age-appropriate attachment needs go unmet. Further, parentified children often continue playing out this familial role in adulthood, typically leading them to feel overly responsible for others, insecure about depending on others, and guilty about having any need of their own. As adults, parentified offspring may describe themselves as feeling empty or “having a hole inside of me” as a result of the deprivation of having given rather than received throughout their childhoods, and as feeling “burned out” as they continue in adulthood to excessively prioritize the needs of others over their own. Commonly, parentified children initially enjoy having such a special role with their caregiver but, as adults, they often come to resent having been deprived of their own childhoods. As adult clients presenting in treatment, they often describe being angry and/or anxious in current relationships because they:

- (1) Do not trust that others will be there for them in times of distress;
- (2) Are anxiously preoccupied about losing relationships when they cannot meet the needs of their significant others; and
- (3) Feel resentful about the unfairness of giving and not getting in important relationships.

This topic is especially important because parentification is pervasive in the background of therapists as well as clients (DiCaccavo, 2006). Some clients seeking treatment were parentified by Neglectful caregivers who asked them to fulfill adult caregiving responsibilities, such as assuming the primary role of feeding and caring for siblings. Perhaps more common and subtle, in contrast, Preoccupied caregivers often confused the parentified child by being warm and communicative, and seemingly responsive to their emotional needs. However, the tables often turned quickly, but subtly, and soon the parent’s need, not the child’s, was being met in their dialogue or interaction. Because their basic relational orientation was to take care of others, children who were parentified often select careers in helping fields such as nursing, ministry, and counseling that extend this caretaking role and fit their childhood script and identity. Although kind and capable, they often feel guilty about saying “no,” setting limits, and responding to their own interests and needs. Because they do not draw boundaries well, they tend to become over-identified with others’ problems and, over time, become especially prone to experiencing burnout in their work. Because they grew up having to take care of their parent, it is now threatening to relinquish this exaggerated sense of responsibility and need to be excessively in charge or in control in their current lives. For example, it may be hard for them to let willing others share in meeting obligations, even though simultaneously they resent having to handle everything themselves. These control issues may also be evidenced in symptoms such as airplane phobias, for example, where these individuals must temporarily relinquish control to the pilot, or to another driver. These clients also have problems in close personal relationships because it is too anxiety-arousing to relinquish the control necessary to be emotionally intimate with someone.

However, in some families—for economic, single-parent, medically challenged, or other valid and necessary reasons—one child may temporarily assume a parental role with younger siblings. This is not the same as parentification because it remains clear that the parent is in charge when the parent is home or present. This child’s role—to assist in the functioning of the home in the caregiver’s absence—is temporary or situational. Furthermore, the child is caring for siblings, not for the emotional needs of the parent. This responsible and helpful child, who still remains a child and is nurtured and given to without the role reversal that occurs for parentified children, often grows up to be an especially resilient and capable person with significant personal strengths.

A cross-cultural example would be children who translate for their parents but don't take over the parenting role.

Parentified clients will be highly sensitive and responsive to the therapist—as they once were to their caregiver. Especially for beginning therapists, it can feel great to work with these clients in the short run because they astutely discern what they need to say or do to help the new therapist feel competent or secure. For example, these clients may talk extensively about the positive changes occurring in their lives, and even exaggerate them, so that their student therapist's supervisor, watching on videotape, will be approving of the therapist! However, if the therapist does not collude in reenacting this relational pattern, clients can begin to explore the far-reaching consequences of having been parentified and having missed or been deprived of their own childhoods. The opportunity to provide an experience of change occurs when the therapist begins to tentatively wonder aloud or make process comments that highlight when this relational pattern may be occurring in the therapist-client relationship as well. For example:

**THERAPIST:** I really appreciate the genuine concern you just expressed for me right there—I know it's caring and genuine. But thinking about the caretaking patterns we have been talking about, and how much that role has been costing you in your life, it leaves me wondering what happens to your needs here if you get oriented to what I may be needing or wanting? What do you think—does my concern make any sense?

To further clarify parentification, let's consider a case study.

*Jasmine was a highly regarded psychiatric nurse. An utterly dependable and take-charge person, she could seemingly handle every situation that arose. In a hospital emergency setting where she had to deal with seriously disordered patients in crisis, her rapid and accurate assessments, good judgment, and compassion for patients had earned her the respect of the entire staff. Although considered a superstar at work, Jasmine sought therapy for her recurrent depression and "burnout."*

*Jasmine was a bright and engaging client who was subtle yet adept at getting the therapist to lead, talk about herself, and become involved in collegial discussions about interesting clinical issues. However, the therapist had formulated working hypotheses about this potential reenactment and was usually effective in recognizing Jasmine's strong "pulls" to repeat this pattern with her. Repeatedly, she looked for points of entry to focus Jasmine inward by saying, for example:*

**THERAPIST:** Help me understand what was going on for you when...

OR

**THERAPIST:** What were you afraid was going to happen if you said no and... ?

*In response to these repeated invitations to consider what she was thinking, feeling, or wanting, instead of living outside of her own skin and being preoccupied with what the therapist might be wanting, Jasmine began expressing that it was new for her to pay attention to herself and her inner experience in this way. And, although she enjoyed this very much, she also found that it made her feel "uncomfortable." The therapist did not press for this internalizing focus when Jasmine did not want it, but instead asked Jasmine on occasion how it was for her to look within at her own experience. It soon became clear to both of them that guilt over being "selfish" and shame over being "the center of attention" were evoked by this internal focus. Thanks to the therapist's affirming responses to these reactions (e.g., "Oh no, I don't think you're being selfish here at all..."), both soon agreed that important new material was cohering.*

*Four weeks into treatment, Jasmine disclosed something that she had never told anyone: Her alcoholic stepfather had sexually assaulted her in her early adolescence. Jasmine successfully fought him off, although she was scratched, had her blouse torn open, and suffered a bloody nose when she literally pushed him away from her. Jasmine recounted her ordeal in detail, but with very little emotion, and the therapist responded effectively in caring and validating ways. The therapist, herself a mother of two daughters, soon asked the question that was burning for her:*

**THERAPIST:** You have kept this painful secret for almost 20 years. I'm glad that you can share it with me now, but it's heartbreaking that you've had to be alone with this for so long. What kept you from telling your mother and asking her for help?

**CLIENT:** I didn't want to put more on her—her life was kind of overwhelming already. She couldn't have done much anyway, and I didn't want her to worry.

*Jasmine's poignant response illustrates the plight of the seriously parentified child. As in most aspects of her relationship with her mother, Jasmine's own profound needs for protection and comfort were set aside in order to meet her parent's need. Here again, we are seeing the heart of the attachment story. Jasmine, like all Insecurely attached children, cannot even imagine the possibility of taking her problem to her attachment figure for help, not even problems as significant as this.*

Therapists will see the problematic consequences of these structural family relationships with blurred intergenerational boundaries and cross-generational alliances operating with many of their clients. This is one reason why therapists need to honor treatment parameters and provide these clients with a CEE by maintaining clear therapeutic boundaries. It is especially valuable for new therapists to have their supervisors actively help them monitor this process dimension of clear therapeutic boundaries with their clients.

## CLOSING

The developmental issues presented in this chapter offer many important parallels to the relationship between the therapist and client. Effective therapists want to be a Safe Haven where clients feel safe bringing their problems and vulnerability. Then, when provided with this safety, clients will be able to utilize the therapist as a Secure Base to explore and try out new ways of responding to themselves and others. We believe this sequence leads to deep exploration in treatment and meaningful change in their lives. Thus, the personal challenge for every therapist is to examine their own capacity to provide both a Safe Haven and a Secure Base to their clients—which has been shaped in their own families of origin. We encourage trainees to strive to be non-defensive and challenge themselves to expand the interpersonal spheres that are anxiety-arousing for them, and to sustain a lifelong intention to broaden their own interpersonal range so they can respond to the wide range of problems that clients present.

We have also addressed the common misconception that adopting a developmental perspective focuses on obtaining psychodynamic insight, and is used to blame parents as “bad” or to make them responsible for clients' lives and choices. Instead, the intention is learning more about what's really troubling clients and gaining a richer context for understanding what isn't working in their lives. The far-reaching characteristics of family life presented here will inform therapists and clarify a treatment focus, helping them make sense of their clients' current problems and maladaptive patterns. Student therapists frequently ask their supervisors, “What should I do?” However, supervisors can offer only broad and general guidelines that are of limited usefulness, until they begin to



*contextualize* clients' problems in a broader developmental/familial/cultural perspective that sheds light on the harder question that needs to be explored first: "What does this mean?"

Finally, for some trainees, it is disruptive to read this developmental material because it violates familial rules and cultural prescriptions to question or examine what transpired in their families of origin. Further, some readers are parents or grandparents themselves. As these readers learn more about family functioning, parenting practices and, especially, their own attachment styles, unwanted guilt may be evoked. Never helpful, this guilt may result from recognizing the previous limits of their own parenting skills, and the problematic consequences that ensued for their own children and loved ones. We hope that such readers will be able to relinquish unrealistic expectations for themselves, and forgive themselves, for not knowing more about child-rearing than they did at the time.

## CHAPTER EXERCISE

What is your current style of adult attachment? If you grew up with an Insecure attachment style, have you been able to make progress toward earned security? If so, what were the experiences or relationships that helped you? If not, what kind of help might you need, or what kind of experiences might you seek, to facilitate greater earned security?

## SUGGESTED READING

Therapists interested in learning more about working with Preoccupied and Dismissive clients, and in preparing case formulations and treatment plans from this relational/attachment perspective, are encouraged to read Daly & Mallinckrodt (2009), "Experienced Therapists' Approach to Psychotherapy for Adults with Attachment Avoidance or Attachment Anxiety," *Journal of Counseling Psychology* 56, 549-63



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes.

# Inflexible Interpersonal Coping Strategies

### CONCEPTUAL OVERVIEW

In this chapter, we continue to help therapists conceptualize their clients' symptoms and problems by focusing on disruptive IWMs and interpersonal coping strategies. They highlight the key concerns that pervade clients' lives and comprise clients' **core conflict**. They are the hub of the wheel that links the different problems and concerns that clients present and provide a focus for treatment.

Clients' core conflicts arise from the combined effects of insecure attachments, problematic child-rearing practices, and faulty structural family relations. When these types of problematic interactions with caregivers were *repetitive or ongoing*, they gave rise to disruptive IWMs with pathogenic beliefs about oneself, faulty expectations of others, and a narrow or skewed view of what will usually occur in close relationships (Wachtel, 2008). To cope with the painful feelings, low self-esteem, and interpersonal threats that accompany these developmental experiences, clients also develop a fixed interpersonal coping style (McCarthy et al., 2008). For example, many clients characteristically: please others, strive to be perfect, and are compliant; others repeatedly take charge, seek control and power; while others consistently remain invisible, avoid all conflict, and exhibit rigid self-sufficiency (Coolidge et al., 2010). The key here is that this **interpersonal coping strategy** is pervasively and rigidly overused—even in situations where it is not necessary or no longer adaptive. (See Millon & Grossman, 2007 for a comprehensive overview of interpersonal coping styles and their implications for adaptive and maladaptive behavior.)

Defenses such as these are necessary for all people at times—they help us cope in difficult situations. Clients will feel deeply understood and “seen” when their counselors can grasp and validate how one of these interpersonal coping styles was a necessary and adaptive way to cope with reality-based problems at one point in time. For example, being pleasing or compliant kept a child from being ignored or criticized by an anxious or depressed parent, or withdrawing and not being noticed was an adaptive strategy to keep from being hit by an alcoholic parent. The problem for most clients, however, is *that these coping strategies are no longer necessary or adaptive in most current relationships*, but they are still being used habitually. These strategies have become ingrained behavior patterns and, rather than continuing to help, they now contribute to many of the

symptoms and problems that clients present in treatment—such as speaking up when needed (Baumeister & Scher, 1988; Pezzarossa et al., 2002; and Wei & Ku, 2007). *The purpose of this chapter is to help therapists recognize and assist clients differentiate with whom they are still adaptive and when they no longer help, and clarify this as an important treatment goal* (Denckla et al., 2015). On the one hand, the counselor aims to help clients sort through current relationships where they may still need to respond in this pleasing or avoidant way (e.g., with an authoritarian, dominating boss). On the other hand, it's important to discriminate other relationships (such as with a new friend or potential partner) where the old coping style (such as always being in control) is no longer needed and only serves to create new problems.

To reach this treatment goal, the therapist's intention is to:

1. *identify the formative patterns and painful feelings that make up the client's core conflict,*
2. *highlight the interpersonal coping strategy the client has learned to cope with these developmental and familial challenges,*
3. *clarify when and how the client's interpersonal coping strategy is being expressed in current interactions with the therapist and others, and*
4. *begin changing this pattern first in the relationship with the therapist, and then with others in the client's life.*

As new therapists become more successful at formulating the client's core conflict and interpersonal adaptation to it, this conceptualization will help counselors recognize themes and find organizing patterns in the disparate material clients present (Barber, 2002; Levensen & Strupp, 2007). As introduced in Chapter 2, client conceptualization is an ongoing process that begins with the initial client contact. General hypotheses about clients' problems are formulated early in treatment, and these *tentative working hypotheses* are further refined or discarded as the therapist learns more about each client. The key concerns that are central to the client's problems emerge and become clear as the therapist focuses the client inward, the client's feelings emerge, and the therapist works with the process dimension to explore the client's reactions to and expectations of the therapist and others. The therapist can then use these key guideposts to clarify central conflicts and focus on changing how they are being played out with the therapist and others.

## INTERPERSONAL FRAMEWORK FOR CONCEPTUALIZING CLIENTS

In this section, we explore an interpersonal framework for conceptualizing the client's core conflict—how it originally developed, the client's interpersonal adaptation for coping with it, and how it is being expressed in current symptoms and problems (Cresswell et al., 2011; Quirk et al., 2015; Rapee, 1997, 2012; Yap & Jorm, 2015). This conceptualization of the client's problems includes four components:

1. The client's unmet attachment needs
2. The client's IWMs and what he or she came to believe about Self and Others based on developmental challenges
3. The client's interpersonal strategy to manage or cope with anxiety
4. An interpersonal resolution of the core conflict

Adapted from Horney's (1970) interpersonal theory, these four components help therapists conceptualize their client problems (see Figure 7.1). After exploring these disruptive IWMs and rigid interpersonal coping strategies more fully, we will apply them to treatment and illustrate them with three case studies. Below we begin by: (a) exploring

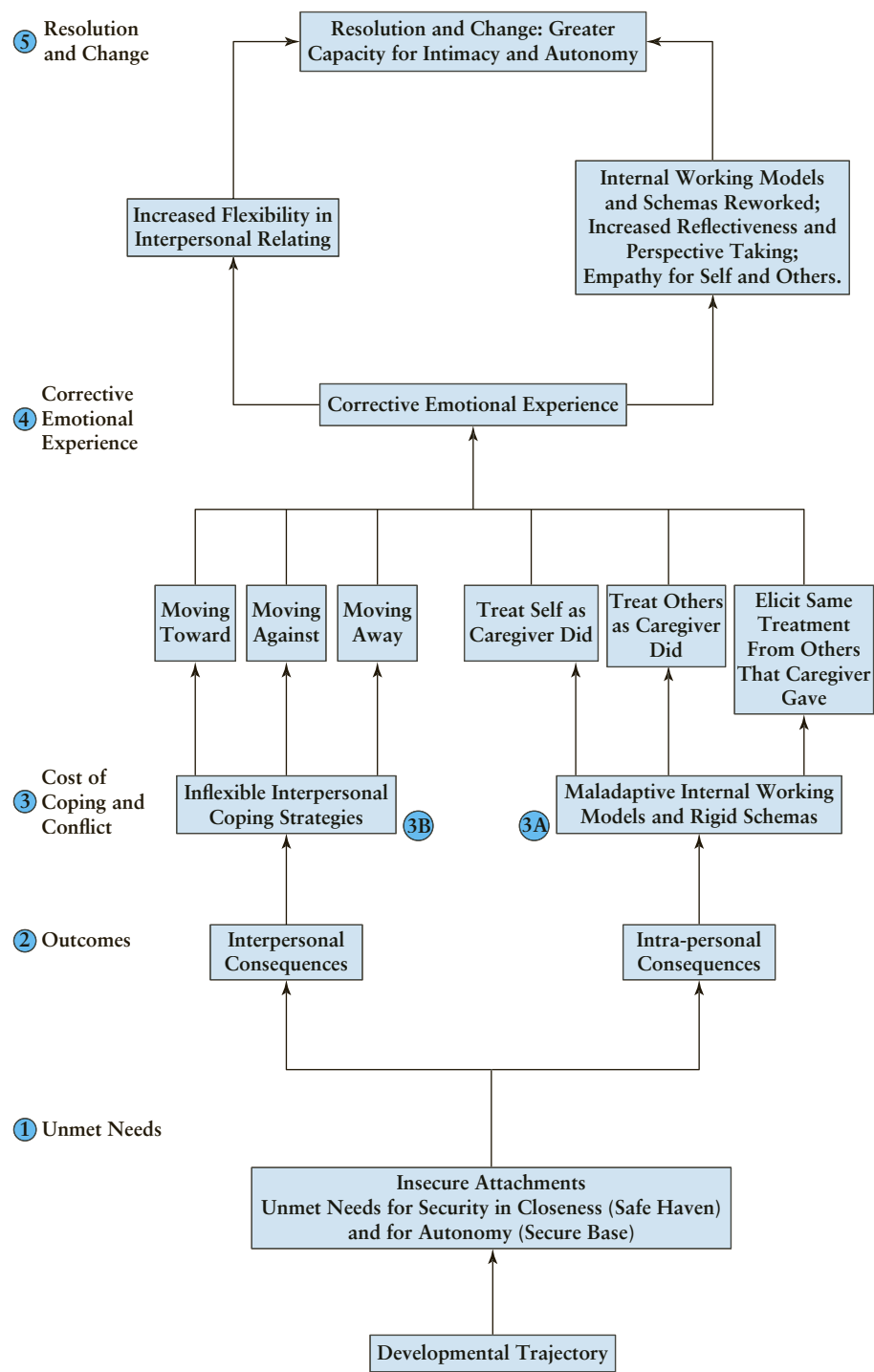


FIGURE 7.1 | INTERPERSONAL MODEL OF THE CHANGE PROCESS

how enduring problems result when one or more of a child’s basic developmental needs go unmet, and then (b) seeing how disruptive IWMs and rigid interpersonal coping strategies lead to the symptoms and problems clients present in treatment.

## UNMET DEVELOPMENTAL NEEDS

Figure 7.1 starts at the bottom of the page and flows upward. It highlights young children's emotional needs and their developmental trajectory when these needs for a secure attachment are not met. If parenting figures are consistently responsive to the children's bids for comfort when distressed (Safe Haven), and support for exploration when curious (Secure Base), children will be able to express these needs freely. Of course, as children develop, they benefit from learning to accept limitations on parents' ability to respond and to tolerate delays in parental response. If parents are not usually responsive, however, anxiety will soon become associated with the child's needs.

Children will then develop IWMs based on their parents' unresponsiveness—which continues to shape their feelings and perceptions about themselves and others on into their adulthood (Bogels & Brechman-Toussaint, 2006; Quirk et al., 2015). For example, consider preschool children whose need for understanding and affection goes unmet from disengaged, authoritarian, or even permissive parents. When such children approach their caregivers for comfort, their need is often rebuffed with statements such as:

**DISENGAGED:** What do you want now? You always need something—get out of here!

**AUTHORITARIAN:** What's wrong with you? Stop that crying right now, or I'll give you something to cry about!

**PERMISSIVE:** It's OK, honey. Go ahead and do whatever you want, I'm kind of busy right now.

If these types of interchanges occur repeatedly, or come to *characterize* parent-child interactions, these children will soon learn to anticipate feeling criticized, rejected, or ignored, and become anxious whenever emotional needs for comfort or soothing are aroused. More enduringly, they will also develop IWMs of Self as unworthy of being responded to and Others as being unable to respond—which they will carry into future relationships (Wachtel, 2008).

Children are biologically organized to continue experiencing attachment needs, but their direct expression is minimized or denied (defended against) by the parents' unresponsiveness (see Part 1 at the bottom of Figure 7.1 on page 226). The attachment-seeking child is then compelled to find some way to cope with this painful set of circumstances. Although clients will present with a wide range of problems that result from the varied developmental challenges presented in Chapter 6, enduring psychological problems often begin when basic childhood needs for secure attachments (both Safe Haven and Secure Base) are not met. This failure to provide children with clear communication and consistent emotional access to their caregiver when distressed, and support for autonomy and differentiation when secure, results in adult clients who present with problems that involve difficulties with commitment and intimacy, or with self-direction and autonomy, or both (Otani et al., 2014). As we saw in Chapter 6, children who grow up in a predictable environment with both affection *and* consistent limits have the best developmental trajectory (Kobak et al., 2006; Sroufe et al., 2005). However, most children experience anxiety when their emotional needs are consistently not met and, later as adults, *they also feel anxious when adult versions of these attachment needs or feelings are activated in current relationships* (e.g., when clients have a serious problem and have difficulty asking a spouse for help or sharing their distress).

Parts 2 and 3 of Figure 7.1 illustrate IWMs and interpersonal coping strategies. On the one hand, clients turn against themselves to minimize the feeling or need that has become anxiety-arousing (e.g., "I hate myself. I have such a big mouth—I'm too much for everybody"). Simultaneously, clients also implement interpersonal coping strategies to manage the unacceptable need and its accompanying anxiety (e.g., "If I just get a little



thinner, then he won't leave me"). Part 4 of Figure 7.1 refers to the CEEs that therapists aim to provide in the therapeutic relationship, and then generalize to their everyday lives, to help clients resolve their core conflicts and live more effectively (Bridges, 2006). We will see how clients' inflexible coping styles are commonly replayed in the current relationship with the therapist, just as they are with others in their everyday lives, providing a powerful opportunity for experiential relearning.

## INTERNAL WORKING MODELS AND INTERPERSONAL COPING STRATEGIES

In Chapter 2, we proposed three domains for conceptualizing clients' problems: (1) maladaptive relational patterns, (2) rigid schemas or IWMs with faulty beliefs about self and unrealistic expectations of others, and (3) core conflicted feelings. All three of these domains are intertwined, but the Insecure attachment based on unmet needs in Part 1 of Figure 7.1 principally evokes the core conflict. As we will see, the faulty beliefs that arise from these core conflicts predominate in Part 3A, and the interpersonal strategies used to cope with them—which are going to be the primary focus of this chapter—are highlighted in Part 3B.

Clients employ two psychological mechanisms to defend against the anxiety associated with expressing, or even experiencing, their unmet attachment needs. First, clients respond to themselves in the same way that significant others originally responded to them (i.e., they develop IWMs that are based on how caregivers treated them; they are not simply irrational beliefs unconnected to their learning histories). The IWMs that clients develop become enduring ways of believing who they are, how others will respond, and regulate their behavior. Second, clients' inflexible and over-generalized coping style is an attempt to diminish anxiety by trying to overcome or manage their personal or emotional needs. Clients avoid their unmet attachment needs because they are too painful and disruptive to continually experience and address. In treatment, this interpersonal coping style often quickly becomes apparent in the way clients interact with the therapist (e.g., characteristically pleasing/complying, controlling/competing, or withdrawing).

These two mechanisms of denying or avoiding anxiety-arousing thoughts, feelings, and needs on the one hand, while trying to overcome them on the other, represent attempts to minimize anxiety and provide a modicum of self-regulation, efficacy and esteem. They are clients' solution to their core conflict, and together they generate many of the symptoms and problems clients present in treatment. Let's see how this defensive system once helped clients function better, but now costs dearly and is no longer adaptive.

## INTERNAL WORKING MODELS SHAPE SYMPTOMS AND PROBLEMS

With the limited skills and means available to them, insecurely attached children try urgently to get their caregivers to respond to their emotional needs but are unable to elicit the help they need. This may be reflected by a general lack of parental responsiveness, parents responding on their own timetable rather than when the child initiates contact or expresses a need, or a punitive or inconsistent parental response. To gain some active mastery over their helplessness and to ward off the anxiety aroused, *such children begin to minimize or sequester away their own emotional need in the same way that their caregivers originally responded to it* (Otani et al., 2014). For example, children may deny their need or distress as insignificant and regard themselves as unimportant ("I'm stupid"; "I don't matter") or may reject it and view themselves contemptuously as demanding and dependent ("I'm too needy"). Routinely, counselors will hear clients say the same hurtful responses toward themselves that others originally expressed toward them "Why are you so bossy!"). In turning against themselves in this way, children are able to ward off the unwanted anxiety that would be engendered if they felt that their developmental

needs were reasonable and legitimate but still viewed as unacceptable to those they need and love most—their caregivers. This adaption provides a sense of control and preserves attachment ties ( “There’s just something wrong with me”), and allows them to continue to view the world and their parents as reasonable and just (Janoff-Bulman, 1979, Weiss, 1995). In this way, a certain kind of attachment security or safety comes from viewing themselves as the problem, rather than the caregiver. This then becomes their IWM for viewing themselves and others; by acting accordingly, they elicit responses consistent with the IWM. For example, when they ask for something (e.g., a pay raise, a day off from work, help on a project) they do it in a way that conveys that they don’t really expect that their request will be granted. Consequently, more often than not, their requests go unmet. This adaptation where children begin to do to themselves, and elicit from others in current relationships, aspects of what was originally done to them also has been referred to in different orientations as modeling and identification with the aggressor.

Therapists will often see clients employing three types of disruptive IWMs that help avoid anxiety or manage distress, but these defenses subsequently cause further problems for them:

1. First, clients respond to themselves in similarly hurtful ways that others have responded to them. When adult offspring of non-responsive parents feel vulnerable or threatened, their anxiety-arousing need for reassurance or affection will still be evoked. They will cope with this anxiety by seeing their need as their caregivers once did—and *respond to themselves in the critical, judgmental or contemptuous way that others once did*. As adults, their emotional needs will still be unacceptable, and they will often say to themselves the same critical or dismissive things they heard from others years ago. This may include adopting the exact words and tone—but without recognizing the source of these internalized “tapes” as that of their own attachment figures. Further, the judgmental, punitive, ignoring, or rejecting affect that clients feel toward themselves as they replay these internalized tapes is the same affect that attachment figures originally expressed toward them years before (e.g., “You are just a pest!”).
2. Second, clients avoid experiencing unwanted developmental feelings and memories by re-creating or reenacting in current relationships the same types of interpersonal scenarios that originally caused distress or fear in earlier, formative relationships. That is, on an interpersonal level, *some clients will say and do to others what was originally done to them*. For example, the child of strictly authoritarian or dismissive parents may grow up to be a parent who similarly disparages the same emotional needs in his or her own children—particularly the child who most resembles the parent in gender, birth order, temperament, skin color and so forth. Especially in the arena of parenting, clients who have not integrated or come to terms with their own conflicted feelings, or clarified a new narrative for their lives that more accurately fits what behaviorally occurred, are prone to reenact the same maladaptive relational patterns that they experienced in their own childhoods (Siegel & Hartzell, 2014). As parents, many clients feel helplessly dismayed as they watch themselves respond to their children in the same hurtful ways that their parents responded to them. Despite sincere pledges that “I will never do to my children what my parents did to me,” variations of the old themes can occur with clocklike regularity. Adult offspring gain or “restore” some control over their own dilemma by repeating what was originally done to them rather than knowing and experiencing what they once saw and felt.
3. Third, some clients avoid the anxiety they experienced developmentally by making it a current issue with others—thereby keep it from being a familial or developmental problem of their own. They do this by *eliciting* the same unsatisfying response from others in current relationships that they received in their attachment dilemma.

For example, unless children with significantly problematic developmental experiences have other reparative relationships (with aunts and uncles, friends, teachers, and mentors) to help them resolve these concerns, they are likely, as adults, to select a marital partner who cannot respond well to their emotional needs. In the event that their spouse is capable of being affectionate or intimate, they may not be able to accept this emotional responsiveness. Why? The possibility of having a response to their old, unmet needs—even a well-intended, benevolent response—will often arouse the shame or evoke the anxiety of the original deprivation. Following their IWMs, such clients also will anticipate that their spouse will reject or control them as others have done in the past—even though the spouse has not been responding in this unwanted way.

Operating on schemas or IWMs that fit past relationships better than current ones, such clients may simultaneously elicit and reject nurturance from their partners. This will certainly frustrate and confuse the spouse, especially if these mixed messages activate the spouse's own problematic relational templates, as they commonly do. Unresolvable marital conflict that results in emotional gridlock occurs when the partners' IWMs dovetail and each presents the other with a version of their old relational pattern (Hendrix, 2001). For example, consider a husband with a Dismissive attachment style who is married to a Preoccupied wife. Confirming each other's worst fears, his lack of responsiveness and communication will make her feel even more anxious and alone, while her need for emotional support and reassurance will be construed by him as more of the intrusive, demanding control he has learned to expect and abhor in relationships. In this mutually escalating cycle, potential outcomes include: ongoing and intensifying conflict that erodes the rewarding qualities in the marriage; emotional disengagement from each other and settling into parallel lives; embroiling children in the ongoing wrangling and pressuring them to take sides; or ending in acrimonious divorce unless counseling helps one or both partners recognize and change their mutual interaction.

Although one of these three modes may be predominant for a particular client, many clients will employ all three mechanisms. In all three maneuvers, however, clients turn a passive experience that originally happened to them into an active experience over which they now have some control. Therapists can begin to help clients change by wondering aloud or being curious about them.

## INTERPERSONAL COPING STRATEGIES

It is important for new therapists to recognize the defensive interpersonal styles that clients adopt to cope with their developmental problems. Children's unmet needs and developmental problems do not just dissipate as they grow up (unless their caregivers change over the years and begin responding in new and better ways, or they have been able to establish reparative relationships with others). These disruptive needs and feelings may be denied but they continue to be evoked in adult relationships, *especially in the adult attachment settings of their own marriages and in parenting*. When these needs and feelings remain too anxiety-arousing or unacceptable to be expressed or dealt with directly in current relationships, clients try to cope with them by adopting various interpersonal coping styles. To illustrate these interpersonal coping strategies, we will focus on Horney's three lifestyle adaptations: moving-toward, moving-away, and moving-against others (Horney 1970). They have been widely adopted by many theorists, and a large body of empirical research finds that these rigid coping strategies have been learned in problematic caregiver-child and insecure attachment relationships. They provide short-term benefits but long-term adjustment costs (Hoffman et al., 2013; Rubino et al., 2004; Schill & Williams, 1993; Wei & Ku, 2007; Gottman & DeClair, 2001).

## INFLEXIBLE INTERPERSONAL COPING STRATEGIES: MOVING-TOWARD, MOVING-AGAINST, AND MOVING-AWAY

Clients adopt a fixed interpersonal style both to reduce the anxiety associated with having unacceptable needs and feelings revealed (both to others and to themselves), and to find some sense of identity, value or self-worth. Let's look first at Horney's three coping styles—they can help therapists so much as they try to understand and respond to their clients.

1. **Moving-Toward:** Some clients learned to cope with problematic familial interactions by **moving-toward** or pleasing people. These clients learned they could earn some needed approval, and diminish the threat of further rejection criticism or being ignored by excessively *complying* with their caregivers and being unfailingly good (e.g., always being helpful and nice, striving to be perfectly well behaved). These children are not learning to be appropriately well behaved. Instead, in order to ward off anxiety and maintain their vulnerable self-esteem, they are giving up too much of themselves and their own identity. In this pervasively pleasing stance of wanting to be liked and accepted, clients have lost their own voice and the ability to have their own mind (Smith, 2007). Generalizing this coping strategy they have adopted with caregivers, they carry over to others in adulthood this pattern of relating in which they always try to please or accommodate others and avoid disagreements. They don't express their own interests or preferences, or assert their own limits and boundaries. These clients have learned to defend against anxiety and win some approval by consistently moving-toward others in a pleasing, servile way. This coping strategy is commonly found in counselors, nurses and others who choose a caretaking career.
2. **Moving-Against:** Other clients have learned that aggression or resistance to parental wishes, if pursued long enough, is a way to ward off pain or manage insecurity. They may have needed permissive caretakers to act more strongly and take effective charge of the family so they could feel secure without having to try and take control of everything themselves. Or, they may have identified with, or were supported by the other parent, in defying a dominating caregiver. These expansive and dominating individuals cope by **moving-against** others. They want to be in complete control of themselves and their emotions at all times and, even more problematic, they dominate, intimidate, or try to exert too much control over others. They approach relationships competitively with the orienting attitude that they must win at all costs, and they often assert themselves aggressively for this purpose. Their mindset is that they need to *prevail* in every situation or conflict. Thus, these clients adopt a repetitive interpersonal style of moving-against others to protect themselves from the unwanted feelings evoked by needing someone, being in situations where they are not in charge. They are motivated more by having power and influence over others than by having intimacy or egalitarian relationships (Fetterman et al., 2015).

The beginning therapist may dread moving-against clients because they can be challenging or critical of the therapist and may try to control or compete with the therapist. Researchers find that many therapists have trouble with clients' anger toward them (Hill et al., 2008) and that these negative, angry, or provocative clients frequently disrupt the treatment process (Binder & Strupp, 1997; Matsakis, 1998). Practicum instructors and supervisors need to help new therapists prepare for these workable, but initially challenging, clients by role-playing and rehearsing effective, non-defensive responses to clients who challenge the new therapist's credentials or competence.

3. **Moving-Away:** In Horney's third interpersonal style, clients have learned that the best way to reduce the interpersonal threats they have grown up with and create some safety for themselves is by **moving away** from others through physical

avoidance, emotional withdrawal, or total self-sufficiency. These clients do everything for themselves and do not ask for help, even when that may be necessary and appropriate. They move through life having no need of others—they often expect rejection or rebuff, and attempt to take care of all their needs on their own. This tendency to move away from people and be avoidant is often shaped by a heightened sensitivity to rejection and shame (Millon, 1999; Otani et al., 2014).

Because life is stressful for everyone and all people need defenses, healthy individuals will also employ these three interpersonal styles at times. When they do not become exaggerated or used pervasively, there are adaptive strength features in them (Smith, 2007). However, they are not an identity or way of living for well-functioning people who use these strategies flexibly to cope with a difficult person or problematic situation. With most clients who enter treatment, however, one style has become predominant or characteristic. It is overused, inflexibly, even in situations where it is not adaptive or necessary and is not helping the situation. As we will be discussing further, the therapist's role is to help clients learn to discriminate or assess more realistically when and with whom they need to cope in one of these ways, and when and with whom they don't.

### **RIGID INTERPERSONAL COPING STYLES: A DEFENSE FRAMED AS A VIRTUE**

When developmental problems were pervasive or severe, clients often adopt one of the three interpersonal styles: moving-toward, moving-against, or moving-away. This **inflexible coping style** is a habitual way of relating to others that is also reflected in vocational, marital, and other defining life choices. There are many exceptions, of course, but individuals who move toward may find that careers in nursing or counseling are a good match for their caretaking skills and needs. Individuals who move against may find that careers in law, medicine, or money management are congruent with their interpersonal style of taking charge and needing to be in control. Individuals who move away may feel comfortable as researchers, artists, technical experts who work alone, or individuals with a meditative lifestyle. In their self-concept, Horney suggests many clients will privately frame this defensive coping style as a special attribute or virtue and use it to feel special and derive some self-esteem or identity from it. However, this unrealistic sense of being special is compensatory and often reflects a shame-based sense of self. These clients' self-worth is brittle and vulnerable—it relies too much on their ability to rise above or disprove their core conflicts by pleasing others, by achieving and succeeding, or by withdrawing and feeling cynically superior.

More contemporary therapists write about the “narcissistic” element in many different types of clients who are defending against a shame-based sense of self (Kohut, 1977; Stolorow et al., 1994). They describe what clients do to “restore” their sense of self-worth or power when they (too readily) feel shamed and over-react to feeling rejected or diminished by others or that they have failed in some way (see also Thomaes et al., 2007)—sensitive issues to work with, certainly, yet relieving for clients when counselors can take the risk and find respectful ways to broach these far-reaching issues.

Even though moving-toward clients are compliant and submissive, they do not experience themselves as servile. Instead, they often feel special—more caring than others who are not as compassionate. They accomplish this distortion by perceiving themselves as selflessly loving; always altruistic, sensitive to the needs of others, or committed to the ideals of peace or justice. In this way, the defensive interpersonal style becomes a way to cope with a lack of self-esteem—a virtuous quality that wins needed approval from others and wards off anxiety and shame. Because this coping style has worked for clients in the past, it can be harder to let go of in treatment.



Similarly, moving-against clients do not see themselves as angry, competitive, self-centered, or demanding, although they frequently behave in these ways. Instead, they see themselves as heroes or heroines—strong leaders entitled to direct others, whom they may regard contemptuously as weak, dependent, or incompetent.

Finally, moving-away clients do not regard themselves as emotionally constricted, as avoidant and too easily hurt, or as being risk-averse and narrowly limiting their lives. Instead, they prize their aloofness, perhaps as evidence that they are more refined, that they do not need others, or hold the illusion that they feel or experience things more deeply than others. They may be cynical and at some level feel better than the ordinary masses, whose involvement in everyday happenings may be seen as mundane and small-minded.

To lessen their anxiety, many clients inflexibly employ these interpersonal coping styles that allow them to feel special, gain a sense of identity and self-esteem, and manage deprivation or minimize problems that originally led to these compensatory adaptations. Their self-esteem, however, is brittle and vulnerable. It is based on an unrealistic sense of being special rather than learning to value themselves and their own authentic feelings.

Further, clients have often developed significant abilities in the service of their coping strategies. For example, many moving-toward clients have not only been able to make others like them but have also become genuinely skilled at caring for others. Likewise, moving-against clients have often been able to achieve some success and power, and may develop significant technical or other abilities along the way. Similarly, although moving-away clients have often been able to remain safely aloof, they may also have developed a rich inner life or been able to cultivate creative or artistic abilities.

On the other hand, however, one of the most important interventions therapists can make is to *help clients recognize the significant emotional price they pay for their interpersonal coping style*. Therapists do this cost/benefit analysis by recognizing and affirming clients' strengths and abilities, while also highlighting how their interpersonal adaptation is creating stress and limiting their lives now. For example:

**THERAPIST:** You've just earned another award at work, where they have nicknamed you the "star," and you do so much for everybody at home. I'm genuinely impressed by how capable you are and how much you are always doing for everyone. But I'm also concerned about how tired you look and how often you are sick. You often use the expression that you're "running on empty." I'm hoping that we can begin to listen to that feeling together here in counseling—I think it's telling both of us something important.

In this way, therapists both affirm the genuine strengths that clients have developed through these coping strategies, but simultaneously help to articulate in an empathic way the significant toll they also take. Later in this chapter we will see how clients can retain the skills and strengths they have developed in these compensatory modes, but without utilizing them in the same inflexible or unidimensional ways as in the past. As before, therapists need to help clients honor their interpersonal coping styles and appreciate rather than pathologize them. They were absolutely necessary at one time and represented the best possible adaptation they could have made at that point in their development. Although they are still adaptive in some problematic relationships, they are overused now and are no longer necessary or helpful in many current situations. *A primary treatment goal for many clients is to learn that more flexible or varied interpersonal coping styles will serve them better.*

To sum up, by turning their defensive interpersonal adaptation into an identity or way of being in the world, clients often gain significant secondary benefits and genuine personality strengths. It's no wonder, then, that some clients initially may be reluctant to explore this adaptation with the therapist. It has become the basis of their identity, the primary source of their self-esteem, and the only way they have had of defending against

their seemingly unresolvable conflicts. Once therapists fully grasp all that this defense has provided for clients, and help clients to appreciate both what it has given and what it has cost, both therapists and clients will be less judgmental or impatient with the clients' symptoms. Both will have greater empathy for the clients' struggles to change and will be able to work collaboratively to develop more flexible coping strategies. Understandably, clients will not want to risk giving up what they have without having something better to replace it. As we will see, however, clients never really succeed in overcoming their core conflicts by using these compensatory adaptations. By acknowledging problems and addressing them directly, clients can develop a wider experiential/interpersonal range that will be an important part of resolving many problems.

### SHOULDs FOR THE SELF AND EXPECTATIONS OF OTHERS

Following Horney, these interpersonal coping strategies often hold unrealistic shoulds for clients themselves, and expectations of others as well. For example, self-effacing, moving-toward clients hold unspoken expectations of unwavering loyalty and approval from others, and they often demand constant reassurances. Expansive, moving-against clients claim leadership and control, and they expect deference from others. And, although it may look as if detached, moving-away clients place no demands on others, they silently hold expectations that others never criticize them or make demands upon them. Of course, the world doesn't respond very well to these covert expectations. They create ongoing conflict and leave clients exposed to continuing frustration, as others frequently do not accept such unrealistic demands and unspoken expectations.

Accompanying clients' unrealistic demands on others is a harsh and uncompromising set of "shoulds" that they place on themselves; these faulty beliefs reflect their impaired self-esteem and diminished sense of self. For example, moving-toward clients suffer under the self-imposed demands that they should be the perfect lover, teacher, spouse, or parent. They must *always* be caring and responsive to the needs of others and never feel angry, compete, or advocate on their own behalf. moving-against clients demand of themselves that they should be able to prevail—overcoming all difficulties and successfully dominate. They must win and be on top, control their feelings and be "strong" at all times, and overcome personal insecurities, fatigue and even physical illness simply by an act of will. moving-away clients pay for their coping strategy by believing that they should be able to work tirelessly and always be productive. They demand that they should be able to endure anything without becoming upset and that they should never need help from anyone. These individuals will not always be able to meet the rigid demands they place on themselves, of course, and inevitable failures will occur.

When such failures occur, intense anxiety, depression, and other symptoms often result, and some clients will enter treatment in crisis. Clients' pathogenic beliefs that if they are unable to successfully please, achieve or remain aloof then they are not love-worthy, cannot protect themselves, or are too needy or demanding for others. In particular, shame is often evoked when interpersonal coping styles fail—which many clients may experience as a "defeat" or present as a "failure experience,"—and contributes to many different symptoms and problems that clients bring to treatment (Tangney & Dearing, 2011; 2002).

As we will see, the "tyranny of the shoulds" is an exacting price to pay for a client's interpersonal coping strategy. One of the most helpful interventions therapists can make is to be able to articulate, empathically, how this coping strategy is central to the client's basic sense of self. At the same time, therapists need to help clients appreciate these interpersonal coping strategies. They do in fact contain real strengths, and therapists can help clients recognize how once they really were necessary and adaptive. However, overused and overgeneralized now, they are no longer necessary or adaptive

in many current situations. Therapists help by a) clarifying the unnecessary price in symptoms and stress that the client is paying for this unidimensional coping strategy and b) helping them discern when and with whom this coping style is still adaptive—and when it is not.

## RESOLVING THE CORE CONFLICT

Part 4 in Figure 7.1 on page 226 illustrates how the client's core conflict can be resolved. Ultimately, clients' coping strategies to work around their core conflicts will not succeed. These compensatory maneuvers will not resolve clients' problems—no matter how successful the client becomes through these interpersonal coping strategies. *The client is still defending against the original conflict rather than understanding and resolving it.* As they repeatedly employ these interpersonal styles—even in situations where they are not needed or are not effective—clients will engender further problems in current relationships, and create personal stress that often contributes to health crises. Let's examine why clients' interpersonal coping strategies and disruptive IWMs will lead to further problems rather than resolution, and then explore what therapists can do to help the client.

First, the “tyranny of the shoulds” places unrelenting demands on the client. These can be heard, for example, when clients enter treatment exclaiming, “I can't do all of this anymore! I can't be a perfect parent, a great spouse, and the most productive person in my office. I can't be everything to everybody!” Clients' quality of life is diminished by these unrealistic expectations. They are depleting and fatiguing, and diminish the potential for personal and professional growth, even if these unrelenting demands clients place on themselves (and on others) do not provoke a crisis that brings them to treatment. The struggle that comes from not having had a Secure Base to venture flexibly into new relationships, is palpable in clients severely bound by “shoulds,” while not having a Safe Haven results in clients' IWMs of Self as deficient, unworthy, and needy.

Clients' demanding expectations of themselves and others will usually fail as well. For example, pleasing clients will not be able to win the love or approval of everyone they need. Expansive clients will not always succeed or be able to make others defer to their demands, and detached clients will encounter criticism from others or situational pressures that force them to compete. Inevitably, clients' attempts to compensate for their lack of Security will lead to conflict at work and at home. Frequently, the precipitating crisis that leads clients to enter treatment is when situational life stressors, developmental transitions in adulthood, or aging/illness have caused their interpersonal coping strategies to fail.

Therapists help clients change when they grasp the discouragement and pain that occur when these coping strategies fail. Kohut (1977) writes eloquently of the narcissistic wound that is so painful to clients' core sense of Self. Even though the demands that clients place on themselves and others are unrealistic, it still can be excruciating when their interpersonal coping strategy fails. Tragically, a few individuals will even attempt suicide in response to such a failure, and others will contemplate it. Why are some clients driven to such an extreme response? In a word, shame. The interpersonal coping style that many clients have adopted is a brittle substitute for the genuine self-esteem or Security they lack. Although the severity of clients' interpersonal coping strategies will vary on a continuum, from many clients' point of view, *they are their ability to please, to achieve, or to remain superior and aloof.* Such clients over-react to seemingly insignificant events (e.g., gaining three pounds over the holidays or losing their patience with a trying two-year-old). This over-reaction occurs because their entire sense of Self is threatened when their interpersonal coping strategy fails and their shame-based sense of self is revealed by their mistakes—which are often framed more globally and self-damningly as “failures.”

To illustrate, recall the client who got an A– rather than the A he demanded of himself and felt like a complete failure. Too often, the therapist’s initial response is to challenge these clients’ overreaction to the grade as catastrophizing and provide a more realistic appraisal of the situation. This well-intended, reality-based response certainly has value, but on its own, it misses the deeper meaning for this particular client. A more productive response may be a bid for an empathic connection. That is, wondering aloud with skillful tentativeness, and collaboratively seeking client’s participation in capturing the real meaning or more life-defining issue at hand, the counselor might ask:

**THERAPIST:** I’m wondering if there is no room for error for you—ever? Does everything always have to be perfect or, seemingly, everything is lost and the consequences seem so dire?

In this way, the therapist is trying to discern collaboratively how all-important this “failure” or disappointment seems to be from the client’s point of view. With a compassionate and curious stance, the therapist continues to invite the client to join and explore how this grade seems to indicate something very important about the client and who he really is, and who has made him feel that way. In this way, the counselor and client often find that this failure experience initially is viewed as “proof” or confirming evidence that something bad or unwanted is true, which commonly reveals an attachment figure’s expectations and not the client’s own choices. Taking other examples:

**CLIENT:** When I got passed over for the promotion, it proved to everyone that I couldn’t make it.

**THERAPIST:** Whose voice do you hear when you say to yourself, “It’s proof that I will never make it”?

**CLIENT:** My father always said *I must be the best or I will just end up a failure.*

**THERAPIST:** I’m sorry that you had that experience. I’m wondering if it’s possible for you to feel differently about yourself than your father did?

OR

**CLIENT:** When I got frustrated and upset like that, I knew none of them would ever like or respect me again.

**THERAPIST:** With whom did you learn that getting upset could never be repaired or recovered from—and damned you to being alone, bad and on the outside forever?

**CLIENT:** Well, if I got upset or mad, my Mother would turn away in disgust and walk out of the room.

**THERAPIST:** It sounds like that might leave you feeling painfully alone, and also deserving of that unwanted response. Is it possible for you to feel differently about yourself than your mother did—to feel some compassion for yourself rather than joining her in disgust for yourself?

In these examples, the therapist and client are trying to hone in and clarify, as precisely as they can, what the disappointing grade, missed promotion, or getting “upset”, seems to say about these clients *and how it defines who they really are*. For example, together they may find that it proves to the client that he is just an “imposter” who really is inadequate and can never do it right, or is letting someone down whose approval he or she “must” earn. This pathogenic belief is true, in the client’s mind, despite the client’s Herculean efforts to overcome or disprove it by always earning As or always being “nice.”

By exploring and clarifying together how completely clients’ sense of Self depends on their ability to achieve or please (which is a different intervention and interpersonal process than interpreting, challenging, or reframing), the therapist reveals the anxious

insecurity and emotional deprivation that originally led to this defensive coping strategy. Therapists use this empathic understanding to provide a Safe Haven and share their compassion both for the pain of the original problem and the burden of trying to overcome or disprove it that clients have suffered with for so long. By being able to help clients articulate or clarify this dilemma, therapists behaviorally demonstrate that *they grasp the real meaning of this prototypic experience that occurs over and over again for clients*. This validation or experience of being seen and understood helps to disprove the pathogenic belief, and therapists will often observe that it diminishes presenting symptoms of anxiety and depression as well. Such empathic understanding also provides a Secure Base for clients from which they can further explore and question these views of themselves and their coping strategy; appreciate how and why they learned it; and start to observe when they see themselves employing it in their daily lives. This, in turn, helps clients begin to change this pattern of relating—often with the therapist and then with others in their lives.

By appreciating the *broader life context* that gives meaning to a “failure experience” when clients’ interpersonal defense fails—an impact that has always seemed so irrational to others as well as to the clients themselves—therapists understand clients in a way that most clients have not experienced before. This empathic understanding is a CEE that helps clients free themselves from the tyranny of the shoulds and the unrealistic demands they place on themselves, and helps them come to terms more realistically with who they are and how they choose to live.

Therapists can help clients *recognize* or start to become aware of when and how their IWMs and interpersonal coping strategies are in play. The therapist does this by *highlighting the themes or patterns* in the interactions that are recurring with others in clients’ lives. The therapist also uses process comments that bring into consideration how these could be occurring and how they could be changed, right now in the way the therapist and client are interacting together. For example:

**THERAPIST:** I’d like to take a risk and share something’s that’s going on for me right now—but I wouldn’t want you to hear it in the wrong way. Maybe it’s completely out to lunch, or maybe it’s telling both of us something important. Can I have your permission to just speak up directly?

**CLIENT:** Yeah, that would be all right.

**THERAPIST:** I’m seeing this issue differently than you are, but as I start to share my perspective, I feel anxious. Like I shouldn’t disagree with you, or you will somehow withdraw from me—or maybe even go away and not come back to our next session. Can we sort this out together? Is this just my mistaken notion, or have others said something like this to you before?

**CLIENT:** Well, yeah, I have heard before that I “go away” pretty easily.

**THERAPIST:** I don’t want you to “go away” from me. Let’s talk about this, and find a better way in here...

Thus, instead of avoiding their real problems in the two defensive ways we have been discussing, the therapists’ goal is to help clients approach them, recognize and begin to understand them, and find that in some relationships the unwanted but familiar response they received in the past does not occur with all people. The interpersonal solution provided by the therapist’s new and different response—the CEE—allows clients to integrate other reactive feelings, helps clients disconfirm pathogenic beliefs about themselves, and alters clients’ faulty expectations about what may occur in relationships with others. Having the experience of change with the therapist also facilitates clients’ ability to begin changing their own rigid and ineffective ways of relating with others in their lives. Now, in parallel, clients often are able to understand and resolve the original



developmental conflict by re-evaluating with the therapist what was once so hurtful or problematic. For example:

**CLIENT:** I'm really not being selfish or doing something wrong if I become more independent and successful, even though this makes my mom feel sad and withdraw sometimes.

In this way, maladaptive relational patterns are broken and new, more flexible interpersonal styles and IWMs become possible. Therapists' compassion and validation also comprise a new and reparative response that disconfirms these faulty beliefs and, in turn, allows clients to be more accepting and forgiving of themselves. Clients can now integrate the previously unacceptable parts of themselves and can better accept both the good qualities and the limitations in other relationships that have been important but conflicted. As this occurs with the therapist, clients are able to begin generalizing this experiential relearning with the therapist to others in their lives. With the therapist's coaching, clients can begin to respond in more adaptive ways that improve their relationships with others and expand the narrow coping style they originally adopted. In the chapters ahead, we will examine more specifically: (1) how the therapeutic relationship can be utilized to help clients resolve their core conflict; and especially, (2) how this emotional relearning with the therapist can be generalized to other relationships in clients' lives. For now, an extended case study of a moving-toward client, and two case summaries of the other coping styles, will help therapists use these concepts to guide their interventions.

## CASE STUDY OF PETER: MOVING-TOWARD OTHERS

### DEVELOPMENTAL HISTORY AND PRECIPITATING CRISIS

Peter was an insecurely attached child whose emotional needs went unmet in his authoritarian home. Further stressors accumulated as he anxiously observed his parents' ongoing wrangling, and contentious divorce when he was 7 years-old. His parents could not work out a cooperative parenting relationship following the breakup and Peter's father did not fulfill an active parenting role. Two years after the divorce, Peter saw his father infrequently and when they were together their interaction was superficial.

Peter's mother was overburdened by the demands of raising three children on her own, working full-time, and trying to make some kind of personal life for herself. Frustrated by the many demands and few pleasures in her life, Peter's mother often made him the target of her resentment. Impatient and irritable toward him, she criticized Peter when things went wrong in her life. She often felt resentful of his needs and, although she tried her best to be fair to the children and give them a good home, she was not very responsive to Peter or affirming of him.

After his father left, Peter quickly learned that taking care of his mother was the best way to ward off her disapproval and to win whatever affection he could. Following the anxious/ambivalent attachment style, he was "preoccupied" with monitoring his mother's moods and concerns. By the age of 10, Peter had already adopted a pervasive interpersonal style of moving-toward people. His teachers described him as an especially responsible and well-behaved boy who was "a pleasure to have in class—always so helpful." Let's turn the clock forward 15 years and see how these developmental challenges are being expressed in Peter's early adulthood.

At the age of 25, Peter is a graduate student in counseling. Becoming a therapist felt like a perfect career choice to Peter. He prided himself on his sensitivity and concern for others and took pleasure in being able to help those in need. He enjoyed being a therapist and felt that he was often able to help his new clients. At least, all of his clients seemed

to like him and they kept returning to their counseling sessions each week. As his second-semester practicum got under way, Peter felt that he was on his way.

Later that semester, though, Peter had a setback. After presenting a videotaped recording of one of his therapy sessions in group supervision, Peter received some unexpected feedback. The practicum instructor told Peter that he was being “too nice” to his clients and that he seemed to need their approval too much. The instructor went on to say that Peter seemed to be afraid to challenge his clients, reassured them too often, and tended to avoid potential misunderstandings or conflicts in the therapist-client relationship that needed to be addressed.

Peter was stunned. Although he had some awareness of his aversion to conflict, he felt hurt and confused by the criticism. It was important to Peter that his instructor like him and approve of his clinical work. Peter tried carefully to explain that the instructor did not understand the close relationship that Peter was developing with his clients or recognize all of the important issues that his clients had been revealing. The instructor responded that this was probably true, but Peter was missing the point. He repeated that Peter needed to think about his reluctance to address interpersonal conflict or say things that his clients might not want to hear. To make matters worse, two students in the practicum group chimed in and agreed with the instructor’s comments. With that, Peter’s anxiety became so high that he could no longer explain himself or try to learn from their comments. He stopped arguing, looked down, and was quiet throughout the rest of the supervision session.

Throughout the next few days, Peter was so dismayed by the criticism that he couldn’t think about anything else. He considered dropping out of the practicum group, but then he decided that if he tried hard enough, he could make the instructor see that his criticism was unfounded. Peter kept searching for a way to discount the feedback and stop the anxiety that was churning inside.

One week later, Peter found out that his girlfriend was having an affair with another student in the program. Although he tried to be understanding at first, he felt shocked and betrayed. He alternated between withdrawing and announcing that their relationship was over and desperately trying to win her back. He became so anxious that he was unable to eat or sleep, let alone study. Peter began hyperventilating, experiencing heart palpitations, and having anxiety attacks.

To make matters worse, Peter tried to keep all of this turmoil to himself. He thought he should remain “calm and together.” He was afraid that his supervisors would not want him to see clients if he was “so messed up” that he was having anxiety attacks himself. But despite his attempts to cover up his distress, his individual supervisor soon asked him what was wrong.

Although he never could have allowed himself to ask for it, Peter desperately wanted his individual supervisor’s support, and he was greatly relieved to receive it. Peter explained how his practicum group was becoming one of the worst failure experiences of his life and how hard it was to accept what his girlfriend had done. The supervisor was supportive but also found a tactful way to say that the practicum instructor’s comments fit with some of his own observations. Because he knew that his individual supervisor liked him, Peter was able to consider the feedback this time. The supervisor suggested that these were important issues for Peter to work with but that they could be dealt with better in his own therapy rather than in supervision. Peter agreed and began seeing his own therapist.

It helped that Peter’s supervisor thought he could be a fine therapist, in spite of his anxiety attacks. However, Peter’s anxiety remained paralyzing as he began his own treatment. Fortunately, Peter’s therapist was skilled and experienced and soon conceptualized the predisposing vulnerability, interpersonal coping strategy, and

situational stressors that precipitated his crisis. Peter's coping strategy of moving-toward people had generally worked well enough for him up to this point. However, both of the crises that Peter had just experienced ran headlong into the heart of his core conflict and his interpersonal style for coping with it. As his ability to manage his core conflict by pleasing others failed, the anxiety associated with his profoundly insecure childhood broke through, and anxiety attacks resulted. Let's try to understand more fully why Peter developed these symptoms.

### **PRECIPITATING CRISIS, MALADAPTIVE RELATIONAL PATTERNS, AND SYMPTOM DEVELOPMENT**

Many people would have coped with the two stressful events that Peter experienced without developing such significant symptoms. As we have seen, however, we understand more when we apply the concept of client response specificity. We will use Peter to illustrate how symptoms develop when situational stressors evoke painful developmental experiences, confirm pathogenic beliefs, or cause interpersonal coping strategies to fail.

The first stressor for Peter was his practicum instructor's critical feedback. Criticism from a respected authority figure would be unsettling for most people, but generally they can cope with it. For someone like Peter, however, such disapproval carried far more weight. Peter's childhood deprivation and strong need for approval, his history of receiving excessive criticism, and his lifestyle adaptation of trying to manage these developmental conflicts by being helpful and nice was not working.

Second, the instructor's feedback triggered Peter's core conflict—anxiety about receiving criticism and feeling rejected—and, simultaneously, weakened his interpersonal defense of pleasing others and accommodating their needs. Seeing how IWMs develop, Peter adopted the attitude toward himself communicated by his early caretakers—that he did not matter enough to be committed to (from his father and his girlfriend) and was not good enough to be approved of and responded to (from his mother), and to expect that others often would be critical of him (like his practicum instructor). His attempt to defend against the anxiety evoked by his father's abandonment and mother's criticism was by trying to be perfect—to please her and others and submerge himself to their needs, so that this self-critical schema could be refuted by their appreciation (Bernstein & Benfield, 2013). This coping style was, of course, a formula for failure: It was impossible for Peter to be liked and appreciated at all times by everyone he met. There was no such thing as constructive criticism for Peter—he construed all disapproval as rejection, and he felt very uncomfortable whenever he could not win everyone's approval. When teaching his first undergraduate course, for example, Peter seemed only to notice the two or three students in the class who seemed disgruntled. He did not meaningfully register that the large majority of students enjoyed the class and thought he was doing a good job.

If not for the subsequent crisis with his girlfriend, Peter probably could have recovered from the first setback without developing symptoms. Most likely, he would have reconstituted his interpersonal coping style and been somewhat successful in winning the approval he needed from others in his life—different instructors, friends, and as it too often goes, perhaps even clients. As we will see, however, the subsequent stressor with his girlfriend also struck at the same core conflict. At that point, his moving-toward coping style toppled, and the intense anxiety associated with his insecure attachment history was evoked. More specifically, this was “shame-anxiety”—anxiety over the threat of having his unlovable and unworthy self exposed. This anxiety over having his shame-worthy self revealed, both to himself and to others, became too intense and broke through in anxiety attacks and symptom formation.

A partner's infidelity will be highly stressful for almost everyone. Here again, though, this particular stressor held far greater significance for Peter when viewed within the

personal context of his developmental history and subjective worldview. Peter had to cope with far more than just the loss of trust with his girlfriend; he also suffered a blow to his identity, his basic sense of self-worth, and his interpersonal coping strategy of being able to please others and win their approval.

In and of themselves, these multiple stressors would precipitate a crisis for many people and lead them to therapy. However, if these situational stressors do not tap into preexisting schemas, repeat familiar but unwanted relational scenarios, and confirm pathogenic beliefs, they will not usually provoke such strong symptoms. The client will often be able to recover in a relatively short time with crisis intervention or short-term supportive therapy. In contrast, *when stressful life events tap squarely into a client's core conflict, a client such as Peter has to cope with far more than just the demands of the current situational stressors.*

Thus, when Peter's girlfriend became sexually involved with another man, he had to cope with much more than just the pain of this betrayal. He had to cope with the even bigger developmental problems that were triggered: his strained attachment history was evoked; his interpersonal coping strategy of pleasing to ward off the criticism and rejection that he expected was failing; and because he received too little empathy as a child, he had virtually no capacity for self-compassion or self-empathy to help see him through difficult times (Neff, 2014).

In this way, clients such as Peter have little to fall back on in times of crisis. As unrealistic as it is, the secondary gains that clients can earn with their interpersonal coping style of moving-toward others and being "giving" is the only identity or sense of self-worth that some clients have been able to garner. It is understandable, then, that Peter was overwhelmed by anxiety attacks. Both of the situational stressors intensified Peter's original conflicts, and they both took away his coping strategy for defending against them. In this light, we see that clients' presenting symptoms are not irrational. Per client response specificity, they make sense once they are understood in the broader context of the client's developmental history and the cognitive schemas, pathogenic beliefs, and inflexible coping styles that result from this life context.

## COURSE OF TREATMENT

Fortunately, Peter's therapist was both knowledgeable and kind. He was genuinely empathic to the pain that these situational crises brought on for Peter but also grasped what made Peter so vulnerable to them. The therapist recognized that the current crisis provided an opportunity to resolve the more important developmental problems that left Peter prone to re-experience crises such as these whenever others left or disapproved of him. In the months that followed, Peter was able to resolve the precipitating crisis. More significantly, he made progress in coming to terms with the impact of his developmental history and the problems that followed from it. As a result, Peter was able to expand his narrow moving-toward coping style and adopt a more flexible interpersonal repertoire. Here and in the next chapter, we will review the course of therapeutic events that allowed Peter to make these far-reaching changes.

From the first session, Peter felt understood and cared about by the therapist. As the therapist's accurate empathy led to a strong working alliance, Peter's anxiety attacks stopped. The distress that had been so disruptive for him steadily subsided as the therapist continued to provide an effective holding environment.

The primary way that the therapist worked in the beginning was to help Peter identify his habitual coping style of pleasing and recognize how automatically and frequently it came into play. Thus, the therapist *named it*, so they could talk about it and begin to explore it together—compassionately and without judgment. This opened the door that changed everything. From these conversations, for example, Peter learned to focus inward

to identify what he was feeling, and what was occurring with others, at the moment just before he felt compelled to employ this coping pattern and please. Usually, at such moments, Peter found that he was expecting others to be disapproving of him or to go away and cut off from him in some way. With help from his therapist, *Peter became increasingly adept at recognizing this sequence* and his propensity to manage his anxiety by engaging in pleasing behavior (Levensen, 2010).

Specifically, as he listened to Peter's narratives, the therapist would highlight when instances of Peter's key relational themes were occurring (Barber et al., 2002). The therapist would point out when Peter was (1) pleasing others, (2) avoiding anger and other interpersonal conflicts, and (3) expecting others to be critical or rejecting. In particular, the therapist used process comments to approach or explore these three repetitive relational themes whenever he thought they might be occurring in their interaction together, or with others (DeFife, Hilsenroth, & Kuutmann, 2014). For example, when the therapist thought that Peter was trying to please him, too, he made this overt in a direct but collaborative manner and invited dialogue about this possibility by saying:

**THERAPIST:** When we're talking, Peter, sometimes I find myself wondering if you might be thinking too much about what I or others might want to hear. What do you think—is there anything to that?

The therapist also highlighted whenever he thought he heard one of these three themes in Peter's relationships with others. For example:

**THERAPIST:** As I listen to you describe this interaction with them, it sounds like you are trying very hard to keep them from being angry at you. I feel like I've heard this before. Does it seem to you like this comes up a lot?

This feedback provided direction for their work together—a treatment focus—and served as the basis for developing mutually held treatment goals. For example, *the therapist encouraged Peter to risk not pleasing him and focus instead on just saying and doing what he wanted in their relationship*. Not having to be vigilant about what the therapist might like or want gave Peter the opportunity to live inside his own skin and have his own mind rather than be so preoccupied about what the other person wanted to hear. This was a CEE for Peter. He could enjoy this relationship where he could be cared about even when he was not trying to please the therapist or act in ways that, at some level, were designed to keep the therapist engaged with him and ensure that he would not leave him or be critical (such as frequently complimenting the therapist or telling him how helpful he is being).

The therapist continued to focus Peter inward. In particular, they explored the feelings that were evoked for Peter when, instead of employing his usual coping style of pleasing others, he risked new, more assertive responses that he had rehearsed with the therapist. Gradually, by focusing on what he was experiencing just before he employed his usual coping patterns, Peter began to clarify the emotional deprivation he had suffered as a child but had always been too ashamed to reveal to others or even acknowledge fully to himself. Relatively quickly, the therapist's accepting presence allowed Peter to overcome the shame of having his pain revealed. Now, for the first time in his life, Peter could be fully seen by someone with all his strengths and problems and continue to be respected. Peter began to understand how he had coped with his childhood predicament by taking care of and pleasing his mother, and then others as well. As this significant progress occurred, however, Peter still continued to struggle with strong feelings of anxiety, shame, and sadness.

These contradictory feelings continued to wax and wane for Peter over the next few months. He was increasingly recognizing how unwanted and alone he had felt as a child



and how ashamed of himself he had always been. Peter also recognized and began to challenge the false belief that he was not loved because he was somehow unworthy—that he did not matter enough to be important to others. As Peter continued to feel “held” by the therapist’s understanding, his long-withheld feelings of sadness over this deprivation and invalidation could be expressed for the first time. Peter successfully moved through his original expectations and coping style as he received a far-reaching CEE. Repeatedly, the therapist’s validating responses disconfirmed Peter’s old relational expectations and were deeply comforting, offering him the affirmation for which he had always longed. This validation also provided him with a sense of being seen or known, and accepted for who he really was for the first time.

Peter began to feel sadness and compassion for himself and mourn his childhood losses—rather than continue to blame himself for what occurred. In response to this primary affect of sadness, two reactive feelings followed closely behind. Peter found himself feeling angry: at his father for walking away from him; and at his mother for making him feel responsible for trying to earn her love, and for having to work so hard to be “good” to ward off her disdain. As soon as this anger emerged, however, Peter became exceedingly anxious—afraid of being left on his own by the therapist, his girlfriend, his parents, and anyone else for being angry with them and “protesting” how they treated him. Each of these three feelings in Peter’s affective constellation—the shame surrounding his unmet emotional needs, his anger over being rejected and dismissed so readily, and the anxiety of being left or emotionally cut off if he protested—was repeatedly aroused in treatment. With the therapist’s help, Peter was gradually able to integrate and resolve these feelings. He was able to experience or know them himself, to risk sharing or revealing them to the therapist, and to discover that he could live through or contain them with the therapist’s support.

The therapist provided a skillful balancing of the good news and the bad news in Peter’s development. He was able to acknowledge the reality that Peter loved his mother and still felt loyal to her despite all that occurred, and that some of the qualities he valued most about himself were qualities derived from her. However, these reality-based strengths were accompanied by the painful realities of Peter’s emotional deprivation and invalidation. This process of acknowledging both aspects helped Peter preserve important aspects of his tie to his mother. Being able to protest with the therapist what had been so unfair and hurtful was not to reject his parents, hold a grudge, feel sorry for himself, or make his parents bad people. Instead, it was to change the narrative of his life and know that these things actually did happen, really were painful, and that he didn’t cause or deserve them. Rather than denying the reality of his circumstances any longer, being able to have the validity of his own feelings and perceptions for the first time led Peter to a stronger stance that he was able to generalize to other relationships. He became more confident and assertive, developed firmer boundaries with others and gained more clarity about who he was, and began to disconfirm the pathogenic belief that he would be criticized or rejected if he wasn’t taking care of others. Peter came to understand that his mother’s on-going criticism and exasperation with him was unwarranted - just the expression of her own hopelessness about ever being loved. Significantly, Peter was able to feel compassion toward her and appreciate the predicament she was in as she was coping with her own long-term depression, yet without denying the reality of the painful consequences this engendered for him. Peter also grasped experientially, for the first time, that his father’s abandonment was his inability to be a parent and had nothing to do with him or his basic worth and lovability.

As a result of these changes, Peter felt less compelled to make others like him, less threatened by interpersonal conflict, and more self-contained. He was also able to keep the best parts of his moving-toward style: He remained a genuinely caring and responsive person. However, this was no longer the unidimensional response pattern that it had been

in the past. Peter no longer compulsively tried to get everyone to like him. He became increasingly more able to address interpersonal conflicts, although this would continue to be challenging for him. Nevertheless, this growing ability was helpful in establishing more mutually rewarding relationships.

Peter will certainly have further crises in his life, especially when circumstances again tap into his old wound, but he is more resilient now and the changes he has made will help him respond to future problems in a more empowered, present-centered manner.

## TWO CASE SUMMARIES

In the case study of Peter, we have seen how the interpersonal model for conceptualizing clients is used with a moving-toward client. Next are two case summaries that highlight salient features in the treatment of a moving-against and a moving-away client.

### JUAN: MOVING-AGAINST OTHERS

*Juan came to therapy because his wife, Maria, had threatened to leave him unless he went to therapy to deal with his jealous, intrusive, and controlling behavior. Upon again finding him “checking up” on her by secretly going through her cell phone and emails, and then erupting in threatening accusations that she was having affairs when she told him to stop, she had, in fact, moved in with her parents for a week until he promised to give therapy a try.*

**CLIENT:** I’m here because my wife is threatening to leave me. She says I’m too possessive and controlling. I’m not sure how therapy will help but when she went back to her parents’ house I agreed to try.

**THERAPIST:** I hear you—you don’t want to be here talking to me, but you are doing this to try and save your marriage. I hope you find it helpful to talk to me, and eventually decide to come for yourself. Your wife says you are too possessive and controlling. Can you tell me more about that?

**CLIENT:** Well, I just got back from Afghanistan and discovered—even though she knows I disapprove—that she started going to school. She says she needs a career. Why does she need a career when I’m here to take care of her and the family? I told her I don’t like it and that she should leave school—and I don’t like or trust the friends she’s making. She says she had to make all the decisions when I was in the military and now she thinks she should be able to make some of the decisions about her life. I have always made the decisions in my home and at my work—except when I was in the service.

**THERAPIST:** You disapprove of her going to school and feel now that you are back, you should make all the family decisions? Did I get that right?

**CLIENT:** Yes, I’m the man of the house. She shouldn’t have started school and been spending time with all of these new people she calls “friends” without my permission. And now that I’m back, she should leave the family decisions about our family life to me and respect that I don’t want her in school. She’s even taking night classes!

**THERAPIST:** How have you responded to her making decisions without checking with you?

**CLIENT:** I told her I don’t like it—things will get out of control. She used to go along with what I decided before I left for Afghanistan, but she has always complained about my needing to be in charge, in control. She used to call me “Mr. Control.” But maybe it’s fine with you for a wife to be disrespectful to her husband—you’re probably just a feminist. But I feel that if I don’t have control in my own house, then what kind of man am I, what kind of life are we going to have?

**THERAPIST:** No, I'm not a feminist who thinks your wife should be disrespectful to you, but I do value reciprocity in relationships more than you do—shared control and mutual respect. So there is a value difference between us that we can acknowledge and work with. But being in control is very important to you. Help me understand that better—have you always been this way?

**CLIENT:** Oh no, I didn't have any control when I was a kid.

**THERAPIST:** No control at all...that doesn't sound good—tell me how that went.

**CLIENT:** Yeah, no control at all, like where I would sleep at night.

**THERAPIST:** You didn't know what bed you were going to sleep in at night?

**CLIENT:** I wish it was just that—not knowing what bed I was going to sleep in, but which house.

**THERAPIST:** “Which house” —that really is “no control.” I'm so sorry—that sounds so very difficult for a child. Tell me more about not having control as a child—not knowing where you were going to sleep—and how it's affecting you now?

**CLIENT:** Well, my mom was unreliable—a flake. Sometimes she was there and helpful and nice—until she found a new boyfriend and would disappear for weeks at a time. I lived mostly with my grandma, but she died when I was five. After that I lived with different family members—whichever would take me in for a while. I didn't have a real home staying with my aunts or uncle, I always felt like the outsider. I had no control, no bed of my own or my own clothes—just “hand-me-downs.” I was alone a lot and people said I was too angry—I got in lots of fights. But I know I was angry about not having a home like other kids—I don't need a shrink to tell me that—and promised myself that it would be different when I have my own family and would have control over things and how we lived.

**THERAPIST:** You've just said so much, Juan, and it's heartbreaking. I'm so sorry you were so alone and painfully on the outside as a child—and things really were out of control, not knowing when your mother would be with you or gone, and who would be taking you in. It makes sense that you want a stable life with Maria now—where things are dependable and in control. But maybe we could explore some better ways to give that to you, and to Maria as well?

**CLIENT:** Well I can see you girls like to stick together, and I don't need you to feel sorry for me, either. But shouldn't a woman, who is being taken care of, want what her husband wants? I'm the man of the house and ought to have the last word. I've always taken charge—since I got emancipated at age 16. I seriously wanted to get out of the back and forth, and stop having people telling me what to do. I started working—at a warehouse, and took charge there too—and they liked it and promoted me—I became the youngest loading supervisor. I should have stayed but they kept telling me to do more, so I got pissed and quit to join the military. I thought it would provide more benefits and pay for college—but they REALLY control you. I hated it, but now that I'm back, I'm not going to have my wife going places and being with people and doing things that I don't know about. She's my wife and she ought to respect what I want.

**THERAPIST:** Sounds like you have strong beliefs about a woman's role. Tell me more about that.

**CLIENT:** Maria says she knew that I was possessive and needed to be in charge but thought I would change once we got married. But now she thinks I've gotten worse since I've been in the military. And she says she liked living more independently—going to school—when I was away and doesn't want to just be home like she should be. Who and what else does she need if she has me?

**THERAPIST:** I understand that your childhood experiences have made you want to make a different and better life for yourself—and certainly respect that. And I can see how having control over things—over Maria—is a way you are trying to bring a feeling of safety and predictability, but it seems to be costing you a lot at the same time.

**CLIENT:** “Costing me,” what do you mean, it doesn't cost me anything. It's the right way.

**THERAPIST:** Well, I'm concerned it may cost you Maria...she is saying she doesn't want this control anymore, and has moved to her parents' home.

**CLIENT:** Well, she should be a good wife. But you're probably on her side. You're a working woman—like Maria wants to be. You'd probably like it if she left me.

**THERAPIST:** Thank you for expressing your concern with me so directly. No, I'm not wishing she would leave you, but that you two could work out some new ways of being together that worked for both of you.

**CLIENT:** So how could that happen—how does this stupid therapy thing work anyway? Obviously you think I'm just a control freak, like her parents do, but you're just trickier about how you say it.

**THERAPIST:** No part of me wants to trick you, Juan. The important relationships in your life have not been trustworthy—so painfully undependable, and I certainly don't want to add to that. I think therapy might help if we could work together to sort out a middle ground where you finally have the stable life you want and deserve, but don't have to always be in control of Maria. How does that sound to you?

**CLIENT:** How could my life possibly be stable if I'm not in control? That's happy-talk, Disneyland, that just doesn't happen.

**THERAPIST:** Yes, I think it would be very hard for you to imagine that possibility, because you have never experienced anything like that. But I think it's certainly possible that you and Maria can share control and have a stable family life. You could know that she is choosing you, and not staying with you out of fear that you'll lose your temper if she doesn't do everything your way. I don't have any guarantees, of course, but I think it's possible your relationship could go better for both of you.

**CLIENT** (*less combative, more thoughtful tone of voice*): Maria has said almost those same words to me (pauses). But when we start to talk, I do get upset—angry I guess, and everything just goes to shit. I don't like this “independence” thing she's on... but ya, I don't want to lose her either. Could the three of us meet together sometimes—it might make it easier for us to talk?

**THERAPIST:** Yes, we're on the same page—I was just going to suggest the same thing. And I'd like to say something else, Juan—about us—that I think is important. Right now, I feel like our relationship, the way you and I are talking or being together, just changed. I feel like you just took a risk with me, let go of some control here in our relationship, and we started working together rather than jockeying or vying back and forth. I liked that change a lot, and I'm wondering if you felt something different happened as well?

**CLIENT:** Yeah, I did, and I liked it too.

Previously, with Peter, we discussed many different facets of his moving-toward interpersonal coping strategy and how it played out in his life and in treatment. In contrast, here with Juan, we are going to use this extended dialogue to try and highlight just one cardinal issue: the process dimension.

Like other moving-against clients, Juan repeatedly responds to the therapist in ways that are challenging (and especially unwanted for most new therapists), just as he does with others in his everyday life. Juan is provocative with the therapist and, at times, overtly disrespectful and insulting—perhaps even contemptuous. Even though he is repeatedly off-putting in this moving-against way, his therapist is non-defensive and doesn't *personalize* his disdain for her and therapy. Instead, she is able to *sustain* an empathic stance toward Juan in the face of this—which is not so easy for most to do. She readily grasps with compassion the meaning of his profound childhood losses (grandma), abandonment (mother), and aloneness (an outsider with the relatives he is shuffled around to). And,

despite her repeated bids to see things from Juan's point of view, he repeatedly brushes off the therapist's empathy and counters by framing the therapist as being against him and on his wife's side.

*As a thought exercise, pause now for a moment and consider how Juan would have made you feel, and how you would have tended to respond to him, if he were your client?*

Moving-against clients like Juan often elicit a fight-or-flight response from others. That is, the initial or automatic response tendency for some is to respond with counter-hostility and spit back, whereas many others will simply withdraw and go away from this difficult individual. Whatever the theoretical orientation of the therapist, *treatment is likely to fail if the therapist responds in either of these ineffective modes and reenacts this familiar but faulty interpersonal process that typically ensues with others in his everyday life.* That is, Juan's therapist did not respond defensively with her own counter-hostility or become embroiled in a control battle with him—as Juan elicits from many. And without getting into a power struggle with him, she did not give in to Juan, either. He did not control or dominate her, and she did not withdraw physically or emotionally give up on him, as Juan often elicits from others. Instead, this therapist was effective in establishing a new and different interpersonal process with Juan that did not repeat either of these two maladaptive scenarios that pervade his life. As the therapist patiently tried to create a different and reparative interpersonal process in their interaction together, Juan made a highly significant shift at the end of the dialogue. He stopped (temporarily) his me-against-you way of interacting with the therapist, relinquished his controlling stance temporarily, and accepted the therapist's repeated bid to work more collaboratively. At this closing moment, their interpersonal process—the way they were interacting together—was behaviorally enacting a resolution of his presenting problem with his wife.

With such significant attachment trauma in his history, Juan will not be able to relinquish his defensive interpersonal coping style for long. However, this success experience, which the therapist “names” and makes overt to help Juan recognize the different way of relating in their relationship, is significant. And, as he and the therapist can continue to find ways to interact in this new and different way, the potential for change with his wife is greatly enhanced.

### MAGGIE: MOVING AWAY FROM OTHERS

*Six months after her adolescent daughter had been date-raped, Maggie, a 38-year-old biracial woman, was still in crisis—as if it all happened yesterday. Nightmares stole her sleep; migraine headaches tormented her days. Feeling so helpless and inadequate to comfort her daughter, and furious at the casual indifference of the lawyer and police, Maggie was afraid her life was spinning out of control. At work, her supervisor's evaluation said she was “irritable, sullen, and difficult for coworkers to interact with” and suggested that she seek counseling. Although she had always “hated” to ask for help with anything, Maggie contacted a therapist when she realized her job was in jeopardy. The therapist was responsive to Maggie's feelings of outrage, guilt, and helplessness evoked by her daughter's tragedy. To her surprise, Maggie felt understood by the therapist. Time and again, the therapist “got it”—understood what something really meant to her—and Maggie actually began looking forward to their meetings. She gradually came to trust the therapist a little more and slowly began to risk investing herself a bit in their relationship.*

During one session, Maggie reported a dream from the previous night. In the dream, Maggie was alone in a vast desert night. No other people existed in this great, silent space. The desert night was black; no light shone from stars or moon. As she walked across the endless sand, a cool, dry wind began to move lightly across her face. Maggie lay down on the sand, closed her eyes, and silently slipped away into the darkness.



Maggie relayed that she had dreamt variations of this dream many times throughout her life. Because this was a recurrent dream, the therapist knew that it held much meaning and, in ways she couldn't fully understand yet, probably encapsulated her core conflict. Hoping that the dream could also provide an avenue for joining her in her central feeling of aloneness, the therapist tried to bridge Maggie's moving-away orientation:

**THERAPIST:** Can you close your eyes and find the dream again?

**MAGGIE:** Uh-huh. (*settles back and closes her eyes*)

**THERAPIST:** Describe what you see to me.

**MAGGIE:** I'm walking, but I'm tired of it; there isn't any place to go anyway. It's quiet and dark, empty. I'm alone. I can feel the breeze. It's sandy and the horizon is a long way off. Now I'm lying down on the sand. I close my eyes, and just slip away somehow, like going to sleep forever.

**THERAPIST:** I don't want you to be there alone. Will you let me join you?

**MAGGIE:** (pauses, then speaks cautiously) Well, thanks, but it feels sort of familiar, sort of safe, I guess, to be just by myself.

**THERAPIST:** You are familiar with being alone, that's the safe zone for you.

**MAGGIE:** Yeah, I guess it is...

**THERAPIST:** ...is that what you want?

**MAGGIE:** (*very long pause, tears in her eyes*) No...actually, no...I really don't want to be alone (*looks at therapist*). I do want to be with you, it's kind of comforting to talk with you, but it's kind of scary, too, sort of vulnerable.

**THERAPIST:** Uh-huh. It's kind of comforting to have me there and not be alone, but that vulnerable feeling is kind of scary too.

**MAGGIE:** Yeah, I guess I've just learned to do it all myself. Being alone, doing it on my own has just become the safe way. But no, I don't really want to be alone anymore.

**THERAPIST:** Good. Will you let me join you and maybe talk about the scary/vulnerable feelings if you want?

**MAGGIE:** Yes...but do you really want to join me?

**THERAPIST:** Yes, Maggie, I really do want to join you, and I don't want to do anything that would hurt or scare you if you invited me in.

**MAGGIE:** Well, maybe that would be nice, to not be alone anymore.

**THERAPIST:** Thank you for taking the risk to let me join you. Can we go back to the dream, but be together in it this time?

**MAGGIE:** All right.

**THERAPIST:** Good. Close your eyes and hold that same, familiar image. But you're not alone this time. I am walking toward you, and I reach my hand out toward yours. Will you take it?

**MAGGIE:** Yeah. It's nice...But I'm kind of scared, too...

**THERAPIST:** Mmm hmm. Now I'm in the dream with you—you're not alone there anymore. We're holding hands and walking together through the desert night. And it feels two ways—kind of nice, and kind of scary.

**MAGGIE:** Yeah, we're walking together. (*opens her eyes and looks at the therapist*) I'm not alone. It's better to have you with me. I think it's more nice than scary...but I sort of feel like crying when I say that...

This joining experience was a turning point in therapy. Maggie had grown up in a silent void, much like the setting in her dream. She had never known her father, and her mother was often “away” pursuing the next new boyfriend in her life. By 10 years of age, Maggie was regularly spending most of the weekends alone, fixing her own meals, and putting herself to bed while her mother was “out.” Maggie felt unwanted and rejected by her mother, and not “safe” with the men her mother brought home—who “weren’t always nice” and whom Maggie sometimes felt “scared” of. The dream reflected the emptiness of her childhood, the lack of comfort and protection she had experienced, and her lifestyle adaptation of moving-away from others. However, the crisis with her daughter overwhelmed her coping strategy to withdraw and be self-sufficient and aloof. In letting the therapist join her in her dark, empty space, Maggie took the enormous personal risk of accepting the human contact she longed for yet had long since resolved to hold away. Of course, this single CEE did not resolve her core conflict or change her interpersonal style for coping with it. However, in big and small ways, similar incidents of sharing continued to occur with the therapist.

Maggie’s sense of aloneness was heightened by her consciousness of being “different” because she was biracial, as her therapist, herself a biracial woman, was well aware. The therapist’s support and ability to articulate all the factors, familial cultural, and racial, that contributed to Maggie’s sense of isolation helped her begin to change in three ways.

First, Maggie’s affective constellation emerged. The profound *sadness* resulting from her childhood neglect began to come to her—as we see at the end of the preceding dialogue. To ensure that the lack of response that Maggie had suffered as a child was not reenacted in their relationship, the therapist was careful to let Maggie know that her feelings were being heard this time. The therapist was skillful in communicating to Maggie that she was not in a dark, silent void anymore but rather in a caring and responsive relationship. Maggie became more comfortable sharing her loneliness with the therapist—and began to question the accompanying beliefs that she “didn’t really matter very much” and that others “wouldn’t be very interested in helping” if she had a problem and needed them. Feeling safe with the therapist allowed her to experience her deprivation and face it rather than dismiss it as she had always done before. She was able to register how painful this had actually been for her. This deep sharing and resonance from the therapist was relieving for her, but it evoked another threatening feeling—the intense *anger* she felt toward her mother for, in effect, walking away from her and for exposing her to all of these men who just kept coming in and out of their apartment. The therapist was affirming of her anger and validated her feelings of not being watched over and protected. For example,

**Maggie:** *I didn’t deserve this—a mother shouldn’t leave her daughter home alone to go chasing after the next stupid boyfriend or bring all those creeps into the house. The one she had around the longest was awful—he knew the bathroom door didn’t lock and he used to always “accidentally” walk in when I was in there. He also used to talk about the size of my breasts and behind—he was a creep and my mom never stopped him from saying those things. I guess she preferred those worthless men to me. Even when they tried to touch me, she would say they were just “teasing.” I hated it.*

Feeling such anger toward her mother evoked the third feeling in her affective constellation—*anxiety*. Painful separation anxieties were aroused by this protest, as Maggie feared that being angry with her mother would only further push her away and leave her back where she started—feeling even more alone. The therapist affirmed each feeling in her affective constellation and, as she did, Maggie progressively became more comfortable with each feeling. In turn, the therapist began to hear narratives in their sessions where Maggie was being a little more forthright, engaging, and confident in her interactions with some others.

Second, as the therapist continued to understand and support Maggie through each feeling in her affective constellation, Maggie began to make further important behavioral changes in how she responded to the therapist. Although Maggie had developed a strong working alliance with the therapist, she was still deeply reluctant to accept help from anyone and held a part of herself back. Interestingly, the therapist's biracial identity had given her an **ascribed credibility**, based on perceived similarities of race and gender, which helped Maggie remain in treatment initially when she was so unsure of this process. Now, however, the therapist's **achieved credibility**—her skillfulness and the effectiveness of her responses—took center stage.

As their working alliance continued, Maggie incrementally relinquished her coping style of moving-away and gradually allowed the therapist to see her, know her, and help her. Maggie became less reserved toward the counselor and, instead, more expressive and responsive than she had been with others. She talked more freely about herself and found herself being curious about the therapist's own personal life. The therapist saw this as positive for this particular client and responded willingly at times. Maggie's nightmares and headaches already had been alleviated, but now a sense of humor and relaxed confidence were emerging as well.

Third, as Maggie talked about feeling "fuller" inside, the therapist observed that changes were occurring in other relationships as well. At work, her supervisor said he was pleased to observe that Maggie was "less irritable and sullen than before"—it was easier for others in the office to talk and work with her. When tensions surfaced with others, Maggie's initial reaction still was to withdraw and "go away inside." However, Maggie became better able to approach problems with others and talk through the interpersonal conflicts that inevitably come up in a busy workplace. For Maggie, it was especially helpful for her to describe a conflict she was having with a coworker or customer and have the therapist suggest new or different ways for her to respond. Together, they would role-play different responses that would keep Maggie involved in the situation, rather than withdraw or remain aloof as she had always done in the past. Maggie relished playing the part of the difficult customer or coworker and have the therapist role-play what Maggie could say or do in that situation. Maggie found this behavioral training invaluable. With frequent success, she regularly found herself using almost the same words and phrases with others that the therapist had modeled.

Especially meaningful for the therapist to observe, Maggie became more accessible—more emotionally present—with her two adolescent children. For the first time, she talked more about her own interests and personal history with them. The nearly grown children welcomed this sharing and, in turn, began to say more about themselves and what was going on in their lives as well. Maggie was feeling closer to her children and, with coaching from the therapist, more capable of helping her daughter with the aftershocks of her assault.

In addition, Maggie began to change how she responded to a man she had started dating. This had been a superficial relationship, like most others in her life had been. Even though they had known each other for more than two years, Maggie now invited more personal sharing and emotional closeness in their relationship and asked for more commitment from him. Specifically, she asked him to talk more about himself and to spend more time with her, which he was able to do. It was exciting but anxiety-arousing to take each of these steps forward. Maggie's old coping style of moving-away was activated each time she took a step toward her boyfriend or others, especially when others did not respond in completely positive ways. Repetitiously, the therapist celebrated her successes in each of these arenas, and patiently helped her work through the setbacks and recover from the disappointments that regularly came along as well. As she continued to make progress and participate more fully in life, however, Maggie tentatively suggested that it might be time to end counseling.

Although things had been going well, Maggie became depressed as they started talking about ending. Even though she had brought up the topic of termination, she missed their next session (“I just completely forgot”), and arrived late to another. As before, the therapist continued to focus Maggie inward, and her profound feelings of being unwanted, alone, unprotected, and vulnerable emerged again. The therapist suggested that they put off setting a termination date for a while and work further with this sadness. With more vividness and detail than before, Maggie recalled her childhood depression. Painful recollections returned—for example, the memory of being eight years old, sitting alone on the living room couch, and listening to the clock tick away the empty afternoon. Maggie sobbed as she recalled her aching wish for a mom she could come home to, which was only reenacted in her unfortunate marital choice. Following her early maladaptive schemas, Maggie had married a salesman whose job took him away from home for extended periods—and he had been preoccupied and emotionally unresponsive to her when he was home as well. Thus, in her first significant love relationship, Maggie chose someone who repeated, rather than resolved, her history of aloneness. However, touching the pain of her childhood neglect so directly, and sharing so fully how it was echoed in the disappointment of her marriage, relieved the depression that the suggestion of terminating had precipitated. Before long, Maggie again felt ready to end and, this time, successfully terminated.

## CLOSING

In this chapter, we have studied an interpersonal model for conceptualizing clients. In an extended case study of Peter, a moving-toward client, we saw how this model can be applied to treatment. We also explored two case summaries that illustrate its application to moving-against (Juan) and moving-away (Maggie) clients. The therapists in these three cases were helpful, in part, because they used their clients’ rigid interpersonal coping styles as an orientating focus for treatment. These therapists were also helpful because they worked collaboratively and engaged the client in recognizing and expanding their coping strategies.

Although new therapists often find these three coping styles relevant with their clients, they can intervene with them ineffectively. To help, Safran and Muran (2000) clarify that the therapist’s premature attempts to identify a relational pattern (that is, force it) usually feels blaming to the client. They encourage collaboration and, again, as Kiesler (1996) and others guide, to share observations with “skillful tentativeness.” Emphasizing the subjectivity of the therapist (e.g., “My sense is...” “What occurs to me...” “I’m wondering if...”) suggests a more egalitarian relationship. This allows clients to be freer to accept, reject, or modify the therapist’s observations and make them their own. Similarly, once therapists identify these patterns, they can press clients to change the patterns too quickly. Instead, a better stance with most clients is to encourage a simple awareness of the relational patterns—merely taking note when clients see themselves responding in this way, rather than pressing clients to change their coping strategy prematurely. For example:

**THERAPIST:** Let’s watch for this and see when it comes up. You know, just pay attention to moments when you find yourself accommodating or trying to please too much—and we’ll start to track that together. As you become more aware of it, I think it will become easier for you to change it and respond differently if you choose.

Looking ahead, new therapists need further guidelines for conceptualizing clients and more help to shape treatment plans that guide where they are going in treatment and what they are trying to accomplish with each client. To do this, further information about

clients' current interpersonal functioning will be provided in Chapter 8. At this point, however, readers are prepared to utilize the guidelines for keeping process notes provided in Appendix A, and for writing case conceptualizations in Appendix B. These guidelines have two overarching goals. First, they will help therapists formulate treatment plans by clarifying the maladaptive relational and cognitive patterns that provide a focus for treatment. Second, they will help therapists recognize the process dimension and intervene more effectively by linking the current interaction with the counselor to the problems clients are having with others.

### CHAPTER EXERCISE

What is your primary or initial interpersonal coping strategy? As a child, why was this adaptive in your family of origin? What does it cost and what does it give you in your life today?

### READING SUGGESTION

Student therapists with two or more years of clinical experience are encouraged to read Lorna Smith Benjamin's (2006) *Interpersonal Reconstructive Therapy*, New York: Guilford. This clinically rich, interpersonal and attachment-oriented book will help readers understand and intervene with their clients. Clearly written, integrative, and wise, this book is a must-read for more experienced graduate student therapists.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes.





## CHAPTER 8

# Relational Themes and Reparative Experiences

### CONCEPTUAL OVERVIEW

This chapter explores further the interpersonal patterns that are causing problems in clients' lives, highlights different ways they can be played out in the therapeutic relationship, and suggests useful ways to respond to these reenactments. In particular, we are going to explore these maladaptive relational patterns to help therapists conceptualize what is really wrong for clients and better understand what is going on in the therapeutic relationship. We will learn how therapists can use their own feelings and personal reactions toward clients as one of the best ways to understand the problems clients have with others. Clients often are adept at getting the therapist to feel or respond toward them along the same problematic lines that occur with others in their lives (e.g., having the therapist—like others in their lives— feel overwhelmed, bored, or controlled by them; helpless or discouraged about being able to help; anxious about offending them or making a mistake). *We will see how the problematic patterns that are disrupting clients' relationships with others can be brought into the therapeutic relationship and played out with the therapist along the process dimension.* Empowered by an understanding of what is going on between them, therapists can formulate better treatment plans and know where they are trying to go and what they are trying to do to help their clients. Throughout, we will suggest different ways that therapists can intervene to change these problematic reenactments by making process comments, providing interpersonal feedback, and using other immediacy interventions to disconfirm IWMs and provide corrective relational experiences.

### THREE WAYS CLIENTS REENACT THEIR PROBLEMS WITH OTHERS IN THE THERAPEUTIC RELATIONSHIP

The problems that clients try to resolve in treatment usually originated in attachment and formative family relationships, and now are being played out in thematically similar ways in current relationships. Routinely, the same interpersonal patterns that are causing problems with others emerge in some form in the therapeutic relationship as well.

As we have seen, clients do not merely talk with therapists about the problems they are having with others (Beatson & Rao, 2014). Clients also recreate relational themes with the therapist that parallel the problematic patterns they are playing out with others. The challenge for the therapist, then, is to find a way to alter or change this pattern—often through process comments and providing CEEs. Providing this experience of change with the therapist—as opposed to merely interpreting or explaining it intellectually—greatly facilitates clients’ ability to change these faulty patterns with their spouses, children, and important others (Rubinstein, 2015). Throughout the course of treatment, the therapist’s aim is to:

1. identify and highlight these maladaptive relational patterns that keep recurring with others,
2. provide a new and better response that does not repeat the familiar scenario in their interactions together, and
3. help generalize this experience of change in the therapeutic relationship to clients’ interactions with others in their everyday lives.

Therapists practicing in varying interpersonal/relational, psychodynamic/inter-subjective, attachment, and cognitive/schema-oriented approaches are all working with three closely related but still distinct ways in which clients bring their problems with others into the therapeutic relationship: *Eliciting Maneuvers*, *Testing Behavior*, and *Transference Reactions*. Additionally, all three topics share an especially important component: each commonly evokes *COUNTERTRANSFERENCE REACTIONS* in the therapist.

Clients often employ eliciting maneuvers (Sullivan, 1968) to avoid anxiety and defend against their problems by getting others to respond in certain predictable ways. At other times, clients will employ testing behavior (Weiss, 1993), to cope with problems. When “testing,” clients behave in certain ways to assess whether the therapist is going to respond in the familiar but problematic ways they have learned to expect, or in the more helpful ways they actually need. The final (and better-known) way that clients bring their conflicts into the therapeutic relationship is through transference reactions or distortions based on their IWMs—systematic misperceptions or cognitive distortions of the therapist. These three concepts illuminate much of what is going on in therapeutic relationships. In all three concepts, the basic assumption is that the client recreates certain problematic relationship patterns with the therapist as a way to *confirm* that the therapist and others are going to respond in the same unwanted way that important attachment figures once did or behaviorally respond in a new, more helpful and corrective way (Castonguay & Hill, 2012; Gelso & Hayes, 2002). Let’s look closely at these important interpersonal processes, and the countertransference issues that all three readily evoke in therapists.

## 1. ELICITING MANEUVERS

Many cognitive and interpersonally oriented therapists have described how clients develop fixed interpersonal styles to avoid anxiety and defend against the unwanted responses they expect from others (see, e.g., Beck et al., 2003; Benjamin, 2006; Millon, 2004 ; Young, 1999). Routinely, clients are inaccurate in their perceptions of others and restricted in their range of emotions. As we saw in the last chapter, they often respond to others inflexibly, using rigid coping styles of moving-toward, moving-away, and moving-against. In part, clients systematically employ these interpersonal styles to:

1. Elicit desired responses from others that will avoid conflict and ward off anxiety and
2. Preclude threatening or unwanted responses from others that will trigger their IWMs and key conflicts.

Thus *an eliciting maneuver is an interpersonal strategy that wards off anxiety and brings about certain desired, safe responses*. However, this defensive maneuver learned in parenting and familial interactions, keeps the client stuck; change, personal growth, and interpersonal relationships remain stymied (Bartsch et al., 2015). To illustrate how eliciting maneuvers serve to defend against problems, we will return to the case study of Peter, the moving-toward client in Chapter 7.

Peter's moving-toward style tended to elicit approval and kindness from others. As a general way of life, Peter was helpful; he sympathized, agreed, and cooperated with people. For example, Peter tried to "understand" his girlfriend's infidelity and to win his instructor's approval. When Peter began to see a therapist, he continued to employ the same interpersonal style that he had used with others throughout his life. As many moving-toward clients do with their therapists, he tried to elicit his therapist's approval by being a "good" client who was quickly getting better—at least until the therapist began using process comments to question this unwanted, symptomatic *compliance*. For example: "Peter, sometimes I wonder if you are trying too hard to figure out what *I* want you to do. Any thoughts about that possibility?"

Peter was rarely forthright, appropriately assertive or angry, or skeptical with anyone in his life. These responses, which a well-functioning person needs to employ at times, were not part of Peter's interpersonal repertoire. For example, he did not communicate how angry he was with his girlfriend or even have much awareness of how angry he actually was with both of his parents. Similarly, he did not set limits with his practicum instructor or classmates regarding how much critical feedback he could incorporate at one time. Thus, *his pleasing interpersonal coping style was designed to elicit approval from others and discourage familiar, angry, or critical responses that would arouse intense anxiety for him*.

If Peter's therapist had merely responded automatically, by simply supporting Peter and meeting his anxious need for approval, without first considering what Peter's interpersonal style tended to elicit from others, Peter would not have changed much in therapy. Peter would have only reenacted in therapy the same pleasing/caretaking/accommodating interpersonal defense that he had used throughout his life. Shoring up these defensive coping styles may be the goal in some time-limited or supportive therapies and can help some clients in crisis regain their equilibrium. However, the therapist wanted to do more than this with Peter. Seeking more enduring changes and not merely symptom relief, the therapist tactfully used process comments to help make Peter's moving-toward style overt as a shared focus for discussion. Thus, instead of automatically responding to what Peter elicited and providing only approval, support, and reassurance, the therapist focused on this eliciting maneuver as a part of Peter's problems that needed to be addressed in treatment. How did this intervention occur?

Over the course of several months, the therapist remained watchful for points of entry to tentatively wonder aloud—in a supportive and curious manner—about instances where Peter's coping style of moving-toward others might be operating with him and with others in Peter's life. Soon, working collaboratively on this, Peter and his therapist began to consider how this was serving to protect and help him avoid his problems, but also causing Peter further conflict and distress. As they "named" this behavior pattern and began watching for it together, Peter increasingly reported instances where he had observed himself using this moving-toward style with others, and recognized how it elicited approval and support from others. He also began to see how this style discouraged the critical or rejecting responses that were so familiar from his childhood, and that were so anxiety-arousing for him. As Peter made progress in treatment, he gradually became less preoccupied with winning approval from others and less worried

about “fixing things” when small interactions with others held some conflict or just didn’t go so smoothly. The treatment goal of expanding his interpersonal range was being met as he continued to report instances where he was feeling less self-conscious than before, speaking up more directly with others, and standing up for himself more assertively when the situation called for it. Peter was excited about these changes and described himself as feeling “less anxious” and “stronger.”

### **Clients’ Eliciting Maneuvers Can Evoke Countertransference in the Therapist.**

Clients’ eliciting maneuvers can evoke countertransference reactions in the therapist that lead to reenacting rather than reparative responses. For example, the eliciting maneuvers used by moving-toward clients like Peter are not personally threatening or challenging for most new therapists to work with. However, *if the therapist has strong needs to be liked or approved of, and is conflict-avoidant in relationships, it is easy to see how Peter and this therapist can merely reenact together what Peter does in other relationships.* This may feel good to both but precludes change for Peter. In contrast, moving-against clients like Juan in Chapter 7 employ eliciting maneuvers that are often challenging for beginning and experienced therapists alike. In the first few minutes of the first counseling session, for example, moving-against clients may do something to take command of the relationship. They may find some way to intimidate or provoke the therapist—making the therapist feel insecure in the therapeutic relationship (or sometimes defensively angry and competitive). For example, clients may insist on sitting in therapists’ chairs, question therapists’ adequacy, diminish their credentials, compare them to “better” or more experienced therapists, or criticize what therapists have just said or done. Consider the following:

**CLIENT:** So, if you’re just a trainee, what makes you think you know enough to be able to help me?

**THERAPIST:** Well, I’ve had some experience before I entered the program, and I have a supervisor to help me.

**CLIENT:** I see. Now, does this supervisor always tell you what to say, or do they let you newbies say what you think sometimes?

This provocative presentation elicits anxiety in many therapists—as it often succeeds in doing with others in clients’ lives. Many therapists respond by trying to be nice in order to stop the hostile challenges: continue along, hoping that ignoring the hostile subtext will stop it; or becoming quiet or withdrawing emotionally. In contrast, some therapists will respond in kind and become judgmental or punitive, and a few will even counter with their own overt hostility (Binder & Strupp, 1997; Henry, Schact & Strupp, 1990; Lambert & Barley, 2002). Treatment will not progress when moving-against clients succeed in eliciting one of these defensive flight-or-fight responses from the therapist—as they commonly do with others. By reenacting this maladaptive relational pattern with the therapist, these clients have successfully neutralized the therapist’s ability to help them (or to hurt them). In this way, they have defended against their anxieties associated with having problems, asking for help, relinquishing some control, getting closer to what’s really wrong, and so forth. Clients’ eliciting maneuvers protect them from their core conflicts, but at the price of change.

Unfortunately, therapists tend to respond reflexively or automatically to clients’ eliciting maneuvers with their own countertransference propensities—that is, with their own characteristic tendencies toward flight or fight (Millon & Grossman, 2007; Mueller & Aniskiewicz, 1986). What can therapists do instead? First, they can try to retain legitimate control of themselves internally by trying to understand what is occurring

right now in their relationship with the client—rather than *personalizing* it so readily and overreacting. It certainly is challenging for new therapists to do this in the beginning, but with experience and help from a supervisor the goal instead is to begin formulating working hypotheses about:

- *the impact this moving-against behavior might have on others in clients' lives (such as making others feel like arguing with or withdrawing from them) and*
- *what conflicts or anxiety-arousing situations clients might be avoiding by these maneuvers (for example, feeling weak or ashamed of needing help or having a problem they can't solve on their own).*

Second, on an interpersonal level, *therapists are trying to find ways to engage moving-against clients in some way in which the interpersonal patterns of domination, intimidation, and competition are not reenacted.* Immediacy interventions offer us many possibilities (Teyber & Teyber, 2014). One option is for the therapist to adopt a curious stance with the client and try to explore, or try to understand internally, clients' responses as diagnostically or therapeutically useful, without avoiding the challenge or becoming defensive (Datz et al., 2016). For example:

**THERAPIST:** (*calmly, straightforwardly*) So, your previous therapist was an experienced psychiatrist, and you're not sure that a younger social worker such as myself can help you. Let's talk together about that—tell me more about your concern.

Another alternative is for the therapist to make a process comment and engage clients in talking together about the current interaction and what's going on between them.

**THERAPIST:** You're speaking to me in a loud, forceful voice right now, and you sound angry. What are your thoughts about what might be going on between us here?

With some clients, the therapist may wish to use a self-involving statement or provide interpersonal feedback to begin exploring the effects of his or her interpersonal style on others and highlighting the relational patterns that ensue. For instance:

**THERAPIST:** Let me take a bit of a risk here and talk about the impact you have on me sometimes. Right now, I'm feeling criticized as you correct me again, as I've felt other times. That leaves me wondering how others in your life usually respond to you when you are critical like this—what usually happens next?

Some of these clients will be unaware of the distancing and disruptive impact their provocativeness/criticalness has on others, and this interpersonal feedback can provide a point of entry to begin exploring this collaboratively. Other moving-against clients will say something that suggests that they “hit first” before someone can get them. The purpose here is not to win the battle for power and control. On a process level, this interaction would only continue or re-enact the same cyclical pattern that recurs with others (Wachtel et al., 2005). Rather, this approach is intended to help the therapist find some way to engage with clients other than automatically responding with counterhostility, control, or withdrawal to the eliciting maneuvers—as others usually do. It will also help clients develop increased awareness of when they employ their coping style, its impact on others, and (most important for change to occur) the price they pay for this coping style. *As clients hear interpersonal feedback from the therapist, become more aware of the impact they are having on others, and begin to reflect on when and why they act this way, they may begin to respond more flexibly and exercise more choice about how they want to respond.*



Still another possibility is for the therapist to explore how clients' eliciting maneuvers affect clients and others. For example:

**THERAPIST:** (*non-defensively*) You have insisted on sitting in my chair, even though I asked you not to. Now that you are there, how does it feel?

Routinely, moving-against clients become aware of feeling alone, empty, or anxious at these moments, and the hollowness of their victory provides a new shared point of entry for the therapist and the client. As all four of these different types of immediacy responses illustrate, there is no simple formula for the “right” way for therapists to respond. Per client response specificity, one type of response will work well with one client and not so well with another. Therapists need to be flexible, assess how clients respond to each intervention, and modify their responses to find what works best for a particular client (Persons & Silberschatz, 1998). Thus, to respond effectively to the eliciting maneuvers these challenging clients present, therapists' intentions are to:

- *attend to their own subjective reactions and recognize what clients are eliciting in them (such as feelings of incompetence or competitiveness);*
- *formulate working hypotheses about how others would typically respond and begin to identify the relational patterns that are occurring with others and causing problems in clients' lives;*
- *try to find another way to respond (e.g., via process comments, self-involving statements, or interpersonal feedback) that does not go down the old familiar path and reenact the same problematic scenario that the client usually elicits from others.*

Although moving-against clients often intimidate new therapists, this doesn't usually last very long. With a little more clinical experience, practice, and rehearsal, and a supportive supervisor, new therapists learn not to charge so readily at this red flag the client is waving. With more exposure to moving-against clients, therapists learn to appreciate that the intensity and rigidity of clients' eliciting maneuvers, no matter how alienating initially, are commensurate with clients' degree of anxiety and conflict. As therapists better understand the purpose of these distancing maneuvers, they begin to find some compassion for the predicament such clients are living out, which is usually the very best way to step out of the old relational pattern being elicited. By flexibly trying out different approaches (it helps to practice by role-playing with classmates and supervisors), it's really not so hard for therapists to find ways to engage these clients that do not reenact either the intimidated/withdrawal or the competitive/hostile patterns that they usually succeed in eliciting from others (Gans, 2011).

As we have seen, the confrontational eliciting maneuvers of moving-against clients—such as Juan in Chapter 7—place considerable stress on many new therapists. Surprisingly, however, the meeker, moving-away clients—such as Maggie in Chapter 7—often pose a greater challenge in the long run. Many who select counseling careers have strong needs for engagement and deeply enjoy close personal relationships. When moving-away clients continue to maintain their characteristic stance of emotional distance, therapists can begin to feel ineffective or unimportant, become discouraged about the therapeutic relationship and helping, and ultimately give up—*disengaging as many others have done before* (Robbins & Jolkovski, 1987). When this occurs, clients' eliciting behaviors have successfully defended them from a meaningful relationship with the therapist and thereby allowed them to avoid anxiety-arousing issues—which were reality-based threats in their history evoked by risking genuine involvement. Thus, the clients' conflicts, or the threats and dangers that engagement has brought in the past, will not be activated, but the therapist's only real vehicle for resolving these problems is closed as well. Rather than

responding automatically to these clients' aloofness by giving up and, too commonly, blaming clients as unmotivated, therapists can use process comments to find other avenues for establishing an alliance. For example:

**EFFECTIVE THERAPIST:** You're very quiet, Mary. Sometimes it almost feels as if you are hardly in the room with me. Would it be OK with you if we talk a little about what it's like for you to come here and take the risk to try and bring me in on your life?

**CLIENT:** I'm not sure what you mean. Am I doing something wrong?

**THERAPIST:** Oh no, you absolutely are not doing anything wrong at all. I guess I'm trying to say that sometimes I feel like I'm not reaching you—or getting close to what really matters in your life, even though I want to. Has anyone else ever said something like this to you—or maybe this is just me?

**CLIENT:** Well, kind of. The only really nice boyfriend I've ever had left because he said he never really felt very important to me, that he gave up trying to "pursue" me.

**THERAPIST:** Thanks for joining me. This is a sensitive thing for us to be talking about, but I think this is important. It sounds like other people feel held away from you in some way or that they have to work to reach you...or maybe that you can "go away" so readily or do without them so easily. I'm trying to understand how this goes in your life. Can you help me say it better?

**CLIENT:** I think I do withdraw from people easily. I know I'm too sensitive—my feelings get hurt so easily...over "nothing" they would say. And sometimes I feel nothing they could say would help me...

**THERAPIST:** I see, you go away when your feelings get hurt, and maybe others don't understand what's really going on for you. We'll have to watch for that here and make sure that doesn't happen between you and me, too. But right now, I'm appreciating that you're choosing not to go away from me, but taking the risk to stay present and engaged with me as we try to sort this through. This is feeling very different right now than how we usually are. I'm feeling more connected with you, and I like it, but I'm wondering what it's like for you?

**CLIENT:** I don't know...I'm not so sure. I just get afraid with other people. I know I shouldn't go away so easily, but it just feels easier...it's safer.

**THERAPIST:** It just "feels safer." And maybe it's not feeling very safe with me either right now.

**CLIENT:** Oh, you're very nice. But it is just easier to go away...it's just what I've always done.

**THERAPIST:** Well, I'm thinking that it's probably easier for you to "go away" because it really hasn't been safe for you in the past. Would you be willing to tell me a little more about how I might hurt you if you stay engaged with me like this, or what others have done in the past that you really did need to get away from? You lead and I'll follow.

Therapists can attend to what clients systematically elicit in others and hypothesize how this interpersonal strategy is likely to affect others and cause problems in their lives. In doing this, therapists attend to clients' developmental history, current life situation, and to the cultural context in which coping styles and eliciting maneuvers are embedded. Importantly, *therapists are also attending to their own feelings that are elicited by their clients*. This self-awareness is one of therapists' most important sources of information about clients and what their interpersonal style tends to elicit or "pull" from others (such as feeling controlled, minimized, pushed away, idealized, etc.). To be able to learn from their own personal reactions to clients, however, requires therapists to be *non-defensive* enough to be able to observe what the client tends to elicit or "pull" in them (Bass, 2007; Gabbard, 2007).

Attending to their own personal reactions in this way will often provide important information about clients' key concerns, coping strategies for defending against old problems, and how the relational themes being played out in the therapeutic relationship may be disrupting other relationships. Doing this is very useful in highlighting the *treatment focus*, as therapists consider:

1. What has brought clients to this particular place in their lives (developmental history);
2. What is causing problems in clients' lives now and how this might be similar to what they have had to deal with developmentally;
3. What the cultural context is in which this all developed;
4. How aspects of these issues are being replayed in the client-therapist relationship; and
5. How to address and clarify what has been difficult for clients and provide an experience in the client-therapist relationship that is different, reparative, and empowering.

Over time, therapists can begin to help clients identify their eliciting mechanisms, recognize the situations or types of interactions where they tend to employ these measures, and understand how these maneuvers have served to create safety in the past. It takes courage for therapists to do this, however, especially as they begin their clinical training. Therapists are trying to tolerate their own unwanted feelings long enough to sort through whether these reactions have more to do with their own lives and personal feelings (i.e., countertransference) or whether they inform therapists about clients' eliciting maneuvers and help them understand what clients tend to evoke in other relationships (Datz et al., 2016). The key element here is for therapists to remain non-defensive in their own reactions so they can reflect upon them and learn more about what may be going on in the therapeutic relationship.

We need to explore this interface further, so let's examine how *clients' eliciting maneuvers often tap into therapists' own personal issues*—activating **countertransference reactions** that lead to reenactments, confirm faulty beliefs and expectations, and impede treatment.

**Two Types of Countertransference: Client-Induced versus Therapist-Induced.** At the beginning of this chapter, we noted that all three topics in this section shared a common denominator: Countertransference runs through each of them. We have just begun to see how eliciting maneuvers can readily evoke countertransference reactions in therapists, and we are going to see this throughout our next two topics of testing behavior and transference reactions. Let's introduce this related new concept here, and begin by distinguishing two different types of countertransference: client-induced countertransference versus therapist-induced countertransference.

As we have seen, clients' eliciting maneuvers tend to evoke certain similar reactions in the therapist and others—which we will call **client-induced countertransference** *because the client tends to induce these reactions in most people*. Things become more complicated when clients' eliciting behaviors happen to tap into therapists' own personal issues or current stressors—which we will call **therapist-induced countertransference** *because it has a lot to do with a particular therapist personally* (Springmann, 1986). With therapist-induced countertransference, it's not just about what clients tend to elicit in others, but how the therapist's own life and personality have been activated and brought into the equation as well.

To illustrate, suppose that a hyperactivating client acts helpless and keeps escalating his distress, and the therapist tries hard to respond better and help but “fails” at each attempt. Ideally, the therapist would retain a certain objectivity, remain actively engaged in the work but not overly invested in the client's response, and keep exploring other ways to respond—for example, by making the process comment that whatever the therapist tries

to do seems to disappoint the client or fails to help. However, what if this therapist grew up with critical caregivers and, no matter what she did, she could never quite do it right or please them? If the therapist's own personal concerns regarding performance demands or criticism are activated in this way, she may lose her effectiveness because the client's eliciting maneuvers have tapped into her own history and personal issues (in other words, "therapist-induced" countertransference). Continuing, the therapist who is now over-reactive to this client's unspoken disapproval may feel inadequate and withdraw—believing falsely that she is failing or incompetent as a therapist, or perhaps feeling resentful and becoming critical toward this client in turn. *When clients successfully elicit the same type of response from the therapist that they usually elicit from others, as is occurring here, the therapist and client are reenacting rather than resolving the problem.* These "enactments" replay unwanted patterns rather than provide a CEE (Aron, 1996; Renik, 1998), and are complex and challenging issues for new therapists. As student therapists learn more about these ways of understanding what may be going on between the therapist and the client, they discern more specific ways to intervene and help their clients change.

Scenarios such as the preceding one—where the client's eliciting maneuvers activate therapist-induced countertransference and lead to reenactments in the therapeutic relationship—are commonplace. For beginning and experienced therapists alike, it's just part of the work. When this type of reenactment occurs, however, and clients' defensive eliciting maneuvers have succeeded in immobilizing or discouraging the therapist, clients often switch to the other side of their ambivalence and try to re-engage the therapist—perhaps by reassuring the therapist about how helpful the sessions are. Clients don't want to end the relationship and *will often try to get the disengaged or discouraged therapist to become involved in the relationship again.* Premature and unnecessary terminations may occur, unfortunately, because the therapist (and perhaps the supervisor) fails to hear clients' renewed bids for engagement because the discouraged or frustrated therapist has given up on the relationship. For example:

**INEFFECTIVE THERAPIST:** This client is hopeless; he isn't ready to change. He's the King of Blame—he just wants to tell everybody else what's wrong with them—including me! No wonder his third secretary just quit and his teenaged son doesn't want anything to do with him.

Therapists will find that these disengaging and subsequent re-engaging maneuvers are especially significant with more troubled clients who are coping with abuse histories.

How do therapists begin to sort through for themselves whether clients' eliciting maneuvers (client-induced), their own personal issues (therapist-induced), or perhaps, some combination of both are evoking their strong reactions? If a particular client makes others feel or react similarly (e.g., others in the client's life are also bored, impatient, or intimidated, as the therapist is made to feel in the session), it is probably client-induced. If not, this reaction probably has more to do with a therapist's own personal life (it is more likely therapist-induced countertransference). Of course, when reenactments and ruptures are being experienced, both may be occurring (Hill & Knox, 2009) and supervision is usually the best place to sort this through (Falender & Shrafransky, 2004).

We will find these two different conceptions of countertransference helpful as we move on to explore clients' testing behavior and their transference reactions.

## II. CLIENTS' TESTING BEHAVIOR

We have seen how clients employ their inflexible coping style to systematically elicit responses from others that may seem self-defeating yet have served to avoid anxiety or support self-esteem in the past. These eliciting maneuvers—or "security operations" as Sullivan (1953) first termed them—are interpersonal defenses.

Eliciting maneuvers are used defensively to avoid anxiety, but at other times clients use different interpersonal strategies to approach and try to resolve problems. Although this may sound contradictory, it reflects clients' ambivalent feelings about treatment—*the hope that things could change and get better versus the schema-driven expectation that the same unwanted outcomes will only occur again*. Clients often carry out this healthy, approach side of their ambivalence by means of **testing behavior** (Sampson, 2005; Silberschatz & Curtis, 1993; Weiss, 1993). In direct or in more covert ways, and especially at the beginning of treatment and when strong emotions have been evoked, clients often tend to reconstruct interpersonal patterns with the therapist that follow those that have been causing problems with others in their lives (Strupp & Binder, 1984; Zilcha-Mano et al., 2014). Clients “test” to ascertain whether the therapist will respond in the familiar but problematic way that others often have in the past. Or, as is hoped, whether the therapist can respond in a different and more reparative way that changes the old predictable scenario—which allows clients to expand IWMs and facilitate progress on their problems. There is nothing casual about this testing for clients; *they are vitally interested in assessing whether the therapist is going to confirm or disconfirm strongly held but faulty expectations and beliefs*. Many clients bring profoundly painful histories of exploitation, denigration, ostracization, and rejection; for these clients, their trust in others has been betrayed. Their testing behavior with the therapist often reflects their intense wishes for healthier, safer relationships—specifically, hoping that the therapist's response will disconfirm their fear of again being violated, disappointed, or hurt. *Through testing, in other words, clients are trying to assess safety/danger in the therapeutic relationship* (Rubinstein, 2015).

Therapists respond effectively and pass tests by *behaviorally disconfirming clients' faulty beliefs and expectations*—and this corrective or real-life experience (not just interpretations, explanations, or reframing) powerfully *sets the change process in motion*. Clients feel safer when the therapist does not respond in the familiar but problematic way that important others often have, and that they have now come to expect from the therapist as well. Immediately following this CEE, clients often make visible progress in treatment. For example, within the next minute or two, the therapist may observe that clients act stronger or behaviorally improve in one of these ways:

- They express more confidence in themselves or in the therapist (“You know, I think I'm just going to go ahead and talk to my wife about this. I've gone around about it in my head for too long now. I'm going to bring this up with her and just put it on the table—it's not good for me to keep avoiding it like this.”)
- They feel better as symptoms such as anxiety or depression diminish (“I feel more settled right now. Maybe he won't ever stop being so bossy, but that's just him—it really doesn't have much to do with me. Maybe he can't change, but I don't have to get so angry and upset when he's demanding like that.”)
- They take the risk of bringing up threatening new material with the therapist (“I've been ashamed to tell you about something that's happening: I've been having an affair for a while now. I know I should be talking to you about it, but I've been afraid to tell you. I guess I've been worried that you'll judge me—you know, you wouldn't respect me anymore if you knew about this.”)
- They act stronger by being more honest or forthright with the therapist and bringing up problems between them (“It's kind of hard to say this because I don't want to hurt your feelings or anything, but I've been thinking for a while that therapy hasn't been working very well for me. You're so quiet—I don't know what you're thinking a lot of the time. I think I need more feedback or something from you.”)

In a healthy attempt to address and resolve problems, clients often actively (but without really being aware of it) elicit the same types of problems with the therapist that



they are having with others. They are actively testing to see whether they can change these problematic relational patterns and disconfirm the pathogenic beliefs that stem from them. However, it is difficult for clients to find disconfirming evidence because repeated confirmation of the beliefs has made clients deaf to such evidence, and selective attention and biased filters also lead clients to keep slotting diverse experiences into the same narrow categories (Beck, 1995). However, the *power of immediate, in vivo relearning with the therapist allows clients to begin questioning, or sometimes even to relinquish, deeply held but faulty beliefs about themselves and expectations of others.*

To illustrate, suppose a young adult client is being seen in a college counseling center for depression. Her therapist has formulated the working hypothesis that this client is ruled by the pathogenic belief that if she has her own mind and does what she wants with life, her mother will be hurt, feel sad, and withdraw. Thus, she believes that she deserves to feel bad whenever she *differentiates* in any way because this is being “selfish” and hurts her mother, who acts as a martyr and induces guilt. How can this hypothesis guide the therapist’s interventions as he tries to pass the tests this client is apt to present? The therapist would likely fail her test, because he would be repeating the dysfunctional interpersonal pattern that the client lives out with others if he expressed caution, worries, or doubts when this inhibited client said something about becoming more independent, speaking up, acting on her own behalf, or pursuing her own interests and goals in some way (rather than submerging her Self to the needs of others). For example:

**CLIENT:** I don’t think I want a career in business after all. I’m not liking these business classes I’ve been taking very much. The other students are fine, but I’m not really like them. I think I might like teaching better—kids have always been fun for me.

**INEFFECTIVE THERAPIST:** Well, maybe education would be a better major for you but, gosh, that job market sure is tight right now, and it hasn’t ever paid as well as it should.

**CLIENT:** (*sinking*) Yeah, I wasn’t thinking about that. I guess I need to be more practical. Maybe teaching wouldn’t work very well for me after all...

The counselor’s reservations would be insignificant to many clients, and even helpful to some, but they are highly problematic for this particular client. In this reenactment, for example, the client’s own choices and interests were disapproved of, which then exacerbated her presenting symptom of depression. In contrast, a helpful, counteractive response, in which the therapist would pass this test—and empower this cautious and compliant client—would involve *expressing interest in the client’s own interests and supporting the client’s initiative whenever it emerged.* The therapist can still express realistic concerns and potential problems, but not until unambiguous support for her own interests and choices have been affirmed and recognized. For example:

**CLIENT:** I don’t think I want a career in business after all. I’m not liking these business classes I’ve been taking very much. The other students are fine, but I’m not really like them. I think I might like teaching better—kids have always been fun for me.

**EFFECTIVE THERAPIST:** Kids are fun for you—I’m seeing that. I’d love to hear about your interest in teaching. Tell me more.

**CLIENT:** Well, I don’t think my parents would like to see me be “just a teacher.” I was a National Merit Scholar, you know. But I think I’d love having my own classroom, and I could have a more balanced life with summers off—you know, so I could have time to raise my own family.

**EFFECTIVE THERAPIST:** You sound excited about this, like it would give you the kind of lifestyle that you choose. You know, you really do sound different right now...your voice is more alive, has more feeling, as you talk about this possibility.

Testing behavior such as this pervades all counseling relationships—from the first session to the last. However, testing is especially likely to occur in the initial sessions, in situations where strong feelings are being evoked, and when the current interaction between the therapist and client parallels relational problems that the client is having with others. Testing will also be more persistent and pervasive when clients are struggling with more serious and painful problems, such as trauma and abuse. And this makes sense when we consider this in their broader life context. Too often, for example, a client's experience goes like this: The client's mother did not believe her as a child when she said her stepfather was "touching" her; in high school her sister (who suffered the same violation) said she "didn't want to talk about this"; her best friend at college anxiously said, "I'm sorry" and immediately changed the topic; and when she vaguely alluded to childhood molestation with her previous therapist, he suggested they remain focused on her presenting symptom of anxiety attacks. *If this client does decide to return to therapy with another therapist at some point, she will appropriately be testing earnestly in the initial phone contact and first session to discern if this therapist, too, does or does not want to hear about what happened to her—so she will not be painfully invalidated again.* Testing is about assessing safety/danger in the interpersonal environment.

Let's consider another example of testing. Once therapists have the concept of testing, many of these tests will be easy to recognize and pass, allowing clients to progress in treatment and make changes in their lives. Suppose a female client, who often has been acting in a passive and dependent way with others in her life, says to her male therapist, "Where should we start today?" Even though he has the best of intentions, the therapist will fail this client's test if he takes the bait and says, "Tell me about..." By telling this client where to begin, even though she has just asked for his direction, the therapist has *confirmed this client's pathogenic belief that she needs to take a subservient role with others and follow or comply with what they want.* The problematic belief that she ought not to assert herself, initiate, or take the lead is confirmed when the therapist tells this client what to talk about (recall that, per client response specificity, this same directive could be helpful to a different client). This client is likely to remain unassertive and dependent in her relationship with the therapist and, as a result, *she will be unable to change this with others in her life.* In contrast, the therapist is likely to pass this test and disconfirm this faulty belief if he repeatedly looks for ways to encourage her own voice and actively supports her initiative whenever she does take the risk of speaking up on her own behalf (which therapists can recognize in the moment if they have conceptualized that this client's problem is about compliance). Thus, when this client tests and compliantly asks him what to talk about or where to begin, the therapist can provide an experience of change by responding as follows:

- *Well, maybe you could just sit quietly for a moment or two, think about the issues or concerns that feel most important to you right now, and bring me in on those.*
- *What would you like us to begin with today?*
- *OK, I do have some possibilities, but maybe I could hear your thoughts first about the best way to use our time today.*
- *I'd like to talk about whatever you think would be most helpful. What might that be for you today—what comes to mind?*

Many compliant clients will not accept these invitations so easily, of course. Often, they will not accept the therapist's first invitation but continue testing to see whether the therapist really means it when he or she shows interest in clients' choices and what seems most important to them:

**CLIENT:** (*vaguely*) Oh, I'm not sure, I can't really think of anything. What do you suggest?

Trying again to change the old relational pattern in their current interaction and engage the client in a new, more egalitarian way, the therapist might pass the test by giving a second bid for the client to take the lead:

**THERAPIST:** (*comfortably and patiently*) OK, let's just wait a minute then. Why don't you take a moment to check in and just be with yourself, and let's see what comes up for you.

**CLIENT:** (*pause*) Well, uh, OK...I guess I haven't been sleeping very well lately. I keep having this bad dream.

**THERAPIST:** The same bad dream keeps coming back—I'm sorry, that's no fun. Let me join you there—tell me about the dream.

With each of these responses, the therapist's intention is to demonstrate that he or she genuinely wants to engage the client in a more collaborative way. Behaviorally, the therapist is demonstrating to the client that he or she does not share the client's belief that she needs to remain dependent or submissive and be told what to do. The therapist is also discerning more clearly as a treatment focus that, too often in other relationships, this is how things went. Repeated interactions of this sort—which consistently disconfirm the client's symptom-engendering belief that she isn't capable of acting independently, that others need her to depend on them or be submissive, or that she doesn't have a right to say what she wants and express her own needs—will be reparative and bolster her self-efficacy and autonomy. As she continues to find it safe to act stronger with the therapist, she can begin to generalize and try this new way of relating with a few carefully selected (safe/affirming) others in her life as well. She and the therapist will begin to sort through and identify others in her life with whom it is safe to act more assertively and independently, and others who undermine her confidence, try to control her, and do in fact want her to remain one-down or to act dependent.

Usually, it is relatively easy for new therapists to identify this type of testing around dependency/compliance issues and respond effectively. In contrast, tests that involve issues of power and control usually are far more challenging for new and experienced therapists alike. Clients with different developmental experiences—for example, those who could “run over” their caregiver, or who wielded too much power and influence in family matters, often test the resolve of the therapist. When these confrontational or moving-against clients can successfully dominate, demean, or control the therapist, things go downhill for therapist and client alike. Before these clients feel safe disclosing personal or vulnerable issues, or even begin to engage in a working alliance, they also need to assess safety/danger in the therapeutic relationship. For these clients, this often means testing whether the therapist can manage their demands and maintain boundaries without becoming angry/punitive or withdrawing. For example, when clients are insulting, provocative, manipulate or push limits with the therapist, as they could their permissive caregiver, they are often testing. Let's consider a case example of this more challenging, provocative type of testing.

*Graduate students in Carl's training program were supposed to videotape all therapy sessions. Early in treatment, however, Carl's client Lucy complained that the videotaping made her “self-conscious.” She said it was already hard for her to talk about what was going on in her life and that the videotaping just made it “impossible” to talk about really important things. With each successive week, Lucy voiced her increasing exasperation with Carl and disappointment in treatment. She told Carl that he was not helping her and threatened to stop treatment if he didn't turn off the video recorder.*

*Carl was torn and truly did not know what to do. He knew the clinic rules and that his supervisor would want him to abide by them. Yet he also felt he should be flexible enough to do what his client needed, and he didn't want her to stop coming and “lose another*

*client.” Without consulting his supervisor, he reached a compromise with Lucy and agreed to turn the videotape off for the last five minutes of each session. Lucy was appreciative and did indeed disclose new material to Carl. Carl felt badly about doing this without his supervisor’s permission, but it also seemed like it was the only way to help his client and keep her in treatment (which was Carl’s need, not Lucy’s).*

*Soon afterward, however, Carl realized that treatment was not going well. Even though she brought up some relevant new issues, Lucy became scattered and started jumping from topic to topic. Carl tried to pin her down and focus on one thing at a time—such as her ever-changing feelings or scattered thoughts, but she slipped away to a different issue each time. Lucy’s distress escalated and she communicated that she was “very upset” and needed more guidance from Carl, yet whatever he did failed to help. Realizing that he had made a mistake and things were getting worse with his client rather than better, Carl knew he had to face his anxiety and talk with his supervisor.*

First, Carl and his supervisor spent some time discussing honestly what Carl’s decision to turn off the tape meant for their relationship and their ability to work collaboratively. With their relationship restored, Carl and his supervisor began exploring what had been going on between Lucy and Carl, and tried to understand what this interaction pattern might mean. To begin, they clarified how badly Carl had always felt about himself when others were critical of his performance. Carl relayed that he just “dreaded” to be in situations where, no matter what he did, he “couldn’t do it right.” Further clarifying their mutual interaction, the supervisor helped Carl recognize how this countertransference issue in his own life was activated by the critical and demanding stance that Lucy took with Carl—and with others in her life. Carl felt empowered by these new ways of thinking about what was going on in the client-therapist relationship and accepted his supervisor’s firm limit that the videotape must remain on throughout every session. To his dismay, however, Lucy did indeed feel angry and betrayed when Carl reinstated this limit at the beginning of the next session. She accused Carl of being “untrustworthy” because he broke their agreement, told him that she didn’t think he was going to become a very good therapist, and informed him that she could no longer talk with him about certain important topics. All of this was difficult for Carl, but the biggest threat came when she told him that she would have to think about whether she wanted to come back next week and keep working with him.

Carl felt “horrible” about her threats and accusations, yet with his supervisor’s support, he tolerated his discomfort, tried to remain as non-defensive as he could, and stuck to the rules this time. Although she continued to blame, threaten, and complain, Lucy did return. In fact, despite her protests, she actually seemed calmer than before and began to focus more productively in therapy. Over the next few weeks, Lucy succeeded in making some important changes in her life. She began to share with Carl how “sickening” it had felt for her to be able to “boss my father around—just like my mother did.” Following this disclosure, which Lucy had felt so ashamed about, she returned to the next session and said that she had apologized to her boyfriend for being “so demanding.” Lucy told her boyfriend that she “didn’t want to be this way anymore—with him or anybody else,” and was trying to change it.

It is adaptive—even vital—for clients like Lucy to test whether the therapist will confirm or disconfirm their pathogenic beliefs. Clients will make progress toward more vulnerable material, or make behavioral changes in their lives, if the current therapeutic relationship is safer than past relationships have been. However, there is limited leverage in merely reassuring clients verbally about deeply held beliefs and expectations. Moreover, most clients are not consciously aware of testing therapists in these ways, and usually it is not helpful to point out testing behavior to them. Instead, as in all human relations, *it is what the therapist does that counts, rather than what the therapist says.*

Therapists pass tests and provide a CEE when they repeatedly respond in ways that behaviorally disconfirm pathogenic beliefs and alter maladaptive relational patterns. Fortunately, even though all therapists fail tests sometimes, they will have many opportunities to recover—just as Carl did. *It is especially important for new therapists to be reassured that they can readily recover from mistakes, failed tests, and reenactments.* They can acknowledge what went wrong and talk it through with the client, or simply respond more effectively the next time this issue or theme comes up—often later in the same session or in the next session. In particular, therapists can recover from these failed tests that, like ruptures, regularly occur for even the most effective and experienced therapists if they:

- *Stay closely attuned for any relational statements—whether overt or subtly embedded, that clients make about the therapist or their interaction together (Client: “You’re probably not the kind of person who would approve of this, but...”).*
- *In each successive interaction, track how the client responds to what the therapist has just said or done (Therapist: “I just asked you to clarify that for me, and you became quiet. I wonder what went on right there for us—maybe something didn’t feel quite right?”).*
- *When therapists are thinking to themselves that, right now, something just isn’t going right, check in and ask the client (Therapist: “Can I check in with you for a minute about how we are doing together here—how things between us are feeling for you as we talk about this?”).*

The therapist passes a client’s test and provides a reparative experience by responding in the way the client needs. The client experiences a new degree of safety in the therapeutic relationship (such as providing limits for Lucy, who held too much power over adults while growing up and did not experience appropriate limits and boundaries). Therapists will often observe clients respond to this new safety by beginning to remember more clearly, feel more deeply, and share more fully. In the next few sessions, therapists will also observe clients taking risks, as Lucy did, and try out new and better ways of responding with others in their lives. Most clients will initiate these changes on their own, saying, for example:

**CLIENT:** Hey, guess what I did this week? I finally spoke up and told my boyfriend...

At this point, the therapist can support the client’s willingness to risk engaging with others in this new and self-affirming way. This then leads to further discussion about *discerning who in the client’s life is capable of responding affirmingly to the changes the client has made and who is not.* Thus, the client can safely process with the therapist how to generalize the new response style and work at consolidating a new set of beliefs about him or herself and others. For instance:

**THERAPIST:** You just did this differently with me—how very nice. You know, that makes me wonder if you couldn’t speak up for yourself better and say more of what you want with your husband, too. What would it be like for you to try this with him, like you just did with me? What might the plusses and minuses be in doing that?

### Helping Therapists Provide Reparative Responses that Pass Clients’ Tests.

How can therapists clarify the interpersonal response that each particular client needs in order to pass a test and provide a reparative experience? To guide their responses, therapists can (1) consider client response specificity and (2) track the moment-to-moment interaction sequences and assess the client’s positive or negative reactions to what the therapist has just said or done. Let’s consider both carefully.



**Client Response Specificity.** As discussed in Chapter 1, therapists need the *flexibility* to be able to respond differently to different clients (see Norcross & Wampold, 2011). In counseling relationships, there are no cookbook formulas for the “right” way to intervene—one size does not fit all. The same therapeutic response that passes a test for one client may reenact problems for another client with different IWMs and expectations (Persons & Silberschatz, 1998). For example, in response to pressure from her client, Nico agreed to reduce her fee substantially. At their next session, however, Nico noticed that her client was wearing an expensive watch, and mentioned that he and his wife were “converting reward points” to fly first-class on their upcoming vacation to Hawaii. Realizing that the client was taking advantage of her, Nico recognized the need to make this treatment issue overt and renegotiate a new fee. This client had been indulged by doting parents and his wife, and now was testing to see if the therapist would just “go along” with him as well. A point of entry soon emerged and Nico was relieved that this potentially challenging conversation went so easily—it was almost as if the client felt reassured by her forthright discussion and limits. Later in the session, the client brought up important new material about feeling both “alone” and “anxious” when he felt he had too much control over others. Working together, they entered a productive new phase of treatment where the client began to explore how this pattern of being demanding or taking advantage was being played out and disrupting his other relationships with people—who also caught on eventually and stopped trusting or liking him.

Based on the client’s willingness to begin looking at these unwanted parts of himself, Nico concluded that by being firm and not allowing the client to take advantage, she had passed the client’s test and provided him with the corrective response he needed. This client was unaware of his own testing behavior with Nico, however, and unable to articulate it in any way. Like most clients, he did not feel safe enough to recognize or discuss this sensitive issue until he was *behaviorally convinced* that he could not manipulate Nico as he could others in his life.

In contrast, the same response of lowering the fee can be helpful for a different client. Chris was coping with a diminished income following a recent divorce and was having trouble making ends meet. Knowing that she grew up with demanding, authoritarian parents who gave her little, the therapist realized that offering Chris a more flexible fee arrangement could be a significant overture. This client would never have asked the therapist for a reduced fee (or asked anyone for help with anything) and was initially unwilling to accept any of the alternative fee arrangements that the therapist suggested. When the therapist asked about her reluctance to even consider his offer, Chris began to cry. As their discussion soon revealed, the therapist’s generous response disconfirmed her pathogenic belief that she “didn’t matter” and evoked the longing she had felt for her parents to “see her” and help when she had a problem. Chris had grown up under the demanding expectations that she had to do everything “perfectly” and should never need any help or support. That was simply the family norm and was what was expected from her. Clearly, changing the payment schedule with this client disconfirmed pathogenic beliefs about herself and faulty expectations about others. Within this session, treatment progressed into more intensive work about her emotional deprivation and resulting belief that she “did not matter much” to others. As these two examples illustrate, *the same response by the therapist will often have very different effects on different clients* (Daly & Mallinckrodt, 2009; Silberschatz, 2005).

Clients’ cultural/ethnic backgrounds add complexity to response specificity. For example, self-disclosure of an issue that the therapist had great difficulty resolving in his own life could be viewed as unprofessional by some traditional Asian clients. However, the same disclosure with some African-American clients may be welcome and convey that the therapist is human and less likely to be judgmental. In this way, racial, gender, religious,

or other differences between therapist and client will affect how the client responds to the therapist's self-disclosure. Following our previous guidelines, the therapist's intention is to pay close attention to how the client responds to this intervention. For example, if the therapist decides that self-disclosure may be productive for a particular client, *the therapist will closely observe how the client reacts to it*—with a helpful new perspective or more involvement in the working alliance, for example, or with greater reserve and emotional distance from the therapist.

Client response specificity puts more demands on the therapist. It takes away the security of having a simple, rule-based approach to therapy. Manualized therapy approaches have been useful in terms of maintaining adherence to the treatment approach—especially in research studies, which was their original purpose. However, manualized treatments fail to honor client response specificity unless they specifically allow for variation based on client needs. The Motivational Interviewing literature has richly informed our understanding of how manualized approaches can be limiting when the client's expressed need or initiative and lead is not taken fully into account and there is a press to strictly follow the manual. These MI researchers report that therapists in their treatment approach were *more likely to bring about effective and lasting change when therapists applied the manual flexibly and tracked the client's response to interventions—attending and adjusting to how the client has responded to what the therapist just tried to do* (see Amrhein et al., 2003; Levant, 2005; Miller & Rose, 2013). Client response specificity requires the therapist to remain close to the unique and personal experience of each individual client. However, within this highly idiographic approach, guidelines are available to help therapists discern more accurately the specific needs of differing clients. In particular, the therapist can learn to assess the specific relational experiences that each individual client needs by identifying the maladaptive interpersonal patterns that tend to recur in three arenas:

1. current client-therapist interactions,
2. relationships with important others in the client's current life, and
3. the client's family of origin, where these IWMs and patterns were originally engendered.

We have begun to see that therapists learn how to pass tests and provide clients with the specific relational experiences they need by assessing how clients react to different interventions from the therapist. Let's go further with it—learning to do this is empowering for new therapists.

**Assessing Clients' Reactions.** How do therapists know whether they have passed or failed the client's test, or determine in general whether they have responded usefully to what the client just said or did? In this section, we provide guidelines to help new therapists assess whether their interventions or responses have been helpful or potentially reenacted some aspect of the client's conflict.

New therapists often have the misconception that there is a single correct way to intervene with clients, and they feel worried or insecure that they haven't done it right. A better approach is to observe how the client reacts to the therapist's response. In this flexible, assessment-oriented approach, the client is determining the best way to intervene—based on how well the client can use what the therapist just said or did to make progress in treatment. With each successive turn of the conversation, therapists are trying to decide what is the best way to respond to this client right now—and choosing to respond by empathizing, challenging, exploring, affirming, reframing, making links, providing feedback, and so forth.

As soon as they respond, however, the therapist's intention is to *attend closely and observe how the client seems to be responding to this intervention*. As the therapist tracks

the moment-to-moment interaction sequences—how the client is responding right now to what the therapist just said or did—clients inform the observing therapist of the responses they can utilize or benefit from. Clients do not usually verbalize this outcome and may not be conscious of it, but *they reliably guide the therapist by their behavioral response to how the therapist just intervened*. Let's look more closely at how this goes.

When the therapist responds effectively to a client's test and disconfirms pathogenic beliefs about self and faulty expectations of others, the client often feels safer and begins to behave in new and more adaptive ways. This progress can be expressed in many different ways. For example, in the minutes after the therapist passes a test, clients may stop complaining about other people and begin talking more directly about their own thoughts and feelings. Or, they may look more honestly at their own contribution to the problem, as Nico's client just demonstrated. Perhaps the client will relate a vignette with more clarity than usual. The interaction with the therapist may acquire more immediacy or intensity and the dialogue becomes less intellectualized or superficially social. Clients will often engage the therapist more directly or be more present, perhaps by expressing warm feelings toward the therapist or by taking the risk to bring up a problem they are having with the therapist. Often, the client's affect will emerge or intensify. Therapists want to take note when clients act stronger in any of these ways. It is a pleasure to see the client is improving, but the client is also teaching therapists what they have just done, and what they can do in the future, to make a difference.

Therapists also learn that they have responded effectively or passed tests when clients bring up relevant new material to explore. When clients initiate new topics in this way, they tend to become especially productive. In particular, clients may make useful connections, on their own, between their current behavior and formative relationships—without the therapist needing to make interpretations or guide the client back to developmental or familial relationships. The insights that clients generate in these circumstances are enlivening and hold real meaning, and they are readily translated into new ways of thinking and acting in their everyday lives. In contrast, *it is not usually very productive for therapists to make historical interpretations or to lead clients back to try and make these familial connections*. Even when these developmental links are accurate, clients do not usually find them to be relevant to their current problems—just abstract ideas or distant possibilities that do not hold real meaning or lead to behavior change. For example, a client might say with exasperation, “My mother? Why are you asking about my mother? I'm 31 years old—what does my mother have to do with anything?!”.

Therapists are usually reassured they have responded effectively or passed the client's test when the client responds by acting stronger in one of these ways. To illustrate, when Carl turned the videotape on for the full session, Lucy complained and even threatened to terminate. But by maintaining these appropriate professional boundaries, Carl behaviorally demonstrated that he was different from Lucy's father and was therefore safer. Carl could tolerate Lucy's disapproval and do what she needed rather than what she demanded. Although Lucy threatened and complained, she also began to work more productively with Carl and began changing with others in her life this way of relating that wasn't working.

If therapists observe how clients respond to their interventions and try to identify what they have just done to pass (or fail) clients' tests, they also will be able to *formulate treatment plans more effectively*. These conceptualizations can then be used to shape subsequent interventions and better provide clients with the responses they need in order to change. For example, at other anxiety-arousing points in treatment, Lucy will again push the limits with Carl. If Carl has learned from their previous interaction, however, he will be better prepared to provide Lucy with the clear limits she needs and is covertly requesting.

What about the other side of the coin? How do therapists know when they have failed the client's test or unwittingly reenacted the client's conflict in some way? In this situation, the therapist will often observe that clients act weaker in some way—such as, becoming *more* compliant, defensive, confused, externalizing, distant, self-doubting, intrapunitive, and so on. Clients may also backtrack from healthy new behavior by undoing or retreating from recent successes, or become less of a partner in the working alliance. If therapists are closely assessing how the client is responding to what they have just said, they will observe that these reactions to failed tests are usually evident within the next minute or two, and may continue into subsequent sessions. By tracking their interpersonal process in this moment-to-moment way, therapists are much better able to recognize when their current interaction may have just failed a client's test. In particular, therapists should consider the possibility that they have unwittingly reenacted some aspect of their clients' conflicts when:

- *clients begin talking about others rather than themselves and the dialogue becomes more superficial;*
- *the therapeutic process becomes repetitive, intellectual, or loses any real focus; or*
- *clients become passive or compliant, lose their initiative, or cannot find meaningful material to discuss.*

We have just seen that when therapists pass tests, clients usually acknowledge the therapist's effectiveness by acting stronger and making progress in treatment. In parallel, when a therapist repeatedly fails a specific test, *the client may inform the therapist that something is awry by recalling other relationships in which the same problematic scenario was being enacted.* That is, the client may begin to tell the therapist vignettes about other people the client has known, or perhaps characters in books or movies, who are enacting the same problematic pattern. *Therapists always want to consider the possibility that, whenever the client is talking about another relationship, the client may be using this as a metaphor, analogue, or encoded reference to describe what is going on between the therapist and the client as well* (Kahn, 1997). Throughout each session, therapists are listening for the relational themes that characterize the vignettes clients choose to relay. For example, if clients repetitively share stories in which trust is betrayed, control battles are being enacted, and others are not responsible or need to be taken care of, they are often expressing how these same patterns are currently being activated or played out with the therapist. Yalom (2003) describes this as listening with “rabbit ears”—always considering the possibility that *whenever clients are talking about another person they may be making a covert reference to the therapist or what is going on between the therapist and client as well.* When this occurs, the therapist can use a process comment to inquire about this possibility, make this relational theme overt, and bring it back into the immediacy of the therapeutic relationship—where something can be done to address and resolve the misunderstanding or repair the rupture in the working alliance. For example:

**THERAPIST:** Both of these people you've just been telling me about failed to hear something important that others were trying to tell them. I'm wondering if something like that could be going on between you and me as well?

**CLIENT:** I'm not sure what you mean?

**THERAPIST:** I'm wondering about the possibility that, like with these other two people, you might have told me something earlier that was important to you, but maybe I didn't hear it very well. Anything come to mind?

**CLIENT:** Well, since you mention it, sort of. I guess sometimes I think you do try to make things seem better than they really are. Like you need me to be happier than I really am, or want to make my problems less than what they really are.

**THERAPIST:** It seems like I'm trying to make things better than they really are. Like I don't want to see how bad it really is for you sometimes? I can sure see why you wouldn't like that, and it's helpful that you're speaking up and talking with me about it. I want to understand this better so I can change it. Can you give me an example—did I do that today?

**CLIENT:** Uh-huh. I told you I was “terrified” when I woke up and they were gone. And you said, “Yeah, I can see how anxious you felt.” Maybe you prefer that I only feel anxious, but I didn't. I felt terrified—that's what I said and that's what I meant, but I don't think that's what you wanted me to feel.

**THERAPIST:** Got it—thank you, I see what you mean now. I don't want to do this and am going to be mindful to change it. If you ever feel I'm still doing it, would you please speak up again, right as it's happening? I don't want this to continue.

**CLIENT:** Yes, I will, and thank you for hearing me. *(pause)* You know, I think I often have the sense that my feelings are “too much” for other people...

Supervisors and theories certainly are useful, but new therapists develop more autonomy and better find their own identity as therapists by learning how to assess client responses and determine for themselves the effectiveness of their interventions. By tracking the therapeutic process in the ways suggested here, therapists become able to assess for themselves whether they are providing clients with the reparative responses they need. However, all of this can seem like too much for a new therapist to be tracking all at once. Although it may feel overwhelming to try and think about so many things while simultaneously interacting and trying to be present with the client, most of this does become second nature with practice and several years of experience. However, as therapists incrementally learn to track and assess client reactions in this way, they will be able to choose more confidently or know more easily how they want to respond next to what the client just said—and that's a very good feeling to have.

### III. TRANSFERENCE REACTIONS

The third way clients bring their problems with others into the therapeutic relationship is through transference reactions. **Transference** means inaccurately displacing or projecting various positive and negative feelings and attitudes toward the therapist that clients have held toward important people in their past (Gelso & Bhatia, 2012; Gelso, Palma & Bhatia, 2013). That is, these distortions have been overgeneralized or *transferred* to the therapist—casting the therapist *inaccurately* into a role that better fits important persons from clients' pasts. It is during vulnerable or distressing, affect-laden moments that clients are most likely to misperceive the therapist and others along old familiar lines. These persistent distortions of the therapist (and others) reflect clients' schemas or IWMs. *The more troubled clients are, the more pervasive their transference distortions will be*—causing significant problems in their close personal relationships and distorting their reactions to the therapist as well (Levy & Scalia, 2012).

As new therapists gain more clinical experience, their worldview often changes as they experience distressed clients having strong reactions to them that do not fit very well with who they are or how they have responded. However, therapists error when they automatically or too readily relegate clients' positive or negative feelings toward them merely to transference distortions. We can make that possibility a working hypothesis, but therapists want to be non-defensive enough to be able to *consider* the potential validity or partial accuracy of those reactions as well. Maybe a client does have a valid complaint and something we have said or done has contributed to the client's (seeming) misperception or misunderstanding. In this egalitarian approach to transference the therapist is never a completely neutral or objective observer but, instead, always considers



the possibility that both client and therapist may be contributing their own biases (Hill & Knox, 2009). In this regard, attachment researchers find that therapists who themselves have Secure attachment styles are better able than insecure therapists (that is, Dismissing or Preoccupied) to remain non-defensive, have the reflective capacity to consider the client's differing point of view, and do not readily provide the automatic responses that the client characteristically tends to elicit or "pull" from others (Dozier et al., 1994; Dozier & Tyrell, 1998; Fonagy et al., 2002).

Therapists can use transference reactions both to conceptualize their clients and to help resolve problems within the therapeutic relationship. An important change has occurred in how therapists work with transference reactions. In decades past, transference reactions were used as a springboard to make interpretations about historical relationships and the genesis of conflicts with parents and early caregivers. Now they are used instead to *understand and change the current interaction between the therapist and the client* (Levenson, 1995). As noted earlier, therapists will often recognize important connections between current problems and developmental relationships, *yet few clients will find these links meaningful without first addressing and resolving these distortions in the real-life relationship with the therapist*. In this more contemporary approach, the therapist is working with transference in the immediacy of the current relationship, rather than through the more distant or intellectualized approach of trying to interpret or lead the client back to historical relationships. For example,

**EFFECTIVE THERAPIST:** What might I be thinking about or feeling toward you as you tell me this?

#### VERSUS

**INEFFECTIVE THERAPIST:** So, you're seeing me as being withholding here...could that perception fit your father better than me?

Researchers find strong empirical support that this here-and-now approach is linked to better treatment outcomes (Andrusyna et al., 2006; Levy et al., 2006). In this contemporary approach to transference, the goal is to:

1. help clients feel safe to explore and discuss personal reactions toward the therapist;
2. work collaboratively to explore mutual contributions to problematic interactions or misperceptions in their relationship; and
3. work together to change interactions that are problematic or resolve misperceptions with the therapist first, and only then explore how these also might play out with others in their lives.

We want to emphasize further that clients' reactions toward the therapist *may* accurately capture the interaction with the therapist—or aspects of it. It is essential for the therapist to acknowledge and consider the potential validity of clients' perceptions. Indeed, without realizing it, we may well have said or done something that contributes to the misperception, misunderstanding, or rupture. Therapists want to be non-defensive and enter into a genuine dialogue with clients to clarify what each was thinking, feeling, and intending during any misunderstanding or rupture of the working alliance. Addressing and resolving differing perceptions and ruptures in this way, as soon as they emerge, is associated with treatment retention and treatment outcome (Muran et al., 2009). Clients feel respected and empowered in this egalitarian relationship, and they often are able to become more authentic with the therapist and, in turn, with others in their lives.

In this way, the therapist is receptive to the potential validity of client reactions and tries to sort through and resolve disagreements or distortions in the current relationship, rather than focusing back on the historical genesis of conflicts. Ironically, just after the therapist and client have addressed and resolved a significant misperception or

misunderstanding between them, clients will often go on to make meaningful connections between formative and current relationships *on their own*. Let's look further at how therapists can intervene with transference to help clients change.

**Transference in Everyday Life.** Transference reactions are commonplace—they simply pervade human relationships. To a greater or lesser degree, they occur for all people at times—including therapists. We all systematically misperceive others as a result of overgeneralizing previous learning to the present situation—*especially when we feel vulnerable or distressed*.

Transference reactions are also likely in other types of emotion-laden situations. For example, transference reactions are especially pronounced when two people are falling in love. Many married couples recall their courtship as the happiest time in their lives. During this time, each member may systematically misperceive the partner as someone who can fulfill unmet developmental needs—most commonly, perhaps, making one feel love-worthy or wanted when one has not felt this way before. This romantic period is usually short-lived for two reasons, however. First, the transference projections break down—they both realize that the other person cannot fulfill their unmet developmental needs, and their old schemas are reinstated as they face this disappointment. If the unmet attachment needs and accompanying transference distortions are strong, the result is often dashed dreams or angry feelings of betrayal. In contrast, if the transference distortions of the partner have not been extreme, they can readily be shed, and a more realistic and enduring relationship can develop. Relinquishing these transference distortions of the new spouse is one of the major psychological tasks of courtship and early marriage.

Second, individuals with Insecure attachment styles often will make poorer relationship choices because *they will not “see” problems, realistic limitations, or red flags in their exciting new partner*. When this occurs, the partner presents problematic aspects of the relationship with attachment figures that often *cannot be registered* until the client has achieved earned security. These individuals need to develop a new and more realistic narrative for their lives where they can accept the good and bad news of their attachment figures, which for many means grieving the loss, or feeling the sadness, over what one once wanted but did not receive.

Perhaps the primary arena in life for transference distortions is in parenting. One type occurs when a parent's unresolved conflict with his or her own parent is projected onto a child who (often selected by birth order, gender, or appearance), most reminds the parent of the grandparent who was most hurtful. Alternatively, as in the heartbreaking vignette that follows, a child “reminds” a parent of the child's father from whom she is now estranged and, in fact, has come to detest.

*Ten-year-old Adam was brought into treatment at the request of his teacher, who reported that Adam was “sullen and withdrawn” and often spent his class time drawing pictures of angry/muscular men, swords and other weapons, and blood. The teacher became increasingly concerned when he began getting into fights with others at school, and made the referral when she noticed that he had cuts on his arms. In evaluating Adam, the therapist ascertained that Adam's mother had become increasingly negative and hostile toward him in the past year (for example, setting up his bedroom out in the garage). She had been telling Adam that she “hated” his worthless father and that Adam was “just like his father” and that “the apple doesn't fall far from the tree.” Adam reported feeling acutely suicidal and had to be hospitalized (where he did, in fact, make an attempt—trying to electrocute himself by inserting a wire paper clip he had been using to “scrape himself” into an electrical outlet). Adam's troubled mother argued against any need for treatment and had been skeptical about his need for hospitalization. She confirmed Adam's belief that “my mother hates me” when she said to the therapist in front of Adam: “Are you sure he isn't just trying to manipulate you—just trying to get attention like his father always did?”*

In treatment, the therapist can systematically utilize the client's transference reactions to better understand the client's problems with others. One of the most effective ways to discern the client's IWMs and faulty expectations and make transference distortions accessible for discussion in treatment is to *check in regularly and ask how the client might be perceiving or reacting to the therapist or what is going on between them right now*. Routinely, aspects of the same patterns and themes that cause problems with others will be activated toward the therapist or played out in the therapeutic relationship (Connolly et al., 2000). If the therapist tracks these transference reactions and successfully engages the client in a dialogue about them, they clarify faulty schemas to the client and help change the problematic relational patterns that are being reenacted with the therapist. As this important relearning occurs with the therapist, the client is empowered to begin changing this pattern with others as well. Let look more closely at how transference helps therapists discern the problem and clarify the treatment focus.

**Utilizing Transference Reactions to Conceptualize Clients.** Attachment histories provide one useful lens for understanding transference reactions and helping us conceptualize our clients. As Bowlby noted (1988), repetitive interactions with early caregivers shape children's expectations, and subsequent transference distortions of, therapists and others. Researchers have supported this, showing that the attachment patterns individuals develop shape both transference reactions for clients and countertransference reactions for therapists (Dozier & Tyrrell, 1998; Mallinckrodt et al., 1995; Martin et al., 2007; Woodhouse et al., 2003). For example, researchers find that Preoccupied clients tend to misperceive the therapist as being unreliable in meeting, or overwhelmed by, their emotional needs. At the same time, problematically, Preoccupied clients are unlikely to address this or other problems with the therapist *for fear of offending and potentially losing the relationship with the therapist*. Thus it becomes especially important that therapists ask these clients directly about their reactions to the therapist and explore their perceptions of the therapist. This is one way to help these clients find that this is a safe or different relationship that challenges their IWMs because, unlike significant others in the past, the therapist is making it unambiguously clear that he or she is not overwhelmed by them—or too caught up in his or her own emotional problems or distress to be able to see and respond to clients in a consistent and dependable way.

Transference distortions for Dismissive clients tend to follow themes that the therapist will see them as being weak, or will be intrusive and controlling, if they are vulnerable or share deeply with the therapist. Dismissive clients also will tend to distort the therapist and others in their lives as being hostile and competitive—when actually it is usually they who are responding this way and causing problems with others by trying to dominate and take control (Mallinckrodt, Daly, & Wang, 2009; Smith et al., 2010).

Therapists may not see clients' transference reactions, and the valuable case formulation/understanding they provide, *unless they actively intervene to draw them out*. Let's look further at how the therapist can highlight or identify clients' transference reactions. Below, we present several ways of making clients' transference distortions overt so clients can collaborate with the therapist to understand and change them.

Primarily, the therapist is looking for tactful but direct ways to inquire about clients' feelings and reactions toward the therapist. As we saw in Chapter 3 when discussing resistance, it is important for both therapists and clients to learn about clients' reactions to being a client and seeking help. Further along in treatment, it is also important to learn about clients' perceptions and reactions toward the therapist. This is especially important when:

- *conflicts or misunderstandings arise between therapists and clients, and*
- *clients have just shared strong emotions or self-disclosed.*

It is during these two types of sensitive interactions that *clients are most likely to misperceive therapists as responding in the same problematic or unwanted way that others have in the past*. As we have just seen, for example, if clients disclose something they feel embarrassed about or enter a strong feeling, they will often be certain (erroneously) that the therapist feels burdened or depressed by their sad feelings. Or, they may inaccurately believe that the therapist doesn't respect them anymore now that they have disclosed this information about themselves. Clients believe the therapist is privately responding in this unwanted way because this is how important others have responded in the past—even *though the therapist doesn't think or feel anything like this and has not communicated disapproval in any way*.

It is difficult for most graduate student therapists to grasp this in the beginning. Because the interaction or sharing seemed so positive to them, and to be so well-received by the client, new therapists find it hard to believe that it could have been misperceived in some problematic way by the client. However, if the therapist doesn't check in with client and ask how it has been for them to share this significant feeling or disclose this important issue with the therapist, some clients will miss, reschedule, or come late to the next session, or return but keep the next session superficial. Thus therapists need to ask about or clarify potential transference distortions with the client and, when necessary, *make it unambiguously clear that they are not thinking or feeling the same unwanted things that important others have in the past*. For example:

**THERAPIST:** That was a lot to share—there's so much sadness there.

**CLIENT:** Yeah, I guess I was crying pretty hard.

**THERAPIST:** Yes, and I'm honored that you choose to share such important feelings with me.

May I check in with you for a minute about this—about how it feels right now to have shared so much and cried so hard with me?

**CLIENT:** Well, you're always very nice, but this has to be pretty tiring for you, too.

**THERAPIST:** Tiring? No, it's not tiring at all. Actually, it's enlivening for me—I feel connected to you right now—that we've shared something that's meaningful for both of us.

**CLIENT:** Really? I don't wear you out with all this?

**THERAPIST:** No, I don't feel worn out or tired in any way. Actually, it's just the opposite; I like being able to respond to your genuine experience.

**CLIENT:** Boy, this is really different for me, that it's OK for you if I feel sad or cry sometimes. I think my Mom needed me to be happy all the time, so I acted that way, even though I wasn't. When looking at pictures of myself, other people often say I look pretty or have a nice smile, but I see a sad face...

Therapists want to **debrief** with clients after sensitive interactions, and check whether clients are distorting or accurately perceiving the therapist. If transference distortions are in play,, therapists want to differentiate themselves from significant others and clients' IWMs. For example,

**THERAPIST:** Maybe you're telling me that others in your life have felt burdened by you, or that your needs made people "tired," but I think I'm different from some important people in your life. I'm not feeling "tired" or burdened or anything like that at all.

At other less significant times as well, therapists will want to check in with clients and explore their reactions toward the therapist. Routinely, the following type of

questions will open some of the most important, and unexpected, information that clients share during treatment—*revealing perceptions and reactions that are central to clients' problems but that would not become accessible if the therapist did not inquire in these ways:*

- *As you were driving to our session today, how did you feel about coming to see me?*
- *When you find yourself thinking about therapy, what kind of thoughts do you have about me and our work together?*
- *I'm wondering what you might be thinking is going on for me right now as we talk about this sensitive issue?*
- *I wanted to check in and get your thoughts about our work together and how you and I are doing together. You know, what you like about our time together, and anything about our interaction that may not feel so good.*

Therapists can also explore what clients *project* onto the therapist—what clients believe the therapist is thinking, expecting, or feeling toward clients at that moment. For instance, therapists can ask:

- *What do you think I am feeling toward you as you tell me that?*
- *How do you think I am going to respond if you do that?*
- *What do you think I expect you to do in that situation?*

Typically, it is uncomfortable for new therapists to break long-held familial and cultural norms and invite clients to talk about “you and me” so directly. Doing so, however, often brings up key issues and concerns that are central to clients' problems and the therapeutic relationship, even though the therapist had no idea this was going on privately for clients. With training and practice, student therapists can learn to become more safely and comfortably forthright with their clients than they have been able to be (or may want to be) with others in their personal life (Spangler, Hill & Dunn et al., 2014).

The distinction between everyday social interaction and clinical intervention is significant here. Because many therapists have a moving-toward interpersonal style, it is especially difficult for them to become more forthright in this way and risk clients' disapproval. By role-playing and rehearsing this interpersonal skill with classmates and supervisors, by viewing recordings of their sessions and considering times when they could have made process-oriented interventions, and in their own personal therapy, new therapists can practice being more direct or straightforward with clients without crossing the line and becoming confrontational or intrusive—which we don't want and never need to do. If trainees practice this intervention with colleagues and take the risk to pose such questions diplomatically and respectfully, clients' transference reactions will be made overt, and *often the therapist will be taken right to the core of clients' concerns*. The new issues that are revealed are often rich in meaning, and routinely surprise the therapist as clients reply:

- *I think you are feeling disappointed in me.*
- *I think you'll act nice but really be wishing inside that I would just stop coming.*
- *You're probably thinking that it's wrong of me to even be thinking about getting a divorce—that it's selfish for me to do that to my children.*

As IWMs and misperceptions such as these are made overt, therapists can clarify their actual responses to clients, which in itself often provides clients with a reparative experience. When transference reactions are revealed in this way, therapists have the opportunity to respond to clients with an immediacy and authenticity that is not often



found in other relationships (Gelso et al., 2013). For example, a therapist might clarify transference distortions by replying:

- *No, I'm not feeling disappointed in you at all. Actually, I'm very pleased with how well you have been doing. I'm wondering where that feeling of "disappointment" comes from, though. Did I do something today that made you think I was disappointed in you, or maybe this comes up with others, too?*
- *I want you to "stop coming"? Oh, no, that's not going on for me at all—I enjoy working with you very much. But I'm so glad you're telling me this—so we can work it through. Do you know when you started thinking this?*
- *No, I wasn't thinking that you are being "selfish." I was thinking about how hard you are trying to do the right thing—to find a balance between your needs and what you want to do for others. Tell me more about "selfish"—It's such a judgmental term. Is that an important or familiar word for you?*

Most beginning therapists find it difficult to explore the possibility *that their clients are having significant feelings and faulty perceptions toward them that are not based on the reality of how they have responded to clients*. With experience, these therapists often become more familiar with clients' distortions and may be better prepared to explore and clarify them. Even though the therapist may never have responded to clients in a way that would reflect any of these problematic perceptions, the discerning therapist still works under the assumption that such client reactions are likely to be present yet covert. If these distortions are not made overt and rectified, the potential for change is limited. For some trainees, this may be the critical point: They can accept intellectually that clients may hold such faulty expectations of others. However, it is often too threatening—evoking too much anxiety over making a mistake or guilt about having done something wrong—to *explore the possibility* that clients may have such unwanted reactions toward the therapist.

Exploring clients' reactions toward the therapist reveals three important types of information. First, the faulty perceptions and expectations that are revealed are often central to clients' presenting problems. They now provide a treatment focus and can be sorted through and resolved in the immediacy of the real-life relationship between the therapist and client.

Second, exploring transference will also highlight how aspects of the same problems that clients are having with others—and that they originally learned in formative relationships—are being reenacted with the therapist. Unless clients' reactions toward the therapist are directly invited and explored, these distortions and reenactments are not likely to be identified or resolved. Here again, transference reactions reveal clients' key concerns and provide an opportunity to resolve them—first, in the relationship with the therapist, and then with others.

Third, as paradoxical as it sounds at first, transference reactions ultimately provide evidence to clients that their feelings and perceptions really are trustworthy. We're back to our theme of self-efficacy. Because clients' expectations and emotional reactions may not accurately fit the current situation in which they are activated, they often do not make sense at first to clients or others. As a result, many clients experience their emotional responses toward spouses, children, employers, and others as "irrational," "stupid," or "crazy." Consequently, they do not trust the validity of their own experience and, as a result, their self-efficacy is undermined. This also occurs in the therapeutic relationship when clients have emotional reactions toward the therapist that seem inappropriate or exaggerated.

Clients' emotional reactions are not irrational, however. Though they may not fully fit the present circumstances, they do make sense when they can be understood within their original developmental context. Real-life events have legitimately caused clients to feel and expect what they do. When therapists help clients link feelings and expectations

to where they originally learned them, they will prove to have been an appropriate reaction to someone else in another time and place (by therapist asking, for example, “*When is the first time you can remember feeling this way? Who were you with and what was happening?*”). Once the therapist and client have been able to clarify and resolve the transference distortion in their current interaction, this developmental link can often be made readily. In contrast, the developmental context will not come alive or be meaningful to clients if it is not first resolved in the current interaction with the therapist. In this way, transference reactions, like resistance, ultimately reveal that clients’ feelings make sense. Consider the following exchange:

- CLIENT:** He doesn’t want to listen to me, maybe I am just too much—too needy, too demanding, too much maintenance...I don’t know...
- THERAPIST:** Have you ever felt that way in here with me? That I find you “too much” in some way?
- CLIENT:** Well, sure, I guess I’ve thought that sometimes, but you can’t say anything because you’re a therapist. You know, you have to be nice—that’s your job.
- THERAPIST:** Well, no, I don’t think that being nice is my job—but giving you honest feedback is. If I thought you were doing something with me that was causing problems with others in your life, I’d tell you. It might not be easy for me, but I absolutely would take the risk and try to find a way to talk with you about it in a constructive way. So, no, I don’t find you needy or demanding or too much with me. Actually, I like it when you speak up for yourself—I think it’s a strong and healthy part of you.
- CLIENT:** I’m glad you’re saying that. I think I go along with other people too much, and don’t say what I want, because I’m so afraid of seeming, you know, selfish or demanding or something. I guess it’s easier for me to say what I want in here with you than it is in the rest of my life.
- THERAPIST [linking interaction with therapist to client’s daily life]:** I’m glad it’s easier with me—but what would you like to be able to say the next time you feel like your boyfriend isn’t listening to you or taking you seriously?
- CLIENT:** Just tell him to stop acting like an asshole.
- THERAPIST:** How do you think that will help your relationship?
- CLIENT:** I guess we’d just get into another fight...do you have any suggestions?
- THERAPIST:** What if, next time, you just said to your boyfriend, “I don’t think you’re really listening to me right now. Can we both try to listen better to each other?” How would it be for you to try something like that?
- CLIENT:** Ya, that’s better—I can say that to him. And I guess if he can’t really listen to me, I probably shouldn’t be thinking about moving in with him.
- THERAPIST:** Yeah, if he can generally respond well when you ask him for what you need, green light; and if he still can’t listen when you ask so specifically and respectfully, red light.

This type of transference clarification gives clients a real-life experience of change; *they learn that their inaccurate expectations or problematic IWMs do not fit in this relationship*. As they have a real-life experience of change with the therapist, they find that at least some relationships, sometimes, can be another way. This experiential relearning, which comes from clarifying the misperception with the therapist, allows many clients to make two important steps. First, they often are able to make meaningful links to formative relationships that shed light on current patterns and problems. Second, by clarifying the transference distortion and finding that the therapist is not responding in the familiar but unwanted way, many clients are empowered to try out these new ways of responding with others that have succeeded with the therapist. Some will initiate this step

on their own (*Client: Guess what I did this weekend...*) and others will risk changing as the therapist makes a suggestion—as occurred in the previous dialogue. (*Therapist: How would it be to say to him...*). Together, the therapist and client then begin the essential process of sorting through who in the client's life is likely to respond well to these changes and who isn't.

## FINDING INTERPERSONAL BALANCE

In this chapter, we are trying to understand the patterns and purposes in clients' interpersonal functioning in order to discern a treatment focus. In particular, we are exploring different ways that clients tend to play out with the therapist interpersonal patterns and themes that also are causing problems with others. However, as we consider clients' eliciting maneuvers, testing behavior, and transference reactions, it draws us further into the closely related issue of countertransference, where therapists get pulled into "enactments" with their clients. We are striving to maintain a balanced level of interpersonal involvement with clients. The therapist's intention is to be empathic, present and emotionally engaged, yet still boundaried or differentiated. This balanced degree of involvement helps therapists manage their own client-induced and therapist-induced countertransference reactions more effectively, and be better prepared to provide clients with a corrective response that does not reenact familiar but unwanted relational scenarios (Gelso & Hayes, 2002; 2007).

To help with this cardinal issue in clinical training, recall the two important attachment constructs introduced previously: Safe Haven and Secure Base. As a Safe Haven, the therapist uses empathic understanding and emotional availability to make it safe for clients to share their distress and feel responded to. In combination, as a Secure Base, the therapist also has the task of supporting autonomy and differentiation while sustaining connection—such as the task of respectfully challenging clients' schemas or questioning the current utility of their interpersonal coping strategies (Yeomans et al, 2013). Thus, the therapist has to monitor these two different trajectories—trying to balance being separate and related at the same time, and avoid two common types of countertransference—**enmeshment** and **disengagement**.

To provide therapeutic relationships that facilitate change, therapists want to track this interpersonal process and intervene to restore a more balanced degree of involvement when this has been lost. On the one hand, if the therapist becomes overly invested in clients' choices or change, this will be an unwanted reenactment for many clients (enmeshment). Especially for clients who grew up with too much parental control or intrusiveness, or too little support for pursuing their own interests and goals, their fear and expectation that they will again be controlled by the therapist (and others in their life they get close to) will be confirmed. On the other hand, if the therapist is emotionally removed or too personally distant (disengagement), a CEE cannot occur either. Although this will be especially problematic for clients who had Authoritarian, rejecting, neglectful, Dismissive, or otherwise distant caregivers, it will also diminish the potential for treatment gains for most clients because the relationship is too insignificant to effect change. That is, even if the disengaged therapist responds in new, healthier ways that counteract what clients have found in the past, it doesn't have much impact because the relationship is not very meaningful to the client. Thus, throughout the course of treatment, the therapist's intention is to monitor the balance of separateness versus relatedness in the therapeutic relationship and maintain an effective interpersonal balance (Kerr & Bowen, 1988; Minuchin & Nichols, 2007).

New therapists want to be patient with their ability to apply this concept with their clients. At times, all therapists will have difficulty maintaining an effective interpersonal

balance with some clients—whether because of client-induced or therapist-induced countertransference. To manage these pulls, the therapist’s aim is to be both a genuine participant in the relationship and, as we are trying to clarify, simultaneously an objective observer of it as well. Sullivan (1968) captured this complex, dual role in his enduring term “participant/observer.” However, combining genuine empathic engagement with an “observing ego” or objective viewpoint is no easy task for beginning (or experienced) therapists. It is a challenging paradox indeed to be close or related (be empathically engaged) and Separate (be differentiated and maintain clear boundaries) at the same time (Jordan et al., 1991).

Without question, this balancing act is one of the most difficult skill sets for new therapists to develop. Perhaps an apt analogy is being a 16-year-old student-driver in a car with a manual (stick shift) transmission. Simultaneously, the new driver is trying to press down on the clutch with her left foot; brake or accelerate with her right foot; steer the car with her left hand; and shift the gearshift with her right hand. She does this while she is trying to look ahead down the road; see where she is going and attend to what is rapidly approaching; and listen to instructions from a driving instructor sitting next to her and barking instructions! In just a short time, of course, the new driver will be able to do this almost automatically—and even change the playlist on the MP3 player, monitor directions on the GPS, and carry on a meaningful conversation at the same time. Similarly, with practice and two or three years of experience, trainees can be reassured that it will become much easier to maintain this self-reflective capacity and simultaneously be both a participant in and an observer of what’s transpiring in the therapeutic interaction.

### ENMESHMENT

At times, all therapists will become over-identified—“enmeshed,” in family systems terms—with certain clients. For example, if a client is well practiced and skillful at getting therapists and others to rescue, direct, or take responsibility, this is an instance of client-induced countertransference. It probably has more to do with what the client elicits in others than with the therapist’s own personal issues. With the help of a supportive supervisor, the therapist can come to recognize that the client is making the therapist feel responsible—just as the client does with others in his or her life. It is relatively easy to begin talking with the client about this relational pattern and changing how it is potentially being reenacted in the therapeutic relationship.

However, this becomes a more complicated issue when the client’s attempts to make the therapist take responsibility, self-disclose, or engage more as friends taps into the therapist’s own personal issues (e.g., perhaps this therapist grew up in a parentified/caretaking role, or has an anxiously Preoccupied attachment style with a high need for approval). With this type of therapist-induced countertransference, therapists lose their objectivity with the client because their own personal concerns have been activated. For instance, the parentified therapists may continue in a caretaking, dependency-fostering role with the client, or the Preoccupied therapists may have difficulty maintaining clear boundaries or challenging the client because they can’t risk the client’s disapproval (Sauer et al., 2003). Or, to use another example, therapists become personally invested in the client choosing to get (or not to get) a divorce—as therapists experience how hurt they have been by their own recent or their earlier parents’ divorce.

In these ways, their own developmental histories will give all therapists a tendency to lose the clarity of their personal boundaries and become over-involved with some clients at times. How can therapists recognize when this therapist-induced type of countertransference is occurring? When therapists find they are frustrated, angry, or critical of a client for not changing, they have become overly invested and usually are trying to manage a personal issue through the client. Similarly, therapists often are enmeshed when

they have dreams about a client, often think about a client outside the therapy session, feel depressed (rather than appropriately concerned) when a client is not changing, or become envious or elated over positive changes in the client's life. Enmeshment is usually occurring when therapists describe the client as being “just like me” and cannot recognize the many differences that, in reality, always exist between therapist and client. This over-involvement, occurs, for example, when a Preoccupied therapist loses clear boundaries with a Preoccupied client who is pressing for intense engagement or even fusion—which is very different than genuine care and liking for the client which, of course, is helpful and appropriate when it is modulated. When enmeshment occurs, therapists usually need the client to change in order to meet their own needs—for example, to shore up their own feelings of adequacy as therapists or to manage personal problems or feelings of their own that are similar to the client's. Recalling the concept of change from the inside out or an internal locus for change, enmeshed therapists are trying to solve an internal problem of their own, externally, through the client.

Treatment progress and change usually halt at the point when therapists become over-involved with a client. When they become enmeshed, therapists lose sight of the fact that clients have their own subjective worldview, shaped by many factors that actually differ from those of the therapist. They tend to become controlling of the client and to project their own problems and solutions onto the client. This response often reenacts the client's developmental predicament in the therapeutic relationship. For example, many clients have grown up in families with overly controlling or intrusive caregivers. When therapists fail to support the client's own autonomy and differentiation, they are responding along the same problematic lines as caregivers did in formative relationships. This reenactment with the therapist will hinder clients as they try to resolve the same types of conflicts around intimacy and control that originally brought them to treatment. One reason for paying attention to maintaining an interpersonal balance, and always being receptive to the possibility of enmeshment occurring, is to safeguard against countertransference and responding to clients along familiar lines that originally contributed to their problems.

When therapists become too close or over-identified, they lose their ability to be a participant-observer. They cannot think objectively about clients or step out of the interpersonal process they are enacting with their clients to consider what their interaction may mean. In parallel, their heightened emotional reactivity to clients will cloud their understanding and diminish their ability to provide a holding environment and remain a “steady presence” to contain clients' feelings and distress (See Fonagy et al., 2008). *Enmeshed therapists lack the reflective capacity to step out of affectively charged interactions sufficiently to reflect on their own thoughts, feelings, and behaviors, and clients' experiences, and respond to what's going on between them.* Instead of accurate empathy, enmeshed therapists will offer clients only a global, undifferentiated sympathy (as in, **Ineffective Therapist:** e.g., “*Oh, I totally get it - I know exactly what you mean!*”) that does not consistently capture the specific, personal meaning that a particular experience holds for a client. Most important of all, perhaps, therapists will fail to discriminate the real differences between themselves and clients that do exist and will begin to see clients as being just like themselves. This over-identification may feel like closeness or deep understanding, and can be reassuring to some clients at first. However, initial positive changes do not sustain well in an enmeshed therapeutic relationship because clients will also need to differentiate—which an enmeshed therapist will have difficulty supporting.

## DISENGAGEMENT

Just as many therapists will become enmeshed with clients at times, many therapists also will hold themselves too far away from certain clients at times. Per client-induced countertransference, therapists will often observe themselves initially distancing from



moving-against and other “negative” clients who are characteristically hostile, critical, competitive, distrustful, controlling, or manipulative (Martin et al., 2007). Disengaged therapists distance themselves just as others in clients’ lives often distance from them. Similarly, it is common for clients with a Dismissive attachment style to evoke feelings in the therapist of boredom, detachment, or being unimportant and unneeded—feelings that these clients elicit in others in their lives, and feelings that also mirror Dismissive clients’ own childhood experiences (Daly & Mallinckrodt, 2009; Slade, 1999). As therapists *recognize what their own personal reactions may be teaching them about clients’ problems with others*, they can begin looking for points of entry to broach this topic and start talking about it (Bennett, 2008).

Therapists also tend to disengage or distance from clients when clients arouse a therapist’s own personal conflicts. For example, the therapist may disengage from the client as the client talks about the impending death of an aging parent. In this situation, the therapist may be activated by his or her own unresolved sadness about significant losses, disappointments, or deprivation. A primary purpose of supervision is to help therapists recognize their own countertransference reactions, find a more appropriate way to manage their own personal feelings (e.g., talking with a supervisor or a colleague about them, or in their own therapy if the issue endures), and empower therapists to be able to re-engage clients with their own boundaries better in place (Falender & Schafransky, 2004). Many studies show that attending to therapists’ countertransference is essential to positive treatment outcomes (Beatson & Rao, 2014).

When therapists cannot be emotionally available or responsive to their clients the working alliance is shallow, or there may be too little empathy to allow some clients to enter treatment after the initial intake or first session. Treatment also loses its intensity and often becomes intellectualized when the therapist cannot be a “participant” in the real relationship with the client (Gelso, 2014; Marmarosh et al., 2009). Therapists need to be able to risk affecting and being affected by the client—nothing ventured, nothing gained.

Therapists also lose their intuition and creativity when they distance themselves from clients. Clients need the engagement that comes with a supportive relationship or safe holding environment necessary to explore affect-laden problems. Counseling is more than an intellectual discourse, and little enduring change will occur in an emotionally disengaged relationship. Without the impetus or real meaning provided by genuine personal involvement, a CEE will not occur, even if the therapist responds in schema-disconfirming ways. The relationship does not matter enough to impact the client and fuel change.

Finally, a therapeutic relationship in which the therapist is too distant or uninvolved will reenact developmental problems for many clients. For example, for clients whose aloof, Dismissive, self-centered, or Authoritarian parents could not meet their childhood needs for emotional support, a distant therapist might be frustrating but ultimately “safe.” That is, their anxieties over their insecure attachments and unmet needs—and the resulting problems associated with trust, intimacy, or making commitments—will not be activated in this therapeutic relationship. This may occur, for example, in a therapeutic match between a Dismissive therapist and a Dismissive client, where both may be too comfortable with distance to engage each other or clients’ problems in a meaningful way (Dozier et al., 1994). Unfortunately, just as clients’ conflicts are not aroused in a disengaged relationship, the opportunity to resolve these issues is lost as well. Our experience as supervisors has been that novice therapists who are consistently unwilling to risk authentic engagement with their clients are less likely to be effective (and to benefit from supervision) than those who are willing to risk a real relationship but tend to become over-involved at times.

## EFFECTIVE MIDDLE GROUND OF BALANCED INVOLVEMENT

To be an effective participant-observer, or sustain an effective balance of being separate yet connected, therapists try to maintain a middle ground of involvement. Although the goal is to maintain this balanced involvement throughout the course of treatment, it is no easy task. Let's look at other ways to achieve this goal.

**Learning How to Manage Our Own Reactivity.** Therapists are continually confronted with personal issues in their own lives by the sensitive, affect-laden material that clients present. For every therapist, it's just part of the work—it comes with the territory. As a basic element of their training, all therapists are encouraged to pay attention to their own countertransference propensities and become more aware of how they tend to respond when clients activate their own anxieties or personal issues. For example, when your client talks about an issue that is difficult for you, pause for a moment and try to identify your initial response tendency.

Some therapists initially withdraw from the client and distance themselves by intellectualizing—for example, by interpreting the client's affect before the client fully experiences it or by ineffectively “explaining” or interpreting what something means. Others become overly problem-solving and prescriptive, telling clients what they should do in a particular situation. Whereas some therapists begin to talk too much, we find that many student therapists become too quiet and unresponsive, leaving clients feeling that they aren't getting any help or don't have a real partner with them in the counseling room. With a little effort and some non-defensiveness, however, therapists can begin to identify their own countertransference propensities and learn how to manage them responsibly. Again, this is so important as studies consistently find that countertransference impacts treatment outcome (Hayes, et al., 2011).

Becoming familiar with our own response tendencies becomes even more important with moving-against and other difficult clients who respond to the therapist with negativity (Binder & Strupp, 1997). Through eliciting maneuvers, testing behavior, and transference distortions, we have seen that some clients will respond in negative ways to therapists. Too many therapists have not been given the practice and preparation they need in order to respond non-defensively to these challenging situations. Therapists-in-training need to role-play or rehearse alternative responses with supervisors that offer more “neutrality” (i.e., not the familiar fight-or-flight responses they typically evoke), and therefore are less likely to repeat the familiar but problematic response sequences that these clients' negativity usually elicits from others. New therapists also are encouraged to anticipate and explore how their own personal issues and response tendencies will be triggered by different kinds of challenging, unwanted responses from the client—such as power struggles, criticism, sexual innuendos, and so forth.

Whereas much is often said about “nice” things such as empathy, genuineness and warmth, or a working alliance, less training is usually provided to help beginning therapists respond to a demanding, passive-aggressive, or demeaning client (the one who comments, for example, “Looks like you've been gaining some weight. What's your husband say about that?”). In this regard, researchers find that therapists do not register or respond to clients' negative reactions toward them very well—*therapists tend to deny or avoid the unwanted remark*. It seems that it is much easier for therapists to miss or gloss over negative/hostile responses from clients than positive responses (Hill et al., 1992). However, we don't want to tune out or miss these important messages from clients. Why? *The negativity coming toward us is often at the heart of what is causing problems in clients' relationships with others.* We need to make the most of the time we have with

clients, and addressing clients' problems with others—especially as they are occurring right now with us—is usually the best way to do this.

Our intention is to approach negativity non-defensively, rather than avoid it and act as if something significant was not being said—even though this may be hard to do in the beginning. Our goal is to work with it in an up-front way that doesn't punish or blame clients in any way but, instead, helps them learn about the problematic impact they are having on others, and better ways to negotiate conflict and manage feelings in relationships. Consider this example:

**CLIENT:** (*critical tone*) Your office is looking pretty tired. This furniture and brown color went out a few decades ago.

**THERAPIST:** (*calmly*) Things look a little out-of-date to you in here.

**CLIENT:** Yeah, you should do something to change it.

**THERAPIST:** No, I don't think I want to make any changes; I'm comfortable here. But maybe it would be helpful for us to talk about this for a minute. You've said things like this to me before, and I'm wondering how others usually respond when you criticize them or tell them what they should do?

**CLIENT:** Geez, Doctor, relax. I'm just trying to be helpful. You shouldn't be so sensitive.

**THERAPIST:** It's OK that you don't like my décor. But I am still wondering how others usually respond when you advise or critique them like that.

**CLIENT:** Well, I don't know, I guess mostly they just do it.

**THERAPIST:** Uh-huh, I can see that—you do have good taste, and express strong opinions in an authoritative voice. But I'm wondering how it affects your relationships. Maybe you're right and this is just me here, but I'm thinking that most people wouldn't like too much of this. What do you think?

**CLIENT:** (*pause*) Well, no, maybe some people at my church might agree with you and say I'm a know-it-all.

**THERAPIST:** Well, I'm not saying that, but I am wondering how this goes with others. So, yeah, I can imagine some people feeling intimidated and wanting to get away from you, and maybe a few feeling resentful and wanting to argue with you. What it's like for you to be seen in this way—how does that feel?

**CLIENT:** Well, when you say it like that, it makes me feel kinda bad—almost like crying...

**THERAPIST:** Crying. Let's sit together with that feeling and just let it be there.

**CLIENT:** (*slowly*) I'm taking over, aren't I...just like my mother did. Giving advice and opinions that others haven't asked for and don't want. Sometimes I wonder if I'm going to ruin my marriage—like she ruined hers...

This therapist was effective because she remained *non-defensive* in the face of the client's criticism and control. And she *approached* this sensitive issue—she took the risk of naming it and asking about it in a respectful manner—rather than just continuing on and acting as if nothing important was happening between them. In contrast, other therapists too often respond ineffectively to such negativity with their own personal or counter-transference reactions, as in the following exchange:

**CLIENT:** Your office is looking a little tired. This furniture and brown color went out a few decades ago.

Three ineffective responses to this client:

(Moving-toward and complying): *Oh, I'm sorry. You know, you're probably right. Maybe it is time for me to update things a bit in here.*

(Moving-away and avoiding): *OK, well uh, what do you think we should work on today?*

(Moving-against and chiding) *Maybe you should spend more time thinking about your own problems and less about what everybody else should do.*

With practice, and by observing instructors role-play or model effective responses, therapists can learn how to *remain non-defensive and approach the negativity*—the key to responding effectively. This approach empowers the therapist to intervene when they provide interpersonal feedback, make process comments, take a curious stance toward what was just said, or simply adopting a neutral stance to question, highlight, or explore this negativity. Although still poorly integrated into clinical training, psychotherapy researchers have been trying for decades to tell practitioners that negativity will occur with many of their clients and therapists need to learn how to deal with it more effectively. Clearly, this is an important part of the treatment process that is difficult for new and experienced therapists alike as they tend to respond with the same fight-or-flight reactions that clients elicit from others and which cause problems in their lives (Safran & Muran, 2003; Strupp & Hadley, 1979).

To help therapists better maintain an effective interpersonal balance with clients, let's look further at what therapists can do to manage their reactivity to clients' negativity. Angus and Kagan (2007) emphasize that it is critical for therapists—early in their training—to begin tracking their response tendencies and countertransference propensities, and they provide guidelines to help with this process. One useful exercise for therapists is to write down their initial reaction propensities in different anxiety-arousing situations—for instance, *what you are most likely to say or do first* when a client is demeaning or critical of you, or telling you how amazing you are and asking if you can meet at Starbucks instead next week.

Alternatively, with a partner, trainees can role-play their responses to varying clients in different challenging situations. For example, one person can role-play being a competitive, intimidating, or insulting client who is questioning whether the therapist is really capable, experienced, or old enough to be able to help. Taking turns, the other trainee practices responding to this challenging client. In small discussion groups of three or four, trainees can provide each other with feedback, or watch themselves being replayed on videotape, and share their observations of the other therapists' responses while role-playing these challenging situations. With practice and guidance from supervisors, new therapists can learn to approach rather than gloss over and avoid these unwanted situations, find non-confrontational, non-blaming ways to make them overt and safe to talk about, and begin to explore them collaboratively with clients. The goal with this approach is to *identify and alter the usual problematic scenario that typically ensues for clients with others*. This gives therapists a chance to provide a new and corrective experience of change—as we saw in the previous dialogue.

**Managing Over-involvement and Under-involvement.** It is also challenging to maintain an effective balance between over-involvement and under-involvement because *clients may try to shift or move the therapist along this continuum of involvement as part of their interpersonal coping strategy*. This fits our term, “client-induced countertransference.” When the therapist has been responding effectively, some clients will become

threatened by anxiety-arousing issues that begin coming up (succeeding and getting better), or the alliance-strengthening and the therapist becoming more important to the clients. To cope with these anxieties that can be evoked by success in treatment, some clients will re-test safety/danger in the therapeutic relationship by trying to engender either enmeshment or disengagement in the therapist. For example, some clients may try to bore the therapist by talking about issues they are not really concerned about (which can be more problematic for a Preoccupied therapist, who needs engagement and intensity). In contrast, other clients may try to elicit too much involvement from the therapist by exaggerating their distress and escalating the urgency for their need for help (which may present more of a challenge for Dismissive therapists, whose propensity is to minimize strong emotions and bids for intense engagement). In these and other ways, clients' eliciting behaviors may press the therapist away from a balanced middle ground of involvement.

If therapists can maintain this effective middle ground of involvement—by remaining both a participant and an observer in the counseling relationship—they can provide therapy that is more even with no stormy ups and downs or lengthy impasses. It is also safer for clients to be vulnerable and trust when the therapist is dependably available without swinging to either side and becoming over-reactive or unresponsive. By offering this steady presence in the face of clients' eliciting maneuvers, the therapist is providing a Safe Haven for the client. More specifically, when therapists can maintain this effective interpersonal balance, clients do not have to be *concerned about disappointing or letting down an over-involved therapist, or eliciting the responsiveness or engagement of an emotionally removed or distant therapist*. In this regard, the attachment researchers have appreciated best the importance of the caregiver's, and the therapist's, *consistent* responsiveness in developing security and trust in relationships (Tufekciogly & Muran, 2015). The key is for therapists to maintain their own personal boundaries (Separateness) while still being empathic and responsive to clients (Relatedness). Therapists are striving to enter clients' subjective worldview—to be fully present and empathically attuned—yet still maintain appropriate self-boundaries and self/other differentiation.

How can we help therapists recognize the interpersonal process they are enacting with their clients and assess their balance of involvement with clients? Therapists can do this by considering the following questions after every session:

1. What are my feelings and personal reactions toward this client?
2. How might my reactions parallel those of significant others in the client's life?
3. Are my feelings typical of my reactions to others, or are they more confined to this particular client?

By writing answers to these questions in their case notes after each session, therapists ensure that they are attending to the process dimension. Asking themselves such questions will also help therapists distinguish their own personal reactions toward the client (therapist-induced countertransference) from the client's eliciting behaviors and interpersonal coping style that affect many people (client-induced countertransference). Building upon these three questions, therapists are encouraged to utilize the guidelines for writing process notes in Appendix A after each session.

All therapists will sometimes become over-involved with a client. Indeed, many theorists have described the therapeutic process *as therapists' ability to become immersed in clients' modes of relating, and then to work their way out of these enactments* (Gill & Muslin, 1976; Levenson, 1982; Mitchell, 1988). New therapists often feel discouraged when their supervisors accurately point out that, here again, they are reenacting aspects of clients' problems in their interpersonal process. By examining their own affective and interpersonal range, anticipating their own countertransference propensities, and tracking



the separateness-relatedness dimension in the therapeutic relationship, new therapists will become increasingly able to maintain an effective interpersonal balance—and thereby become more effective therapists.

When therapists realize that they have become over- or under-involved with a client and find that they cannot realign the relationship on their own with a process comment, it is best to consult with a colleague or supervisor to help reestablish clear boundaries. One of the most productive uses of supervision is *to help therapists regain control of their own emotional reactions so that they can reestablish an effective degree of involvement with a client who has successfully pushed them away or drawn them in too much*. Sustaining this interpersonal balance is one of the best ways to provide a reparative relationship and keep from reenacting the familiar but problematic scenarios that occur with others.

## AMBIVALENCE: RESPONDING TO BOTH SIDES OF CLIENTS' EXPERIENCE

Therapists can better understand their clients' problems when they recognize the ambivalent nature of many conflicts. Often, there are two sides to clients' most enduring problems and core conflict. Before many problems can be resolved, therapists need to be able to capture and respond to both sides of their dilemma. Therapists do this by responding with interest, curiosity, and an open-minded, exploratory stance for *both sides* rather than persuading or pressing clients, or subtly leading and cajoling them, toward one side or the other (Miller & Rollnick, 2014; 2002).

Often, because of their own countertransference, therapists may fail to recognize both sides of their clients' conflict. Sometimes, because they believe they know what is “right” and know what clients “should” do, therapists press clients toward only one side of their ambivalence about a decision (e.g., an ineffective therapist might say: *Just leave your boyfriend... You need to stay in school... You should apply for that position... You shouldn't buy him that car... It's a mistake to get married so young...*). Therapists are more helpful when they can articulate and highlight more clearly, and then help clients explore and sort through for themselves, the two opposing feelings, competing needs, or contradictory beliefs that make up the double-edged nature of the binding conflict they commonly are grappling with. For example:

**EFFECTIVE THERAPIST:** Yes, I can see what an important but difficult decision this is for you. I'm not invested in the choice you make. I just want to help you sort through the mixed feelings and all of your pros and cons about this decision. Let's explore both sides: tell me about the part of you that wants to leave him, and the part of you that doesn't.

Colloquial expressions convey the two-sided nature of conflict—we often say that someone is “stuck between a rock and a hard place.” Such expressions capture the push-pull nature of conflict—that you “can't live with them and you can't live without them” or are “damned if you do and damned if you don't.” Similarly, therapists working within different theoretical perspectives have used differing terms to describe this. Long ago, for example, the behavioral therapists Dollard and Miller (1950) highlighted how clients were often struggling with “approach-approach” or “avoid-avoid” conflicts, while communication theorists and early family therapists highlighted double-bind interactions and paradoxical communications (Bateson, 1972). We have also seen that contemporary attachment researchers reveal the immobilizing plight of “Category D” children who are attached to a caretaker who is sometimes helpful but, at other times, also frightens or hurts them (Hesse & Main, 1999). To these children, seemingly, there is no solution to this double bind. They think, “I can't walk away from this caregiver who I depend on, yet at

the same time, I can't be close and need this person who frightens or hurts me so much." In this regard, clients with the most intractable symptoms, and who tend to internalize or repeat their caregivers' own problematic parenting or symptomatic behavior, grew up with these highly ambivalent relationships (Benjamin, 2006). Indeed, life is easier when caregivers are generally good or generally problematic. In contrast, when there is too much good and bad news mixed together, problems become more complex and harder to resolve (e.g., when a client is molested by her stepfather who, in other ways, is the most responsive and helpful adult in her life).

There are many different facets to this issue of the two-sided nature of conflict. One aspect is how therapists often fail to appreciate that some clients have concerns both about being hurt by the therapist as they have been hurt by others, and about hurting others by doing normal, appropriate things (Weiss, 1993). The counseling field has well-developed concepts regarding clients' expectations about being hurt, misunderstood, or betrayed, by the therapist—as they have been in other important relationships (i.e., Transference). In contrast, far less attention has been paid to some clients' worry and guilt about hurting the therapist and others (and their internalized attachment figures) by doing well in life. This commonly occurs, for example, when these clients do better than their parents and surpass their caregiver in some arena such as having greater financial success, enjoying a happier marriage, or simply living longer or better than their caregivers. It may also occur when clients succeed at reaching a sought-after goal, appropriately stand up for themselves or advocate on their own behalf, and pursue their own interests or career choices.

When these clients make progress and improve in treatment (e.g., by having a relationship with someone who treats them well), they may retreat from these success experiences. They may sabotage themselves, be unable to sustain progress toward their own chosen goals or, most commonly, simply be unable to enjoy or feel good about the success they achieve. Therapists will observe that some clients will become depressed as a result of separation guilt or survivor guilt. Even more will become anxious as success threatens their attachment ties to internalized caregivers who did not support their individuation, or if that success deviates from their prescribed familial role (e.g., to take care of others and submerge their needs rather than pursue their own interests or goals). Clearly, the therapist doesn't want to sit back and relax as clients begin to make progress in treatment and improve their lives—this is often where the real work begins! Clients' contradictory or self-defeating behaviors do make sense, but not until we understand the two-sided, push-pull, or double-binding nature of their conflicts.

With greater appreciation for ambivalence or the "shadow side," therapists will be more effective when they can take a neutral, exploratory stance and invite clients to explore *all* of their competing or contradictory thoughts and feelings regarding this issue. For example:

**THERAPIST:** It sounds like one part of you wants to get married but another part of you isn't sure. Let's let both sides have their full voice. Tell me about the part of you that wants to marry him and the part that doesn't, so we can sort through all of your concerns about this.

Therapists provide a Secure Base for exploration, and often a CEE, by welcoming both sides of their clients' ambivalence and responding to the opposing or contradictory feelings that accompany both sides. As clients find that they have the support of the therapist to explore all of their concerns about a decision, they are empowered to clarify their own preferences, take responsibility for their own decisions, and act on their own choices. In contrast, therapists will not be effective when they only respond to one half of their clients' ambivalence, do not encourage clients to explore all sides of the issue, or jump in

and subtly advocate or overtly tell clients what to do. In their Motivational Interviewing work with problem drinkers, Miller and Rollnick have found strong empirical support for *listening for and then supporting clients' own intrinsic motivation to change rather than trying to instill it or "motivating" clients to make a particular decision*, such as to stop drinking (2013; 2002). When therapists cannot "hear" both sides of their clients' ambivalence, or when they press clients to act or choose the way the therapists want, therapists' own countertransference is usually in play, as the following vignette illustrates.

*Marie, a depressed, 25-year-old graduate student, entered time-limited (16 sessions) treatment at the Student Counseling Center on campus. During the first few sessions, she explored how angry she was at her mother. Her mother expected Marie to be "perfect," insisted that Marie never had any problems, and always needed Marie to be "happy." In counseling, Marie began to realize how much she resented her mother for denying so many of her true feelings and for having so many expectations of how she "should" be. Recalling similar issues in her own childhood, Marie's therapist resonated with her anger and actively supported her indignation.*

At first, it was liberating for Marie to have her long-suppressed anger affirmed. She was both excited and relieved to be realizing that she no longer needed to keep fulfilling her mother's unrealistic expectations. This good feeling was short-lived, however. In the next few sessions, Marie's newfound freedom gave way to a growing despondency and return to her long-standing dysthymia. The therapist thought that Marie was worried about becoming the "bad child" in the family for defying her mother's expectations, feeling disloyal to her family for complaining about her mother to the therapist, and feeling guilty about being angry with her mother.

While asking about and looking for opportunities to explore these reasonable possibilities, the therapist continued to draw out and encourage Marie's anger. However, moving in a different direction than the therapist, Marie began to share fond memories about her mother and recalled special times they had spent together baking and making cookies after school. The therapist was not very responsive to Marie's positive feelings toward her mother, however. The therapist thought that Marie's fond recollections were part of denying her anger toward her mother, avoiding the reality-based conflicts in their relationship, and was concerned that Marie would continue to be ruled by her mother's problematic expectations.

As too often happens, Marie's therapist only recognized one side of her ambivalence and failed to respond to the feelings on both sides of her conflict. Yes, Marie was angry with her mother, and these feelings needed to be explored and affirmed. At the same time, however, Marie also knew that her mother had offered her many fine things as a parent and she did not want to forgo the good things they had shared. Marie's conflict was that she wanted to keep the good things she had experienced with her mother without having to take on or adopt the problematic aspects of their relationship as well. On the one hand, Marie wanted to reject the unrealistic demands to be perfect and happy that her mother had placed on her. Yet on the other hand, Marie became depressed when it seemed that the only way to do this was to give up her identification with her mother altogether. This had been the most important relationship in her life and, understandably, Marie did not want to risk losing the good parts of this relationship as well.

Realizing that time was short and treatment was stagnating, Marie's therapist sought consultation from a colleague. As an outside observer, it was easy for the colleague to recognize both sides of Marie's ambivalence. The colleague encouraged the therapist to *give Marie support for simultaneously having contradictory feelings of appreciation and anger toward her mother*. At first, the therapist was reluctant and began arguing with the colleague. Soon, however, she began to consider the possibility that her own countertransference might be operating and that, because of her own history, it was easier

to support Marie's anger toward her mother than her appreciation for her. As a result of this helpful consultation, the therapist watched at their next session for a point of entry to invite and affirm the other side of Marie's feelings in the following way:

**THERAPIST:** It sounds like you feel two different ways toward your mother at the same time, and that makes sense to me. You're angry with her for expecting you to be perfect and happy all the time, but you also treasure the many fond times you've had together. Tell me more about your angry and your loving feelings toward her—bring me in on both sides of this.

This new approach had an immediate impact on Marie. As the therapist encouraged Marie to explore both sides of her feelings and, in effect, gave her permission to love her mother and be angry with her at the same time, she felt enlivened and began to make real progress. Marie began to clarify what qualities of her mother she wanted to keep and make a part of herself (such as her mother's easy laughter and genuine warmth at times) and what aspects of her mother she wanted to discard (her mother's preoccupation with appearances and what other people might think). This process of differentiation also allowed Marie to clarify which issues she wanted to bring up, talk about, and try to change in her relationship with her mother, and which issues she preferred to let be and simply accommodate herself to.

Marie continued to make progress in treatment as the therapist stayed this course and provided support for both sides of her ambivalence. At times, for example, the therapist would observe, "It really made you mad when she did that." However, this would soon be followed by the reflection, "I can see how important you two are to each other, and how much you love her—even though these problems get in the way sometimes." Therapy was successfully terminated at the 16th session. Marie was no longer depressed and had better integrated aspects of her own identity as a woman. In the months following termination, Marie was continuing to initiate a rapprochement with her mother, was sorting through what could change and be better now in their relationship and what couldn't, and was exploring realistically how much authenticity she could have, including the closeness they could share now at this different point in their lives.

As Marie demonstrates, therapists help clients change when they recognize the two-sided structure of their clients' conflicts and help clients explore and integrate both sides of their ambivalent feelings. Trained to look for pathology, therapists tend to miss personal strengths or positive aspects of conflicted relationships. However, clients will make far more progress when therapists can actively encourage clients to keep the good parts of the parental (or spousal) relationship while they are realistically examining the problematic parts. In this regard, it usually is very helpful if *therapists can tell clients explicitly that they are not being disloyal if they talk about problems with a parent or spouse, and that therapists know that clients' complaints do not reflect all of their feelings about this person*. This means clients can be disappointed or angry with someone they love while still remaining connected to other, more rewarding aspects of the person. This, in turn, will also allow clients to still feel worthwhile even though they are disappointed with themselves about something.

## CLOSING

With these conceptual guidelines in mind, the next chapter returns more directly to the treatment process and explores the process dimension in more depth. Working in the here-and-now with the process dimension can be helpful to therapists working within any treatment approach. However, if therapists are to use process-oriented interventions and the therapeutic relationship to effect change, they need to be able to talk with clients

about their current interaction and what may be going on between them. These immediacy interventions are powerful, yet challenging, for most new therapists to adopt because it often feels uncomfortable to meta-communicate and “talk about you and me.” Thus in Chapter 9 we explore therapists’ concerns about working with the process dimension and provide guidelines to help therapists use process comments more effectively.

### CHAPTER EXERCISE

Describe your family of origin in terms of being disengaged or enmeshed (or for a few, both). Provide an example of how this was communicated to you. How do you think your family system of relating has had an impact on your most important current relationships? How has this affected both your ability to become personally or emotionally engaged with your clients *and* simultaneously provide clear boundaries?

### SUGGESTED READING

Hayes, J.A., Gelso, C.J., & Hummel, A.M. (2011). Managing countertransference. *Psychotherapy*, 48, 88-97. This helpful article offers different ways of thinking about countertransference and practical guidelines to help therapists manage their own.



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes.



# PART IV

## RESOLUTION AND CHANGE

### **CHAPTER NINE**

Working With the Process Dimension

### **CHAPTER TEN**

Working-Through and Termination





## CHAPTER 9

# Working With the Process Dimension

### CONCEPTUAL OVERVIEW

The three previous chapters helped therapists conceptualize their clients' problems and formulate treatment plans; we now focus further on the process dimension and how change occurs. Therapists need to empower clients by helping them develop new narratives for their lives that allow them to make sense of what occurred in significant relationships to shape who they have become and the problems they are having (Crits-Christoph et al., 1999). Unless clients develop self-narratives that are more realistic and fit closer with the good and bad news they have actually experienced in their attachment relationships, and are defined less by family family myths and roles, wishes and denial, and other distorting inaccuracies, it will be harder for them to guide where they are going in the future or to sustain changes after treatment has stopped. In this way, seeking a more realistic assessment of the formative experiences and interaction patterns that clients have experienced helps both clients and the therapist understand how they got here—where early maladaptive schemas and problematic IWMs developed and why current relationships are being framed and recreated in the way they are (Bowlby, 1988; Luborsky et al., 1997; Mallinckrodt, 2010). However, a consistent or primary focus on historical relationships and what happened in the past will not produce change and, too often, only serves to help clients avoid the anxiety of addressing current problems.

Change often occurs as clients first find new responses to their problems in their relationship with the therapist and, then, with the therapist's guidance, work through similar issues with others beyond the therapy room. To highlight the power of this experiential relearning over just intellectually interpreting, reframing, or explaining what something means, the attachment researchers put it best: You do not get an empathic child by teaching or admonishing the child to be empathic; you get an empathic child by *being empathic* with the child (Karen, 1998). The therapist's intention is to make this experience of change an ongoing pattern of interaction in the therapeutic relationship—one trial learning is not sufficient. As this occurs, the focus of treatment shifts to helping clients *generalize* this in vivo relearning to other arenas in their lives where, in parallel, similar themes are being played out and contributing to clients' presenting symptoms. Conversely, if the therapist and client merely talk about important issues and behavioral options, but the therapist does not actively help the client link these new ways of relating

with the therapist to other relationships where the same kinds of interactions are causing problems, change will be limited. Thus, the purpose of this chapter is to further clarify the process dimension and illustrate how therapists working within different theoretical orientations can use the therapeutic relationship to help clients change (Gold, 2011; Goldfried, 2004; Levenson, 2010; Sampson, 2005).

## RESOLVING PROBLEMS THROUGH THE INTERPERSONAL PROCESS

In this section, we see how clients' problems are brought into the therapeutic relationship, and how therapists can provide a new relational experience that does not go down old familiar lines. Putting these pieces together serves to introduce the next stage of treatment—providing a CEE and helping clients apply this relearning to other relationships in their daily lives. Following this overview, we will look at four extended case vignettes that illustrate implementation of these constructs.

### RESPONDING TO CLIENTS' CONFLICTS IN THE THERAPEUTIC RELATIONSHIP

Ongoing, problematic interaction patterns with caregivers give rise to developmental conflicts that clients have not been able to resolve on their own. Symptoms and problems develop as clients try to cope with difficult feelings and faulty beliefs by adopting fixed interpersonal coping strategies. Although these coping strategies once were necessary and adaptive, they are no longer necessary and create problems in many current relationships. The therapist's first task is to establish a collaborative working alliance with clients by making accurately empathic connections. This is consistent with the empirical literature that indicates that the therapist-client alliance is a highly significant predictor of treatment outcome, *more so than any theoretical orientation* (Barber et al., 2009; Norcross & Lambert, 2014; Norcross & Wampold, 2011). That is, the therapist's empathic attunement facilitates clients' self-reflection/perspective taking ability and their sense of agency (Allen, 2013; Bandura, 2006) as well as their motivation for engagement in treatment (Westin et al., 2004).

Once an alliance is established, client and therapist then work together to identify and change the maladaptive relational patterns that are occurring with others and in the therapeutic relationship, as well as the conflicted feelings and pathogenic beliefs that accompany them. As this work proceeds, the therapist's intention is to watch for and recognize the *pull* from the client to reenact these relational themes in the therapeutic relationship. The therapist's goal, then, is to respond differently than others usually have and change the familiar but problematic scenario—providing a different type of relationship that resolves, rather than reenacts, clients' cyclical relational themes (Beebe et al., 2005; Goldfried, 2004; Sampson, 2005; Wachtel, 2005). As therapists respond in ways that counteract the problematic expectations learned in the past, clients experience new ways of relating and begin to question or evaluate the costs and gains of their habitual coping styles. In this process, clients are empowered to choose more flexibly how they want to respond in current relationships, and to develop more realistic, affirming and compassionate self-concepts (Gilbert, 2014).

The therapist's primary role is not to give advice, explain, reassure, interpret, self-disclose, or focus on the behavior or motives of others, although each of these responses will be effective at times. Instead, the therapist encourages clients to take ownership of the treatment process by setting the direction for therapy (Miller & Rollnick, 2013). The therapist does this by helping clients identify the issues or concerns that clients feel are most pressing or important right now, and then joining or responding to their initiative.

The therapist continues to focus clients inward, reflects the core affective messages, links clients' maladaptive relational patterns with others to their own current interaction, and maintains a collaborative, working alliance by ensuring that clients are active participants who feel *ownership* of the change process. As clients' faulty beliefs, expectations, and coping styles recur throughout the narratives they relate, themes emerge that clarify the treatment focus and provide direction for the ongoing course of therapy (Barber et al., 2009). For example, the therapist will be able to do the following:

- *Point out when and how clients employ their interpersonal coping strategy, and begin to discern when it is helpful and with whom it is no longer useful.*
- *Help clients become aware of how they respond to themselves internally in the same problematic ways that others originally responded to them.*
- *Help clients explore why they become anxious at a particular time, what types of responses they tend to elicit from the therapist and others to manage this anxiety, and how they systematically avoid certain interpersonal modes or feelings.*

As we have seen, the client and therapist do not just talk about issues in therapy; they actually relive them in the therapeutic relationship. This occurs in three ways.

### BRINGING CONFLICTS TO THE THERAPEUTIC RELATIONSHIP

First, transference reactions bring clients' core conflicts into the current relationship with the therapist (Connolly et al., 2000). As clients' conflicts begin to emerge in therapy, they will become increasingly concerned that the therapist has been responding—or is going to respond—in the same unwanted ways that significant others have in the past. These fears and misperceptions follow from clients' cognitive schemas—influenced by developmental experiences, *which are most likely to occur when strong feelings have been evoked and clients feel distressed or vulnerable* (Quirk et al., 2015). In some cases, treatment will reach an impasse until the therapist can help clients differentiate the therapist's actual intentions from the problematic responses that clients expect and have in fact received in other important relationships. For example:

**THERAPIST:** As you tell me about this, what do you think I might be feeling about you?

**CLIENT:** Well, I guess I'm concerned that you might be a little disappointed in me.

**THERAPIST:** Disappointed? Oh, no, not at all. Actually, I was thinking...

Second, some clients will systematically elicit responses from the therapist that pull the therapist into reenacting their old scenarios or confirming their problematic expectations of others. When therapy stalls or ends prematurely, clients often have reenacted their generic conflict, evoking responses from the therapist similar to those they have received from significant others in their lives. This unwanted reenactment occurs when clients elicit responses from the therapist that are thematically similar to those they have received from others in the past. Clients elicit these familiar yet problematic responses from the therapist for different reasons—sometimes defensively, to avoid the internal aspects of their conflicts, and sometimes more adaptively, by testing whether they can obtain a better response to their familiar relational scenario than they have come to expect from others. For instance:

**THERAPIST:** How do others usually respond when you talk to them like you're talking to me right now?

**CLIENT:** They probably don't like it much—you know, everybody just gets pissed with me.



**THERAPIST:** I can see how that happens, and I have some ideas about how we could try to make that different here in therapy—in our relationship. I think if we could start talking about what’s going on between us and change how this goes in here, that would be a very helpful step to start changing this with others in your life. How does that sound to you?

Third, the way in which the therapist responds to clients—the interpersonal process—may unwittingly reenact their conflicts. As we have emphasized, clients’ relationships with their therapists need to enact or provide, as specifically as possible, different and more helpful responses to their conflicts than they have received in the past. However, it is not always easy to provide this CEE. Many clients will expect, and some will successfully elicit, similar types of problematic responses from the therapist that they have received from others. The task for therapists, then, is *to identify how their interpersonal process with clients may be reenacting some aspect of clients’ problems with others and, when this is occurring, to use a process comment to make this interaction overt as a topic for discussion*. The therapist then works together with clients (and with the help of a supervisor when the clients’ issues have evoked the therapist’s own matching or reciprocal conflicts) to establish a different pattern of interaction that does not repeat the old relational scenario. As we have seen, however, this idea is far easier to say than to put into practice. Consider the following exchange:

**THERAPIST:** I just disagreed with you—I’m wondering how that was for you?

**CLIENT:** Well, kind of different, I guess. I’m not used to people speaking up and disagreeing with me much.

**THERAPIST:** Yes, I can see why—you are so forceful when you speak. I hear you that you want to “communicate better” with your 15-year-old daughter, but it makes sense to me when she says that she feels “dominated” by you and reluctant to disagree or speak up. I’m a 45-year-old, educated professional, and you’re a challenge even for me.

**CLIENT:** We need to work on this, don’t we? I think this is what my wife has been trying to tell me for a long time.

When therapists enact a resolution of clients’ conflicts rather than a repetition of them, clients have received far more than just an explanation for their problems. They have experienced a meaningful relationship in which their old conflicts have been aroused, but this time they have found a better outcome than what usually occurs. They have been able to reveal themselves, to speak up or act more boldly, and ask for help or express a need without receiving the unwanted response they have come to expect. When this occurs in the context of a meaningful relationship, such in vivo or experiential relearning is powerful indeed.

Experiencing an interaction with the therapist that is incompatible with their early maladaptive schemas does not make up for the deprivations or disappointments that have shaped clients’ lives, of course. However, it does behaviorally demonstrate that change can occur—that at least some relationships, sometimes, can play out in a different and better way. This CEE begins to expand clients’ schemas for what can occur between people in relationships and broaden their interpersonal and affective range. For example, they can voice their opinion with some others without being diminished or interrupted; they don’t have to fear being left if they disagree or prioritize their own needs at times; and they don’t have to be considered selfish or arrogant if they feel good about an accomplishment, and so forth.

At this critical juncture in treatment, when clients are having a meaningful experience of change with the therapist and the therapeutic process is providing a resolution of

clients' conflicts, two important things occur. First, *intervention techniques from varying theoretical orientations all become more effective* (Ackerman & Hilsenroth, 2013). That is, cognitive, interpretive, self-monitoring, educational, skill development, and other interventions all can be used more productively by clients because the useful new content in these interventions is congruent with the corrective process they are enacting. Second, as clients have repeated experiences of change with the therapist, the therapist can help clients begin to apply this in vivo relearning to other arenas in their lives where similar conflicts are being enacted. This next period of therapy—transferring this experiential relearning that has occurred with the therapist to other relationships—is called the *working-through* phase of treatment and will be the focus of the next chapter.

With this overview of the change process in mind, let's now examine more closely how therapists can use the interpersonal process they enact with their clients to effect change.

### USING THE PROCESS DIMENSION TO FACILITATE CHANGE

To facilitate change, therapists are:

1. listening keenly to identify the core messages about Self (e.g., *It's weak to ask for help or need anything*) and Others (e.g., *They don't want to listen to you, they won't believe you*) that clients learned in childhood; and
2. sustaining a focus on providing unique responses that differ from and disconfirm these problematic expectations (e.g., *It's safe to share with some others, I won't always be controlled or criticized*).

Providing this in vivo or experiential relearning is the core component of change in the interpersonal process approach.

Within this context, interventions from other theories can be helpfully integrated. Following Rogers (1975), (and the Motivational Interviewing approach), it is essential for the therapist to listen empathically and work collaboratively. Also, it is helpful with some clients to suggest psychodynamic interpretations and facilitate client insight about the sources or development of their problems (Hill et al., 2007). Behavioral and cognitive therapists significantly help clients by training new relaxation, desensitization, and assertiveness skills; cognitively reframing situations; and suggesting new, more adaptive behavioral responses for clients to try between sessions. Each of these and other interventions will certainly be useful, but each can become more effective when the therapist is also attending to the interpersonal process that is being enacted with the client.

To help, we look below at four different case examples to illustrate how the process dimension can be used to facilitate change.

**Example 1: Reenacting the Problem—An Inability to Recognize the Interpersonal Process.** Therapists-in-training usually find it easy to understand the process dimension conceptually, and in group supervision, to observe when clients' problematic relational patterns are being reenacted with another therapist. It is, however, far more difficult for therapists to recognize how the interpersonal process they are enacting with their own clients may be reenacting the conflict rather than resolving it.

*Kareina, a 30-year-old mixed race woman, had been struggling with a generalized anxiety disorder and an eating disorder for the past 10 years. Bright, attractive, and extremely hard-working, she began treatment by expressing how much pain she was in as a result of her bingeing and purging. Kareina explained that when this symptom initially started, "it was my friend"—because the purging temporarily stopped the anxiety that simply became*

overwhelming for her at times. As a six-year-old child, she thought the anxiety “was going to make my head explode.” Kareina loved and respected her parents, but struggled intensely with both of them. Her Black father and Hispanic mother both had emigrated to the U.S. from Central America in their early teens—virtually without assistance or familial support. They made their way by “working the hardest” since they were just children to overcome real poverty and succeed in making a stable financial life for their family now. But their strong adaption that helped them succeed now held a big cost. Both parents, in their own different ways, were highly critical, demanding, and controlling of Kareina. They overtly demanded “perfection” in everything she did, and emotions elevated intensely if Kareina fell short in some way. First, either or both parents would express disappointment or frustration with her, and then withdraw in exasperation. Then, Kareina felt painfully responsible as her parents began to argue and berate each other’s parenting methods, which sometimes escalated to hitting walls or throwing dishes. Kareina would listen from her room and cry—feeling responsible and to blame for her parents’ intense conflict, and anxiously worried that the yelling and breaking might escalate to physical violence.

Kareina was deeply ashamed of her eating disorder and, even though she had tried to keep it a secret, people figured it out as her ongoing lethargy, health problems, and preoccupation about food and eating kept giving her secret away. Her mother had recently walked in on her in their family bathroom while Kareina was throwing up and “talked” to her that she “must stop this.” Kareina was distraught but she could not stop bingeing and purging. She decided to enter therapy and during their first session she told the therapist, “Everybody ends up disappointed in me, no matter how much I do. Like my best friend distanced from me when I told her I had an eating disorder, and now my boyfriend is stepping away, too “because I spend too much time in the bathroom.” And then I feel so alone and anxious, and just loathe myself. So that’s when I start purging every day again—and that’s my friend because it’s the only thing that stops the anxiety.”

The therapist was moved by the client’s plight but, unfortunately, lost the clarity of her boundaries and became overly invested in having her change. The therapist found it difficult to listen to how distraught and hopeless she sounded, and she began reassuring her that things would get better. The therapist disclosed that she had had an eating disorder of her own years ago and that she knew what Kareina needed to do in order to stop binge-purging. The therapist and client readily established a friendly rapport in which Kareina successfully elicited a great deal of support and encouragement from the responsive therapist. Kareina enjoyed this support greatly and sincerely tried to follow the therapist’s advice.

Things went better for a few weeks but then, as usual, Kareina “lost it” and began bingeing and purging again. Kareina didn’t feel comfortable telling her—fearing disappointment and disapproval again, and started arriving late for her sessions. Then, when she missed one of them, the therapist expressed that she felt “let down.” Kareina acknowledged that she had not been able to “maintain our agreement” (to not binge and purge until she called the therapist first) but had begun bingeing and purging again, secretly. The therapist felt frustrated, in part, because she had extended herself to Kareina more than she usually did with other clients. She felt that Kareina was letting them both down and told her so.

Unwittingly, the therapist had become “disappointed” with the client, as others had, and she induced guilt over the impact of her “counter-productive” behavior—which Kareina experienced as again being shamefully “imperfect”: (THERAPIST: After all our work together, I don’t know how you could do this without calling me first?). As she did with others, Kareina was contrite and tried to elicit her therapist’s support again. However, Kareina did not show up for her next appointment, and did not return to therapy: (CLIENT (thinking to herself): I’m just hopeless, I can’t do anything right. Everybody gets fed up with me, even this therapist).

This interpersonal process reenacted rather than resolved Kareina’s conflict in two ways. First, she initially elicited sympathy from the therapist, but the therapist’s

support eventually turned subtly toward control, criticism, and emotional withdrawal when Kareina could not perform and meet the therapist's expectations. This reenacted a familiar and painful scenario for Kareina that confirmed her pathogenic belief that she always let others down. Relationships don't sustain, conflict can't be worked out—the specific relational pattern that she had forewarned the therapist about during their initial session.

Second, the therapist benevolently tried to rescue this client, who was highly distressed and overtly asking for help and direction. From the beginning, however, the therapist took control of treatment and gave Kareina a program for what she “should” do to get better without asking this compliant client—who had received virtually no support for having her own initiative or voice in her life—for her ideas and suggestions for what would help. In addition to not working collaboratively, the therapist did not watch for points of entry when Kareina did act more assertively or independently in the session or with others, and respond affirmingly to these healthy expressions of her own identity. Thus, their interpersonal process paralleled the similarly controlling interaction that Kareina had received from her parents, who were well-intended, but could not serve as a Secure Base and support her own exploration, efficacy, and autonomy.

In reviewing this case several weeks later, the therapist reported, “The client just wasn't ready to stop her binge-purge behavior yet. She's going to have to get worse and really hit bottom before she'll be able to stop denying there's a problem and do something about it.” The therapist failed to recognize that the interpersonal process they had enacted in treatment unwittingly repeated the client's familiar but problematic interpersonal scenario and prevented change from occurring. As is often the case when treatment fails, the therapist attended only to the content of what they talked about (her eating problem) and not as well to how their process might be replaying aspects of the client's interpersonal problems (such as finding disappointment and blame when she failed to meet the therapist's expectation). How could this premature termination (discontinuance) been prevented? Following the sequence we keep highlighting, *the therapist could have first provided a safe haven by offering an empathic response, and then provide a secure base by making a process comment—to explore with immediacy and real personal intensity for both, the reenactment transpiring in their current interaction.* For example:

**THERAPIST:** I'm so sorry you had a setback and began binge/purging again. But my first concern right now is about you and me. It's seeming to me that you didn't feel safe bringing this to me—or were afraid I would respond in some unwanted way? Can we talk together about why it wasn't ok to tell me?

**CLIENT:** Well, I guess I thought you'd be disappointed...I didn't keep our agreement...

**THERAPIST** (graciously, genuinely): Uh huh, I'd be disappointed in you. Thank you, I'm so glad we're talking about this...

All therapists, beginning and experienced alike, will reenact aspects of clients' patterns and problems at times without realizing that this parallel is occurring. From an emotionally neutral vantage point—such as watching a recording of another therapist's session—it is relatively easy to see how the old pattern is being reenacted in the interpersonal process. In humbling contrast, it can be exasperatingly difficult for therapists to see their own interpersonal process while engaged in an intense, affect-laden relationship. It is challenging for new therapists to attend both to the content of what is being discussed and to the process that is being enacted as the discussion proceeds. Clearly, therapists-in-training are facing a complex challenge in their professional development here, and they need to be patient with themselves. With some practice and experience, however,

developing therapists will often find that they have the most potent impact when they adopt a process-oriented approach and become comfortable using these immediacy interventions (Spangler, Hill & Dunn et al, 2014; Kuutmann & Hilsenroth, 2012). Student therapists will benefit from further reading on this topic, rehearsal and practice with classmates, watching the demonstration videos provided here and by others, role-play and modeling from practicum instructors, and the guidance of a supportive supervisor. (See the practical guidelines and exercises suggested by Hill, 2014 (see chapter 14) and Angus and Kagan, 2007).

**Example 2: Process Comments Keep Therapists from Repeating Unwanted Patterns.** In the previous example, the therapeutic process reenacted the client's old scenario. In the next illustration, a similar dynamic begins, but this time the therapist recognizes the interpersonal process being enacted with the client and uses a process comment to realign their problematic interaction.

Many clients enter treatment because of depression. They feel sad, believe they are bad, and experience themselves as helpless to change their circumstances. In many cases, these clients communicate their very real suffering to the therapist with emotional pleas for help. However, with some Preoccupied and Fearful clients in particular, whatever the therapist tries does not work. The therapist's efforts to meet the client's request for help may be met with some resistant version of "Yes, but..." A problematic cycle begins: This client, who may be presenting with both anxiety and depression—perhaps an agitated depression—feels increasingly distressed and intensifies her plea for help. Some clients like these might even exclaim that they "can't go on" and express suicidal ideation in a hyperactivating attempt to get their departing partner to re-engage. In response, the concerned therapist becomes more anxious, more active, and tries even harder to find some way to help. However, nothing the therapist does has any impact or provides any relief.

As these clients reject the help that they actively have been eliciting, the therapist's own personal need to be helpful is frustrated. In a developmentally appropriate way, for example, the new therapist's own tenuous confidence or fledgling sense of adequacy as a helper may be threatened by these clients' responses—especially if the student therapist isn't feeling supported by his or her supervisor. Or, other therapists who were parentified in their family of origin may feel distressed about failing to fulfill their familial role and meet the emotional needs of others. When clients' eliciting behavior has tapped into the therapist's own personal issues in one of these ways, some therapists will work harder to please; others will withdraw and emotionally disengage; and a few will become punitive and critical toward the client. When any of these occur, clients do not experience a reparative relationship with the therapist.

*Pause now and think about your own response tendencies for a moment, and consider how you would tend to respond, both internally and behaviorally, to this mixed message from your client.*

Treatment will stall when the therapist remains stuck and continues to please, be critical of, or disengage from the client. The clients' conflict is reenacted as they again feel dependent on a relationship with someone who is critical, too easily controlled to be trustworthy, inconsistent or emotionally unavailable, or threatens to terminate the relationship. What is the alternative? A more effective intervention is for the therapist to make a process comment (metacommunicate) and invite a dialogue with clients about what might be going on in their present interaction. Rather than focus solely on the content of what they are talking about (e.g., depression), *the therapist can wonder aloud or tentatively inquire about how they seem to be responding to each other*. This is often the best way to effectively change or alter the reenactment. For instance:



**THERAPIST:** Let's talk about what is going on between us right now and see if we can understand what's happening in our relationship—it might have something to do with your depression. It seems to me that you keep asking me for help, but you also keep saying “Yes, but...” Maybe I over-react and get a little frustrated then, feeling that you don't allow me to help. What do you see going on between us?

With a process comment of this type, the therapist is inviting the client to be a collaborator and join in trying to understand their mutual interaction. The therapist is also providing interpersonal feedback and helping the client look at how his depression is expressed to and experienced by others. (This client recently said, “My wife used to be supportive, but it seems like she's giving up on me and kinda going away.”) As we will see, the process comment also allows the therapist to break the cycle, at least for the moment, of the client's escalating need and the therapist's increasing feeling of being stuck and unable to help.

How does this process comment help the client to change? In this moment, the client is experiencing a relationship with the therapist that provides a new and different response to his old relational pattern. The therapist has remained engaged—in this moment by inviting the client to jointly explore the impasse—and being receptive to the possibility that both may be contributing to it. The therapist's consistent availability, in the face of consistent rebuffs, differs from the sequence the client has often experienced in other important relationships: initial attempts to help, followed by subsequent frustration for the helpers who become critical and blaming, and ultimately ending in others withdrawing from him. It signals a sustained interest in the client and commitment to being a collaborative partner in understanding what is happening.

This is indeed a new and different experience for someone who typically expects, elicits, and then experiences abandonment or rejection at this juncture. The client has not experienced this consistent or steady engagement (without the therapist and others over-reacting on the one hand or giving up on the other), or the secure holding environment it provides, in past relationships. If this corrective stance toward the client continues and is repeated in many other ways—large and small—throughout treatment, the client is living out a resolution of his core conflict in the relationship with the therapist. When clients have this real-life experience of change, rather than passively or intellectually hearing advice, reassurance, or interpretations, they are empowered to begin making similar changes with others in their lives.

A key concept in the interpersonal process approach is that *the capacity for change increases as the therapeutic relationship becomes more meaningful*—that is, as the therapist becomes someone who matters to the client (see important discussions of the “real relationship” in psychotherapy by Gelso, 2011 and Gelso & Samstag, 2008). When an interpersonal solution is enacted in a valued relationship with the therapist, the therapeutic alliance is strengthened and change is facilitated in two ways. First, this new type of relationship provides interpersonal safety for the client (Weiss, 2005). The interpersonal safety of this new context permits clients to come to terms with the core conflicted feelings and pathogenic beliefs that accompany their maladaptive relational patterns (Westin et al., 2004). In other words, clients have the supportive relationship necessary to experience and integrate those feelings and situations that previously needed to be disavowed or split off as they were too threatening to be dealt with.

Second, the experience of change with the therapist *demonstrates* to clients, rather than merely tells them, that current relationships can be different than they have learned to expect. It does this by expanding their cognitive schemas for relationships and by altering their core beliefs about the possibilities that relationships hold for them (e.g., “I do

matter and can be cared about”; “I can say what I want and not be left”; “I do have the right to set limits and stop letting my children run over me”) Next, with the therapist’s assistance, clients can begin to transfer this relearning and establish better relationships with others along these more flexible and self-affirming lines. As an illustration, let’s consider the depressed client’s response to the therapist’s process comment:

**CLIENT:** Yeah, I do keep saying “Yes, but...” to you—like my wife always says I say to her. So this is all just pointless—you’re frustrated now, too. If I’m doing this with you, too, then there’s no way I’m going to get better. Maybe we should just forget the whole thing and stop now.

**THERAPIST:** Oh, no—that’s not what I’m suggesting at all. It makes sense that you and I are having a bit of the same problems in here together that you have with other people. In a way, it is a problem, but in another way, it gives us the opportunity to resolve your problem right here in our relationship.

**CLIENT:** How?

**THERAPIST:** If you and I can work out a better way of doing this in our relationship—you know, find a way to keep this old pattern from repeating in our relationship—I think it will go a long way toward helping you change this with your wife and others.

**CLIENT:** How can we do that now if we haven’t been able to up to this point?

**THERAPIST:** Well, I think it’s very possible. In fact, I think we are breaking that pattern right now, just by talking about the way we interact together. Tell me, how is it to be talking with me about our relationship and the way we respond to each other?

**CLIENT:** It’s different, but I like it. You’re pointing out something, but I don’t feel you’re being punitive.

**THERAPIST:** Good, I don’t feel punitive. And I like the way we are interacting, too. I feel like I’m working with you—and not being pushed away as I have felt before. And I think if you do this with others, it would be easier for them to remain engaged with you without getting frustrated or withdrawing. What do you think?

In this example, the therapist has used a process comment to effectively alter the current interaction with the client. For the moment, the process comment has kept their interaction from repeating the client’s familiar predicament. Some variation on the old “Yes, but...” pattern will probably soon reappear, and the therapist will have to make another process comment and work through a similar cycle again. However, with the process comment, the therapist is temporarily providing the client with a different response that does not fit very well in the client’s old scenario. If the therapist continues to find ways to provide these types of *counteractive or corrective responses* throughout treatment (i.e., remain engaged and not withdraw from the client), the client will experience a reparative relationship and, as we will focus on in the next chapter, begin changing this pattern in other relationships as well (Bridges, 2006). The next example illustrates a more macro-perspective that tracks the process dimension over the course of treatment.

**Example 3: The Process Dimension as a Means to Bridge Differing Theoretical Orientations.** There are many ways to help clients change, and therapists working from different theoretical orientations can all help. Every theoretical approach clarifies some aspects of clients’ problems, and every theoretical approach has limitations. Researchers from an integrative perspective find that *“master” therapists representing different approaches tend to act more like each other than like the less skillful adherents of their own treatment models* (Goldfried et al., 1998). Researchers also consistently find

that within each theoretical orientation, the primary component of change is not the theory *per se* (e.g., client-centered versus psychodynamic) but how effectively individual therapists apply it in their therapeutic relationships (Norcross & Lambert, 2014; Norcross & Wampold, 2011). That is, *some cognitive behavioral therapists are far more effective than other cognitive behavioral therapists and account for most of the positive treatment effect* in psychotherapy outcome studies. Furthermore, it is the relationship between therapist and client, and common factors such as empathy, genuineness, and positive regard, that are most powerful in treatment outcome (Luborsky et al., 1997; Norcross, 2011; Teyber & McClure, 2000; Wampold, 2015).

Decades ago, Kiesler (1966) identified this issue and called it the **uniformity myth**—*as if all therapists were equally effective and differences in treatment outcome were due to theoretical orientation rather than more and less effective therapists within each brand of treatment*. Kiesler and many others since have encouraged psychotherapy outcome researchers to study *within-group differences* (i.e., more and less effective psychodynamic therapists, and more and less effective client-centered therapists), where large differences in treatment effectiveness are consistently found. This is far more informative than continuing to focus on *between-group differences* (e.g., cognitive-behavioral versus interpersonal-dynamic treatment approaches), where only very small or no differences in treatment effectiveness are consistently found. In other words, *variability in outcome is primarily due to therapist effects rather than theoretical orientation* (Anderson et al., 2009; Okiishi et al., 2003).

Summarizing this research literature, Lambert & Ogles (2004) argue that *instead of continuing to try to identify empirically supported treatments we should be trying to identify empirically supported therapists*. In sum, regardless of theoretical orientation or treatment approach, change is likely to occur when therapists can establish a strong working alliance through empathic understanding and repair this alliance when it is ruptured (Angus & Kagan, 2007; Horvath, Del Re, Fluckinger, & Symonds, 2011; Norcross, 2010, 2002; Wampold, 2001). Working within any theoretical modality or treatment length, therapists will find that change is also more likely to occur when the therapeutic relationship provides a resolution of clients' conflicts rather than a repetition of them.

*After two years in analytically oriented therapy, Rachel still could not take charge of her life because her therapist, like her husband, engaged in interpretations that told her why and how she behaved the way she did. Rachel's response to the therapist matched her response to her husband—she took his views as “the truth.” This was exacerbated by her difficulty getting her children to do what she asked, fueling her sense of being incompetent. She also complained of her stultifying daily routine as a homemaker, yet she was never able to do anything to improve it. At her husband's insistence, she would periodically enroll in a class or interview for an office job but she seldom followed through on his suggestions.*

*In therapy, Rachel had spent many hours exploring her childhood. Her therapist was skillful in seeing unifying themes in the recollections that she shared with him. For example, her therapist astutely observed how she was subtly discouraged from initiating activities on her own as a child, and how she was not allowed to feel good about her success experiences. Any time she expressed an enthusiastic interest, tried out a new venture, or achieved something she was proud of as a child, her parents did not notice it—or, if they did notice, they somehow didn't seem very happy about it.*

*Rachel's therapist once explained to her, in a sensitive and nonjudgmental way, that she had a passive-dependent personality style. Rather than feeling labeled or put down, Rachel was impressed that her therapist seemed to understand her so well. He knew so many things about her without her even having to tell him. Although her problems had not changed much yet, she still believed her therapist would cure her. He was so bright and insightful, and she felt he genuinely cared about her. He didn't like to tell her what to do,*

*but when things became too much, he could usually help by explaining what the problem really meant. It was reassuring to be with him, and Rachel did not know what she would do without him.*

*It was Rachel's husband, Frank, who finally ran out of patience with the slow course of treatment. After two years, he was fed up with the unending therapy bills and his wife's unrelenting discontent. His wife's helpless dissatisfaction felt like an unspoken but unending demand for him to love her more, give her more, or somehow fill up her life. He was tired of these subtle, nagging demands and he wanted a change.*

*A man of action, Frank obtained the name of a cognitively oriented therapist on the faculty of a nearby college. A friend told Frank that this therapist was a "problem-solving realist" who could make things happen. This sounded like just the right approach to him. Frank insisted that Rachel stop treatment with her present therapist and begin with the new therapist. Initially, Rachel was sickened at the thought of leaving her therapist, but she sensed that Frank was truly at the end of his rope. Although she still believed in her therapist and felt loyal to him, she was afraid that Frank might actually leave her if she did not go along with this demand.*

*After her first two sessions with the new therapist, Rachel was surprised to think that perhaps Frank might have been right after all. The new therapist wasted no time in taking charge of the situation. It was actually encouraging to have the therapist outline a treatment plan with steps for her to follow, and tracking more concretely with a thought record. The therapist discussed a list of specific treatment goals with her and they planned a set of graduated assignments for meeting these goals on a scheduled timetable.*

*In their first hour together, the therapist had Rachel role-play how she responded to her children when they disobeyed her. Then, with the therapist serving as a model and coach, they rehearsed more assertive responses that Rachel could try with her children. The therapist also had Rachel enroll in an assertiveness-training class that the therapist was running for some other clients. Each week, Rachel was also to complete a homework assignment. For the second week, she was to call one new person she might like to get to know better and ask her to lunch. She was supposed to report back to the therapist on this assignment at the beginning of their next appointment.*

*Frank was encouraged by this practical, problem-solving approach to his wife's problems. He began to think that something might change after all. Rachel was surprised to find that she actually felt hopeful, too. She felt reassured by her new therapist's goal-oriented, problem-solving approach. In fact, Rachel became determined to make this therapy work, even though she had not wanted anything to do with it initially. She promised herself that she was going to try to do everything the therapist asked of her.*

*Therapy progressed well for the first few weeks, but things soon started to slow again. Without really knowing why, Rachel began to find it hard to muster the energy to attend the assertiveness class. She knew her therapist would be disappointed in her but she just couldn't seem to help it. Although she felt guilty and confused about it, Rachel began to come late to some of her therapy sessions. Over the next month, she began to re-schedule or miss them altogether and soon faded out of treatment. Unwittingly, this "take-charge" approach taken by Rachel's new therapist conveyed to her that she was incapable of independent action and that she needed his direction—as she did in complying with her husband.*

Both of these therapists failed to have a significant impact on Rachel's problems, even though the treatment approaches they used were seemingly very different. The analytically oriented therapist might attribute the unsuccessful outcome to the difficulty in restructuring this client's basic dependency needs. The cognitive-behavioral therapist might note that Rachel was not sufficiently motivated to change because she was receiving too many secondary gains from her help-seeking behavior. If we look at the interpersonal process that transpired between both therapists and the client, however, a different

picture emerges. These two therapists actually responded to Rachel in a very similar—and problematic way.

Let's look carefully at their interpersonal process. With both therapists, Rachel reenacted the same maladaptive relational pattern that she had with her children and with her husband. Her presenting problem was that her children ran over her and she could not make them listen to her. Her disturbing degree of compliance with her take-charge husband was a profound example of the same problem. Therapy failed because her pattern of being passive and compliant was reenacted with both therapists. Unwittingly, both therapists provided a **hierarchical relationship** in which she remained the passive helpee led by an “expert” who was in charge and going to cure her. This interpersonal process reinforced her pathogenic belief that the source of strength and the ability to solve problems did not reside in her but in the therapist or others. She was not a collaborative partner in her own treatment, so agency, motivation, and commitment to change were difficult to sustain (Horvath & Bedi, 2002; Rollnick & Miller, 2013).

In order for Rachel to change, she needs to experience a therapeutic relationship that behaviorally affirms her own efficacy. This would be a relationship in which she is actively encouraged to initiate what she wants to explore in treatment, invited to set personal limits and share control with the therapist, and is more overtly supported and encouraged by the therapist in making her own decisions and choices. The client's symptoms will visibly improve in the context of a new interpersonal process in which the therapist:

- *encourages her to act more independently in their relationship, and*
- *joins with her in collaboratively exploring the anxiety and guilt that arise each time she tries to act more assertively or independently with the therapist and with others in her life.*

This experience of change with the therapist behaviorally challenges her IWMs and shows her that her own strengths are valued and can be used, in conjunction with the therapist's skill and understanding, in a more collaborative and productive relationship.

Unfortunately, both of Rachel's therapists were comfortable with the hierarchical helper-helpee relational process and did not bring it up as a focus for treatment. For example:

**THERAPIST:** Can we talk together about how we are working together? I feel like we are interacting as if I am the teacher or leader and you are the student or follower. And I've heard this in other relationships you have talked about. I don't think this is good for us—I'd like for us to be collaborators—more equal partners in our work together. What do you think—what do you see going on between us?

Neither therapist offered the metacommunication that, in certain ways, their current interaction was reenacting the same pattern that was problematic for Rachel in other relationships. If the therapists had asked Rachel what *she* wanted and supported her efforts to express her own voice, this would have given Rachel a sense of efficacy. Such a process comment, tactfully and tentatively shared, would have allowed Rachel to begin looking at this symptomatic pattern in a supportive environment. More importantly, it would have given her permission to change her dependent and compliant coping style with the therapist and to begin expressing her own feelings, interests, and authentic voice *in their relationship*.



The sequence is often important: If Rachel can experience support for her own initiative first in her relationship with the therapist, she can then feel more empowered to generalize and begin to do this in her relationships with others. The new experience with the therapist delivers two very important messages: (1) that some relationships can be different (i.e., Rachel will not be abandoned, ridiculed, or ignored if she asserts herself and has a “voice” with the therapist—and perhaps some others as well); and (2) she is already succeeding and experiencing empowerment, now, by acting as more of an agent in her own life and future trajectory than she has before (Bandura, 2006).

What do these changes in the ways she interacts with the therapist have to do with solving the real-life problems with others that brought her to treatment? As long as the therapists were telling her what to do, Rachel could not set limits with her children, take a more assertive stance with her husband, become more aware of her own genuine interests and goals, or follow through and act on what she wanted to do. That is, the highly relevant content of what she discussed with both therapists (issues about autonomy, assertiveness, and her own identity) was not matched by the interpersonal process they enacted. *Unless Rachel has the actual experience of behaving as an active, equal participant in her relationship with the therapist, she will not be able to adopt this stronger stance in other areas of her life.* In other words, the process must be congruent with the content.

To enact a more egalitarian or collaborative relationship with someone like Rachel is not a simple task for the therapist, however. It requires therapeutic skill and thoughtful monitoring of the process dimension. In response to her life experiences, Rachel has become accomplished at getting her therapists, her husband, her children, and others to lead, direct, and take responsibility for her. If these therapists had encouraged her to initiate more in the session, and then focused together on exploring her reluctance to lead, Rachel’s core conflict would have emerged quite overtly in the therapeutic relationship. That is, as soon as either of the therapists invited her to follow her own agenda and bring up whatever she felt was most important to talk about, encouraged her to disagree with or express any dissatisfaction she may have with the therapist, or celebrated whenever she tested and tried acting stronger - more independent or assertive in the therapeutic relationship, Rachel would have become anxious. At that moment, either therapist could have acted on this point of entry and focused her inward on this anxiety so that they could begin exploring together what the threat or danger was for her to step out of her compliant and help-seeking mode (e.g., the belief that others would leave her if she wasn’t always nice, pleasing and compliant; that she would be acting like her demanding and dominating mother—of whom she was very afraid of being like; that she was being selfish or others would think she acting angrily if she spoke up for herself).

Simultaneously, either therapist could have given Rachel permission to act more assertively within the therapeutic relationship and responded to her in ways that facilitated this new behavior in their interaction together. The therapist could do this in two ways. First, the therapist could watch for and affirm this effective new stance whenever it emerged in their relationship or with others. Second, the therapist could respond to instances in which Rachel “undid” herself by retreating to the safer, nonassertive mode just after she had risked acting in a stronger way with the therapist. Given Rachel’s life experiences and interpersonal coping style, she was likely to become apologetic or confused, or to act dependently and ask for direction, soon after disagreeing with the therapist, making an insightful connection on her own, or redirecting the session more toward her own interests or concerns at that moment. If therapists have formulated working hypotheses about these propensities, they can track this potential reenactment in their interpersonal process. If so, they will be prepared to help their client identify this problematic pattern *as it is occurring*, and then explore the threat or danger she feels when she has just retreated from her new, stronger stance with the therapist. Let’s illustrate this immediacy intervention:

**THERAPIST:** You just took a risk with me, and acted stronger by disagreeing with me, and I want you to know that I liked that very much. But I also want to check in and ask: What do you think I might be going on for me after you just acted more assertively with me like that?

**CLIENT:** Uh, well, I'm not sure...(pause). Maybe you don't really like it, but you probably can't say that...so you could think I'm being a little pushy, like my mother, and you know I don't want to be that way.

**THERAPIST:** No, you were not pushy at all, and I genuinely liked it. I welcome you speaking up and saying more of what you think and feel with me.

**CLIENT:** Well, ok, but my father really did leave us - saying he couldn't stand my mother's pushiness anymore.

**THERAPIST:** I hear you, once there were very big consequences for speaking up—for saying what you wanted. Are you concerned I'll leave our relationship if you express yourself here?

**CLIENT:** I don't know...others have.

**THERAPIST:** I'm not going away, but others have in the past. And some others in your current life may not like it if you get stronger, but I think some others in your life will. We need to start sorting through who in your current life will support this, and who will punish it. But first, let's agree that anytime I think you are being too pushy or demanding, or you are worrying that something like that is going on, that both of us will stop right then and bring it up. That way, I can be responsible for my response, and you can say whatever you want without worrying about what's going on for me. How would that be?

**CLIENT:** Yeah, let's try that.

In most therapeutic relationships, therapists and clients will temporarily reenact clients' core conflicts in their interpersonal process—it just happens. However, in successful therapy of every theoretical orientation, the therapist and client do not *continue* to reenact the maladaptive relational pattern in an ongoing way. Instead, they are able to recognize and name this recurrent pattern (e.g., “going along”) and work out a different type of relationship that changes the course of the old familiar scenario. As Intersubjective therapists describe it, **deconstruct** or disembed from this enactment. Once clients find that their conflicts can be activated or come into play with the therapist, but do not have to result in the same hurtful or frustrating outcomes they have come to expect, their schemas and expectations expand and become more flexible. At this pivotal point in treatment, it is relatively easy for therapists to move to the next phase of treatment (i.e. the Working-Through phase) and help clients generalize this experiential relearning to other relationships in their lives.

#### Example 4: Resolving Sexual Abuse by Working with the Process Dimension.

Excessive performance demands often leave trainees feeling pressure to do something to make their clients change. Unfortunately, these internal pressures on the therapist are often translated into interpersonal pressures on clients to change, often before either therapists or clients know what's really wrong, let alone what they want to do about it. This is often evidenced by a premature emphasis on intervention techniques and usually at the expense of too little exploration and understanding. Unless the therapist has conceptualized what has gone awry for clients in other relationships and considered how these relational themes could be reenacted in their therapeutic process, intervention techniques will often fail. In contrast, if therapists have been *assessment-oriented* and asked themselves, “*What experiences has this client had to bring him to this place in his life?*” and has been able to generate two or three good working hypotheses about the interaction between themselves and the client—and the specific relational experiences that this client needs in order to

change—it is usually easy to find effective ways to intervene. Thus, therapists are encouraged to ask themselves repeatedly, “What does this mean?” rather than, “What should I do?” As we will continue to see, the second question is usually answered by the first.

Therapists can employ a wide range of techniques from different theoretical modalities (Wachtel, 2008; 1997). The key, then, is to observe closely clients’ responses to what they have just said or done (Curtis & Silberschatz, 1997; Weiss, 1993). That is, the therapist evaluates the effectiveness of each successive intervention in terms of clients’ ability to utilize each type of response to make progress in treatment. Based on this behavioral feedback from clients, *therapists need the personal flexibility to modify their interventions* to provide the responses that work best for this particular client (Binder, 2004; Teyber & McClure, 2000). Unfortunately, researchers find that many therapists do not flexibly alter their interventions to match clients’ needs, but dogmatically stick to the same approach whether the client is finding it helpful or not (Ackerman & Hilsenroth, 2001; Najavits & Strupp, 1994).

To illustrate the need to use our understanding to guide our interventions, we examine two critical incidents in the treatment of an incest survivor. In these incidents, the therapist uses intervention techniques that have a highly significant impact on the client: validating the resistance and role-playing. It is not the intervention techniques in themselves that facilitate change, however. Both interventions are effective because they follow from the therapist’s understanding of what is transpiring in the current interaction, and because the therapist then is able to work with these issues in the immediacy of the client-therapist relationship.

*Early in treatment, Sandy had confided to her male therapist that she was an incest survivor. Grasping this profound betrayal, he anticipated that trust was likely to become a central issue in their relationship. Therapy had gotten off to a good start but, before long, progress began to slow as the material that Sandy presented became repetitious. About this time, Sandy recounted two different narratives in which the relational theme was feeling unsafe with men. Based on this and other material they had been talking about, the therapist hypothesized that, without being aware of it, Sandy was using these vignettes to broach the topic of trust in their relationship. As their relationship was becoming more important to Sandy, it seemed that her deep concerns about safety and betrayal were now being activated with him.*

*Responding to these embedded messages about their relationship, the therapist began to talk with Sandy about trust between them and asked about the different thoughts and feelings she was having toward him. Sandy genuinely liked the therapist and was finding him helpful. However, when the therapist explored the trust issue and asked specifically whether she felt safe with him, her affirmative response sounded half-hearted and unconvincing. It soon became clear to both of them that Sandy was emotionally removing herself from the therapist as they talked more directly about her abuse and safety in their relationship.*

*The therapist responded to Sandy’s concern by affirming both sides of her ambivalent feelings and, used immediacy to work with her concerns directly in terms of their relationship.*

**THERAPIST:** I know that one part of you likes and trusts me, but it makes sense to me that another part of you doesn’t feel safe. I think that both sides of your feelings toward me are valid and important for us to work with.

**SANDY:** *(sheepishly nodding and gesturing vaguely to indicate that this was true)*

Because they had been talking about issues of trust and betrayal for some time and groundwork had been prepared, the therapist thought that this might be an opportunity to try to go further with these issues. Rather than trying to talk her out of these

concerns or convince her of his trustworthiness, the therapist validated Sandy's distrust and actually went on to articulate and develop it more fully:

**THERAPIST:** If I violated your trust in some way after you had taken the risk to ask me to help you, it would be very bad for you. If I tried to approach you sexually or foster any other kind of relationship between us, it would hurt you very much. Maybe it would even hurt you so much that you might not be able to risk trusting or asking for help again—from anybody.

**SANDY:** (*tearing, looks at the therapist and slowly nods in agreement*)

The therapist continued to elaborate Sandy's concerns about mistrust and exploitation from men in general within the immediacy of their relationship:

**THERAPIST:** If I took advantage of our relationship in some way, I think you would feel hopelessly betrayed. I think you might become very depressed again, enter into other relationships that would be hurtful or exploitative, and may even start thinking again that you do not want to be alive.

As the therapist spoke and made the concerns that they had been discussing with others overt in terms of their relationship, he observed that her entire demeanor changed. She remained tearful but became alert and present, solemnly nodding agreement with what he was saying. As they continued to talk together about this, the therapist went on to describe his own attitude, and reframe her distrust as a strength:

**THERAPIST:** Yes, I can see how much it would hurt you if I betrayed your trust, and no part of me wants you to have that wounding experience again. In fact, I respect the cautious part of you that isn't sure about trusting me. That distrustful part of you is your ally—it's a strength of yours. We need her—because she is committed to not letting you get hurt again. That's why it's so important that we go at your speed in here, that you have control over what we talk about, and that you can say no to me and know that I will honor your limits.

After the therapist responded so affirmingly to Sandy's concerns, and worked with this previously unspoken but dreaded expectation of betrayal in terms of their relationship, important changes occurred. Congruent with the literature that use of immediacy increases clients' willingness to explore more deeply, and also their ability to experience and tolerate more intense affects, Sandy began to share more of her story (Hill et al, 2013; Hill et al, 2008; Kuutmann & Hilsenroth, 2011; Mayotte-Blum et al, 2012). Whereas in the past Sandy had only alluded vaguely to her childhood abuse, she now chose to share it more explicitly. Over the next weeks, she recounted in painful detail and with strong emotions how she had been molested over a period of years by her stepbrother, who was 12 years older. When she first went to her mother for help, her mother was not supportive, denied that the abuse was occurring, and told Sandy to "never talk about anything like that again." Sandy did not even consider seeking protection from her stepfather, who had always been distant and unresponsive. After failing to receive help, Sandy recalled sitting alone on the floor of her closet for long periods with the door closed, feeling afraid, ashamed, and "out of it." Sandy said that this was the point in her life when she resolved that she was "always going to just be alone." In response to this deep sharing, the therapist was compassionate, validated her experience, and began to help her with the many significant connections she began to make between this familial tragedy and the symptoms and problems she was struggling with in her current life.

The therapist had already learned in their work together that role-playing was an effective intervention that Sandy enjoyed and found useful. During one of these sessions, the therapist talked with Sandy about using a role-playing technique to provide a different response to the abuse she had suffered, and Sandy welcomed the suggestion.

**THERAPIST:** I wish someone could have been there to stop him and protect you. No one was there for you but, if I had been there, I would have walked into your bedroom when he was there, turned on the light, and in a loud voice commanded, “Stop it! Get away from her and leave her alone right now! I see what you’re doing and it’s not fair. You’re hurting her, and I won’t allow it.”

After speaking this forcefully, as if he were actually saying it to the perpetrator, the therapist rolled Sandy’s coat into a ball. Using it as a little Sandy doll, he spoke to it reassuringly:

**THERAPIST:** You’re safe now, and he’s gone for good. I’m going to call the police and help protect you so you’ll never have to worry about him hurting you again.

As the therapist metaphorically gave Sandy the protective response that she desperately longed for as a girl but did not get, the full intensity of her isolation and shame was evoked. As these important feelings ran their course, Sandy became more composed and said, “I’m not all alone anymore. I’m going to be all right.” The therapist, still holding the rolled-up jacket tenderly, asked Sandy if she could join in and help him take care of this little girl who still needed help. Sandy’s affirmative response was almost joyous, and the therapist carefully tucked it in her arms.

**THERAPIST:** This is the little girl you were. She needs you to open up your heart to her and give her a home. You need to take care of her. Have compassion for her rather than shame—like I’m feeling right now. Do not push her away anymore like they did then—and you have done since. You need to hold her, talk to her, and listen to what she tells you. I want you to join me in taking care of this part of you, so this little girl is no longer alone behind the closet door.

Sandy readily accepted this responsibility and later bought a doll that she used to represent the part of her that needed to be cared for but had not been protected. She had always thought of herself as “unattractive” but went to considerable effort to find and purchase a doll for herself that she felt was pretty.

Sandy entered therapy almost 25 years after the abuse, anxious, unassertive, and almost incapable of leaving home alone. Just weeks after this session, however, she got her driver’s license renewed and began driving again for the first time in several years. She also got a job as a waitress—her first paid employment in six years. Although she had always foiled any type of success for herself, she enrolled in a local community college and began receiving As in many of her classes. Whereas Sandy had felt helpless, had characteristically acted as a victim, and had been repeatedly taken advantage of by others, she increasingly became more assertive and expansive. All of Sandy’s problems did not go away, of course, but long-standing symptoms were resolved and her life turned forward on a new trajectory.

What allowed Sandy to get stronger in these significant ways? Several factors made this role-play intervention effective. By using this technique, the therapist brought Sandy’s conflict into the therapeutic relationship and provided her with the reparative experiences she needed. In sharp contrast to what occurred in her family, this time Sandy experienced protection, validation, appropriate boundaries, and a supportive holding environment. This corrective experience with the therapist disconfirmed her pathogenic belief that she did not matter enough to be protected or looked after. The therapist’s compassionate response also helped Sandy shift from an identification with her parents’ rejection and dismissal of her—and her own resulting shame and self-loathing—to an identification with the therapist and his compassion for her (Neff, 2011). Eventually, this healthy new identification led her to be able to care better for herself and believe that she did matter and was not alone with her problems for the first time in her life. These far-reaching consequences were set in motion by the therapist’s affirming response to Sandy’s vulnerability. Even though it was enacted in role-play, the therapist provided Sandy with a CEE that played an important part in allowing her to become stronger and more capable of protecting herself.



In this section, readers have been developing their ability to “see” and think about the interpersonal process that is transpiring between themselves and the client. When therapists do not think their interpersonal process is corrective or facilitative, (e.g., Therapist thinking to herself: “I’m not liking this...we aren’t going anywhere...”) their best way of intervening and changing it is usually with a process comment. Let’s explore this more fully.

## USING PROCESS COMMENTS TO PROVIDE AN INTERPERSONAL SOLUTION

Now, with more exposure to process comments (also referred to as **immediacy interventions** in the counseling literature and metacommunication in the family systems literature) we can explore further how therapists can use them throughout treatment to facilitate change. Researchers find that immediacy interventions can be used to strengthen the working alliance, increase session-depth and discussion of more personal issues, help clients experience thoughts and feelings more fully, help therapists and clients restore ruptures and negotiate problems in the therapeutic relationship, respond to resistance, and facilitate other interventions (Hill et al., 2008; Hill & Knox, 2009; Hill et al., 2013; Kuutmann & Hilsenroth, 2011; Mayott-Blum et al., 2012; Muran et al., 2009). These Immediacy Interventions are powerful interventions that bring real focus and intensity, but they are not for the faint of heart and are commonly viewed as the most challenging interventions for new therapists to learn. As we have seen, process comments require therapists to break the social rules and intervene in the here-and-now by talking with clients about what may be going on between them in their current interaction. For example:

**THERAPIST:** I’m wondering how you are feeling about me, and about us, right now—since you have just risked sharing so much?

In this way, all process comments make the interaction between the therapist and client overt and put the relationship on the table as a topic for discussion. As we explore different types of process comments in this section, we are always keying on the distinction between the overtly spoken *content* of what is discussed and the *process* dimension of how the therapist and client interact. As new therapists begin to try out these process-oriented interventions with their clients, the most common challenge is to contain their anxiety and work through how process comments often conflict with familial rules and cultural prescriptions. For many trainees it was simply unacceptable or taboo to talk forthrightly with family members about their relationship and current interaction. Also, many trainees initially fear that it is too blunt or disrespectful to talk about “you and me.” The *way* we say it and the language we choose is very important (Wachtel, 2014). We have provided many process comments throughout the preceding chapters, in part, to illustrate that they can be respectful, collaborative invitations for straight talk, and should never be confrontational or intrusive in any way. Next, we (1) look at further guidelines for utilizing process comments effectively; (2) highlight different types of process comment; and (3) see how process comments can be used to make many other therapeutic interventions more effective.

**1. Guidelines for Utilizing Process Comments.** The first guideline to help begin using these powerful interventions is to *discourage trainees from using these types of process comments in their personal lives*. These immediacy interventions we are focusing on really are challenging to learn and apply in the beginning, although they will become second nature with a few years of practice and experience. Learning to work in this process-oriented way will be easier if you restrict process commentary to your clinical work. A clear boundary between your personal and professional life will help keep things simpler.

Second, many trainees inaccurately slot these discussions about process interventions into their own familiar but problematic experiences of being painfully confronted, put on

the spot, or intruded upon. Process interventions should never be confrontational, intrusive, or judgmental in any way. They are merely observations, tentatively suggested, about what *may* be occurring between the therapist and client. Therapists are judiciously using their own experience of clients to *tentatively wonder aloud about what might be going on between them* right now, and offer this as an invitation for further dialogue and mutual sharing of perceptions. As we have seen, process comments offered *tentatively* (“It sounds like...”; “I’m wondering if...”; “Maybe...”) are more effective than direct challenges (“I think you...”) (Miller, Benefield, & Tonigan, 1993). For example:

**THERAPIST:** As you’re talking about him and what went on between you two, I’m wondering if you might be saying something about our relationship as well. Has anything like that ever come up between us, too?

**CLIENT:** Well, yeah, you’re the therapist—you’re always in control of what goes on in here.

**THERAPIST:** Thank you for speaking up so directly. I’m glad we’re talking about this right now so we can change it. It feels like I’m the one in control here. That wouldn’t feel very good—tell me more about this.

It is anxiety-arousing for most new therapists to pose questions such as this and ask the client so directly about what *may* be going on between “you and me.” For most of us, it just goes against the grain of how we have been socialized to talk and interact. However, that simple process comment *revealed significant unspoken conflict between the therapist and client that otherwise would have continued, underground, to hamper treatment.*

Preparing clients for process comments during the initial session is important. Immediacy interventions don’t work so well when clients aren’t expecting or prepared for them. When process comments that ask about our relationship or current interaction come out of the blue, clients *are* often surprised. They can be taken off-guard and, unsure of the therapist’s intentions, may feel defensive—as new therapists often fear. We don’t want this to occur, and the therapist can prevent it by *educating or preparing clients* by talking with them about this straightforward way of interacting during the initial intake or first session. For example, *the therapist can establish a new norm by overtly redefining the social rules that govern how directly the therapist and client will interact:*

**THERAPIST:** One of the most helpful things about therapy is that you and I can change the social rules a little bit and talk more directly with each other than people usually do. One of the best ways you can help me to help you is to speak up whenever something I’m doing isn’t being helpful, doesn’t make sense, or just doesn’t feel right to you. How would it be for you to speak up with me like that and tell me if I was doing something that wasn’t working for you?

**CLIENT:** Well, kinda mixed. When I start talking to my wife or friends about how depressed I feel, I can tell they don’t really want to hear about it. They’re nice people but they feel uncomfortable. So I like what you say about straight talk between us, that’s what I need. But I’m a Christian and I was raised to be respectful...so it might be kinda hard for me be critical of you...

**THERAPIST:** I wouldn’t feel like you were criticizing me or being disrespectful at all. Actually, I’d feel like we were working together. If I were doing something that wasn’t working for you, I wouldn’t want you to get mad and go away, or be nice and go along. I’d want you to speak up and tell me so I could change it and be more helpful.

**CLIENT:** Well, OK, when you put it that way, it sounds good.

**THERAPIST:** Good. I think it will help our work if we can talk frankly about our relationship at times, check in about how you and I are doing together, or if I can give you feedback about something I’m seeing or thinking about. Do you have any other thoughts or concerns I can answer about us working together in this forthright way?

In addition to educating clients about the treatment process, therapists can also ease the transition toward more forthright communication about “you and me” by providing **contextual remarks** or prefatory comments that facilitate the bid for more open communication. For example, the first few times the therapist is going to make an immediacy intervention, he or she can create safety for clients by *offering an introductory remark that acknowledges the shift to a different type of discourse*:

**THERAPIST:** May I break the social rules for a minute and ask about something that might be going on between us?

Clients will respond well to process comments when therapists respond in a respectful manner, share their observations tentatively (e.g., *Sometimes I find myself wondering if...*); invite collaboration (e.g., *That’s what I’m seeing—what do you think?*); and provide these types of contextual remarks (e.g., *I’d like to speak more forthrightly about this than people usually do, would that be OK?*).

These contextual comments help clients understand the therapist’s good intentions as they help clients shift from surface conversation to a more straightforward approach that invites deeper personal engagement. Rather than being threatened by this invitation, most clients will welcome and be reassured by this bid to talk about what may be occurring right now.

To work effectively with the process dimension, therapists also must balance the challenge of metacommunication with being *supportive and protective of clients’ self-esteem* (Kiesler, 1996). Process comments, like any other interventions, can be made in blunt, accusatory, or otherwise ineffective ways. Warmth, tact, curiosity, and a sense of humor can all go a long way toward making every intervention more effective. Finally, therapists also can *check in* with their clients and talk about the process intervention they just made. Consider the following:

**THERAPIST:** I’m wondering how it was for you when I just asked how you were feeling about our work together? I know people don’t usually speak so directly about what’s going on between them. Your thoughts?

**2. Different Types of Process Comments.** Therapists can utilize different types of process comments or Immediacy Interventions to achieve different goals. Let’s consider several.

Donald Kiesler, a wise pioneer in this work, discusses “therapeutic impact disclosure.” With this type of process comment, therapists use their own immediate experience of clients—or what is going on between them right now—to *provide feedback* about how a client’s way of relating is affecting the therapist (and likely others, too) (Kiesler, 1986; Kiesler & Van Denburg, 1993). Here, the therapist is disclosing selectively chosen feedback about the impact the client is having at this moment. For example:

**THERAPIST:** I feel like I’m having a hard time getting to know you—like there’s a wall or distance between us. What do you think, have you ever heard anything like this from others too?

**CLIENT:** Ya, others have said that—I don’t let people get too close.

**THERAPIST:** Help me understand get closer and know you better?

**CLIENT:** I’m just not very open, I guess, it’s always been kinda hard for me to trust people...

Often, others in clients’ lives don’t want to give such feedback that could help clients, fearing it would “hurt their feelings” or “make them mad.” Therapists don’t want to avoid interpersonal feedback that could help with problems in clients’ lives, and we can do this with sensitivity to clients’ self-esteem. Providing contextual remarks, asking the client for permission to say something or making a prefatory comment that the therapist

would like to say something that the client could take the wrong way—make all the difference. For example:

**THERAPIST:** Right now, Ruben, I'm feeling something that might help us understand what's going wrong with some other people in your life. But I'm afraid if I start talking about my experience and offer this feedback, you could misunderstand my good intentions and feel blamed—which I absolutely do not want to do. So, may I have your permission to speak forthrightly and offer some feedback about how you affect me sometimes, and maybe others as well?

Object relations and communication theorists use the term “metacommunication” to provide feedback about their current interaction, especially as a way to *register the unspoken emotional quality of a relationship*. For instance:

**THERAPIST:** I could be wrong about this, Pat, but I have a feeling I would like to check out with you. Sometimes I feel that if I don't always take your side completely, you won't say anything to me but you'll feel offended—as if I've been disloyal. Like, if I say it sounds like your husband has a good point there and that seems like a reasonable thing for you to consider, you'll walk out the door at the end of the session and I won't see you again. Is this just me or is there something to it?

Here, as always, the challenge in providing interpersonal feedback is to provide contextual remarks that cue clients what you are about to do, make your good intentions overt, and check in with clients afterwards and ask how this was for them. These simple steps allow therapists to provide honest feedback and be helpful to clients without being disrespectful or blaming—as it may have been in other relationships.

Another way therapists can use process comments to give clients useful interpersonal feedback that others are reluctant to provide is by acknowledging embedded messages. This type of process comment makes overt the unspoken subtext or relational statement about “you and me” that may be present in what was just said. For example:

**CLIENT:** People are so critical. If you say anything to them, they just start telling you what you should do.

**THERAPIST:** “People” are critical, and they tell you what to do too much. I can see why you wouldn't like that. You know, I'm wondering if something like that has ever gone on between us. I wouldn't want to be critical or tell you how you should live your life, but maybe I've made a mistake and done that without realizing it. Please be honest with me—has there been a time when I've done that? If so, it would help our work together if we could talk about it.

**CLIENT:** Well, by the look on your face, I could tell that you were really thinking I did it wrong.

**THERAPIST:** I'm glad we're talking about this—I think there's been a misunderstanding. I don't know what the look on my face was that you were referring to, but no, I wasn't thinking you did it wrong. How about this: The next time you see “that look” on my face or think I'm disapproving of you, would you stop right then and tell me so we can sort it out?

Therapists can find safe ways to make these relationally-defining messages overt and approach, rather than avoid, such important embedded messages about “you and me,” rather than acting as if nothing important is occurring. Process comments can also be used to provide interpersonal feedback to clients that *highlight contradictions, incongruities, or mixed messages in what the client just said*, or help clients recognize discrepancies between what they are saying and doing. For instance:

**THERAPIST:** You are telling me such a sad thing, Paul, but you are saying it in an off-handed way—as if it doesn't really matter. Help me understand the two different messages I'm getting—such a heartbreaking story being told in a half-interested manner?

The challenge in providing interpersonal feedback, and for all process comments, is *the way we say it*. Tone of voice and nonverbal gesture often communicate more than our words. New therapists also will find the interpersonal feedback they are providing clients will be more helpful and effective if offered *with a bid for the client to join and collaborate in refining it to make it more accurate*. However, doing so may collide with an interpersonal coping strategy that many trainees bring to their training program: to be “nice,” and *avoid potential interpersonal conflict*. In order for many trainees to integrate process-oriented interventions into the way they work, instructors and supervisors will need to provide understanding and support for working through this common countertransference propensity.

Let’s make an important distinction between the commonly used intervention, Self-disclosure, and a seemingly similar process-oriented intervention, Self-Involving Statements, which are actually quite different. Whereas self-disclosing statements refer to the therapist’s own past or personal life experiences, (e.g., **THERAPIST:** *My father did that, too...*), self-involving statements express some of the counselor’s selected thoughts or filtered emotional reactions to what clients have just said or done. Self-disclosing comments often take clients’ focus away from their own experience and shift it onto the therapist, which is especially familiar and problematic for clients who were Parentified or have a Preoccupied attachment style. In contrast, *self-involving statements keep the focus on clients and reveal information about what is happening in the relationship or how something clients have said or done is affecting the therapist*. For example:

**THERAPIST:** No, I don’t feel “judgmental” about what you did with him Saturday night, but I am feeling concerned about your safety and how you put yourself in situations where you could be hurt.

Using self-involving statements to share certain personal reactions to what clients have just said or done conveys personal involvement and resonance, and highlights relational patterns that are causing problems with others. Consider the following:

**THERAPIST:** Right now, Bob, I’m feeling uncomfortable. You’re talking to me in a loud voice right now—which seems very angry. How do others usually react to you when you do this?

Therapists offer clients a gift when they provide such invaluable interpersonal feedback which others have shrunk from sharing. Our intention is to find supportive, noncritical ways to help clients see themselves from others’ eyes, learn about the problematic impact they have on others (such as regularly making others feel intimidated, overwhelmed, or bored), and function better by developing greater reflective capacity (Fonagy, Gergely, Jurist, & Target, 2002).

Use-of-Self is another type of process comment that help therapists work with resistance, and they can make overt unspoken problems or ruptures in the therapeutic alliance—giving therapists the opportunity to sort them through and resolve them (Safran, Muran & Proskurov, 2009). Indeed, *the best time to use process comments in general is when the therapist is feeling that things are not going very well*. For example, Use-of-Self is a helpful option for trainees when they feel the interaction has become repetitive or “stuck,” or treatment has lost its focus. Therapist judiciously use their own personal experience in the moment as a way to intervene and help. Consider the following:

**THERAPIST:** (*thinking to himself: I don’t think this is taking us anywhere...so much happened last week—but we’re not going anywhere today...*). I’m feeling a bit lost right now. I’m not quite sure where we are going with this, or maybe I’m not getting what’s most important for you here. Can you help me out?

**CLIENT:** Yeah, I’m just wandering all over the place—I can’t keep my mind on anything lately. Maybe I’ve been more anxious than I’ve realized since...



As therapists become more experienced and confident, it will be much easier for them to take the risk of “not knowing” and explore more open-endedly what may be occurring in the client-therapist relationship. When feeling confused or stuck, for example, more confident therapists can thoughtfully use their own subjective experience to formulate and share their questions and concerns—filtered parts of their own internal dialogue—with clients. For example:

**THERAPIST:** I felt like we were really engaged—focused and working well—at the beginning of the hour. But now it feels like something has changed—as if I lost you somewhere. Is this my misunderstanding, or maybe you’re feeling something’s not quite right either? What do you see going on between us?

With such Use-of-Self, trainees often have the best opportunity to make their most significant interventions. To illustrate:

**THERAPIST:** Right now, I’m wondering if you might be thinking that I, too, want you to “just forget about this and let it go”—like your mother said when you told her that your uncle had molested you.

**CLIENT:** Well of course you do—isn’t that what therapists always do—just tell you to forgive and forget or you’ll never get over it!

**THERAPIST:** No, I’m not thinking anything like that at all. I support this strong part of you that doesn’t want to give in to the denial and go along with the pressure to stop disrupting the family...

It can be a profound experience for clients when therapists are able to disconfirm their misperceptions in this way. As we see here, such Use-of-Self often gives therapists the opportunity to provide a far-reaching CEE.

**3. Process Comments Facilitate a Variety of Interventions.** These here-and-now immediacy interventions that address “you and me” also help therapists utilize many other interventions more effectively. Yalom (2005) captures this best by *calling process comments the “fuel cells” that make diverse interventions more potent*; others would call them a “moderator variable” that shape the effectiveness of different interventions. Let’s look briefly at how this occurs with several of the interventions introduced previously.

Process comments facilitate empathy and help repair ruptures (Mayotte-Blum, et al., 2012). Early in treatment, therapists are trying to establish a working alliance with clients, primarily through accurate empathy. To provide this, therapists’ intention is to repeatedly give clients the experience that, yet again as they talk about the next issue or experience, “My therapist *gets* me.” It’s not so easy to do this, however. Accurate empathy is much more than a “simple reflection” where the therapist basically restates what the client just said. Clients don’t like hearing very many of those. Empathy is far more challenging to provide. It is a “complex reflection” that discerns the key concern, primary feeling, or basic meaning in what clients just said. To be able to capture this consistently, therapists need help, and process comments provide it. Rather than thinking of empathy as a personal characteristic, therapists can approach empathy as a *collaborative interpersonal process* between the therapist and client. To illustrate:

**THERAPIST:** As I listen, it sounds like you felt unimportant, almost erased, when he said that. Am I getting that right, or can you help me say it better?

Accurate empathy helps establish the working alliance, but the therapist’s empathic stance routinely falters as “ruptures” in the working alliance, or simple misunderstandings,

commonly occur (Safran et al., 2009). Unfortunately, researchers find that clients often do not voice their concerns and bring up the problems they are having with their therapists and, in turn, therapists often avoid or do not ask about them (Hill, Thompson, Cogar, & Denman, 1993). Unacknowledged and unresolved misunderstandings between client and therapist are not benign; they undermine the working alliance and lead to poor treatment outcomes (Johnson, Taylor, Tzanetos, Rhodes, & Geller, 1995). Rather than ignoring the conflict, or responding in kind with blame or disengagement, therapists can use process comments to provide a relationally affirming way to address the problem as it is occurring. The goal is to neutrally observe, adopt a curious stance, or tentatively wonder aloud about potential problems or misunderstandings that may be occurring. For instance:

**THERAPIST:** Right now, it seems to me as though you keep asking me for direction, but I keep trying to get you to lead and then join what you initiate. Let's work this out better—what do you see going on between us?

Treatment stalls unless therapists and clients can talk about problems in their relationship and sort them through. Working them out as they arise also provides a valuable social laboratory where clients can learn how to address and resolve conflict with others. However, most new therapists find it far easier to be supportive than to inquire about potential conflict between “you and me” or try to sort through problems directly. As with their clients, many trainees did not learn to address or find that they could resolve problems in their families of origin where, too often, *there was no secure mechanism to restore the ruptures that occurred with their own attachment figures.*

Process comments also provide a way to respond to strong feelings. We have encouraged student therapists to invite and overtly welcome the full intensity of whatever feelings the client is experiencing into the immediacy of the therapist-client relationship (Greenberg, 2014; 2002). Often, the most significant way therapists can respond to clients' strong feelings is through Use-of-Self and self-involving statements that include selective disclosure of personal reactions. For example:

**THERAPIST:** I can see how much this has hurt you, and how sad you're feeling. I feel connected with you right now, and hope you're not feeling alone as you said you have felt in the past. How is it for you to risk sharing this special part of yourself with me?

**CLIENT:** I am sad, but this is different. I'm not alone with it ...it's not so overwhelming.

Responding in this new and different way to clients' feelings is often one of the powerful ways for therapists to provides an impactful CEE (Bridges, 2006).

In addition, actively exploring and trying to understand the current interaction between the therapist and the client also facilitates work with transference or schema/IWM distortions. To illustrate:

**THERAPIST:** How do you think I am going to react if you decide to do that? What am I going to be thinking or feeling, privately, toward you?

**CLIENT:** Well, I know you're a good person, but I know that inside you will have to be judging me...

Early in their training, it seems unimaginable for many new therapists to ever pose such a question. However, process comments are powerful *uncovering* techniques. They highlight clients' IWMs—their pathogenic beliefs about themselves and faulty expectations of the therapist and others—which helps to clarify the treatment focus. For most trainees, the slowest skill to develop is learning how to develop a treatment focus—it is

complex and initially ambiguous. However, process comments that inquire about clients' perceptions of therapists' thoughts and reactions will reveal clients' key concerns and help both therapist and client discern what's really wrong (Bennett et al, 2008).

Finally, let's highlight how process comments help bring about the CEE (Berman et al, 2012). A unifying theme in all of these process-oriented interventions is to *link clients' issues and concerns with others to what is currently going on between the therapist and the client*. In this way, clients' problems with others are not just talked about abstractly; they often are re-experienced and potentially resolved in the real-life relationship with the therapist. In turn, this in vivo or experiential relearning with the therapist empowers clients to begin changing how they are responding to similar problems with significant others in their lives.

To illustrate, suppose a client continues to relay narratives where the basic relational theme is that he is repeatedly getting locked in control battles with others. To begin resolving this client's problem with others, the therapist's intention is to find an effective way (not blaming or confrontational but tactful) to make this control issue overt and put it on the table for discussion. The therapist does this by wondering aloud or tentatively inquiring about this possibility, so the therapist and client together can begin to explore it directly in terms of their relationship.

**THERAPIST:** Does this type of control battle that you have been describing with others ever get going between us, too? Does it ever seem like you and I are jockeying over-control in our relationship?

Yes, making such a process comment and bringing clients' concerns with others into the here and now can seem a dreadful prospect to new therapists. However, by judiciously taking this risk, the therapist can bring real-life immediacy to therapist-client interactions. Problems with others are not just being talked about intellectually; instead, for the first time perhaps, they are being addressed honestly and constructively with someone *as they occur*. If therapists wait for their own sense of good timing (e.g., the therapist finds herself feeling: This might work right now), the potential for change is at hand.

New therapists need examples of addressing and sorting through conflict with clients, so let's consider a range of three progressively more challenging responses to the therapist's query about control issues in their relationship.

### RESPONSE 1: A DISCONFIRMING RESPONSE IS LEAST CHALLENGING

An example of a disconfirming response from the client might be:

**CLIENT:** No, I don't feel like we are in a control battle—that's not going on here.

Per client response specificity, that would be enough for most clients. However, because control has been a problem in so many other relationships for this client, the therapist doesn't want to settle for just a superficial understanding, but explores further. For example, the therapist can use the client's response to begin exploring what is different about their relationship.

**THERAPIST:** Good, I'm glad that's not a problem for us. Any ideas about what makes our relationship different?

The client's answer often provides useful information:

**CLIENT:** You treat me with respect; that's what's different.

Honing a treatment focus, the therapist can follow up on this comment by exploring the client's concerns about not being respected in other relationships—a significant

relational statement and point of entry. The therapist also can build on it to establish that mutually respectful relationships could be developed with some others in his life as well. Further, the therapist is modeling for this client how he can find more effective ways to talk with others when he feels that an unwanted control battle is coming into play.

## RESPONSE 2: CLIENTS MAY AVOID THE IMMEDIACY OF THEIR CONFLICTS BY DISCOUNTING THE THERAPIST'S RELEVANCE

**CLIENT:** We're not in a control battle because this isn't a real relationship. You're just my therapist.

Clients make far more significant gains in treatment when this (common) distancing defense is addressed and resolved. At this point, it is essential to clarify that the therapist and the client really are two people who have been having a relationship for some time, even though there are constraints on their relationship (e.g., **Therapist:** *Yes, I am your therapist, so in many ways our relationship is different from a social one. I'm wondering, though, what about this makes it not feel like a real relationship.*). The therapist can then begin to explore the threats aroused for the client by more meaningful involvement. For example:

**THERAPIST:** If this relationship did feel more "real" to you, what would be different between us...or what might go wrong here that goes wrong with others?

**CLIENT:** Well, then I might have to worry more about what you might need or want from me.

The therapist and the client can then agree to watch for these newly highlighted concerns that come into play in the client's daily life, and address them if they arise in their relationship. It is usually more effective if therapists do not bring up these relational patterns in the abstract (e.g., at the beginning of the next session because the therapist has been thinking about them) but wait until they think the client might be experiencing them, right now - in their current interaction, before addressing them.

## RESPONSE 3: CLIENTS MAY RESPOND TO THE THERAPIST'S QUESTION AFFIRMATIVELY

**CLIENT:** (*in an exasperated tone*) Of course you're in control! You insist that we stop at 10 to the hour, whether I'm finished or not. And you're subtle about it, but I see you trying to take control all the time and make things go where you want.

Many new therapists fear such an "accusing" response and may eschew process comments to safeguard against such criticism. Too often, student therapists take clients' comments at face value and believe that they have done something wrong or have been too controlling if the client disapproves of them in this way. Perhaps the therapist actually has been too controlling—this possibility must always be considered and "owned" if necessary. If, upon reflection, it still seems that clients' reactions have more to do with their own cognitive schemas than they do with the reality of how the therapist has responded, however, rather than posing a problem, this actually provides an important point of entry.

Openly acknowledging that the same conflict clients have with others is also occurring right now with the therapist creates the opportunity to explore this issue directly and begin working together to try and resolve it in their relationship. Why do we keep returning to this same point? As long as this client is feeling controlled by the therapist, the therapist is not going to succeed in helping the client change this problem with others either. However, by utilizing the therapeutic relationship as a social learning lab, therapists can

use their interactions with clients to begin changing this maladaptive pattern or schema in the following ways:

1. By remaining non-defensive and accepting the validity of clients' concerns whenever possible:

**THERAPIST:** Yes, we do have to stop at 10 to the hour, and that is an unnatural ending for you. I can see how the control issue is brought up by those time constraints.

2. By exploring clients' perceptions further and working with them to understand them better:

**THERAPIST:** What do you think is going on for me when I try to take something we talk about in a certain direction? What does it seem like I am trying to do at those times—what might my intentions be?

3. By “differentiating” the therapist from other figures in clients' lives:

**THERAPIST:** Yes, I have had ideas about where we should go, but I have also been interested in following your lead, too. In fact, I may be different from some other people in your life because I genuinely liked it when you disagreed with me last week and told me what you thought. I like it when neither of us feels controlled and we can both say what we want. It makes our relationship feel more engaged and alive to me. And how is this for you?

4. By offering to be sensitive to this concern in their relationship, expressing a willingness to handle the issue differently in the future, and inviting clients to tell the therapist right then whenever they think this conflict is occurring between them—so together they can talk it through and change it before continuing:

**THERAPIST:** I don't want you to be controlled in our relationship as you have been in others. That's no good for you or anybody else. Let's try to do something about it. From now on, I will watch the clock and let you know when it's five minutes before we have to stop. And any time you feel like I am directing you away from where you want to go or being controlling in any way, tell me, and we'll stop right then, sort through what's happening, and change it. What else could we do to help with this? Tell me your ideas.

Adopting these approaches will work easily with some clients, whereas other clients will insist on reestablishing the same constricted relational patterns. However, if therapists are willing to remain non-defensive and tolerate their own discomfort for a few moments, most clients will become significantly more engaged in the therapeutic relationship and motivated to explore this and other related issues more fully. As clients repeatedly find that the expected but unwanted old scenario does not repeat with the therapist, they experience greater **interpersonal safety** than they have known before. As a result, important new issues, pathogenic beliefs, and previously threatening feelings are now safer to emerge, providing new material for exploration and clarifying the treatment focus. Routinely, *many clients will make attempts during the next week to try out this new way of relating with others and test out what they have just experienced with the therapist.* Clients often will return to the next session or two and report on their attempts to change these same types of patterns and respond in a different way to someone in their life. Next, we explore several reasons therapists are reluctant to work with the process dimension and offer some practical guidelines to help therapists take the leap and begin utilizing process comments with their clients.



## THERAPISTS' INITIAL RELUCTANCE TO WORK WITH THE PROCESS DIMENSION

*Karen, a first-year practicum student, was seeing her first client. Everything her client talked about seemed unimportant—she just rambled on about nothing. Karen felt bored, and even though she knew inside that the client also felt like this wasn't going anywhere, neither one wanted to say anything. A few sessions went by like this and Karen's awkward, bad feeling just got worse. She stopped enjoying their sessions and almost began to dread them, knowing that nothing meaningful was occurring and feeling like she was failing. Karen developed a knot in her stomach as she sat politely with the client and worried to herself: "Being a nice, empathetic person who just listens isn't going anywhere. Maybe I'm in the wrong field—I'm not sure I can do this..."*

*Karen's supervisor listened to her distress and tried to help. He suggested that Karen talk with the client about what she was feeling and check things out—maybe the client was feeling the same way, too? For example, the supervisor suggested that Karen reflect aloud on their interaction, and he tried to show her how by role-playing things she might say, such as:*

*"Sometimes I'm wondering if what you're talking about feels like what's really most important for you";*

*"How does it feel for you to talk with me—or others—about problems or personal things that are meaningful for you?"; or*

*"What's it like for you to come each week and talk with me? What feels good and what isn't being helpful?"*

*Karen knew that this is what she should do, but there was absolutely no way she could bring herself to say things like that. She couldn't be so forthright; she couldn't do that with her client or with anyone—she just couldn't. Karen had never talked this way in her family or with any of her friends, and felt torn apart about by the possibility that she might hurt her client's feelings by being so direct, or not know what to do next if the client did respond.*

*Karen's client didn't show up, or call to cancel, for their fourth session. Karen wasn't surprised and thought this was coming, and felt like she had failed, knowing that this could, and should, have been so different. Her supervisor was kind, and offered a suggestion—and a challenge. He said that Karen could try calling the client and inviting the client to come back and attend just one more session, to talk about what did and did not feel good about coming, to consider other ways they could work together if the client wanted to continue, or to have the opportunity to say goodbye, end their relationship, and get another referral if the client did not want to continue. The hope of being able to restore the relationship outweighed her anxiety, and Karen called the client and said almost the same words her supervisor had role-played with her. Karen hit the issue on the mark when she said they could talk about other ways of working together. The client asked, "What do you mean by that?", and Karen spoke up directly and said she was often quiet in their sessions and the client might benefit if she was a more active therapist and provided more feedback. The client responded immediately, saying Karen was "a very nice person, but she did need more help." The client agreed to attend one more session, and they continued this forthright discussion about how they could work together more effectively. Karen was more active, forthright, and engaging in this session and the client decided to continue in treatment. Karen felt hugely relieved after the session and, for the first time, had the good feeling that she had been a helpful therapist.*

In this section, we explore why some beginning therapists, like Karen, may be reluctant to make process-oriented interventions and talk with clients about their relationship

and interaction together. Although the best vehicle for effecting change is often the therapeutic relationship, it can be anxiety-arousing for new therapists to take the plunge and say, for example:

**THERAPIST:** Does that kind of problem—feeling ignored or dismissed, ever go on here, too—you know, between us?

Talking together so directly about issues between two people breaks the cultural norms and family rules that most therapists grew up with. As a result, and because of the immediacy that such forthright communication creates, some beginning therapists are reluctant to make process comments in the beginning and explore how clients may be experiencing parallel problems with the therapist that they are having with others (Spangler et al, 2014). Let's examine six concerns new therapists often have about beginning to use immediacy Interventions.

**1. Uncertainty about When to Intervene.** Because new therapists have not had much experience attending to the process dimension or tracking it with clients, they are often unsure when or how to use process comments. As trainees gain more experience and begin to integrate all of the complex new information their clinical training presents, most will become more comfortable intervening with the process dimension. The usual sequence is, at first, therapists will find that they are beginning to recognize when something important is going on between them, but they cannot speak up and apply what they are observing. Although this is often frustrating for new therapists, developmentally, it's a good place to be. Typically, if they can "see" the process dimension, it is often just weeks or months later until they will feel confident enough to begin speaking up and inquiring about this possibility with clients. It is something of a personal risk for therapists to suggest:

**THERAPIST:** I'm thinking about something that might be going on between us right now, and wondering if...

Although therapists will be accurate with some of their process observations, the client will not resonate with others. Therapists have not failed in any way or made a mistake when the observation they have tentatively suggested is inaccurate; they are simply trying to understand, and clients usually appreciate these good intentions. As emphasized earlier, the therapist's aim is not to be "right." Instead, he or she is trying to initiate a mutual collaboration or dialogue with clients—so that together they can consider and explore what may be going on between them. For instance:

**THERAPIST:** (*non-defensively, in a friendly and welcoming tone*) OK, what I'm suggesting doesn't quite fit. Help me out. What are your words for what might be going on between us here?

New therapists should not be in a hurry to make process comments or use other interventions that create immediacy until they feel ready to do so. Forcing such responses will not be good for therapists or effective for clients. If therapists do not feel in control of choosing how and when they intervene with their clients they are disempowered and, in turn, will not be capable of empowering their clients.

How can therapists discern when clients' conflicts are being reenacted between them? Let's follow this sequence. First, it is helpful if the therapist has tried to identify clients' maladaptive relational patterns with others. By formulating working hypotheses about what goes wrong with others—and how those same patterns or similar themes could come about

in their interaction—the therapist is better prepared to respond to what a particular client is likely to present in treatment. Thus, when therapists can anticipate the types of expectations, schema-driven distortions, and reenactments that a client is likely to enact with them, therapists do not have to think on their feet as much. They can better see what is going on between them or hear what the client is really saying—right now as the client is saying it, rather than “getting it” later (e.g., a trainee reviewing a tape recording of his session might think to himself: “She’s telling me that ‘everybody’ judges her. She’s probably telling me that she’s worried that I’m judging her and being critical, too. Why do I get it now, when it’s too late, and not right then when she’s saying it to me? This always happens!”).

Second, these working hypotheses will help therapists make sense of their own experience and recognize when they are starting to feel or respond as others do with this client. For example, when clients are leading the therapist to feel bored, impatient, or overwhelmed, these working hypotheses will help the therapist consider the possibility that he or she is beginning to react as others in the client’s life often do—that is, that his or her feelings and reactions toward the client may be signaling that the same scenario that causes problems with others may be underway here, too.

Third, this is a good time for the therapist to consult with a supervisor and check out together what may be occurring in the therapeutic interaction. If it does seem that a reenactment may be underway, supervisees can explore any concerns they may have about making this interaction overt the next time it comes up and talking it through with clients. Looking for effective ways to explore this possibility, therapists can also role-play or rehearse alternative responses with the supervisor. Again, it is not realistic for therapists-in-training to be able to explore and try out these process-oriented interventions without active guidance from a supportive supervisor.

Fourth, with this preparation, therapists can wait to broach this possibility with clients the next time they feel this reenactment may be occurring. *It is more effective to intervene at the moment when the interaction is occurring (immediacy) rather than bringing it up in the abstract* (e.g., at the beginning of the next session). Recall the therapist earlier who realized, after the session, that his client likely was feeling judged or criticized by him as well. He did not start the next session by bringing up this possibility and talking with the client about it—as he wanted to do. Instead, he listened to his supervisor’s advice to wait until he thought this issue might be occurring between them again, which did indeed happen about 20 minutes into their next session, and successfully inquired about this possibility *as it was occurring*—which holds so much more salience and meaning for the client.

Fifth, beginning therapists are encouraged to wait until they feel that this intervention (or any other) is likely to work. We need to respect our own sense of timing. As emphasized, it is important that therapists *choose* whether to make this or any other intervention, and when to try it—compliance is just as problematic for therapists as it is for clients.

It will often take a year or two before these responses feel natural and come easily—but they will. Good advice for the novice therapist is to practice and rehearse with classmates, ask your instructors/supervisors to role-play these immediacy interventions with you, and watch demonstration videos—it’s very different to see rather than just read about them (see Spangler et al., 2014 for further guidelines).

**2. Fear of Offending the Client.** Some beginning therapists are concerned that clients will feel therapists are being too blunt, personal, or intrusive if they ask, “How does it feel to be talking with me about this?”

Speaking respectfully yet forthrightly about what may be going on may break unspoken social rules, and many trainees are concerned that clients may be surprised or taken back by this. To help ease the transition toward this more genuine dialogue, the first few times that therapists make a process comment they can create safety by offering an introductory remark that acknowledges the shift to another level of discourse:

**THERAPIST:** Can I be forthright with you and speak directly about something?

Therapists will not be too blunt, take clients off guard, or offend clients if they respond respectfully, invite collaboration (e.g., “*What do you think?*”), and provide contextual remarks that help clients recognize therapists’ good intentions. Rather than being threatened by this invitation for more forthright communication, most clients appreciate this more authentic and substantive discussion. Welcoming it from the beginning, they are reassured by the therapist’s willingness to get down to business and talk in plain terms about what’s really wrong in their lives—something they have often wanted but not been able to do with others.

**3. Therapists’ Own Insecurities and Countertransference Issues.** To work with clients in the highly personal manner described here, therapists are challenged to take the risk of engaging in a genuine relationship with clients. In the interpersonal process approach, therapy is a two-way street where each participant is willing to be affected by the other. Process comments lose their impact when therapists are not willing to be emotionally present and engaged in the “real relationship” with the client (Gelso, 2014, 2011; Gelso & Samstag, 2008). Clients receive a contradictory or mixed message from therapists when therapists make a process comment and invite a more genuine dialogue, but do not follow through. This mixed message occurs when therapists become defensive and distance themselves from the client or when therapists are unwilling to consider the potential validity of the client’s criticism or dissatisfaction. For example:

**THERAPIST:** What do you think as I say that to you?

**CLIENT:** Well, I feel like you’re criticizing me a little bit there, and I guess it doesn’t feel very good.

**INEFFECTIVE THERAPIST:** (*defensive tone*) Well, of course I’m not being critical—geez, you really are sensitive to feeling criticized! Do you overreact like this to other people, too?

In contrast, when therapists are willing to remain non-defensive and talk with clients about their participation in problems and what is occurring between them, sessions become more intense and productive—and most clients appreciate the genuineness and honesty. As an alternative to the previous therapist’s reaction, a non-defensive therapist might respond:

**EFFECTIVE THERAPIST:** I’m sorry it feels like I’m critical—that sure wouldn’t feel very good. I don’t want to be critical in any way, but I do want to give you feedback in a way that’s not judgmental. So, thanks for being honest with me, it’s helpful, and please tell me more about the part that felt “critical.”

Therapists’ willingness to be personally affected by clients and to look at their own contribution to their interpersonal process will at times be anxiety-arousing. We wouldn’t want it to be otherwise—nothing ventured, nothing gained. *An impactful CEE does not occur unless the relationship is significant and holds real meaning for both the client and the therapist.* By looking honestly at their own contribution to problems or misunderstandings in the relationship, therapists facilitate an egalitarian relationship that holds genuine meaning for both. Of course, this increasing mutuality will activate therapists’ own personal problems or countertransference issues at times (Gelso & Hayes, 2007; Hayes,

Gelso & Hummel, 2011; Mitchell, 1993). For example, in order to respond more authentically, therapists need to be able to relinquish hierarchical control over the relationship, which will arouse anxiety for some therapists who need to be the authority, in control of what happens next, or one-up in relationships (e.g., Dismissive therapist Marmarosh et al, 2014). Therapists may also be concerned that this genuine responsiveness or emotional presence with clients will lead to a loss of appropriate therapeutic boundaries and result in over-involvement or acting out. However, by consulting with a supervisor and applying the guidelines on optimum interpersonal relatedness from Chapter 6 and countertransference in Chapter 8, therapists will be able to recognize when their own countertransference issues are prompting them to become too close to—or too distant from—the client.

In making process comments, how do therapists know whether what is going on is due to client-induced or therapist-induced countertransference? For example:

**THERAPIST** (internally): Is this just me, or does she make everybody feel this way?!

First, new therapists need a supportive supervisor who helps them track the therapeutic process, especially in the beginning. It is unrealistic to expect trainees to be a participant-observer who can maintain a balance of emotional relatedness and objectivity without ongoing assistance. This is a normative, developmental issue in clinical training. Although new therapists should be thinking about this distinction and working diligently to clarify what is the client's issue versus what is their own, they should not feel badly if they are having trouble distinguishing this in the beginning. It is complex and ambiguous, and it becomes easier as you learn better to recognize your countertransference propensities, so let your supervisor help you.

Second, beginning therapists can safeguard against confusing their own and clients' issues by tracking and observing the therapist-client relationship, but not jumping to conclusions or intervening as soon as they think something significant may be occurring. It is more effective for therapists to generate working hypotheses first, and then wait to see whether the observation also applies to subsequent exchanges between therapist and client. *If the issue or concern is relevant for clients, it will repeat as a theme or pattern. If it does not keep coming up, then it is likely that therapists' own issues are involved or that it is not an important concern and this hypothesis should be discarded.* Thus, therapists don't wish to be cavalier about making process comments. When in doubt, it is best to wait, gather additional information about the therapeutic process, and/or consult with a supervisor before raising issues with clients. Remember, too, that all process comments are simply observations. They are offered to the client *tentatively*, as possibilities for joint exploration and mutual clarification—not as truth or fact.

Across varying treatment approaches, there is strong empirical evidence that the therapist's relationship with the client may be the most significant means of effecting change (Barber et al., 2009; Najavits & Strupp, 1994; Norcross & Beutler, 2011). At the same time, one of the reasons for treatment to terminate prematurely is that clients' interpersonal coping strategies have activated therapists' own conflicts and they have become stuck in a reenactment. Therapists always need to remain open to the possibility that their own countertransference issues may have come into play, and should generate working hypotheses about how each client's conflicts could be reenacted in treatment (see Appendix B, Part 6). Likewise, beginning and experienced therapists also honor the therapeutic enterprise when they:

- *make a lifelong commitment to exploring and remaining open to their own countertransference propensities,*
- *consult with supervisors and colleagues as an ongoing career activity, and*
- *seek their own personal therapy when countertransference issues persist.*



Countertransference issues are most likely to create problems when clinicians disregard them. *The red flag goes up when clinicians are not willing to consider their own participation or potential contribution to the conflict.* In contrast, therapists who are aware that they are susceptible to countertransference and who discuss possible instances of countertransference with supervisors as they arise should feel unfettered about working with clients in a process-oriented manner.

**4. Concern about Appearing Confrontational.** The most common problem we find in helping new therapists explore and try out process comments is they misconstrue them as *confrontations*. Many new therapists are reluctant to make a process comment because they anticipate that clients will be mad about being “confronted,” view the therapist as hostile or accusatory, and will leave treatment. *There should be nothing confrontational about a process comment or any other immediacy intervention.* A process comment is an invitation from the therapist to the client for a dialogue about what may be going on between them right now. If you think that the process comment you are about to make is going to make a client feel accused or blamed, or that it will put the client on the spot, then don’t make it. Instead, wait for another time that feels better or, preferably, talk with the client about your reservations about sharing this observation. For example:

**THERAPIST:** There’s something I’m thinking about right now, but I’m hesitant to talk about it with you. I suppose I’m concerned that you might feel criticized or blamed—confronted in some way, which is not my intention. Can we talk about this?

Some therapists may also worry that responding more forthrightly, accurately reflecting the key concern or core message, or capturing the central feeling in what clients just said, may hurt clients. These expectations of client vulnerability are often inaccurate, and reflect therapists’ own countertransference issues about becoming more effective, having more impact on clients, or acting stronger. Virtually all clients will welcome the invitation for a more authentic, forthright dialogue about what’s really wrong in their lives and about what’s going on between the therapist and the client. In fact, we believe that one reason why clients drop out of treatment prematurely is because they don’t get this straight talk and direct personal engagement with the therapist. As clients find that it is safe and helpful to speak with the therapist about what goes on between them at times, new therapists will be reassured that they can be both sensitive and direct at the same time.

A different concern about hurting clients—and being hurt, takes on far more personal significance for some therapists with a Fearful attachment style who grew up with **double binding family communications**. Confusingly, there was a great discrepancy between what was actually being done (e.g., the child routinely being ostracized, ridiculed, threatened or hit and what was being said and presented to others (e.g., “We are a close and loving family. There are no problems, everyone is fine.”). The essential element with these discrepant messages is the clearly understood but unspoken family rule that the child cannot acknowledge the incongruence in any way. For example, the child in such a double binding situation *cannot make these contradictory messages overt* by metacommunicating and saying, “You’re telling me to clean my plate, but later you’ll make fun of me for being fat. Stop it—you’re driving me crazy!”

Such double binding communications occur in many dysfunctional families, and are routine in substance-abusing and in physically and sexually abusive families. An enduring legacy of such mystifying developmental experiences (i.e., “What’s real?!”) is that, years later, these therapists still may be intensely afraid that if they break the family rule and speak directly about what is behaviorally transpiring, something very dangerous will result—even though they cannot quite name the threat. Old threats can be evoked—but also resolved—by breaking double binds and no longer complying with or being ruled by

covert family rules about how members must communicate. Thus therapists with these developmental experiences may find it threatening (or liberating as they work toward earned security) to begin using process comments and make overt what is occurring. However, such countertransference issues are most appropriately resolved in therapists' own counseling, rather than in the supervisory relationship.

**5. Feeling Inadequate: “What do I say next?”** Some developing therapists recognize that process comments enliven the therapeutic interaction, bring clients' feelings and problems into bolder relief, and make them accessible for the therapist and client to work on in the here-and-now. But this good news now poses a new problem for an inexperienced therapist—as expressed in the following questions:

“*What do I say next?*” (when clients share strong feelings)

OR

“*What do I do now?*” (when clients begin talking about a significant problem in more depth).

With little training and experience, understandably, novice therapists are in a new and challenging position at these important moments. Uncertain about how to proceed or what to say next, some new therapists step away from process comments to avoid these anxious moments where they don't know what step to take next and help the client move forward or make progress, and feel they are failing. However, putting on the brakes and remaining on the surface by avoiding interventions that will bring forth clients' feelings and problems with more clarity and immediacy is not the way to cope. Not knowing what to do or say next is a developmental stage in clinical training, and as we will illustrate next, it is resolved as new therapists get better at developing a *treatment focus* (Tufekcioglu & Muran, 2015).

As supervisors, we can still recall decades ago when we were trainees struggling with these same anxious feelings. We feel empathy for our trainees, and we'd like to focus on two topics to try and help. First, when supervisees are saying, “What should I do next?”, they usually are really saying, “I don't understand what this means.” So, rather than tell our supervisees what to say next, which doesn't empower them, we first try to help them understand what clients just said. This will give them clear direction for what to say or for where they are trying to go. For example:

**SUPERVISEE:** I don't know what to say right here.

**SUPERVISOR:** What do you think the client just said to you? What might that have meant?

**SUPERVISEE:** I don't really know...what do you think?

**SUPERVISOR:** I heard the client saying—in so many words, that she was afraid of being left—afraid of being alone. She mentioned that her sister is joining the army and moving to Georgia. That's a significant loss for many teen-aged girls—but a far more profound loss for this girl. As you recall from the intake we discussed, her mother overdosed and died when she was 10. So right here, I'm hearing this client telling you that the painful and far more significant loss of her mother is also being evoked by her sister moving—and that combined it's just too much. What do you think?

**SUPERVISEE:** Yes, I didn't connect that bigger but unspoken part about her mother in the session, but as you say it now, that's exactly what's going on. But what should I have said? This will probably come up again.

**SUPERVISOR:** Yes, I think so too. I think we are trying to hear what was most important—most meaningful to the client, in what the client just said. If we can grasp that, usually by considering the broader life context or related themes, then that's what we want to respond to.

**SUPERVISEE:** Yeah, but what would you say?

**SUPERVISOR:** I would try to reflect all of her feelings of loss, but in a modest or graduated way. And things always work best when we approach them with empathy. So I would say something like: “Your sister is joining the army and moving far away from you. And it sounds like you don’t know when you will be seeing her next. It makes sense to me that you are distressed about this, about this, especially because you have already lost so much.” How would something like that work for you?

**SUPERVISEE:** Yeah, that gets right to the heart of what she’s really telling me...it is her sister—but this conversation is also about her Mom. It makes perfect sense now, but why can’t I hear this on my own? It’s so frustrating!

**SUPERVISOR:** You’re doing fine. In a year or two, as you keep learning how to listen for themes and develop a treatment focus, you will be much better at hearing what your clients are saying to you as they are saying it.

In their next session, the supervisee waited until the client brought up her sister’s move again, made a response similar to the supervisor’s suggestion, and the client moved into her deep and unresolved sadness, *and her previously unacknowledged anger*, over her mother’s death. Greatly advancing treatment, the client also expressed her shame-based belief that she was “worthless...unlovable” because her mother “chose drugs over me.”

The supervisor was able to hear the core meaning in what the client said (attachment trauma), which led simply and clearly to what the supervisee wanted to say and do next. The supervisor was able to provide this fuller understanding because she could **contextualize** what the client just said (i.e., think about the meaning of her sister moving within the broader developmental context of having lost her mother). Knowing that developmental experience, the supervisor was attentive to themes of loss that this client might present—and the supervisor had highlighted the importance of this theme to the supervisee before their first session. Understandably, however, learning to listen for relational patterns, affective themes, and trying to understand the client’s presenting problem within the broader developmental context of her life is a new way of thinking that takes most therapists two or three years to learn and apply. As this conceptualizing ability develops, student therapists will get better at refining a treatment focus, hearing the core concern in what the client just expressed, and knowing what they want to say or do next—just as this supervisee succeeded in doing.

Second, when supervisees ask, “What should I say next?”, we find that they often have useful questions and apt observations in mind that they don’t consider using. Drawing more on Use-of-Self, we often ask supervisees what they are thinking or wondering about as they are listening to the client and feeling stuck or uncertain of how to proceed. It just doesn’t occur to many supervisees to try and find a way to utilize the observations or connections they are making as they are listening to clients. Although a few may be too quick to speak without filtering their own responses, *the much bigger problem we see is student therapists being overly inhibited with clients and too reluctant to use their own experience and engage clients with their own thoughts, questions, and perceptions*. This Use-of-Self is much different than being directive, self-disclosing, or problem-solving.

Finally, as developing therapists become more experienced and confident, it will be easier for them to take the risk of “not knowing” and explore more open-endedly what may be occurring in the client-therapist relationship. When feeling confused, for example, a more experienced therapist may be able to inquire simply, “How did we end up here, anyway?”

**6. Concerns about Owning Personal Power.** A pervasive but generally unacknowledged issue in clinical training is therapists’ concerns about owning their own personal power. Many new therapists feel uncomfortable about allowing themselves to become

someone who is important to clients and having a significant impact on their lives. Especially in the beginning, it is also anxiety-arousing to make strong interventions that can influence clients profoundly and to accept the responsibility that comes with exercising such personal power. As a result, many beginning therapists over-qualify their comments and water down the impact of their interventions.

Therapists' own effectiveness can arouse anxiety for many reasons. Some clinicians, for instance, were parentified or aggrandized as children. For these therapists, legitimate competencies can be readily exaggerated into unrealistic all-powerful or all-responsible grandiosities. Initially, as children, it may have been exciting to be special and powerful *vis-à-vis* their parents in this way, but it soon becomes lonely, intimidating, or burdensome. As a result, these therapists may undo, or quickly retreat from, strong interventions they make or may avoid them altogether.

Some developing therapists are reluctant to have a significant impact on clients because of their own separation anxiety or separation guilt. For these therapists, competent or independent functioning in their families of origin threatened their emotional ties to parental caregivers (i.e., they lacked a Secure Base). As the young child began to explore independently and move toward greater autonomy, caregivers may have looked sad or hurt (instilling separation guilt), withdrawn physically or emotionally (instilling separation anxiety), demanded more or belittled the child's attempts at mastery (instilling feelings of inadequacy and shame), or otherwise paired anxiety with healthy competence and individuation strivings. Therapists for whom these types of developmental experiences were ongoing or characteristic may retreat from their own effective interventions, and frequently may be heard making comments like, "I don't know what to do," "I'm afraid of hurting the client," or "I'm so screwed up myself that I have no right to try to help somebody else." All therapists have their own personal problems and limitations. However, when such comments persist, it may indicate that therapists want to avoid the anxiety evoked by effectively addressing clients' problems—in the same way that effectively differentiating from their attachment figures and demonstrating competence and independence was (and is) anxiety-evoking because it violated unspoken "rules of attachment."

What can help trainees with these concerns? An affirming supervisory relationship is crucial if therapists are to embrace their own personal power and be as effective as they can be. The supervisor-supervisee relationship is most productive when supervisees:

- *feel personally supported by the supervisor,*
- *receive conceptual information and practical guidelines when needed,*
- *obtain nonjudgmental assistance in sorting out their own conflicted reactions toward clients that have been triggered by clients' eliciting maneuvers or by therapists' own personal issues, and*
- *are able to address and resolve the interpersonal conflicts (restore ruptures) that are likely to arise in the supervisor-supervisee relationship.*

Ideally, supervisors will help supervisees evaluate the effectiveness of interventions and consider alternatives without taking away supervisees' own initiative. Supervisors do best when they collaborate actively so therapists feel ownership of the treatment process. (See the Motivational Interview literature for the powerful effects of using this collaborative approach (Miller & Rollnick; Miller & Rose, 2009, 2013; Moyers et al., 2005)).

It may take several years before therapists can allow themselves to have as significant an impact on clients as possible and to possess their own personal power fully; this is a gradual developmental process. If student therapists do not progress along this feeling-of-adequacy dimension as they move through their training, they should discuss this issue with their supervisors as they go through their training program and/or seek treatment for themselves.

## CLOSING

This chapter has drawn primarily from process-oriented concepts in the group therapy, family therapy, and existential psychotherapy, and from the counseling literature on Immediacy interventions. As developing therapists begin to work in other rewarding treatment modalities such as couple counseling, group therapy and family therapy, attending to the process dimension will become even more central. In all three modalities, the therapist is sharing observations about the way clients are interacting (their interpersonal process) rather than just the content of what they are discussing (e.g., **Marital Counselor:** *You two are arguing about who takes out the garbage, but I'm wondering if the argument is really about who has the power to make household decisions or tell the other what to do. What do you two think – would it be helpful to talk about that more directly?*). As in individual therapy, process comments that describe the current interaction with the therapist, or what is transpiring between group members or family members, can be used to identify and change relational patterns.

The underlying assumption in all interpersonally oriented treatment approaches is that the therapeutic relationship will come to resemble other prototypic relationships in clients' lives. Clients' problems will emerge in the therapeutic relationship, especially if therapists track the process dimension and attend to how their on-going interaction together could be similar to and reenacting, or different from and reparative, what the client has experienced in other important relationships. If therapists are willing to make process comments and work with clients' problems in the immediacy of the therapeutic relationship, they can offer clients a genuinely meaningful, real-life experience of trust, empowerment, and change.

## CHAPTER EXERCISE

Imagine you had a situation where someone you trusted and cared about was going to give you feedback that would be difficult to hear but was intended to help you. How would you like this benevolent person to bring up and talk with you about that feedback? What could that person do that would both make it easier, and more difficult, for you to be able to consider and discuss it non-defensively?

## SUGGESTED READING

We suggest that straight talk and immediacy interventions facilitate more helpful relationships, and invite trainees to read further on this topic (See Clara Hill's (2014) *Helping Skills*, chapter 14). Simultaneously, we encourage clients and therapists to develop self-compassion for the struggles and challenges they face on this journey toward self-awareness and change. See Kristin Neff, *Self-compassion*, (New York: HarperCollins, 2011).



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## CHAPTER 10

# Working-Through and Termination

### CONCEPTUAL OVERVIEW

*Working-through* and *Termination* are two distinct phases of treatment. As clients continue to change dysfunctional patterns with the therapist and find that familiar but problematic scenarios do not reoccur in this relationship, they begin to generalize this experience of change beyond the therapy setting. Based on the experience that emotional needs can be met and conflicts can be resolved in the therapeutic relationship, clients begin to explore how they can make the same types of changes in other relationships as well. This working-through phase of treatment is an exciting period of growth and change *as clients try out with others the emotional relearning that has occurred with the therapist*. As progress is consolidated in this stage, treatment evolves to a natural close.

Next and finally, the termination phase provides an opportunity to re-experience and further resolve conflicts, internalize the helping relationship the client has had with the therapist, and successfully end treatment. Although termination is considered an ending, it is in reality a transitional phase to clients' next stage of development. It ends the treatment phase where therapist and client have worked together to address the clients' presenting problems; it also signals the next stage of life where clients feel more contained and better able to manage life with more balance and flexibility. This also includes knowing and feeling comfortable with asking for help, should that become needed.

### THE COURSE OF CLIENT CHANGE: AN OVERVIEW

New therapists usually lack a conceptual overview of how client change comes about. Often, because they have not worked with many clients or may not have experienced their own successful therapy, student therapists do not have a sense of the ordered sequence in which change typically occurs. We begin this section on working-through by providing an overview of the change process. We will describe when and how clients tend to resolve their presenting problems and adopt new, more adaptive responses with others.

For some clients, change begins as soon as they decide to enter treatment. These clients are in the committed, or action, stage of the change process (Prochaska & Norcross, 2006). They recognize that a problem exists and are no longer going to deny or avoid their

problem. Furthermore, they are now ready to devote time and energy to addressing the problem(s). These clients, by entering treatment, are also acknowledging that they cannot resolve their problems alone and need help. Some clients recognize that their resolution to seek help is a healthy step forward, and they feel good about making this decision. Sadly, others may experience it as a failure, as evidence of their inadequacy, disloyalty to their family, or as a violation of their cultural values and beliefs. When clients can allow themselves to feel good about the decision to seek help, or when therapists can help clients reframe their need for help so they can have more self-empathy and self-compassion (Neff, 2011), some initial relief of symptoms—such as anxiety or depression—may result. This internal commitment to seek help is a crucial first step. Change is less likely to occur if clients enter treatment because someone else is telling them to. For example, Therapist in the initial intake:

**THERAPIST:** OK, that's what your wife thinks you should do. But I would like to hear what you want to work on in therapy—what you want to change, what is causing you trouble?

Some changes in feelings and behavior may also occur during clients' initial sessions with the therapist. Clients are reassured when the therapist:

- *invites them to express their concerns directly and fully;*
- *listens intently with respect and empathy for their distress;*
- *is able to enter their subjective worldview and grasp the core meaning that these concerns hold for them;*
- *works collaboratively with clients on clarifying what's wrong and needs to change in a way that yields hope;*
- *helps them feel connected to a caring other; and*
- *demonstrates a practical ability to help solve at least some small part of clients' immediate problem or symptom (e.g., by providing practical information about discipline/child-rearing, teaching relaxation techniques for anxiety, identifying thought processes or behavior patterns that are maladaptive, role-playing new behaviors that will help with an upcoming situation).*

Consider the following case example:

*Ella, a 20-year-old African-American student at a community college, came to the initial session saying she wanted to know how to help her depressed cousin who was isolating himself from others. As the intake progressed, Ella repeated, "He doesn't really know what a hard time really means." Taking the cue and responding to this point of entry, the therapist gently asked, "What has been hard at times for you, Ella?" She began to tear and explained that she and her older sister had been repeatedly ridiculed and "shoved around" by their stepfather. Neither she nor her sister had ever told anyone or even talked to each other about it except to acknowledge to each other minimally what was happening at the time. As soon as they were able to work and could afford an apartment, however, they moved out of the family home together.*

**THERAPIST:** You told no one....Did your mother or biological father know you were being mistreated?

**ELLA:** I don't know...(tears become sobs)...I think maybe...

**THERAPIST:** You're not quite sure....Do you think they didn't know?

**ELLA:** My dad wasn't in the picture by then. He was gone. I didn't tell my mom, though, even though I think she knew. I wanted to protect her from really knowing.

**THERAPIST:** You didn't tell her because you wanted to protect her...you didn't want her to *really* know?

**ELLA:** Yes, I wanted her to be safe...safe from really knowing...keep her protected, you know.

**THERAPIST:** You wanted to protect her from having to deal with all of this, and maybe that left you feeling alone and needing someone to protect you and keep you safe? Ella, this is difficult to think about....I'm glad you are bringing me in on this—you've been alone with it for so long. What's been the hardest part?

**ELLA:** She knew, I know my mother knew. She had to hear it, and see the bruises we had. I just didn't want to make her feel badly because if I was to say, "Ma, can't you see what he's doing to us?" what could she do? Besides, if I did say anything to her, she'd have to choose, us or him.

**THERAPIST:** You're saying she didn't want to hear about this physical abuse because it would disrupt her marriage, and that you think she might have chosen him over you and your sister? Am I getting that right?

**ELLA:** Yeah, I think she ignored what was going on for us just to keep her life easier. That's probably been the hardest part. Even now, my sister won't talk about any of it. He was one of those ragers, you know. It's probably why both of us still feel scared all the time....

As clients find that the therapist can understand their experience and respond compassionately to them, a working alliance is established that also reduces anxiety and depression. Such validation and support do not resolve most clients' conflicts but does engender hope and help to relieve their distress. Finding this Safe Haven in the therapeutic relationship, clients are no longer alone in their problems—they have the benevolent ally they need who can see and understand what's wrong. As we have emphasized, *the essence of a secure attachment is that children are secure in the expectation that when they are distressed, the caregiver will see or register the distress and try to help them solve their problem* (a secure attachment has nothing to do with common misconceptions about warmth, friendliness, being nice, or "bonding" through common interests and activities). The key, which is also our role as therapists, is to provide clients the sense of safety that they are no longer facing their problems alone—as with Ella.

In the therapeutic relationship, needs, concerns, and fears can be shared, fully registered and understood, and empathically responded to. This then would be followed with helping clients discern who in their environment is also safe to share their concerns with—and who isn't. This process of providing clients with a Safe Haven to bring their emotional needs and struggles to is also accompanied by actively listening for and encouraging their own voice—to clarify and claim what they like, believe in, and choose. The therapist's task goes beyond empathic attunement and also includes a Secure Base - respectfully challenging clients' faulty schemas and beliefs about how relationships must and always will be (Bernier & Dozier, 2002; Bowlby, 1988; Dozier & Tyrrell, 1998). Here the client is encouraged and supported in developing a differentiated sense of self. This does not mean disconnection from others; rather, it refers to connection to a coherent, reflective, and integrated inner voice that is the foundation for self-efficacy and self-agency. Taken together, these interventions form the bedrock of therapeutic effectiveness for therapists working within most theoretical orientations and treatment modalities.

Change occurs as clients experience the therapist as a "go-to" person when in need and a source of support in exploration. The next phase where changes are observed is when the therapist succeeds in focusing clients inward on their own thoughts, feelings, and response patterns and away from their preoccupation with the problematic behavior of others. This seismic shift occurs as clients explore their own internal and interpersonal reactions, begin to recognize how their behavior contributes to an interpersonal conflict, and realize they may be able to *change their own part* in the unwanted, problematic prototypic interactions that keep occurring. Adopting an internal focus for change is also an important way to identify maladaptive schemas and reframe how clients think about

their problems (Levenson, 2010). Redefining the conflict with the other person as, in part, an internal problem usually reveals a wider array of new and more adaptive alternatives that clients can begin to try out with others.

When the client experiences the therapeutic relationship as different and finds that familiar but unwanted expectations are not confirmed, the door to change opens with others in two major ways and we enter the working-through phase of treatment. First, *the therapist begins in earnest to assist the client to actively transfer the client's here-and-now relearning to other relationships, as in the following exchange:*

**THERAPIST:** Good, it's getting clear between us that I'm really not judging or criticizing you. How is this going with others in your life?

**CLIENT:** I'm not sure what you mean.

**THERAPIST:** Who consistently does this to you? And who in your life doesn't?

**CLIENT:** (*chuckles*) The list of doers and non-doers. Well, my dad is definitely at the top of that list of doers, and I'm thankful to say that my boyfriend is a non-doer.

Second, under their own initiative, many clients will come back to the next session, following a CEE with the therapist, and share successes and failures in their attempts to relate in this new way with others in their life. For example:

**CLIENT:** I tried speaking up with my girlfriend this week, you know, like I did with you.. It felt good—she listened to me this time. She said it was because I was talking about “our problems” and not just “her problem”.

At this point in the change process, the therapist often acts as a “coach” to help clients deal with the positive and negative responses of others to their new ways of relating. Through behavioral rehearsal or role-playing, and direct guidance or teaching, the therapist helps clients transfer the new ways of interacting that have occurred with the therapist to others in their lives, as with the following statement:

**THERAPIST:** I can see how she might have appreciated you speaking to her differently, not blaming. Using those “I” statements we talked about really are different than starting a sensitive conversation with “You...”

With this overview of the sequence of change in mind, let's now enter the working-through phase of therapy more closely, and explore how the changes that occur with the therapist can be generalized to others.

## WORKING-THROUGH

### THE WORKING-THROUGH PROCESS

When clients change, it usually begins in the therapeutic relationship. Some clients, especially those who generally function well, can successfully adopt new behavior with just the therapist's validation and support, enhanced perspective taking, and education/skill training. When the new behavior is integrally linked to their core conflict, however, most clients will need to practice this new response in the therapeutic relationship. Thus, *therapists will need to encourage clients to try out new ways of responding with them in the therapy setting* and check out with clients if they have slotted the therapist's response into old schemas and the problematic ways that clients have grown to expect from others. It is crucial that therapists convey verbally *and* enact behaviorally their support of their clients' new behavioral repertoire, and that clients register that this new response does not fit the old template. Therapists can anticipate that their clients will soon test them and

try out these anxiety-arousing new responses (e.g., by being more assertive or disagreeing with the therapist). If therapists pass the clients' test—respond affirmingly to the new behavior, rather than unwittingly repeat unwanted but familiar responses—then clients have the real-life experience that change can occur.

This CEE is the linchpin in clients' reworking of their inflexible interpersonal coping strategies, rigid schemas, and dysfunctional internal working models. As previously observed, one corrective episode with the therapist is not usually sufficient to effect sustained change. Most clients will need to reenact this and other reparative relational themes multiple times in the therapeutic relationship. As a rule of thumb, the more clients have been hurt or maltreated, the more often they will need to re-experience a new and safer response that doesn't fit the old schema or template. However, *once clients have seen that at least one relationship can be different, therapists can actively engage clients in generalizing this new experience of change beyond the treatment setting*. Thus, working-through is not about gaining insight or exploring the past—it is about linking the changes that have occurred with the therapist to changes with significant others in their current lives. Typically—but by no means always—clients try out new ways of responding with others in the following progression.

Change often occurs first in clients' relationships with the therapist—especially if the therapist is working actively with “you and me” and asking about and sorting through what's going on this relationship. Clients then change with acquaintances they do not know well or with others who are not especially important to them—i.e., where the stakes are not so high if things go poorly. This is often followed by change with supportive others who are important to the client, such as caring friends, teachers, or mentors. Some clients will become interested in trying to change old response patterns with the developmental figures with whom the conflicts originally arose, such as caregivers and important family members. In contrast, other clients will want to explore potential changes in their internal reactions and behavioral responses to primary others with whom the conflict is currently being lived out, such as spouses or children, and subsequently transition to formative issues with developmental figures. These final two arenas are the most challenging because the consequences have so much greater import. Although this may co-occur to some extent for many clients, as a rule of thumb, clients generally will try to make changes first in the interpersonal sphere that feels safest or most likely to succeed.

During the working-through stage of therapy, some clients will rapidly assimilate the new ways of responding and readily apply them throughout their lives. Other clients, who have more pervasive conflicts or who have been traumatized or deprived more severely, will work through their problems more slowly (i.e., less capacity for trust to use the therapist as a Safe Haven or a Secure Base). An example would include adult clients with a Fearful attachment style, who have experienced overt rejection from a parent, or grew up in families where familial rules prohibited talking about or making overt mistreatment that was occurring. Others who have been deeply “un-done” by pervasive invalidation of their experience (and been left anxiously unsure of the validity of their own thoughts, feelings, and perceptions), will need to confront and address the same fears and expectations repeatedly. The therapist's aim is to clarify how the same relational pattern, affective theme, or faulty belief is being played out in this particular situation too, and help clients find a better way to respond to this manifestation or exemplar of their core conflict. In this regard, the core conflict will be repeatedly expressed in four areas of client functioning:

- *In the current interaction or interpersonal process that is transpiring with the therapist;*
- *When reviewing crisis events in clients' lives or the crisis events that originally prompted clients to seek treatment;*
- *In current relationships with friends and significant others in which clients' emotional problems are being activated;*
- *In developmental relationships with attachment figures.*



For many clients, their attachment style and history will have an impact on this working-through phase. For example, clients with secure attachments are better able to risk exploring their issues in more depth, address and sort through real conflict with therapists, and better use therapists as a Secure Base from whom to effectively “launch” (Mallinckrodt et al., 2005; 2014; Romano et al., 2008). That is, they are able to be emotionally close to their therapists and are also able to transfer what they have learned in their relationship with the therapist to relationships outside the therapy setting.

In contrast, clients who have preoccupied attachment styles, while able to go to their therapists to express their distress and needs, are anxious that the therapist will not be consistently available and that their needs will not be met. They often talk about their problems in diffuse, overwhelming terms, or in a global or superficial manner. Coupled with their inability to trust others, it is difficult for these clients to accept and internalize therapists’ ability to sustain a dependable and benevolent responsiveness toward them and so they have difficulty developing a sustained, coherent sense of self. Thus, working-through—which requires becoming more reflective and considering different perspectives, exploring new ways of being with others (based on real rather than magnified needs), and discerning more selectively with whom to disclose and trust—will be a significant challenge for these clients.

Unlike clients who have a preoccupied attachment style, those who have a dismissing style are likely to have difficulty recognizing and disclosing their needs, and engaging emotionally with the therapist. As we have seen, these clients are especially apprehensive about intimacy and vulnerability. The relational message they convey, overtly and nonverbally, is that they need no one and can manage on their own. One of the primary tasks here is to help these clients find that “being-in-relationship” does not inevitably hold the threats of intrusive control from others, or insensitivity or indifference to their distress when it does emerge. The other task is to help these clients see that disclosing needs and asking for help sometimes is separate from “weakness.” Dismissive clients fail to see that realistically assessing strengths and needs and asking for help when appropriate is a healthy and natural process.

Thus, the best way to help both Preoccupied and Dismissive clients become more capable of sustained intimate relationships is to help them have a meaningful relationship with their therapists—one that is safe or reparative because it isn’t overwhelming or distant. Only then, as they are having the real-life experience that the unwanted responses they expected are not occurring in this relationship, can these clients begin to translate this reparative experience to relationships outside the therapy room where they repeatedly have been hurt in these familiar ways. This resolving process, or *earned security*, is a healthy sign of emotional well-being and it facilitates improved functioning in many ways (Farber & Metzger, 2009; Mallinckrodt et al., 2005; 2014; Townsend & McWhirter, 2005).

In terms of the treatment process, many therapists find that the working-through phase is one of the most rewarding phases of treatment. Therapists have the opportunity to take pleasure in clients’ newly obtained efficacy and celebrate with them as they successfully adopt new responses in progressively more challenging situations. During this stage, as change occurs, therapists can also become more actively involved in answering direct questions and in providing suggestions and information to help clients enact new behavior or solve problems. For example:

**THERAPIST:** Maybe you could try saying something like this to her...

Therapists also can provide more interpersonal feedback about how clients come across to the therapist and others (Brammer & MacDonald, 1996). For example:

**THERAPIST:** Hey, that wasn’t that “small voice” we’ve been talking about. You sounded strong and clear as you said that to me—it held real conviction. It’s great to see you like this. Have you been acting stronger like this with others, too?

Therapists often find that cognitive interventions and behavioral procedures for rehearsing alternative responses are especially helpful during this phase. Once the interpersonal process is enacting a corrective experience, clients become more responsive to other types of intervention, such as assertiveness training and parenting education, self-monitoring and self-instructional training, role-playing and other modeling techniques, and educational inputs or readings (Gold, 2011, 2005; Goldfried & Davila, 2005). The nature of the therapeutic relationship also changes during this period. As clients improve through the working-through period, they will increasingly perceive the therapist in more realistic terms. Once the transference projections and eliciting maneuvers have been jointly identified and are being resolved, therapists can disclose more personal information to clients and enjoy further mutuality. However, therapists are still prepared to respond to the reenactments, faulty beliefs, maladaptive coping patterns, and resistance that clients will continue to present, especially when distressed.

As clients live out and continue to experience a different and corrective relationship with their therapist, they will find that new ways of relating with others in their lives are possible. This is a critical juncture in treatment. Now, the therapist's task becomes helping clients anticipate and negotiate the successes and failures that are likely to follow. As rigid schemas and IWMs expand, clients need to be informed that, although some individuals will respond positively to the clients' new changes, others will not. It certainly will be empowering when some people respond affirmingly to clients' healthy new behavior. However, therapists must also help clients discern who is likely to respond poorly (e.g., when a client stops adopting a pervasive caretaking role and begins to say no sometimes). Therapists also need to help clients anticipate realistically how it will feel, and how they are likely to react, if the other person responds in the familiar, unwanted way to their healthier new behavior. This is especially important in primary relationships with spouses and parents, where it can be so deeply discouraging when clients, yet again, receive the same unwanted but familiar responses. To illustrate, consider the following statement:

**THERAPIST:** It's great that your husband has been able to listen to you better, and take your concerns more seriously now that you are expressing them more directly. But I'm wondering how this will go with your mother when she visit this coming three-day weekend. What if you speak up more strongly for yourself with her, as you have been doing with me and others, but she keeps turning everything back around to her needs, what she wants, and what you "should" do? Let's think this through before you see her.

Therapists can help clients anticipate potential disappointments, which are an inevitable part of the working-through and change process. As clients find that they can safely respond in new ways with the therapist, and then make successful changes with certain supportive individuals in their current lives, *their expectations of change in some other relationships may become unrealistically high*. For example, suppose an adult survivor of childhood abuse has been deeply understood and validated by an effective therapist. Following this success with the therapist, the client risks disclosing this shame-laden secret to her husband, and then to her best friend, and they both respond affirmingly as well. Next, the client's lifelong wish that the abusing parent can now acknowledge the reality of what occurred long ago is evoked, or that the other parent, who could not be protective at the time, will now hear or believe the client.

With expectations buoyed by successes with others, this client may address parents or other family members, *only to encounter the same invalidation, scapegoating, and threats of ostracization that she received decades ago*. Or, some clients may be profoundly discouraged to find that, unlike the therapist and some others in their lives, their partner cannot change or respond positively to their healthy new behavior. Thus, in order to

manage these disappointments that are an inevitable part of the change process, therapists can prepare clients in the following ways:

1. By helping clients *realistically anticipate* how each new person is likely to respond to their changes. For example, if the client is a married woman, what will her husband probably say and do when she acts more assertively with him for the first time?

**CLIENT:** He's likely to smile at me like I'm a cute child or something, and then just change the topic as if I didn't even say anything;

OR

**CLIENT:** He's likely to say, "You've gotten so selfish and stopped thinking about anybody else since you got that degree and started working."

2. By helping clients spell out in detail how they are likely to feel, and *what they are likely to say and do*, if they receive a version of the old unwanted response to their healthier new behavior.

**CLIENT:** I think I'd feel ashamed. You know, feel stupid for trying to stand up for myself. And then just get quiet and withdraw inside—and go along again like I've always done;

OR

**CLIENT:** I'd feel anxious and keep trying to convince him to understand how I feel but, you know, then I'll just give up and go get a glass of wine—or maybe two or three.

3. By role-playing and providing new, more adaptive responses that, after rehearsing in therapy, clients can use at this discouraging moment, instead of repeating what they have usually done in such circumstances in the past. Recalling the concept of "change from the inside out," the key concept is trying to help clients change their own response in problematic interactions, rather than focusing on the often futile attempt to get others to respond in new or better ways. For example:

**CLIENT:** I'm trying to help our marriage and tell you something important right now, but you're not taking me seriously. I don't like feeling dismissed like this. Are you willing to take this more seriously or should we just stop this conversation now?

OR

**CLIENT:** I'm trying to tell you something about me, but you keep making this conversation about you. Would you just listen and hear me out first, and then I will listen to whatever you want to say next. Let's make it so we can both feel heard.

Clients will continue to have experiences with others that reenact hurtful old scenarios and confirm their pathogenic beliefs. However, by helping clients walk through these same three steps throughout the working-through stage, therapists can help them find ways of changing their own internal and interpersonal responses—even when others are unable to change. For example, clients can learn that they are not to blame for, or deserving of, the way they were mistreated—even though family members still cannot affirm them. Or, maybe a married woman who's been belittled by her husband can learn that she can still advocate for herself. Although her husband might not be able to change, she does not have to go along or comply with his dismissal in the same disempowered way she has in the past; she can establish new relationships with others where her limits or opinions can be respected. In this process, clients become less anxious and depressed

and are empowered as they learn that they can change their own responses, even when significant others in their lives are not supportive.

Through each setback with a significant other, therapists and clients both gain further awareness of clients' habitual response patterns, deeper access to the painful feelings and faulty assumptions that accompany them, and have more opportunities to continue exploring new responses that clients might try out the next time they are in that situation. Therapists can explore each successive step with clients by saying:

- *What were you thinking and feeling inside when he dismissed you like that?*
- *Tell me what you did when he said that to you.*
- *What would you like to be able to say to him instead the next time this comes up?*
- *Would you like to role-play that together and try it out to see how it might go?*

Although clients are improving during the working-through period, they will still be struggling with their core conflicts, especially when they feel vulnerable or distressed. Therapists will need to continue helping them to work through the distortions, faulty expectations and nonproductive response patterns that stem from their old schemas and IWMs. In addition, most clients will become discouraged at times by the repetitious working-through process. Seeing that a variation of the same old conflict or scenario is confronting them again, clients may lament that “nothing has changed” and feel it is futile to keep trying. Therapists can make no guarantees of change, of course, and cannot assume responsibility for clients' motivation to continue. At these critical junctures, however, therapists want to reach out and extend themselves to their clients. Only as student therapists incrementally develop a new worldview and increasingly grasp the profound influence of early maladaptive schemas in shaping the subjective experience of “reality” can they be empathic with clients' discouragement. Although clients may realize it intellectually, experientially it really does seem to them as if this is the only way that relationships can be:

**CLIENT:** Why do I *always* have to be the responsible one who takes care of everything!

Therapists help clients by holding steadfast that there can be another another way - that some some relationships can be different. Therapists can help clients identify instances where other (reparative) ways have occurred, affirm their appreciation of how frustrated or discouraged clients may be feeling right now, and convey that they remain committed to working together with clients to find a better way. It is therapists' resolve to effect change in the relationship with the client, and their personal commitment to helping this client change with others, that pulls the client through these expectable crisis points in the working-through phase. And crucially, it is therapists' ability to continue interacting with the client in reparative ways that provide hope that some relationships can be different and change can occur. In providing these CEEs, therapists provide clients with a safe person that can help them fundamentally reorganize how they see themselves and others, and meaningfully integrate their thoughts and feelings into a more secure and cohesive self (Elliott et al., 2004; Pos et al., 2003).

As an illustration of the working-through phase, we now consider an extended case example in which the client changes first in her relationship with the therapist and then, with the therapist's assistance, is able to extend that change to other relationships in her life. The episode in which client change first emerges—the *critical incident*—may be an overt, interpersonal conflict with the therapist or a compelling, transference-laden misunderstanding. More likely, however, as in this case study, it is a subtle aspect of the interaction with the therapist that could easily go unnoticed unless the therapist, informed by working hypotheses, has been prepared to look for this relational theme or pattern.

*Tracy, a 26-year-old, moving-toward client habitually responded to others in a compliant, pleasing manner. She usually went along with others, acting as if her wishes and feelings didn't matter. Tracy told her therapist that she didn't believe that others would be very interested in listening to her or doing what she wanted to do—she long had felt that either she had to “go along or be alone.” Collaboratively, they had linked Tracy's compliant behavior to her dysthymia, especially her presenting symptom of crying spells, and went on to explore Tracy's low self-esteem and feelings of worthlessness. In addition, the therapist actively encouraged Tracy to respond differently in their relationship. The therapist behaviorally demonstrated that she cared about what Tracy had to say and invited her to express her own opinions in counseling—even if that meant disagreeing with the therapist. During their next session, Tracy tested to see whether the therapist meant what she said and risked this significant new behavior in the therapeutic relationship:*

**TRACY:** You're right. I guess I never have felt very good about myself.

**THERAPIST:** From other things you've said, it seems to me that your mother just wasn't there for you as a child. You didn't get the support you needed from her. Maybe we should explore that further.

**TRACY:** No, I don't really want to talk about her. I remember her more fondly. My father is the one who was pretty harsh.

**THERAPIST:** (*exclaiming happily*) You just said “no”, you don't want to talk about your mother and that I was wrong about her!

**TRACY:** I'm sorry; sure, we can talk about her. What do you want to know?

**THERAPIST:** You just disagreed with me! You just told me you didn't want to do what I wanted to do and suggested what you thought would be better. That's great!

**TRACY:** What?

**THERAPIST:** You just did what we have been talking about. You expressed your own opinion, said what you thought, and disagreed with me. I am so happy for you!

**TRACY:** Aren't you put off by that? Didn't I seem disrespectful?

**THERAPIST:** Oh, no, we can disagree and still be close. That was a strong thing you just did.

**TRACY:** So...I guess what you're saying is that maybe it is OK for me to say what I think sometimes?

**THERAPIST:** Sure it is! I care about what you think. I want to know what you feel, and I want to do things your way, too.

**TRACY** (*tearing*): But you're safe. It's easier to do that with you—you're not like other people.

**THERAPIST:** OK, I am “safe,” and it is easier to do things like that with me than with other people. But you have changed to be able to do that with me; you've grown. You couldn't do that before, you know.

**TRACY:** Yeah, that's right.

**THERAPIST:** And if you can value yourself enough to express what you think in here with me, then you can begin to do that out there with certain others as well.

**TRACY:** You really think so?

**THERAPIST:** Yes, I think you can. If you can do it with me, you can do it with some of them.

**TRACY:** Oh, I would love to be able to do this with my boyfriend.

**THERAPIST:** Tell me how this usually plays out with your boyfriend.



**TRACY:** *(describes the usual scenario)*

**THERAPIST:** OK, what do you want to be able to say to him instead?

**TRACY:** *(describes an alternative scenario)*

**THERAPIST:** Would it be helpful to you if we role-play that together? You can be him—and say the things that he’s likely to say that will be hard for you to deal with. And I’ll take your part and role-play ways you might want to respond to him. Then we can talk through together what might work for you and what wouldn’t—where the problems might develop if you decide to try some of that with him.

Why is the therapist so enthusiastic about such a seemingly small change? Is this single manifestation of a new behavior all that is necessary for Tracy to resolve her problems? No, but this highly significant event brings her to the working-through phase of treatment. Within the safe orbit of the therapeutic relationship, Tracy was able to try out a new behavior that taps into the most significant issue in her life and that arouses intense anxiety. For Tracy, adopting such a seemingly insignificant self-assertive behavior arouses lifelong feelings of worthlessness. At the same time, she is discarding the primary means she has developed to protect herself from these painful feelings—that is, to “move toward” and go along with others. In order to resolve her painful conflict, Tracy will have to experience the following sequence repeatedly:

1. *Her old relational pattern is activated—she has to “go along” and act as if everything is OK even when things are painfully wrong.*
2. *She tries out a new and more adaptive response with the therapist, which is to assert her own preference.*
3. *She receives a different and more satisfying response from the therapist, who affirms Tracy’s initiative.*
4. *She generalizes the new behavior to others beyond the therapy setting by considering how she typically behaves with significant others in her life and, specifically, what she might want to say and do instead the next time she is in this situation with her boyfriend. This repetitious reworking of the new behavior and the old conflict is typical of the working-through phase.*

The following week, Tracy was able to initiate the same type of assertive behavior with her boyfriend for the first time. This successful experience, in turn, encouraged her to confront the same issue in another more threatening relationship—with her father. During the next session, the following exchange took place:

**TRACY:** *(beaming)* Guess what? I did it! I’ve been dying to tell you all week!

**THERAPIST:** *(enthusiastically)* Ha! What did you do?

**TRACY:** I did what we talked about with my boyfriend. He was talking about something he thought we should do, and I disagreed with him—just like we role-played, and told him what I thought we should do instead. It felt great, and I don’t think he minded, either.

**THERAPIST:** Good for you! You’re on your way.

**TRACY:** I can’t believe how easy it was. *(pause)* I wish it could be that easy with my father. He’s always putting me down when I say something.

**THERAPIST:** Maybe you’re ready to start changing your relationship with your father as well. How do you respond to him when he puts you down?

**TRACY:** I get real quiet when he does that. I feel like crying, but I don’t. I wish I could just tell him that I don’t like it and I want him to stop.

**THERAPIST:** Yeah, speaking up at the time he does that and setting limits with him would certainly change your relationship. It would be saying to him—and to yourself—that you value who you are and what you have to say, and you’re not going to “go along” and let him hurt you like that anymore.

**TRACY:** But I could never do that. I’d like to, but I just couldn’t.

**THERAPIST:** Yes, this sounds challenging. You haven’t been able to speak up with him in the past, but you have been changing in here with me, and now with your boyfriend, too. So maybe you can do something different with your father as well. What holds you back from setting limits with him? What are you most afraid of?

**TRACY:** He wouldn’t take me seriously; he’d just laugh at me.

**THERAPIST:** How would that make you feel?

**TRACY:** Worthless. (*begins crying*)

**THERAPIST:** So that’s where that awful feeling comes from.

**TRACY:** Yeah. (*long pause*) It makes me mad, too.

**THERAPIST:** Yes, that makes absolute sense. It hurts you very much when he diminishes and devalues you like that, and you have every right to be angry.

**TRACY:** It’s not fair! I don’t want to let him do that anymore. It’s not good for me.

**THERAPIST:** That’s right, it’s not good for you, and you don’t deserve it. Right now, you are making the transition from feeling worthless to feeling worthy—that you do matter. Now that you are seeing this so clearly, maybe you don’t have to go along with it the way you used to.

**TRACY:** But what could I do?

**THERAPIST:** Let’s think this through and explore some options together. What would you like to do the next time he does that?

**TRACY:** Well, maybe I could just tell him to stop it—tell him that I don’t want to be treated like that anymore.

**THERAPIST:** Yes. That would be a strong and appropriate response on your part. But before you try it, let’s explore further how that conversation might play out. Maybe we can identify the trouble spots and role-play some responses to help you through them.

In ever-widening circles such as these, clients confront and work through the same relational patterns in the different spheres of their lives.

Notice that the critical incident in this case was very subtle: When the therapist suggested that Tracy talk about her mother, Tracy said she didn’t want to. Therapists could *easily miss such slight shifts in behavior unless they have formulated working hypotheses about how the client’s conflicts with others are likely to be reenacted in the therapeutic relationship*. If Tracy’s therapist hadn’t anticipated that Tracy would tend to comply with her, and if the therapist hadn’t been alert for any budding sign of self-direction or assertiveness from Tracy, this particular opportunity for change would most likely have been lost. The therapist might have responded by focusing on the content (by saying, for example, “Well, ok, but I think it’s important to look at your relationship with your mother because...”), rather than on the process (Tracy’s self-assertion). In that case, Tracy would have complied with the therapist and dutifully talked about her relationship with her mother. Then, however, the interpersonal process between Tracy and her therapist would be repeating her problematic pattern, and little change with others in her everyday life would result.

When working with a client who seems unable to change, the therapist may eventually grow discouraged and become frustrated with, or disengaged from, the client. In

those circumstances, the therapist should consult with a supervisor and consider how their interpersonal process subtly may be reenacting the client's maladaptive relational patterns. That would explain why the client to change, even though the therapist and client continue to talk about adopting more assertive behaviors with others, the therapist sincerely supports and encourages this, and the client learns useful skills that should facilitate the new behavior. Such subtle reenactments along the process dimension are expectable. Further, if therapists utilize process notes (Appendix A) and generate working hypotheses (Appendix B), they can identify possibilities for how each particular client's conflicts might be expressed or played out in the therapeutic relationship. They can then check these possibilities out with the client at opportune times. By considering potential issues and themes that may be especially important to listen and watch for with this client, such tentative working hypotheses will help therapists better recognize such reenactments in the moment as they are occurring, and be prepared to respond more effectively to them.

In the working-through phase, the therapeutic action is primarily in the present. Many of the significant events that take place in treatment are current interactions with the therapist that disconfirm pathogenic beliefs and maladaptive schemas. As these reparative interactions occur, they also lead to internal reworkings of clients' schemas as new behavior comes into play (Akhtar, 2007). These cognitive changes are necessary to help clients assimilate and maintain these new and more effective ways of relating after treatment ends. The process of experiencing a different response from the therapist, along with new ways of thinking about self and others, contributes meaningfully to affect self-regulation. Following these changes in clients' relationship with the therapist, the next step is to generalize these corrective experiences and expand clients' interpersonal range beyond the therapy setting as they now try out these new ways of thinking, feeling, and behaving with others. As we have seen, clients will have both successes and disappointments as some individuals will welcome clients' changes, whereas others will insist on continuing along well-worn, problematic lines.

As clients explore the potential for change in current relationships and come to terms more realistically with the possibilities and limits of these relationships, two new subphases in the working-through process may emerge. First, clients, on their own, often start to look back in time in order to understand the formative experiences that originally shaped the problems they now are resolving. To elucidate this process, we will examine therapeutic guidelines to help with this **family-of-origin work**. Second, as clients develop a more realistic narrative that helps them better make sense of their life experiences and how they have become who they are, they also begin to look ahead. They begin to think more about what they want their life to be in the future and to reformulate plans to fit the person they are becoming. This subphase, often ushered in by the emergence of the **Dream**, will be explored second.

### FAMILY-OF-ORIGIN WORK

It can be liberating for clients to explore the familial interactions and developmental experiences that shaped their current conflicts. As they gain an understanding of the family rules, roles, and childhood dilemmas that shaped how they learned to cope and adapt, they become more accepting of the choices, compromises, and "best effort" adaptations they have had to fashion in their lives (Russell & Fosha, 2008). During this time, therapists can help clients feel more empathy for:

- *themselves and what they have endured or suffered,*
- *how they have chosen to cope, and*
- *the personal limitations or difficult life circumstances that led their caregivers to respond in the problematic ways they did.*

Clearly, thinking about clients' experiences within this developmental context can be profoundly enriching. In most cases, however, it is not productive for therapists to *lead* clients back in time, make historical interpretations, or try too readily to make links between current relational patterns and formative family relationships. Although these developmental connections may indeed be highly accurate, many clients will not find them helpful or relevant to their current problems if they come from the therapist. Per client response specificity, some clients may be able to utilize familial connections—and therapists can assess whether they are likely to be helpful for this particular client. However, many clients will not find these developmental links relevant to their current problems—especially early in treatment, and when they have not been jointly or collaboratively formulated, even though they may be quite accurate. For example:

**THERAPIST:** I keep hearing the same theme: that what you are complaining about with your spouse seems in some ways to be similar to what used to occur with your mother.

**CLIENT:** My mother? I don't see what my mother has to do with this. She was pretty great, and my wife's driving me crazy. Besides, she passed away almost 10 years ago, so I'm really not sure why we're talking about my mother.

**THERAPIST:** Sure, what I'm suggesting doesn't resonate for you—doesn't ring true. So let's get back to your thoughts. How do you make sense of what's going on between you and your wife? You've been thinking about this a lot—how do you understand it?

Many clients will not be able to use this type of developmental connection to make progress in treatment. Although the familial suggestion may be apt, and at a later point in treatment it may shed some light on the scenario that's recurring, it is often too far from their current experience to hold meaning for them. Instead, a more collaborative, *mutual exploration* of familial and developmental experiences may teach therapists (and clients) a great deal about the faulty schemas and coping strategies that clients originally learned, how these are shaping problems in current relationships, and the corrective experiences that clients need to find now in treatment with the therapist. Although this developmental understanding will not help some clients with their current problems, it does offer therapists a great deal of help in generating working hypotheses about the themes and patterns that are likely to go on between them in their interpersonal process. *Toward this different purpose of assessment and formulating a treatment focus*, for example, the therapist might ask: “*What was it like to be a child growing up in your family?*” or “*Tell me about your parents' marriage. Can you bring it to life for me?*” To illustrate more fully:

**THERAPIST:** Tell me how your parents responded, what they would say and do, when you were successful or felt proud of an accomplishment.

**CLIENT:** I'm not sure what you mean—why are you asking about that now?

**THERAPIST:** I'm wondering if this depression didn't start right after you got that promotion and pay raise? That possibility is what's leading me to ask, “What was the look on your mother's face when you showed her that spelling award years ago?”

**CLIENT:** (*pensively*) Well, I don't think it was a very happy face. I don't know why, but she almost seemed sort of sad or hurt.

**THERAPIST:** Uh-huh, your success seemed to hurt her in some way, maybe almost as if she might have felt it as a loss. But whatever caused her to respond that way, it sure would make it hard for a girl to feel good about her success experiences. How did your father respond when you told him about winning?

**CLIENT:** There was nothing subtle about that—he didn't like it and told me to stop bragging in front of my brother.

Clearly, this client will need her therapist to take unambiguous pleasure in her success experiences and personal strengths. Early in treatment, however, most clients will not be able to make meaningful bridges between current and past relationships from this developmental exploration. When the therapist highlights seemingly obvious connections, the relational patterns may not come alive for clients until they first have experienced a corrective response to these old relational patterns with the therapist. Here is the key: *Following this corrective experience, many clients on their own will recall and initiate leading the therapist back in time to the developmental experiences that originally shaped the current problem being discussed.* These client-initiated explorations, which usually occur immediately after clients have a CEE with the therapist, or have successfully adopted a new response with others, continue throughout the working-through period. In this sequence, the explorations are not sterile interpretations or abstract concepts. Instead, they are enlivening for clients, feel salient and informative, and contribute to productive change with others.

Let's pause and highlight the important sequence of change that is being clarified here. In contrast to many other counseling theories, the interpersonal process approach does not suggest that behavior change leads to insight or, conversely, that insight leads to behavior change. Both of these change processes certainly occur, but a different mechanism of change is emphasized here. *Both meaningful insight and sustainable behavior change follow or result from clients' new or reparative experience with the therapist* (Levenson, 2003; Wachtel, 2008; Weiss, 1993).

**Addressing Family-of-Origin Issues with Primary Caregivers.** As clients make their own meaningful links between formative and current relationships, which therapists can join in expanding and developing, clients often ask therapists what they should do about problems and patterns that are continuing in their current relationships with family members and attachment figures. We now examine two broad guidelines to help clients with this family-of-origin work. First, recall the “internal focus for change” described in Chapter 4. We will again see how clients can learn to change how *they* respond in current problematic interactions with family members—rather than engage in futile attempts to try to get parents or others to be different and change how they have always responded. Second, we look at the concept of “grief work,” in which clients mourn and come to terms with what they have missed developmentally. Working in both of these areas results in an interpersonal and internal resolution of family dynamics that helps clients attain earned security.

Clients who struggle with more pervasive or long-standing problems usually have family-of-origin work to do. Many clients do not wish to discuss problems from the past with caregivers or family members, and therapists should not press clients to do so. However, other clients will want to acknowledge or address past conflicts with caregivers and other family members, as a way to stop current and ongoing participation in them or simply to better resolve them. It is deeply gratifying—and a powerful impetus for change—when such rapprochements succeed.

As we emphasize, however, such current resolutions of historical problems often do not work out or *sustain* over time. Sometimes caregivers have grown and changed over the years and can talk more openly or non-defensively for the first time about hurtful interactions that once occurred. However, in many cases, and especially with more serious problems or abuse, *clients will often receive the same invalidating, guilt-inducing, or threatening responses as they did years ago in childhood.* This is especially painful when clients' motivation for talking with the attachment figure is the unrecognized wish to get the parental approval, validation for what did occur, or protection that they missed developmentally and have always longed to receive. If therapists have not *realistically*



*prepared* clients for the possibility that this deep disappointment may well occur, clients may despair and feel hopeless about change occurring in any relationship. Often, *clients will be breaking unspoken but strongly held family rules against acknowledging that any family problems or that a certain specific conflict with a parent ever existed* and, in many cases, punitive **homeostatic mechanisms** will be set in motion.

For example, when Tracy did choose to speak up and set limits with her father when he put her down, he again disparaged her—just as he had for the past 25 years. As if that doesn't make change difficult enough, look what happens next in the family system. Pressuring Tracy to just go along with this mistreatment as if nothing significant is occurring, *her mother threatened to disown her for being so disrespectful and ungrateful, and the next day her depressed and obese sister telephoned from another city and tried to make her feel guilty for "hurting Daddy" and "stirring up trouble."* For clients like Tracy, change is not easy and problems are not simple. They are deeply embedded in a larger familial and cultural context that, sometimes, works actively and overtly against changes in treatment in order to maintain status quo in the family and keep clients in their same childhood role.

As we see here, the problem is not simply the parent's mistreatment of the child. If that occurs *but the mistreatment can be "named" or spoken about in the family*, the child has a problem but it is much easier to resolve. That is, it's not just the content—(what happened), but the process—(how others responded to it) that shapes the meaning of what occurred. Instead, far more serious consequences—such as the shame-based sense of self, bulimia and long-standing dysthymia that Tracy suffered—result when either of the following occur: (1) the parent and extended family system make the adult offspring again feel responsible for, or deserving of, the parent's original and current mistreatment; or (2) the child and the child's experience are invalidated—as if everyone seems to be in agreement that nothing significant or problematic occurred (mystification).

Clients often feel hopeless and blame themselves when parental caregivers and the broader family system cannot change, especially if therapists haven't prepared them for this possibility. Although these clients may go on to act helpless in other current relationships, in actuality they are not. Their resolution does not rest on getting the parent/family to change or acknowledge realistically what occurred. Instead, it rests on how clients change their own behavior and how they now respond in current interactions with living parents or, equally important, in their ongoing internal relationship with deceased caregivers. In order for clients to make enduring changes in these formative relational conflicts, they can change their own responses in these prototypical interactions—regardless of how others continue to respond. For example, Tracy's father was too limited to be able to hear her concern and talk with her about the problem. However, Tracy stopped going along with or participating in her father's disparagement of her. Instead, she was able to change how she responded to him, and began setting limits with her father by bravely metacommunicating with him, as in the following example:

**TRACY:** I keep asking you to stop it when you put me down, and you laugh at me and keep doing it. I can't make you stop, but I don't have to pretend it's OK anymore. It hurts, and I don't like it.

Although her father stayed in the same disparaging mode and again made fun of her, things did change profoundly for Tracy when she changed how she responded to him in their current interaction. As she was able to sustain this stronger stance toward him and the family system over the next few months, she felt less insecure and intimidated with them—and with others in her everyday life, than she had ever been. Her long-standing feelings of worthlessness and depression significantly improved, and her intermittent battles with bulimia largely disappeared. These far-reaching changes occurred for Tracy even though her father—as well as her mother and sister who pressured her to comply and "go along" again—could not change.

**Grieving for the Caretaking They Needed but Did Not Receive.** As they work through family-of-origin issues, clients make further progress by changing how they respond in current interactions with significant others in their lives. Clients also change by doing **grief work**—coming to terms with the feelings left in them by hurtful or emotionally depriving caregiver-child interactions. Clients do not need to discuss with caregivers problems in historical relationships. However, they do need to:

1. *stop disavowing and acknowledge to themselves what was legitimately wrong;*
2. *stop their own participation in any ongoing or current mistreatment; and*
3. *grieve for the support, protection, or validation they wanted but have missed.*

That is, they have to grieve the loss of the “wished for” caregiver. Many adult clients struggle mightily with highly ambivalent relationships with their painfully inconsistent caregivers. Often it is especially important to help them become aware that sometimes the unrealistic feeling of hope for their idealized parent can emerge when they glimpse brief episodes of kindness in their caregiver that regularly occur, yet the caregiver cannot *sustain* this benevolent responsiveness and soon returns to the usual stance of being largely self-centered, overtly favoring a sibling, undermining independence or success, responding contemptuously, and so forth. Helping clients be realistic in their appraisals, providing support in their grief, and changing their own current responses to their caregivers’ predictable behavior helps clients sustain emotional stability in the face of hurtful familial interactions. Thus, regardless of whether formative attachment figures can change—and whether they are living or deceased—clients can resolve these problems through their own internal work.

Exploring the potential for better relationships with family members is often important but, as we are emphasizing, it may or may not lead to improved relations. Often, aging caregivers and other family members cannot change, so *therapists need to prepare adult clients that when they introduce healthier new behavior into their family systems it will often be met with significant resistance*. Even if the family system has improved over time, however, grown offspring will still function better in current relationships when they can more realistically acknowledge what was wrong (and what was right) in the past and mourn what they missed developmentally - moving-toward earned security. Recalling Safe Haven, the therapist’s intention is to provide a supportive holding environment that allows clients to come to terms with the sadness, anger, shame, and other feelings that were too threatening or unacceptable to contain on their own before.

However, to succeed in this work, therapists are encouraged to remember that *clients will be safe to discuss the problematic aspects of the caregiver (or spouse) only after being reassured that therapists also appreciate and affirm the good or well-intended parts of these relationships as well*. The therapist’s goal is to integrate the ambivalence without making clients’ attachment figures or themselves “all good” or “all bad”, which are the dichotomizing or splitting defenses that clients have relied on in the past.

In another way, therapists must also help clients come to terms with both the good news and the bad news about their current situations. The good news is that *through grief work, clients can disconfirm the pathogenic belief that they were in some way to blame for their abandonment, exploitation, rejection, or parentification*. Clients remain “unresolved” for trauma or loss if they still believe they were somehow responsible or to blame (shame-worthy) for their own childhood maltreatment. The bad news is that clients’ developmental needs—such as the need for secure attachment ties—were not adequately met then, and these original developmental needs cannot now be met or fulfilled in current adult relationships (e.g., with a spouse, friend, therapist, or exciting new partner). Only after such losses or deprivation can be acknowledged and mourned, rather than disavowed, can clients go on for the first time to get appropriate and realistic adult versions of their emotional needs met in current relationships. Finally, once clients have

accepted both the good news and the bad news associated with their caregivers, they are then free to accept some of those same characteristics in themselves, and relinquish their own rigidity and expectations of perfectionism in themselves.

**Countertransference Impedes Family-of-Origin Work.** Two countertransference propensities can prevent therapists from helping clients achieve resolution of such family-of-origin work. First, because of their own splitting defenses or strong dichotomous thinking, therapists sometimes do not have the personal integration to help clients accept both what was good and what was problematic in their development. Therapists' countertransference propensities may lead them to make one of two errors. On the one hand, *some therapists characteristically want to downplay or minimize the extent and continuing influence of painful developmental experiences. For instance:*

**INEFFECTIVE THERAPIST:** That was then. You need to forget about what happened in the past. Let's try to do something about the problems you're having now. Besides, forgiveness is such a liberation.

On the other hand, in contrast to this denial, other therapists only want to blame the caregivers, make them "bad," and ignore the strengths, positive contributions and love that also were usually present in conflicted attachment relationships, as in the following example:

**INEFFECTIVE THERAPIST:** (*judgmental tone*) Your father was just toxic! What's wrong with him? I can't imagine anyone wanting anything to do with him after that.

Instead, therapists need to affirm the full reality of what went wrong in formative relations, while appreciating the need to support whatever positive ties can be preserved. Except in extreme circumstances, therapists generally want to discourage clients from breaking off all contact with conflicted caregivers if possible. When clients cut off all communication with family members, meaningful and sustained change is more difficult. Clients will struggle with self-acceptance and coming to terms realistically with their own strengths and limitations. Further, therapists often will be idealized and clients will be compelled to recreate this "split" in other relationships, with idealization and devaluation applied to themselves and to others. Therapists will be far more effective—and clients will be better able to stay with this difficult work—when therapists can acknowledge realistically what was wrong without demonizing parental figures and making them "all bad". For example:

**EFFECTIVE THERAPIST:** I can see how much it hurt you when he did that. He does go way over the top sometimes. Those times must have been really tough for you. I wonder what was going on for him when he lost it like that—you know, what led him to such a disconnected place that he was so oblivious to the pain he was causing you?

## VERSUS

**INEFFECTIVE THERAPIST:** I can't believe he did that to you. What a destructive SOB.

The second countertransference propensity that interferes with family-of-origin work is that *therapists often want to bypass the work of mourning*. As highlighted above, clients become able to acknowledge realistically to themselves the ways in which they were hurt developmentally or missed what they needed, painful feelings of loss are evoked. Sometimes therapists find it difficult to help clients come to terms with accepting their deprivation or how much they were hurt. They can do so, however, simply by being

able to remain emotionally present and compassionately affirming. Most clients do not want to end the hope or wish of getting what they missed; a few will even become angry when therapists acknowledge these disappointments or limitations.

The problem occurs when clients' reluctance to see and feel their sadness or loss coincides with therapists' own reluctance to be responsive to these difficult feelings. Routinely, when this occurs and therapists either move clients away from their sadness or grief - or over-react to it, therapists' own unfinished family-of-origin and grief work are being defended against. Typically, therapists with a preoccupied attachment style are activated by the client's loss, deprivation and separation/abandonment themes and hyperactivate themselves—no longer able to remain differentiated and provide a secure holding environment. Alternately, therapists with a dismissive attachment style minimize and avoid the client's sadness and fail to respond empathically to these vulnerable feelings (Mikulincer & Shaver, 2005; Rubino et al., 2000). When Dismissive therapists try to bypass the work of mourning and move too quickly to a problem-solving approach, clients may not be able to act on or sustain practical suggestions—even though they may be useful ideas. Only after original losses have been acknowledged and integrated (grieved) in a supportive relationship can clients open up their adult emotional needs and feel sustained by new relationships in a healthy way they have not been able to risk or find before. As clients improve in these significant ways, they achieve earned security and become able to leave the past behind—to turn the page and begin looking ahead to a different and better future.

### THE DREAM: THE FUTURE

Clients make behavioral changes and feel more well-being as they work through their problems. As they improve, the focus of therapy often expands beyond developmental conflicts and current interpersonal problems. In a very positive transition, the therapeutic dialogue often begins to include future plans that reflect clients' life-enhancing aspirations and goals. Referencing the literature on the psychology of hope, for example, therapists can help clients explore how they would like to be—"imagining possible selves" and clarifying what they might like to become (Russell & Fosha, 2008; Snyder, 1994). Similarly, therapists can work in an existential tradition with issues regarding personal choice and explore the question: How can clients create more meaning in their lives, live more authentically, and make the most of the time and relationships they have?

This freedom to explore what they really want—what matters most to them in life, develop their own personal voice more fully, and risk making choices and life changes based on self-discovery—often follows experiencing the therapist as a Secure Base and learning that change does not lead to the unwanted outcomes they had learned to expect. This process is liberating and allows for more authentic and meaningful living. In general, therapists are watching for points of entry throughout treatment when they can acknowledge health features or affirm personal strengths in clients. One special opportunity to provide this Secure Base occurs during this exciting transition from solving problems to leading a fuller and more meaningful life—which is often signaled by the emergence of the Dream.

In his classic books, *The Seasons of a Man's Life* and *The Seasons of a Woman's Life*, Levinson (1978, 2000) describes the profound influence of the Dream in shaping the structure of adult life and the course of adult personality development. As Levinson points out, the Dream does not refer to casual waking or sleeping dreams but, in the largest sense, *to the kind of life that one wants to lead*. At first, the Dream may be poorly articulated and tenuously connected to reality, but it holds imagined possibilities of self in the adult world that generate vitality. It is central in Dr. Martin Luther King Jr.'s historic "I Have a Dream" speech or in Delmore Schwartz's story "In Dreams Begin Responsibilities."

The Dream has roots in the grandiose and unrealistic hero fantasies of adolescence, but it is more than that. In early adulthood, the Dream still has the quality of a vision—of

who and how we would like to be in the world. In this way, the Dream is inspiring and sustaining for the individual, even though it may be mundane to others. For example, individuals have Dreams to be a good mother or responsible husband, a respected community leader, an ethical attorney, a skilled craftsman, a successful but honest businessperson, an artist or a spiritual leader. *If the individual is to have purpose and a sense of being alive, occupational and marital choices need to incorporate some aspects of the Dream.* When the Dream has been abandoned or set aside, life will not be infused with vitality and meaning, even though the person may be successful.

Up until the working-through phase of therapy, therapists are usually responding to the despair of broken dreams, the disillusionment of unfulfilling dreams, and the cynicism of abandoned dreams. Often, the life-infusing Dream that Levinson has articulated has not yet been addressed in treatment. In the working-through phase, however, clients are resolving their problems and emerging as healthier individuals. At this point, therapists can help clients to better articulate and reclaim their Dream and to find ways of incorporating aspects of their Dream in their everyday lives. To achieve this, therapists first need to be able to differentiate the Dream from the broken dreams that contribute to clients' presenting symptoms. These broken dreams usually reflect clients' failed attempts to overcome or rise above conflicts through their interpersonal coping styles, as discussed in Chapter 7.

In the process of working-through, therapists repeatedly help clients recognize these compensatory strivings to cope and adapt by pleasing others (moving-toward), achieving success and power (moving-against), or becoming safely aloof and cynically superior (moving-away). Therapists help clients progressively relinquish these inflexible coping styles and adopt a wider interpersonal range. In doing so, clients stop avoiding or minimizing their generic conflict, which presents the opportunity for clients' core conflicts to be expressed and resolved within the therapeutic relationship—and then generalized to others.

However, therapists can anticipate that when clients relinquish these defensive coping strategies they may experience a sense of failure or loss of self-esteem. Thus, one important component of helping clients resolve their problems is to replace these defensive, unrealistic strivings with clients' own attainable, yet sustaining, Dream. Clients need to be actively encouraged at every stage of treatment, but especially here, to explore and formulate their *own* personal dreams based on their own genuine interests. Many clients have never had the opportunity in their families of origin to know themselves, to clarify what their genuine interests and passions might be, or to choose their own goals and pursue their own ambitions. This phase of therapy enlivens clients as therapists provide a CEE and support this exploration. Here, the client's "Dream" is encouraged and facilitated.

As clients begin to improve during the working-through phase, the Dream will often emerge. To help clients change and create a more meaningful life for themselves, therapists can support and encourage the Dream in many ways. First, *therapists can listen actively for the issues that genuinely enliven clients, engage their intrinsic interests, and bring them pleasure.* By attending closely to what clients really want to do and inquiring about what feels right for them, therapists can help clients discover and articulate their Dream. More specifically, therapists can ask questions that will help clients clarify what they like, find interesting, or want in order to make a better life for themselves. For example, clients may respond:

- *I want to stop losing my temper—I want my children's respect back.*
- *I want to make peace with my father and bury the hatchet.*
- *I don't want to keep driving to this same dead-end job. I want to finish school and get my credential.*
- *I want to be less reclusive and spend more time with my family and a few true friends.*

For other clients, their Dream is completely undeveloped and they cannot articulate what they want in any meaningful way. Often, their caregivers didn't track their experience



or keep their “mind” in mind very well, and didn’t empathically register what they were feeling or join them in their interests. These clients, with a less developed sense of Self, are unaware not only of what they want to do in life but, more basically, even of what they like or dislike. Therapists can help these clients begin to attend to and discover their feelings, interests, and values, and ultimately formulate and pursue their Dream by repeatedly:

- *inviting clients to attend to what they are experiencing at different moments;*
- *orienting clients to listen to themselves and become mindful of what they want to do in different situations—even if they cannot always act on their wishes (e.g., to realize, “I’m not enjoying this right now—I want to leave”);*
- *being interested in and entering the clients’ subjective experience (“joining” them);*
- *calling attention to, and trying to participate supportively in, whatever seems to hold interest, meaning or enjoyment for clients;*
- *acknowledging and taking overt pleasure in what clients do well; and*
- *encouraging clients to act on their own feelings or preferences when possible (e.g., for some, just beginning with something as simple as recognizing, “I’m sleepy—I’m going to take a nap”).*

Although many clients have a more developed Self than this, some will need help to begin attending to their own internal experience in these very basic ways. It is rewarding for therapists to help clients *differentiate a self* by responding—perhaps for the first time in their lives—to their own feelings and interests. Clinicians working within many theoretical orientations have emphasized this process, such as the concept of “mirroring” in Self-psychology (Kohut, 1977), “experiencing” in the client-centered tradition (Carkhuff, 1999), or “attuned responsiveness” in attachment theory (Bowlby, 1988).

As clients begin to experience their inner life more fully, and the therapist responds affirmingly, their Dream will start to emerge. At this point, therapists can do several things to help clients bring aspects of their Dream into their current lives. For example, therapists can provide a “practicing sphere” in which clients can discuss and explore various possibilities of their Dream without having to become committed prematurely to any action. Therapists can also help clients tailor their Dream to reality and find ways to express aspects of their Dream in their everyday lives. Although most clients can’t quit their day job and become, say, full-time performers, a client who loves music can sing in the choir, take piano lessons, put together a garage band, or get a degree in music. Therapists can also help clients identify training or education routes that will facilitate realization of their Dream, and give clients permission to develop close relationships with others who can act as mentors in their chosen fields and pursuits.

Finally, clients will confront the same core conflicts in exploring their Dream that they have experienced in other aspects of their lives. Thus, therapists will also have to help clients work through variations of their old conflicts that are aroused by pursuing their Dream. In particular, therapists will have to help clients differentiate between their Dream and their interpersonal coping style and family role. This might include strivings to feel secure by being loved by everyone, to disprove their shame and gain a sense of personal adequacy or worth by compulsively pursuing wealth or power, or to escape criticism or rejection by being safely aloof and removed. Progress usually will not be simple or straightforward, however. For many clients, *actively pursuing what they really want to do, or successfully attaining what they want in life, commonly threatens attachment ties and arouses both separation anxiety and separation guilt—leading clients to retreat from progress or undo their success in treatment.* However, as clients successively work through these conflicts and find that they do have a right to their own life, they can get better at following through on realistic plans for attaining aspects of their Dream without self-sabotage. With this success, treatment evolves to a natural conclusion.

## TERMINATION

### CLIENT ENDS TREATMENT EARLY

Premature termination (attrition) has long been recognized as a serious problem in psychotherapy (Arntz et al., 2015; Garfield, 1994; Gelso & Woodhouse, 2002; Reis & Brown, 2006). However, there is no consensus in the field on the definition of premature termination (Dew & Brown, 2005) or real understanding of why it occurs so frequently. Significantly, “drop-outs” are not even reported in the majority of treatment outcome studies (Ravitz et al., 2011).

Does premature termination refer to dropout after the first session or dropout any time prior to clients meeting their treatment goals? Bergin and Garfield (1994) have suggested “premature termination” be considered whenever an individual has attended a session and then discontinues on his or her own by failing to come to future scheduled appointments. According to published data, disturbingly, *approximately 30 percent of clients drop out after completing only an intake*, and 40 to 60 percent before experiencing sustainable benefits. And, on average, clients attend only three to six treatment sessions (Callahan & Hynan, 2005; Garfield, 1994; Hansen et al., 2002; Wierzbicki & Pekarik, 1993). Why are we failing to help so many clients? A substantial focus of the early dropout literature has been on demographic and lifestyle characteristics of those who terminate early. These include people who come from deprived backgrounds, those of minority status, individuals who have a history of substance abuse, and those with a history of involvement with the criminal justice system (see O’Brien et al., 2009). Researchers also find that premature termination is associated with non-contemplators, mandated to treatment, male gender, high anger-hostility, child maltreatment (especially physical abuse) and loss, and other characteristics (Arntz, Rutenfrans & Bloo, et al., 2015).

Although these and other client and therapist features are important, we believe the primary issue for therapists is understanding what clients are actually experiencing in treatment; in particular, considering what clients are not getting or finding in their therapy that leads to such extensive dissatisfaction and disengagement (Muran et al., 2005; Safran et al., 2005; Swift & Callahan, 2008). Research suggests that one important factor implicated in early termination of treatment results from clients’ unmet expectations—i.e., their expectations of what they would gain from being in treatment don’t match what they actually experience. Many therapists have interpreted this to mean they need to engage in further “role induction” (i.e., giving clients more explanations about what therapy entails). We believe, however, that role induction is but a small and structural feature of therapy and not the major reason clients disengage early. We believe that it is the therapy process itself and clients’ *experience*, especially in that important first session as therapists are trying to begin a working alliance, that is key to whether clients will continue in treatment (Norcross & Wampold, 2011).

As clinical supervisors and training clinic directors, we find that beginning therapists struggle with knowing or understanding their own role vis-à-vis the client. They consider one stance to be that of functioning in a guidance or directive mode (here they would tell clients, from a more “expert” perspective, how to structure their day, give them homework, advise them on decisions, etc.). Another role they often take is a passive, “neutral” stance, where they are simply in the same room with the clients, are respectful and nice but convey very little of themselves to clients, avoid questions clients ask as much as possible, superficially reflect what clients say without capturing the most important message or emotion (simple vs. complex reflection), and function as non-directively as possible. Their hope here is that clients will somehow know they are trying to be fully accepting, nonjudgmental, and are not trying to direct clients’ lives or tell them what to do. Still others fall back on what they know best and already do well—adopting the familiar role

of “friend.” These trainees are supportive, conversational and sociably on the surface—avoiding points of entry for distress and conflict that may be awkward or too personal.

The problem with each of these stances, however, is that clients come to therapy because they have real problems and they want to find: a responsive and engaged person who is fully present, clearly wants to be with them, and enters their experience with genuine interest and concern (not self-disclosure); establishes credibility by demonstrating some real understanding about what’s going on for them; has the capacity to more clearly discern the key issues or grasp what’s really distressing them; and has enough expertise to offer them something helpful in a practical way right from the first session. They want the experience of being seen and heard - fully met, and want to feel that their therapist is trying to understand *their* personal issue or problem as they view it and join with them in clarifying what they need. The goal here is that *before clients leave the session, the therapist needs to give clients the experience that there is someone here who is worth all the trouble and expense of coming to talk to.* And, rather than just making assumptions if the session seemed to go well, brings this up and asks clients whether this has occurred (e.g., “How was this first session for you today? What felt good, and what could I do differently next time to be more helpful?”). Thus, in this first session in particular, but also in subsequent sessions, therapists need to:

- *Give clients a sense of hope that they can get the help they need in this setting by “hearing” what is most pressing, painful, or important for them right now;*
- *Help clients clarify what’s really wrong and what they can begin doing now to help with this issue;*
- *Work with clients to discern what clients need most, today, before they leave each session;*
- *Provide hope that by working together it is possible to find ways to make the future be different and see that it does not have to repeat the past; nor do they have to face their challenges alone;*
- *Actively engage with clients and provide them with the sense that therapists are going to be a highly involved collaborators with them on this journey—actively sharing ideas and questions, observations, alternatives, differing points of view, and feedback, but without making decisions or telling them “what to do” or how to lead their lives; and especially,*
- *Re-define the social rules and make it safe to talk more forthrightly together about what is going on in the client-therapist relationship, clarifying that this will be done collaboratively and respectfully, and never in a confrontational, argumentative or blaming manner.*

We find that when student therapists (1) stretch their personal comfort zone and actively engage with clients in these ways right from the beginning; and (2) are able to “get” what is most important to the client and provide empathic understanding, few clients drop out prematurely.

In this vein, Sue & Zane (2009) write eloquently about the importance of offering the client a “Gift”—this needs to be something that demonstrates behaviorally that you, as a therapist, are someone who has the ability to be helpful and make a difference (i.e., demonstrate or achieve credibility). This capacity to offer a Gift, and your behavioral communication that you are going to be a present and actively engaged helper, will have your clients leaving the session with a sense of hope; it also increases clients’ likelihood of returning. This Gift can include:

- *simply giving clients the experience of feeling affirmed, such as understanding and validating the burdensome impact on clients of growing up as a parentified child, or the lifelong anxiety instilled in a child by lying in bed at night and listening to elevated parental wrangling;*

- *letting certain clients know that you understand that some children were not protected, got hit or molested, and that you are a safe person to talk with about those issues that many others have not wanted to hear;*
- *providing clients with basic educational information about sex education or sexual dysfunction so they no longer feel so different or alone;*
- *role-playing how to approach an upcoming situation or person that is particularly troubling;*
- *providing an overwhelmed parent with positive parenting strategies or other ways to regain control and discipline a child, and so forth.*

An important component of all sessions, but the initial session in particular, is that clients leave feeling a sense of connection, collaboration, and hope that they will get the help they need, and not feeling that, primarily, *they are merely answering questions for the therapist so he or she can complete the agency's intake form or make a provisional diagnosis for a new patient chart.* Find a balance: make real contact with clients, provide a Gift if you can, and then tell them that you need to shift gears and ask some different questions in order to complete some needed paperwork. For example:

**THERAPIST:** Welcome. I'm glad you're here. We spoke briefly on the phone, but let's just get to work and try to understand what's troubling you. Why don't you tell me more about what's wrong and bringing you in, and then we can start sorting things through together.

You want your clients to have the feeling that “this is a person who has something to offer me that other people in my life can't offer me right now, and I can benefit from what is being offered.” In terms of role induction, it is useful to communicate to clients that improvement or recovery from treatment takes longer than most expect (e.g., 50 percent of clients improve after eight sessions and most need 13 to 18 sessions; see Swift & Callahan, 2008). Clients may also benefit from knowing that therapy may evoke painful feelings or difficult memories but that you will be with them through all the challenging moments of this journey. As we have seen, however, for therapists working within a process-oriented approach, *role induction for clients also emphasizes education about the therapeutic relationship and redefining social norms to include straight talk about what might be going on between the client and the therapist.* Because we find that this is so effective in successfully helping clients enter treatment, and it seems that few do it with any depth or real engagement with clients' responses, let's illustrate it again. Toward the end of the initial intake appointment, the intake worker can say:

**THERAPIST:** Bob, here's the best way that you can help the therapist help you. We can be more direct here in therapy, and you can let the therapist know what you need, and what is and what's not working for you. Things won't go as well if you just try to be nice or polite, and don't tell the therapist if somethings isn't working for you. How does this sound—is this something you could do?

**CLIENT:** Well, I don't usually talk that that way to others, but I can see that it might be helpful. But what if I say something I don't like or isn't working for me, and the therapist continues the same way—you know, doesn't change?

**THERAPIST:** If you bring up a problem with the therapist or some dis-satisfaction with treatment, and the therapist doesn't listen to your feedback, talk it through with you, and try to respond to it in a way that works better for you, it's probably not a good match. I'd recommend that you ask to be transferred to a different therapist who can work with you as a partner, and can hear and respond to your needs.

Stemming the premature terminations that pervade our field necessitates that in the initial sessions therapists go beyond being “nice” or “passively” present. Therapists need

to be actively engaged, assume a consistent **mentalizing perspective** in which they have the client's "mind in mind," so that what is most distressing or urgent for this client is carefully discerned and clearly articulated (Allen, 2013; Fonagy & Bateman, 2006). Following this sustained accurate empathy, therapists will need to respond in the corrective, reparative ways we have been discussing. If this is accomplished, the client and therapist will have co-created a productive therapeutic space where termination is most likely to occur only after meaningful and enduring treatment goals have been met.

### CLIENT AND THERAPIST TALK ABOUT THERAPY ENDING

Termination is an important phase of therapy and needs to be negotiated thoughtfully (Binder, 2004; Gelso & Woodhouse, 2002; Wachtel, 2002). Ending the therapeutic relationship will often be of great significance to clients. For some, this may be the first positive ending of a relationship they have ever experienced. How it is managed is so important that it often influences how well clients will be able to manage future separations, endings, and losses in their lives. If carefully planned, it provides an opportunity to convey to clients that they have been valued, allows therapists and clients time to talk together about areas of growth and anticipate where clients' future challenges may emerge, influences clients' ability to enter into new relationships in the future, and provides a healthy model for ending relationships (Many, 2009; Rappleyea et al., 2009). Most beginning therapists greatly underestimate what an important experience a successful termination can be for clients. It holds the potential to diminish the changes that have come about in treatment or to help clients consolidate and further their gains—and better be able to maintain these changes after treatment has ended. Therapists will want to make the most of this significant opportunity for further change.

Early in their training, therapists often ask, "How will I know when it's time to end therapy?" It is time to end when clients have achieved significant relief from their symptoms, can respond more flexibly in current situations rather than slotting diverse experiences into narrow, predisposed categories, and have begun to take steps toward promising new directions in their lives. Therapists know that clients are ready to terminate when they receive converging reports of client change from three different sources:

- *from clients—when clients report that they consistently feel better and can respond in more adaptive ways to previously problematic situations;*
- *from their own observations—when clients can consistently respond to the therapist in new ways that expand their old coping styles and do not reenact their maladaptive relational patterns; and*
- *from significant others in clients' lives—when people give clients feedback that they are different or make comments such as, "You never used to do that before."*

Before going on to discuss what therapists need to do to make terminations successful, we must distinguish between two types of endings: natural and unnatural.

The termination sequence just described is a **natural ending** because the clients' work is finished. One of therapists' primary goals in these natural endings is to affirm both sides of clients' feelings about ending—wanting to go but also wanting to be supported. Thus, *therapists are trying to provide a secure base during the termination phase* by taking unambiguous pleasure in clients' independence and actively supporting their movement out "on their own." However, a Secure Base also means providing continuing support as clients differentiate and emancipate, so *to the extent possible*, therapists can let clients know that they will accept their request for help or contact in the future if the need arises. Also, clients' old schemas, transference distortions, and faulty expectations will often be reactivated toward therapists as they begin to discuss ending. To correct these transference distortions, therapists need to clarify overtly that they will not be



disappointed, burdened, or troubled (as many clients will inaccurately expect) if clients want to send a letter to share important life events such as marriages and births, or to contact the therapist for help or a referral if they have problems in the future.

The key issue here is that so many clients have not had permission to be both separate and related at the same time. For example, growing up, becoming more independent, and differentiating from their caregiver by pursuing their own interests, goals, and relationships often meant losing their emotional support and attachment ties. Or, staying connected and emotionally supported too often meant being compliant and remaining dependent or not having their own voice, views, or feelings distinct from those of the people they were with. Thus, the termination phase provides these clients with an important opportunity to further increase their capacity to be both separate and related at the same time, and enhance their capacity both for greater autonomy and intimacy in the future.

Therapists offer many clients a far-reaching reparative experience when clients are assured of the therapist's support for both sides of their ambivalent feelings. Many clients benefit greatly from finding that therapists take pleasure in their success and welcome their independence yet are still available and interested in their well-being if clients need to contact them (or another therapist) in the future. With this support for both sides of this separateness-relatedness continuum, clients can further internalize the therapist and keep all that was offered, solidify their own sense of self and personal efficacy, and successfully terminate.

Unfortunately, most graduate student therapists do not have the satisfaction of seeing many cases evolve to a natural close. Instead, therapy often ends before clients are finished for a number of reasons. As discussed previously, sometimes clients initiate the termination and simply do not return to the next scheduled appointment. In better circumstances, therapy ends because graduate students must move on to another placement, the school year ends, or the training clinic provides only time-limited services. Sometimes the therapist and client have made substantial gains when treatment has to stop, and other times the treatment process may be only in midstream and much is unfinished. In any case, significant therapeutic gains can still be made during the termination phase. To accomplish this more difficult task, however, it is essential that therapists and clients address and work through the complex issues that an unnatural or externally imposed ending arouses. Because graduate student therapists often have to initiate terminations before clients are ready, we will focus on **unnatural endings** in the discussion that follows. It is necessary to examine unnatural or imposed endings closely because *they routinely evoke angry, blaming, and distancing reactions in clients*. These expectable responses are often difficult for new and unexpecting therapists. Recalling "client negativity", this can readily activate therapists and evoke guilt, defensiveness, and other countertransference reactions that may diminish gains that have been made.

When it has come to hold meaning for both, it is often difficult for both therapists and clients to end their relationship. This occurs commonly because the ending stirs up unresolved losses that are far more intense and significant than either the therapist or the client had anticipated (e.g., evoking loved ones who have died; divorces, where a child loses a relationship with a parent in the aftermath of the breakup; important relationships that ruptured and could not be repaired; emotional deprivation and other lasting disappointments with a caretaker; attachment figures who became emotionally unavailable because of alcohol or drug abuse; parental anxiety and awkward disengagement as a child enters puberty). As a result, *therapists and clients regularly collude to deny the reality of the impending ending and avoid the difficult feelings it arouses for both of them*.

When therapists and clients stumble in this way and do not address the ending squarely, their interpersonal process will often reenact aspects of clients' core conflicts and prevent them from resolving their problems as fully as they could have. As we

highlight, this becomes especially important with clients who have a history of painful and unresolved interpersonal losses, which so many clients bring to treatment. In particular, clients who received little validation or support for previous interpersonal losses now need a new and reparative type of ending with the therapist that differs from the unacknowledged, unpredictable, unplanned, and unpaced losses in their pasts (Many, 2009). Thus, *the single most important guideline for negotiating a successful termination is to unambiguously acknowledge and sensitively address the reality of the ending*. Even with clients who are ready to terminate treatment, old issues of loss and abandonment are unexpectedly but commonly evoked as termination approaches. Consider the following:

**EFFECTIVE THERAPIST:** We have three more sessions left before the school year ends and we have to stop. Let's talk about what this ending is going to mean for both of us.

The therapist and client can then discuss all of the client's positive and negative reactions about the ending, *especially the client's emotional reactions toward the therapist*. Although therapists and clients may both want to avoid this topic, therapists cannot let that happen. Once a therapist and client mutually decide that the client is ready to terminate, or that the therapist must terminate because of external constraints, they need to establish the specific date for the final session and then explore the client's reactions to ending their relationship. For example:

**THERAPIST:** Our last session will be in three weeks, on Thursday, December 9, at 4:00. What are your thoughts as I acknowledge that?

Because therapists and clients need time to work through their ending, the final session should be scheduled at least two or more weeks hence. If a client announces, out of the blue, that he or she has decided to terminate today and this is the last session, the therapist should invite and encourage the client to return just one more time in order to sort through what this ending might mean for the client and to have a better opportunity to say goodbye to each other. We always want to support clients' individuation and autonomy, but this type of abrupt and unilateral ending is usually a red flag for some type of reenactment or rupture.

In some cases, it will be difficult for therapists to set a specific termination date and discuss the ending because this arouses therapists' own conflicts over the ending—or more commonly, endings in general. Further, setting a clear termination date will also often bring to the fore clients' presenting symptoms and core conflicts again. For some clients, it may be necessary to rework aspects of these issues once more. It is not uncommon for some clients to retreat temporarily from the changes they have made when termination becomes a reality, or to have difficulty attending the next session. In particular, therapists may need to share with certain clients (e.g., clients who have experienced extensive loss in their lives that they are concerned that clients might “forget” the last session and will want to talk through this unwanted possibility with them). For example:

**THERAPIST:** If I am sitting here alone next Thursday at 4:15, what will have kept you from coming?

For all of these reasons, *therapists need to acknowledge the termination forthrightly*, count down to the final sessions, and invite clients to discuss their reactions to the termination by asking questions and making statements such as:

- *After today, we will have only three sessions left. How is it for you to hear me say that?*
- *We have only two sessions left now, and I think it's important that we talk together about our ending. What comes up for you when I acknowledge that soon we are going to have to stop working together?*

- *Next week will be our last session. We've accomplished some things together, but problems remain and our work is unfinished. What do you find yourself thinking about our relationship and the time we have spent together?*
- *This is the last time we are going to be able to meet. I'm feeling some sadness about that, and I'm wondering what kind of feelings you're having.*

This type of *countdown* precludes any ambiguity, bargaining, or denial that clients may have about the ending. For example, it will prevent clients from asking, at the end of the last session, “So, will I be seeing you next week?” It also keeps the therapist from acting out and avoiding the separation as well—for example, by saying at the end of the last session, “Oh, I see our time is up. Well, I guess that’s it.”

When therapists do not address the termination directly they are often avoiding their own separation anxieties or unresolved losses. They may have their own unresolved feelings from unwanted and painful endings in their own lives that were not responded to supportively and that they had to negotiate on their own—which is problematic because they dovetail so often with their clients’ own unresolved feelings over past painful endings. Consequently, the termination phase is especially likely to evoke therapists’ own potential for counter-transference, and expressed via their attachment style. For example, Dismissive therapists may not be able to approach and empathize with the client’s sad or vulnerable feelings about endings, loss and grief. Preoccupied therapists can hyperactivate as their heightened sensitivity to loss and abandonment are activated. This challenges their ability to differentiate their own feelings from the client’s distress and provide a holding environment. In these ways, unresolved conflicts over ending relationships may be the most common countertransference issue that therapists bring to treatment. When this occurs, therapists can reassure themselves that these are common and understandable problems to have, *but they also need to do something about them*. They can seek consultation from a supervisor or colleague, or better, find a therapist who can understand and help resolve this. Studies find graduate clinicians’ personal therapy is associated with significantly better treatment outcomes and lower drop-out rates for their clients (Gold & Hilsenroth, 2009; Gold et al., 2014).

Because both clients’ transference distortions and therapists’ countertransference reactions are especially likely to occur around unnatural terminations, and at the end of time-limited treatments, we need to explore these issues further. Both therapists and clients often have experienced painful endings with significant others as “just happening to them.” In many cases, they:

- *were not prepared in advance for the separation,*
- *did not understand when or why this particular ending was occurring, or*
- *were not able to participate in the leave-taking by discussing it with the departing person or by saying goodbye.*

Such problematic endings have left many therapists and clients feeling powerless in regard to some of the most important experiences in their lives. In contrast, the approach suggested here enhances clients’ efficacy by allowing them to be active, informed participants in the ending. Clients know when the ending is occurring (experience some predictability), have the safety to express their feelings (whether appreciative, disappointed, or angry), receive a non-defensive response, and have input on future challenges. Why is this so important? Especially with unnatural endings, they often activate early maladaptive schemas and leave clients feeling that *this relationship has just failed in the same way that others have in the past*. Let’s explore this expectable problem more fully.

As the old saying goes, children are supposed to grow up and put their feet under their own table, but it’s hard to get up from the table when you’re still hungry. The emotional deprivation that many clients bring to therapy is activated when treatment ends

prematurely or before they feel ready to stop. Routinely, *clients' anger and disappointment about what they have wanted but missed in formative relationships—even their sense of abandonment or betrayal—may be directed toward therapists*, no matter how clearly the treatment parameters were explained at the beginning of treatment. The following guilt-inducing accusations from clients can be very difficult to hear for new and experienced therapists alike:

- *You're just like my mother. You're leaving me, too.*
- *Our relationship never meant much to me, anyway.*
- *Nothing's really changed for me in therapy, I'm just the same.*

The success of treatment in these cases depends in part on *therapists' ability to remain non-defensive and tolerate clients' protests*—rather than feel guilty, become defensive, and make the following ineffective attempt to talk clients out of their feelings:

**INEFFECTIVE THERAPIST:** But you have changed. You're not as depressed as you used to be, and now you can do \_\_\_\_\_, and before you couldn't do that.

Instead of these defensive, futile attempts, therapists need to begin by acknowledging clients' immediate feelings. Consider the following response:

**EFFECTIVE THERAPIST:** You're really angry at me as our relationship is ending, and I respect those important feelings. Right now, it just seems like nothing meaningful has occurred and that nothing has gotten better for you.

By accepting clients' angry protests in these ways—without agreeing that their factual content is true—therapists give clients a reparative response that doesn't match their IWMs or previous experiences. In turn, this affirmation of their current frustration or disappointment may allow clients to connect their strong emotional reactions to the developmental figures who originally disappointed or left them. Thus, *what therapists often find hardest about unnatural endings—and about time-limited treatment—is managing their own guilt or defensiveness in the face of clients' anger over the ending*. However, by drawing on the guidelines outlined here, therapists can help clients see how this ending is in fact different in some important ways from other unwanted endings in their past—even though it feels as if the same unwanted scenario is happening to them again. Therapists can help resolve this **unfinished grief work** from the past in the following ways, by:

- *talking with clients about the ending in advance;*
- *inviting clients to share their angry, disappointed, or sad feelings and being able to accept those feelings without becoming defensive;*
- *talking with clients about the meaning the termination holds for them, and clarifying how it is similar to, and different from, other important endings in their lives;*
- *validating clients' experiences by acknowledging the ways that this ending may be evoking other painful or unwanted endings with others;*
- *discussing the meaning this relationship has held for the therapist and sharing some of the therapist's own feelings about ending this relationship; and*
- *ensuring that therapists and clients can say goodbye to each other.*

In other previous endings that have been problematic, clients were not able to have such experiences with the departing person. Although most clients will have difficulty recognizing these differences initially, therapists offer clients an opportunity to resolve long-standing problems by helping them differentiate this type of mutually acknowledged ending from the incomplete or unsatisfying endings with which they have had to cope in the past. In this way, successfully managing the termination also becomes the prototype for learning how to cope effectively with future endings and losses. One of the

illuminating pioneers in short-term treatment (Mann, 1973), and others since (Binder, 2004), write informatively about these issues and the strong feelings of loss that are activated for so many clients during termination.

In the face of clients' withdrawal or blaming accusations that their termination is "just like" prior losses and disappointments, therapists, too, may lose sight of the real differences that exist between this termination and clients' problematic past endings. Therapists need to keep in mind that this ending really isn't "just like" previous losses (e.g., there was a date mutually set for the ending; the client can, within reason of a few weeks' time, choose the exact ending date; they are talking about the ending and the client is not suddenly being left without explanation or preparation). When therapists lose sight of these real differences, they are far more likely to accept the clients' blame, feel guilty, and avoid the ending—or respond defensively and invalidate the clients' sad, disappointed, or angry feelings by trying to talk clients out of them. When this occurs, paradoxically, therapists make clients' accusations come true: They metaphorically reenact important aspects of clients' past conflicted endings. Therapists do better by acknowledging the similarity of clients' feelings at termination and at past separations, and then exploring with clients what they can do together to make this termination different and better from such problematic endings in the past.

As therapists track clients' reactions to the ending, they will often see how clients' core conflicts are simultaneously being aroused in two contexts. First, clients' primary feelings about termination (such as guilt over feeling better and no longer needing the therapist; or feeling rejected, abandoned, or let down by the therapist) may match clients' feelings about the formative relationship in which the core conflict originally arose. Second, clients' reaction toward termination may also match the feelings that were aroused in the crisis situation that originally brought them to treatment. By responding affirmingly to clients' feelings, and clarifying their connection to these two sources when appropriate, therapists help clients further resolve their core conflicts.

Finally, to help with all of this challenging material, clinical researchers (Gelso & Woodhouse, 2002; Marx and Gelso, 1987) emphasize three main steps in effective terminations:

1. *looking back and reviewing what has changed;*
2. *looking ahead and making realistic plans for coping with the problems that are likely to come up; and*
3. *saying goodbye.*

Exploring effective terminations further, a useful intervention for the termination phase is to go through a **Review-Predict-Practice sequence** with all clients. Therapists review progress, accomplishments, successful changes and transitions, and unfinished issues with clients. Especially important, *therapists help clients predict the challenging events or anxiety-arousing situations that are likely to activate old wounds or bring up their dysfunctional coping patterns in the future.* To help clients maintain the changes they have achieved, it is essential to anticipate these potentially problematic situations or relational scenarios. Therapists want to identify these activating circumstances or triggers as specifically as possible. As illustrated with Tracy, earlier in this chapter, the therapist might ask:

**THERAPIST:** What is your Dad likely to say and do if you speak up and set limits with him? And how is that going to make you feel inside, and how are you likely to behave—what are you initially going to want to say and do, when he does that?

Finally, practice in dealing with these **triggering issues** or familiar patterns also prepares clients to succeed on their own. Therapists can provide this by role-playing effective and ineffective responses to the threatening or unwanted interactions that clients are likely to face in the future.



## ENDING THE RELATIONSHIP

Although some clients will re-experience their presenting problems at the time of termination, many will not. Both clients and therapists still need to talk about the ending, however. For example, therapists can talk with clients about the different ways in which they have seen clients change over the course of treatment. Therapists can also acknowledge the limitations of their work together and the unfinished issues that clients will need to continue to work with on their own or perhaps with another therapist at some point in the future. Fearing that they are being disloyal, some clients will need explicitly spoken permission from therapists to work with another therapist if they have problems in the future. Therapists need to reassure these clients that they would be happy to know that clients are taking good care of themselves and are getting the help they need. Recollections of close moments, awkward misunderstandings, risks ventured, and humorous incidents also can be shared. Therapists can also share some of their own feelings about clients, including the significance of a particular relationship for the therapist and what the therapist has learned, enjoyed and will remember about this person.

In natural and unnatural endings, we have seen that termination brings therapists and clients back to the issue of being both separate and related. In the case of natural endings, therapists will want to give clients permission to leave. Clients need to know with absolutely no uncertainty that the therapist enjoys their success, is pleased by their independence, and takes pleasure in seeing clients become committed to new relationships and activities. Therapists want to be a Secure Base and help clients celebrate and feel supported as they transition out of treatment into a new phase of life. Clients:

- *having connected with the therapist, can now feel more competent or confident about relating to others;*
- *having struggled with emotional or behavioral dysregulation and needed help with this, can now self-regulate with more balance or flexibility; and*
- *having arrived at an understanding that change is also an opportunity, can view this “loss” as signaling the transition to the next stage in life (in much the same way leaving home to attend college consists of both “loss” and “success”—so that feeling nervous or sad, as well as feeling excited and delighted, both make sense).*

A thoughtful termination is extremely helpful in clients’ ability to trust in the possibility that future relationships can be nurturing and stable. The model presented by the therapist is important—it conveys that stable, emotionally nurturing, and attuned relationships do exist, and that these can end in caring, manageable, and affirming ways. Addressing termination overtly is also an opportunity to observe mutually clients’ “developmental progression” and how much more connected they now are to their own voice and authentic self or how much more confident they may now be about engaging with others. Although treatment is often incomplete when the relationship comes to an end, when clients can live out with others what they have experienced with the therapist, and therapists can appreciate what they have learned from and given to the client, both will have been enriched by this relationship.

## CLOSING

This book has followed the course of therapy from beginning to end and provided a foundation for clinical training. Just as clients still have unresolved problems when treatment ends, many questions about therapy still remain unanswered for therapists-in-training. Challenging personal issues and complex interpersonal processes have been introduced here, and therapists will need further reading, supervision, and experience to help with the myriad exceptions that occur in every therapeutic relationship. However, readers have

learned essential guidelines for understanding the interaction taking place in therapeutic relationships, and for using the process dimension to help clients change. The conceptual framework presented here will help new therapists work more effectively with their clients and, we hope, better integrate training from a variety of theoretical perspectives.

The theme of this text is that enduring and substantive change occurs within the context of significant relationships. Therapists working with client-centered, cognitive and behavioral approaches, interpersonal and dynamic frameworks, attachment, family systems and other theoretical traditions can all utilize the interpersonal process approach to make their own work more effective. Accordingly, therapists are encouraged to use themselves, and the relationships they provide, to help clients change.

### CHAPTER EXERCISE

Thinking of a major loss you have experienced (e.g., the death of someone important to you or unexpectedly and suddenly being left by a significant other), what would you have liked others to have said to you? If you didn't want anyone to try and respond to your loss, what contributed to that feeling in you?

### SUGGESTED READINGS

Developing therapists who have found the ideas presented here helpful, but new and challenging, are encouraged to read the chapters you need help with from the excellent text on basic skill development, *Helping Skills: Facilitating Exploration, Insight, and Change*, 4/E (Hill, 2014). More experienced trainees who would like to enter further into this way of working with clients are encouraged to read: *Interpersonal Reconstructive Therapy* (Benjamin, 2006), *Between Therapist and Client* (Kahn, 1997), *Brief Dynamic Therapy* (Levenson, 2010), and *A Secure Base: Clinical applications of attachment theory* (Bowlby, 1988).



Visit [CengageBrain.com](http://CengageBrain.com) for a variety of study tools and useful resources such as interactive exercises, flashcards, and quizzes.



## APPENDIX A

# Process Notes

Client \_\_\_\_\_ Session Number \_\_\_\_\_ Date \_\_\_\_\_

1. **Issues Presented.** Key concerns or distress expressed by client or noted by therapist:

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2. **Themes.** Repetitive patterns that the client presents recurrently (e.g., feeling alone, unappreciated, overwhelmed):

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3. **Relationship and Resistance.** Did the client make any overt statements or covert references (embedded messages) about you or your interaction together (e.g., difficulty trusting, not feeling heard or understood)? Did the client exhibit any ambivalence or concerns about being in treatment (e.g., feeling disloyal to family, discomfort with asking for help)?

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4. **Interpersonal Process.** In this session, did you and the client become more engaged with each other? What can you learn about your client's problematic interactions with others from your own personal reactions toward the client (e.g., do you feel bored, frustrated, protective)? How is your interaction or internal response similar to or different from the way others respond to the client?

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5. **Transference and Countertransference.** Are there characteristic ways in which the client tends to distort or misperceive you and others? Does this client respond to you as he or she does to significant others—or differently? Are there ways in which your

own personal history or current life circumstances are activated by this client and influence how you respond to him or her?

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6. **Evaluating Interventions.** What were your primary interventions (e.g., empathic understanding, reframing, role-playing)? What was effective or ineffective? What might you want to do differently next time?

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7. **Treatment Focus.** Did the client present issues in this session that were closely related to your treatment focus? Were you able to use these as points of entry to intervene and provide more focus and direction in this session?

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8. **Critical Incidents.** Identify potential legal, ethical, or reporting concerns for consultation with supervisor (e.g., child, elder, and dependent abuse; danger to self and others):

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# Guidelines for Treatment Planning

Although writing a case formulation is often challenging, completing one is important because it helps therapists provide more focused treatments.

1. **Formulation of the problem(s).** Summarize the client's presenting problem(s). Why is the client seeking treatment now? What organizing themes or history characterize the client's most important experiences with others?
2. **Treatment focus.** Identify the maladaptive relational patterns and beliefs that recur for the client. Thinking of the client's IWMs, ask yourself the following questions: (1) What does the client want from others (e.g., to be cared for, to have autonomy supported)? (2) What does the client expect from others (e.g., to be criticized, ignored, abandoned)? (3) What is the client's experience of Self in relationship to others (e.g., a sense of being unimportant, burdensome)? (4) What conflicted feelings are typical for this client (e.g., anger, anxiety, shame)? (5) What interpersonal coping strategies are characteristic for this client (e.g., to comply and go along, to withdraw and convey no need for others, to dominate and take control)? (6) What do these coping strategies tend to elicit from the therapist and others (e.g., disinterest, advice giving)? Use your responses to these questions to identify two or three primary treatment focuses that clarify what's wrong and needs to change for this client.
3. **Developmental context.** What are the familial and cultural issues that have shaped the client's presenting problems and worldview? How did the family nurture, discipline and communicate? In other words, how did the client's early maladaptive schemas, interpersonal coping strategies, and attachment styles develop?
4. **Therapeutic process.** Describe the quality of the working alliance and how you and the client have interacted together. How are you trying to make the client's relational experiences with you repair or disconfirm problematic experiences in other relationships? When the client is distressed, how does he or she tend to misperceive and react to you? Have you been able to address potential problems between you and work through rupture-repair episodes?
5. **Goals and interventions.** As you focus on the two to three core treatment issues identified in item 2, indicate how you plan to address these. Articulate as clearly as possible the specific corrective experiences that this client needs from you to meet the treatment goals identified. How are you helping this client to generalize new experiences



with you to relationships outside of therapy? Consider also what referrals might be appropriate (e.g., group counseling, medication). In sum, clarify where you want to go with this client and how you plan to get there.

6. **Impediments to change.** Anticipate why treatment could fail. If the client terminates prematurely or treatment is not successful, what factors might contribute? What client characteristics are most likely to impede sustaining a strong working alliance? Are there any significant personal, familial, or cultural factors that may make it difficult for this client to acknowledge having problems or to ask for help? Are you able to talk with the client about how you work together and what you could do to make treatment more helpful? Most importantly, suggest how the client's maladaptive relational patterns with others could subtly be reenacted with you in the therapeutic relationship.



## GLOSSARY

**Accurate empathy:** the therapist's ability to understand or discern the primary meaning, or correctly perceive the subjective experience the client presents, with clarity and specificity.

**Achieved credibility:** the credibility achieved by therapists due to their skillfulness and the effectiveness of their responses, rather than merely their status, role or similarity to the client.

**Active stance:** the balanced therapeutic stance that is active, responsive, and engaging—yet neither primarily directive nor non-directive.

**Adult attachment styles:** the relational styles or patterns of attachment in adult attachment relationships; including Secure, Preoccupied, Dismissive, and Fearful.

**Affect regulation:** children's increasing capacity to independently and successfully manage disruptive feelings of distress, fear, vulnerability or need on their own. Children's capacity for self-regulation is based largely on internalizing their caregiver's ability to consistently hear their distress and respond by providing comfort or "psychological holding."

**Affective constellations:** a sequence of interrelated feelings that often occur together as a predictable, patterned sequence. The first or initial presenting feeling on the surface is often the least difficult to express and covers deeper, more vulnerable feelings.

**Affective containment:** a holding or psychologically safe environment provided by the therapist that facilitates clients' ability to address, explore, more fully experience and share their cardinal issues.

**Ambivalence:** the competing, mixed or even contradictory feelings that clients experience with regard to a particular situation or decision, or toward their therapist and the therapy.

**Ambivalent feelings:** the simultaneous conflicting feelings experienced by clients during the course of their therapy.

**Anxiety:** the elevated state of apprehension experienced by clients, which is often exacerbated because the client cannot name the danger or identify the source of their fear.

**Ascribed credibility:** the credibility that clients ascribe to their therapist based on the therapist's role, professional credentials and status, or perceived similarities such as gender, ethnicity and religion.

**Attuned:** A state of being attentive and aware of the specific feelings or personal experience of the other.

**Attuned responsiveness:** the ability of therapists to respond readily and positively to the client's emotional needs by accurately discerning the client's needs or distress.

**Authoritarian:** an approach to discipline that is strict and controlling, and also lacks warmth and explanation for rules. It can often be punitive and is used by parents to enforce obedience and compliance in their children.

**Authoritative:** an effective child-rearing style with firm limits and reliably enforced rules. However, these effective parents also are affectionate, clearly communicate their expectations, and explain the reasons for their rules and decisions.

**Between-group differences:** the differences in the outcome of therapy based on therapists' use of different theories or treatment approaches, such as cognitive-behavioral versus interpersonal-dynamic treatments.

**Case formulation or case conceptualization:** the therapist's formulation of clients' issues and functioning, including how clients' problem came about developmentally, and how this is now contributing to or amplifying symptoms and problems in their current life.

**Change talk:** clients' expression of their own desire or motivation to make changes in their attitude or behavior.

**Characterological affect:** the recurrent affective theme or predominant feeling that some clients may experience as a defining or primary aspect of their existence.

**Client response specificity:** the construct used by therapists to flexibly tailor their responses to better fit the specific needs of each individual client.

**Client-induced countertransference:** the therapist's reaction to the client that occurs when the client's eliciting maneuvers or coping styles typically evoke similar reactions in the therapist and in others.

**Collaborative:** where the therapist and client work together as partners rather than the therapist directing or leading the client from a position of "expert" and prescribing solutions or providing the client with answers.

**Compacted phrase:** a phrase that clients tend to repeat that encapsulates or condenses the meaning or primary emotional reactions the client has toward the stressful events or interaction patterns that have repeatedly occurred.

**Compliance:** Disposed to passively follow the therapist's lead during therapy, believing they need to suppress their own interests and goals and seek direction from the therapist. Problematically, these clients have learned that it is unacceptable for them to assert their own thoughts and preferences, so they follow rather than participate collaboratively with the therapist.

**Compliance issues:** the symptomatic tendency of some clients to believe they must give up their own self-direction, Self or own voice and go along with others in close relationships, and only do what others want and expect of them.

**Compliant:** See Compliance, Compliance issues.

**Conditions of worth:** the behavior or roles that children learn they must do to maintain their parents' approval, and to avoid—or cope with, the painful separation anxieties evoked when parents employ disciplinary techniques based on love withdrawal.

**Containment:** the safety, support and validation that therapists offer clients so they can experience their emotions more fully in the moment without fear of judgment or criticism.

**Contextual remarks:** prefatory comments that facilitate the bid for more open communication between therapists and their clients. These comments help clients understand their therapists' good intentions as they shift from surface conversation to a more straightforward approach that invites deeper personal engagement.

**Contextualize:** listening for relational patterns and affective themes, and trying to understand the client's problems or subjective experience within the broader developmental and cultural context of his or her life.

**Core conditions:** Carl Rogers proposed three necessary ingredients for therapeutic change—empathy, genuineness, and unconditional positive regard. Extensive empirical support for therapists conveying these basic attitudes and behavioral characteristics to clients, and for empathy in particular, has been found.

**Core conflict:** the most basic or central issue that links the different problems and concerns that clients present which, in turn, provides a focus for treatment.

**Core meaning:** the most basic feeling, key concern, or central meaning in the issues that a client experiences.

**Corrective Emotional Experience (CEE):** a reparative, real life experience with the therapist that resolves rather than seemingly repeats a familiar but maladaptive relational pattern. The therapists new and reparative response disconfirms the clients' feared but expected responses, which are based on unwanted, hurtful past experiences.

**Cost/benefit analysis:** Therapists help clients do a cost/benefit analysis by having them evaluate both the costs and the benefits of the decisions they are considering, as

well as the relational patterns or coping styles they have adopted.

**Countdown:** when therapists acknowledge forthrightly that therapy is about to terminate, overtly count down to the final session, and repeatedly invite clients to discuss their reactions to the termination.

**Countertransference:** the therapist's personal reactions to the client based on the therapist's own life history and unresolved issues or current personal stressors.

**Countertransference reactions:** see Countertransference.

**Credibility:** the extent to which therapists develop Achieved Credibility by behaviorally demonstrating that they are trustworthy and capable of providing assistance versus Ascribed Credibility which is based only on degrees, titles and roles.

**Deactivating:** a coping strategy or form of affect regulation observed in dismissive clients in which they minimize their needs, suppress feelings of vulnerability, and see themselves as strong and competent.

**Deactivation:** See Deactivating.

**Debrief:** feedback provided to the client regarding an important event or experience that just occurred, often used to help the client understand the event more fully or clarify their perception of what occurred.

**Deconstruct:** to consider the developmental, familial and cultural influences that contribute to a client's feeling or experience, that helps both therapist and client understand a reaction or response that doesn't initially make sense until viewed or framed within this broader perspective.

**Differentiate:** See Differentiation.

**Differentiation of a self:** to become one's own person and be able to recognize and act on one's personal likes and dislikes rather than automatically complying with familial rules, roles, and expectations. The ability to discriminate one's own feelings from others and not over-identify with or over-react to others.

**Differentiation:** to clarify differences in beliefs, values or feelings between oneself and others. To be able to discriminate one's own perspective from that of others', the capacity to know or have one's own mind and function more independently.

**Disempowered:** to feel powerless and incapable of acting on one's behalf; inefficacy.

**Disengagement:** the tendency to be emotionally removed or too personally distant, often lacking in empathy—both for oneself and for others.

**Disengaged Parenting:** a neglective style of parenting in which the parents are emotionally uninvolved with the child, and fail to provide guidance or show warmth toward the child. They are usually passive, self-absorbed and unresponsive, but can be overtly rejecting.

**Dismissive:** See Dismissive client.

**Dismissive client:** clients who are low in overt or expressed attachment anxiety (although they experience high physiological distress or cortisol activation when their needs come into play in relationships), and high in avoidance of relationships.

**Double binding family communications:** the discrepant or contradictory communication within a family which places the child in a confusing lose—lose position. S/he is “damned if you do” say something and equally “damned” if you fail to do or say the same thing. The key feature in double binding communications is that the offspring feels bound by not being allowed to meta-communicate and make this double bind overt—such as by being able to say, “no matter what I say, it will be wrong.”

**Early maladaptive schemas:** the stable and enduring beliefs and expectations that develop from ongoing patterns of parent–child interaction that are inaccurate, hurtful, and dysfunctional, and which extend on into adulthood.

**Earned security:** the sense of security that clients achieve by addressing the problematic life experiences that originally engendered their insecure attachments, and working through them later in life through reparative relationships with others who are more responsive, empathic and affirming.

**Eliciting:** the defensive/coping behavior of evoking or trying to bring about a certain type of response in others—such as intimidation or caretaking.

**Eliciting maneuvers:** see Eliciting.

**Eliciting pull:** the feelings or reactions drawn out in others, the way the client makes the therapist tend to want to respond in a certain way—such as withdrawing, giving up, or feeling bored. The force or effect of eliciting maneuvers on the other person.

**Emancipation Conflicts:** anxiety or problems that emerge as late adolescents/young adults explore and try to develop their own beliefs, interests and values, and struggle to negotiate the developmental hurdle of emancipation or launching from their families-of-origin and establish a more independent adult life.

**Embedded message:** a message that is not conveyed directly but is underlying in what is said, such as making a stereotypic, derogatory comment about females in general that also serves as a covert communication to this client’s female therapist.

**Emotional regulation:** the capacity to modulate, regulate or effectively exert self-control of one’s emotions, impulses or behavioral reactions.

**Empathic understanding:** a genuine feeling of warmth, concern, and acceptance of others. This is accompanied by an accurate, specific and in-depth understanding of clients’ problems within a developmental, familial and cultural

contextualization—which enables therapists to make sense of clients’ seemingly irrational responses.

**Empathy:** See Empathic understanding.

**Encoded reference:** the analogues or metaphors used to indirectly describe what is going on between the therapist and the client. This occurs when the client is discussing another relationship that serves as the overt context for the parallel reference to the therapist or what is transpiring between the therapist and client.

**Enmeshment:** emotional over-involvement or binding closeness that doesn’t allow for healthy autonomy or differentiation, which may be expressed through differing opinions, points of view or the ability to express disagreement with certain family members.

**Excessive accommodation:** the compliance dynamics of clients that leads to the belief that they need permission for all they do, and therefore submerge their own goals and interests to “go along” with the demands of others. Oftentimes, this includes asking the therapist to tell them what to do, while simultaneously resisting the control they have just elicited.

**Experiential relearning:** the process by which a client learns new behaviors, attitudes, or beliefs based on in vivo re-learning or the reparative relational experiences the therapist behaviorally provides.

**External validation:** the feedback from an external source that affirms the legitimacy or appropriateness of something the client has thought, felt or done.

**Family-of-origin work:** the psychological work associated with exploring and better understanding how family interactions and relationships have shaped clients in becoming who they are and the problematic ways they are still responding in the present.

**Fearful:** See fearful client.

**Fearful client:** clients with a Fearful attachment style lack a consistent strategy for coping and utilize less adaptive, vacillating or unstable coping styles. Confusing to themselves and others, these back and forth clients are both high in attachment anxiety and in attachment avoidance. Maddeningly, they cannot sustain being close or apart.

**Flexibility:** therapists’ ability to relinquish control and tolerate ambiguity. Rather than dogmatically following one theoretical orientation, therapists assess the client’s responses to their interventions and modify them to work more effectively for this particular client.

**Genograms:** a three or four generational, graphical representation of family roles, rules and relationships that highlight problematic patterns that repeat across generations and continue to shape clients’ current functioning.

**Gift:** especially at the beginning of treatment, therapists aim to provide clients with a “gift,” a practical skill or a coping strategy that offers some immediate help and engenders

hope, which fosters the working alliance by helping clients feel seen and responded to.

**Grief work:** helping clients better come to terms with unresolved feelings of loss and longing for a loved one who has died or physically left, and from emotional deprivation resulting from unresponsive attachment figures.

**Guilt:** a feeling of remorse for a transgression.

**Hierarchical relationship:** a one-up/one down relationship in which status or authority takes precedence. People involved in a hierarchical relationship are perceived as holding different levels of authority based on some predetermined basis such as power, position or role of “expert.”

**Holding environment:** a safe haven of empathic understanding and attuned responsiveness that therapists provide to help modulate or contain the client’s distress or disruptive feelings.

**Homeostatic mechanisms:** the process of the family system working against change and responding in predictable and patterned ways to get a family member to return to their former role. Resisting change and trying to get the transforming individual back to their previous behavior and restore the status quo in family relations.

**Hyperactivating:** See Hyperactivation.

**Hyperactivation:** a type of affect regulation in which clients with a preoccupied attachment style typically escalate emotions or over-react in order to elicit reassurance and responsiveness from others, because they fear abandonment and not being seen or heard.

**Idiographic:** a highly individualistic approach that emphasizes the personal experience or subjective worldview of each individual client, focusing on individual differences rather than large group characteristics.

**Immediacy:** See Immediacy interventions.

**Immediacy interventions:** here-and-now interventions that focus on exploring together and trying to understand what is transpiring right now between the therapist and client. Often used to help negotiate and resolve problems or misunderstanding in the therapeutic relationship, and help make it safe to speak forthrightly about feelings, concerns and expectations in the moment.

**Incongruence:** marked discrepancy between what the client is saying (content) and the manner or way they are expressing it (process).

**Inflexible coping style:** a habitual way of relating to others that is pervasively used in relationships even though it is no longer necessary or effective. This rigid coping style is also reflected in vocational, marital, and other defining life choices.

**Intergenerational boundaries:** delineating the sub-systems of the family system, such as containing marital conflict within the parental subsystem without triangulating

children into taking sides, or having unclear boundaries between parental and child sub-systems and having children assume caretaking responsibilities for adults.

**Internal dissonance:** the disempowerment, uncertainty and self doubt, and pervasive anxiety that is characteristic of clients who grew up feeling repeatedly invalidated in dysfunctional families.

**Internal focus:** considering one’s own contributions to a relational problem and trying to change one’s own internal and interpersonal responses, rather than trying to control and change the other person’s decisions and behavior.

**Internal working models:** the mental representations of close interpersonal relationships, especially those between parents and young children, and how these templates for what is usually going to transpire between Self and Others shape current functioning.

**Internalized contempt:** when attachment figures repeatedly respond to children with contempt, children adopt this rejecting/shaming stance and respond to themselves with the same disgust/criticism.

**Interpersonal coping strategy:** the habitual coping strategies employed in interpersonal relationships to ward off anxiety and maintain self esteem.

**Interpersonal process:** the manner or way that the client and therapist interact with one another, rather than the content of what they are talking about.

**Interpersonal safety:** the sense of security or well-being that one develops in relationships that have proven to be trustworthy, validating or protective.

**Intrapsychic dynamics:** the conscious and unconscious interplay between a rich myriad of feelings, beliefs, wishes, fears and expectations that often are in conflict with each other.

**Invalidation:** to dismiss, weaken, or engender uncertainty in another’s personal or subjective experience.

**In vivo relearning:** experiential re-learning, unlike verbal explanations or interpretations, where clients live through a behavioral or real-life experience of change in their interaction/relationship with the therapist that disconfirm schemas or faulty beliefs they have learned to expect from others.

**Love withdrawal:** the discipline technique in which parents do not focus specifically on unacceptable behavior but instead globally withdraw feelings of love and approval from the child, engendering anxiety as children’s emotional connection to their caregivers is threatened.

**Mentalizing perspective:** secure attachment relationships foster perspective taking abilities in children and greater capacity for empathy and seeing their own, and other’s, experience more flexibly and accurately.

**Metacommunicate:** to talk together directly and discuss therapist - client communication dynamics as they occur



in the moment—to talk about how we are talking to each other.

**Misattunement:** the inability of a parent to recognize and respond to the current emotional needs of the child. Failure to provide accurate empathy contributes to children both losing self awareness and the validity of their own experience, and learning to expect unresponsiveness from others, fostering insecure attachments.

**Moving against:** a fixed and pervasive interpersonal strategy for coping with others by asserting dominance or aggression as a characteristic way of relating to others.

**Moving away:** a fixed and pervasive interpersonal strategy for coping and adapting via distancing from others through physical avoidance, emotional withdrawal, or rigid self-sufficiency.

**Moving toward:** a fixed and pervasive interpersonal strategy for coping by repeatedly trying to please others, exercising excessive compliance and avoiding conflict, which causes clients to lose their own identity and autonomy.

**Mystification:** when children have repeatedly been invalidated, or told that what they really did see and hear was not true or did not occur, children lose the validity of their own experience and perceptions—living anxiously uncertain and disempowered.

**Naming:** highlighting or giving a name to more readily identify clients' maladaptive relational patterns, so that clients can begin to recognize and anticipate when these patterns are coming into play, and begin to exercise more choice in finding new and better ways to respond that could change the familiar but unwanted scenario.

**Narcissistic wound:** the pain experienced by clients when their interpersonal coping strategies fail, injuring their feelings of worth, esteem, and their core sense of Self.

**Natural ending:** A natural ending in treatment occurs when the therapist and client have worked through problems sufficiently and mutually agreed to terminate. An unnatural ending occurs when treatment ends before the client's work is finished because of time limited treatment parameters or relocation.

**Non-defensive:** Therapists' ability to sustain a curious or exploratory stance, and be less prone to personalizing and argue, criticize or withdraw in response to clients' criticism, dissatisfaction with some aspect of treatment, or testing behavior.

**Parentified:** see Parentification.

**Parentification:** the role-reversal that may occur in families without clear intergenerational boundaries, in which the child takes on the role of meeting the caregiver's emotional needs, rather than adults responding to the child's needs.

**Participant/observer:** a stance which involves being in a relationship experientially, while simultaneously remaining

an observer and thinking about what might be going on in the treatment process at the same time.

**Pathogenic beliefs:** the faulty and symptom engendering beliefs, learned in repetitive, affect-laden interactions with attachment figures, that help to create and sustain maladaptive relational patterns.

**Performance demands:** the unrealistic or exaggerated expectations new therapists can place upon themselves to know more or be able to perform more than what is realistically expected—engendering anxiety and self-doubt.

**Permissive:** an indulgent style of parenting that does not enforce limits and rules consistently, and does not place age-appropriate expectations for mature and responsible behavior onto children.

**Personalize:** to take personally and over-react to challenging or provocative behavior from the client, rather than remaining non-defensive and sustaining a neutral, exploratory or curious stance toward the client that is different than the reactivity the client usually elicits from others.

**Personalizing:** see Personalize.

**Points of entry:** therapists' ability to recognize the opportunities that clients present where therapists can choose to intervene and successfully engage the client in more significant issues and concerns.

**Predominant affect:** the main feeling state that characterizes the client's daily functioning. The primary feeling which much of a client's experience revolves around, such as being shame-prone or anxiously preoccupied as a way of life.

**Premature terminations:** attrition or discontinuance of treatment. The client drops out of treatment within the first few sessions without making gains and often without notifying the therapist.

**Preoccupied:** an adult attachment style characterized by on-going anxiety about not being responded to or left. They are prone to escalate emotions with frequent crises, and hold a worried state of mind that leaves them frequently entangled in angry, idealizing, or worrisome pre-occupations about others in current and past relationships.

**Preoccupied client:** a client who is low on avoidance, but high on anxiety. These clients do not avoid emotional needs or avoid seeking help from others, although they are overtly anxious and worried when approaching others with problems or concerns.

**Process comments:** the present-focused, immediacy interventions that invite clients to explore or sort through with the therapist what is transpiring between them right now. Often phrased as talking about you-and-me and what is going on between us in the here-and-now.

**Process dimension:** the manner or way in which a therapist and a client interact with each other, which may either change and help resolve (collaborative, respectful or

non-judgmental interaction that disconfirms feared expectations) or reenact (competitive, indifferent, judgmental or intrusive interaction that is familiar but unwanted) important aspects of a client's problem.

**Protest:** an attachment term to describe the young child's expression of anger or cueing that the attachment figure is not responding to their distress.

**Real relationship:** the personal relationship or authentic connection that develops between the therapist and client as two individuals, as distinct from the other two domains of therapeutic relationships—the working alliance and the transference/schema distortion dimensions.

**Recurrent themes:** the repeating patterns and themes expressed in the narratives that clients relay that help therapists make sense of the client's experience and better understand what is most important or central in the wide-ranging material that clients present.

**Reenactment:** re-creating or re-living old relational scenarios or maladaptive relational patterns in the way the therapist and clients are interacting together, which impedes change and confirms the problematic experiences and expectations from past relationships—leading clients to believe that change is not possible, therapy cannot help, and relationships with others cannot be different and better now.

**Reflective:** the capacity to see things from a variety of perspectives, and be able to dis-embed from one's own strong emotional reactions and more flexibly consider them within a broader context and from different vantage points.

**Reparative interpersonal process:** the way in which the therapist and client are interacting together is a different and better way of interacting than the client has experienced in the past. This new way of interacting facilitates change by experientially disconfirming hurtful or problematic relational patterns the client has experienced with significant others in the past and come to expect from others now in their current lives.

**Resistance:** the clients' reluctance to enter treatment or explore a problem, usually based on fears that once were reality-based, of receiving the same type of unwanted responses they have experienced with significant others in the past when they have asked for help, shared their distress or vulnerability, or risked trust and genuine involvement in a relationship.

**Review-predict-practice sequence:** the intervention sequence designed to make the termination phase more effective. In this sequence, therapists, therapists review progress, accomplishments, successful changes and transitions, and unfinished issues with clients. They also help clients predict the challenging events or anxiety-arousing situations that are likely to activate problems in the future, and rehearse more effective responses to them.

**Role induction:** educating a client about the therapy process prior to beginning treatment, thus taking a step toward establishing a collaborative relationship where the client knows what to expect and is invited to speak up about questions or concerns the client may have about the therapist or any aspect of treatment.

**Ruptures:** in the context of therapist-client relationships, the term refers to misunderstandings, mistakes, or other interpersonal conflicts that may be occurring and disrupt cooperative collaboration between the therapist and client. In order to restore a working alliance the "rupture" needs to be discussed and "repaired."

**Sabotaging:** some clients learned in significant relationships that success, achievement, happiness or greater independence is envied or disliked by important others, which results in some clients sabotaging their own success in order to abate anxiety by complying with binding familial roles and rules, and undoing the success experiences that once threatened their attachment ties.

**Safe haven:** the empathy and emotional responsiveness that an attachment figure reliably offers when a young child is frightened, distressed or vulnerable. In parallel, therapists intend to provide this experience of empathy, validation and acceptance for clients when they are distressed.

**Secure:** an attachment style for adults who are free from excessive anxiety in intimate or close attachment relationships. Secure clients function independently but can seek help when distressed—they discuss relationships clearly and openly, exhibit reflective capacity, and show empathy for others and themselves.

**Secure base:** the active support that caregivers offer to facilitate a child's exploration and increasing independence, and that therapists try to provide for clients as they explore their concerns and attempt change.

**Secure clients:** See Secure.

**Self-efficacy:** an individual's sense of his or her own competence or ability to manage challenging situations effectively and successfully cope with stressors.

**Self-involving comment:** an immediacy intervention in which therapists disclose how the client is affecting them at that moment. With this Use-of-Self, therapists judiciously provide interpersonal feedback about their own reactions to the client that invite more meaningful dialogue about what may be transpiring between them right now, and explore how the client may be affecting others as well.

**Separation guilt:** the symptomatic guilt that often ensues in offspring who are not permitted culturally sanctioned avenues for emancipation, individuation and couple formation during late adolescence and early adulthood. This commonly occurs when there has been a primary cross-generational alliance between caregiver and child.

**Shame:** the toxic feeling that our true Self is deeply and irreparably defective, inherently flawed or inadequate. This is a

global and pervasive judgment of Self as bad, and not just limited to a specific mistake or failure as occurs with the far less problematic feeling of guilt.

**Shame-anxiety:** the strong anxiety that is evoked by the threat of having one's inadequate, flawed, or bad self exposed to others or even revealed to oneself.

**Shame-prone, shame-proneness:** the pervasive and readily activated shame-based sense of Self that has developed in clients who have been frequently shamed in their attachment relationships, stemming from unrelenting criticism or from repeated experiences of being responded to with contempt by caregivers.

**Shame-rage cycle:** one of the faulty coping strategies that clients develop to cope with their shame-proneness. The shame-rage cycle characterizes "road rage" and many domestic violence episodes of battering and physical abuse. As the shame-prone individual feels "dissed" or shamed too readily, and over-reacts to the unintended humiliation by trying to dis-prove that they can be hurt or ridiculed by igniting with rage, aggression or power assertion. Thus they "restore" their sense of worth and truncate their feelings of shame by attempting to prove to themselves and others that they are strong and not weak or bad.

**Structural family relations:** the relatively enduring patterns of alliances, coalitions, and loyalties that exist in a family, and the boundaries that delineate these different subsystems within the family.

**Tentative working hypotheses:** the general hypotheses formulated about the client's problems, strengths and coping strategies by therapists at the beginning of treatment. These initial hypotheses will need to be refined, modified, or discarded in an on-going manner as further information is obtained about each client's personality and problems.

**Testing behavior:** a client behaves in a certain way in order to gauge or assess whether the therapist is going to respond in the familiar but problematic way the client has learned to expect, or in different and more helpful ways which s/he actually needs and wants. More commonly used by more troubled clients with trauma and abuse histories.

**The Dream:** a vision of the kind of life that one wants to lead and the kind of person one wishes to become.

**Therapist-induced countertransference:** "Therapist-induced countertransference" occurs when the therapist's own personal history or problems have been activated by something the client has said or done—this is the traditional conceptualization of counter-transference. However, it must be distinguished from "client-induced countertransference" which occurs when clients' eliciting maneuvers or testing behavior evokes the same types of responses from the therapist that the client characteristically elicits from others in their lives—and this has more to do with the client's interpersonal functioning than the therapist's own personality or dynamics.

**Toxic or core shame:** toxic or core shame refers to a deeply held, pervasive and wounding contempt for oneself that contributes to many serious symptoms and problems, whereas situational embarrassment or innocuous experiences of self-consciousness are commonplace and largely insignificant for treatment.

**Transference:** See Transference reactions.

**Transference reactions:** the systematic misperceptions or cognitive distortions towards the therapist (and others) that are based on the client's internal working models or schemas. The client transfers or over-generalizes learning from earlier relationships onto the therapist (or other person) that is not reality-based.

**Treatment focus:** the therapist's clarity about the core problem and understanding of what most needs to change for the client. Treatment focus provides direction for where the therapist and client are trying to go and the goals they are trying to accomplish with each client.

**Triangulated:** the symptomatic familial process of involving a third person in a relationship to provide a diversion when too much conflict or closeness arouses anxiety between two individuals.

**Triggering issues:** identifying the types of situations or issues that are challenging for clients and lead them to act out and respond in familiar but problematic ways.

**Unfinished grief work:** coming to terms with the painful and often threatening or unacceptable feelings of sadness caused by emotional deprivation or unwanted/conflicted endings in the past—such as interpersonal loss through death or abandonment, or emotional loss through parental rejection or neglect.

**Uniformity myth:** the myth that all therapists are equally effective and that differences in treatment outcome are due to theoretical orientation rather than to more and less effective therapists within each brand of treatment, as the empirical literature repeatedly finds.

**Unnatural endings:** See Natural endings.

**Unspoken family rule:** a covert rule that all family members know about but do not talk about overtly which often governs important behavior in dysfunctional families, such as: what feelings can and can't be talked about; who is allowed to grow up and leave home in late adolescence and who isn't; or who can talk to whom about what topics. When these unspoken family rules are violated, predictable and patterned homeostatic mechanisms swing into play to stop the rule-breaking behavior and restore the status quo.

**Use-of-self:** a type of process comment where therapists draw on their own current subjective experience—observations, questions or feelings they may be having as the client is speaking, to clarify what is transpiring between the therapist and client; such as, what the client is intending or

meaning, or to try and change what they are doing together when it does not seem productive.

**Validation:** an affirming approval or sanction by a secondary person. This has an especially significant impact when an important experience has been denied or minimized by significant others.

**Virtue:** when clients adopt a fixed interpersonal coping strategy (always being kind, strong or independent), they often frame it to themselves as a strength or virtue rather than more realistically as a defensive strategy - an attempt to cope with chronic stressors and ongoing anxiety.

**Within-group differences:** the differences in the treatment outcomes of a group of therapists who share the same theoretical orientation, in order to discern therapists who are more and less effective at utilizing the same treatment manual or theoretical approach. This research paradigm is not widely used, even though it consistently accounts for far more variance in treatment outcomes than measuring between-group differences (for instance, contrasting the effectiveness of client-centered therapists versus cognitive behavioral therapists).

**Working alliance:** The therapeutic relationship has been conceptualized as being comprised of three distinct components: The Working Alliance, the Real Relationship, and the Transference–Countertransference dimension. The working alliance consists of the bond between therapist and client, agreement on treatment goals, and consensus on the tasks or interventions that will be used to facilitate change. The bond or connection between the therapist and client is the most important component of the working alliance.

**Working hypotheses:** See Tentative working hypotheses.

**Working-through:** a stage of therapy when the client is trying out new ways of responding to others. In this phase clients are experiencing successes and disappointments with different people in their lives, and the therapist is serving as a coach or ally as the client repetitively tries to change old response patterns with new and better ways of responding.

**Worldviews:** an individual's subjective perception of the world and life in general, based on culture, class, gender and other shaping influences.



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# NAME INDEX

## A

Ackerman, S., 44, 82, 90, 299, 310  
Adachi, K., 202  
Adams, K., 196  
Adelson, E., 66  
Adelson, J., 202  
Adrian, S., 80  
Agrawal, H., 187, 195  
Ahmed, E., 103  
Ain, S., 14  
Ainsworth, M., 189, 194  
Akhtar, S., 345  
Albus, K., 193, 197, 198  
Alexander, F., 10  
Alimohamed, S., 38  
Allen, J., 7, 155, 296, 357  
Allen, J. G., 187, 191  
Allen, J. P., 191, 200  
Allen, J. W., 53, 119, 206  
Allison, E., 114  
Alpert, J. L., 103  
Altenstein, D., 8  
Amrhein, P., 40, 269  
An, J., 206, 207  
Anderson, T. C., 161, 305  
Andrusyna, T. P., 273  
Angus, L., 49, 213, 286, 302, 305  
Aniskiewics, A. S., 256  
Antonishak, J., 200  
Archineiga, G. M., 161  
Arkowitz, H., 69, 98, 133  
Armour, C., 168  
Arntz, A., 354  
Aron, L., 39, 261  
Atkinson, L., 195  
Atwood, G. E., 232

## B

Bachelor, A., 16  
Bagby, R., 195  
Bakermans-Kranenburg, M., 168  
Balcom, D., 165  
Baldwin, S., 14, 22, 31, 37  
Balfour, L., 196  
Ballueta, N., 196  
Bandura, A., 12, 111, 158, 296, 308

Barber, J., 224, 225, 242, 262, 275, 296, 297, 327  
Barends, A., 14  
Barker, C., 351  
Barkham, M., 53, 152  
Barley, D., 37, 256  
Bartholomew, K., 11, 185, 186, 190, 195, 199  
Bartsch, D., 255  
Bass, A., 259  
Bateman, A., 53, 119, 195, 206, 357  
Bateson, G., 288  
Baumeister, R., 147, 154, 225  
Baumrind, D., 204  
Beatson, J., 254, 283  
Beavers, W. R., 217  
Beck, A., 27, 62, 254, 263  
Beckner, V. L., 7, 83  
Bedi, R. P., 14, 22, 38, 82, 307  
Beebe, B., 296  
Beitman B. D., 111, 137  
Benbassat, N., 206  
Bender, D., 80  
Bender, H. L., 200  
Benefield, R. G., 314  
Benfield, J., 240  
Benjamin, J., 135  
Benjamin, L. S., 26, 196, 251, 254, 289, 364  
Bennett, S., 283, 320  
Berant, E., 184  
Berger, U., 275, 283  
Bergin, A., 354  
Berk, L. E., 200  
Berman, M., 320  
Bernecker, S., 195  
Bernett, L., 197, 273, 283  
Bernier, A., 196  
Bernier, A. T., 10, 197, 335  
Bernstein, M., 240  
Beutler, L., 38  
Beutler, L. E., 72  
Beutler, L. F., 49, 327  
Bhatia, A., 272, 278  
Bien, K., 185, 327  
Binder, J. L., 11, 12, 83, 90, 176, 231, 256, 262, 284, 310, 357, 362  
Bitter, J. R., 30, 215  
Bjorguinsson, T., 62  
Blain, J., 295

Blehar, M., 189, 194  
 Blissett, J., 202, 204  
 Bloo, J., 354  
 Bögels, S., 227  
 Bordin, E. S., 38  
 Borman-Spurrell, E., 191  
 Bornstein, R., 225  
 Boszormenyi-Nagy, I., 219  
 Bowen, M., 30, 45  
 Bowlby, J., 49, 71, 142, 187, 275, 295, 335, 353, 364  
 Boyda, D., 185, 197  
 Braithwaite, V., 103  
 Brammer, L. M., 338  
 Brandchaft, B., 232  
 Brechman-Toussaint, M., 227  
 Brennan, K., 193  
 Bretherton, I., 187  
 Breuer, J., 69  
 Bridges, M., 10, 228, 304, 319  
 Brisch, K. H., 169  
 Bromberg, P., 61, 165, 179  
 Brown, L. G., 42, 43, 198, 354  
 Bruch, M. A., 125  
 Brugha, T., 207  
 Buchheim, A., 275, 283  
 Budge, S., 6, 7  
 Bushman, B., 147, 154  
 Butler, S., 137

## C

Caligor, E., 278  
 Callahan, C., 202  
 Callahan, J., 354  
 Callahan, P. E., 81  
 Cannolly, M., 224  
 Carkhuff, R. R., 53, 132, 154  
 Carlson, E., 198  
 Carlson, J., 15, 31, 54, 61, 94, 219  
 Carter, J., 206  
 Cartwright, D., 80  
 Cassidy, J., 24, 71, 169, 170, 184, 227  
 Cassidy, P. R., 8  
 Castle, D., 191, 196  
 Castonguay, L. G., 11, 254, 304  
 Catley, D., 49  
 Cervone, D., 125  
 Chatters, L.M., 204  
 Chen, E., 190, 194, 195  
 Choi, G., 197  
 Christian, A., 225, 227, 297  
 Cibelli, C., 198  
 Clark, C., 193  
 Clarkin, J. F., 273  
 Cloitre, M., 198  
 Coble, H., 275  
 Coccia, C., 204  
 Cogar, M. M., 80, 151, 319  
 Comas-Diaz, L., 94

Comtois, K. A., 144  
 Condun, M-C., 207  
 Connell, G.M., 204  
 Connolly, H. B., 275, 297  
 Connolly, M., 296, 297, 327  
 Connors, M., 184, 190, 191  
 Consedine, N., 200  
 Constantine, M., 94  
 Coolidge, F., 224  
 Cooney, T., 206, 207  
 Cooper, G., 169, 192  
 Cooper, P., 225  
 Corbett, M. M., 80, 88, 284  
 Costa, P., 204  
 Cowan, C., 217  
 Cowan, P., 217  
 Cox, C., 95  
 Crago, M., 49  
 Cresswell, C., 225  
 Crethar, H., 15, 31, 54, 61, 94, 219  
 Crits-Christoph, P., 9, 275, 295, 296, 297, 327  
 Crockett, M., 145  
 Crook, R. E., 275  
 Crook-Lyon, R., 80, 299  
 Crowley, S., 208  
 Csillik, A., 126, 138  
 Cue, K. L., 197, 273, 283  
 Cui, M., 204  
 Curtis, J. T., 64, 262, 310

## D

Daeppen, J. B., 138  
 Dakof, G., 207  
 Daly, K. D., 60, 188, 193, 197, 223, 268, 275, 283  
 Daniel, S. I. F., 184, 195  
 D'Aniello, C., 7  
 Darling, C., 204  
 Darling, N., 202, 204, 206  
 Datz, F., 257  
 Davies, M., 255  
 Davila, J., 339  
 Davis, D., 254  
 Dearing, R., 102, 109, 165, 167, 234  
 DeClair, J., 230  
 DeFife, J., 242  
 Della Rosa, A., 225, 230  
 Del Re, A., 38, 39, 305  
 Denckla, C., 225  
 Denman, D. W., 80, 151, 319  
 DeRubeis, R., 242  
 Dew, S. E., 354  
 Diamona, G. M., 38  
 DiCaccavo, A., 220  
 DiClemente, C., 72  
 Diener, M. J., 63, 144  
 Diguier, L., 295, 305  
 Dimeff, L. A., 144



Dollard, J., 288  
 Domenech-Rodriguez, M., 208  
 Donovanick, M., 208  
 Dornbusch, S., 202, 204, 206  
 Dozier, M., 10, 190, 191, 193, 195, 197, 198, 273, 275, 283, 335  
 Draguns, J., 31  
 Drapeau, M., 185  
 Dubas, J., 207, 208  
 Dunn, M., 277, 302, 325

## E

Eagle, M. N., 25, 169  
 Easterbrooks, M., 198  
 Egan, G., 67, 70, 73  
 Ehrenberg, M., 217, 219  
 Eichelsheim, V., 207, 208  
 Eisenberger, N., 145  
 Elklit, A., 168, 185, 197  
 Elliott, R., 80, 144, 148, 165, 171, 174, 339  
 Ellis, A., 62  
 Ellison, W., 195  
 Enten, R., 202  
 Erikson, E., 37  
 Escuderom V., 38  
 Esterson, A., 51, 211  
 Estey, A., 224  
 Ethier, N., 8  
 Eubanks-Carter, C., 89, 90, 196,  
 273, 313  
 Everson, E., 80

## F

Fahmy, R., 354  
 Falender, C., 177, 261, 283  
 Fallof, R., 191  
 Farber, B. A., 338  
 Farber, B. A., 119  
 Fauth, J., 175, 179  
 Fearon, P., 351  
 Feeney, B., 184, 188  
 Feeney, J., 194  
 Feiring, C., 187  
 Feldman, L., 138  
 Fetterman, A., 231  
 Feldman, S., 119  
 Fey, A., 62  
 Finkelhor, D., 103  
 Fiori, K., 200  
 Fitzpatrick, K., 207  
 Fitzpatrick, M. R., 152, 185, 338  
 Flores, E., 206  
 Florsheim, P., 14  
 Fluckiger, C., 80, 185, 215  
 Flückiger, C., 38, 39, 305  
 Foltz, C., 242  
 Fonagy, O., 191, 196, 198  
 Fonagy, P., 7, 26, 53, 114, 119, 155, 168, 174, 187, 194, 195,  
 196, 198, 206, 273, 282, 317, 357

Ford, J., 198  
 Forman, E. M., 354  
 Fosha, D., 170, 345, 351  
 Fragoso, J. M., 161  
 Fraiberg, S., 66  
 Fraley, R., 207  
 Frank, A., 295  
 Freeman, A., 254  
 French, T., 10  
 Freud, S., 69  
 Friedlander, M. L., 38, 95, 142, 145, 177  
 Fromm-Reichmann, F., 12  
 Fryers, T., 202, 207

## G

Gabbard, G. O., 38, 72, 259  
 Galassi, J. P., 125  
 Gans, J., 258  
 Gantt, D., 275  
 Garfield, S. L., 43, 354  
 Geller, J. D., 319  
 Geller, S. M., 39, 146, 148, 171, 172  
 Gelso, C. J., 14, 38, 272, 278, 283, 284, 292, 303, 326, 327, 354,  
 357, 362  
 Gendlin, P. E. T., 111, 154  
 George, C., 198  
 Gergeley, G., 155, 168, 174, 196, 273, 317  
 Gerlsma, P., Gold, S. H., Hilsenroth, M. J., Kuutmann K., &  
 Owen, J. J., 202  
 Gerris, J., 207, 208  
 Gerson, R., 30  
 Gibbons, B. B., 9  
 Gibbons, M., 224  
 Gilbert, P., 106, 107, 158, 296  
 Gill, M., 287  
 Gilligan, C., 67  
 Gilligan, J., 104  
 Gladis, L., 296, 297, 327  
 Gmel, G., 138  
 Golan, M., 202  
 Gold, J., 71, 145, 176, 184, 296, 339  
 Gold, S., 360  
 Goldenberg, H., 29, 217  
 Goldenberg, I., 29, 217  
 Gold, S. H., 360  
 Goldfried, M. R., 142, 145, 296, 304, 339  
 Golding, L., 275  
 Goldman, R. N., 63, 111, 144, 148, 165, 171,  
 339, 341  
 Gorman, B. S., 90, 273, 313  
 Gormley, B., 281  
 Göstas, M. W., 43  
 Gottman, J., 230  
 Greenberg, L. S., 39, 63, 98, 104, 111, 127, 137, 138, 144, 146,  
 148, 162, 165, 171, 172, 319, 339, 341  
 Greenson, R., 38  
 Grossman, S., 224, 256  
 Groth, M., 119

Grunebaum, J., 219  
 Guame, J., 138  
 Guilliland, B., 42  
 Gunderson, J., 187, 195

## H

Hadley, S., 286  
 Haley, J., 45  
 Hall, S., 49  
 Hampson, R. B., 217  
 Hamre, B., 202  
 Hansen, N. B., 354  
 Harris, K. J., 49  
 Harris, S. M., 357  
 Hart, J., 62  
 Hartzell, M., 229  
 Hatcher, R., 14  
 Hauser, S. T., 191  
 Haycraft, E., 202, 204  
 Hayes, H., 278, 326  
 Hayes, J. A., 175, 179, 278, 284, 292, 326–327  
 Heagerty, P., 144  
 Heller, R., 191  
 Henderson, C., 207  
 Henderson, J. C., 43  
 Hendricks, M. N., 155  
 Hendrickson, S. M. L., 331  
 Hendrix, H., 230  
 Henry, W., 256  
 Herman, J., 198  
 Hersoug, A., 38  
 Hess, S., 80, 299  
 Hesse, E., 25, 197, 198, 199, 288  
 Hetherington, L., 38  
 Hickson, M., 148, 171  
 Highlen, P., 95  
 Hill, C. E., 9, 10, 11, 15, 63, 80, 88, 142, 150, 151, 231, 254, 261, 273, 277, 284, 299, 302, 311, 313, 319, 320, 325, 331, 364  
 Hilsenroth, M. J., 44, 63, 82, 90, 144, 242, 299, 302, 310, 311, 313, 360  
 Hinnen, C., 26  
 Hobson, R., 191, 196  
 Hoeve, M., 207, 208  
 Hoffman, K., 169, 192  
 Hoffman, M. A., 63, 230  
 Hoglend, P., 38  
 Holmes, B., 187, 195  
 Holtforth, M., 185  
 Holtforth, M., 215  
 Holtforth, M. C., 8  
 Hopwood, C., 8  
 Horney, K., 225, 230  
 Horowitz, L. M., 185, 186, 190, 195, 199  
 Horowitz, M., 11  
 Horvath, A. O., 14, 22, 38, 39, 82, 127, 305, 307  
 Houck, J., 40, 174

Howard, R., 191, 196  
 Hummel, A. M., 14, 284, 292, 327  
 Hunter, M., 217, 219  
 Hynan, M., 354

## I

Imel, Z., 7, 14, 22, 31  
 Imm, G., 104  
 Ingersoll-Dayton, B., 204  
 Ingram, B., 7, 22  
 Inman, A., 175  
 Ivery, M., 18  
 Ivey, A., 18  
 Iwakabe, S., 104, 152

## J

Jacobs, T. J., 296  
 Jacobsen, B., 113  
 Jacobvitz, D., 187  
 Jackson, J., 320  
 James, R. K., 42  
 Janoff-Bulman, R., 210, 229  
 Janzen, J., 185, 338  
 Jenkins, S., 218  
 Jeong, J., 184, 338  
 Johnson, B., 319  
 Johnson, M. E., 137  
 Johnson, S., 170, 211  
 Joireman, J., 103  
 Jolkovski, M. P., 179, 258  
 Jones, R., 103  
 Jordan, J., 281  
 Jorm, A., 225  
 Joyce, A. S., 80  
 Joyce, P., 206  
 Judge, A., 63  
 Jung, 1998, 219  
 Jurist, E. J., 155, 174, 196, 273, 317

## K

Kagan, F., 49, 213, 286, 302, 305  
 Kahn, M., 271, 364  
 Kaltreider, N., 11  
 Kaplan, A., 281  
 Karen, R., 295  
 Karver, M. S., 38  
 Kashubeck, S., 161  
 Kasper, L., 9  
 Kathleen, M., 202  
 Katznelson, H., 206  
 Kazantzis, N., 135  
 Kegel, A., 80  
 Keller, H., 200  
 Kellis, E., 135  
 Kelly, A. E., 151  
 Kelly, S. M., 137  
 Kerig, P., 32, 202  
 Kernberg, O. F., 273

Kerr, M. E., 30, 45  
 Khurgin-Bott, R., 119  
 Kiesler, D. J., 8, 9, 73, 135, 251, 305, 315  
 Kitamura, T., 202  
 Kivlahan, D. R., 9, 14, 144, 185, 327  
 Kivlingham, D. M., 338  
 Kjellin, L., 43  
 Klosko, H. S., 27  
 Knoblauch, S., 296  
 Knox, S., 80, 261, 273, 299, 313  
 Kobak, R., 227  
 Kohlenberg, R., 77, 146  
 Kohut, H., 232, 235, 353  
 Korman, L. M., 341  
 Krauge, N., 204  
 Kraus, J., 90, 135  
 Krieger, T., 8  
 Kruk, J., 111, 257, 296  
 Krupnick, J., 11  
 Ku, T., 77, 225, 230  
 Kuutmann, K., 242, 302, 311, 313  
 Kuwabara, H., 202

## L

Ladany, N., 175, 177  
 Lafferty, P., 49  
 Laing, R. D., 51, 211  
 Lambert, M., 6, 7, 22, 37, 256, 296, 305  
 Lambert, M. J., 305, 354  
 Lamborn, S., 202, 204, 206  
 Landis, J., 242  
 Larose, S., 196  
 Lasca, F., 196  
 Lazarus, A. O., 16, 62, 162  
 Lazarus, C., 62  
 Lee, R., 165  
 Lee, S., 190, 191, 195  
 Lee, W., 30, 45  
 Lehmann, M., 311, 313, 318  
 Leigh, T., 191, 196, 198  
 Levant, R. F., 269  
 Levenson, H., 11, 16, 48–49, 60, 83, 137, 142, 170, 183, 217, 225, 242, 273, 287, 296, 336, 347, 364  
 Levine, A., 191  
 Levinson, D., 351  
 Levy, K. N., 195, 272, 278  
 Lewis, H. B., 165  
 Lewis, M., 103, 187  
 Liddle, H., 207  
 Lieberman, M., 145  
 Ligiero, D. P., 275  
 Linehan, M., 171  
 Linehan, M., 51, 52  
 Linehan, M. M., 51, 52, 144  
 Liotti, G., 198  
 Liu, J., 320  
 Loborsky, L., 275, 297

Löffler-Staska, H., 257, 260  
 Lohr, M., 147, 154  
 Lomax, L., 190, 191, 195  
 Lopez, F. G., 281  
 Lorentzen, S., 38  
 Love, K., 202, 204  
 Luborsky, L., 273, 295, 305  
 Ludlow, A., 202  
 Luty, S., 206  
 Lyons-Ruth, K., 195, 198, 227

## M

MacDonald, G., 338  
 Magai, C., 200  
 Main, M., 25, 189, 194, 197, 198, 199, 288  
 Majors, R., 283  
 Malik, M., 38  
 Mallinckrodt, B., 60, 184, 185, 187, 188, 189, 190, 194, 195, 197, 200, 223, 268, 275, 283, 295, 338  
 Mann, J., 362  
 Many, M. M., 357, 359  
 Markin, R., 14, 283  
 Markowitz, T., 63  
 Marmar, C., 11  
 Marmarosh, C., 185, 283, 327  
 Martin, A., 275, 283  
 Martin, S., 225, 227, 297  
 Martin, T., 40  
 Marx, S. A., 362  
 Matos, M., 106  
 Matsakis, A., 231  
 Matsumoto, V., 227, 228, 232  
 Maughan, B., 191, 196  
 Maunder, R., 354  
 Mayo, M. S., 49  
 Mayotte-Blum, J., 311, 313, 318  
 McArthur, L., 14  
 McBride, C., 195, 354  
 McCarthur, K., 262  
 McCarthy, K., 224  
 McClure, F., 48, 94, 103, 158, 165, 305, 310  
 McCrae, R., 204  
 McCroskey, J., 148, 171  
 McElhaney, K. B., 200  
 McEwan, K., 106  
 McGoldrick, M., 30  
 McKinney, C., 207  
 McKinney, M., 111, 257, 296  
 McLellan, A. T., 295, 305  
 McWhirter, B., 119, 338  
 Meissner, W. W., 81  
 Mellon, J., 137  
 Messman, J., 209  
 Metzger, T. A., 119, 338  
 Mikulincer, M., 184, 185, 189, 190, 191, 193, 194, 347  
 Miller, A., 31, 103

Miller, J., 281  
 Miller, N., 288  
 Miller, W. R., 14, 37, 39, 40, 49, 69, 70, 98, 111, 133, 146, 269, 288, 296, 314, 331  
 Millon, T., 224, 232, 254, 256  
 Mills, R., 104  
 Milrod, B., 63  
 Minuchin, S., 30, 45, 217  
 Mitchell, S., 327  
 Mitjavila, M., 196  
 Moleiro, C., 72  
 Morgan, Z., 207  
 Morrison, K., 296, 303  
 Mounts, N., 202, 204, 206  
 Mowen, T., 207  
 Moyers, T., 40, 98, 174  
 Moyers, T. B., 40, 98, 146, 174, 331  
 Msetfi, R. M., 275  
 Mueller, W. J., 256  
 Mulder, R., 206  
 Munholland, K., 187  
 Muran, J. C., 9, 38, 89, 90, 112, 196, 251, 273, 286, 287, 313, 317, 319, 329, 354  
 Murphy, S., 185, 197  
 Murray, L., 225  
 Muslin, H., 287  
 Myers, L., 52, 95  
 Myers, S., 67

## N

Najavits, L., 18, 90, 310, 327  
 Najavits, L. (Najavits in text), 217  
 Nathanson, D. L., 164  
 Neander, K., 43  
 Neff, K., 76, 312, 334  
 Nelson, D., 175, 179  
 Nelson, E., 207  
 Nelson, M., 177  
 Nelson, S., 305  
 Neuzil, P., 224  
 Nichols, M. P., 30, 45, 217  
 Nienhuis, J. B., 38, 241  
 Noller, P., 194  
 Norcross, J. C., 6, 7, 16, 22, 37, 72, 268, 296, 305, 327, 333, 354

## O

Obegi, J., 184  
 O'Brien, A., 354  
 O'Brien, K. M., 150  
 Ochoa, S., 196  
 Ode, S., 231  
 Ogawa, J., 198  
 Ogden, T. H., 24  
 Ogles, B. M., 305  
 Ogrodniczuk, J. S., 80  
 Okiishi, J., 305  
 Olatunji, B., 147, 154

Olthof, T., 232  
 Orlinsky, D. E., 72  
 Otani, K., 227, 228, 232  
 Ozer, E., 206

## P

Palma, B., 272, 278  
 Palmer, M., 269  
 Parish, M., 169  
 Parker, S., 167  
 Parth, K., 257, 260  
 Pasch, L., 206  
 Pasternak, A., 219  
 Patrick, M., 191, 196  
 Patterson, C. L., 305  
 Pederson, P., 15, 31, 54, 61, 94, 149, 219  
 Pekarik, G., 354  
 Pereg, D., 189, 194  
 Perrin, M., 217, 219  
 Perry, J., 198  
 Persons, J. B., 7, 83, 258, 268  
 Peterson, B., 202  
 Petry, S., 30  
 Pezzarossa, B., 225, 230  
 Pham, T., 273  
 Pietromonacco, P. (Pietromonaco), 138  
 Pinger, V., 262  
 Piper, W. T., 80  
 Polly, R., 296  
 Popp, C., 137  
 Porter, M. J., 338  
 Pos, A. E., 341  
 Potter-Efron, R., 103, 104  
 Powell, B., 169, 192  
 Powers, T., 137  
 Priel, B., 206  
 Prochaska, J., 16, 72, 333  
 Procter, S., 106  
 Proeve, M., 255  
 Proskurov, B., 38, 317, 319

## Q

Quilty, L., 195  
 Quirk, S., 225, 227, 297

## R

Rao, S., 254, 283  
 Rapee, R., 225  
 Rappleyea, D., 357  
 Raskin, J., 49  
 Raue, P. J., 304  
 Ravitz, P., 354  
 Regan, A. M., 150  
 Rehm, M., 204  
 Reis, B. F., 42, 43, 198, 354  
 Renik, O., 261  
 Renk, K., 207  
 Renn, P., 145

Rennestad, M., 65  
 Reynolds, S. K., 144  
 Rhodes, R. H., 80  
 Rhodes, R., 319  
 Rholes, W., 189  
 Rice, S., 174  
 Richmond, V., 148, 171  
 Ridley, C. R., 137  
 Ritchie, K., 168  
 Rivas, A., 106  
 Robbins, S. B., 179, 258  
 Roberts, R., 255  
 Robins, R., 165  
 Robinson, M., 231  
 Rogers, C., 14, 39, 54, 208, 216, 299  
 Roisman, G., 207  
 Rollnick, S., 14, 39, 40, 49, 69, 70, 98, 133, 288, 296, 307, 331  
 Romano, V., 338  
 Ronnestad, M. H., 72  
 Rose, G., 37, 40, 69, 111, 269, 331  
 Rosenberg, S. E., 11, 64  
 Rosenthal, S., 187  
 Roth, A., 7  
 Roth, T., 351  
 Rothrauff, T., 206, 207  
 Rubino, A. I., 225, 230  
 Rubino, G., 351  
 Rubinstein, T., 254, 262  
 Russell, E., 345, 351  
 Rustin, J., 296

## S

Sadler, P., 8  
 Safran, J., 9, 38, 89, 90, 135, 251, 286, 317, 319  
 Safran, J. D., 90, 196, 273, 313, 354  
 Said, U., 257, 260  
 Sakado, K., 202  
 Sakamoto, A., 202  
 Sampson, H., 16, 64, 213, 262, 296  
 Sampson, H. W., 18  
 Samstag, L., 9, 89, 303, 326  
 Samstag, L. W., 90, 273, 313, 354  
 Sanderman, R., 26  
 Sanders, S., 142, 145  
 Sanfer, J., 103  
 Sathe, S. K., 204  
 Satir, V., 30, 215  
 Sato, T., 202  
 Sauer, E. M., 281  
 Scalia, J., 272  
 Schacht, T., 256  
 Schact, T. E., 137  
 Schatzman, M., 51  
 Scheff, T. J., 164  
 Scher, S. J., 225  
 Schier, K., 219  
 Schill, T., 230  
 Schlosser, L. Z., 275

Schore, A. N., 148, 171, 187  
 Schore, J., 120  
 Schroeder, R., 207  
 Schwartz, S., 207  
 Scott, L. N., 195, 273  
 Segal, D., 224  
 Seligman, D. A., 295, 305  
 Seligman, M., 6  
 Shafransky, E., 177, 261, 283  
 Shah, R., 207  
 Shapiro, D., 53, 152  
 Shapiro, V., 66  
 Shaver, J., 8  
 Shaver, P. R., 24, 71, 169, 170, 184, 185, 189, 190, 191, 193, 194, 347  
 Shaw, B. A., 204  
 Shaw Welch, S., 144  
 Sheehan, A. H., 95  
 Sheldon-Keller, 196  
 Sherlin, M., 168, 185, 197  
 Shirk, S. R., 38  
 Siegel, D., 170, 229  
 Silberschatz, G., 16, 19, 62, 64, 135, 258, 262, 268, 310  
 Silberschatz, G. I., 95  
 Sim, W., 10, 231, 311, 313, 320  
 Simon, J. L., 104  
 Simon, K., 357  
 Simpson, J., 189  
 Singh, S. P., 354  
 Siqueland, L., 295  
 Siracusano, A., 230  
 Skovholt, T., 65  
 Slade, A., 185, 283  
 Slavin-Mulford, J., 311, 313, 318  
 Slicker, E. K., 200  
 Smeenk, W., 207, 208  
 Smith, A. E., 275  
 Smith, J., 64  
 Smith, W. F., 231, 232  
 Snyder, C. R., 351  
 Solomon, J., 198  
 Solomon, M., 170  
 Sorter, D., 296  
 Soth, A. M., 111  
 Soucyn, N., 196  
 Spangler, P., 10, 231, 277, 302, 311, 313, 320, 325  
 Speight, S., 95  
 Spiegel, D., 103  
 Spieker, S., 207  
 Sprangers, M., 26  
 Springmann, R., 260  
 Sroufe, L., 168, 198, 207, 227  
 Stacey, J., 225  
 Stahl, J., 10, 231, 311, 313  
 Stalikas, A., 152  
 Steele, M., 191, 196, 198  
 Stegge, H., 232



Steinberg, L., 202, 204, 206, 207  
 Stevens, C., 9, 89  
 Stolorow, R. D., 232  
 Stovall-McClough, K., 193, 197, 198  
 Straus, B., 275, 283  
 Stricker, G., 176  
 Striver, P., 281  
 Strupp, H., 12, 16, 18, 90, 137, 176, 217, 225, 231, 256, 262, 284, 286, 310, 327  
 Strupp, H. H., 137  
 Strupp, H. P., 60  
 Stupar-Ruthenfrans, S., 354  
 Sue, D. W., 47, 61, 94, 149, 161  
 Sue, S., 13, 31, 63, 123, 355  
 Sullivan, C., 10, 231, 311, 313  
 Sullivan, H. S., 23, 136, 254, 261, 281  
 Surrey, J., 281  
 Sutherland, O., 142, 145  
 Suzuki, A., 227, 228, 232  
 Swift, J., 354  
 Swift, J. K., 43  
 Symonds, D., 38, 39, 305

## T

Tager, J., 165  
 Tagney, J., 109  
 Talebi, H., 72  
 Tang, T. Z., 273  
 Tangney, J., 102, 165, 234  
 Tangney, J. P., 103  
 Tantillo, M., 103  
 Target, M., 155, 168, 174, 187, 194, 196, 198, 273, 317  
 Tasca, G.A., 168, 196  
 Taylor, E., 319  
 Teague, G., 191  
 Teyber, E., 10, 48, 94, 103, 152, 158, 165, 205, 208, 213, 217, 218, 219, 231, 257, 305, 310, 311, 313  
 Teyber, F., 152, 213, 257  
 Thomaes, S., 232  
 Thomas, K., 8  
 Thomas, P., 202, 204  
 Thomas, R., 167  
 Thompson, B. J., 80, 88, 151, 284, 319  
 Thompson-Brenner, H., 296, 303  
 Thornberry, I., 200  
 Tomkins, S., 103  
 Tompkins, M. A., 7, 83  
 Tonigan, J., 40  
 Tonigan, J. S., 314  
 Townsend, K., 338  
 Tracey, J., 165, 167  
 Truax, C. B., 53  
 Tsai, M., 77, 146  
 Tschann, J., 206  
 Tufekcioglu, S. (Tufekciogly), 112, 287, 329  
 Tyrone, G. S., 86  
 Tyrrell, C., (this name seems to be et al in ref, but in text only one name available), 190, 191, 197, 273, 275, 335

Tzanetos, T., 319

## U

Uji, M., 202  
 Ulrich, D., 219

## V

Van Denburg, T., 315  
 van der Kolk, B., 198  
 vander Laan, P., 207, 208  
 Van Dijke, A., 198  
 Van IJendoorh, M., 168, 209  
 Van Vieet, M., 184, 188  
 Van Zeijl, J., 209  
 Varghese, A., 218  
 Vasquez, M., 47

## W

Wachtel, P., 73, 95, 224, 227, 310, 313, 347  
 Wachtel, P. L., 111, 257, 296  
 Wall, S., 189, 194  
 Waller, G., 207  
 Wallerstein, R., 11  
 Wallin, D., 193–194  
 Walling, B., 104  
 Wampold, B. E., 6, 7, 14, 22, 31, 38, 39, 43, 268, 296, 305, 354  
 Wang, C., 200, 275  
 Wasserman, R. H., 273  
 Waters, E., 189, 194  
 Watkins-Lewis, K., 202  
 Watson, J., 111, 144, 148, 165, 171, 339  
 Weathers, R., 103, 165  
 Wei, M., 77, 225, 230  
 Weiler, H., 104  
 Weinberger, J., 144  
 Weinfeld, N., 198  
 Weir, D., 225, 227, 297  
 Weishaar, M. E., 27  
 Weiss, J., 62, 64, 135, 218, 229, 254, 262, 289, 303, 310, 347  
 Wells, M., 103  
 Wenar, C., 32, 202  
 West, M., 196  
 Westin, D., 296, 303  
 Whelton, W. J., 148  
 White, M., 215  
 White, B., 177  
 White, C., 67  
 White, M., 357  
 White, W. L., 98  
 Wiberg, B., 43  
 Wierzbicki, M., 354  
 Williams, D., 230  
 Williams, E. N., 63  
 Willutzki, U., 72  
 Wilner, N., 11  
 Winnicott, D. W., 52, 172  
 Winograd, G., 86

Winston, A., 90, 273, 313, 354  
 Wolfe, B. E., 137  
 Wolitzky, D., 25  
 Woodhouse, S., 354, 357, 362  
 Woodhouse, S. S., 275  
 Woody, E., 8  
 Woody, G., 295, 305

## Y

Yahne, C., 269  
 Yalom, I., 85, 113, 271, 318

Yap, M., 225  
 Yard, S., 77, 146  
 Yeomans, F., 273, 278  
 Young, J. E., 27, 70, 254

## Z

Zachariades, F., 168  
 Zalaquett, C., 18  
 Zane, N., 13, 31, 63, 123, 355  
 Zilcha-Mano, S., 262  
 Zubriggen, E., 202

# SUBJECT INDEX

## A

Abuse. *See* Sexual abuse  
Accurate empathy, 53  
Achieved credibility, 13, 31, 250  
Adult Attachment Interview (AAI), 186–187  
Affect

- affective constellations, 161–167
- characterological, 63, 159–161, 167
- containment, 98, 169, 170–173
- expanding and elaborating, 152–154
- experiencing rather than talking about, 154–157
- identifying predominant, 157–161
- recurrent affect themes, 63–65
- regulation, 169

## Ambivalence

- in attachment style, 25
- exploring ambivalent feelings, 70
- responding to clients' conflict, 288–291
- working with client's, 144–145

## Anger

- anger-sadness-shame constellation, 162–165
- sadness-anger-guilt constellation, 166–167

## Anxiety

- in basic attachment ties, 25
- shame-anxiety, 105–107

## Ascribed credibility, 250

## Assessing client reactions, 269–272

## Attachment styles

- clinical guidelines, 212–217
- in adults: dismissive, 185, 188–193
  - fearful, 186, 197–200
  - preoccupied, 186, 193–197
  - secure, 185–188
- in children: ambivalent, 25
  - avoidant, 24–25
  - disorganized, 25
  - secure, 24

## Attachment theory, 24–25

- internalizing attachment relationship, 25–27
- secure *vs.* insecure, 24

## B

Between-group differences, 305

## C

Case formulation/conceptualization 6–7

- interpersonal model for conceptualizing clients, 225–226
- using transference reactions, 275–280

Change process, interpersonal model, 226  
Client response specificity, 15–19

- assessing client reactions, 269–272
- testing behavior, 268–269

Client's salient feeling, 148–152  
Cognitive behavioral therapy, 27–28. *See also* Early maladaptive schemas (EMs)  
Cognitive domain, 24  
Collaborative alliance. *See* Working alliance  
Collaborative relationship, 14  
Compacted phrase, 159  
Compliance, 156  
Compromise solution, 26  
Conditions of worth in parenting, 208–212  
Conflicts

- emancipation, 217–219
- over initiating, 47–48
- responding to, in therapeutic relationship, 296–297

Confrontations, concern about, 328–329  
Contemporary short-term relational approach, 11  
Contempt, 103. *See also* Shame  
Corrective emotional experience, 10–15, 267–280 *See also* Experiential Relearning

- using process dimension to provide, 13–14

Corrective interpersonal process, 22  
Countertransference, 14–15, 175–177, 260–261

- 254–261, 281–283, 286–288
- client- *vs.* therapist-induced countertransference, 260–261
- in family-of-origin work 350–351
- issues/therapists fears, 325–328

Cultural competence, 31  
Cultural norms and values, 28–29

## D

De-activating in Attachment, 168, 188  
Debriefing, 276  
Dichotomous thinking, 31–32  
Differentiation, 178  
Directive/nondirective styles, 39–43

Disengagement, 282–283  
 Dream, the, 351–353

## E

Early maladaptive schemas (EMs), 27–28  
 Early parent–child relationships, 24  
 Earned security, 184–185  
 Eliciting maneuvers, 254–261  
 Emancipation conflicts, 217–219  
 Embedded messages, 58  
 Empathic understanding, 48–50  
 Enmeshment, 281–282  
 Experiential relearning, 10–15, 27, 267–280

## F

Facilitate change, in process dimension, 299  
 Family-of-origin work, 345–351  
     conflict with primary caregivers, 347–348  
     developmental familial experience, 32–33  
     grieving for caretaking, 349–350  
 Family systems  
     cultural context, 30–32  
     familial rules and roles, 175–177  
     invalidation of client's experience, 50–54  
     lifelong impact of family experience, 32–33  
     strengths, 31–32  
 Flexibility, 61

## G

Grief work, 361  
 Guilt, 70 *vs.* shame, 103  
 Sadness-anger-guilt constellation, 166–167

## H

Hierarchical model or relationship, 20, 37, 73, 133, 307  
 Homeostatic mechanism, 348  
 Hyper-activating in Attachment, 168, 188

## I

Idiographic approach, 16–17  
 Immediacy interventions, 313–320, *See also*  
     Process Comments  
     uncertainty about when to intervene, 324–325  
 Inclusive cultural empathy, 31  
 Incongruence, 155  
 Initial interview, 43–48  
 Initiating, conflicts over, 47–48  
 Insecure attachment, 24  
 Intergenerational boundaries, 217  
 Internal focus for change, 110–142  
     client's reluctance to adopt, 122–125  
     effective interventions, 130–132  
     enlisting clients to solve their problems, 132–136  
     focusing client's inward, 112–118, 140–141  
     fostering client's initiative, 125–132  
     ineffective interventions, 129–130  
     resolving client's conflict, 133–136

therapist–client relationship, 127–129  
 therapist's reluctance to adopt, 118–122  
 tracking client's anxiety, 136–141  
 Internal working models (IWMs), 24, 228–230  
     maladaptive relational patterns, 240–241  
     resolving core conflict, 235–238  
     shoulds for self, entitlement for others, 234–235  
     unmet developmental needs, 227–235

## Interpersonal balance

disengagement, 282–283  
 effective middle ground, 284–286  
 enmeshment, 281–282  
 over-involvement, 286–288  
 under-involvement, 286–288

## Interpersonal conflict, 88–89

## Interpersonal consequences, unwanted, 145–148

## Interpersonal coping strategies, 209, 224–251

moving toward, moving against, or moving away, 231–232  
 moving against, 244–247  
 moving away, 247–251  
 moving toward, 238–244

## Interpersonal domain, 23–24

## Interpersonal feedback, 155

## Interpersonal patterns, 61–62

## Interpersonal process approach, 8–19

theoretical and historical context, 22–33

## Invalidation, 51

## In vivo relearning, *also* Experiential Relearning, 27

## Involvement

disengagement, 282–283  
 effective middle ground, 284–286  
 enmeshment, 281–282

## L

Love withdrawal, in parenting, 208–212  
 Loyalties, family, 85, 217

## M

Metacommunication, 9  
 Middle ground of effective involvement, 284–286  
 Mis-attunement, 51  
 Multiple stressors, 158–159  
 Mystification, 210–212

## N

Natural endings, 357  
 Non-defensive therapist, 9

## O

Object relations theory, 24–25  
 Participant/ Observer, 6  
 Open-ended invitations, 152–153

## P

Parentification of children, 29, 176, 219–222  
 Parenting styles  
     authoritarian, 201–203

authoritative, 207–208  
 disengaged, 205–207  
 permissive, 203–205  
 Pathogenic beliefs, 62–63  
 Performance anxieties  
   causes and management of, 4–6  
   concerns about personal power, 330–331  
   countertransference issues and, 325–328  
   fear of offending client, 325–326  
   fear of revealing inadequacies, 329–330  
 Personalizing, 132  
 Pervasive coping style, 210  
 Points of entry, 120–122  
 Power and control, 330–331  
 Pre-contemplators, 72  
 Premature termination, 354–357  
 Process comments, 9  
   facilitating interventions, 318–320  
   guidelines for utilizing, 313–315  
   preventing reenactment with therapist, 302–304  
   providing interpersonal solution, 313–320  
   types of, 315–318  
 Process dimension, 8–10, 295–331  
   bridging theoretical orientations, 304–309  
   facilitating change, 299  
   recognizing interpersonal process, 299–302  
   reenacting problem in interpersonal process, 299–313  
   therapists reluctance to work with, 323–331  
 Psychologically based resistance, 81

## R

Real relationship, 38  
 Recurrent themes  
   affective, 63–65  
   identifying, 60–65  
 Reenactments of client's conflict  
   eliciting maneuvers, 254–261  
   testing behavior, 261–272  
   transference reactions, 272–280  
 Rejection/Shame schema, 28  
 Relational patterns, 23–24, 29. *See also* Interpersonal coping strategies  
   maladaptive, 11, 14–15, 19  
   repetitive themes, 61–65  
 Resistance, 69–108  
   clients' concerns about therapy, 93–94  
   client's reluctance to work with, 75–80  
   common themes in, 70  
   cultural differences, 94–95  
   exploring danger/identifying threat, 99–101  
   exploring in first session, 88–95  
   formulating case conceptions, 81–83  
   fueled by shame, 102–108  
   identifying, 80–81  
   in initial telephone contact, 84–87  
   interpersonal conflict, 88–89  
   interventions, 99, 101–102

responding to, 83–84  
 testing therapist's adequacy, 92–93  
 therapist's reluctance to work with, 72–75  
 Ruptures, 9  
   ruptures, reenactments, and resistance, 95–102

## S

Sadness  
   anger-sadness-shame constellation, 162–165  
   sadness-anger-guilt constellation, 166–167  
 Safe haven, 49, 71, 174, 184  
 Schemas, 27–28  
 Secure base, 71, 184  
 Self-blaming distortion, 27  
 Self-defeating behavior, 107  
 Self-efficacy, 37  
 Separateness/relatedness, 30  
 Separation guilt, 156, 218  
 Sexual abuse, 309–313  
 Shame, 102–108 *vs.* guilt, 103  
   anger-sadness-shame constellation, 162–165  
   defending against, framing as virtue, 232–234  
   shame-anxiety, 105–107  
   shame-prone sense of self, 104  
   shame-rage cycle, 104–105  
   shame-worthiness, 104  
 Shoulds, tyranny of, 234–235  
 Splitting defenses, 26  
 Strain trauma, 32  
 Structural family relations, 217

## T

Termination, 354–363  
   client and therapist discuss ending, 357–362  
   ending the relationship, 363  
   premature, 354–357  
 Testing behavior, 261–272  
   assessing clients' reactions, 269–272  
   client response specificity, 268–269  
   therapist passing client's test, 267  
 Theoretical orientations, bridging, 304–309  
 Therapeutic alliance. *See* Working alliance  
 Therapeutic relationship  
   affirmative responses to clients, 321–322  
   evoking & responding to clients' conflicts, 296–299  
   shared control, 127–129  
 Transference reactions, 38, 272–280  
   contemporary approach, 273  
   definition, 272  
   in everyday life, 274–275  
   therapeutic goals for, 273  
   using to conceptualize clients, 275–280  
 Triangulation, 45–46

## U

Understanding  
   client's subjective meaning, 50–54  
   communicating overtly, 54–60



Understanding (*Continued*)  
    empathic, 48–50  
    pre-conditions of change, 66–67  
Unfinished grief work, 361  
Unnatural endings, 358  
Unspoken family rule, 211  
Unwanted interpersonal consequences,  
    145–148

## V

Validation of client's experience, 50–54

## W

Working alliance, 37–67  
    collaboration, in initial interview, 43–48  
    collaboration *vs.* directive/nondirective styles, 39–43  
    communicating understanding, 54–60  
    demonstrating understanding, 50–54  
    empathic understanding as basis, 48–50  
    identifying recurrent themes, 60–65  
    performance anxieties, 65  
Working-through, 333–353













